

Auszug aus der UAW-Datenbank des BfArM

Stand der Datenbank: 07.03.2016

Einzelfallausdrucke

Paracetamol: Inland, ab 1992, schwere Hautreaktionen

Hinweise zur Interpretation der Daten:

Es handelt sich bei den Berichten sowohl um Spontanmeldungen als auch um Berichte aus systematisierten Untersuchungen, aus Projekten oder aus der Literatur. Die Meldungen sind dem Bundesinstitut für Arzneimittel und Medizinprodukte durch pharmazeutische Unternehmer, durch die Arzneimittelkommissionen der Heilberufe, direkt durch niedergelassene oder in Krankenhäusern tätige Ärzte sowie durch sonstige Meldequellen, z. B. von Patienten, berichtet worden. Es muss beachtet werden, dass es sich um Verdachtsfälle unerwünschter Arzneimittelwirkungen handelt, dass also ein Kausalzusammenhang im Einzelfall nicht sicher belegt ist. Ferner lassen sich auf der Grundlage von Spontanberichten keine Aussagen darüber machen, wie häufig eine bestimmte unerwünschte Wirkung unter der Anwendung eines Arzneimittels vorkommt, und auch vergleichende Angaben darüber, wie viel mal häufiger eine bestimmte unerwünschte Wirkung bei einem Arzneimittel im Verhältnis zu einem anderen Medikament auftritt, sind anhand solcher Berichte kaum möglich.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-92003793

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 63 Year weight (kg): 106 BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Epidermal necrolysis	19911124				Unknown
2 Stevens Johnson syndrome	19911124				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
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DE-BFARM-92003793

1	acetylcysteine acetylcysteine	Oral	003 Df dosage form/Day	19911113-19911129	Cough
2	adalat nifedipine	Oral	003 Df dosage form/Day	-	Essential hypertension, unspecified
3	adumbran oxazepam	Oral	001 Df dosage form/Day	-	Sleep disturbances
4	atosil promethazine hydrochloride	Oral	020 Gtt drop(s)/Day	19911113-19911121	Other and unspecified special symptoms or syndromes, not
5	catapresan 300 clonidine	Oral	003 Df dosage form/Day	-	Essential hypertension, unspecified
6	cedur bezafibrate	Oral	001 Df dosage form/Day	19911011-19911012	Other and unspecified hyperlipidaemia
7	diazepam diazepam	Oral	009 Df dosage form/Day	19911119-19911129	Other general symptoms
8	dulcolax bisacodyl	Oral	1 Df dosage form/	19911117-19911117	Constipation
9	heparin heparin	Subcutaneous	015 Kiu iu (1000s)/Day	19911111-19911129	Thrombosis
10	insulin insulin	Subcutaneous	080 lu international unit(s)	-	Diabetes mellitus
11	kalinor-brausetabletten potassium carbonate; potassium citrate	Oral	001 Df dosage form/Day	19911113-19911129	Potassium deficiency
12	lidocaine lidocaine	Intramuscular	001 Df dosage form/Day	19911001-19911111	Backache, unspecified
13	lopirin cor captopril	Oral	001 Df dosage form/Day	19911117-19911129	Heart failure, unspecified
14	moduretik amiloride hydrochloride; hydrochlorothiazide	Oral	001 Df dosage form/Day	-	Essential hypertension, unspecified
15	novodigal beta-acetyldigoxin	Oral	0.2 Mg milligram(s)/Day	-	Heart failure, unspecified
16	paracetamol paracetamol	Unknown	/	19911114-	Other general symptoms
17	penicillin benzylpenicillin sodium	Intravenous (not otherwis	003 Df dosage form/Day	19911111-19911129	Erysipelas
18	prednisolon prednisolone	Intramuscular	001 Df dosage form/Day	19911001-19911111	Backache, unspecified
19	psyquil triflupromazine	Rectal	1 Df dosage form/	19911121-19911121	Other general symptoms
20	pulmidur forte theophylline	Oral	002 Df dosage form/Day	19911113-19911129	Asthma, unspecified
21	solosin theophylline	Oral	001 Df dosage form/Day	19911113-19911129	Asthma, unspecified
22	zyloric 300 allopurinol	Oral	001 Df dosage form/Day	19911113-19911129	Hyperuricaemia

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Elephantiasis			k.A.	concomitant disease

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-92900174

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 23 Year weight (kg): 69 BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19911028				recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 aequamen betahistine mesilate	Oral	002 Df dosage form/Day	19911003-19911023	Difficulty in walking
2 aspirin acetylsalicylic acid	Oral	001 Df dosage form/Day	19911023-	Pyrexia of unknown origin
3 ben-u-ron paracetamol	Oral	001 Df dosage form/Day	19911023-	Pyrexia of unknown origin
4 claforan cefotaxime sodium	Intravenous (not otherwis	002 G gram(s)/Day	19911024-19911029	Pneumonia, organism unspecified
5 daraprim pyrimethamine	Oral	100 Mg milligram(s)/Day	19911003-19911031	Unspecified infectious and parasitic diseases
6 durenat sulfametoxydiazine	Oral	001 Df dosage form/Day	19911003-19911031	Unspecified infectious and parasitic diseases
7 gutron midodrine hydrochloride	Oral	002 Df dosage form/Day	19911024-19911029	Hypotension, unspecified
8 leucovorin folinic acid	Oral	001 Df dosage form/Day	19911003-19911031	Other B-complex deficiencies
9 tegretal 400 retard carbamazepine	Oral	800 Mg milligram(s)/Day	19911003-19911031	Epilepsy, unspecified
10 valdispert valerian extract	Oral	003 Df dosage form/Day	19911003-19911031	Sleep disturbances

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Toxoplasmosis, unspecified			k.A.	concomitant disease
Other causes of encephalitis			k.A.	concomitant disease
HIV infection NOS			k.A.	concomitant disease

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-92901282

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes					

Patient

Initials:	Sex: Male	Birth: privacy	Age: 33 Year	weight (kg): 82	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19910502				recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 aspirin acetylsalicylic acid	Oral	/	19910502-	Pyrexia of unknown origin
2 baypen mezlocillin sodium	Intravenous (not otherwis	/	19910501-19910502	Pyrexia of unknown origin
3 ben-u-ron paracetamol	Unknown	/	19910502-	Pyrexia of unknown origin
4 cebion ascorbic acid	Oral	/	199104-	Unspecified immunity deficiency
5 rulid roxithromycin	Oral	/	19910502-	Pyrexia of unknown origin

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-92003685

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.
 Yes Yes

Patient

Initials: Sex: Male Birth: privacy Age: 51 Year weight (kg): 75 BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19920310		3 Day		recovered/resolved
2 Epidermal necrolysis	19920310		3 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ass acetylsalicylic acid	Oral	/	-	Influenza with other respiratory manifestations
2 nasivinetten gegen schnupfen 0,05% oxymetazoline hydrochloride	Nasal	/	-	
3 neurocil levomepromazine maleate	Oral	250 Mg milligram(s)/Day	-	Paranoid state, simple
4 paracetamol paracetamol	Oral	/	-	
5 pepdul famotidine	Oral	001 Df dosage form/Day	-	
6 tavor lorazepam	Oral	004 Mg milligram(s)/Day	-	

Drugs (concomitant)

7 fluanxol flupentixol dihydrochloride	Intramuscular	80 Mg milligram(s)/Month	-	Mild mental retardation
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Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Chronic bronchitis, unspecified			k.A.	concomitant disease

Primary source

Qualification: Physician observestudytype:
 Literature:

FIRST LOOK

Country: Deutschland

Type: Report from study

case: DE-BFARM-92006324

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.
 Yes Yes

Patient

Initials: Sex: Male Birth: privacy Age: 11 Month weight (kg): 7 BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Epidermal necrolysis	19850222				recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 luminaletten phenobarbital	Oral	/	198502-198502 -	Unspecified psychophysiological
2 novalgin metamizole	Oral	/	19850217-19850228 -	Influenza with other respiratory manifestations
3 paracetamol paracetamol	Rectal	/	19850217- -	Influenza with other respiratory manifestations

Drugs (concomitant)

4 erythromycin erythromycin	Unknown	/	-	
5 josamycin josamycin	Unknown	/	-	
6 lanitop metildigoxin	Oral	2.5 Df dosage form/Day	-19850302	Ventricular septal defect
7 wilprafen josamycin	Unknown	/	198502-198502 -	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Acute upper respiratory infections of unspecified site			k.A.	concomitant disease
Pneumonia, organism unspecified			k.A.	concomitant disease
Primary pulmonary hypertension			k.A.	concomitant disease

Primary source

Qualification: Physician observestudytype:
 Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-92008029

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No						

Patient

Initials:	Sex: Male	Birth: privacy	Age: 10 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19910529				recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ben-u-ron paracetamol	Oral	4 Df dosage form/	19910522-19910528	Acute nasopharyngitis (common cold)
2 bidocef saft cefadroxil	Oral	001 Df dosage form/Day	19910528-19910529	Acute nasopharyngitis (common cold)
3 bronchopront tropfen ambroxol hydrochloride	Oral	/	19910528-19910530	Cough
4 otriven xylometazoline hydrochloride	Nasal	/	19910528-19910530	Chronic rhinitis

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-92008035

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No						

Patient

Initials:	Sex: Male	Birth: privacy	Age: 16 Year	weight (kg): 70	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19910525		3 Week		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 amoxicillin amoxicillin	Oral	/	19910525-19910528	Unspecified infectious and parasitic diseases
2 diazepam diazepam	Rectal	1 Df dosage form/	-	-
3 paracetamol paracetamol	Rectal	/	-	Pyrexia of unknown origin

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Unspecified diseases due to mycobacteria			k.A.	concomitant disease

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-92008038

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.
 Yes Yes

Patient

Initials: Sex: Male Birth: privacy Age: 9 Year weight (kg): 34 BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Epidermal necrolysis	19910221		1 Month		recovered/resolved with sequel

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ben-u-ron paracetamol	Rectal	/	-	Acute nasopharyngitis (common cold)
2 mucosolvan ambroxol hydrochloride	Oral	/	-	Acute nasopharyngitis (common cold)
3 otriven xylometazoline hydrochloride	Nasal	/	-	Acute nasopharyngitis (common cold)
4 paracetamol paracetamol	Rectal	/	-	Acute nasopharyngitis (common cold)
5 rhinotussal carbinoxamine maleate; dextromethorphan hydrobromide; phenylephrine hydrochloride	Oral	/	-	Acute nasopharyngitis (common cold)
6 soledum cineole	Oral	/	-	Acute nasopharyngitis (common cold)
7 vivimed bucetin; caffeine; phenacetin; propyphenazone; sulfamidopyrine sodium; thiamine mononitrate	Oral	/	-	Acute nasopharyngitis (common cold)

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-92008041

Serious Yes	Death Yes	Lifethr. Yes	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Female	Birth: privacy	Age: 69 Year	weight (kg): 49	BMI_int:	Age fetus:
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Patientdeath

deathdate: 199102	deathcause: Cardiac arrest
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19910131				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 adumbran oxazepam	Oral	/	19901228-19901228	-
2 agarol paraffin, liquid; phenolphthalein	Oral	/	19910103-19910103	Unspecified functional disorder of stomach
3 aldactone spironolactone	Oral	/	-	Oedema
4 alimix cisapride monohydrate	Oral	/	19910111-19910131	-
5 antra omeprazole	Oral	/	-19901226	-
6 bronchicum ammonia; cimicifuga racemosa root; cortex quebracho; ephedrine hydrochloride; eucalyptus oil; grindelia; menthol; pimpinella root; sodium bromide; thyme; thyme extract	Oral	/	-19901228	Bronchitis, not specified as acute or chronic
7 dulcolax bisacodyl	Rectal	/	19910103-19910103	-
8 fluimucil acetylcysteine	Oral	/	19910101-19910111	Bronchitis, not specified as acute or chronic
9 ismo 20 isosorbide mononitrate	Oral	/	-	Coronary atherosclerosis
10 lactulose lactulose	Oral	/	-	Cirrhosis of liver without mention of alcohol
11 lasix furosemide	Oral	/	-19910110	Oedema
12 paracetamol paracetamol	Oral	/	-19901229	-
13 paracodin dihydrocodeine bitartrate	Oral	/	19910102-19910113	Bronchitis, not specified as acute or chronic
14 silomat clobutinol hydrochloride	Oral	/	19901229-19901230	Bronchitis, not specified as acute or chronic
15 ulcogant sucralfate	Oral	/	19910105-19910112	-
16 valdispert valerian extract	Oral	/	-19910113	-

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Malignant neoplasm of female breast, unspecified			k.A.	concomitant disease
Other malignant neoplasm of unspecified site			k.A.	concomitant disease

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-92008047

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 44 Year weight (kg): 49 BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19910513				Unknown
2 Epidermal necrolysis	19910513				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 acetylcysteine acetylcysteine	Oral	/	19910430-19910515 -	Bronchitis, not specified as acute or chronic
2 ambroxol ambroxol	Oral	/	19910430-19910514 -	Bronchitis, not specified as acute or chronic
3 calcium calcium	Oral	/	-19910423 -	Disorders of calcium metabolism
4 heparin heparin	Subcutaneous	/	19910511- -	Thrombosis
5 kalinor-brausetabletten potassium carbonate; potassium citrate	Oral	/	-19910423 -	Hypokalaemia
6 lactulose lactulose	Oral	/	- -	
7 paracetamol paracetamol	Rectal	/	- -	Pyrexia of unknown origin
8 pipril piperacillin sodium	Intravenous (not otherwis /		-19910419 -	Pyrexia of unknown origin
9 rocephin ceftriaxone sodium	Intravenous (not otherwis /		19910429-19910507 -	Pyrexia of unknown origin
10 staphylex flucloxacillin sodium	Intravenous (not otherwis /		-19910419 -	Pyrexia of unknown origin
11 targocid teicoplanin	Intravenous (not otherwis /		19910429-19910507 -	Pyrexia of unknown origin

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-92008052

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes	Yes						

Patient

Initials:	Sex: Female	Birth: privacy	Age: 69 Year	weight (kg):	BMI_int:	Age fetus:
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Patientdeath

deathdate:	deathcause: Erythema multiforme
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19901024		1 Week		Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
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1	acetylcysteine acetylcysteine	Unknown	/	19900928-19901005 -
2	aldactone spironolactone	Unknown	/	19901005-19901025 Oliguria -
3	alt-insulin insulin	Subcutaneous	/	-19901005 Diabetes mellitus without - mention of complication
4	atosil promethazine hydrochloride	Oral	/	19901004-19901004 -
5	augmentan amoxicillin trihydrate; clavulanic acid	Unknown	/	19900928-19901003 Pyrexia of unknown origin -
6	ciprobay ciprofloxacin hydrochloride	Unknown	/	19900928-19901003 Pyrexia of unknown origin -
7	depot-h-insulin "hoechst" insulin human	Subcutaneous	/	19901010-19901025 Diabetes mellitus without - mention of complication
8	digimerck digitoxin	Oral	/	- Heart failure, unspecified -
9	dipiperon pipamperone	Unknown	/	19901003-19901003 -
10	dobutrex dobutamine hydrochloride	Intravenous (not otherwis	/	-19901004 Unspecified circulatory system - disorder
11	dominal prothipendyl hydrochloride	Oral	/	19901005-19901009 -
12	dopamin dopamine hydrochloride	Intravenous (not otherwis	/	-19901004 Unspecified circulatory system - disorder
13	dulcolax bisacodyl	Oral	/	19901012-19901012 Constipation -
14	euglucon n glibenclamide	Oral	/	19901005-19901025 Diabetes mellitus without - mention of complication
15	eunerpan melperone hydrochloride	Intravenous (not otherwis	/	19901001-19901005 -
16	fentanyl fentanyl	Intravenous (not otherwis	/	-19900929 -
17	furosemid furosemide	Unknown	030 Mg milligram(s)/Day	19901005-19901025 Oliguria -
18	imodium loperamide hydrochloride	Oral	/	19901012-19901012 Other noninfective - gastroenteritis and colitis
19	kalinor-brausetabletten potassium carbonate; potassium citrate	Oral	/	19901005-19901009 Hypokalaemia -
20	koffein caffeine	Oral	/	19901005-19901009 -
21	lasix furosemide	Intravenous (not otherwis	/	-19901005 Oliguria -
22	liquemin heparin	Subcutaneous	/	- Thrombosis -
23	metoclopramide metoclopramide	Oral	/	19901012-19901012 -
24	paracetamol paracetamol	Rectal	/	19901002-19901002 Pyrexia of unknown origin -
25	protactyl promazine hydrochloride	Intravenous (not otherwis	/	19900928-19901001 -
26	rohypnol flunitrazepam	Intravenous (not otherwis	/	-19900928 -
27	solu-decortin-h prednisolone sodium succinate	Intravenous (not otherwis	/	-19900928 -
28	truxal chlorprothixene hydrochloride	Oral	/	-19900928 -

29	ulcogant sucralfate	Oral	/	-19901016 -	
30	valium diazepam	Intravenous (not otherwis	/	19901004-19901004 -	
31	zyloric allopurinol	Oral	/	-19900930 -	Hyperuricaemia

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-92008065

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No						

Patient

Initials:	Sex: Male	Birth: privacy	Age: 24 Year	weight (kg): 84	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19911007		2 Week		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 acetylcysteine acetylcysteine	Oral	500 Mg milligram(s)/Day	19910910-19911007	Chronic sinusitis, unspecified
2 doxy-wolff 100 doxycycline hyclate	Oral	2 Df dosage form/	19911007-19911007	Chronic sinusitis, unspecified
3 floxal augentropfen ofloxacin	Ophthalmic	/	19911007-19911007	Other and unspecified conjunctivitis
4 imurek azathioprine	Oral	050 Mg milligram(s)/Day	-	Congenital factor VIII disorder
5 milgamma benfotiamine; cyanocobalamin	Oral	003 Df dosage form/Day	-19910916	Mononeuritis of unspecified site
6 paracetamol paracetamol	Oral	001 Df dosage form/Day	19910930-19910930	Headache
7 therapin ascorbic acid; cetylpyridinium chloride	Oral	004 Df dosage form/Day	-	Throat pain
8 vitamin c ascorbic acid	Oral	003 G gram(s)/Day	-19911007	

Drugs (concomitant)

9 *multivitamine	Oral	001 Df dosage form/Day	-19911004
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Primary source

Qualification: Physician observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-92008066

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No						

Patient

Initials:	Sex: Male	Birth: privacy	Age: 3 Year	weight (kg): 15	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19911009		2 Week		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 mono praecimed paracetamol	Oral	/	19911007-19911007	Pyrexia of unknown origin
2 monomycin-saft erythromycin ethylsuccinate	Oral	/	19911007-19911009	Enlargement of lymph nodes

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-93004929

Serious Yes	Death	Lifethr. No	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Male	Birth: privacy	Age: 14 Year	weight (kg): 65	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19920131				recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ben-u-ron paracetamol	Oral	/	19920126-	-
2 codipront codeine; phenyltoloxamine	Oral	/	19920126-	-
3 erythromycin erythromycin	Oral	/	19920129-	-
4 fluimucil acetylcysteine	Oral	/	19920126-	-
5 optipect tropfen aminophenazone; ammonia; ammonium chloride; camphor; ephedrine hydrochloride; eucalyptus oil; eucalyptus tincture; liquorice liquid extract; menthol; peppermint oil; primula tincture; salvia tincture	Oral	/	19920126-	-
6 otriven xylometazoline hydrochloride	Nasal	/	19920126-	-
7 transpulmin-balsam camphor; chamomile; cineole; menthol; salvia	Topical	/	19920126-	-

Primary source

Qualification: Physician observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-93004930

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.
 Yes No

Patient

Initials: Sex: Male Birth: privacy Age: 13 Year weight (kg): 53 BMI_int: 19.23 Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19910430		1 Month		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ben-u-ron paracetamol	Oral	/	19910424- -	Pyrexia of unknown origin
2 hexoral hexetidine	Buccal	/	19910424- -	Throat pain
3 hustentee	Oral	/	19910424- -	Cough

Drugs (concomitant)

4 baycillin propicillin potassium	Oral	/	19910430- -	Pyrexia of unknown origin
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Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Stomatitis			k.A.	concomitant disease

Primary source

Qualification: Physician observestudytype:
 Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-93004891

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes	Yes	Yes					

Patient

Initials:	Sex: Female	Birth: privacy	Age: 77 Year	weight (kg): 62	BMI_int:	Age fetus:
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Patientdeath

deathdate: 19920704	deathcause: Heart failure, unspecified
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19920616				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 anti-phosphat algedrate	Oral	/	-	Renal failure, unspecified
2 depuran anise oil; carum carvi; senna fruit extract	Oral	/	-	Constipation
3 diclofenac diclofenac	Oral	/	19920613-19920613	Bronchitis, not specified as acute or chronic
4 isoket retard 40 isosorbide dinitrate	Oral	/	-	Hypotension, unspecified
5 lactulose lactulose	Oral	/	-	Constipation
6 paracetamol paracetamol	Oral	/	-	Other general symptoms
7 rulid roxithromycin	Oral	/	19920613-19920613	Bronchitis, not specified as acute or chronic

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Unspecified adverse effect of drug, medicament and biologi			k.A.	concomitant disease
unbekannt			k.A.	concomitant disease: z.n.lyell-syndrom
Coronary atherosclerosis			k.A.	concomitant disease
Unspecified adverse effect of drug, medicament and biologi			k.A.	concomitant disease

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-93004899

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes					

Patient

Initials:	Sex: Female	Birth: privacy	Age: 67 Year	weight (kg): 57	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19920619				recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 carnigen oxilofrine hydrochloride	Oral	060 Gtt drop(s)/Day	19920601-	Hypotension, unspecified
2 claforan cefotaxime sodium	Intravenous (not otherwis	004 G gram(s)/Day	19920601-19920606	Unspecified cause of encephalitis
3 effortil etilefrine hydrochloride	Oral	060 Gtt drop(s)/Day	19920601-	Hypotension, unspecified
4 gentamicin gentamicin	Intravenous (not otherwis	160 Mg milligram(s)/Day	19920601-19920604	Unspecified cause of encephalitis
5 imurek azathioprine	Oral	002 Df dosage form/Day	-	Sicca syndrome
6 lecicarbon lecithin; sodium bicarbonate; sodium phosphate monobasic (anhydrate)	Rectal	/	19920602-19920602	Constipation
7 musaril tetrazepam	Oral	004 Df dosage form/Day	-	Hypermobility syndrome
8 paracetamol paracetamol	Rectal	/	19920601-19920615	Other general symptoms
9 pepdul famotidine	Oral	001 Df dosage form/Day	-	Unspecified functional disorder of stomach
10 ultralan fluocortolone; fluocortolone caproate	Oral	001 Df dosage form/Day	-	Sicca syndrome

Drugs (concomitant)

11 tavegil clemastine	Oral	1 Df dosage form/	19920619-19920619	Rash and other nonspecific skin eruption
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Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Unspecified diffuse connective tissue disease			k.A.	concomitant disease
Lumbago			k.A.	concomitant disease

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-93004907

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		Yes	Yes					

Patient

Initials:	Sex: Female	Birth: privacy	Age: 30 Year	weight (kg): 50	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Epidermal necrolysis	19920921				Unknown
2 Stevens Johnson syndrome	19920921				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ben-u-ron paracetamol	Rectal	001 Df dosage form/Day	19920912-19920913	
2 daraprim pyrimethamine	Oral	050 Mg milligram(s)/Day	-	
3 diflucan fluconazole	Oral	2 Df dosage form/Week	-	Candidiasis of unspecified site
4 ferro sanol ferroglycine sulfate complex	Oral	001 Df dosage form/Day	19920909-19920919	
5 fortecortin dexamethasone	Oral	012 Df dosage form/Day	-19920913	
6 leucovorin folinic acid	Oral	/	-19920919	
7 maaloxan aluminium hydroxide; aluminium hydroxide gel; magnesium hydroxide	Oral	003 Df dosage form/Day	-19920913	Chronic ulcer of skin, unspecified site
8 paracetamol paracetamol	Oral	001 Df dosage form/Day	19920910-19920912	
9 paspertin metoclopramide hydrochloride	Unknown	1 Df dosage form/	19920913-19920913	
10 pentacarinat pentamidine isethionate	Oral	300 Mg milligram(s)/	19920907-19920907	Rheumatoid arthritis
11 phenhydan phenytoin	Oral	003 Df dosage form/Day	19920910-	
12 rescuvinol calcium folinate	Oral	005 Mg milligram(s)/Day	19920920-	
13 rovamycin spiramycin	Oral	1.5 G gram(s)/Day	-	Toxoplasmosis, unspecified
14 sobelin clindamycin hydrochloride	Oral	900 Mg milligram(s)/Day	-19920909	
15 sultanol salbutamol	Respiratory (inhalation)	/	19920907-19920907	
16 tavegil clemastine	Oral	/	-19920909	Rash and other nonspecific skin eruption
17 zovirax aciclovir	Intravenous (not otherwis	1.5 G gram(s)/Day	19920907-19920909	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Epilepsy, unspecified			k.A.	concomitant disease
AIDS related complex			k.A.	concomitant disease

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-93011028

Serious Yes	Death	Lifethr. No	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Male	Birth: privacy	Age: 14 Year	weight (kg): 65	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19930526				not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 doxy-n-tablinen doxycycline	Oral	004 Df dosage form/Day	19930524-19930528	Unspecified infectious and parasitic diseases
2 paracetamol paracetamol	Oral	002 Df dosage form/Day	19930520-19930522	Unspecified infectious and parasitic diseases

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Chronic rhinitis			k.A.	concomitant disease
Acute bronchitis			k.A.	concomitant disease

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-93011029

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No						

Patient

Initials:	Sex: Male	Birth: privacy	Age: 30 Year	weight (kg): 80	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19930214				not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 paracetamol paracetamol	Oral	/	19930211- -	Bronchitis, not specified as acute or chronic
2 p-mega-tablinen phenoxymethylpenicillin potassium	Oral	003 Df dosage form/Day	19930211-19930215 -	Influenza with other respiratory manifestations
3 thomapyrin acetylsalicylic acid; caffeine; paracetamol	Oral	1 Df dosage form/	19930211- -	Influenza with other respiratory manifestations
4 vitamin c ascorbic acid	Oral	/	19930126- -	

Drugs (concomitant)

5 *expectorantia	Intravenous (not otherwis /		19930214-19930214 -	Influenza with other respiratory manifestations
6 muco panoral bromhexine hydrochloride; cefaclor	Oral	2 Df dosage form/	-19930215 -	Cough
7 mucophlogat ambroxol hydrochloride	Oral	2 Df dosage form/	19930214-19930214 -	Influenza with other respiratory manifestations

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Bacterial infection, unspecified, in conditions classified else			k.A.	concomitant disease

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-93900822

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		Yes	Yes					

Patient

Initials:	Sex: Male	Birth: privacy	Age: 16 Year	weight (kg): 85	BMI_int: 26.23	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Epidermal necrolysis	19930902				not recovered/not resolved
2 Stevens Johnson syndrome	19930902				Unknown
3 Rash	19930902				not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diazepam diazepam	Rectal	010 Mg milligram(s)/Day	-19930812	
2 doxycycline doxycycline	Oral	010 Mg milligram(s)/Day	19930808-19930812	Pneumonia, organism unspecified
3 lamictal lamotrigine	Oral	200 Mg milligram(s)/Day	19930816-19930902	Epilepsy, unspecified
4 leptilan valproate sodium	Oral	100 Mg milligram(s)/Day	19930630-19930905	
5 liskantin primidone	Oral	750 Mg milligram(s)/Day	19930630-19930905	Other forms of epilepsy
6 paracetamol paracetamol	Oral	/	19930831-19930901	
7 rivotril clonazepam	Oral	006 Mg milligram(s)/Day	19930604-19930905	Acute, but ill-defined, cerebrovascular disease

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Unspecified infective arthritis			k.A.	concomitant disease
Pyelonephritis or pyonephrosis, not specified as acute or ch			k.A.	concomitant disease
Infantile hemiplegia			k.A.	concomitant disease
Meningitis due to other nonbacterial organisms classified el			k.A.	concomitant disease
Cerebral degeneration, unspecified			k.A.	concomitant disease
Unspecified mental retardation			k.A.	concomitant disease

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-93011099

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No						

Patient

Initials:	Sex: Male	Birth: privacy	Age: 6 Year	weight (kg): 16	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Erythema multiforme	19921212		14 Day		recovered/resolved
2 Erythema exudativum	19921212		14 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ben-u-ron paracetamol	Rectal	/	19921208-19921208	-
2 berotec fenoterol hydrobromide	Respiratory (inhalation)	4 Df dosage form/	19921209-19921209	-
3 erythrocin erythromycin	Oral	/	19921209-19921210	-
4 monomycin-saft erythromycin ethylsuccinate	Oral	/	19921210-	-
5 muco panoral bromhexine hydrochloride; cefaclor	Oral	/	19921207-19921208	-
6 ozothin terpin hydrate; turpentine oil	Intravenous (not otherwis /		19921209-	-
7 spizef cefotiam hydrochloride	Intravenous (not otherwis /		19921208-	-
8 vomex a dimenhydrinate	Oral	/	19921207-19921208	-

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Acute nasopharyngitis (common cold)			k.A.	concomitant disease
Nausea and vomiting			k.A.	concomitant disease
Pneumonia due to other specified organism			k.A.	concomitant disease
Nausea and vomiting			k.A.	concomitant disease

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-93012754

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No						

Patient

Initials:	Sex: Male	Birth: privacy	Age: 27 Year	weight (kg): 53	BMI_int: 18.34	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Erythema exudativum	19930225				recovered/resolved
2 Erythema multiforme	19930225				recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ben-u-ron paracetamol	Rectal	001 Df dosage form/Day	19930221-19930226 6 Day	
2 hustentropfen	Oral	/	19930208-19930225 18 Day	Cough
3 japanisches heilpflanzenoel peppermint oil	Oral	/	19930208- -	
4 spalt n caffeine; paracetamol; phenazone	Oral	/	19930208- -	Headache
5 wick formel 44 plus dextromethorphan hydrobromide; paracetamol	Oral	/	19930218-19930226 9 Day	

Drugs (concomitant)

6 candio-hermal nystatin	Oral	003 Df dosage form/Day	199302- -	Stomatitis
7 doxycycline doxycycline	Oral	002 Df dosage form/Day	199302- -	
8 spersallerg antazoline hydrochloride; tetrazoline hydrochloride	Ophthalmic	001 Df dosage form/Day	199302- -	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-94001641

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		Yes	Yes					

Patient

Initials:	Sex: Male	Birth: privacy	Age: 4 Year	weight (kg): 23	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19930305		3 Week		recovered/resolved
2 Epidermal necrolysis	19930305		3 Week		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 captin zaepfchen 500 mg paracetamol	Rectal	1.2 G gram(s)/Day	19930223-19930228	Pyrexia of unknown origin
2 erythromycin erythromycin	Oral	/	19930227-	Acute upper respiratory infections of other multiple sites
3 leptilan valproate sodium	Oral	003 Df dosage form/Day	-	Pyrexia of unknown origin
4 rulofer ascorbic acid; ferrous fumarate	Oral	003 Df dosage form/Day	-	Iron deficiency anaemias

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-94001642

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes					

Patient

Initials:	Sex: Male	Birth: privacy	Age: 11 Year	weight (kg): 29	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19920825				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 arnika arnica tincture	Oral	/Day	19920803-	
2 dantamacin dantrolene sodium	Intravenous (not otherwis	/Day	19920805-	Abnormal involuntary movements
3 dormicum midazolam maleate	Oral	/Day	-19920806	
4 fentanyl fentanyl	Intravenous (not otherwis	/Day	-19920806	
5 lioresal baclofen	Intravenous (not otherwis	/Day	19920808-	Abnormal involuntary movements
6 mucosolvan ambroxol hydrochloride	Oral	/Day	19920731-	Cough
7 paracetamol paracetamol	Rectal	500 Mg milligram(s)/	-	Other general symptoms
8 phenobarbital phenobarbital	Oral	/Day	-	Epilepsy, unspecified
9 pipril piperacillin sodium	Oral	/Day	-19920731	Unspecified infectious and parasitic diseases
10 sterinor sulfadiazine; tetroxoprim	Intravenous (not otherwis	/Day	19920812-19920820	Urinary tract infection, site not specified
11 valium diazepam	Rectal	5 Mg milligram(s)/	19920731-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Traumatic shock			k.A.	concomitant disease
Intracranial injury of other and unspecified nature without m			k.A.	concomitant disease

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Report from study

case: DE-BFARM-94900218

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes					

Patient

Initials:	Sex: Male	Birth: privacy	Age: 13 Year	weight (kg): 38	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Erythema multiforme	19930312		19 Day		recovered/resolved
2 Erythema exudativum	19930312		19 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 codipront codeine; phenyltoloxamine	Oral	/	19930310-19930314	Influenza with other respiratory manifestations
2 kepinol sulfamethoxazole; trimethoprim	Oral	/	19930310-19930314	Influenza with other respiratory manifestations
3 panoral cefaclor	Oral	/	19930312-19930314	Pneumonia, organism unspecified
4 paracetamol paracetamol	Oral	/	19930310-19930314	Influenza with other respiratory manifestations

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Lichenification and lichen simplex chronicus			k.A.	concomitant disease
Other atopic dermatitis and related conditions			k.A.	concomitant disease

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-94003570

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 1 Year weight (kg): 9.3 BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19940122				recovered/resolved
2 Blepharitis	19940122				recovered/resolved
3 Stomatitis	19940122				recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 acetylcysteine acetylcysteine	Respiratory (inhalation)	006 ml millilitre(s)/Day	19940116-19940125	Bronchitis, not specified as acute or chronic
2 amoxypen amoxicillin trihydrate	Oral	003 Df dosage form/Day	19940116-19940123	Other diseases of respiratory system, not elsewhere
3 aspirin 100 acetylsalicylic acid	Oral	/	-	-
4 ergenyl valproate sodium	Oral	030 Gtt drop(s)/Day	19940117-19940124	Grand mal
5 liskantin primidone	Oral	250 Mg milligram(s)/Day	19931222-19940119	Epilepsy, unspecified
6 novalgin tropfen metamizole sodium	Oral	/	-	-
7 paracetamol paracetamol	Unknown	/	-	-

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Convulsions			k.A.	concomitant disease

Primary source

Qualification: Physician observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-94005683

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes					

Patient

Initials:	Sex: Male	Birth: privacy	Age: 17 Year	weight (kg): 75	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19921219		2 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ben-u-ron paracetamol	Oral	001 Df dosage form/Day	-	Chronic gingivitis
2 ciprobay ciprofloxacin hydrochloride	Oral	002 Df dosage form/Day	-	Chronic gingivitis
3 spalt acetylsalicylic acid	Oral	/	-	Unspecified disorder of the teeth and supporting structures
4 tramal tramadol hydrochloride	Unknown	001 Df dosage form/Day	-	Chronic gingivitis

Drugs (concomitant)

5 clydoval stoffangabe (teilweise) unklar	Oral	/	-	Chronic gingivitis
6 wasserstoffperoxid hydrogen peroxide	Oral	/	-	Chronic gingivitis

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-95003062

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Birth: privacy Age: weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Toxic epidermal necrolysis					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 atosil promethazine hydrochloride	Oral	/	-	-
2 elobact cefuroxime axetil	Oral	/	-	-
3 mucosolvan saft ambroxol hydrochloride	Oral	/	-	-
4 paracetamol paracetamol	Rectal	/	-	-
5 rhinoprnt spray tetryzoline hydrochloride	Respiratory (inhalation)	/	-	-

Primary source

Qualification:

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-95003069

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 4 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Respiratory distress	19930221				Unknown
2 Stevens Johnson syndrome	19930312				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 bactrim sulfamethoxazole; trimethoprim	Intravenous (not otherwis /		-	
2 ben-u-ron paracetamol	Rectal	/	-	
3 claforan cefotaxime sodium	Intravenous (not otherwis /		-	
4 fenistil dimetindene maleate	Oral	/	-	
5 fluimucil acetylcysteine	Intravenous (not otherwis /		-	
6 fortecortin dexamethasone	Intravenous (not otherwis /		-	
7 luminal phenobarbital	Intravenous (not otherwis /		-	
8 moronal nystatin	Oral	/	-	
9 muciteran 100 acetylcysteine	Oral	/	-	
10 phenaemalotten phenobarbital	Intravenous (not otherwis /		-	
11 refobacin gentamicin sulfate	Intravenous (not otherwis /		-	
12 zovirax aciclovir	Intravenous (not otherwis /		-	

Primary source

Qualification:

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-95900369

Serious Yes	Death Yes	Lifethr. No	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Male	Birth: privacy	Age: 63 Year	weight (kg): 74	BMI_int:	Age fetus:
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Patientdeath

deathdate:	deathcause: Other diseases of lung, not elsewhere classified
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Respiratory insufficiency					Unknown
2 Stevens Johnson syndrome	19940311				Unknown
3 Respiratory disorder					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
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1	acc acetylcysteine	Oral	001 Df dosage form/Day	19940304-19940312	Asthma, unspecified
2	arelix piretanide	Oral	/Day	19940303-19940308	Oedema
3	atosil promethazine hydrochloride	Oral	010 Gtt drop(s)/Day	19940304-19940305	Other and unspecified special symptoms or syndromes, not
4	bambec bambuterol hydrochloride	Oral	/Day	19940225-19940316	Asthma, unspecified
5	bayotensin nitrendipine	Oral	001 Df dosage form/Day	19940217-19940302	Essential hypertension, unspecified
6	ben-u-ron paracetamol	Rectal	3*100 Mg milligram(s)/	19940222-19940316	Pyrexia of unknown origin
7	bronchoparat theophylline sodium aminoacetate	Intravenous (not otherwis	/Day	19940217-19940225	Asthma, unspecified
8	bronchoretard theophylline	Oral	/Day	19940225-19940317	Asthma, unspecified
9	bronchospasmin reproterol hydrochloride	Intravenous (not otherwis	/Day	19940217-19940225	Asthma, unspecified
10	ciprobay ciprofloxacin hydrochloride	Intravenous (not otherwis	001 Df dosage form/Day	19940217-19940220	Other diseases of respiratory system, not elsewhere
11	dehydrobenzperidol droperidol	Intravenous (not otherwis	0.5 Df dosage form/Day	19940219-19940221	Other general symptoms
12	dipidolor piritramide	Intravenous (not otherwis	/Day	19940217-19940220	Other general symptoms
13	dormicum midazolam maleate	Intravenous (not otherwis	/Day	19940218-19940224	Other and unspecified special symptoms or syndromes, not
14	eugalac lactulose	Oral	003 Df dosage form/Day	19940217-19940312	Constipation
15	heparin heparin	Subcutaneous	/Day	19940217-19940317	Thrombosis
16	kalinor-brausetabletten potassium carbonate; potassium citrate	Oral	002 Df dosage form/Day	19940304-19940307	
17	ketanest ketamine	Intravenous (not otherwis	/Day	19940217-19940218	Other and unspecified special symptoms or syndromes, not
18	lasix furosemide	Intravenous (not otherwis	002 Df dosage form/Day	19940217-19940225	Oedema
19	lecicarbon lecithin; sodium bicarbonate; sodium phosphate monobasic (anhydrate)	Rectal	2*1 Df dosage form/	19940307-19940311	Constipation
20	mucosolvan ambroxol hydrochloride	Intravenous (not otherwis	/Day	19940217-19940225	Asthma, unspecified
21	propulsin cisapride monohydrate	Oral	030 ml millilitre(s)/Day	19940217-19940302	Other specified functional disorders of intestine
22	sab simplex dimeticone	Oral	020 Gtt drop(s)/Day	19940217-19940222	Bloating
23	sanasthmax dossier-aerosol beclometasone dipropionate	Respiratory (inhalation)	004 Df dosage form/Day	19940304-19940317	Asthma, unspecified
24	solu-decortin prednisolone sodium succinate	Intravenous (not otherwis	100 Mg milligram(s)/Day	19940217-19940226	Asthma, unspecified
25	ulcogant-suspension sucralfate	Oral	005 ml millilitre(s)/Day	19940217-19940225	
26	zienam 500 cilastatin sodium; imipenem	Intravenous (not otherwis	001 Df dosage form/Day	19940218-19940228	Other diseases of respiratory system, not elsewhere

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-95002423

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Birth: privacy Age: weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ben-u-ron paracetamol	Unknown	/	-	-
2 erythromycin erythromycin	Oral	/	-	-

Primary source

Qualification: observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-95900196

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes							

Patient

Initials:	Sex: Female	Birth: privacy	Age: 78 Year	weight (kg):	BMI_int:	Age fetus:
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Patientdeath

deathdate:	deathcause: Other diseases of lung, not elsewhere classified
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Death NOS					Unknown
2 Respiratory disorder					Unknown
3 Stevens Johnson syndrome	19921003				Unknown
4 Toxic epidermal necrolysis					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
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1	actrapid human insulin human	Intravenous (not otherwis /	-
2	arterenol norepinephrine hydrochloride	Intravenous (not otherwis /	-
3	ben-u-ron paracetamol	Oral /	-
4	boro-scopol hyoscine borate	Ophthalmic /	-
5	depuran anise oil; carum carvi; senna fruit extract	Oral /	-
6	dopamin dopamine hydrochloride	Intravenous (not otherwis /	-
7	dormicum midazolam maleate	Intravenous (not otherwis /	-
8	dytide h hydrochlorothiazide; triamterene	Oral /	-
9	fentanyl fentanyl	Intravenous (not otherwis /	-
10	fluimucil acetylcysteine	Intravenous (not otherwis /	-
11	fortecortin mono 40 dexamethasone sodium phosphate	Intravenous (not otherwis /	-
12	gastrozepin pirenzepine dihydrochloride	Intravenous (not otherwis /	-
13	gelonida acetylsalicylic acid; codeine phosphate; paracetamol	Oral /	-
14	heparin heparin	Intravenous (not otherwis /	-
15	hypnomidate etomidate sulfate	Intravenous (not otherwis /	-
16	isoptin verapamil hydrochloride	Intravenous (not otherwis /	-
17	lasix furosemide	Oral /	-
18	mucosolvan ambroxol hydrochloride	Intravenous (not otherwis /	-
19	neurotrat forte dragees cyanocobalamin; pyridoxine hydrochloride; thiamine hydrochloride	Oral /	-
20	norcuron vecuronium bromide	Intravenous (not otherwis /	-
21	novodigal beta-acetyldigoxin	Oral /	-
22	pipril piperacillin sodium	Intravenous (not otherwis /	-
23	rapifen alfentanil hydrochloride	Intravenous (not otherwis /	-
24	refobacin 80 gentamicin sulfate	Intravenous (not otherwis /	-
25	remestan temazepam	Oral /	-
26	staphylex flucloxacillin sodium	Intravenous (not otherwis /	-
27	succinyl suxamethonium chloride	Intravenous (not otherwis /	-
28	vibramycin doxycycline hyclate	Oral /	-

Qualification: Physician
Literature: observestudytype:

observestudytype:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-95901063

Serious Yes	Death Yes	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Female	Birth: privacy	Age: 77 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Leukocytosis	19950519				Unknown
2 Thrombocytopenia	19950519				Unknown
3 Stevens Johnson syndrome	19950519				Unknown
4 Lyell syndrome	19950519				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
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1	antra	Oral	/	-
	omeprazole			-
2	aspisol	Intravenous (not otherwis	/	-
	acetylsalicylate lysine			-
3	bifiteral	Oral	/	-
	lactulose			-
4	calciparin	Subcutaneous	/	-
	heparin calcium			-
5	cebion	Intravenous (not otherwis	/	-
	ascorbic acid			-
6	ciprobay 200	Unknown	/	-
	ciprofloxacin lactate			-
7	dormicum	Intravenous (not otherwis	/	-
	midazolam maleate			-
8	dulcolax	Oral	/	-
	bisacodyl			-
9	dytide h	Oral	/	-
	hydrochlorothiazide; triamterene			-
10	eunerpan	Oral	/	-
	melperone hydrochloride			-
11	gastrosil	Oral	/	-
	metoclopramide hydrochloride			-
12	heparin	Subcutaneous	/	-
	heparin			-
13	normofundin	Intravenous (not otherwis	/	-
	calcium chloride; magnesium chloride anhydrous; potassium chloride; sodium acetate; sodium chloride; sorbitol; xylitol			-
14	paracetamol	Unknown	/	-
	paracetamol			-
15	polybion	Intravenous (not otherwis	/	-
	vitamins nos			-
16	tramal	Oral	/	-
	tramadol hydrochloride			-
17	xylocain	Intravenous (not otherwis	/	-
	lidocaine hydrochloride			-
18	zantic	Intravenous (not otherwis	/	-
	ranitidine hydrochloride			-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-95005913

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
No								

Patient

Initials:	Sex: Female	Birth: privacy	Age: 40 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19931219				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
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1	agarol paraffin, liquid; phenolphthalein	Oral	/	-
2	akrinor injektionsloesung cafedrine hydrochloride; theodrenaline hydrochloride	Intravenous (not otherwis	/	-
3	atropine sulfate atropine sulfate	Intravenous (not otherwis	/	-
4	calcium calcium	Oral	/	-
5	decadron-phosphat 8 mg dexamethasone sodium phosphate	Intravenous (not otherwis	/	-
6	dobutrex dobutamine hydrochloride	Intravenous (not otherwis	/	-
7	dopamin dopamine hydrochloride	Intravenous (not otherwis	/	-
8	dulcolax bisacodyl	Rectal	/	-
9	expafusin calcium chloride; glucose; hetastarch; potassium chloride; sodium chloride; sodium lactate	Intravenous (not otherwis	/	-
10	fentanyl fentanyl	Intravenous (not otherwis	/	-
11	forene isoflurane	Respiratory (inhalation)	/	-
12	fortecortin dexamethasone	Oral	/	-
13	gelonida na acetylsalicylic acid; codeine phosphate; paracetamol	Oral	/	-
14	heparin-natrium heparin sodium	Subcutaneous	/	-
15	hexoral hexetidine	Buccal	/	-
16	imeson nitrazepam	Oral	/	-
17	lactuflor lactulose	Oral	/	-
18	liquidepur anise oil; carum carvi; senna liquid extract	Oral	/	-
19	minirin desmopressin acetate	Nasal	/	-
20	muskel trancopal chlormezanone	Oral	/	-
21	nimotop s nimodipine	Intravenous (not otherwis	/	-
22	onkohaes hetastarch; sodium chloride	Intravenous (not otherwis	/	-
23	optalidon special noc dragees caffeine; dihydroergotamine mesilate; propyphenazone	Oral	/	-
24	optalidon special noc suppositorien dihydroergotamine mesilate; propyphenazone	Rectal	/	-
25	pancuronium pancuronium	Intravenous (not otherwis	/	-
26	paracetamol paracetamol	Rectal	/	-
27	phenhydan phenytoin	Oral	/	-

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-95005914

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 68 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19940110				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 adumbran oxazepam	Oral	/	-	-
2 ben-u-ron paracetamol	Oral	/	-	-
3 clexane 20 enoxaparin	Subcutaneous	/	-	-
4 cytotec 200 misoprostol	Oral	/	-	-
5 endoxan cyclophosphamide	Oral	/	-	-
6 fluimucil granulat acetylcysteine	Oral	/	-	-
7 gallo-merz cellulase; dehydrocholic acid; dimeticone; pancreatin; turmeric	Oral	/	-	-
8 harzol sitosterols	Oral	/	-	-
9 mucosolvan tropfen ambroxol hydrochloride	Oral	/	-	-
10 muskel trancopal chlormezanone	Oral	/	-	-
11 rantudil forte acemetacin	Oral	/	-	-
12 rohypnol flunitrazepam	Oral	/	-	-
13 strogen uno sabal extract	Oral	/	-	-
14 ultralan-oral 5 fluocortolone	Oral	/	-	-
15 valoron tilidine hydrochloride	Oral	/	-	-
16 voltaren 50 diclofenac sodium	Oral	/	-	-
17 zantic ranitidine hydrochloride	Oral	/	-	-
18 zinacef cefuroxime	Intravenous (not otherwis /		-	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-95006170

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 53 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19950918				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 clin-sanorania 300 clindamycin hydrochloride	Oral	/	-	-
2 paracetamol paracetamol	Unknown	/	-	-

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-96005159

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 10 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Erythema exudativum	19930807				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ben-u-ron paracetamol	Rectal	/	-	
2 durapenicillin phenoxymethylpenicillin potassium	Oral	/	-	
3 erythromycin erythromycin	Oral	/	-	

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-96008601

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 48 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Erythema multiforme	19960403				Unknown
2 Erythema exudativum	19960403				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 aspro acetylsalicylic acid	Oral	/	-	-
2 paracetamol paracetamol	Oral	/	-	-

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-96009330

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Birth: privacy Age: 10 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	1992	2012	1		Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ben-u-ron paracetamol	Oral	/	-	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-96009331

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Birth: privacy Age: 10 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19920906				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 aspirin 100 acetylsalicylic acid	Oral	/	-	-
2 ben-u-ron paracetamol	Rectal	/	-	-

Primary source

Qualification: Physician observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-96009333

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 87 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19931031				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Acetylsalizylsaeure acetylsalicylic acid	Oral	/	-	-
2 ben-u-ron paracetamol	Rectal	/	-	-
3 calciparin heparin calcium	Subcutaneous	/	-	-
4 digotab acetyldigoxin	Oral	/	-	-
5 dilzem retard diltiazem hydrochloride	Oral	/	-	-
6 isoket isosorbide dinitrate	Oral	/	-	-
7 mono mack isosorbide mononitrate	Oral	/	-	-
8 novodigal beta-acetyldigoxin	Oral	/	-	-
9 novodigal mite beta-acetyldigoxin	Oral	/	-	-
10 supracombin forte sulfamethoxazole; trimethoprim	Oral	/	-	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-96009335

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Birth: privacy Age: 9 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19910311				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 esberitox apis mellifica; baptisia; crotalus; echinacea spp; lachesis; silicon dioxide, colloidal; thuja	Oral	/	-	-
2 intal aerosol cromoglicate sodium	Respiratory (inhalation)	/	-	-
3 paracetamol paracetamol	Rectal	/	-	-
4 sultanol salbutamol	Respiratory (inhalation)	/	-	-

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-96018835

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes							

Patient

Initials:	Sex: Female	Birth: privacy	Age: 34 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Pancreatitis acute	19960711				Unknown
2 Stevens Johnson syndrome	19960711				Unknown
3 Toxic epidermal necrolysis	19960711				Unknown
4 Gamma globulin decreased	19960711				Unknown
5 Hepatitis cholestatic	19960711				Unknown
6 Shock lung	19960711				Unknown
7 Corneal perforation	19960711				Unknown
8 Pancytopenia	19960711				Unknown
9 Oedema pulmonary	19960711				Unknown
10 Urinary tract infection	19960711				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 buscopan	Oral	/	-	
hyoscine butylbromide			-	
2 diazepam	Oral	/	-	
diazepam			-	
3 doxycycline	Oral	/	-	
doxycycline			-	
4 korodin	Oral	/	-	
camphor; convallaria tincture; crataegus extract; menthol; valerian tincture			-	
5 kytta-balsam f	Topical	/	-	
methyl nicotinate; symphytum officinale			-	
6 muskel trancopal	Oral	/	-	
chlormezanone			-	
7 paracetamol	Oral	/	-	
paracetamol			-	
8 yxin	Ophthalmic	/	-	
tetryzoline hydrochloride			-	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Report from study

case: DE-BFARM-96019757

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 78 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19960727				Unknown
2 Toxic epidermal necrolysis	19960727				Unknown
3 Coagulation disorder	199607				Unknown
4 Anaemia	199607				Unknown
5 Rash bullous	19960727				Unknown
6 Vesiculobullous rash	19960725				Unknown
7 Hypokalaemia	199607				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
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1	Acetylsalizylsaeure acetylsalicylic acid	Oral	/	-
2	augmentan i.v. 2,2 g amoxicillin sodium; clavulanate potassium	Intravenous (not otherwis	/	-
3	ben-u-ron 1000 mg paracetamol	Rectal	/	-
4	bupivacaine hydrochloride bupivacaine hydrochloride	Intravenous (not otherwis	/	-
5	decortin-h 5 mg prednisolone	Oral	/	-
6	dipidolor piritramide	Intravenous (not otherwis	/	-
7	effortil etilefrine hydrochloride	Intravenous (not otherwis	/	-
8	euphylong theophylline	Oral	/	-
9	fraxiparin 0,3 nadroparin calcium	Subcutaneous	/	-
10	furosemide furosemide	Oral	/	-
11	gastrozepin pirenzepine dihydrochloride	Oral	/	-
12	haes-steril 10% hetastarch; sodium chloride	Intravenous (not otherwis	/	-
13	irenat sodium perchlorate	Oral	/	-
14	myxofat acetylcysteine	Oral	/	-
15	sostril 300 mg ranitidine hydrochloride	Oral	/	-
16	spasmo-lyt dragees trospium chloride	Oral	/	-
17	spasmo-urgenin tc trospium chloride	Oral	/	-
18	spizef cefotiam hydrochloride	Intravenous (not otherwis	/	-
19	tarivid ofloxacin	Oral	/	-
20	zyloric allopurinol	Oral	/	-

Drugs (concomitant)

21	refobacin gentamicin sulfate	Intravenous (not otherwis	/	-
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Primary source

Qualification: Physician observestudytype: Other studies
 Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-96020743

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 77 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19910613				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 neuro-ratiopharm cyanocobalamin; pyridoxine hydrochloride; thiamine hydrochloride	Oral	/	-	-
2 paracetamol paracetamol	Unknown	/	-	-
3 phenhydan phenytoin	Oral	/	-	-
4 roekan ginkgo tree leaves extract	Oral	/	-	-

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-96901568

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 53 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Toxic epidermal necrolysis	19960204				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 paracetamol paracetamol	Oral	/	-	
2 tegretal 200 retard carbamazepine	Oral	/	-	

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-97000059

Serious Yes	Death Yes	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Male	Birth: privacy	Age: 45 Year	weight (kg): 68	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19960110				Unknown
2 Toxic epidermal necrolysis	19960110				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
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DE-BFARM-97000059

1	acimethin methionine, active	Oral	003 Df dosage form/Day	19951229-19960101	Urinary tract infection, site not specified
2	adriamycin doxorubicin	Intravenous (not otherwis /		19960106-19960108	Burkitt's tumour
3	ampho-moronal amphotericin b	Unknown	/	19951213-19960101	Other and unspecified diseases of the oral soft tissues
4	amphotericin b amphotericin b	Respiratory (inhalation)	/	19951213-19951214	Unspecified infectious and parasitic diseases
5	antra 40 omeprazole	Intravenous (not otherwis 080 Mg milligram(s)/Day		19951213-19951218	Unspecified functional disorder of stomach
6	aracell cytarabine	Unknown	/	19951215-	Burkitt's tumour
7	bepanthen dexpanthenol	Topical	/	19951213-19960101	Other and unspecified diseases of the oral soft tissues
8	bepanthen roche tabletten panthenol	Oral	/	19951213-19960101	
9	betaisodona loesung povidone-iodine	Buccal	/	-	Other and unspecified diseases of the oral soft tissues
10	buscopan hyoscine butylbromide	Intravenous (not otherwis 002 ml millilitre(s)/Day		19951214-19951219	Unspecified functional disorder of stomach
11	ciprobay 500 ciprofloxacin hydrochloride	Oral	/	-	Unspecified infectious and parasitic diseases
12	clont metronidazole	Oral	002 G gram(s)/Day	19951219-19951222	Unspecified infectious and parasitic diseases
13	Cyclophosphamid cyclophosphamide	Oral	/	19960103-19960108	Burkitt's tumour
14	dexamethasone dexamethasone	Unknown	/	19951215-19960103	Burkitt's tumour
15	diflucan fluconazole	Oral	200 Mg milligram(s)/Day	19951213-19951226	Candidiasis of unspecified site
16	dolantin pethidine hydrochloride	Intravenous (not otherwis /		-	Other general symptoms
17	fortecortin dexamethasone	Oral	/	-	Burkitt's tumour
18	fortum ceftazidime	Intravenous (not otherwis 006 G gram(s)/Day		19951217-19951226	Unspecified infectious and parasitic diseases
19	fraxiparin nadroparin calcium	Subcutaneous	001 Df dosage form/Day	19951213-19951214	Thrombosis
20	furosemid furosemide	Intravenous (not otherwis /		-	
21	gernebcin tobramycin sulfate	Intravenous (not otherwis 002 G gram(s)/Day		19951224-19960109	Unspecified infectious and parasitic diseases
22	leucovorin folinic acid	Oral	/	19960103-19960122	
23	lipovenoos glycerol; lecithin; soya oil	Intravenous (not otherwis 200 ml millilitre(s)/Day		19960106-19960110	Feeding difficulties and mismanagement
24	liquemin heparin	Subcutaneous	012 Kiu iu (1000s)/Day	19960101-19960103	Thrombosis
25	liquidepur anise oil; carum carvi; senna liquid extract	Oral	/	19951222-19951226	Constipation
26	magnesium verla kautabletten magnesium hydrogen aspartate	Oral	003 Df dosage form/Day	19951217-19960103	Disorders of magnesium metabolism
27	methotrexate methotrexate	Unknown	015 Mg milligram(s)/Day	19951215-19960103	Burkitt's tumour
28	multibionta ascorbic acid; calcium pantothenate;	Intravenous (not otherwis 001 Df dosage form/Day		19960103-19960110	Unspecified vitamin deficiency

cyanocobalamin; nicotinamide; pyridoxine
hydrochloride; retinol palmitate; riboflavin; rutoside;
thiamine hydrochloride; tocopheryl acetate

29	navoban tropisetron hydrochloride	Intravenous (not otherwis 005 Mg milligram(s)/Day	19960101-19960110 Nausea and vomiting
30	novalgin metamizole	Intravenous (not otherwis /	19960103-19960107 Other general symptoms
31	nutriflex basal electrolytes nos; glucose	Intravenous (not otherwis /	19951223-19951224 Feeding difficulties and mismanagement
32	paracetamol paracetamol	Rectal /	19951222-19951222 Other general symptoms
33	solu-decortin prednisolone sodium succinate	Intravenous (not otherwis 100 ml millilitre(s)/Day	19951223-19951225
34	staphylex infusion 2 g flucloxacillin sodium	Intravenous (not otherwis 006 G gram(s)/Day	19951217-19951227 Unspecified infectious and parasitic diseases
35	tavegil clemastine	Intravenous (not otherwis /	19951223- Unspecified pruritic disorder
36	uromitexan mesna	Intravenous (not otherwis /	19960103-19960109 Unspecified adverse effect of drug, medicament and
37	vitintra ergocalciferol; phytomenadione; retinol; retinol palmitate	Intravenous (not otherwis 001 Df dosage form/Day	19960106-19960108 Feeding difficulties and mismanagement
38	zienam 500 cilastatin sodium; imipenem	Intravenous (not otherwis 002 G gram(s)/Day	19951224-19960101 Unspecified infectious and parasitic diseases
39	zovirax aciclovir	Intravenous (not otherwis 1.5 G gram(s)/Day	19951213-19960107 Herpes labialis
40	zyloric 300 allopurinol	Oral 001 Df dosage form/Day	19951213-19951222 Hyperuricaemia

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-97000096

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 40 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Toxic epidermal necrolysis	19931021				Unknown
2 Maculopapular rash	19931021				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 paracetamol paracetamol	Oral	/	-	-
2 perenterol yeast dried	Oral	/	-	-
3 sirtal carbamazepine	Oral	/	-	-
4 venopyronum adonis vernalis glycosides; convallaria glycosides; horse chestnut extract; oleandrin; rutoside; squill extract	Oral	/	-	-
5 zovirax aciclovir	Topical	/	-	-

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-97000099

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 21 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Epidermal necrolysis	19930413				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 codipront codeine; phenyltoloxamine	Oral	/	-	-
2 paracetamol paracetamol	Unknown	/	-	-

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-97000100

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 45 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19940116				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 apis-homaccord homeopatics nos	Oral	/	-	-
2 ass acetylsalicylic acid	Oral	/	-	-
3 augmentan amoxicillin trihydrate; clavulanic acid	Oral	/	-	-
4 diclo divide diclofenac sodium	Oral	/	-	-
5 paracetamol paracetamol	Oral	/	-	-
6 solidagoren n herbal extracts nos	Oral	/	-	-

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-97000103

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 32 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Toxic epidermal necrolysis	19940210				Unknown
2 Macular rash	19940210				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 cafergot n caffeine; ergotamine tartrate	Rectal	/	-	-
2 dalmadorm flurazepam hydrochloride	Oral	/	-	-
3 elzogram 1,0 cefazolin sodium	Intravenous (not otherwis	/	-	-
4 fortecortin mono 8 dexamethasone sodium phosphate	Intravenous (not otherwis	/	-	-
5 liquemin heparin	Subcutaneous	/	-	-
6 nitrazepam nitrazepam	Oral	/	-	-
7 paracetamol paracetamol	Unknown	/	-	-
8 phenhydan phenytoin	Oral	/	-	-
9 ulcogant-suspension sucralfate	Oral	/	-	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-97000104

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 3 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19930912				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ben-u-ron paracetamol	Rectal	/	-	-
2 sigaprim sulfamethoxazole; trimethoprim	Oral	/	-	-

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-97000115

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 11 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Erythema multiforme	19951229				Unknown
2 Erythema exudativum	19951229				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 contramutan n aconite; adonis vernalis glycosides; belladonna alkaloids; echinacea spp; eupatorium perfoliatum; myristica	Oral	/	-	-
2 paracetamol paracetamol	Oral	/	-	-
3 pinimenthol salbe camphor; eucalyptus oil; menthol; pine oil; pumilio pine oil; turpentine oil	Topical	/	-	-
4 silomat clobutinol hydrochloride	Oral	/	-	-

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-97000117

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 26 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Macular rash	19960116				Unknown
2 Toxic epidermal necrolysis	19960116				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ben-u-ron	Oral	/	-	
paracetamol			-	
2 cotrimoxazol	Oral	/	-	
sulfamethoxazole; trimethoprim			-	

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-97001281

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 39 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19940930				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ampho-moronal amphotericin b	Oral	/	-	-
2 ampho-moronal amphotericin b	Oral	/	-	-
3 aureomycin chlortetracycline	Ophthalmic	/	-	-
4 ben-u-ron paracetamol	Unknown	/	-	-
5 biklin amikacin sulfate	Intravenous (not otherwis	/	-	-
6 calcium brausetabletten calcium	Oral	/	-	-
7 diflucan 200 fluconazole	Oral	/	-	-
8 fortum ceftazidime	Intravenous (not otherwis	/	-	-
9 foscavir foscarnet sodium	Intravenous (not otherwis	/	-	-
10 gastrosil metoclopramide hydrochloride	Oral	/	-	-
11 ibuprofen 400 mg ibuprofen	Oral	/	-	-
12 kalinor-brausetabletten potassium carbonate; potassium citrate	Oral	/	-	-
13 klacid clarithromycin	Oral	/	-	-
14 musaril tetrazepam	Oral	/	-	-
15 novalgin tropfen metamizole sodium	Oral	/	-	-
16 sobelin soluble 900 clindamycin phosphate	Intravenous (not otherwis	/	-	-
17 tavegil clemastine	Intravenous (not otherwis	/	-	-
18 vancomycin vancomycin	Intravenous (not otherwis	/	-	-
19 vergentan alzapride hydrochloride	Intravenous (not otherwis	/	-	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-97001399

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 22 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19931027				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Acetylsalizylsaeure acetylsalicylic acid	Oral	/	-	-
2 bepanthen roche tabletten panthenol	Oral	/	-	-
3 bromuc 200 brausetabletten acetylcysteine	Oral	/	-	-
4 mucosolvan ambroxol hydrochloride	Oral	/	-	-
5 novalgin metamizole	Intravenous (not otherwis	/	-	-
6 paracetamol paracetamol	Oral	/	-	-
7 paracodin n dihydrocodeine	Oral	/	-	-

Primary source

Qualification: Physician observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-97002109

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 4 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19950408				Unknown
2 Toxic epidermal necrolysis	19950408				Unknown
3 Conjunctivitis					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 gruencef cefadroxil monohydrate	Unknown	/	-	-
2 isocillin phenoxymethylpenicillin potassium	Oral	/	-	-
3 paracetamol paracetamol	Rectal	/	-	-
4 sultanol salbutamol	Respiratory (inhalation)	/	-	-
5 zinacef cefuroxime	Intravenous (not otherwis	/	-	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-97002114

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 7 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Erythema multiforme	19960104				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 acc acetylcysteine	Oral	/	-	
2 klacid clarithromycin	Oral	/	-	
3 paracetamol paracetamol	Rectal	/	-	

Primary source

Qualification: Physician observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-97002119

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 44 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Toxic epidermal necrolysis	19960415				Unknown
2 Stevens Johnson syndrome	19960415				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ben-u-ron 1000 mg paracetamol	Rectal	/	-	-
2 ciprobay ciprofloxacin hydrochloride	Oral	/	-	-
3 dormo-puren nitrazepam	Oral	/	-	-
4 encephabol forte pyritinol hydrochloride	Oral	/	-	-
5 lasix furosemide	Intravenous (not otherwis	/	-	-
6 timonil 300 retard carbamazepine	Oral	/	-	-

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-97002122

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 53 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19960415				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 paracetamol paracetamol	Oral	/	-	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-97002123

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 38 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19960413				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ben-u-ron paracetamol	Rectal	/	-	-
2 tegretal carbamazepine	Oral	/	-	-

Primary source

Qualification: Physician observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-97002146

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 43 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19961127				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 acetylcysteine acetylcysteine	Oral	/	-	-
2 elobact cefuroxime axetil	Oral	/	-	-
3 paracetamol paracetamol	Oral	/	-	-

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-97002147

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 1 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Maculopapular rash	19960925				Unknown
2 Stevens Johnson syndrome	19960925				Unknown
3 Fever	19960924				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
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1	aldactone for injection potassium canrenoate	Intravenous (not otherwis /	-
2	arterenol norepinephrine hydrochloride	Intravenous (not otherwis /	-
3	atosil promethazine hydrochloride	Intravenous (not otherwis /	-
4	ben-u-ron paracetamol	Rectal /	-
5	buscopan hyoscine butylbromide	Rectal /	-
6	catapresan clonidine	Intravenous (not otherwis /	-
7	claforan cefotaxime sodium	Intravenous (not otherwis /	-
8	dormicum midazolam maleate	Intravenous (not otherwis /	-
9	fentanyl "janssen" fentanyl citrate	Intravenous (not otherwis /	-
10	fluimucil acetylcysteine	Intravenous (not otherwis /	-
11	hydergin dihydroergocornine mesilate; dihydroergocristine mesilate; dihydroergocryptine mesilate	Intravenous (not otherwis /	-
12	intralipid glycerol; lecithin; soya oil	Intravenous (not otherwis /	-
13	kalitrans citric acid; potassium bicarbonate	Oral /	-
14	lanitop metildigoxin	Intravenous (not otherwis /	-
15	lasix furosemide	Intravenous (not otherwis /	-
16	luminal phenobarbital	Intravenous (not otherwis /	-
17	moronal suspension nystatin	Unknown /	-
18	mucosolvan ambroxol hydrochloride	Intravenous (not otherwis /	-
19	novalgine metamizole	Intravenous (not otherwis /	-
20	paracetamol paracetamol	Rectal /	-
21	refobacin gentamicin sulfate	Intravenous (not otherwis /	-
22	rohypnol flunitrazepam	Intravenous (not otherwis /	-
23	sab simplex dimeticone	Oral /	-
24	solu-decortin prednisolone sodium succinate	Intravenous (not otherwis /	-
25	somsanit oxybate sodium	Intravenous (not otherwis /	-
26	spizef cefotiam hydrochloride	Intravenous (not otherwis /	-
27	staphylex flucloxacillin sodium	Intravenous (not otherwis /	-
28	suprarenin epinephrine	Intravenous (not otherwis /	-

Qualification: Physician observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-97006262

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 63 Year weight (kg): 64 BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19940814		7 Week		recovered/resolved
2 Leucopenia					Unknown
3 Exanthema	19940714				Unknown
4 Burning sensation skin	19940714				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
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1	antra omeprazole	Oral	/	19940729-19940821	-
2	aspisol acetylsalicylate lysine	Intravenous (not otherwis	/	19940808-19940808	-
3	bayotensin nitrendipine	Oral	/Day	19940729-19940821	Essential hypertension, unspecified
4	benuron paracetamol	Rectal	/	19940729-19940814	Pyrexia of unknown origin
5	calcium carbonate calcium carbonate	Oral	/	19940729-19940821	Renal failure, unspecified
6	cymeven ganciclovir sodium	Intravenous (not otherwis	/	19940805-19940821	Congenital cytomegalovirus infection
7	fragmin dalteparin sodium	Subcutaneous	/	19940804-19940821	-
8	imurek azathioprine	Oral	025 Mg milligram(s)/Day	19940729-19940804	-
9	lamisil terbinafine hydrochloride	Oral	/	1994-19940731	Dermatomycosis, unspecified
10	lasix furosemide	Oral	/	1994-19940819	Renal failure, unspecified
11	recormon epoetin beta	Subcutaneous	/	19940819-19940819	-
12	refobacin gentamicin sulfate	Intravenous (not otherwis	020 Mg milligram(s)/Day	19940802-19940821	Other and unspecified mitral valve diseases
13	rocaltrol 0,25 calcitriol	Oral	0.5 Mg milligram(s)/Day	19940729-19940821	Renal failure, unspecified
14	tavegil clemastine	Oral	/	19940731-19940821	Unspecified pruritic disorder
15	tenormin 50 atenolol	Oral	050 Mg milligram(s)/Day	19940729-19940821	Essential hypertension, unspecified
16	Urbason methylprednisolone	Oral	004 Mg milligram(s)/Day	19940729-19940821	-
17	vancomycin vancomycin	Intravenous (not otherwis	1 G gram(s)/	19940802-19940805	Other and unspecified mitral valve diseases
18	vancomycin "lilly" vancomycin	Oral	001 G gram(s)/Day	19940802-19940821	Other and unspecified mitral valve diseases
19	zienam 500 cilastatin sodium; imipenem	Intravenous (not otherwis	/	19940729-19940821	Other and unspecified mitral valve diseases

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Cardiac hypertrophy			k.A.	concomitant disease

Primary source

Qualification: Physician observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-97006274

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 31 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19960212				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diazepam diazepam	Oral	/	-	-
2 paracetamol paracetamol	Oral	/	-	-
3 remedacen dihydrocodeine	Oral	/	-	-
4 timonil 300 retard carbamazepine	Oral	/	-	-

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-97006277

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 40 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Erythema multiforme severe	19960224				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 aristamid sulfisomidine	Topical	/	-	-
2 aspirin plus c acetylsalicylic acid; ascorbic acid	Oral	/	-	-
3 bepanthen roche augen- und nasensalbe panthenol	Ophthalmic	/	-	-
4 bepanthen roche tabletten panthenol	Oral	/	-	-
5 bromuc granulat acetylcysteine	Oral	/	-	-
6 codipront kapseln codeine; phenyltoloxamine	Oral	/	-	-
7 dolo-dobendan benzocaine; cetylpyridinium chloride	Oral	/	-	-
8 heparin calcium heparin calcium	Subcutaneous	/	-	-
9 novalgin tropfen metamizole sodium	Oral	/	-	-
10 novaminsulfon metamizole	Oral	/	-	-
11 otrivin xylometazoline hydrochloride	Nasal	/	-	-
12 paracetamol paracetamol	Oral	/	-	-
13 paracodin dihydrocodeine bitartrate	Oral	/	-	-
14 rulid 300 roxithromycin	Oral	/	-	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-97006282

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 26 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19960220				Unknown
2 Skin erythema	19960220				Unknown
3 Macular rash	19960220				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ampho-moronal amphotericin b	Oral	/	-	-
2 candio-hermal nystatin	Oral	/	-	-
3 emovate creme clobetasone butyrate	Topical	/	-	-
4 isocillin 1,2 mega phenoxymethylpenicillin potassium	Oral	/	-	-
5 otrivin xylometazoline hydrochloride	Nasal	/	-	-
6 paracetamol paracetamol	Oral	/	-	-
7 penicillin benzylpenicillin sodium	Intravenous (not otherwis	/	-	-
8 tabalon ibuprofen	Oral	/	-	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-97006284

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 32 Month weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Macula lutea degeneration	19941217				Unknown
2 Toxic epidermal necrolysis	19941217				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 chloraldurat chloral hydrate	Oral	/	-	
2 imeson nitrazepam	Oral	/	-	
3 luminal phenobarbital	Oral	/	-	
4 paracetamol paracetamol	Oral	/	-	
5 tavegil clemastine	Oral	/	-	
6 theralene alimemazine tartrate	Oral	/	-	

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-97006287

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 56 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19960316				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 aciclovir aciclovir	Oral	/	-	-
2 paracetamol paracetamol	Oral	/	-	-
3 vitamin b complex calcium pantothenate; nicotinamide; pyridoxine hydrochloride; riboflavin; thiamine hydrochloride	Oral	/	-	-

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-97006351

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 43 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19960222				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 bromazaniol bromazepam	Oral	/	-	-
2 novalgin tropfen metamizole sodium	Oral	/	-	-
3 paracefan clonidine hydrochloride	Oral	/	-	-
4 paracetamol paracetamol	Oral	/	-	-
5 rivotril clonazepam	Oral	/	-	-
6 tegretal suspension carbamazepine	Oral	/	-	-
7 voltaren diclofenac sodium	Oral	/	-	-

Primary source

Qualification: Physician observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-97006354

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 84 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19951229				Unknown
2 Toxic epidermal necrolysis	19951229				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 maninil 1,75 glibenclamide	Oral	/	-	-
2 milgamma na benfotiamine; pyridoxine hydrochloride	Oral	/	-	-
3 paracetamol paracetamol	Oral	/	-	-
4 tegretal 200 carbamazepine	Oral	/	-	-
5 valoron tilidine hydrochloride	Oral	/	-	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-97006355

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes							

Patient

Initials:	Sex: Male	Birth: privacy	Age: 44 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Lyell syndrome	19950610				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 agarol paraffin, liquid; phenolphthalein	Intravenous (not otherwis /		-	
2 aspisol acetylsalicylate lysine	Intravenous (not otherwis /		-	
3 bisacodyl bisacodyl	Unknown /		-	
4 ciprobay ciprofloxacin hydrochloride	Intravenous (not otherwis /		-	
5 cotrim forte sulfamethoxazole; trimethoprim	Oral /		-	
6 esprenit ibuprofen	Oral /		-	
7 fenistil dimetindene maleate	Intravenous (not otherwis /		-	
8 fortum ceftazidime	Intravenous (not otherwis /		-	
9 furorese injekt furosemide sodium	Intravenous (not otherwis /		-	
10 konakion phytomenadione	Intravenous (not otherwis /		-	
11 Kortison cortisone acetate	Oral /		-	
12 lasix furosemide	Intravenous (not otherwis /		-	
13 nepresol dihydralazine sulfate	Intravenous (not otherwis /		-	
14 novalgin metamizole	Intravenous (not otherwis /		-	
15 paracetamol paracetamol	Oral /		-	
16 pipril piperacillin sodium	Intravenous (not otherwis /		-	
17 reparil escin	Intravenous (not otherwis /		-	
18 rivotril clonazepam	Oral /		-	
19 theralene alimemazine tartrate	Oral /		-	
20 vancomycin vancomycin	Intravenous (not otherwis /		-	

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-97007208

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Birth: privacy Age: weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Thrombopenia					Unknown
2 Headache					Unknown
3 Pyrexia abnormal					Unknown
4 White blood cell decreased					Unknown
5 Drug eruption					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ibuprofen	Unknown	/	-	
ibuprofen			-	
2 paracetamol	Unknown	/	-	
paracetamol			-	

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-97008939

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 20 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19771017				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 globocef cefetamet pivoxil hydrochloride	Oral	/	-	-
2 paracetamol paracetamol	Oral	/	-	-
3 zithromax azithromycin	Oral	/	-	-

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-98000010

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 66 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19950513				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 adalat nifedipine	Oral	/	-	-
2 atosil promethazine hydrochloride	Oral	/	-	-
3 ben-u-ron paracetamol	Unknown	/	-	-
4 cytotec 200 misoprostol	Oral	/	-	-
5 diclophlogont ampullen diclofenac sodium	Intramuscular	/	-	-
6 digimerck digitoxin	Intravenous (not otherwis	/	-	-
7 elantan 20 isosorbide mononitrate	Oral	/	-	-
8 furosemide furosemide	Intravenous (not otherwis	/	-	-
9 gelonida acetylsalicylic acid; codeine phosphate; paracetamol	Oral	/	-	-
10 isoptin verapamil hydrochloride	Intravenous (not otherwis	/	-	-
11 lasix 40 furosemide	Oral	/	-	-
12 neurocil levomepromazine maleate	Oral	/	-	-
13 novodigal beta-acetyldigoxin	Intravenous (not otherwis	/	-	-
14 supracombin sulfamethoxazole; trimethoprim	Oral	/	-	-
15 tranxilium clorazepate dipotassium	Oral	/	-	-
16 valoron n naloxone hydrochloride; tilidine hydrochloride	Oral	/	-	-
17 voltaren diclofenac sodium	Unknown	/	-	-

Primary source

Qualification: Physician observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-98000014

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes	Yes	Yes					

Patient

Initials:	Sex: Male	Birth: privacy	Age: 25 Year	weight (kg):	BMI_int:	Age fetus:
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Patientdeath

deathdate: 19930106	deathcause: Multiorgan failure
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Lyell's syndrome	19921230	19930106	8 Day		fatal
2 Multiple organ failure	19921230				Unknown
3 Leucopenia	19921230				Unknown
4 Epidermal necrolysis	19921230				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 antra pro infusione omeprazole sodium	Intravenous (not otherwis /		-	
2 baypen mezlocillin sodium	Intravenous (not otherwis /		-	
3 bepanthen roche loesung panthenol	Intravenous (not otherwis /		-	
4 clont metronidazole	Intravenous (not otherwis /		-	
5 fortral for injection pentazocine lactate	Intravenous (not otherwis /		-	
6 insulin actrapid human insulin human	Intravenous (not otherwis /		-	
7 klacid clarithromycin	Oral	/	-	
8 konaktion phytomenadione	Oral	/	-	
9 multibionta zur infusion ascorbic acid; dexpantenol; nicotinamide; pyridoxine hydrochloride; retinol palmitate; riboflavin sodium phosphate; thiamine hydrochloride; tocopheryl acetate	Intravenous (not otherwis /		-	
10 myambutol ethambutol dihydrochloride	Oral	/	-	
11 paracetamol paracetamol	Unknown	/	-	
12 paspertin metoclopramide hydrochloride	Intravenous (not otherwis /		-	
13 prostigmin injection neostigmine metilsulfate	Intravenous (not otherwis /		-	
14 rifampicin rifampicin	Oral	/	-	
15 temgesic buprenorphine	Intravenous (not otherwis /		-	
16 thioacetazon thioacetazone	Oral	/	-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Atypical mycobacteriosis			Unknown	
Allergy			Unknown	
Diabetes mellitus			Unknown	concomitant disease
HIV infection NOS			Unknown	concomitant disease
Kaposi's sarcoma			Unknown	concomitant disease

Primary source

Qualification: Physician observestudytype:
Literature:

Narratives (nur bei elektronischem Eingang)

Schering-Plough, 26.11.2008 DE-01054554290-2008SP023612

Sender diagnosis: not reported

Narrative

THIS IS A VERBATIM REPORT THAT ORIGINATED IN GERMANY FROM RECKITT BENKISER WITH REPORT NUMBER PR/980204/16.

PATIENT HOSPITALISED DUE TO 16/12/92 DUE TO ABDOMINAL PAIN. DIAGNOSED ATYPICAL MYCOBACTERIAL INFECTION AND RETROPANCREATIC ABSCESS. GIVEN MEDICATION INCLUDING TEMGESIC IV (24, 25 AND 31 DEC 92). ALSO RECEIVED RADIOTHERAPY FOR KAPOSIS SARCOMA. 30 DEC DEVELOPED MACULOPAPULOUS EXANTHEMA WITH ITCHING. 1 JAN 93 CUTANEOUS CONDITION DETERIORATED ERYTHRODERMA BULLOUS CHANGES, CONJUNCTIVITIS AND EROSIONS OF ORAL AND GENITAL MUCOUS MEMBRANES. DIAGNOSED AS LYELLS SYNDROME (TOXIC EPIDERMAL NECROLYSIS). PATIENT DIED 6 JAN 93 DUE TO MULTI-ORGAN FAILURE.

COMMENTS FROM RECONCILIATION/LEGACY TRANSFER GERMANY: MALE AGED 25 YEARS WITH ADVANCED AIDS DISEASE, KAPOSIS SARCOMA AND TUBERCULOSIS, HOSPITALISED WITH RETROPANCREATIC ABSCESS. MEDICATION: ACTRAPID INSULIN IV, OMEPRAZOLE, RIFAMPICIN, ETHAMBUTOL, MEZLOCILLIN, CLARITHROMYCIN, THIOACETACON. ANALGESIA STARTED ON 27.12.92 WAS PARACETAMOL SUPPOSITORIES, BUPRENORPHINE IV 3 TIMES AND PENTAZOCINE. PATIENT DEVELOPED TOXIC EPIDERMAL

Sender's comment:

The serious events of toxic epidermal necrolysis (Lyell's syndrome) and multi-organ failure are not listed within the Company Core Data Sheet (CCDS). The reporter did not provide causality assessment. Paracetamol and pentazocine were considered co-suspect drugs. Confounders include medical history of HIV infection, atypical mycobacterial infection, Kaposi's sarcoma and diabetes. Review of this single case report does not change the existing safety profile of the product.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-98000015

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 12 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19951030				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 capval noscaphine resin	Oral	/	-	-
2 dolo-dobendan benzocaine; cetylpyridinium chloride	Oral	/	-	-
3 mucosolvan ambroxol hydrochloride	Oral	/	-	-
4 paracetamol paracetamol	Unknown	/	-	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-98000017

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 6 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Lyell syndrome	19970205				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 cotrimox-wolff saft sulfamethoxazole; trimethoprim	Oral	/	-	-
2 mucosolvan ambroxol hydrochloride	Oral	/	-	-
3 paracetamol paracetamol	Unknown	/	-	-
4 saccharomyces boulardii saccharomyces boulardii	Oral	/	-	-

Primary source

Qualification: Physician observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-98000021

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 6 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Skin erosion					Unknown
2 Buccal inflammation					Unknown
3 Stevens Johnson syndrome	19970122				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ambroxol	Oral	/	-	
ambroxol			-	
2 bromhexin 8 berlin-chemie	Oral	/	-	
bromhexine hydrochloride			-	
3 orelox junior	Oral	/	-	
cefprozime proxetil			-	
4 paracetamol	Oral	/	-	
paracetamol			-	

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-98006131

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Female	Birth: privacy	Age: 12 Year	weight (kg): 48	BMI_int: 18.07	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Erythema multiforme severe	19980328		1 Month		recovered/resolved
2 Oral mucosal eruption	19980328		1 Month		recovered/resolved
3 Stevens Johnson syndrome	19980328		1 Month		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
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1	acesal	Oral	/	-	
	acetylsalicylic acid			-	
2	berlocombin 200	Oral	/	-	
	sulfamerazine; trimethoprim			-	
3	betaisodona loesung	Topical	/	-	
	povidone-iodine			-	
4	colistin parenteral	Intravenous (not otherwis	/	-	
	colistin mesilate sodium			-	
5	dexamethasone	Oral	/	-	
	dexamethasone			-	
6	disoprivan	Intravenous (not otherwis	/	-	
	propofol			-	
7	dolantin	Intravenous (not otherwis	/	-	
	pethidine hydrochloride			-	
8	dormicum	Oral	/	-	19980316-19980316
	midazolam maleate			-	
9	endoxan	Intravenous (not otherwis	/	-	
	cyclophosphamide			-	
10	esmeron	Intravenous (not otherwis	/	-	
	rocuronium bromide			-	
11	fenistil tropfen	Oral	/	-	
	dimetindene maleate			-	
12	furesis comp	Oral	/	-	
	furosemide; triamterene			-	
13	hydergin	Intravenous (not otherwis	/	-	
	dihydroergocornine mesilate; dihydroergocristine mesilate; dihydroergocryptine mesilate			-	
14	ifosfamide	Intravenous (not otherwis	/	-	
	ifosfamide			-	
15	leucovorin	Intravenous (not otherwis	/	-	Non-Hodgkin's lymphoma
	folinic acid			-	
16	marophen	Unknown	/	-	
	chlorphenethazine hydrochloride			-	
17	mtx	Unknown	/	-	
	methotrexate sodium			-	
18	novalgin	Oral	/	-	19980312-
	metamizole			-	
19	paracetamol	Rectal	/	-	Pain
	paracetamol			-	
20	prothazin	Intravenous (not otherwis	/	-	
	promethazine hydrochloride			-	
21	solu-decortin-h	Intrathecal	/	-	
	prednisolone sodium succinate			-	
22	sostril	Oral	/	-	
	ranitidine hydrochloride			-	
23	sufenta mite 10	Intravenous (not otherwis	/	-	
	sufentanil citrate			-	
24	temgesic	Intravenous (not otherwis	/	-	19980316- Prophylaxis
	buprenorphine			-	
25	tramal tropfen	Oral	/	-	Pain
	tramadol hydrochloride			-	
26	uromitexan 100 mg	Intravenous (not otherwis	/	-	
	mesna			-	
27	vetren	Intravenous (not otherwis	/	-	
	heparin sodium			-	
28	vincristine sulfate	Intravenous (not otherwis	/	-	
	vincristine sulfate			-	

29 **zyloric**
 allopurinol

Oral

/

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Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-98003738

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 58 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Toxic epidermal necrolysis	19960401				Unknown
2 Stevens Johnson syndrome	19960401				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ibuhexal 600	Oral	/	-	
ibuprofen			-	
2 Kodeinphosphat	Oral	/	-	
codeine phosphate			-	
3 paracetamol	Oral	/	-	
paracetamol			-	
4 thomapyrin	Oral	/	-	
acetylsalicylic acid; caffeine; paracetamol			-	

Primary source

Qualification: Physician observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-98003750

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.
YES

Patient

Initials: Sex: Female Birth: privacy Age: 23 Year weight (kg): BMI_int: Age fetus: 6 Week

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Toxic epidermal necrolysis	19960606				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ben-u-ron paracetamol	Rectal	/	-	-
2 cebion 500 mg n forte sodium ascorbate	Intravenous (not otherwis	/	-	-
3 sinupret ascorbic acid; gentiana; primula flowers; rumicis asetosa; sambucus flowers; verbenaherb	Oral	/	-	-
4 vomex a suppositorien dimenhydrinate	Rectal	/	-	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-98003759

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 31 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Toxic epidermal necrolysis	19960716				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 paracetamol paracetamol	Oral	/	-	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-98900372

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 58 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Bullous eruption					Unknown
2 LDH increased					Unknown
3 Toxic epidermal necrolysis	19960501				Unknown
4 Itching	19960430				Unknown
5 Canker sores oral					Unknown
6 Exanthema					Unknown
7 Stevens Johnson syndrome	19960501				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
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1	adumbran	Oral	/	-
	oxazepam			-
2	azucalcit	Subcutaneous	/	-
	calcitonin, salmon			-
3	bactrim roche	Oral	/	-
	sulfamethoxazole; trimethoprim			-
4	baldrian	Oral	/	-
	valeriana officinalis root			-
5	ciprobay 500	Oral	/	-
	ciprofloxacin hydrochloride			-
6	fenistil-retard	Intravenous (not otherwis	/	-
	dimetindene maleate			-
7	ludiomil injektionsloesung	Intravenous (not otherwis	/	-
	maprotiline mesilate			-
8	maalox 70	Oral	/	-
	aluminium hydroxide; aluminium hydroxide gel; magnesium hydroxide			-
9	novalgin tropfen	Oral	/	-
	metamizole sodium			-
10	paracetamol	Oral	/	-
	paracetamol			-
11	saroten	Oral	/	-
	amitriptyline hydrochloride			-
12	saroten retard	Oral	/	-
	amitriptyline hydrochloride			-
13	solu-decortin	Intravenous (not otherwis	/	-
	prednisolone sodium succinate			-
14	tensobon comp	Oral	/	-
	captopril; hydrochlorothiazide			-
15	timonil	Oral	/	-
	carbamazepine			-
16	tramal tropfen	Oral	/	-
	tramadol hydrochloride			-
17	tranquase	Oral	/	-
	diazepam			-
18	valoron	Oral	/	-
	tilidine hydrochloride			-

Primary source

Qualification: Physician observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-98005969

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 13 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Fatigability generalized	19980331				Unknown
2 Fever	19980402				Unknown
3 Stevens Johnson syndrome	19980403				Unknown
4 Epidermal necrolysis	19940403				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 aspirin "bayer"	Oral	/	-	
acetylsalicylic acid			-	
2 ben-u-ron	Unknown	/	-	
paracetamol			-	

Primary source

Qualification: Physician observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-98006116

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 32 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19971129				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 paracetamol paracetamol	Oral	/	-	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-98006121

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 53 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19980109				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 amoxicillin amoxicillin	Oral	/	-	-
2 ass acetylsalicylic acid	Oral	/	-	-
3 paracetamol paracetamol	Oral	/	-	-
4 paracodin dihydrocodeine bitartrate	Oral	/	-	-

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-98006128

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 9 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19980125				Unknown
2 Toxic epidermal necrolysis	19980125				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ambroxol	Oral	/	-	
ambroxol			-	
2 mcp-tropfen	Oral	/	-	
metoclopramide hydrochloride			-	
3 paracetamol	Oral	/	-	
paracetamol			-	

Primary source

Qualification: Physician observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-98006137

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes							

Patient

Initials:	Sex: Male	Birth: privacy	Age: 8 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19971122				Unknown
2 Cardiac failure	19971206				Unknown
3 Pruritis	19971106				Unknown
4 Multiple organ failure	19971206				Unknown
5 Toxic epidermal necrolysis	19971118				Unknown
6 Maculopapular rash	19971106				Unknown
7 Circulatory failure	19971206				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
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1	ambisome amphotericin b	Intravenous (not otherwis /	-
2	atosil tropfen promethazine hydrochloride	Oral /	-
3	diflucan fluconazole	Intravenous (not otherwis /	-
4	dobutrex dobutamine hydrochloride	Intravenous (not otherwis /	-
5	dopamin dopamine hydrochloride	Intravenous (not otherwis /	-
6	dormicum midazolam maleate	Intravenous (not otherwis /	-
7	euphyllin aminophylline	Intravenous (not otherwis /	-
8	fortum ceftazidime	Intravenous (not otherwis /	-
9	kepinol sulfamethoxazole; trimethoprim	Oral /	-
10	lasix furosemide	Intravenous (not otherwis /	-
11	neupogen filgrastim	Subcutaneous /	-
12	neurocil levomepromazine maleate	Intravenous (not otherwis /	-
13	novalgine metamizole	Intravenous (not otherwis /	-
14	novodigal beta-acetyldigoxin	Intravenous (not otherwis /	-
15	paracetamol paracetamol	Oral /	-
16	refobacin gentamicin sulfate	Ophthalmic /	-
17	riopan magaldrate	Oral /	-
18	suprarenin epinephrine	Intravenous (not otherwis /	-
19	tavegil clemastine	Intravenous (not otherwis /	-
20	TAZOBAC piperacillin sodium; tazobactam sodium	Intravenous (not otherwis /	-
21	valium diazepam	Intravenous (not otherwis /	-
22	vancomycin vancomycin	Intravenous (not otherwis /	-
23	vetren heparin sodium	Intravenous (not otherwis /	-
24	vidisept cetrimonium chloride; polyvidone	Ophthalmic /	-
25	zantic ranitidine hydrochloride	Oral /	-
26	zovirax aciclovir	Intravenous (not otherwis /	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-98006138

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Male	Birth: privacy	Age: 51 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Exanthema	19971124				recovered/resolved
2 Red eye	19971124				recovered/resolved
3 Urethritis	19971124				recovered/resolved
4 Toxic epidermal necrolysis	19971124				recovered/resolved
5 Stevens Johnson syndrome	19971124				recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
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1	akrinor injektionsloesung cafedrine hydrochloride; theodrenaline hydrochloride	Intravenous (not otherwis /	19971104-19971121	Unspecified circulatory system disorder
2	antra pro infusione omeprazole sodium	Intravenous (not otherwis 040 Mg milligram(s)/Day	19971123-19971201	Peptic ulcer unspecified as acute or chronic, without
3	arterenol norepinephrine hydrochloride	Intravenous (not otherwis /	19971120-19971123	Unspecified circulatory system disorder
4	atenativ antithrombin iii	Intravenous (not otherwis 3*1 Kiu iu (1000s)/	19971104-19971118	Embolism and thrombosis of unspecified artery
5	beloc metoprolol tartrate	Intravenous (not otherwis /	19971111-19971121	Atrial fibrillation and flutter
6	bricanyl 0,5 mg injektionsloesung terbutaline sulfate	Intravenous (not otherwis /	19971123-19971201	Other diseases of trachea and bronchus, not elsewhere
7	clont metronidazole	Intravenous (not otherwis 001 G gram(s)/Day	19971108-19971117	Pneumonia, organism unspecified
8	dipidolor piritramide	Intravenous (not otherwis /	19971107-19971201	Other general symptoms
9	disoprivan 2% propofol	Intravenous (not otherwis /	19971106-19971116	Other and unspecified special symptoms or syndromes, not
10	dobutrex dobutamine hydrochloride	Intravenous (not otherwis /	19971121-19971121	Heart failure, unspecified
11	dormicum midazolam maleate	Intravenous (not otherwis /	19971107-19971201	
12	euphyllin aminophylline	Intravenous (not otherwis 200 Mg milligram(s)/	19971123-19971124	Other diseases of trachea and bronchus, not elsewhere
13	faktor xiii factor xiii (fibrin stabilising factor)	Intravenous (not otherwis /	19971111-19971111	Congenital deficiency of other clotting factors
14	fortecortin dexamethasone	Intravenous (not otherwis 4* /	19971104-19971112	Cerebral oedema
15	fortum ceftazidime	Intravenous (not otherwis 004 G gram(s)/Day	19971108-19971117	Pneumonia, organism unspecified
16	Glyceroltrinitrat glyceryl trinitrate	Intravenous (not otherwis /	19971123-19971125	Heart failure, unspecified
17	humanalbumin albumin human	Intravenous (not otherwis 400 ml millilitre(s)/	19971106-19971124	Other disorders of plasma protein metabolism
18	hydergin dihydroergocornine mesilate; dihydroergocristine mesilate; dihydroergocryptine mesilate	Intravenous (not otherwis 3* /	19971115-19971201	Intracranial injury of other and unspecified nature without
19	Hydrokortison hydrocortisone	Intravenous (not otherwis /	19971107-19971125	Cerebral oedema
20	hypnomidate etomidate sulfate	Intravenous (not otherwis /	19971104-19971128	
21	insulin insulin	Intravenous (not otherwis /	19971105-19971201	Abnormal glucose tolerance test
22	isoptin injektionsloesung verapamil hydrochloride	Intravenous (not otherwis 3* /	19971113-19971124	Atrial fibrillation and flutter
23	ketanest ketamine	Intravenous (not otherwis 048 ml millilitre(s)/Day	19971123-19971201	Other general symptoms
24	lasix furosemide	Intravenous (not otherwis /	19971107-19971129	Oedema
25	norcuron vecuronium bromide	Intravenous (not otherwis /	19971106-19971121	Other disorders of muscle, ligament, and fascia
26	novalgin metamizole	Intravenous (not otherwis 6*1 G gram(s)/	19971104-19971201	Pyrexia of unknown origin
27	novodigal beta-acetyldigoxin	Intravenous (not otherwis /	19971109-19971201	Heart failure, unspecified
28	paracetamol paracetamol	Oral /	19971106-19971201	

			-
29	pentaglobin immunoglobulin human normal	Intravenous (not otherwis 300 ml millilitre(s)/	19971117-19971118
30	suprarenin epinephrine	Respiratory (inhalation) /	-
31	Urbason methylprednisolone	Intravenous (not otherwis 3*500 Mg milligram(s)/	19971108-19971123 Unspecified circulatory system disorder
32	vancomycin vancomycin	Intravenous (not otherwis 002 G gram(s)/Day	19971107-19971125 Cerebral oedema
33	verapamil verapamil	Intravenous (not otherwis 10 Mg milligram(s)/	-
34	zienam cilastatin sodium; imipenem	Intravenous (not otherwis 003 G gram(s)/Day	19971108-19971118 Pneumonia, organism unspecified
35	zinacef cefuroxime	Intravenous (not otherwis 4.5 G gram(s)/Day	19971118-19971118 Atrial fibrillation and flutter
			-
			19971117-19971125 Pneumonia, organism unspecified
			19971103-19971108 Unspecified infectious and parasitic diseases
			-

Drugs (concomitant)

36	ake 1100 amino acids nos; electrolytes nos; glycerophosphate sodium; sorbitol; xylitol	Intravenous (not otherwis /	19971103-19971104	Volume depletion
			-	
37	aminomix amino acids nos; minerals nos	Intravenous (not otherwis /	19971105-19971201	
			-	
38	brevimytal methohexital sodium	Intravenous (not otherwis /	19971103-19971106	
			-	
39	calcium gluconate calcium gluconate	Intravenous (not otherwis /	19971105-19971120	Disorders of calcium metabolism
			-	
40	dipeptamin amino acids nos	Intravenous (not otherwis 100 ml millilitre(s)/	19971124-19971124	
			-	
41	dolantin pethidine hydrochloride	Intravenous (not otherwis /	19971115-19971201	Pyrexia of unknown origin
			-	
42	dopamin dopamine hydrochloride	Intravenous (not otherwis /	19971103-19971121	Unspecified circulatory system disorder
			-	
43	ebrantil urapidil	Intravenous (not otherwis /	19971107-19971107	Hypertension arterial
			-	
44	fentanyl fentanyl	Intravenous (not otherwis /	19971103-19971201	
			-	
45	gastrozepin pirenzepine dihydrochloride	Intravenous (not otherwis /	19971103-19971120	Peptic ulcer unspecified as acute or chronic, without
			-	
46	glucose 5% glucose monohydrate	Intravenous (not otherwis /	19971106-19971121	
			-	
47	haemaccel 35 polygeline	Intravenous (not otherwis 500 ml millilitre(s)/	19971103-19971119	Volume depletion
			-	
48	inzolen aspartate zinc; cobalt hydrogenaspartate; copper hydrogenaspartate; magnesium aspartate; manganese hydrogenaspartate; potassium hydrogen aspartate	Intravenous (not otherwis /	19971105-19971110	
			-	
49	kaliumchlorid potassium chloride	Intravenous (not otherwis /	19971103-19971201	Hypokalaemia
			-	
50	liquemin heparin	Intravenous (not otherwis /	19971103-19971201	
			-	
51	mannit 20% mannitol	Intravenous (not otherwis 125 ml millilitre(s)/Day	19971103-19971107	
			-	
52	mucosolvan ambroxol hydrochloride	Intravenous (not otherwis 144 ml millilitre(s)/Day	19971103-19971120	
			-	
53	nacl 0,9% sodium chloride	Intravenous (not otherwis /	19971104-19971128	
			-	
54	ringer calcium chloride; potassium chloride; sodium chloride; sodium chloride compound injection	Intravenous (not otherwis /	19971103-19971201	Volume depletion
			-	
55	ulcogant sucralfate	Intravenous (not otherwis 012 G gram(s)/Day	19971103-19971129	Peptic ulcer unspecified as acute or chronic, without
			-	
56	vitamin c ascorbic acid	Intravenous (not otherwis 005 G gram(s)/Day	19971104-19971125	Ascorbic acid deficiency
			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Osteoarthritis, localized, not specified whether primary or s			k.A.	concomitant disease

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-98006360

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 5 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19951011				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ben-u-ron soft paracetamol	Oral	/	-	-
2 paracetamol paracetamol	Unknown	/	-	-

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-98006369

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 32 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19951219				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 acetylcysteine acetylcysteine	Oral	/	-	-
2 capval noscaphine resin	Oral	/	-	-
3 doxycycline doxycycline	Oral	/	-	-
4 paracetamol paracetamol	Oral	/	-	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-98007004

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes							

Patient

Initials:	Sex: Female	Birth: privacy	Age: 70 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19970510				Unknown
2 Toxic epidermal necrolysis	19970510				Unknown
3 Multiple organ failure	19970514				Unknown
4 Stomatitis	199705				Unknown
5 Conjunctivitis	199705				Unknown
6 Renal insufficiency	19970514				Unknown
7 Tumor lysis syndrome	19970514				Unknown
8 Sepsis	19970514				Unknown
9 Leucopenia	19970514				Unknown
10 Fever	19970507				Unknown
11 Pancreatitis	19970507				Unknown
12 Diarrhea	19970507				Unknown
13 Lip oedema	19970508				Unknown
14 Rash bullous	19970510				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
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1	alexan cytarabine	Intrathecal	/	-
2	amphotericin b amphotericin b	Intravenous (not otherwis	/	-
3	arterenol norepinephrine hydrochloride	Intravenous (not otherwis	/	-
4	ben-u-ron paracetamol	Oral	/	-
5	cernevit	Intravenous (not otherwis	/	-
6	diflucan fluconazole	Intravenous (not otherwis	/	-
7	digimerck digitoxin	Intravenous (not otherwis	/	-
8	dobutrex dobutamine hydrochloride	Intravenous (not otherwis	/	-
9	dopamin dopamine hydrochloride	Intravenous (not otherwis	/	-
10	dormicum midazolam maleate	Intravenous (not otherwis	/	-
11	etomidat-lipuro etomidate	Intravenous (not otherwis	/	-
12	fentanyl fentanyl	Intravenous (not otherwis	/	-
13	fortecortin dexamethasone	Intrathecal	/	-
14	haes-steril 10% hetastarch; sodium chloride	Intravenous (not otherwis	/	-
15	heparin-natrium heparin sodium	Intravenous (not otherwis	/	-
16	insulin velasulin human insulin human	Intravenous (not otherwis	/	-
17	lasix furosemide	Intravenous (not otherwis	/	-
18	meronem meropenem	Intravenous (not otherwis	/	-
19	methotrexat methotrexate	Intrathecal	/	-
20	moduretik amiloride hydrochloride; hydrochlorothiazide	Oral	/	-
21	neupogen 30	Subcutaneous	/	-
22	noctamid-2 lormetazepam	Oral	/	-
23	novalgin injektionsloesung metamizole	Intravenous (not otherwis	/	-
24	pentacarinat pentamidine isethionate	Respiratory (inhalation)	/	-
25	saroten dragees 10 mg amitriptyline hydrochloride	Oral	/	-
26	solu-decortin prednisolone sodium succinate	Intravenous (not otherwis	/	-
27	sostril ranitidine hydrochloride	Intravenous (not otherwis	/	-
28	vancomycin vancomycin	Intravenous (not otherwis	/	-

29	zienam	Intravenous (not otherwis /	-
	cilastatin sodium; imipenem		-
30	zovirax	Intravenous (not otherwis /	-
	aciclovir		-
31	zyloric	Oral /	-
	allopurinol		-

Primary source

Qualification: Physician observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-98007059

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes							

Patient

Initials:	Sex: Female	Birth: privacy	Age: 48 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Diarrhoea	19970904				Unknown
2 Exanthema	19970918				Unknown
3 Itching	19970913				Unknown
4 Buccal inflammation	19970920				Unknown
5 Conjunctivitis	19970918				Unknown
6 Mucositis NOS	19970831				Unknown
7 Circulatory failure	19971015				Unknown
8 Renal failure NOS	19971015				Unknown
9 Sepsis					Unknown
10 Infection fungal					Unknown
11 Toxic epidermal necrolysis	19970918				Unknown
12 Cardiac failure	19971015				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
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1	alt-insulin insulin	Subcutaneous /	-
2	buscopan hyoscine butylbromide	Intravenous (not otherwis /	-
3	ciprobay ciprofloxacin hydrochloride	Intravenous (not otherwis /	-
4	clont i.v. metronidazole	Intravenous (not otherwis /	-
5	diazepam diazepam	Oral /	-
6	diflucan fluconazole	Intravenous (not otherwis /	-
7	dilaudid hydromorphone hydrochloride	Intravenous (not otherwis /	-
8	dopamin dopamine hydrochloride	Intravenous (not otherwis /	-
9	erythrozytenkonzentrat red blood cells, concentrated	Unknown /	-
10	folsan folic acid	Oral /	-
11	fortum ceftazidime	Intravenous (not otherwis /	-
12	lasix furosemide	Intravenous (not otherwis /	-
13	maaloxan aluminium hydroxide; aluminium hydroxide gel; magnesium hydroxide	Oral /	-
14	meronem meropenem	Intravenous (not otherwis /	-
15	nizax lilly nizatidine	Oral /	-
16	novaminsulfon metamizole	Intravenous (not otherwis /	-
17	paracetamol paracetamol	Oral /	-
18	paspertin metoclopramide hydrochloride	Intravenous (not otherwis /	-
19	praxiten oxazepam	Oral /	-
20	presomen estrogens conjugated	Oral /	-
21	sandimmun neoral ciclosporin	Oral /	-
22	sandimmun optoral ciclosporin	Oral /	-
23	saroten amitriptyline hydrochloride	Oral /	-
24	sempera itraconazole	Oral /	-
25	tazobac 4,5 g piperacillin sodium; tazobactam sodium	Intravenous (not otherwis /	-
26	thrombozytenkonzentrat platelets, human blood	Unknown /	-
27	Urbason methylprednisolone	Intravenous (not otherwis /	-
28	vergentan alizapride hydrochloride	Intravenous (not otherwis /	-

Primary source

Qualification: Physician
Literature: observestudytype:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-98007052

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes							

Patient

Initials:	Sex: Female	Birth: privacy	Age: 88 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19971019				Unknown
2 Blistering	19971019				Unknown
3 Red eye	19971020				Unknown
4 Buccal inflammation	19971020				Unknown
5 Maculopapular rash	19971019				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 allopurinol allopurinol	Oral	/	-	
2 ben-u-ron paracetamol	Unknown	/	-	
3 ciprobay 200 ciprofloxacin lactate	Intravenous (not otherwis	/	-	
4 digimerck minor digitoxin	Oral	/	-	
5 furorese furosemide	Oral	/	-	
6 glibenclamide glibenclamide	Oral	/	-	
7 heparin heparin	Subcutaneous	/	-	
8 tavegil clemastine	Oral	/	-	
9 zyrtec cetirizine hydrochloride	Oral	/	-	

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-98007053

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes							

Patient

Initials:	Sex: Male	Birth: privacy	Age: 53 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Buccal inflammation	19971015				Unknown
2 Genital eruption male	19971016				Unknown
3 Blistering	19971008				Unknown
4 Exanthema	19971008				Unknown
5 Toxic epidermal necrolysis	19971008				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 acetylcysteine acetylcysteine	Oral	/	-	
2 adalat retard nifedipine	Oral	/	-	
3 adalat t 10 nifedipine	Oral	/	-	
4 antra 20 omeprazole	Intravenous (not otherwis	/	-	
5 heparin-natrium heparin sodium	Intravenous (not otherwis	/	-	
6 nystatin nystatin	Oral	/	-	
7 paracetamol paracetamol	Rectal	/	-	
8 spizef cefotiam hydrochloride	Intravenous (not otherwis	/	-	
9 tramal tropfen tramadol hydrochloride	Oral	/	-	

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Report from study

case: DE-BFARM-98007055

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 26 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Blisters	19971215				Unknown
2 Red eye	19971215				Unknown
3 Buccal inflammation	19971216				Unknown
4 Exanthema	19971215				Unknown
5 Fever	19971212				Unknown
6 Stevens Johnson syndrome	19971115				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 acc long acetylcysteine	Oral	/	-	-
2 ciprobay ciprofloxacin hydrochloride	Intravenous (not otherwis	/	-	-
3 doreperol n hexetidine	Buccal	/	-	-
4 erythrocin erythromycin	Intravenous (not otherwis	/	-	-
5 grippostad c caffeine; chlorphenamine maleate; paracetamol	Oral	/	-	-
6 novalgin tropfen metamizole sodium	Oral	/	-	-
7 paracetamol paracetamol	Rectal	/	-	-
8 polyspectran tropfen gramicidin; neomycin sulfate; polymyxin b sulfate	Intraocular	/	-	-
9 rocephin ceftriaxone sodium	Intravenous (not otherwis	/	-	-
10 zinacef cefuroxime	Intravenous (not otherwis	/	-	-
11 zovirax aciclovir	Intravenous (not otherwis	/	-	-

Primary source

Qualification: Physician observestudytype: Other studies

Literature:

FIRST LOOK

Country: Deutschland

Type: Report from study

case: DE-BFARM-98007058

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes							

Patient

Initials:	Sex: Female	Birth: privacy	Age: 84 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Exanthema	19970806				Unknown
2 Toxic epidermal necrolysis	19970806				Unknown
3 Fever	19970809				Unknown
4 Lip swelling	19970808				Unknown
5 Stomatitis	19970808				Unknown
6 Stevens Johnson syndrome	19970806				Unknown
7 Swallowing impaired	19970806				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 bvk roche plus c biotin; calcium pantothenate; cyanocobalamin; nicotinamide; pyridoxine hydrochloride; riboflavin; thiamine mononitrate	Unknown	/	-	-
2 diflucan 200 fluconazole	Oral	/	-	-
3 novalgine metamizole	Oral	/	-	-
4 paracetamol 500 "bayer" paracetamol	Oral	/	-	-
5 saroten amitriptyline hydrochloride	Oral	/	-	-
6 stangyl tropfen trimipramine mesilate	Oral	/	-	-
7 tramal tramadol hydrochloride	Oral	/	-	-
8 zentropil phenytoin	Oral	/	-	-

Primary source

Qualification:	Physician	observestudytype:	Other studies
Literature:			

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-98007002

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 26 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Erythema multiforme severe	19970414				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Kodeinphosphat codeine phosphate	Oral	/	-	-
2 paracetamol paracetamol	Oral	/	-	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-98007003

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes							

Patient

Initials:	Sex: Male	Birth: privacy	Age: 67 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Oral mucosal eruption	19970503				Unknown
2 Stevens Johnson syndrome	19970503				Unknown
3 Toxic epidermal necrolysis	19970503				Unknown
4 Conjunctivitis	19970503				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 arthro akut	Oral	/	-	
lonazolac calcium			-	
2 duraglucon n mite	Oral	/	-	
glibenclamide			-	
3 furo 40 von ct	Oral	/	-	
furosemide			-	
4 paracetamol	Oral	/	-	
paracetamol			-	

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-98007010

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 7 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Canker sores oral	19970608				Unknown
2 Conjunctivitis	19970610				Unknown
3 Cough	19970608				Unknown
4 Pyrexia abnormal	19970608				Unknown
5 Stevens Johnson syndrome	19970608				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ben-u-ron paracetamol	Rectal	/	-	-
2 fluimucil granulat acetylcysteine	Oral	/	-	-
3 herviros aminoquinuride hydrochloride; tetracaine hydrochloride	Dental	/	-	-
4 infectomycin plus saft erythromycin estolate	Oral	/	-	-
5 monomycin-saft erythromycin ethylsuccinate	Oral	/	-	-
6 paracetamol paracetamol	Rectal	/	-	-
7 sultanol salbutamol	Respiratory (inhalation)	/	-	-
8 wick vaporub camphor; eucalyptus oil; menthol; turpentine oil	Respiratory (inhalation)	/	-	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-98007011

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 8 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Erythema multiforme severe	19970622				Unknown
2 Fever	19970621				Unknown
3 Conjunctivitis	19970623				Unknown
4 Buccal inflammation	19970622				Unknown
5 Elevated sedimentation rate	19970622				Unknown
6 CRP positive	19970622				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 paracetamol	Rectal	/	-	
paracetamol			-	
2 penicillin-v	Oral	/	-	
phenoxymethylpenicillin			-	

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-98007394

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 4 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19970821				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ben-u-ron 250 mg paracetamol	Rectal	/	-	-
2 myxofat 100 acetylcysteine	Oral	/	-	-
3 panoral tropfen cefaclor monohydrate	Oral	/	-	-
4 sedotussin 20 mg pentoxyverine	Rectal	/	-	-

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-98007422

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 8 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Erythema multiforme severe	19971107				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ass acetylsalicylic acid	Oral	/	-	-
2 ben-u-ron saft paracetamol	Oral	/	-	-
3 monomycin-saft erythromycin ethylsuccinate	Oral	/	-	-
4 mucosolvan saft ambroxol hydrochloride	Oral	/	-	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-98008071

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes	Yes	Yes					

Patient

Initials:	Sex: Female	Birth: privacy	Age: 17 Year	weight (kg): 54	BMI_int:	Age fetus:
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Patientdeath

deathdate: 19960829	deathcause: Shock without mention of trauma
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Multiple organ failure	19960829				Unknown
2 Shock septic	19960829				Unknown
3 Stevens Johnson syndrome	19960704				Unknown
4 Toxic epidermal necrolysis	19960704				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
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1	amphotericin b amphotericin b	Intravenous (not otherwis /	-	Other and unspecified mycoses
2	antra omeprazole	Intravenous (not otherwis /	-	Chronic ulcer of skin, unspecified site
3	atosil promethazine hydrochloride	Intravenous (not otherwis /	-	Nervousness
4	ben-u-ron paracetamol	Oral /	-	Other general symptoms
5	ciprobay ciprofloxacin hydrochloride	Intravenous (not otherwis /	-	Unspecified immunity deficiency
6	cotrimoxazol sulfamethoxazole; trimethoprim	Oral /	-	Unspecified infectious and parasitic diseases
7	cymeven ganciclovir sodium	Intravenous (not otherwis /	-	Unspecified immunity deficiency
8	diazepam diazepam	Unknown /	-	Unspecified psychophysiological
9	diflucan fluconazole	Oral /	-	Candidiasis of unspecified site
10	dolantin pethidine hydrochloride	Intravenous (not otherwis /	-	Other general symptoms
11	dormicum midazolam maleate	Intravenous (not otherwis /	-	Nervousness
12	fortum ceftazidime	Intravenous (not otherwis /	-	Unspecified immunity deficiency
13	gernebcin tobramycin sulfate	Intravenous (not otherwis /	-	Unspecified immunity deficiency
14	lactulose lactulose	Oral /	-	Constipation
15	lasix furosemide	Intravenous (not otherwis /	-	Oedema
16	liquemin heparin	Intravenous (not otherwis /	-	Coagulation defects, other and unspecified
17	meronem meropenem	Intravenous (not otherwis /	-	Unspecified immunity deficiency
18	neupogen filgrastim	Subcutaneous /	-	Agranulocytosis
19	neurocil levomepromazine maleate	Intravenous (not otherwis /	-	Anxiety states
20	novalgin metamizole	Intravenous (not otherwis /	-	Pyrexia of unknown origin
21	paspertin tropfen metoclopramide hydrochloride	Oral /	-	Nausea and vomiting
22	pipril piperacillin sodium	Intravenous (not otherwis /	-	Unspecified immunity deficiency
23	prednisolon-21-hydrogensuccinat natriumsalz prednisolone sodium succinate	Intravenous (not otherwis /	-	Unspecified disorder of immune mechanism
24	rekawan potassium chloride	Oral /	-	Potassium deficiency
25	targocid teicoplanin	Intravenous (not otherwis /	-	Unspecified immunity deficiency
26	tavegil clemastine	Intravenous (not otherwis /	-	Unspecified pruritic disorder
27	vancomycin vancomycin	Intravenous (not otherwis /	-	Unspecified immunity deficiency
28	zantic injektionsloesung ranitidine hydrochloride	Intravenous (not otherwis /	-	Chronic ulcer of skin, unspecified site

29 [zienam](#)

Intravenous (not otherwis /

-

Unspecified immunity deficiency

cilastatin sodium; imipenem

-

Medical History and concurrent conditions

MedDRA-term

Begin

End

Cont.

Comment

Myeloid leukaemia, chronic

k.A.

concomitant disease

Primary source

Qualification:

Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-98008088

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 26 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Exanthema	19960529				Unknown
2 Erythema multiforme severe	19960529				Unknown
3 Conjunctivitis	19960529				Unknown
4 Bullous eruption	19960529				Unknown
5 Buccal inflammation	19960529				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 aspirin acetylsalicylic acid	Oral	/	-	-
2 mucosolvan ambroxol hydrochloride	Oral	/	-	-
3 paracetamol paracetamol	Oral	/	-	-
4 paracodin dihydrocodeine bitartrate	Oral	/	-	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-98008095

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes	Yes	Yes					

Patient

Initials:	Sex: Female	Birth: privacy	Age: 34 Year	weight (kg): 54	BMI_int: 18.04	Age fetus:
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Patientdeath

deathdate:	deathcause: Shock without mention of trauma
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Cheilitis	19960824				Unknown
2 Blistering	19960819				Unknown
3 Stevens Johnson syndrome	19960819		7 Day		Unknown
4 Shock circulatory	19960824				Unknown
5 Toxic epidermal necrolysis	19960819				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
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1	actrapid human insulin human	Subcutaneous	/	-	Abnormal glucose tolerance test
2	allopurinol 300 mg allopurinol	Oral	/	-	Hyperuricaemia
3	amphotericin b amphotericin b	Intravenous (not otherwise)	/	-	Other and unspecified mycoses
4	ancotil flucytosine	Intravenous (not otherwise)	/	-	Other and unspecified mycoses
5	antra pro infusione omeprazole sodium	Intravenous (not otherwise)	/	-	Chronic ulcer of skin, unspecified site
6	augmentan amoxicillin trihydrate; clavulanic acid	Intravenous (not otherwise)	/	-	Unspecified immunity deficiency
7	bifiteral lactulose	Oral	/	-	Constipation
8	ciprobay ciprofloxacin hydrochloride	Intravenous (not otherwise)	/	-	Unspecified immunity deficiency
9	claforan cefotaxime sodium	Intravenous (not otherwise)	/	-	Unspecified immunity deficiency
10	diflucan 200 fluconazole	Oral	/	-	Candidiasis of unspecified site
11	dipidolor piritramide	Intravenous (not otherwise)	/	-	Other general symptoms
12	dolantin pethidine hydrochloride	Intravenous (not otherwise)	/	-	Other general symptoms
13	dopamin dopamine hydrochloride	Intravenous (not otherwise)	/	-	Shock without mention of trauma
14	dormicum midazolam maleate	Intravenous (not otherwise)	/	-	Unspecified psychophysiological
15	gentamycin-ampullen gentamicin sulfate	Intravenous (not otherwise)	/	-	Unspecified immunity deficiency
16	imodium loperamide hydrochloride	Oral	/	-	Diarrhoea
17	isoptin injektionsloesung verapamil hydrochloride	Intravenous (not otherwise)	/	-	Essential hypertension, unspecified
18	lasix furosemide	Oral	/	-	Volume depletion
19	leucomax 300 molgramostim	Subcutaneous	/	-	Unspecified immunity deficiency
20	liquemin heparin	Subcutaneous	/	-	Thrombosis
21	meronem meropenem	Intravenous (not otherwise)	/	-	Unspecified immunity deficiency
22	methotrexat methotrexate	Intrathecal	/	19960730-	Burkitt's tumour
23	metronidazol-ratiopharm 500 metronidazole	Intravenous (not otherwise)	/	-	Unspecified immunity deficiency
24	msi mundipharma morphine sulfate	Intravenous (not otherwise)	/	-	Other general symptoms
25	multibionta zur infusion ascorbic acid; dextranthenol; nicotinamide; pyridoxine hydrochloride; retinol palmitate; riboflavin sodium phosphate; thiamine hydrochloride; tocopheryl acetate	Intravenous (not otherwise)	/	-	Unspecified vitamin deficiency
26	neupogen filgrastim	Subcutaneous	/	-	Agranulocytosis
27	novalgin metamizole	Intravenous (not otherwise)	/	-	Other general symptoms

28	opium tropfen opium alkaloids total	Oral	/	-	Other general symptoms
29	pantozol pantoprazole sodium	Oral	/	-	Chronic ulcer of skin, unspecified site
30	paracetamol paracetamol	Oral	/	-	Other general symptoms
31	paspertin metoclopramide hydrochloride	Intravenous (not otherwis	/	-	Unspecified functional disorder of stomach
32	psyquil triflupromazine	Intravenous (not otherwis	/	-	Unspecified psychophysiological
33	sobelin soluble clindamycin phosphate	Intravenous (not otherwis	/	-	Unspecified immunity deficiency
34	solu-decortin h 50 mg prednisolone sodium succinate	Intravenous (not otherwis	/	-	Unspecified pruritic disorder
35	suprarenin epinephrine	Intravenous (not otherwis	/	-	Shock without mention of trauma
36	tavegil clemastine	Intravenous (not otherwis	/	-	Unspecified pruritic disorder
37	vancomycin vancomycin	Intravenous (not otherwis	/	-	Unspecified immunity deficiency
38	vitalipid vitamins nos	Intravenous (not otherwis	/	-	Unspecified nutritional deficiency
39	zienam cilastatin sodium; imipenem	Intravenous (not otherwis	500 Mg milligram(s)/	19960816-19960816	Unspecified immunity deficiency
40	zofran ondansetron hydrochloride	Intravenous (not otherwis	/	-	Nausea and vomiting

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Pneumonia in aspergillosis			k.A.	concomitant disease
Aspergillosis			k.A.	concomitant disease
Burkitt's tumour			k.A.	concomitant disease

Primary source

Qualification: Physician observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Report from study

case: DE-BFARM-98008110

Serious Yes	Death	Lifethr.	Hospital.	Disabling	Congenital	Other Yes	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Male	Birth: privacy	Age: 19 Month	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19961213				Unknown
2 Angioedema	19961213				Unknown
3 Ascites	19961213				Unknown
4 Mouth irritation	19961213				Unknown
5 Respiratory system disorder	19961213				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
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1	acc acetylcysteine	Intravenous (not otherwis /	-
2	aldactone spironolactone	Intravenous (not otherwis /	-
3	ambisome amphotericin b	Intravenous (not otherwis /	-
4	arterenol norepinephrine hydrochloride	Intravenous (not otherwis /	-
5	bactrim sulfamethoxazole; trimethoprim	Intravenous (not otherwis /	-
6	dopamin dopamine hydrochloride	Intravenous (not otherwis /	-
7	fentanyl fentanyl	Intravenous (not otherwis /	-
8	fluconazole fluconazole	Oral /	-
9	fortum ceftazidime	Intravenous (not otherwis /	-
10	hydromedin etacrynic acid	Intravenous (not otherwis /	-
11	lasix furosemide	Intravenous (not otherwis /	-
12	luminal phenobarbital	Intravenous (not otherwis /	-
13	methotrexat-dinatrium methotrexate sodium	Intravenous (not otherwis /	-
14	morphine hydrochloride morphine hydrochloride	Intravenous (not otherwis /	-
15	mucosolvan ambroxol hydrochloride	Intravenous (not otherwis /	-
16	novalgin metamizole	Intravenous (not otherwis /	-
17	pancuronium pancuronium	Intravenous (not otherwis /	-
18	paracetamol paracetamol	Oral /	-
19	phenhydan phenytoin	Unknown /	-
20	prostavasin alprostadiil	Intravenous (not otherwis /	-
21	racementhine-hydrochlorid epinephrine hydrochloride	Respiratory (inhalation) /	-
22	sobelin clindamycin hydrochloride	Intravenous (not otherwis /	-
23	soluvit vitamins nos	Intravenous (not otherwis /	-
24	TOBRAMYCIN tobramycin	intraperitoneal /	-
25	triglobe soft sulfadiazine; trimethoprim	Oral /	-
26	Urbason methylprednisolone	Intravenous (not otherwis /	-
27	vitintra ergocalciferol; phytomenadione; retinol; retinol palmitate	Intravenous (not otherwis /	-
28	vp-16 etoposide	Intravenous (not otherwis /	-

29	vp-16 etoposide	Intravenous (not otherwis /	-
30	zantic ranitidine hydrochloride	Intravenous (not otherwis /	-
31	zienam cilastatin sodium; imipenem	Intravenous (not otherwis /	-
32	zovirax aciclovir	Intravenous (not otherwis /	-

Primary source

Qualification: Physician observestudytype: Other studies
 Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-98008111

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes							

Patient

Initials:	Sex: Male	Birth: privacy	Age: 58 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19961213				Unknown
2 Toxic epidermal necrolysis	19961213				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 adumbran	Oral	/	-	
oxazepam			-	
2 anafranil 25	Oral	/	-	
clomipramine hydrochloride			-	
3 ben-u-ron	Oral	/	-	
paracetamol			-	
4 bepanthen roche tabletten	Oral	/	-	
panthenol			-	
5 dulcolax	Rectal	/	-	
bisacodyl			-	
6 heparin	Subcutaneous	/	-	
heparin			-	
7 hexoraletten	Buccal	/	-	
benzocaine; chlorhexidine hydrochloride; sodium ascorbate			-	
8 nizatidine	Oral	/	-	
nizatidine			-	
9 zentropil	Oral	/	-	
phenytoin			-	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-98008117

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 37 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Erythema multiforme severe	19970122				Unknown
2 Red eye	19970122				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 acc acetylcysteine	Oral	/	-	-
2 amoxicillin amoxicillin	Oral	/	-	-
3 codipront codeine; phenyltoloxamine	Oral	/	-	-
4 doxycycline doxycycline	Oral	/	-	-
5 paracetamol paracetamol	Oral	/	-	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-98008314

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 59 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Red eye	19970429				Unknown
2 Stevens Johnson syndrome	19970429				Unknown
3 Mucosal inflammation	19970429				Unknown
4 Exanthema	19970429				Unknown
5 Blistering	19970429				Unknown
6 Toxic epidermal necrolysis	19970429				Unknown
7 Stomatitis	19970429				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 caffeine caffeine	Oral	/	-	-
2 diclofenac diclofenac	Intramuscular	/	-	-
3 novalgin metamizole	Oral	/	-	-
4 paracetamol paracetamol	Oral	/	-	-
5 propyphenazone propyphenazone	Oral	/	-	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-98008320

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 9 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19961114				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 globocef cefetamet pivoxil hydrochloride	Oral	/	-	-
2 paracetamol paracetamol	Oral	/	-	-
3 timonil retard carbamazepine	Oral	/	-	-

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-98008321

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 67 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Toxic epidermal necrolysis	19970828				Unknown
2 Stevens Johnson syndrome	19970828				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ass 100 acetylsalicylic acid	Oral	/	-	-
2 eusaprim sulfamethoxazole; trimethoprim	Oral	/	-	-
3 furosemid furosemide	Oral	/	-	-
4 paracetamol paracetamol	Unknown	/	-	-
5 tramal tropfen tramadol hydrochloride	Oral	/	-	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-98008326

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 88 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 SGOT increased	19961226				Unknown
2 Stevens Johnson syndrome	19961230				Unknown
3 Diarrhea	19970101				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
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1	aldactone 100 spironolactone	Oral	/	-
2	ben-u-ron paracetamol	Oral	/	-
3	bvk roche for injection biotin; cyanocobalamin; nicotinamide; panthenol; pyridoxine hydrochloride; riboflavin sodium phosphate; thiamine hydrochloride	Intravenous (not otherwis	/	-
4	cefuroxime cefuroxime	Intravenous (not otherwis	/	-
5	cytotec 200 misoprostol	Oral	/	-
6	digimerck digitoxin	Intravenous (not otherwis	/	-
7	heparin heparin	Unknown	/	-
8	humatin paromomycin sulfate	Oral	/	-
9	lactulose lactulose	Oral	/	-
10	lasix furosemide	Oral	/	-
11	lopirin cor captopril	Oral	/	-
12	multibionta ascorbic acid; calcium pantothenate; cyanocobalamin; nicotinamide; pyridoxine hydrochloride; retinol palmitate; riboflavin; rutoside; thiamine hydrochloride; tocopheryl acetate	Intravenous (not otherwis	/	-
13	osyrol 100 - lasix furosemide; spironolactone	Oral	/	-
14	refobacin gentamicin sulfate	Intravenous (not otherwis	/	-
15	tarivid ofloxacin	Intravenous (not otherwis	/	-
16	vitintra ergocalciferol; phytomenadione; retinol; retinol palmitate	Intravenous (not otherwis	/	-
17	zienam cilastatin sodium; imipenem	Intravenous (not otherwis	/	-

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-98008650

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes							

Patient

Initials:	Sex: Female	Birth: privacy	Age: 86 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Toxic epidermal necrolysis	19980926				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 acercomp mite	Oral	/	-	
hydrochlorothiazide; lisinopril dihydrate			-	
2 Acetylsalizylsaeure	Oral	/	-	
acetylsalicylic acid			-	
3 paracetamol	Oral	/	-	
paracetamol			-	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-98008658

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 1 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19980318				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ben-u-ron paracetamol	Rectal	/	-	-
2 gelonida na saft acetylsalicylic acid; codeine phosphate; paracetamol	Oral	/	-	-
3 nystatin "lederle" nystatin	Oral	/	-	-
4 staphylex infusion-loesung flucloxacillin sodium	Intravenous (not otherwis	/	-	-
5 staphylex trockensaft flucloxacillin sodium	Oral	/	-	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-98008663

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes	No	Yes	No	No	No		

Patient

Initials:	Sex: Male	Birth: privacy	Age: 61 Year	weight (kg): 86	BMI_int: 28.08	Age fetus:
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Patientdeath

deathdate: 19961001	deathcause: Unknown cause of death
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Unknown cause of death	19961001				fatal
2 Buccal inflammation	19960913				Unknown
3 Necrolysis epidermal	19960913				Unknown
4 Blisters	19960913				Unknown
5 Exanthema	19960913				Unknown
6 Stevens Johnson syndrome	19960913				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
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DE-BFARM-98008663

1	ALEXAN cytarabine	Intravenous (not otherwis / Injection	19960829-19960905 8 Day	Acute myelomonocytic leukaemia
2	ALLOPURINOL allopurinol	Oral / Tablet	19960831- -	Hyperuricemia
3	ALLOPURINOL allopurinol	Oral / Tablet	19960823-19960825 3 Day	Hyperuricemia
4	ARA-CELL cytarabine	Intrathecal / Injection	19960910-19960913 4 Day	Acute myelomonocytic leukaemia
5	ATOSIL promethazine	Intravenous (not otherwis / Injection	19960830-19960830 -	Unrest
6	CALCIMAGON calcium carbonate	Oral 2*1 Df dosage form/ Day Tablet	19960905-19960907 3 Day	Hypocalcaemia
7	CEFTRIAXONE SODIUM ceftriaxone	Intravenous (not otherwis 2 G gram(s)/ Day Injection	-19960825 -	Drug use for unknown indication
8	COTRIM FORTE sulfamethoxazole; trimethoprim	Oral 3*1 Df dosage form/ Day Tablet	19960909-19960909 -	Drug use for unknown indication
9	COTRIM FORTE sulfamethoxazole; trimethoprim	Oral 3*1 Df dosage form/ Day Tablet	19960905-19960905 -	Drug use for unknown indication
10	DAUNORUBICIN daunorubicin	Intravenous (not otherwis / Injection	19960824-19960826 3 Day	Acute myelomonocytic leukaemia
11	DAUNORUBICIN daunorubicin	Intravenous (not otherwis / Injection	19960903-19960905 3 Day	Acute myelomonocytic leukaemia
12	dexamethason dexamethasone	Intravenous (not otherwis / Injection	19960910-19960910 -	Drug use for unknown indication
13	DEXAMETHASON dexamethasone	Intravenous (not otherwis / Injection	19960913-19960913 -	Drug use for unknown indication
14	EPIDROPAL allopurinol	Oral 300 Mg milligram(s)/ Day Tablet	19960824-19960826 3 Day	Acute myelomonocytic leukaemia
15	LASIX furosemide	Intravenous (not otherwis 40 Mg milligram(s)/ Hour Injection	19960830- -	Water retention
16	LASIX furosemide	Intravenous (not otherwis 40 Mg milligram(s)/ Hour Injection	19960826-19960828 3 Day	Water retention
17	MERONEM meropenem	Intravenous (not otherwis 3*500 Mg milligram(s)/ Da Injection	19960825- -	Drug use for unknown indication
18	methotrexat methotrexate	Intrathecal /	- -	
19	morphine sulfate morphine sulfate	Rectal /	- -	
20	MSR morphine sulfate	Rectal 2*1 Df dosage form/ Day Suppository	19960826-19960830 5 Day	Pain
21	MTX INJECTION methotrexate	Intrathecal / Injection	19960910-19960913 4 Day	Acute myelomonocytic leukaemia
22	MULTIBIONTA vitamins nos	Intravenous (not otherwis / Injection	19960830- -	Vitamin deficiency
23	mupirocin mupirocin	Nasal 3* / Day Ointment	19960906- -	Drug use for unknown indication
24	NOVALGIN metamizole	Oral / Oral drops	19960826-19960826 -	Pain
25	PARACETAMOL paracetamol	Rectal 2*1 Df dosage form/ Day Suppository	19960907-19960910 4 Day	Drug use for unknown indication
26	PIMAFUCIN natamycin	Oral 4*2 Df dosage form/ Day Tablet	19960904-19960907 4 Day	Candida of mouth
27	REFOBACIN gentamicin	Intravenous (not otherwis 240 Mg milligram(s)/ Day Injection	-19960825 -	Drug use for unknown indication
28	RIOPAN magaldrate	Oral 4* / Day Gel	19960913-19960913 -	Stomach function disorder

29	rocephin ceftriaxone sodium	Intravenous (not otherwis /	-	-
30	SAROTEN amitriptyline	Intravenous (not otherwis / Injection	19960907-19960910 4 Day	Depression
31	TRAMAL tramadol	Oral / Oral drops	19960904-19960904	Pain
32	vancomycin vancomycin	Intravenous (not otherwis / Injection	19980902-19980902	Drug use for unknown indication
33	vancomycin vancomycin	Intravenous (not otherwis / Injection	19960907-19960907	Drug use for unknown indication
34	VANCOMYCIN vancomycin	Intravenous (not otherwis / Injection	19960825-19960827 3 Day	Drug use for unknown indication
35	vitintra ergocalciferol; phytomenadione; retinol; retinol palmitate	Intravenous (not otherwis / Injection	19960830- -	Drug use for unknown indication
36	ZYLORIC allopurinol	Oral 300 Mg milligram(s)/ Day Tablet	-19960823 -	Hyperuricemia

Drugs (concomitant)

37	aminomix amino acids nos; minerals nos	Intravenous (not otherwis 80 ml millilitre(s)/ Hour Injection	19960823- -	Parenteral nutrition
38	ampho-moronal amphotericin b	Oral 6*1 Df dosage form/ Day Oral suspension	19960823-19960828 6 Day	Candidal infection NOS
39	CYTARABIN cytarabine	Intravenous (not otherwis / Injection	19960824-19960830 7 Day	Acute myelomonocytic leukaemia
40	DIARROENT MONO colistin	Oral 4*1 Df dosage form/ Day Tablet	19960823-19960907 16 Day	GI infection
41	kalium-duriles potassium chloride	Oral 2 Df dosage form/ Day Tablet	-19960823 -	Hypokalemia
42	kcl potassium chloride	Intravenous (not otherwis / Injection	19960823-19960827 5 Day	Dialysis
43	lipofundin glycerol; lecithin; soya oil	Intravenous (not otherwis / Injection	19960830- -	Drug therapy
44	LIQUEMIN heparin sodium	Intravenous (not otherwis 400 lu international unit(s) Injection	19960823-19960905 14 Day	Thrombosis prophylaxis
45	MUCOSOLVAN ambroxol hydrochloride	Intravenous (not otherwis 2*1 Df dosage form/ Day Injection	-19960823 -	
46	nacl sodium chloride	Intravenous (not otherwis / Injection	19960830- -	Dehydration
47	PEPDUL famotidine	Intravenous (not otherwis / Injection	19960823- -	Prophylaxis
48	sterofundin electrolytes nos	Intravenous (not otherwis 80 ml millilitre(s)/ Hour Injection	19960823-19960902 11 Day	
49	ZOFRAN ondansetron	Intravenous (not otherwis / Injection	19960823-19960828 6 Day	Nausea

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Tonsillectomy			No	
Acute myelomonocytic leukaemia			Unknown	
Coronary disease			Unknown	
Hypokalemia			Unknown	
Peritonsillar abscess			Unknown	
Renal failure			Unknown	

Primary source

Qualification:	Physician	observestudytype:	Other studies
Literature:			

Narratives (nur bei elektronischem Eingang)

ROCHE, 26.09.2008 DE-ROCHE-111229

Sender diagnosis: not reported

Narrative

INTENSIVE MONITORING STUDY TEN/SJS, REPORTED BY A PHYSICIAN AND BUNDESINSTITUT FUER ARZNEIMITTEL UND MEDIZINPRODUKTE (GERMANY, REFERENCE: 98008663).

A 61-YEAR-OLD MALE PATIENT (PT # 717153) EXPERIENCED STEVENS JOHNSON SYNDROME, WHICH LED TO PROLONGED HOSPITALISATION AND DIED DUE TO AN UNKNOWN CAUSE FOLLOWING THE USE OF ROCEPHIN (CEFTRIAXONE) FOR AN UNKNOWN INDICATION. FURTHER SUSPECT DRUGS WERE REFOBACIN (GENTAMYCIN), ZYLORIC (ALLOPURINOL), ALLOPURINOL TABLETS, EPIDROPAL (ALLOPURINOL), DAUNORUBICIN, MERONEM (MEROPENEM), VANCOMYCIN, LASIX (FUROSEMIDE), MSR SUPPOSITORIES (MORPHINE SULPHATE), NOVALGIN (METAMIZOLE SODIUM), AN UNKNOWN CONCOMITANT DRUG FOR CYTOSTATIC TREATMENT, DEXAMETHASON (DEXAMETHASONE), MULTIBIONTA (ASCORBIC ACID/COLECALCIFEROL/DEXPANTHENOL/NICOTINAMIDE/PYRIDOXINE/RETINOL/RIBOFLAVIN SODIUM PHOSPHATE/THIAMINE/VITAMINE E), VITINTRA (ERGOCALCIFEROL/PHYTOMENADIONE/RETINOL), ALEXAN (CYTARABINE), ATOSIL (PROMETHAZINE), PIMAFUCIN (NATAMYCIN), TRAMAL (TRAMADOL), COTRIM FORTE (SULFAMETHOXAZOLE/TRIMETHOPRIM), CALCIMAGON (CALCIUM CARBONATE), MSL NASAL OINTMENT, PARACETAMOL, SAROTEN (AMITRIPTYLINE), ARA-CELL (CYTARABINE) INTRATHECAL, MTX (METHOTREXATE) AND RIOPAN (MAGALDRATE).

MEDICAL HISTORY INCLUDED CORONARY HEART DISEASE, RENAL FAILURE, HYPOKALAEMIA, ACUTE MYELOMONOCYTIC LEUKAEMIA (DIAGNOSED IN AUG 96), TONSILLECTOMY DUE TO PERITONSILLAR ABSCESS ON 14 AUG 96.

ON AN UNSPECIFIED DATE ROCEPHIN IV 2G OD AND REFOBACIN 240 MG OD WERE STARTED FOR AN UNKNOWN INDICATION. ZYLORIC 300 MG OD WAS STARTED FOR HYPERURICAEMIA.

16 AUG 96: THE PATIENT WAS HOSPITALISED FOR ACUTE MYELOMONOCYTIC LEUKAEMIA.

23 AUG 96: ZYLORIC WAS STOPPED. ALLOPURINOL PO (DOSE NOT STATED) WAS STARTED FOR HYPERURICAEMIA.

24 AUG 96: EPIDROPAL PO 300 MG OD AND DAUNORUBICIN IV 1 G/DAY WERE STARTED FOR ACUTE MYELOMONOCYTIC LEUKAEMIA.

25 AUG 96: ROCEPHIN, REFOBACIN AND ALLOPURINOL WERE STOPPED. MERONEM IV 500 MG TID AND VANCOMYCIN IV 1 G/DAY WAS STARTED FOR AN UNKNOWN INDICATION.

26 AUG 96: EPIDROPAL AND DAUNORUBICIN WERE STOPPED. LASIX IV 40 MG/H FOR OEDEMA AND MSR 2 X 1 SUPPOSITORIES FOR PAIN WERE STARTED. THE PATIENT RECEIVED A SINGLE DOSE OF NOVALGIN (30 DROPS).

27 AUG 96: VANCOMYCIN WAS STOPPED. AN UNKNOWN CONCOMITANT IV DRUG FOR CYTOSTATIC TREATMENT WAS STARTED.

28 AUG 96: THE UNKNOWN DRUG AND LASIX WERE STOPPED.

29 AUG 96: THE PATIENT RECEIVED A SINGLE DOSE (4 MG/DAY) OF DEXAMETHASON IV. ALEXAN IV 200 MG/DAY WAS STARTED FOR ACUTE MYELOMONOCYTIC LEUKAEMIA.

30 AUG 96: MSR SUPPOSITORIES WERE STOPPED. MULTIBIONTA IV 1 AMP/DAY FOR SUBSTITUTION OF VITAMINES AND VITINTRA IV 1 AMP/DAY FOR AN UNKNOWN INDICATION WERE STARTED. THE PATIENT RECEIVED A SINGLE DOSE OF ATOSIL IV (1 AMP/DAY) FOR RESTLESSNESS. LASIX IV 40 MG/H WAS RESTARTED.

31 AUG 96: ALLOPURINOL PO (DOSE NOT STATED) WAS RESTARTED.

2 SEP 96: THE PATIENT RECEIVED A SINGLE DOSE OF VANCOMYCIN IV (1 G/DAY).

3 SEP 96: DAUNORUBICIN IV 100 MG/DAY WAS RESTARTED.

4 SEP 96: THE PATIENT RECEIVED A SINGLE DOSE OF TRAMAL PO (20 DROPS/DAY) FOR PAIN. PIMAFUCIN PO 4 X 2 TABLETS/DAY WAS STARTED FOR ORAL CANDIDIASIS.

5 SEP 96: DAUNORUBICIN AND ALEXAN WERE STOPPED. THE PATIENT RECEIVED A ONE DAY COURSE OF COTRIM FORTE PO (1 TABLET TID) FOR AN UNKNOWN INDICATION. CALCIMAGON PO 1 TABLET BID WAS STARTED FOR HYPOCALCAEMIA.

6 SEP 96: MSL NASAL OINTMENT 3 X/DAY WAS STARTED FOR AN UNKNOWN INDICATION.

7 SEP 96: PIMAFUCIN AND CALCIMAGON WERE STOPPED. PARACETAMOL SUPPOSITORIES 2 - 3/DAY WERE STARTED FOR AN UNKNOWN INDICATION. THE PATIENT RECEIVED ANOTHER SINGLE DOSE OF VANCOMYCIN IV (1G/DAY).

8 SEP 96: SAROTEN IV 25 MG/DAY WAS STARTED FOR DEPRESSION.

9 SEP 96: THE PATIENT RECEIVED ANOTHER ONE DAY COURSE OF COTRIM FORTE PO (1 TABLET TID).

10 SEP 96: PARACETAMOL AND SAROTEN WERE STOPPED. THE PATIENT RECEIVED A SINGLE DOSE OF DEXAMETHASON IV (4 MG/DAY), A SINGLE DOSE OF ARA-CELL INTRATHECAL (40 MG/DAY) AND A SINGLE DOSE OF MTX INTRATHECAL (15 MG) FOR ACUTE MYELOMONOCYTIC LEUKAEMIA.

13 SEP 96: THE PATIENT RECEIVED A SINGLE DOSE OF DEXAMETHASON IV (4 MG/DAY), A SINGLE DOSE OF ARA-CELL INTRATHECAL (40 MG/DAY), A SINGLE DOSE OF MTX INTRATHECAL (15 MG/DAY) AND A ONE DAY COURSE OF RIOPAN (1 SACHET TID) FOR STOMACH COMPLAINTS. HE PRESENTED WITH EXANTHEMA AND BLISTERS.

IT WAS NOT STATED, WHETHER ALLOPURINOL, MERONEM, LASIX, MULTIBIONTA, VITINTRA AND MSL NASAL OINTMENT WERE MAINTAINED AFTER 13 SEP 96.

16 SEP 96: CREATININE WAS 5.7 MG/DL, UREA WAS 111 MG/DL. NO NORMAL RANGES WERE PROVIDED.

19 SEP 96: THE PATIENT PRESENTED WITH EROSION GENITAL MUCOSA.

21 SEP 96: THE PATIENT PRESENTED WITH EPIDERMOLYSIS.

24 SEP 96: THE PATIENT PRESENTED WITH EROSION ORAL MUCOSA. SYMPTOMATIC TREATMENT WAS GIVEN.

1 OCT 96 (EST): THE PATIENT DIED DUE TO AN UNKNOWN CAUSE.

JAN 97: A TEAM OF EXPERTS ASSESSED THE CASE RETROSPECTIVELY AND DESCRIBED IT AS CERTAIN CASE OF STEVENS JOHNSON SYNDROME. THE EVENT OF STEVENS JOHNSON SYNDROME WAS CONSIDERED SERIOUS AS IT REQUIRED HOSPITALISATION.

NO INFORMATION WAS PROVIDED ON THE OUTCOME OF STEVENS JOHNSON SYNDROME.

NO DRUG RELATIONSHIP FOR THE SUSPECT DRUGS WAS PROVIDED FOR EVENT OF DEATH OF UNKNOWN CAUSE. THE REPORTER LISTED THE DRUG RELATIONSHIP FOR STEVENS JOHNSON SYNDROME AS POSSIBLE FOR ROCEPHIN, REFOBACIN, DAUNORUBICIN, MERONEM, MSR SUPPOSITORIES, VANCOMYCIN, UNKNOWN DRUG, DEXAMETHASON, MULTIBIONTA, VITINTRA, ALEXAN, ATOSIL, PIMAFUCIN, CALCIMAGON, MSL NASAL OINTMENT, ARA-CELL AND RIOPAN. THE DRUG RELATIONSHIP FOR ZYLORIC, ALLOPURINOL, EPIDROPAL AND COTRIM FORTE WAS LISTED AS PROBABLE AND THE DRUG RELATIONSHIP FOR LASIX, NOVALGIN, TRAMAL, PARACETAMOL, SAROTEN AND MTX WAS LISTED AS HIGHLY POSSIBLE.

ADVERSE EVENT INFORMATION ORIGINALLY RECEIVED ON 21 DECEMBER 1998 HAS BEEN UPDATED FOR THIS CASE AS A RESULT OF THE RETROSPECTIVE IMPLEMENTATION OF A ROCHE POLICY CHANGE (EFFECTIVE JULY 2007). LABELLING FOR DEATH NOS, DEATH CAUSE UNKNOWN, OR DEATH UNEXPLAINED, HAS BEEN UPDATED. IN ADDITION, THE FOLLOWING CHANGES WERE MADE: REPORTER INFORMATION, EVENT TERM UPDATED FROM GERMAN TO 'DEATH OF UNKNOWN CAUSE', LABELLING FOR THE DRUG ALLOPURINOL HAS BEEN ASSESSED (PREVIOUSLY NOT ASSESSED).

Sender's comment:

CIOMS TEXT: STEVENS-JOHNSON SYNDROME IS LISTED WHILE DEATH OF UNKNOWN CAUSE IS NOT LISTED IN THE CDS FOR CEFTRIAXONE AND THE IDS FOR ALLOPURINOL. FURTHER INFORMATION REGARDING THE CAUSE OF DEATH WOULD ALLOW MORE COMPLETE ASSESSMENT OF THIS CASE. BASED UPON THIS SINGLE REPORT, THERE IS NO CHANGE IN THE OVERALL SAFETY PROFILE FOR THIS PRODUCT.

POSSIBLE ALTERNATIVE CAUSES FOR THE EVENT OF STEVENS-JOHNSON SYNDROME INCLUDED THE DRUGS MEROPENEM, VANCOMYCIN AND FUROSEMIDE.; ***LOCAL ASSESSMENT: SUBMITTED FOR CEFTRIAXON

INTENSIVE MONITORING STUDY TEN/ SJS
REFERENCE: 98008663

FOLLOW- UP TO 08 JAN 99

LOCAL LABEL STATUS:
STEVENS-JOHNSON SYNDROME= LABELLED
DEATH UNKNOWN CAUSE= NOT LABELLED

EINZELFALLBEOBACHTUNG/ KEIN HANDLUNGSBEDARF

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-98008667

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 10 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Erythema multiforme severe	19970911				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 amoxicillin amoxicillin	Oral	/	-	-
2 ben-u-ron paracetamol	Rectal	/	-	-
3 eryhexal soft erythromycin ethylsuccinate	Oral	/	-	-
4 fluimucil acetylcysteine	Oral	/	-	-

Primary source

Qualification: Physician observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-98008671

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 11 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19950329				Unknown
2 Fever	19950329				Unknown
3 Conjunctivitis	19950329				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ben-u-ron paracetamol	Unknown	/	- -	

Primary source

Qualification: Physician observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-98008684

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 7 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19950908				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 aspirin acetylsalicylic acid	Oral	/	-	
2 paracetamol paracetamol	Oral	/	-	

Primary source

Qualification: Physician observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99000717

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 37 Year weight (kg): 66 BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19981028				Unknown
2 Red eye	19981031				Unknown
3 SGOT increased	19981031				Unknown
4 SGPT increased	19981031				Unknown
5 Gamma GT increased	19981031				Unknown
6 LDH increased serum	19981031				Unknown
7 Cough	19981019				Unknown
8 Dysaesthesia	19981031				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 paracetamol paracetamol	Oral	003 Df dosage form/Day	19981005-19981025	Headache
2 viramune nevirapine	Oral	400 Mg milligram(s)/Day	-19981101	HIV infection NOS

Drugs (concomitant)

3 videx tabletten-150 didanosine	Oral	003 Df dosage form/Day	19981005-19981102	HIV infection NOS
4 zerit 40 mg-kapseln stavudine	Oral	002 Df dosage form/Day	19981005-19981102	HIV infection NOS

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Cellulitis and abscess of unspecified sites			k.A.	concomitant disease
Pyrexia of unknown origin			k.A.	concomitant disease
Unspecified septicemia			k.A.	concomitant disease
Lymphadenitis, unspecified, except mesenteric			k.A.	concomitant disease
Staphylococcus infection in conditions classified elsewhere			k.A.	concomitant disease

Primary source

Qualification: Physician observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99000794

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 9 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19980801				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ben-u-ron paracetamol	Rectal	/	-	-
2 clindamycin clindamycin	Intravenous (not otherwis	/	-	-

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99000798

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes					

Patient

Initials:	Sex: Male	Birth: privacy	Age: 63 Year	weight (kg): 65	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Oral mucosal eruption	19981108				Unknown
2 Leukocytosis	19981108				Unknown
3 Stevens Johnson syndrome	19981109				recovered/resolved
4 Urea blood level increased	19981108				Unknown
5 Creatinine increased	19981108				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
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1	acc acetylcysteine	Oral	/	-	Abnormal sputum
2	actrapid human insulin human	Intravenous (not otherwis	/	-	Diabetes mellitus
3	amoxicillin amoxicillin	Oral	/	-	Bronchitis, not specified as acute or chronic
4	aspisol acetylsalicylate lysine	Intravenous (not otherwis	/	-	Angina pectoris
5	ass acetylsalicylic acid	Oral	/	-	Coronary atherosclerosis
6	corvaton retard molsidomine	Oral	/	-	Coronary atherosclerosis
7	diazepam diazepam	Oral	/	-	Other and unspecified special symptoms or syndromes, not
8	ebrantil urapidil	Intravenous (not otherwis	/	19981107-19981107	Angina pectoris
9	furosemide furosemide	Intravenous (not otherwis	/	-	Heart failure, unspecified
10	Glyceroltrinitrat glyceryl trinitrate	Oral	/	-	Angina pectoris
11	heparin-natrium heparin sodium	Subcutaneous	/	-	Thrombosis
12	isoket 20 isosorbide dinitrate	Oral	/	-	Coronary atherosclerosis
13	lasix 40 furosemide	Intravenous (not otherwis	/	-	Heart failure, unspecified
14	locol 40 fluvastatin sodium	Oral	/	-	Pure hypercholesterolaemia
15	mcp-tropfen metoclopramide hydrochloride	Oral	/	-	Nausea and vomiting
16	Morphin morphine	Intravenous (not otherwis	/	-	Other general symptoms
17	novalgin tropfen metamizole sodium	Oral	/	-	Other general symptoms
18	oedemase furosemide	Oral	002 Df dosage form/Day	-	Oedema
19	optipect n tropfen ammonium chloride; camphor; eucalyptus oil; levomenthol; liquorice liquid extract; peppermint oil; salvia tincture	Oral	/	-	Cough
20	pantoprazole pantoprazole	Oral	/	-	Gastric ulcer, unspecified as acute or chronic, without
21	paracetamol paracetamol	Unknown	/	-	Pyrexia of unknown origin
22	perlinganit glyceryl trinitrate	Intravenous (not otherwis	/	-	Angina pectoris
23	zyloric 300 allopurinol	Oral	/	-	Hyperuricaemia

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99000802

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 47 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 SGPT increased	19981213				Unknown
2 LDH increased	19981213				Unknown
3 Stevens Johnson syndrome	19981212				Unknown
4 Epidermal necrolysis	19981212				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 allobeta 300 allopurinol	Oral	/	-	-
2 diclofenbeta uno diclofenac sodium	Oral	/	-	-
3 herz ass-ratiopharm acetylsalicylic acid	Oral	/	-	-
4 meditonsin h homeopatics nos	Oral	/	-	-
5 paracetamol paracetamol	Oral	/	-	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99000892

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 56 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 AST increased	19980827				Unknown
2 Stevens Johnson syndrome	19980826				Unknown
3 ALT increased	19980827				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ass acetylsalicylic acid	Oral	/	-	-
2 paracetamol paracetamol	Oral	/	-	-
3 tegretal 400 retard carbamazepine	Unknown	/	-	-
4 thym-uvocal thymus extract	Oral	/	-	-
5 transpulmin baby eucalyptus oil; pine oil	Respiratory (inhalation)	/	-	-

Primary source

Qualification: Physician observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99000893

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 37 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19980818				Unknown
2 Leukocytosis	19980821				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 amoxicillin	Oral	/	-	
amoxicillin			-	
2 paracetamol	Oral	/	-	
paracetamol			-	

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99000894

Serious Yes	Death	Lifethr. No	Hospital. Yes	Disabling	Congenital	Other	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Female	Birth: privacy	Age: 58 Year	weight (kg): 60	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 SGPT increased	19970424				Unknown
2 Leukocytosis	19970430				Unknown
3 Stevens Johnson syndrome	19970410				recovered/resolved
4 Gamma GT increased	19970424				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
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1	atropine sulfate atropine sulfate	Intravenous (not otherwis /	-	
2	ben-u-ron paracetamol	Unknown /	-	Pyrexia of unknown origin
3	dipidolor piritramide	Intravenous (not otherwis /	-	Other general symptoms
4	disoprivan propofol	Oral /	-	
5	dormicum 7,5 midazolam maleate	Oral /	-	Unspecified psychophysiological
6	elzogram 2,0 cefazolin sodium	Intravenous (not otherwis /	-	Unspecified infectious and parasitic diseases
7	fluimucil acetylcysteine	Intravenous (not otherwis /	-	
8	fluimucil granulat acetylcysteine	Oral /	-	Acute upper respiratory infections of unspecified site
9	fortecortin dexamethasone	Oral /	-	Cerebral oedema
10	gastrozepin pirenzepine dihydrochloride	Intravenous (not otherwis /	-	Unspecified functional disorder of stomach
11	haes-steril 10% hetastarch; sodium chloride	Intravenous (not otherwis /	-	Volume depletion
12	heparin heparin	Intravenous (not otherwis /	-	Thrombosis
13	liquemin n heparin sodium	Subcutaneous /	-	Thrombosis
14	mucosolvan ambroxol hydrochloride	Oral /	-	
15	mucosolvan saft ambroxol hydrochloride	Oral /	-	Acute upper respiratory infections of unspecified site
16	nimbex cisatracurium besilate	Intravenous (not otherwis /	-	Unspecified disorder of muscle, ligament, and fascia
17	phenhydant phenytoin	Oral /	-	Other musculoskeletal symptoms referable to limbs
18	rohypnol 1 flunitrazepam	Oral /	-	
19	ulcogant-suspension sucralfate	Oral /	-	Unspecified functional disorder of stomach
20	ultima remifentanyl hydrochloride	Intravenous (not otherwis /	-	Other specified complications of procedures, not elsewhere
21	zantic ranitidine hydrochloride	Oral /	-	Unspecified functional disorder of stomach

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99000897

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 15 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Lymphocytosis	19971210				Unknown
2 Leukocytosis	19971210				Unknown
3 Stevens Johnson syndrome	19971124				Unknown
4 LDH increased	19971210				Unknown
5 SGPT increased	19971210				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 amoxypen saft	Oral	/	-	
amoxicillin trihydrate			-	
2 ben-u-ron	Oral	/	-	
paracetamol			-	

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99000941

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 4 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Fever	19980920				Unknown
2 Leukocytosis	19980925				Unknown
3 Stevens Johnson syndrome	19980921				Unknown
4 Lymphocytes raised	19980925				Unknown
5 Vomiting	19980920				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 captin	Unknown	/	-	
paracetamol			-	
2 ecolicin-augentropfen	Ophthalmic	/	-	
colistin; erythromycin lactobionate			-	
3 klacid	Oral	/	-	
clarithromycin			-	
4 mucosolvan saft	Oral	/	-	
ambroxol hydrochloride			-	

Primary source

Qualification: Physician observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99000942

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 52 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stomatitis	19980926				Unknown
2 Red eye	19980926				Unknown
3 Stevens Johnson syndrome	19980926				Unknown
4 Fissures of skin	19980926				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 bleomycin	Intravenous (not otherwis /		-	
bleomycin			-	
2 paracetamol	Rectal	/	-	
paracetamol			-	
3 Urbason	Intravenous (not otherwis /		-	
methylprednisolone			-	
4 uromitexan	Intravenous (not otherwis /		-	
mesna			-	
5 vincristine	Intravenous (not otherwis /		-	
vincristine			-	

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99900093

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes							

Patient

Initials:	Sex: Male	Birth: privacy	Age: 71 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Back pain	19980714				Unknown
2 Stevens Johnson syndrome	19980828				Unknown
3 Dyspnoea	19980714				Unknown
4 Exanthema	19980828				Unknown
5 Blistering	19980905				Unknown
6 Buccal inflammation	19980914				Unknown
7 Toxic epidermal necrolysis	19980828				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
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1	acc long acetylcysteine	Oral	/	-
2	ass acetylsalicylic acid	Oral	/	-
3	augmentan amoxicillin trihydrate; clavulanic acid	Intravenous (not otherwis	/	-
4	ben-u-ron 1000 mg paracetamol	Unknown	/	-
5	dilzem retard diltiazem hydrochloride	Oral	/	-
6	dipidolor piritramide	Intravenous (not otherwis	/	-
7	lasix furosemide	Intravenous (not otherwis	/	-
8	neurocil levomepromazine maleate	Oral	/	-
9	noctamid lormetazepam	Oral	/	-
10	novodigal 0,2 beta-acetyldigoxin	Intravenous (not otherwis	/	-
11	osmosteril 20% mannitol	Intravenous (not otherwis	/	-
12	paracetamol paracetamol	Unknown	/	-
13	paspertin metoclopramide hydrochloride	Intravenous (not otherwis	/	-
14	ranitidine ranitidine	Oral	/	-
15	sobelin soluble 600 clindamycin phosphate	Intravenous (not otherwis	/	-
16	sultanol fertiginhalat salbutamol sulfate	Respiratory (inhalation)	/	-
17	tramal tramadol hydrochloride	Oral	/	-
18	vancomycin vancomycin	Intravenous (not otherwis	/	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99001184

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 15 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19980929				Unknown
2 LDH increased	19981008				Unknown
3 Lip swelling	19980930				Unknown
4 Balanitis	19981016				Unknown
5 Blistering	19980903				Unknown
6 Red eye	19980902				Unknown
7 Buccal inflammation	19980930				Unknown
8 Lymphocytes raised	19981004				Unknown
9 Leucopenia	19981004				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 aspirin plus c acetylsalicylic acid; ascorbic acid	Oral	/	-	-
2 paracetamol paracetamol	Oral	/	-	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99001240

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 67 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Fever	19980126				Unknown
2 Toxic epidermal necrolysis	19980126				Unknown
3 Stevens Johnson syndrome	19980126				Unknown
4 LDH increased	19980126				Unknown
5 Cough	19980126				Unknown
6 Leucopenia	19980126				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 isoqid isoniazid	Oral	/	-	-
2 mucosolvan s ambroxol hydrochloride	Oral	/	-	-
3 paracetamol paracetamol	Oral	/	-	-
4 zyloric 300 allopurinol	Oral	/	-	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99001242

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 22 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Red eye	19970523				Unknown
2 Leukocytosis	19970523				Unknown
3 Leucopenia	19970523				Unknown
4 Respiration labored	19970517				Unknown
5 Erythema multiforme severe	19970522				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 acc acetylcysteine	Oral	/	-	-
2 ambroxol ambroxol	Oral	/	-	-
3 ass acetylsalicylic acid	Oral	/	-	-
4 paracetamol paracetamol	Oral	/	-	-
5 theophyllin theophylline	Oral	/	-	-
6 unacid ampicillin sodium; sulbactam sodium	Intravenous (not otherwis	/	-	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99001244

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 58 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Toxic epidermal necrolysis	19980624				Unknown
2 Stevens Johnson syndrome	19980624				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
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1	atosil	Intravenous (not otherwis /	-
	promethazine hydrochloride		-
2	atropine sulfate	Oral /	-
	atropine sulfate		-
3	ben-u-ron	Oral /	-
	paracetamol		-
4	decortin-h	Oral /	-
	prednisolone		-
5	dipidolor	Intravenous (not otherwis /	-
	piritramide		-
6	dolantin	Intravenous (not otherwis /	-
	pethidine hydrochloride		-
7	dormicum	Intravenous (not otherwis /	-
	midazolam maleate		-
8	etomidate	Intravenous (not otherwis /	-
	etomidate		-
9	fentanyl	Intravenous (not otherwis /	-
	fentanyl		-
10	floxal augensalbe	Ophthalmic /	-
	ofloxacin		-
11	heparin calcium	Subcutaneous /	-
	heparin calcium		-
12	isoflurane	Respiratory (inhalation) /	-
	isoflurane		-
13	konakion mm	Intravenous (not otherwis /	-
	phytomenadione		-
14	liquemin n	Intravenous (not otherwis /	-
	heparin sodium		-
15	lisino	Oral /	-
	loratadine		-
16	marcumar	Oral /	-
	phenprocoumon		-
17	nemexin	Oral /	-
	naltrexone hydrochloride		-
18	novalgin	Oral /	-
	metamizole		-
19	refobacin	Intravenous (not otherwis /	-
	gentamicin sulfate		-
20	staphylex	Oral /	-
	flucloxacillin sodium		-
21	tavegil	Intravenous (not otherwis /	-
	clemastine		-
22	tranxilium	Oral /	-
	clorazepate dipotassium		-
23	zienam	Intravenous (not otherwis /	-
	cilastatin sodium; imipenem		-

Primary source

Qualification: Physician observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99001245

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 4 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Fever	19981224				Unknown
2 Lymphocytes raised	19981227				Unknown
3 LDH increased	19981227				Unknown
4 Toxic epidermal necrolysis	19981225				Unknown
5 SGOT increased	19981227				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 capval-saft noscaphine hydrochloride	Oral	/	-	-
2 paracetamol paracetamol	Unknown	/	-	-
3 pen-lich ts phenoxymethylpenicillin potassium	Oral	/	-	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99001426

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 6 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Fever	19980814				Unknown
2 Leukocytosis	19980909				Unknown
3 LDH increased	19980819				Unknown
4 Vomiting	19980814				Unknown
5 Nausea	19980814				Unknown
6 Conjunctivitis	19980815				Unknown
7 Stevens Johnson syndrome	19980815				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 paracetamol	Unknown	/	-	
paracetamol			-	

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99001429

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 16 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Erythema multiforme severe	19990107				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 kamillosan chamomile; chamomile oil; hexylresorcinol	Respiratory (inhalation)	/	-	-
2 Kodeinphosphat codeine phosphate	Oral	/	-	-
3 paracetamol paracetamol	Oral	/	-	-

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99001433

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 69 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Corneal ulceration	19980412				Unknown
2 Conjunctivitis	19980412				Unknown
3 Stevens Johnson syndrome	19980412				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Acetylsalizylsaeure acetylsalicylic acid	Oral	/	-	-
2 ben-u-ron paracetamol	Unknown	/	-	-
3 capozide 25 captopril; hydrochlorothiazide	Oral	/	-	-
4 diflucan fluconazole	Oral	/	-	-
5 heparin-natrium heparin sodium	Subcutaneous	/	-	-
6 lasix furosemide	Oral	/	-	-
7 solu-decortin-h prednisolone sodium succinate	Intravenous (not otherwis	/	-	-
8 zyloric 300 allopurinol	Oral	/	-	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99001467

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 14 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Erythema multiforme severe	19980727				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 bromhexine hydrochloride bromhexine hydrochloride	Oral	/	-	
2 paracetamol paracetamol	Oral	/	-	

Primary source

Qualification: Physician observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99001474

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 36 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19980807				Unknown
2 Leucopenia	19980814				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ass acetylsalicylic acid	Oral	/	-	-
2 ben-u-ron paracetamol	Oral	/	-	-

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99001590

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes							

Patient

Initials:	Sex: Male	Birth: privacy	Age: 62 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19980501				Unknown
2 Peritonitis	19980501				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
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1	ampicillin	Intravenous (not otherwis /	-
	ampicillin		-
2	ben-u-ron saft	Oral /	-
	paracetamol		-
3	bepanthen	Oral /	-
	dexpantenol		-
4	claforan	Intravenous (not otherwis /	-
	cefotaxime sodium		-
5	combaren	Oral /	-
	codeine phosphate; diclofenac sodium		-
6	cotrim forte	Oral /	-
	sulfamethoxazole; trimethoprim		-
7	imeson	Oral /	-
	nitrazepam		-
8	lasix	Intravenous (not otherwis /	-
	furosemide		-
9	laxoberal	Oral /	-
	sodium picosulfate		-
10	mogadan	Oral /	-
	nitrazepam		-
11	myrrhentinktur	Unknown /	-
	herbal extracts nos		-
12	nizax	Oral /	-
	nizatidine		-
13	novalgin injektionsloesung	Intravenous (not otherwis /	-
	metamizole		-
14	novalgin tropfen	Oral /	-
	metamizole sodium		-
15	oxazepam	Oral /	-
	oxazepam		-
16	paspertin	Intravenous (not otherwis /	-
	metoclopramide hydrochloride		-
17	psyquil	Intravenous (not otherwis /	-
	triflupromazine		-
18	refobacin	Intravenous (not otherwis /	-
	gentamicin sulfate		-
19	staphylex	Intravenous (not otherwis /	-
	flucloxacillin sodium		-
20	tarivid	Intravenous (not otherwis /	-
	ofloxacin		-
21	tavegil	Oral /	-
	clemastine		-
22	tramal tropfen	Oral /	-
	tramadol hydrochloride		-
23	Urbason	Intravenous (not otherwis /	-
	methylprednisolone		-
24	zyloric 300	Oral /	-
	allopurinol		-
25	zyrtec	Oral /	-
	cetirizine hydrochloride		-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99001898

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes	Yes	Yes					

Patient

Initials:	Sex: Male	Birth: privacy	Age: 26 Year	weight (kg):	BMI_int:	Age fetus:
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Patientdeath

deathdate: 19941111	deathcause: Hepatic coma
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19941101				Unknown
2 Exanthema	19941101				Unknown
3 Stomatitis	19941101				Unknown
4 Coma hepatic	19941101				Unknown
5 Sepsis	19941101				Unknown
6 Blistering	19941101				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 atosil tropfen promethazine hydrochloride	Oral	/	19941025-19941026	Unspecified functional disorder of stomach
2 augmentan amoxicillin trihydrate; clavulanic acid	Intravenous (not otherwis	/	19941023-19941029	Unspecified infectious and parasitic diseases
3 ben-u-ron paracetamol	Rectal	/	19941021-19941023	Pyrexia of unknown origin
4 bifiteral lactulose	Oral	/	19941023-19941031	Unspecified chronic liver disease without mention of
5 dopamin dopamine hydrochloride	Intravenous (not otherwis	/	19941027-	Renal failure, unspecified
6 furorese furosemide	Oral	/	19941028-19941029	Oedema
7 humatin paromomycin sulfate	Oral	/	19941023-19941031	Encephalopathy, unspecified
8 konakion phytomenadione	Unknown	/	-	Deficiency of vitamin K
9 lasix furosemide	Intravenous (not otherwis	/	19941024-19941031	Oedema
10 novalgin injektionsloesung metamizole	Intravenous (not otherwis	/	19941030-19941030	Pyrexia of unknown origin
11 quantalan colestyramine	Oral	/	19941031-	Cirrhosis of liver without mention of alcohol
12 rocephin ceftriaxone sodium	Intravenous (not otherwis	/	19941030-	Unspecified infectious and parasitic diseases
13 voltaren diclofenac sodium	Rectal	/	19941101-19941101	Other general symptoms

Drugs (concomitant)

14 l-polamidon levomethadone	Unknown	/	-	
15 maalox aluminium hydroxide; aluminium hydroxide gel; magnesium hydroxide	Unknown	/	19941005-19941006	Unspecified functional disorder of stomach

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Drug dependence of morphine type			k.A.	concomitant disease

Primary source

Qualification: Physician observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99001902

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 1 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19940511				Unknown
2 SGOT increased	19940511				Unknown
3 SGPT increased	19940511				Unknown
4 Gamma GT increased	19940511				Unknown
5 Leukocytosis	19940511				Unknown
6 Leucopenia	19940511				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 amoxicillin amoxicillin	Oral	/	-	-
2 paracetamol paracetamol	Rectal	/	-	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99001904

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 8 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Conjunctivitis	19940402				Unknown
2 Lymphopenia	19940407				Unknown
3 Leukocytosis	19940407				Unknown
4 Erythema multiforme severe	19940331				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ben-u-ron	Unknown	/	-	
paracetamol			-	
2 larylin hustenloeser	Oral	/	-	
benzocaine; cetylpyridinium chloride; dropropizine; guaifenesin			-	
3 paediathrocin	Oral	/	-	
erythromycin ethylsuccinate			-	

Primary source

Qualification: Physician observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99001912

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 37 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Gamma GT increased	19940729				Unknown
2 LDH increased	19940729				Unknown
3 SGPT increased	19940729				Unknown
4 SGOT increased	19940729				Unknown
5 Leucopenia	19940729				Unknown
6 Leukocytosis	19940729				Unknown
7 Stevens Johnson syndrome	19940727				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 amoxicillin amoxicillin	Unknown	/	-	-
2 diclofenac-ratiopharm sf diclofenac sodium	Intramuscular	/	-	-
3 paracetamol paracetamol	Unknown	/	-	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99001913

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 20 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Exanthema	19950107				Unknown
2 Gamma GT increased	19950107				Unknown
3 Erythema multiforme severe	19950107				Unknown
4 Leucopenia	19950107				Unknown
5 Genital lesion NOS	19950107				Unknown
6 SGPT increased	19950107				Unknown
7 Stomatitis	19950107				Unknown
8 Conjunctivitis	19950107				Unknown
9 SGOT increased	19950107				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ass acetylsalicylic acid	Unknown	/	-	-
2 ben-u-ron paracetamol	Oral	/	-	-
3 fluimucil 200 brausetabletten acetylcysteine	Oral	/	-	-
4 nystatin "lederle" nystatin	Oral	/	-	-
5 tarivid 200 ofloxacin	Oral	/	-	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99001914

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 4 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Papular rash	19940621				Unknown
2 Mouth irritation	19940621				Unknown
3 Stevens Johnson syndrome	19940603				Unknown
4 Toxic epidermal necrolysis	19940603				Unknown
5 Conjunctivitis	19940621				Unknown
6 Leucopenia	19940621				Unknown
7 Leukocytosis	19940621				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ben-u-ron	Unknown	/	-	
paracetamol			-	
2 isocillin	Oral	/	-	
phenoxymethylpenicillin potassium			-	
3 vividrin nasenspray gegen heuschnupfen	Nasal	/	-	
cromoglicate sodium			-	

Primary source

Qualification: Physician observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99001917

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes							

Patient

Initials:	Sex: Female	Birth: privacy	Age: 58 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Exanthema	19950331				Unknown
2 Blisters	19950331				Unknown
3 Stevens Johnson syndrome	19950117				Unknown
4 Leukocytosis	19950129				Unknown
5 Lymphocytosis	19950202				Unknown
6 LDH increased	19950203				Unknown
7 Creatinine blood increased	19950204				Unknown
8 BUN increased	19950203				Unknown
9 Pneumonia	19950117				Unknown
10 Toxic epidermal necrolysis	19950117				Unknown
11 Renal insufficiency	19950331				Unknown
12 Hypoxic brain damage	19950331				Unknown
13 Itching	19950331				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
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1	ass 100	Oral	/	-
	acetylsalicylic acid			-
2	augmentan	Oral	/	-
	amoxicillin trihydrate; clavulanic acid			-
3	ben-u-ron	Rectal	/	-
	paracetamol			-
4	dulcolax	Rectal	/	-
	bisacodyl			-
5	erypo	Subcutaneous	/	-
	epoetin alfa			-
6	fluimucil granulat	Oral	/	-
	acetylcysteine			-
7	imurek	Oral	/	-
	azathioprine			-
8	lasix	Oral	/	-
	furosemide			-
9	metalcaptase	Oral	/	-
	penicillamine			-
10	novalgin injektionsloesung	Intravenous (not otherwis /		-
	metamizole			-
11	pursennid	Oral	/	-
	senna leaf			-
12	remestan	Oral	/	-
	temazepam			-
13	rohypnol	Oral	/	-
	flunitrazepam			-
14	tazobac 4,5 g	Intravenous (not otherwis /		-
	piperacillin sodium; tazobactam sodium			-
15	telebrix gastro	Oral	/	-
	ioxitalamate meglumine			-
16	trental 400	Oral	/	-
	pentoxifylline			-
17	xanef cor 2,5 mg	Oral	/	-
	enalapril maleate			-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99001923

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes		Yes					

Patient

Initials:	Sex: Female	Birth: privacy	Age: 85 Year	weight (kg): 46	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19950425				Unknown
2 Fever	19950426				Unknown
3 Conjunctivitis	19950427				Unknown
4 Toxic epidermal necrolysis	19950425				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 allopurinol allopurinol	Oral	300 Mg milligram(s)/Day	19950418-19950502	Hyperuricaemia
2 cytotec misoprostol	Oral	400 µg microgram(s)/Day	19950410-19950423	Unspecified functional disorder of stomach
3 dulcolax bisacodyl	Rectal	001 Df dosage form/Day	19950416-	Constipation
4 euphyllin retard aminophylline	Oral	001 Df dosage form/Day	19950410-19950426	Dyspnea
5 ferro sanol duodenal ferroglycine sulfate complex	Oral	002 Df dosage form/Day	19950410-19950418	Anaemia, unspecified
6 gastrogratin meglumine amidotrizoate; sodium amidotrizoate	Oral	/	19950416-19950418	Nonspec abnormal findings on radiol and oth exam of abdo
7 isoglaucan 1/4% augentropfen clonidine hydrochloride	Ophthalmic	003 Df dosage form/Day	-	
8 lasix furosemide	Oral	001 Df dosage form/Day	19950410-19950503	Oedema
9 mono-embolex nm fertigspritze certoparin sodium	Subcutaneous	001 Df dosage form/Day	19950407-19950420	Thrombosis
10 paracetamol paracetamol	Rectal	/	-	Pyrexia of unknown origin
11 propulsin cisapride monohydrate	Oral	030 ml millilitre(s)/Day	19950421-19950423	Unspecified functional disorder of stomach
12 tarivid ofloxacin	Oral	002 Df dosage form/Day	19950414-19950418	Unspecified infectious and parasitic diseases

Drugs (concomitant)

13	calcium carbonate calcium carbonate	Unknown	/	-
14	dolantin pethidine hydrochloride	Unknown	/	-
15	dopamin dopamine hydrochloride	Unknown	/	-
16	eugalac lactulose	Unknown	/	-
17	ferrolecit sodium ferric gluconate complex	Unknown	/	-
18	gastrozepin pirenzepine dihydrochloride	Unknown	/	-
19	Kaliumchlorid potassium chloride	Unknown	/	-
20	laxoberal sodium picosulfate	Unknown	/	-
21	lipovenoos glycerol; lecithin; soya oil	Unknown	/	-
22	mucosolvan ambroxol hydrochloride	Unknown	/	-
23	multibionta ascorbic acid; calcium pantothenate; cyanocobalamin; nicotinamide; pyridoxine hydrochloride; retinol palmitate; riboflavin; rutoside; thiamine hydrochloride; tocopheryl acetate	Unknown	/	-
24	riopan magaldrate	Unknown	/	-
25	tavegil clemastine	Unknown	/	-
26	tramal tramadol hydrochloride	Unknown	/	-
27	transpulmin suppository camphor; chamomile; cineole; guaiacol; isothipendyl hydrochloride; polidocanol; salvia	Unknown	/	-

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Chronic renal failure			k.A.	concomitant disease
Ulcus ventriculi			k.A.	concomitant disease
Atrial fibrillation and flutter			k.A.	concomitant disease

Primary source

Qualification: Physician observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99001924

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes							

Patient

Initials:	Sex: Female	Birth: privacy	Age: 67 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Tonsillitis	19930411				Unknown
2 Death NOS	19930411				Unknown
3 Circulatory failure	19930411				Unknown
4 Headache	19930423				Unknown
5 Leukocytosis	19930425				Unknown
6 BUN increased	19930502				Unknown
7 Toxic epidermal necrolysis	19930423				Unknown
8 Creatinine blood increased	19930502				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 amoxyphen amoxicillin trihydrate	Oral	/	-	-
2 diutensat hydrochlorothiazide; triamterene	Oral	/	-	-
3 paracetamol paracetamol	Unknown	/	-	-
4 remid 100 allopurinol	Oral	/	-	-
5 veratide hydrochlorothiazide; triamterene; verapamil hydrochloride	Oral	/	-	-

Primary source

Qualification: Physician observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99001927

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 50 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Sore throat	19970919				Unknown
2 Fever	19970920				Unknown
3 LDH increased					Unknown
4 Gamma GT increased					Unknown
5 SGOT increased					Unknown
6 Stevens Johnson syndrome	19970920				Unknown
7 Leukocytosis					Unknown
8 Hepatic failure	19970920				Unknown
9 SGPT increased					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 acetylcysteine	Oral	/	-	
acetylcysteine			-	
2 orelox	Oral	/	-	
cefprozime proxetil			-	
3 paracetamol	Oral	/	-	
paracetamol			-	
4 rulid	Oral	/	-	
roxithromycin			-	
5 silomat tropfen gegen reizhusten	Oral	/	-	
clobutinol hydrochloride			-	

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99001931

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 15 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Erythema exudativum	19981229				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ben-u-ron paracetamol	Oral	/	-	-
2 bronchipret primula rhizome; thyme extract	Oral	/	-	-
3 meditonsin h homeopatics nos	Oral	/	-	-

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99001937

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 83 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Gamma GT increased	19941017				Unknown
2 SGPT increased	19941017				Unknown
3 SGOT increased	19941016				Unknown
4 Toxic epidermal necrolysis	19941010				Unknown
5 LDH increased	19941016				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 agarol emulsion paraffin, liquid; phenolphthalein	Oral	/	-	-
2 dalmadorm flurazepam hydrochloride	Oral	/	-	-
3 paracetamol paracetamol	Oral	/	-	-
4 pepdul mite famotidine	Oral	/	-	-
5 pepsin-wein ethanol; pepsin	Unknown	/	-	-
6 prednisolon-21-hydrogensuccinat natriumsalz prednisolone sodium succinate	Intravenous (not otherwis	/	-	-
7 sostril filmltabletten ranitidine hydrochloride	Oral	/	-	-
8 vergentan alozapride hydrochloride	Oral	/	-	-
9 vomex a injektionsloesung i.v. dimenhydrinate	Intravenous (not otherwis	/	-	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99001938

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 9 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Erythema exudativum	19941014				Unknown
2 Pneumonia	19941012				Unknown
3 Leukocytosis					Unknown
4 Fever	19941006				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ambroxol	Unknown	/	-	
ambroxol			-	
2 aspecton n	Unknown	/	-	
saponin; thyme extract			-	
3 ben-u-ron saft	Oral	/	-	
paracetamol			-	
4 infectomycin plus saft	Oral	/	-	
erythromycin estolate			-	
5 olynth 0,1%	Nasal	/	-	
xylometazoline hydrochloride			-	
6 paracetamol	Unknown	/	-	
paracetamol			-	

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99001943

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 53 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19941015				Unknown
2 Leukocytosis	19941015				Unknown
3 Lymphopenia	19941011				Unknown
4 SGOT increased	19941024				Unknown
5 SGPT increased	19941024				Unknown
6 Gamma GT increased	19941024				Unknown
7 Fever	19941015				Unknown
8 Genital lesion NOS	19941015				Unknown
9 Stomatitis	19941015				Unknown
10 Exanthema	19941015				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 paracetamol	Unknown	/	-	
paracetamol			-	
2 phenhydan	Oral	/	-	
phenytoin			-	
3 tarivid	Oral	/	-	
ofloxacin			-	
4 zentropil	Oral	/	-	
phenytoin			-	

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99002159

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 58 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Conjunctivitis	19940913				Unknown
2 Exanthema	19940913				Unknown
3 Toxic epidermal necrolysis	19940913				Unknown
4 Stevens Johnson syndrome	19940913				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 aspirin	Oral	/	-	
acetylsalicylic acid			-	
2 paracetamol 500 "bayer"	Oral	/	-	
paracetamol			-	

Primary source

Qualification: Physician observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99002512

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 32 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Erythema exudativum	19981126				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ass acetylsalicylic acid	Oral	/	-	-
2 paracetamol paracetamol	Oral	/	-	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99002522

Serious Yes	Death	Lifethr. No	Hospital. Yes	Disabling	Congenital	Other	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Female	Birth: privacy	Age: 37 Year	weight (kg): 45	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19970405				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 adalat nifedipine	Oral	/	19970323-19970323	Hypertension arterial
2 dehydrobenzperidol droperidol	Intravenous (not otherwis	/	19970325-19970325	
3 diazepam diazepam	Oral	/	19970324-	
4 doloreduct-500 paracetamol	Oral	/	19970326-	Other general symptoms
5 dulcolax bisacodyl	Oral	/	19970403-	Constipation
6 fentanyl fentanyl	Intravenous (not otherwis	/	19970325-19970325	
7 fortecortin dexamethasone	Intramuscular	2*20 Mg milligram(s)/	19970321-19970325	Cerebral oedema
8 haes-steril hetastarch; sodium chloride	Intravenous (not otherwis 2 l	litre(s)/	19970325-19970325	
9 heparin-natrium heparin sodium	Subcutaneous	015 Kiu iu (1000s)/Day	19970325-	Thrombosis
10 klyσμα disodium hydrogenorthophosphate; potassium sorbate; sodium phosphate monobasic (anhydrate)	Rectal	1 Df dosage form/	19970324-19970331	Constipation
11 lecicarbon lecithin; sodium bicarbonate; sodium phosphate monobasic (anhydrate)	Rectal	/	19970329-	Constipation
12 norcuron vecuronium bromide	Intravenous (not otherwis	/	19970325-19970325	
13 osmofundin mannitol; sodium acetate trihydrate; sodium chloride	Intravenous (not otherwis 125 ml	millilitre(s)/	19970325-19970325	Benign intracranial hypertension
14 propofol 1 % fresenius propofol	Intravenous (not otherwis 250 Mg	milligram(s)/	19970325-19970325	
15 rivotril clonazepam	Intravenous (not otherwis 1 Df	dosage form/	19970327-19970327	Epilepsy, unspecified
16 sab simplex dimeticone	Oral	/	19970330-	Meteorism
17 spizef cefotiam hydrochloride	Intravenous (not otherwis 2*2 G	gram(s)/	19970325-19970325	
18 succicuran suxamethonium chloride	Intravenous (not otherwis	/		
19 tranquase diazepam	Oral	/	19970308-	

Drugs (concomitant)				
20	fortecortin dexamethasone	Oral	/	19970308- - Cerebral oedema
21	glucose 5% glucose monohydrate	Intravenous (not otherwis	500 ml millilitre(s)/Day	19970324-19970324 -
22	Glukose, Elektrolyte electrolytes nos; glucose	Intravenous (not otherwis	500 ml millilitre(s)/Day	19970328-19970403 -
23	Kontrastmittel	Intra-arterial	/	19970313-19970313 -
24	lecicarbon lecithin; sodium bicarbonate; sodium phosphate monobasic (anhydrate)	Rectal	/	19970329- - Constipation
25	sab simplex dimeticone	Oral	/	19970330-19970330 Meteorism -
26	sostril ranitidine hydrochloride	Intravenous (not otherwis	1 Df dosage form/	19970313-19970313 -
27	sostril 150 mg ranitidine hydrochloride	Oral	002 Df dosage form/Day	19970308- -
28	tagamet cimetidine	Intravenous (not otherwis	1 Df dosage form/	19970313-19970313 -
29	Volumenersatzmittel	Intravenous (not otherwis	/	19970325-19970325 -

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99002523

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 85 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Leukocytosis	19980305				Unknown
2 Stevens Johnson syndrome	19990216				Unknown
3 Fever	19980215				Unknown
4 Toxic epidermal necrolysis	19990216				Unknown
5 Urea blood level increased	19980318				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 altinsulin insulin	Subcutaneous	/	-	-
2 ass 100 acetylsalicylic acid	Oral	/	-	-
3 cotrim forte sulfamethoxazole; trimethoprim	Unknown	/	-	-
4 diazepam diazepam	Oral	/	-	-
5 fenistil dimetindene maleate	Intravenous (not otherwis	/	-	-
6 fenistil dimetindene maleate	Oral	/	-	-
7 novalgin tropfen metamizole sodium	Oral	/	-	-
8 paracetamol paracetamol	Unknown	/	-	-
9 pepdul famotidine	Oral	/	-	-
10 refobacin creme gentamicin sulfate	Topical	/	-	-
11 sostril 300 mg ranitidine hydrochloride	Intravenous (not otherwis	/	-	-
12 tramal tropfen tramadol hydrochloride	Oral	/	-	-

Drugs (concomitant)

13 rheogen aloes extract; belladonna extract; rhubarb	Unknown	/	-	-
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Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99002534

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 76 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Toxic epidermal necrolysis	19990316				Unknown
2 Stevens Johnson syndrome	19990316				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 acc 200 acetylcysteine	Oral	/	-	-
2 acercomp hydrochlorothiazide; lisinopril dihydrate	Oral	/	-	-
3 antra mups 20 mg omeprazole magnesium	Oral	/	-	-
4 azumetop-retard metoprolol tartrate	Oral	/	-	-
5 bactrim forte sulfamethoxazole; trimethoprim	Oral	/	-	-
6 beloc zok metoprolol succinate	Oral	/	-	-
7 cytotec 200 misoprostol	Oral	/	-	-
8 irenat sodium perchlorate	Oral	/	-	-
9 metoprolol metoprolol	Oral	/	-	-
10 norvasc 5 mg amlodipine besilate	Oral	/	-	-
11 paracetamol-ratiopharm paracetamol	Oral	/	-	-
12 stangyl 25 trimipramine maleate	Oral	/	-	-
13 tavor 1,0 lorazepam	Oral	/	-	-
14 tramal tropfen tramadol hydrochloride	Oral	/	-	-
15 zithromax azithromycin	Oral	/	-	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99002539

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 58 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Conjunctivitis	19990216				Unknown
2 Stevens Johnson syndrome	19990214				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 atosil tropfen promethazine hydrochloride	Oral	/	-	-
2 bactrim forte sulfamethoxazole; trimethoprim	Oral	/	-	-
3 chloraldurat "rot" chloral hydrate	Oral	/	-	-
4 diazepam diazepam	Unknown	/	-	-
5 lamictal 200 lamotrigine	Oral	/	-	-
6 paracetamol paracetamol	Unknown	/	-	-
7 phenhydan phenytoin	Oral	/	-	-
8 remestan temazepam	Oral	/	-	-
9 rivotril 0,5 clonazepam	Oral	/	-	-
10 tavor 1,0 lorazepam	Oral	/	-	-
11 valiquid 0,3 diazepam	Oral	/	-	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99002542

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 16 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19990122				Unknown
2 Toxic epidermal necrolysis	19990122				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 bronchicum elixir plus plantago lanceolata; primula fluid extract; thymus vulgaris fluid extract	Oral	/	-	-
2 paracetamol paracetamol	Oral	/	-	-
3 prospan akut husten-brausetabletten hedera helix	Oral	/	-	-
4 sinupret forte dragees ascorbic acid; gentiana; primula flowers; rumicis asetosa; sambucus flowers; verbena herb	Oral	/	-	-

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99002548

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes							

Patient

Initials:	Sex: Female	Birth: privacy	Age: 77 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Leucopenia	19990522				Unknown
2 Toxic epidermal necrolysis					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 allopurinol	Oral	/	-	
allopurinol			-	
2 duravolten	Oral	/	-	
diclofenac sodium			-	
3 esidrix	Oral	/	-	
hydrochlorothiazide			-	
4 furosemide	Oral	/	-	
furosemide			-	
5 paracetamol	Oral	/	-	
paracetamol			-	

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99004055

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 16 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19980128				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 bronchicum elixier n cortex quebracho; grindelia; pimpinella root; primula tincture; thyme tincture	Oral	/	-	-
2 paracetamol paracetamol	Rectal	/	-	-
3 pcf-tropfen homeopatics nos	Oral	/	-	-
4 sedotussin 75 forte pentoxiverine citrate	Oral	/	-	-

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99004058

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 54 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19980315				Unknown
2 Toxic epidermal necrolysis	19980315				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 acc acetylcysteine	Oral	/	-	-
2 antra 20 omeprazole	Oral	/	-	-
3 bactrim forte sulfamethoxazole; trimethoprim	Oral	/	-	-
4 bcnu carmustine	Intravenous (not otherwis	/	-	-
5 beloc-zok mite metoprolol succinate	Oral	/	-	-
6 chloraldurat 500 chloral hydrate	Oral	/	-	-
7 dexamethasone dexamethasone	Oral	/	-	-
8 glycerol glycerol	Unknown	/	-	-
9 imeson nitrazepam	Oral	/	-	-
10 laxoberal sodium picosulfate	Oral	/	-	-
11 lefax dimeticone; silicon dioxide, colloidal	Oral	/	-	-
12 nasivinetten oxymetazoline hydrochloride	Nasal	/	-	-
13 paracetamol paracetamol	Oral	/	-	-
14 timonil carbamazepine	Oral	/	-	-
15 zofran ondansetron hydrochloride	Intravenous (not otherwis	/	-	-
16 zofran ondansetron hydrochloride	Oral	/	-	-

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99004059

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 39 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Blistering	19980404				Unknown
2 Exanthema	19980404				Unknown
3 Toxic epidermal necrolysis	19980404				Unknown
4 Stevens Johnson syndrome	19980404				Unknown
5 Lip ulceration	19980404				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 finlepsin carbamazepine	Oral	/	-	-
2 novalgin metamizole	Unknown	/	-	-
3 paracetamol paracetamol	Rectal	/	-	-

Primary source

Qualification:

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99004064

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 12 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Erythema exudativum	19980729				Unknown
2 Conjunctivitis					Unknown
3 Stomatitis					Unknown
4 Blisters					Unknown
5 Erythema multiforme	19980729				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 amoxicillin amoxicillin	Oral	/	-	-
2 mucosolvan soft ambroxol hydrochloride	Oral	/	-	-
3 paracetamol paracetamol	Oral	/	-	-
4 rulid roxithromycin	Oral	/	-	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99004066

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 11 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Erythema exudativum	19990109				Unknown
2 Erythema multiforme	19990109				Unknown
3 Lip ulceration	19990109				Unknown
4 Exanthema	19990109				Unknown
5 Stomatitis	19990109				Unknown
6 Eye irritation	19990109				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ben-u-ron saft paracetamol	Oral	/	-	-
2 cefasept homeopatics nos	Oral	/	-	-
3 codipront saft codeine; phenyltoloxamine	Oral	/	-	-
4 pulmonium-hustensaft n petasites hybridus; plantago lanceolata	Oral	/	-	-
5 rulid tabletten fuer kinder roxithromycin	Oral	/	-	-

Primary source

Qualification:

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99005906

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 10 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Allergic reaction	19991115				Unknown
2 Leukocytosis	19991115				Unknown
3 Erythema multiforme severe	19991115				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 eibisch-hustensaft	Oral	/	-	
althaea syrup			-	
2 klacid saft	Oral	/	-	
clarithromycin			-	
3 paracetamol-ratiopharm	Oral	/	-	
paracetamol			-	
4 silomat	Oral	/	-	
clobutinol hydrochloride			-	

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-00000145

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 23 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Macular rash	19991207				Unknown
2 Conjunctivitis	19991207				Unknown
3 Skin erythema	19991207				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 paracetamol	Unknown	/	-	
paracetamol			-	

Primary source

Qualification: Physician observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-00001263

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.
 Yes No

Patient

Initials: Sex: Female Birth: privacy Age: 70 Year weight (kg): 47 BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Bullous eruption	20000404		10 Day		recovered/resolved
2 Coagulation factors decreased	20000404		10 Day		recovered/resolved
3 Creatine kinase increased	20000404		10 Day		recovered/resolved
4 Haemorrhage NOS	20000404		10 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 firin norfloxacin	Oral	/	-	
2 marcumar phenprocoumon	Unknown	/	-	
3 norfloxacin norfloxacin	Oral	002 Df dosage form/Day	20000331-20000403	Urinary tract infection, site not specified
4 paracetamol paracetamol	Oral	002 G gram(s)/Day	-	
5 tarivid ofloxacin	Oral	002 Df dosage form/Day	20000403-20000405	Urinary tract infection, site not specified

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-00001481

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes					

Patient

Initials:	Sex: Male	Birth: privacy	Age: 60 Year	weight (kg): 83	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19990611		25 Day		Unknown
2 Red eye	19990611		25 Day		Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 cibadrex benazepril hydrochloride; hydrochlorothiazide	Oral	001 Df dosage form/Day	-	Hypertension arterial
2 codiovan hydrochlorothiazide; valsartan	Oral	001 Df dosage form/Day	-	Hypertension arterial
3 milurit 300 allopurinol	Oral	001 Df dosage form/Day	19990513-	Gout, unspecified
4 paracetamol paracetamol	Oral	/	-	Rheumatic fever without mention of heart involvement

Drugs (concomitant)

5 colchysat buerger colchicine	Oral	/	-
6 rewodina dual 75 kaps diclofenac sodium	Oral	/	-

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-00001480

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 57 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 SGPT increased	19990707				Unknown
2 Stevens Johnson syndrome	19990707				Unknown
3 Gamma GT increased	19990707				Unknown
4 Leukocytosis	19990707				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 paracetamol	Oral	/	-	
paracetamol			-	

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-00001483

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 8 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19990616				Unknown
2 Conjunctivitis	19990616				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ben-u-ron	Rectal	/	-	
paracetamol			-	
2 carbamazepine	Oral	/	-	
carbamazepine			-	
3 timonil	Oral	/	-	
carbamazepine			-	
4 toxi-loges	Oral	/	-	
homeopatics nos			-	

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-00001495

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 55 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Skin disorder	19990329				Unknown
2 Conjunctivitis	19990331				Unknown
3 Urethral burning on micturition	19990330				Unknown
4 Erythema multiforme severe	19990329				Unknown
5 Skin depigmentation	19990330				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 chlorhexamed fluid chlorhexidine gluconate	Oral	/	-	-
2 migraenerton metoclopramide hydrochloride; paracetamol	Oral	/	-	-
3 paracetamol paracetamol	Oral	/	-	-
4 timonil 300 retard carbamazepine	Oral	/	-	-
5 tramal tropfen tramadol hydrochloride	Oral	/	-	-

Primary source

Qualification: Physician observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-00001505

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 15 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Erythema multiforme severe	19991022				Unknown
2 Leukocytosis	19991022				Unknown
3 Lymphocytes raised	19991022				Unknown
4 Infection viral	19991022				Unknown
5 SGOT increased	19991022				Unknown
6 LDH increased	19991022				Unknown
7 SGPT increased	19991022				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 fluimucil 200 granulat acetylcysteine	Oral	/	-	-
2 lindoxyl loesung ambroxol hydrochloride	Oral	/	-	-
3 paracetamol 500 paracetamol	Oral	/	-	-
4 rhinotussal saft carbinoxamine; dextromethorphan; phenylpropanolamine	Oral	/	-	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-00001506

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 36 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19991028				Unknown
2 Gamma GT increased	19991019				Unknown
3 SGPT increased	19991016				Unknown
4 Lymphocytosis	19991016				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 amoxicillin amoxicillin	Oral	/	-	-
2 cotrim sulfamethoxazole; trimethoprim	Oral	/	-	-
3 paracetamol paracetamol	Oral	/	-	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-00001507

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 11 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 LDH increased	19991029				Unknown
2 Lymphocytosis	19991027				Unknown
3 Leukocytosis	19991025				Unknown
4 Erythema multiforme severe	19991021				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ambroxol	Oral	/	-	
ambroxol			-	
2 frubienzym halsschmerz-tabletten	Oral	/	-	
cetylpyridinium chloride; lysozyme			-	
3 paracetamol 500	Oral	/	-	
paracetamol			-	

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-00001510

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 4 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Toxic epidermal necrolysis	20000116				Unknown
2 Stevens Johnson syndrome	20000116				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 paracetamol	Oral	/	-	
paracetamol			-	

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-00200094

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Male	Birth: privacy	Age: 37 Year	weight (kg): 82	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Erythema multiforme severe	19991220		11 Day		recovered/resolved
2 Conjunctivitis	19991220		11 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 aspirin plus c acetylsalicylic acid; ascorbic acid	Oral	1 Df dosage form/	19991220-19991220	Influenza with other respiratory manifestations
2 paracetamol-ratiopharm paracetamol	Oral	001 Df dosage form/Day	19991217-19991218	Influenza with other respiratory manifestations

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-00200095

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Male	Birth: privacy	Age: 51 Year	weight (kg): 69	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Irritation eye	19990901				Unknown
2 Swallowing difficult	19990901				Unknown
3 Toxic epidermal necrolysis	19990901				Unknown
4 Fever	19990901				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 adumbran oxazepam	Oral	001 Df dosage form/Day	19990822-19990823	Sleep disturbances
2 ciprobay ciprofloxacin hydrochloride	Oral	1.5 G gram(s)/Week	19990806-19990818	Unspecified infectious and parasitic diseases
3 colistin colistin	Intravenous (not otherwis	004 Miu iu(1,000,000s)/De	19990819-19990823	
4 combactam sulbactam sodium	Intravenous (not otherwis	1 G gram(s)/Day	19990815-19990824	Unspecified infectious and parasitic diseases
5 gernebcin tobramycin sulfate	Intravenous (not otherwis	240 Mg milligram(s)/Day	19990815-19990824	Unspecified infectious and parasitic diseases
6 paracetamol paracetamol	Oral	1 G gram(s)/	19990815-19990815	Other general symptoms
7 phenhydan phenytoin	Oral	100 Mg milligram(s)/Day	19990701-	Grand mal
8 pipril piperacillin sodium	Intravenous (not otherwis	012 G gram(s)/Day	19990815-19990824	Unspecified infectious and parasitic diseases
9 zyloric allopurinol	Oral	600 Mg milligram(s)/Day	19990806-19990824	Other abnormal blood chemistry
10 zyrtec cetirizine hydrochloride	Oral	/	19990820-19990824	Unspecified pruritic disorder

Drugs (concomitant)

11	aminomix amino acids nos; minerals nos	Intravenous (not otherwis /	-19990823	
12	anaesthesin benzocaine	Oral /	-19990824	Other and unspecified diseases of the oral soft tissues
13	fenistil gel dimetindene maleate	Topical /	19990820-19990824	Unspecified pruritic disorder
14	fortecortin dexamethasone	Oral 8 Mg milligram(s)/Day	-	
15	imodium loperamide hydrochloride	Oral 2 Df dosage form/	19990818-19990918	Other noninfective gastroenteritis and colitis
16	interferon alfa-2a interferon alfa-2a	Intravenous (not otherwis 3 l litre(s)/Day	19990812-19990815	Neoplasm of unspecified nature, site unspecified
17	kalinor-brausetabletten potassium carbonate; potassium citrate	Oral 2 Df dosage form/	-19990824	Hypokalaemia
18	liquemin n heparin sodium	Intravenous (not otherwis /	-19990824	
19	mono-embolex nm certoparin sodium	Subcutaneous .5 ml millilitre(s)/Week	-19990823	
20	moronal suspension nystatin	Oral 001 Df dosage form/Day	-19990824	Thrush
21	Multivitamine ascorbic acid; ergocalciferol; folic acid; nicotinamide; panthenol; retinol; riboflavin; thiamine hydrochloride	Oral /	19990818-19990824	Other and unspecified diseases of the oral soft tissues
22	neupogen filgrastim	Subcutaneous 001 Df dosage form/Day	19990814-19990823	Agranulocytosis
23	nizax nizatidine	Oral 300 Mg milligram(s)/Day	-	
24	subcutin n-loesung benzocaine	Oral /	-19990824	Other and unspecified diseases of the oral soft tissues
25	valdispert valerian extract	Oral 1 Df dosage form/	-19990823	Other and unspecified special symptoms or syndromes, not
26	valoron n naloxone hydrochloride; tilidine hydrochloride	Oral 30 Gtt drop(s)/	19990815-19990815	Other general symptoms

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-00200096

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes					

Patient

Initials:	Sex: Male	Birth: privacy	Age: 32 Year	weight (kg): 70	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19991004		19 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 aspirin acetylsalicylic acid	Oral	500 Mg milligram(s)/	19990927-19990927	Unspecified inflammatory and toxic neuropathies
2 noctamid lormetazepam	Oral	1 Df dosage form/	-	Sleep disturbances
3 paracetamol paracetamol	Oral	001 G gram(s)/Day	19990928-	Unspecified inflammatory and toxic neuropathies
4 viramune nevirapine	Oral	001 Df dosage form/Day	19990916-	HIV infection NOS

Drugs (concomitant)

5 combivir lamivudine; zidovudine	Oral	002 Df dosage form/Day	-	HIV infection NOS
6 inirase saquinavir	Oral	004 Df dosage form/Day	-19990915	HIV infection NOS
7 norvir ritonavir	Oral	010 ml millilitre(s)/Day	-19990915	HIV infection NOS
8 taxilan perazine	Oral	1 Df dosage form/	-	
9 ziagen abacavir sulfate	Oral	002 Df dosage form/Day	-	HIV infection NOS

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Opportunistic mycoses			k.A.	concomitant disease

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-00008394

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 40 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Lyell syndrome	20000814		27 Day		Unknown
2 Stevens Johnson syndrome	20000814		27 Day		Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 lamictal lamotrigine	Oral	/	-	Other forms of epilepsy
2 paracetamol paracetamol	Rectal	/	-	Pyrexia of unknown origin
3 zentropil phenytoin	Oral	/	-	Unspecified mental retardation

Drugs (concomitant)

4 ergenyl valproate sodium	Unknown	/	-	Epilepsy, unspecified
5 novothyral levothyroxine sodium; liothyronine sodium	Unknown	/	-	Unspecified hypothyroidism

Primary source

Qualification:

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-00008839

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 31 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Erythema multiforme severe	20000515				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 benuron paracetamol	Oral	/	-	
2 musaril tetrazepam	Oral	/	-	
3 zyloric allopurinol	Oral	/	-	

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-01000192

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes					

Patient

Initials:	Sex: Female	Birth: privacy	Age: 60 Year	weight (kg): 77	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Gamma-glutamyltransferase incr.	20000925				recovered/resolved
2 Stevens Johnson syndrome	20000925				recovered/resolved
3 CRP positive	20000925				recovered/resolved
4 Buccal mucosa ulceration	20000925				recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 bepanthen roche tabletten panthenol	Oral	/	-	Throat pain
2 dolo-dobendan benzocaine; cetylpyridinium chloride	Oral	/	-	Throat pain
3 maaloxan forte aluminium hydroxide; aluminium hydroxide gel; magnesium hydroxide	Oral	/	-	Unspecified functional disorder of stomach
4 paracetamol paracetamol	Oral	/	-	Other general symptoms
5 phenhydantoin phenytoin	Oral	/	-	Epilepsy, unspecified
6 remestan mite temazepam	Oral	/	-	Sleep disturbances
7 tramadol tramadol	Oral	/	-	Other general symptoms
8 zyrtec cetirizine hydrochloride	Oral	/	-	Unspecified pruritic disorder

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Malignant neoplasm of brain, unspecified			k.A.	concomitant disease
Hepatitis non-A non-B			k.A.	concomitant disease
Colitis, enteritis, and gastroenteritis of presumed infectious			k.A.	concomitant disease
unbekannt			k.A.	concomitant disease: z.n.radiatio

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-01000193

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes					

Patient

Initials:	Sex: Male	Birth: privacy	Age: 20 Year	weight (kg): 50	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19990927		24 Day		recovered/resolved
2 Conjunctivitis	19990927				recovered/resolved
3 Lip swelling	19990928				recovered/resolved
4 Leucopenia	19990927				recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 amoxicillin	Unknown	/	-	Influenza with other respiratory manifestations
amoxicillin			-	
2 paracetamol 500	Unknown	/	-	Influenza with other respiratory manifestations
paracetamol			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Iron deficiency anaemias			k.A.	concomitant disease

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-01000197

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes		Yes					

Patient

Initials:	Sex: Female	Birth: privacy	Age: 53 Year	weight (kg): 78	BMI_int:	Age fetus:
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Patientdeath

deathdate:	deathcause: Other specified septicaemias
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 SGPT increased	20000122				Unknown
2 SGOT increased	20000124				Unknown
3 Gamma GT increased	20000124				Unknown
4 Stevens Johnson syndrome	20000116		16 Day		Unknown
5 Lymphocytes raised	20000120				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 aspirin acetylsalicylic acid	Oral	/	-	Unspecified otitis media
2 ass 100 acetylsalicylic acid	Oral	/	-	Myeloid leukaemia, acute
3 augmentan amoxicillin trihydrate; clavulanic acid	Intravenous (not otherwis	/	-	Unspecified otitis media
4 gelonida acetylsalicylic acid; codeine phosphate; paracetamol	Oral	/	-	Unspecified otitis media
5 paracetamol paracetamol	Oral	/	-	Unspecified otitis media

Primary source

Qualification:	Physician	observestudytype:
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Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-01000201

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes					

Patient

Initials:	Sex: Male	Birth: privacy	Age: 8 Year	weight (kg): 26	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Fever	20000205		17 Day		recovered/resolved
2 Oedema	20000205		17 Day		recovered/resolved
3 Conjunctivitis	20000205		17 Day		recovered/resolved
4 Buccal mucosa ulceration	20000205		17 Day		recovered/resolved
5 Erythema multiforme severe	20000205		17 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ambroxol ambroxol	Oral	/	-	Acute upper respiratory infections of unspecified site
2 enelfa saft paracetamol	Oral	/	-	Pyrexia of unknown origin
3 lemocin cetrimonium bromide; lidocaine; tyrothricin	Oral	/	-	Acute upper respiratory infections of unspecified site
4 paracetamol paracetamol	Rectal	/	-	Pyrexia of unknown origin

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-01000212

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes					

Patient

Initials:	Sex: Female	Birth: privacy	Age: 13 Year	weight (kg): 41	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Leukocytosis	20000314		22 Day		recovered/resolved
2 Stevens Johnson syndrome	20000314		22 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 mucosolvan saft ambroxol hydrochloride	Oral	/Day	20000308-20000310	Cough
2 neuranidal acetylsalicylic acid; caffeine; paracetamol	Oral	.5 Df dosage form/	20000306-20000306	Headache
3 paracetamol 500 paracetamol	Oral	003 Df dosage form/Day	20000308-20000310	Pyrexia of unknown origin
4 roxigruen roxithromycin	Oral	001 Df dosage form/Day	20000310-20000314	Acute upper respiratory infections of unspecified site

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-01003215

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes					

Patient

Initials:	Sex: Female	Birth: privacy	Age: 28 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Mucosal bulla	20010305				recovered/resolved
2 Rash haemorrhagic	20010305				recovered/resolved
3 Fever	20010305				recovered/resolved
4 CRP positive	20010305				recovered/resolved
5 LDH increased urine	20010305				recovered/resolved
6 Stevens Johnson syndrome	20010305				recovered/resolved
7 Conjunctivitis	20010305				recovered/resolved
8 Toxic epidermal necrolysis	20010305				recovered/resolved
9 SGPT increased	20010305				recovered/resolved
10 SGOT increased	20010305				recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 paracetamol	Oral	/	-20010308	Cephalgia
paracetamol			-	
2 tegretal	Oral	200 Mg milligram(s)/Day	20010223-20010308	Epilepsy, unspecified
carbamazepine			-	

Drugs (concomitant)

3 novalgin	Unknown	/	-	Pyrexia of unknown origin
metamizole			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Erythema multiforme			k.A.	concomitant disease
Viral hepatitis B without mention of hepatic coma			k.A.	concomitant disease
Adjustment reaction, unspecified			k.A.	concomitant disease

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-01008522

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		Yes	Yes					

Patient

Initials:	Sex: Female	Birth: privacy	Age: 4 Year	weight (kg): 12	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Erythema multiforme severe	20000215		12 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 erythromycin erythromycin	Oral	/	20000215-20000219	Acute nasopharyngitis (common cold)
2 paracetamol paracetamol	Unknown	/	20000213-	Acute nasopharyngitis (common cold)

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Colitis, enteritis, and gastroenteritis of presumed infectious			k.A.	concomitant disease

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-01008524

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes	Yes	Yes					

Patient

Initials:	Sex: Male	Birth: privacy	Age: 73 Year	weight (kg): 85	BMI_int:	Age fetus:
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Patientdeath

deathdate: 20000630	deathcause: Shock without mention of trauma
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Toxic epidermal necrolysis	20000616		15 Day		Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ciprobay ciprofloxacin hydrochloride	Oral	/Day	20000613-20000617	Pyrexia of unknown origin
2 erythrocin erythromycin	Oral	/Day	20000610-20000613	Pyrexia of unknown origin
3 novalgin filmdabletten metamizole	Oral	001 G gram(s)/Day	20000610-20000613	Pyrexia of unknown origin
4 novalgin tropfen	Oral	005 ml millilitre(s)/Day	20000528-20000528	Pyrexia of unknown origin
5 novalgin 1 g-injektionloesung	Intravenous (not otherwis	001 G gram(s)/Day	-	Pyrexia of unknown origin
6 omniflora bifidobacterium bifidum; lactobacillus gasseri	Oral	/Day	-	Other noninfective gastroenteritis and colitis
7 paracetamol paracetamol	Oral	001 G gram(s)/Day	-	Pyrexia of unknown origin
8 timonil 300 retard carbamazepine	Oral	/Day	20000508-20000603	Other forms of epilepsy
9 tranxilium 10 clorazepate dipotassium	Oral	/Day	-	

Drugs (concomitant)

10 ben-u-ron 1000 mg paracetamol	Oral	001 G gram(s)/Day	-	Other general symptoms
11 biftoral sirup lactulose	Oral	030 ml millilitre(s)/Day	-	Constipation
12 dermatop salbe prednicarbate	Topical	/Day	20000527-20000606	Dermatitis due to drugs and medicaments taken internally
13 fraxiparin 0,3 nadroparin calcium	Subcutaneous	0.3 Mg milligram(s)/Day	20000523-20000606	Embolism and thrombosis of unspecified site
14 gastrozepin pirenzepine dihydrochloride	Intravenous (not otherwis	/Day	20000520-20000527	Chronic ulcer of skin, unspecified site
15 tavegil clemastine	Oral	002 Df dosage form/Day	20000527-20000603	Dermatitis due to drugs and medicaments taken internally
16 Urbason methylprednisolone	Oral	100 Mg milligram(s)/Day	20000530-20000602	Dermatitis due to drugs and medicaments taken internally
17 zienam cilastatin sodium; imipenem	Intravenous (not otherwis	003 G gram(s)/Day	20000524-20000527	Pyrexia of unknown origin

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Cytomegalic inclusion disease			k.A.	concomitant disease
Oesophagitis NOS			k.A.	concomitant disease
Ulcer of oesophagus			k.A.	concomitant disease

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-01008526

Serious Yes	Death	Lifethr. Yes	Hospital. Yes	Disabling	Congenital	Other	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Female	Birth: privacy	Age: 64 Year	weight (kg): 95	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	20000908		19 Day		recovered/resolved
2 Toxic epidermal necrolysis	20000908		19 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 adumbran oxazepam	Oral	/	200008- -	
2 aquaphor 10 xipamide	Oral	/	20000814-20000907 -	Oedema
3 cibacen 10 benazepril hydrochloride	Oral	/	20000814-200008 -	Essential hypertension, unspecified
4 clont metronidazole	Oral	/	20000904-20000905 -	Infectious colitis, enteritis, and gastroenteritis
5 imeson nitrazepam	Oral	/	200008- -	Sleep disturbances
6 klacid clarithromycin	Oral	/	200009- -	Other specified gastritis
7 omeprazole omeprazole	Oral	/	200009- -	Other specified gastritis
8 paracetamol paracetamol	Unknown	/	200009- -	Other general symptoms
9 tavegil clemastine	Oral	/	200008- -	Unspecified pruritic disorder
10 tramal tramadol hydrochloride	Oral	/	200008- -	Other general symptoms
11 tramal 100 tramadol hydrochloride	Subcutaneous	/	200009- -	Other general symptoms
12 zyloric 300 allopurinol	Oral	/	200009- -	Hyperuricaemia

Drugs (concomitant)

13 antra 20 omeprazole	Oral	/	20000829- -	Infectious colitis, enteritis, and gastroenteritis
14 clexane enoxaparin	Subcutaneous	/	20000814- -	Coagulation defects, other and unspecified
15 irenat sodium perchlorate	Oral	/	200008- -	Thyrotoxicosis without mention of goitre or other cause
16 mcp metoclopramide hydrochloride	Oral	/	20000814- -	Nausea and vomiting

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont. k.A.	Comment
Hyponatraemia				concomitant disease

Primary source

Qualification:	Physician	observestudytype:
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Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-01008529

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes	Yes	Yes					

Patient

Initials:	Sex: Male	Birth: privacy	Age: 44 Year	weight (kg): 85	BMI_int:	Age fetus:
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Patientdeath

deathdate: 20000928	deathcause: Shock without mention of trauma
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	20000913		16 Day		Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
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1	ampho-moronal amphotericin b	Unknown	/	-	Other and unspecified mycoses
2	antra 40 mg omeprazole	Unknown	/	-	Chronic ulcer of skin, unspecified site
3	aredia pamidronate disodium	Intravenous (not otherwis	/	20000909-20000909	Hyperkalaemia
4	arterenol norepinephrine hydrochloride	Unknown	/	-	
5	beloc zok metoprolol succinate	Unknown	/	-	Cardiac dysrhythmia, unspecified
6	ben-u-ron paracetamol	Unknown	/	-	Other general symptoms
7	combactam 1 g sulbactam sodium	Unknown	/	-	Pneumonia, organism unspecified
8	dexamethasone dexamethasone	Unknown	/	-	Malignant neoplasm of liver, specified as secondary
9	diflucan 100 fluconazole	Unknown	/	-	Other and unspecified mycoses
10	fentanyl fentanyl	Unknown	/	-	
11	fragmin dalteparin sodium	Unknown	/	-	Embolism and thrombosis of unspecified site
12	heparin-natrium heparin sodium	Unknown	/	-	Embolism and thrombosis of unspecified site
13	ibuprofen ibuprofen	Unknown	/	-	Other general symptoms
14	kalinor-brausetabletten potassium carbonate; potassium citrate	Oral	/	-	Hypokalaemia
15	lasix injektionsloesung furosemide sodium	Unknown	/	-	Oedema
16	meronem meropenem	Unknown	/	-	Other specified septicaemias
17	methotrexat methotrexate	Unknown	/	-	Secondary malignant neoplasm of kidney
18	noctamid lormetazepam	Unknown	/	-	Sleep disturbances
19	noctazepam oxazepam	Unknown	/	-	
20	novalgin tropfen metamizole sodium	Oral	/	-	Pyrexia of unknown origin
21	pipril 4 g piperacillin sodium	Intravenous (not otherwis	004 G gram(s)/Day	20000909-20000911	Pneumonia, organism unspecified
22	tazobac piperacillin sodium; tazobactam sodium	Unknown	/	-	Diarrhoea
23	udicil cytarabine	Unknown	/	-	Other malignant lymphomas
24	valdispert valerian extract	Unknown	/	-	
25	zyloric 300 allopurinol	Oral	/	-	Hyperuricaemia

Drugs (concomitant)

26	nutriflex	Unknown	/	-
	amino acids nos; electrolytes nos; glucose			-
27	soluvit	Unknown	/	-
	vitamins nos			-
28	tavanic 500	Unknown	/	-
	levofloxacin			-
29	voltaren emulgel	Topical	/	-
	diclofenac diethylamine			-

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Colitis, enteritis, and gastroenteritis of presumed infectious origin			k.A.	concomitant disease
Renal failure, unspecified			k.A.	concomitant disease
Atrial fibrillation and flutter			k.A.	concomitant disease

Primary source

Qualification: Physician observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-01008536

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes					

Patient

Initials:	Sex: Female	Birth: privacy	Age: 7 Year	weight (kg): 25	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	20001027		21 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 paracetamol paracetamol	Oral	/	-	Pyrexia of unknown origin
2 spasmo-mucosolvan ambroxol hydrochloride; clenbuterol hydrochloride	Oral	/	-	Acute bronchitis

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
unbekannt			k.A.	concomitant disease: z.n. harnwegs-infektion
unbekannt			k.A.	concomitant disease: z.n. epstein-barr-virus

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-01008542

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		Yes	Yes					

Patient

Initials:	Sex: Female	Birth: privacy	Age: 4 Year	weight (kg): 17	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	20000611		26 Day		recovered/resolved
2 Erythema exudativum	20000611		26 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 cotrimoxazol sulfamethoxazole; trimethoprim	Oral	002 Df dosage form/Day	20000531-20000612	Unspecified otitis media
2 paracetamol paracetamol	Rectal	100 Mg milligram(s)/	20000611-20000611	Pyrexia of unknown origin

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-01010450

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes					

Patient

Initials:	Sex: Male	Birth: privacy	Age: 71 Year	weight (kg): 72	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	20010507		17 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 aerius desloratadine	Oral	/	-	Unspecified pruritic disorder
2 allopurinol-ratiopharm 300 allopurinol	Oral	/	-	Hyperuricaemia
3 beloc zok metoprolol succinate	Oral	/	-	Hypertension arterial
4 paracetamol paracetamol	Oral	/	-	Headache
5 rulid roxithromycin	Oral	/	-	Acute nasopharyngitis (common cold)
6 tri.-thiazid "stada" hydrochlorothiazide; triamterene	Oral	/	-	Hypertension arterial

Drugs (concomitant)

7 doxazosin doxazosin	Oral	/	-	Hypertension arterial
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Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Glaucoma, unspecified			k.A.	concomitant disease
Unspecified disorder of eyelid			k.A.	concomitant disease
Hyperplasia of prostate			k.A.	concomitant disease
Acute nasopharyngitis (common cold)			k.A.	concomitant disease

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-01010459

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes					

Patient

Initials:	Sex: Female	Birth: privacy	Age: 51 Year	weight (kg): 83	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Oedema NOS					recovered/resolved
2 Stevens Johnson syndrome	20000413		19 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ben-u-ron paracetamol	Rectal	/	-	Other general symptoms
2 cephalexin cefalexin	Oral	/	-	Unspecified otitis media
3 ibuprofen 400 ibuprofen	Oral	/	-	Other general symptoms

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-01010463

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes					

Patient

Initials:	Sex: Male	Birth: privacy	Age: 8 Year	weight (kg): 27	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Erythema multiforme severe	19990407		15 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 mucosolvan saft ambroxol hydrochloride	Oral	/	-	Cough
2 paracetamol paracetamol	Unknown	/	-	Pyrexia of unknown origin

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Pneumonia due to other specified organism			k.A.	concomitant disease

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-01010464

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes					

Patient

Initials:	Sex: Male	Birth: privacy	Age: 15 Year	weight (kg): 45	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19991203		11 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 acc acetylcysteine	Oral	/	-	Cough
2 paracetamol paracetamol	Oral	/	-	Pyrexia of unknown origin

Drugs (concomitant)

3 meditonsin h homeopatics nos	Oral	/	-	Cough
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Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-01010467

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes					

Patient

Initials:	Sex: Male	Birth: privacy	Age: 14 Year	weight (kg): 52	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19990910		14 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 kamistad-gel herbal extracts nos; lidocaine; thymol	Dental	/	-	Stomatitis
2 mucosolvan saft ambroxol hydrochloride	Oral	/	-	Cough
3 paracetamol paracetamol	Oral	/	-	Pyrexia of unknown origin

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Influenza with other respiratory manifestations			k.A.	concomitant disease
Other atopic dermatitis and related conditions			k.A.	concomitant disease

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-01010471

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		Yes	Yes					

Patient

Initials:	Sex: Male	Birth: privacy	Age: 21 Year	weight (kg): 60	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	20001222		4 Month		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ampho-moronal amphotericin b	Oral	/	-	
2 aspisol acetylsalicylate lysine	Intravenous (not otherwis	/	-	Other general symptoms
3 berlosin injekt metamizole	Intravenous (not otherwis	/	-	
4 combactam 1 g sulbactam sodium	Intravenous (not otherwis	/	-	
5 diflucan soft fluconazole	Oral	/	-	
6 dormicum midazolam maleate	Intravenous (not otherwis	/Day	-20001220	Other general symptoms
7 espumisan emulsion dimeticone; silicon dioxide, colloidal	Oral	/	-	
8 fenistil dimetindene maleate	Intravenous (not otherwis	/	-	
9 furesis comp furosemide; triamterene	Oral	/	-	
10 furosemide furosemide	Intravenous (not otherwis	/	-	
11 heparin heparin	Intravenous (not otherwis	/	-	
12 kalymin pyridostigmine bromide	Intravenous (not otherwis	/	-	
13 ketanest ketamine	Intravenous (not otherwis	/	-	
14 luminal phenobarbital	Intravenous (not otherwis	/	-	Other forms of epilepsy
15 pancuronium bromide pancuronium bromide	Intravenous (not otherwis	/	-	
16 paracefan clonidine hydrochloride	Intravenous (not otherwis	/	-	
17 paracetamol paracetamol	Oral	/	-	Other general symptoms
18 pipril 3 g piperacillin sodium	Intravenous (not otherwis	/	-	
19 sinophenin promazine hydrochloride	Intravenous (not otherwis	/	-	

Drugs (concomitant)

20	aminophylline aminophylline	Oral	/	-
21	bifiteral sirup lactulose	Oral	/	-
22	cerucal metoclopramide hydrochloride	Intravenous (not otherwis	/	-
23	cordanum talinalol	Oral	/	-
24	dopacard dopexamine hydrochloride	Intravenous (not otherwis	/	-
25	fentanyl fentanyl	Intravenous (not otherwis	/	-
26	hydergin dihydroergocornine mesilate; dihydroergocristine mesilate; dihydroergocryptine mesilate	Intravenous (not otherwis	/	-
27	insulin human insulin human	Intravenous (not otherwis	/	-
28	intralipid glycerol; lecithin; soya oil	Intravenous (not otherwis	/	-
29	inzolen aspartate zinc; cobalt hydrogenaspartate; copper hydrogenaspartate; magnesium aspartate; manganese hydrogenaspartate; potassium hydrogen aspartate	Intravenous (not otherwis	/	-
30	konakion phytomenadione	Intravenous (not otherwis	/	-
31	mucosolvan ambroxol hydrochloride	Intravenous (not otherwis	/	-
32	nimotop s nimodipine	Intravenous (not otherwis	/	-
33	panthenol panthenol	Intravenous (not otherwis	/	-
34	prednisolut prednisolone sodium succinate	Intravenous (not otherwis	/	-
35	sanostol ascorbic acid; colecalciferol; retinol; thiamine	Oral	/	-
36	solvit n ascorbic acid; biotin; cyanocobalamin; folic acid; nicotinamide; orotic acid; panthenol; pyridoxine hydrochloride; riboflavin sodium phosphate; thiamine hydrochloride	Intravenous (not otherwis	/	-
37	ulcogant-suspension sucralfate	Oral	/	-
38	vioxx 12,5 mg rofecoxib	Oral	/	-
39	vitalipid adult ergocalciferol; phytomenadione; retinol palmitate; tocopherol	Intravenous (not otherwis	/	-

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-02005472

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes					

Patient

Initials:	Sex: Male	Birth: privacy	Age: 41 Year	weight (kg): 70	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	20000613				recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac	Intramuscular	/	-	Low back pain
diclofenac			-	
2 paracetamol	Oral	/	-	
paracetamol			-	

Drugs (concomitant)

3 omeprazol	Oral	/	-
omeprazole			-

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Alcohol dependence syndrome			k.A.	concomitant disease
Alcoholic fatty liver			k.A.	concomitant disease

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-02005480

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		Yes	Yes					

Patient

Initials:	Sex: Male	Birth: privacy	Age: 10 Year	weight (kg): 25	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	20000706		22 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ben-u-ron soft paracetamol	Oral	/	-	Pyrexia of unknown origin

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Pneumonia due to other specified organism			k.A.	concomitant disease
Streptococcus infection in conditions classified elsewhere a			k.A.	concomitant disease

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-02005483

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		Yes	Yes					

Patient

Initials:	Sex: Male	Birth: privacy	Age: 3 Year	weight (kg): 17	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	20000820		37 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ben-u-ron saft paracetamol	Oral	/	-	Pyrexia of unknown origin

Drugs (concomitant)

2 bachblueten tropfen herbal extracts nos	Oral	/	-	General physical health deterioration
3 mucosolvan ambroxol hydrochloride	Respiratory (inhalation)	/	-	Acute nasopharyngitis (common cold)
4 Multivitamine ascorbic acid; ergocalciferol; folic acid; nicotinamide; panthenol; retinol; riboflavin; thiamine hydrochloride	Oral	/	-	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-02005484

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes					

Patient

Initials:	Sex: Male	Birth: privacy	Age: 32 Year	weight (kg): 58	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	20000820		15 Day		recovered/resolved
2 Toxic epidermal necrolysis	20000820		15 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 paracetamol	Oral	/	-	Headache
paracetamol			-	

Drugs (concomitant)

2 decortin	Oral	/	-	Unspecified allergic alveolitis
prednisone			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Other specified disease of white blood cells			k.A.	concomitant disease
Influenza with other respiratory manifestations			k.A.	concomitant disease

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-02005493

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes					

Patient

Initials:	Sex: Male	Birth: privacy	Age: 9 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	20001003		14 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ben-u-ron saft paracetamol	Oral	/	-	
2 paracodin dihydrocodeine bitartrate	Oral	/	-	Pyrexia of unknown origin

Drugs (concomitant)

3 bepanthen dexpanthenol	Unknown	/	-
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Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Diaphoresis			k.A.	concomitant disease
Pneumonia due to other specified organism			k.A.	concomitant disease

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-02005499

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes					

Patient

Initials:	Sex: Male	Birth: privacy	Age: 9 Year	weight (kg): 36	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	20000921		15 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ben-u-ron paracetamol	Unknown	/	-	Pyrexia of unknown origin

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-02005503

Serious Yes	Death Yes	Lifethr.	Hospital. Yes	Disabling	Congenital	Other	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Female	Birth: privacy	Age: 85 Year	weight (kg):	BMI_int:	Age fetus:
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Patientdeath

deathdate: 20001220	deathcause: Renal failure, unspecified
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	20001206				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ass 100 acetylsalicylic acid	Oral	200 Mg milligram(s)/Day	-	-
2 ciprobay ciprofloxacin hydrochloride	Oral	500 Mg milligram(s)/Day	20001203-20001206	Acute upper respiratory infections of other multiple sites
3 oedemase furosemide	Oral	/	-	Heart failure, unspecified
4 paracetamol paracetamol	Rectal	002 G gram(s)/Day	20001203-	-

Drugs (concomitant)

5 digimerck digitoxin	Oral	/	-	-
6 dipiperon pipamperone	Oral	/	-	Unspecified psychosis
7 dulcolax bisacodyl	Rectal	005 Mg milligram(s)/Day	-	Constipation
8 lactulose lactulose	Oral	040 ml millilitre(s)/Day	-	-
9 madopar benserazide hydrochloride; levodopa	Oral	188 Mg milligram(s)/Day	-	Paralysis agitans

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Pneumonia, organism unspecified			k.A.	concomitant disease

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-03003413

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 14 Year weight (kg): 60 BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stomatitis	20030107				Unknown
2 Angina tonsillaris	20030107				Unknown
3 Agranulocytosis	20030107		14 Day		recovered/resolved
4 Gingivitis	20030107				Unknown
5 Gingiva hypertrophia	20030107				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 antibiotika	Oral	/	20021229-20030105	Acute tonsillitis
2 fevarin fluvoxamine maleate	Oral	/Day	20020928-20030108	Depression
3 metamizol metamizole	Oral	/	20021008-	Other general symptoms
4 paracetamol paracetamol	Oral	45 ml millilitre(s)/	20021229-20030107	Pyrexia of unknown origin
5 piritramide piritramide	Intravenous (not otherwis /		20020926-20021006	Other general symptoms
6 tramal tramadol hydrochloride	Oral	/	20021008-	Other general symptoms
7 tramal tramadol hydrochloride	Intravenous (not otherwis /		20020926-20021008	Other general symptoms

Drugs (concomitant)

8 diclofenac diclofenac	Oral	100 Mg milligram(s)/Day	20021004-20030108	Other general symptoms
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Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Other specified paralytic syndromes			k.A.	concomitant disease
unbekannt			k.A.	concomitant disease: z. n. selbstmordversuch 09.02
Paraplegia			k.A.	concomitant disease

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-03014409

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes					

Patient

Initials:	Sex: Female	Birth: privacy	Age: 44 Year	weight (kg): 52	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Erythema exudativum	20010402		3 Week		recovered/resolved
2 Erythema multiforme severe	20010402		3 Week		recovered/resolved
3 Stevens Johnson syndrome	20010402				recovered/resolved
4 Lip swelling	20010402				recovered/resolved
5 Lip burning sensation of	20010402				recovered/resolved
6 Red eye	20010402				recovered/resolved
7 Buccal mucosa ulceration	20010402		3 Week		recovered/resolved
8 Conjunctivitis	20010402				recovered/resolved
9 Nasal irritation	20010402				recovered/resolved
10 Fever	20010402				recovered/resolved
11 Rigors	20010402				recovered/resolved
12 Pneumonia	20010402				recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 acc for injection acetylcysteine sodium	Intravenous (not otherwis	002 Df dosage form/Day	20010324-20010326	Bronchitis, not specified as acute or chronic
2 acc pulver acetylcysteine	Oral	001 Df dosage form/Day	20010327-20010327	Bronchitis, not specified as acute or chronic
3 adumbran oxazepam	Oral	001 Df dosage form/Day	20010327-20010403	Sleep disturbances
4 ben-u-ron 1000 mg paracetamol	Rectal	003 G gram(s)/Day	20010325-20010404	Pyrexia of unknown origin
5 cefpodoxime proxetil cefpodoxime proxetil	Oral	400 Mg milligram(s)/Day	20010327-20010402	Pneumonia, organism unspecified
6 euthyrox 50 levothyroxine sodium	Oral	050 µg microgram(s)/Day	20010324-20030404	
7 fragmin p dalteparin sodium	Subcutaneous	001 Df dosage form/Day	20010322-20010403	Thrombosis
8 Kodeinphosphat codeine phosphate	Oral	020 Gtt drop(s)/Day	20010331-20010403	Bronchitis, not specified as acute or chronic
9 mucosolvan inhalationsloesung ambroxol hydrochloride	Respiratory (inhalation)	002 Df dosage form/Day	20010328-20010403	Bronchitis, not specified as acute or chronic
10 mucosolvan saft ambroxol hydrochloride	Oral	030 ml millilitre(s)/Day	20010328-20010403	Bronchitis, not specified as acute or chronic
11 orelox 200 filmtabletten cefpodoxime proxetil	Oral	400 Mg milligram(s)/Day	20010327-20010402	Pneumonia
12 paspertin injektionsloesung metoclopramide hydrochloride	Intravenous (not otherwis	002 Df dosage form/Day	20010324-20010326	Unspecified functional disorder of stomach
13 paspertin tropfen metoclopramide hydrochloride	Oral	60 Gtt drop(s)/	20010326-20010326	Unspecified functional disorder of stomach
14 zinacef cefuroxime	Intravenous (not otherwis	003 G gram(s)/Day	20010325-20010326	Pneumonia

Drugs (concomitant)

15	dynexan a gel	Dental	/	20010402-	Stomatitis
	benzalkonium chloride; lidocaine hydrochloride			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
unbekannt			k.A.	concomitant disease: v.a. windpocken

Primary source

Qualification: Physician observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-03014766

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes					

Patient

Initials:	Sex: Male	Birth: privacy	Age: 10 Year	weight (kg): 30	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	20011109				not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 paracetamol paracetamol	Oral	/	- -	Influenza with other respiratory manifestations

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Pneumonia due to other specified organism			k.A.	concomitant disease

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-03014768

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes					

Patient

Initials:	Sex: Female	Birth: privacy	Age: 13 Year	weight (kg): 43	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	20011118		17 Day		Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 aspirin acetylsalicylic acid	Oral	/	-	Other general symptoms
2 kepinol sulfamethoxazole; trimethoprim	Oral	/	-	Urinary tract infection, site not specified
3 kepinol forte sulfamethoxazole; trimethoprim	Oral	/	-	Urinary tract infection, site not specified
4 paracetamol paracetamol	Oral	/	-	Pyrexia of unknown origin
5 penicillin "ratiopharm" benzylpenicillin sodium	Oral	/	-	Streptococcal sore throat

Primary source

Qualification: Physician

observestudytype:

Literature:

case: DE-BFARM-03014771

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other	Pregnancy	Med. conf.
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Initials: Sex: Male Birth: privacy Age: 12 Year weight (kg): BMI int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Erythema multiforme severe	20010627		24 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 cotrim sulfamethoxazole; trimethoprim	Oral	/	20010625-20010627	Pneumonia, organism unspecified
2 paracetamol paracetamol	Unknown	/	-	Pyrexia of unknown origin
3 penhexal phenoxymethylpenicillin potassium	Oral	/	-	Pneumonia, organism unspecified

MedDRA-term	Begin	End	Cont.	Comment
Pneumonia due to other specified organism			k.A.	concomitant disease
Other specified delays in development			k.A.	concomitant disease

Qualification:	Physician	observestudytype:
Literature:		

case: DE-BFARM-03014774

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other	Pregnancy	Med. conf.
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Initials: Sex: Male Birth: privacy Age: 8 Year weight (kg): 45 BMI int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Erythema multiforme severe	20010810		22 Day		Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ben-u-ron paracetamol	Oral	/	-	Pyrexia of unknown origin
2 klacid clarithromycin	Oral	/	-	Acute bronchitis
3 mucosolvan ambroxol hydrochloride	Oral	/	-	Acute bronchitis
4 paracetamol paracetamol	Oral	/	-	Pyrexia of unknown origin

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-03014784

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes					

Patient

Initials:	Sex: Female	Birth: privacy	Age: 10 Year	weight (kg): 30	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Erythema multiforme severe	19981222		21 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 mucosolvan saft ambroxol hydrochloride	Oral	/	-	Cough
2 paracetamol 500 paracetamol	Unknown	/	-	Pyrexia of unknown origin
3 phenoxymethylpenicillin phenoxymethylpenicillin	Oral	/	-	Cough

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-03014798

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		Yes	Yes					

Patient

Initials:	Sex: Male	Birth: privacy	Age: 10 Year	weight (kg): 40	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	20000809		1 Month		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 bisolvon bromhexine hydrochloride; erythromycin	Oral	/	-	Unspecified infectious and parasitic diseases
2 dolo-dobendan benzocaine; cetylpyridinium chloride	Oral	/	-	Throat pain
3 paracetamol paracetamol	Unknown	/	-	
4 p-mega-tablinen phenoxymethylpenicillin potassium	Oral	/	-	Throat pain
5 prospan hedera helix	Oral	/	-	Throat pain

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-03014800

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes					

Patient

Initials:	Sex: Male	Birth: privacy	Age: 11 Year	weight (kg): 38	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	20000426		23 Day		not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 acc 200 acetylcysteine	Oral	/	-	Acute bronchitis
2 paracetamol paracetamol	Oral	/	-	Pyrexia of unknown origin
3 rulid 300 roxithromycin	Oral	/	-	Acute bronchitis

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-03014802

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes					

Patient

Initials:	Sex: Male	Birth: privacy	Age:	weight (kg): 62	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome			1 Month		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 paracetamol paracetamol	Oral	/	-	Other general symptoms
2 ranitidine ranitidine	Oral	/	-	
3 zentropil phenytoin	Oral	/	-	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-03014961

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes	Yes	Yes					

Patient

Initials:	Sex: Female	Birth: privacy	Age: 71 Year	weight (kg): 75	BMI_int:	Age fetus:
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Patientdeath

deathdate: 20010916	deathcause: Pulmonary embolism
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	20010907				Unknown
2 Toxic epidermal necrolysis	20010907				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 adumbran oxazepam	Oral	/	20010818-20010828	Other unknown and unspecified cause of morbidity or mortality
2 ben-u-ron paracetamol	Rectal	001 G gram(s)/Day	20010822-20010822	Other general symptoms
3 irenat sodium perchlorate	Oral	20 Df dosage form/	20010904-20010904	Thyrotoxicosis without mention of goitre or other cause
4 lasix furosemide	Oral	040 Mg milligram(s)/Day	-	-
5 liquemin heparin	Subcutaneous	010 Iu international unit(s)	20010817-	Thrombosis
6 novalgin metamizole	Rectal	001 G gram(s)/Day	20010906-20010906	Other general symptoms
7 novalgin metamizole	Oral	/	-	Other general symptoms
8 omeprazol omeprazole	Oral	080 Mg milligram(s)/Day	20010817-	Other unknown and unspecified cause of morbidity or mortality
9 paspertin metoclopramide hydrochloride	Oral	/	20010828-20010902	Nausea and vomiting
10 refobacin gentamicin sulfate	Intravenous (not otherwis	001 Df dosage form/Day	20010812-	Pyrexia of unknown origin
11 rocefin ceftriaxone sodium	Intravenous (not otherwis	002 G gram(s)/Day	20010812-20010825	Pyrexia of unknown origin
12 saroten amitriptyline hydrochloride	Oral	025 Mg milligram(s)/Day	20010903-	Depression
13 sobelin clindamycin hydrochloride	Intravenous (not otherwis	600 Mg milligram(s)/Day	20010905-	Pyogenic arthritis, site unspecified
14 tramal tramadol hydrochloride	Oral	/	20010818-20010820	Other general symptoms
15 vancomycin vancomycin	Intravenous (not otherwis	001 G gram(s)/Day	20010824-	Pyrexia of unknown origin
16 vomex a dimenhydrinate	Rectal	001 Df dosage form/Day	20010827-20010827	Nausea and vomiting

Drugs (concomitant)

17	beloc zok metoprolol succinate	Oral	/	20010811-20010908	Coronary atherosclerosis
				-	
18	bifiteral lactulose	Oral	/	20010811-20010823	Constipation
				-	
19	magnesium stoffangabe (teilweise) unklar	Oral	/	20010811-20010811	Disorders of magnesium metabolism
				-	
20	mono-embolex certoparin sodium	Subcutaneous	/	20010811-20010811	Thrombosis
				-	
21	pantozol pantoprazole sodium	Oral	/	20010811-20010817	Unspecified functional disorder of stomach
				-	
22	staphylex flucloxacillin sodium	Oral	/	20010811-20010811	Pyrexia of unknown origin
				-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Endocarditis			k.A.	concomitant disease
Staphylococcus infection in conditions classified elsewhere			k.A.	concomitant disease
Quadriplegia			k.A.	concomitant disease
Unspecified monoarthritis			k.A.	concomitant disease
unbekannt			k.A.	concomitant disease: mitralklappen-endocarditis chronisch ni
Unspecified monoarthritis			k.A.	concomitant disease
Paralysis, unspecified			k.A.	concomitant disease
Acute renal failure, unspecified			k.A.	concomitant disease

Primary source

Qualification: Physician observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-03015161

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes	Yes	Yes					

Patient

Initials:	Sex: Female	Birth: privacy	Age: 73 Year	weight (kg): 60	BMI_int:	Age fetus:
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Patientdeath

deathdate: 20010725	deathcause: Multiple organ failure
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Dermatitis exfoliative	20010713				Unknown
2 Respiratory disorder NOS	20010713				Unknown
3 Multiple organ failure	20010713				Unknown
4 Skin erythema	20010713				Unknown
5 Epidermal necrolysis	20010713				Unknown
6 Blistering	20010713				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
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1	ampho-moronal suspension amphotericin b	Oral	200 Mg milligram(s)/Day	20010703-	Thrush
2	ass acetylsalicylic acid	Oral	100 Mg milligram(s)/Day	20010616-20010714	Coronary atherosclerosis
3	benuron paracetamol	Oral	500 Mg milligram(s)/	20010711-20010711	Other general symptoms
4	ceftriaxone sodium ceftriaxone sodium	Intravenous (not otherwis	002 G gram(s)/Day	20010629-20010703	Pyrexia of unknown origin
5	ciprobay ciprofloxacin hydrochloride	Oral	500 Mg milligram(s)/Day	20010620-20010711	Other cholecystitis
6	corvaton molsidomine	Oral	008 Mg milligram(s)/Day	20010703-	Coronary atherosclerosis
7	decortin prednisone	Intravenous (not otherwis	50 Mg milligram(s)/	20010703-20010703	Dyspnea
8	diazepam diazepam	Oral	003 Df dosage form/Day	20010616-20010703	Unspecified psychophysiological
9	digoxin digoxin	Oral	0.2 Mg milligram(s)/Day	20010703-20010714	Coronary atherosclerosis
10	elantan isosorbide mononitrate	Oral	/	20010703-20010714	Coronary atherosclerosis
11	eunerpan melperone hydrochloride	Oral	/Day	20010706-20010712	Unspecified psychophysiological
12	fragmin dalteparin sodium	Subcutaneous	2.5 Kiu iu (1000s)/Day	20010703-20010714	Thrombosis
13	furosemide furosemide	Intravenous (not otherwis	/Day	20010703-20010714	Essential hypertension, unspecified
14	Hefe getrocknet yeast dried	Oral	/Day	20010711-20010714	Other noninfective gastroenteritis and colitis
15	Isosorbidmononitrat isosorbide mononitrate	Oral	/Day	20010703-20010714	Coronary atherosclerosis
16	lasix furosemide	Intravenous (not otherwis	120 Mg milligram(s)/Day	20010703-20010714	Essential hypertension, unspecified
17	mcp metoclopramide hydrochloride	Oral	075 Gtt drop(s)/Day	20010704-20010712	Nausea and vomiting
18	melperone hydrochloride melperone hydrochloride	Oral	/Day	20010706-20010712	Other and unspecified special symptoms or syndromes, not
19	metolazone metolazone	Oral	2.5 Mg milligram(s)/Day	20010712-20010713	Essential hypertension, unspecified
20	molsidomine molsidomine	Oral	/Day	20010703-20010714	Coronary atherosclerosis
21	mono mack isosorbide mononitrate	Oral	001 Df dosage form/Day	20010703-	Coronary atherosclerosis
22	norvasc amlodipine besilate	Oral	010 Mg milligram(s)/Day	20010703-20010714	Coronary atherosclerosis
23	novodigal beta-acetyldigoxin	Oral	0.2 Mg milligram(s)/Day	20010703-	Coronary atherosclerosis
24	paracetamol paracetamol	Oral	500 Mg milligram(s)/Day	20010711-20010711	Other general symptoms
25	perenterol saccharomyces boulardii	Oral	300 Mg milligram(s)/Day	20010711-	Other noninfective gastroenteritis and colitis
26	plavix clopidogrel sulfate	Oral	075 Mg milligram(s)/Day	20010704-20010714	Coronary atherosclerosis
27	Prednisolon Natriumsuccinat prednisolone sodium succinate	Intravenous (not otherwis	50 Mg milligram(s)/	20010703-20010703	Dyspnea
28	rocephin ceftriaxone sodium	Intravenous (not otherwis	002 G gram(s)/Day	20010629-20010703	Pyrexia of unknown origin

29	tranxene clorazepate dipotassium	Oral	010 Mg milligram(s)/Day	20010710-20010714	Other and unspecified special symptoms or syndromes, not
30	tranxilium clorazepate dipotassium	Oral	010 Mg milligram(s)/Day	20010710-	Unspecified psychophysiological
31	zaroxolyn metolazone	Oral	2.5 Mg milligram(s)/Day	20010712-20010713	Essential hypertension, unspecified
Drugs (concomitant)					
32	amaryl glimepiride	Oral	001 Df dosage form/Day	20010616-20010703	Diabetes mellitus
33	atorvastatin calcium atorvastatin calcium	Unknown	20 Mg milligram(s)/	20010703-	
34	bromazepam bromazepam	Unknown	/	20010713-20010714	
35	captopril captopril	Oral	001 Df dosage form/Day	20010616-	Coronary atherosclerosis
36	carbimazole carbimazole	Oral	/Day	20010621-20010714	Thyrototoxicosis without mention of goitre or other cause
37	concor bisoprolol fumarate	Oral	005 Mg milligram(s)/Day	20010616-	Coronary atherosclerosis
38	delix ramipril	Oral	005 Mg milligram(s)/Day	20010616-	Coronary atherosclerosis
39	glimepiride glimepiride	Oral	3 Mg milligram(s)/	20010703-	
40	haldol haloperidol	Oral	003 Gtt drop(s)/Day	20010713-	Unspecified psychophysiological
41	insulin glargine insulin glargine	Subcutaneous	016 Df dosage form/Day	20010629-	Diabetes mellitus
42	irenat sodium perchlorate	Oral	/Day	20010618-	Thyrototoxicosis without mention of goitre or other cause
43	Kaliumchlorid potassium chloride	Unknown	/	20010703-20010714	
44	kalium-duriles potassium chloride	Oral	008 Df dosage form/Day	20010703-	
45	lantus insulin glargine	Subcutaneous	/Day	20010616-20010629	Diabetes mellitus
46	lexotanil bromazepam	Oral	0.5 Df dosage form/Day	20010713-	Unspecified psychophysiological
47	mediabet metformin hydrochloride	Oral	1.5 Df dosage form/Day	20010616-	Diabetes mellitus
48	pantozol pantoprazole sodium	Oral	002 Df dosage form/Day	20010616-	Unspecified disorder of stomach and duodenum
49	sortis atorvastatin calcium	Oral	020 Mg milligram(s)/Day	20010616-20010703	Other and unspecified hyperlipidaemia

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Calculus of gallbladder without mention of cholecystitis			k.A.	concomitant disease
unbekannt			k.A.	concomitant disease: z.n. myokard-infarkt 29/06/03

Primary source

Qualification: Physician
Literature: observestudytype:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-03015293

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes					

Patient

Initials:	Sex: Female	Birth: privacy	Age: 62 Year	weight (kg): 100	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Toxic epidermal necrolysis	20010324		54 Day		recovered/resolved
2 Stevens Johnson syndrome	20010324		54 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 acerbon lisinopril	Oral	005 Mg milligram(s)/Day	20010305- -	Hypertension arterial
2 adumbran oxazepam	Oral	001 Df dosage form/Day	20010305-20010305 -	Sleep disturbances
3 ass 500 acetylsalicylic acid	Oral	500 Mg milligram(s)/	20010306-20010311 -	Headache
4 aurorix moclobemide	Oral	300 Mg milligram(s)/Day	20010305- -	Depression
5 cotrim forte sulfamethoxazole; trimethoprim	Oral	002 Df dosage form/Day	20010319-20010322 -	Cystitis, unspecified
6 dormicum midazolam maleate	Oral	7.5 Mg milligram(s)/Day	20010320-20010325 -	Other unknown and unspecified cause of morbidity or mortality
7 fortecortin dexamethasone	Oral	/	20010306-20010307 -	Malignant neoplasm of brain, unspecified
8 imeson nitrazepam	Oral	001 Df dosage form/Day	20010307-20010311 -	Other unknown and unspecified cause of morbidity or mortality
9 innohep tinzaparin sodium	Subcutaneous	001 Df dosage form/Day	20010305- -	Thrombosis
10 novalgin metamizole	Oral	040 Df dosage form/Day	20010304-20010304 -	Other general symptoms
11 pantozol pantoprazole sodium	Oral	001 Df dosage form/Day	20010305- -	Unspecified functional disorder of stomach
12 paracetamol paracetamol	Oral	/	20010307-20010308 -	Other general symptoms
13 paspertin metoclopramide hydrochloride	Oral	020 Df dosage form/Day	20010319-20010319 -	Nausea and vomiting
14 phenhydan phenytoin	Oral	/	20010305-20010325 -	Convulsions
15 zentropil phenytoin	Oral	/	20010222-20010305 -	Convulsions

Drugs (concomitant)

16	atosil promethazine hydrochloride	Oral	/	20010305-20010305	Other and unspecified special symptoms or syndromes, not
17	bifiteral lactulose	Oral	/	20010306-20010306	Constipation
18	doxepin doxepin	Oral	/	20010305-20010305	Other and unspecified special symptoms or syndromes, not
19	fentanyl fentanyl	Intravenous (not otherwis	/	20010228-20010228	
20	liquemin n heparin sodium	Subcutaneous	/	20010226-20010305	Thrombosis
21	pancuronium pancuronium	Intravenous (not otherwis	/	20010228-20010228	
22	pepdul famotidine	Oral	/	20010226-20010305	Unspecified functional disorder of stomach
23	rohypnol flunitrazepam	Oral	/	20010228-20010228	Sleep disturbances
24	systral gel chlorphenoxamine hydrochloride	Topical	/	20010318-20010325	Unspecified pruritic disorder
25	thiopental thiopental	Intravenous (not otherwis	/	20010228-20010228	
26	valdispert valerian extract	Oral	/	200103-20010309	Other and unspecified special symptoms or syndromes, not

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Neoplasm of unspecified nature of brain			k.A.	concomitant disease
Contact dermatitis and other eczema due to other specified			k.A.	concomitant disease
Contact dermatitis and other eczema, unspecified cause			k.A.	concomitant disease
Obesity			k.A.	concomitant disease
Hemiplegia, unspecified			k.A.	concomitant disease

Primary source

Qualification: Physician observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-03015398

Serious Yes	Death	Lifethr. Yes	Hospital. Yes	Disabling	Congenital	Other	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Female	Birth: privacy	Age: 16 Year	weight (kg): 57	BMI_int: 18.19	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	20010408		39 Day		recovered/resolved
2 Thrombocytopenia	200104		6 Week		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
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DE-BFARM-03015398

1	ben-u-ron paracetamol	Rectal	500 Mg milligram(s)/Day	20010320-20010320	Other general symptoms
2	ben-u-ron paracetamol	Oral	500 Mg milligram(s)/Day	20010321-20010328	8 Day
3	ben-u-ron paracetamol	Oral	001 G gram(s)/Day	20010331-20010331	-
4	ben-u-ron paracetamol	Rectal	001 G gram(s)/Day	20010408-20010408	-
5	chloral hydrate chloral hydrate	Oral	002 G gram(s)/Day	20010327-20010327	Other and unspecified special symptoms or syndromes, not
6	chloral hydrate chloral hydrate	Rectal	/	20010327-20010330	4 Day
7	dibro-be mono potassium bromide	Oral	/	20010328-20010409	Epilepsy, unspecified
8	diclofenac diclofenac	Oral	150 Mg milligram(s)/Day	20010323-20010326	Other general symptoms
9	ergenyl valproate sodium	Intravenous (not otherwis	800 Mg milligram(s)/Day	20010329-20010329	-
10	ergenyl valproate sodium	Intravenous (not otherwis	800 Mg milligram(s)/Day	20010327-20010327	Epilepsy grand mal
11	frisium clobazam	Oral	010 Mg milligram(s)/Day	20010327-20010328	Other and unspecified special symptoms or syndromes, not
12	heparin sodium heparin sodium	Oral	002 Mg milligram(s)/Day	20010321-20010321	Embolism and thrombosis of unspecified site
13	heparin sodium heparin sodium	Oral	002 Mg milligram(s)/Day	20010326-20010330	5 Day
14	luminal phenobarbital	Intravenous (not otherwis	600 ml millilitre(s)/Day	20010320-20010320	Epilepsy grand mal
15	novalgin metamizole	Oral	500 Mg milligram(s)/Day	20010324-20010324	-
16	novalgin metamizole	Oral	500 Mg milligram(s)/	20010321-20010321	Other general symptoms
17	phenhydan phenytoin	Intravenous (not otherwis	/	20010320-20010322	Epilepsy grand mal
18	phenhydan phenytoin	Intravenous (not otherwis	/	20010326-20010328	3 Day
19	rivotril clonazepam	Intravenous (not otherwis	2.5 Mg milligram(s)/Day	20010327-20010327	Epilepsy grand mal
20	tavor lorazepam	Oral	002 Mg milligram(s)/Day	20010408-20010408	-
21	tavor lorazepam	Oral	002 Mg milligram(s)/Day	20010322-20010322	Epilepsy grand mal
22	timonil retard carbamazepine	Oral	100 lu international unit(s)	20010323-20010409	Embolism and thrombosis of unspecified site
23	zentropil phenytoin	Oral	/	20010329-20010408	Epilepsy, unspecified
					11 Day

Drugs (concomitant)

24	aciclovir aciclovir	Intravenous (not otherwis 1.5 G gram(s)/Day	20010320-20010324	Herpes simplex without mention of complication
25	decortin-h prednisolone	Oral 100 Mg milligram(s)/Day	20010408-20010408	Rash and other nonspecific skin eruption
26	dolantin pethidine hydrochloride	Intravenous (not otherwis 015 Mg milligram(s)/Day	20010323-20010323	Other general symptoms
27	dolantin pethidine hydrochloride	Intravenous (not otherwis 020 Mg milligram(s)/Day	20010408-20010408	
28	kavacur kava-kava rhizoma	Oral 045 Mg milligram(s)/Day	20010312-20010320	Other and unspecified special symptoms or syndromes, not
29	paediafusin calcium chloride dihydrate; glucose; magnesium chloride hexahydrate; malic acid; sodium acetate; sodium glycerophosphate	Intravenous (not otherwis 600 ml millilitre(s)/Day	20010320-20010323	
30	ringer lactado "grifols" calcium chloride dihydrate; potassium chloride; sodium chloride; sodium lactate	Intravenous (not otherwis 680 ml millilitre(s)/Day	20010323-20010324	
31	tbg Basisloesung	Intravenous (not otherwis 300 ml millilitre(s)/Day	20010408-20010409	
32	tbg Basisloesung	Intravenous (not otherwis 900 ml millilitre(s)/Day	20010409-20010409	
33	valium diazepam	Intravenous (not otherwis 010 Mg milligram(s)/Day	20010408-20010409	Partial epilepsy, with impairment of consciousness
34	zyrtec cetirizine hydrochloride	Oral 010 Mg milligram(s)/Day	20010408-20010408	Rash and other nonspecific skin eruption

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Allergic rhinitis, cause unspecified			k.A.	concomitant disease
Partial epilepsy, without mention of impairment of conscious			k.A.	concomitant disease

Primary source

Qualification: Physician observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-03016250

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes					Yes

Patient

Initials:	Sex: Female	Birth: privacy	Age: 85 Year	weight (kg): 54	BMI_int: 22.48	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	20030704	20030717	14 Day		not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 clexane enoxaparin	Subcutaneous	/	20030610-20030623 14 Day	Prophylaxis
2 coversum cor perindopril erbumine	Oral	1 Df dosage form/Day	20030628-20030705 8 Day	Hypertension
3 euthyrox levothyroxine sodium	Oral	/	20030607-20030705 29 Day	Thyroid disorder
4 lopirin cor captopril	Oral	4 Df dosage form/Day	20030618-20030627 10 Day	Hypertension
5 movicol macrogol; potassium chloride; sodium bicarbonate; sodium chloride	Oral	/	20030625-20030705 11 Day	Constipation
6 nacl-traegerloesung sodium chloride	Intravenous (not otherwis /		20030608-20030705 28 Day	
7 Natriumchlorid sodium chloride	Intravenous (not otherwis /		20030608-20030703 26 Day	Thyroid disorder
8 Natriumchlorid sodium chloride	Oral	/	20030607-20030705 29 Day	
9 norvasc amlodipine besilate	Oral	/	20030619-20030705 17 Day	
10 norvasc 5 mg amlodipine besilate	Oral	15 Mg milligram(s)/Day Tablet	20030611-20030616 6 Day	Hypertension
11 paracetamol paracetamol	Oral	1 G gram(s)/Day	20030624-20030629 6 Day	Pain
12 phenhydan phenytoin	Oral	300 Mg milligram(s)/Day	20030611-20030705 25 Day	Epilepsy
13 remergil mirtazapine		/	20030619-20030624 2 Year	Depression
14 thrombophob heparin sodium	Subcutaneous	10000 Iu international unit	20030624-20030705 12 Day	Thrombosis prophylaxis
15 vioxx dolor 50 mg tabletten rofecoxib	Oral	050 Mg milligram(s)/Day	20030702-20030704 3 Day	Backache, unspecified
16 ximovan zopiclone	Oral	1 Df dosage form/Day	20030626-20030626 -	Sleep disorder

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Pain			Yes	
Sleep disorder			Unknown	
Thyroid disorder			Yes	hyper o hypo thyroidism
Hyponatremia			Yes	
Depression			Unknown	
Hematoma subdural			Unknown	concomitant disease
Constipation			Yes	

Primary source

Qualification: Physician observestudytype:
Literature:

Narratives (nur bei elektronischem Eingang)

PFIZER INC, 15.02.2008 DE-PFIZER INC-2008011464

Sender diagnosis: not reported

Narrative

This is a report from the German Documentation Centre of Severe Skin Reactions - a sponsored project set up to collect suspected drug induced severe skin reactions leading to hospital admission (their local case # 9150065): This female patient at the time 85 years old was treated with NORVASC from 11JUN2003 to 16JUN2003 as well as from 19JUN2003 to 05JUL2003 and developed Stevens-Johnson-Syndrome beginning on 04JUL2003. The patient with known hypertension, hyperthyroidism (or hypothyroidism) and hyponatremia was admitted to hospital for unknown reason(s) where she stayed in the surgical ward from 01JUN2003 to 05JUN2003, in the neurologic department from 06JUN2003 to 24JUN2003 and in the medical department from 24JUN2003 to 06JUL2003. On 04JUL2003 the patient developed pruritus and generalised blotches on her skin and, during further course the first blisters on 06JUL2003 due to which she was transferred to the dermatologic ward. The adverse event was categorised as "not life-threatening". The event was documented to have lasted "until 17JUL2003" (date of discharge) and, contradictory, the outcome of the event documented as "not yet resolved" at time of documentation. According to the documentation the patient showed no signs of an infection or any other conspicuousness in association with the symptoms. On the basis of the detailed listing of all medications administered between 08JUN2003 and diagnosis on 06JUL2003 the following information was known, apart from NORVASC therapy dates: Since 07 or 08JUN2003 respectively the patient had been receiving Natriumchlorid p.o. nearly continuously until 05JUL2003 as well as intermittent NaCl i.v., Euthyrox from 07JUN2003 to 05JUL2003 due to hyper(hypo?)thyroidism, Clexane s.c. from 10JUN2003 to 23JUN2003 and Thrombophob s.c. from 24JUN2003 to 05JUL2003 for prophylaxis of thrombosis. For prophylaxis of seizure she received Phenhydan p.o. from 11JUN2003 to 05JUL2003. The hypertension had been treated with Lopirin Cor from 18JUN2003 to 27JUN2003 and with Coversum Cor as of 28JUN2003 until 05JUL2003. Additionally the patient received Remergil from 19JUN2003 to 24JUN2003 for depression, Paracetamol p.o. on 24JUN2003, 27JUN2003 and 29JUN2003 for pain as well as Vioxx from 02JUL2003 until 04JUL2003. Due to constipation the patient was treated with Movicol from 25JUN2003 to 05JUL2003. Because of sleep disorder she received Ximovan once on 26JUN2003. The causality was assessed by the experts in AUG2003; within the graduation reaching from 1 ("definite") to 7 ("not related") the causal relationship was assessed as 2 ("very probable") with Phenhydan, as 5 ("possible") with NORVASC, Euthyrox, Lopirin, Thrombophob, Paracetamol, Movicol, Ximovan and Coversum and as 6 ("questionable") with NaCl, Clexane, Remergil and Vioxx.

Company Clinical Evaluation Comment

The possibility that any of the suspect drugs may have contributed to the onset of the event cannot be excluded, based on temporal association and positive dechallenge. The specific role of amlodipine is difficult to assess.

Sender's comment:**Reporter's comment:**

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-03016936

Serious Yes	Death Yes	Lifethr. Yes	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Female	Birth: privacy	Age: 74 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Toxic epidermal necrolysis	20011007				Unknown
2 Respiratory disorder	20011007				Unknown
3 Multiple organ failure	20011007				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 amlodipine	Oral	010 Mg milligram(s)/Day	20010703-20010714	Coronary atherosclerosis
amlodipine			-	
2 amphotericin b	Oral	/	20010703-20010714	Candidiasis of unspecified site
amphotericin b			-	
3 aspirin	Oral	/Day	-20010714	
acetylsalicylic acid			-	
4 ciprofloxacin	Oral	500 Mg milligram(s)/Day	20010707-20010711	Other cholecystitis
ciprofloxacin			-	
5 clopidogrel	Oral	/Day	20010704-20010714	
clopidogrel			-	
6 clorazepate dipotassium	Oral	/	20010710-20010714	Restless leg syndrome
clorazepate dipotassium			-	
7 dalteparin sodium	Subcutaneous	/	20010703-20010714	Thrombosis
dalteparin sodium			-	
8 diazepam	Oral	/Day	-20010707	
diazepam			-	
9 frusemide	Intravenous (not otherwis	/Day	20010703-20010714	Essential hypertension,
furosemide			-	unspecified
10 Hefe	Oral	/Day	20010711-20010714	Diarrhoea
yeast			-	
11 Isosorbidmononitrat	Oral	/Day	20010703-20010714	
isosorbide mononitrate			-	
12 lenoxin	Oral	/	20010703-20010714	Coronary atherosclerosis
digoxin			-	
13 melperone hydrochloride	Oral	/Day	20010706-20010712	
melperone hydrochloride			-	
14 metoclopramide	Oral	/	20010704-20010712	Nausea and vomiting
metoclopramide			-	
15 molsidomine	Oral	/Day	20010703-20010714	
molsidomine			-	
16 paracetamol	Oral	500 Mg milligram(s)/	-	Other general symptoms
paracetamol			-	
17 prednisolon	Intravenous (not otherwis	50 Mg milligram(s)/	20010703-20010703	Left heart failure
prednisolone			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Diabetes mellitus			k.A.	concomitant disease
Thyrotoxicosis without mention of goitre or other cause			k.A.	concomitant disease
Other and unspecified hyperlipidaemia			k.A.	concomitant disease

Primary source

Qualification: observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-03018901

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		Yes						

Patient

Initials:	Sex: Female	Birth: privacy	Age: 24 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Erythema exudativum	20031123				not recovered/not resolved
2 Exanthema	20031123				not recovered/not resolved
3 Gamma-glutamyltransferase incr.	20031123				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 paracetamol paracetamol	Unknown	001 G gram(s)/Day	20031014-20031123	
2 rebif interferon beta-1a	Subcutaneous	/	20031014-20031124	Multiple sclerosis

Drugs (concomitant)

3 pantoprazole sodium pantoprazole sodium	Unknown	040 Mg milligram(s)/Day	-	
4 prednisolone prednisolone	Unknown	1 G gram(s)/	200309-20031123	Multiple sclerosis
5 tetrazepam tetrazepam	Unknown	050 Mg milligram(s)/Day	20031007-20031123	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-04000318

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Female	Birth: privacy	Age: 1 Year	weight (kg): 8	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	20010527		11 Day		recovered/resolved
2 Gastroenteritis	20010520				recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 nurofen fuer kinder fiebersaft ibuprofen	Oral	/	-	Pyrexia of unknown origin
2 paracetamol paracetamol	Unknown	/	-	Pyrexia of unknown origin

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Water excessive loss of			k.A.	concomitant disease
Lichenification and lichen simplex chronicus			k.A.	concomitant disease

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-04000327

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes					

Patient

Initials:	Sex: Male	Birth: privacy	Age: 39 Year	weight (kg): 69	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	20011201		20 Day		not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 baldrian valeriana officinalis root	Oral	/	-	Other and unspecified special symptoms or syndromes, not
2 carbamazepine carbamazepine	Oral	/	-	Other forms of epilepsy
3 ergenyl chrono valproate sodium; valproic acid	Oral	/	-	
4 leptilan 300 valproate sodium	Oral	/	-	
5 mcp-tropfen metoclopramide hydrochloride	Oral	/	-	
6 paracetamol stada paracetamol	Oral	/	-	Acute nasopharyngitis (common cold)
7 phenytoin phenytoin	Oral	/	-	

Drugs (concomitant)

8 kamillosan chamomile; chamomile oil; hexylresorcinol	Buccal	/Day	-	Dysphagia
9 orfiril valproate sodium	Oral	/	-	Other forms of epilepsy
10 ranitidine ranitidine	Oral	/	-	
11 taxofit vitamin e kapseln tocopheryl acetate	Oral	/	-	Weariness

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-04000336

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		Yes	Yes					

Patient

Initials:	Sex: Male	Birth: privacy	Age: 12 Year	weight (kg): 35	BMI_int: 14.95	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	20020119		17 Day		recovered/resolved
2 Toxic epidermal necrolysis	20020119		17 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 cec	Oral	/	-	Bronchitis
cefaclor monohydrate			-	
2 equisil soft	Oral	/	-	Cough
			-	
3 iberogast	Oral	/	-	Cough
herbal extracts nos			-	
4 paracetamol 500	Unknown	/	-	Pyrexia of unknown origin
paracetamol			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Debility, unspecified			k.A.	concomitant disease
Chronic sinusitis, unspecified			k.A.	concomitant disease

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-04000337

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes					

Patient

Initials:	Sex: Female	Birth: privacy	Age: 23 Year	weight (kg): 65	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Erythema multiforme severe	20020124				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 codipront codeine; phenyltoloxamine	Oral	/	-	Cough
2 diclo-puren 50 diclofenac sodium	Unknown	/	-	Pain in limb
3 paracetamol 500 paracetamol	Oral	/	-	Pain in limb
4 tramal tramadol hydrochloride	Unknown	/	-	
5 zithromax azithromycin	Oral	/	-	Influenza with other respiratory manifestations

Drugs (concomitant)

6 orfiril long 300 mg valproate sodium	Oral	/	-	Epilepsy, unspecified
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Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-04000343

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes					

Patient

Initials:	Sex: Male	Birth: privacy	Age: 38 Year	weight (kg): 75	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	20020226		19 Day		recovered/resolved
2 CRP positive	20020226		19 Day		recovered/resolved
3 LDH increased serum	20020226		19 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 acc acetylcysteine	Oral	/	-	Unspecified infectious and parasitic diseases
2 aspirin plus c acetylsalicylic acid; ascorbic acid	Oral	/	-	Debility, unspecified
3 paracetamol paracetamol	Oral	/	-	Unspecified infectious and parasitic diseases

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Pneumonia due to other specified organism			k.A.	concomitant disease

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-04000350

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes	Yes	Yes					

Patient

Initials:	Sex: Female	Birth: privacy	Age: 75 Year	weight (kg): 75	BMI_int:	Age fetus:
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Patientdeath

deathdate:	deathcause: Shock without mention of trauma
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Toxic epidermal necrolysis	20020412		2 Month		not recovered/not resolved
2 Stevens Johnson syndrome	20020412		2 Month		not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 furorese furosemide	Intravenous (not otherwis	080 Mg milligram(s)/Day	20020320-20020323	Frequency of urination and polyuria
2 lasix injektionsloesung furosemide sodium	Intravenous (not otherwis /		-	Frequency of urination and polyuria
3 mcp metoclopramide hydrochloride	Intravenous (not otherwis /		-	Nausea and vomiting
4 nitrazepam nitrazepam	Oral	/	-	Sleep disturbances
5 novalgin metamizole	Oral	/	-	Other general symptoms
6 pantozol 40 mg pantoprazole	Oral	040 Mg milligram(s)/Day	20020409-20020413	Unspecified functional disorder of stomach
7 paracetamol paracetamol	Oral	/	-	Other general symptoms
8 prednisolon prednisolone	Oral	/	-	Other specified disorders of skin
9 tramal tramadol hydrochloride	Oral	/	-	Other general symptoms

Drugs (concomitant)

10 antra omeprazole	Intravenous (not otherwis /		-	
11 baldrian valeriana officinalis root	Oral	/	-	Sleep disturbances
12 clont metronidazole	Intravenous (not otherwis /		-	Bacterial infection, unspecified, in conditions classified
13 orgaran danaparoid sodium	Subcutaneous	/	-	Thrombosis
14 paspertin metoclopramide hydrochloride	Intravenous (not otherwis /		-	Nausea and vomiting
15 periplasmal amino acids nos; electrolytes nos; sorbitol; xylitol	Intravenous (not otherwis /		-	
16 solu-decortin-h prednisolone sodium succinate	Intravenous (not otherwis /		-	Other specified disorders of skin
17 vancomycin vancomycin	Intravenous (not otherwis /		-	Other specified septicaemias

Primary source

Qualification: Physician
Literature:

observestudytype:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-04000775

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes			Yes					

Patient

Initials:	Sex: Female	Birth: privacy	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Facial swelling					Unknown
2 Skin necrosis					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 paracetamol 500 hexal paracetamol	Oral	500 Mg milligram(s)/Day	-	Pyrexia of unknown origin
			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Unspecified adverse effect of drug, medicament and biologi			k.A.	concomitant disease
Unspecified disorder of kidney and ureter			k.A.	concomitant disease

Primary source

Qualification:	observestudytype:
Literature:	

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-04003543

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	Yes		

Patient

Initials:	Sex: Female	Birth: privacy	Age: 62 Year	weight (kg): 94	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Lip swelling	20040229			17 Day	not recovered/not resolved
2 Marginal keratitis					not recovered/not resolved
3 Pain of skin	20040221			9 Day	not recovered/not resolved
4 Rash pruritic	200402				not recovered/not resolved
5 Skin disorder	20040225			13 Day	not recovered/not resolved
6 Skin reaction	20040221			9 Day	not recovered/not resolved
7 Vesicle	20040228			16 Day	not recovered/not resolved
8 Heartburn					not recovered/not resolved
9 Flu-like symptoms	20040213			1 Day	not recovered/not resolved
10 Fever	20040220			8 Day	not recovered/not resolved
11 Eye redness	20040302			19 Day	not recovered/not resolved
12 Conjunctivitis					not recovered/not resolved
13 Chills	20040220			8 Day	not recovered/not resolved
14 Transaminases increased	20040301			18 Day	not recovered/not resolved
15 Creatinine increased	20040301			18 Day	not recovered/not resolved
16 Pruritus	20040221			9 Day	not recovered/not resolved
17 Mucosal erosion	20040229			17 Day	not recovered/not resolved
18 ESR increased	20040305			22 Day	Unknown
19 CRP increased	20040301			18 Day	not recovered/not resolved
20 Stevens Johnson syndrome	20040221	20040312	21 Day	11 Day	not recovered/not resolved
21 Allergic exanthema	20040215	20040218	4 Day	3 Day	not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 fenistil dimetindene maleate	Intravenous (not otherwis	1 Df dosage form/Day Solution for injection	20040216-20040216	Itching 1440 Minute
2 PARACETAMOL paracetamol	Oral	/ Tablet	20040212-20040212	Fever 1440 Minute
3 PARACETAMOL paracetamol	Oral	/ Tablet	20040216-20040218	Fever 3 Day
4 prednisolon prednisolone	Oral	/	20040216-20040227	Allergic exanthema 12 Day
5 solu-decortin-h prednisolone sodium succinate	Intravenous (not otherwis	250 Mg milligram(s)/Day Solution for injection	20040216-20040216	Exanthema 1440 Minute
6 VOLTAREN RESINAT diclofenac resinate	Oral	2 Df dosage form/Day Capsule	20040213-20040213	Fever 1440 Minute

Drugs (concomitant)

7 concor plus bisoprolol fumarate; hydrochlorothiazide	Unknown	/	-	Hypertension
8 KETEK telithromycin	Oral	1 Df dosage form/Day	20040224-20040224	Purulent bronchitis 1440 Minute

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Dermatitis			Unknown	due to drugs and medicaments taken internally
Hypertension arterial			Unknown	concomitant disease
Upper respiratory tract infection			Unknown	

Primary source

Qualification: Physician observestudytype:
 Literature:

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 06.02.2008 DE-002147023-PHNU2004DE01207

Sender diagnosis: not reported

Narrative

Initial report received on 12 Mar 2004. Physician reported about patient who took one Voltaren (diclofenac) dose, about 10-12 days ago, for flu symptoms. Shortly thereafter, the patient showed an "extreme" skin reaction, and antibiotic treatment with Ketek (telithromycin) was started. The patient was hospitalized on 1 Mar 2004, and high dose prednisolone over 3 days (starting with 250mg/d, then tapering) was given. Due to affection of the conjunctiva, a dermatological consultation was requested, and Ketek (telithromycin) was stopped. Histology confirmed the diagnosis of Stevens Johnson syndrome. The patient remained on topical treatment, and the condition greatly improved during the following days. The patient's discharge was foreseen for 12 Mar 2004. Both Voltaren and Ketek were named as suspected drugs.

Follow up received on 24 Mar 2004 (written documentation from internal specialist): The patient took Voltaren on 22 Feb 2004, for rheumatic pains due to upper respiratory tract infection. Stevens Johnson syndrome started on ("about") 23 Feb 2004 and lasted for "more than 14 days". According to reporter, pruritic rash started after Voltaren use, and the picture of Stevens Johnson syndrome occurred one day after starting Ketek therapy. On an attached lab sheet, the following "lab data relevant for the AE" were reported: increased creatinine, increased transaminases, CRP increase and ESR increase. The condition did not abate after stopping therapy (no rechallenge since). Outcome was indicated as "not yet recovered". The reporting physician rated causality for Voltaren as probable (Ketek was not explicitly named as suspect drug).

Follow up received on 4 Feb 2008 from RegiSCAR project (internal case number 9150136). The Voltaren drug was specified as Voltaren Resinat (diclofenac, colestyramin); and according to this information (in contrast to previous data), Voltaren Resinat was used on 13 Feb 2004, for fever. No previous Voltaren drug use was recorded in this follow up. Fenistil (dimetinden maleate) was used on 16 Feb 2004, due to pruritus. On 13 Feb 2004, flu like symptoms occurred; and allergic exanthema was present from 15 until 18 Feb 2004. Chills and fever occurred on 20 Feb, and skin pains, pruritus, generalized macules and spots were visible on 21 Feb 2004. Purulent bronchitis was seen on 23 Feb, and skin changes further spread on 25 Feb 2004. The patient later also presented heartburn, marginal keratitis, eye redness, lip swellings and mucosal erosions. The first vesicle was seen on 28 Feb 2004. The patient was hospitalised on 1 Mar 2004. Outcome was reported as "not yet recovered". By the expert from the reporting center, the following drugs were mentioned as suspected (i.e. with a causality of at least possible): Voltaren Resinat, Fenistil inj., paracetamol, Solu-Decortin (prednisolone-21-hydrogensuccinate), and prednisolone; Ketek was not regarded as suspected. As other contributing or causal factor, flu infection was mentioned.

Sender's comment:

Serious adverse event [hospitalisation], Stevens-Johnson syndrome assessed as listed according to the Core Data Sheet.
 Reporting healthcare professional causality is suspected.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-05001020

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes					

Patient

Initials:	Sex: Male	Birth: privacy	Age: 18 Year	weight (kg): 54	BMI_int: 18.25	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Drug eruption	20050113		9 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 elobact 500 cefuroxime axetil	Oral	003 Df dosage form/Day	20050111-20050114	Acute tonsillitis
2 paracetamol paracetamol	Oral	001 G gram(s)/Day	20050111-20050114	Acute tonsillitis
3 tantum verde benzydamine hydrochloride	Oral	004 Df dosage form/Day	20050111-20050114	Acute tonsillitis

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-05006031

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Female	Birth: privacy	Age: 62 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Pyrexia					Unknown
2 Vomiting NOS					Unknown
3 Nausea					Unknown
4 Eyelid oedema					Unknown
5 Exfoliation NOS					Unknown
6 Generalised erythema					Unknown
7 Delayed type hypersensitivity					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 paracetamol	Oral	/	-	Headache
paracetamol			-	

Primary source

Qualification:	Physician	observestudytype:
Literature:	Contact dermatitis 2000; Jul; 43(1): 60-1 Irion R.	

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-05006468

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes			Yes			Yes		

Patient

Initials:	Sex: Male	Birth: privacy	Age: 39 Year	weight (kg): 68	BMI_int: 20.31	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Eosinophilia	20050313	20050402	21 Day	2 Day	recovered/resolved
2 Stevens-Johnson syndrome	20050313	20050404	23 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 cotrimoxazol sulfamethoxazole; trimethoprim	Oral	/	20010601-	
2 ibuprofen ibuprofen	Oral	/	-	Toothache
3 kaletra lopinavir; ritonavir	Unknown	/	20050113-20050316 63 Day	HIV infection
4 paracetamol paracetamol	Oral	/	-	Pain NOS

Drugs (concomitant)

5 combivir lamivudine; zidovudine	Oral	/	20050113-20050316 63 Day
6 prothipendyl prothipendyl	Oral	/	20050312-20050312 -

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Polytoxicomania			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-05009958

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No				Yes		

Patient

Initials:	Sex: Female	Birth: privacy	Age: 24 Year	weight (kg): 55	BMI_int: 20.2	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens-Johnson syndrome	20050703			6 Day	not recovered/not resolved
2 Blisters	20050703			6 Day	not recovered/not resolved
3 Buccal mucosa ulceration	20050703			6 Day	not recovered/not resolved
4 Maculopapular rash	20050703			6 Day	not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 isoniazid isoniazid	Oral	0.3 G gram(s)/Day	20050628-20050703 6 Day	Tuberculosis
2 paracetamol paracetamol	Oral	1.5 G gram(s)/Day	20050628-20050703 6 Day	Tuberculosis
3 pyrazinamide pyrazinamide		1.5 G gram(s)/Day	20050621- -	Pulmonary tuberculosis
4 rifampicin rifampicin	Oral	0.4 G gram(s)/Day	20050628-20050703 6 Day	Tuberculosis

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-07063959

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	Yes	Yes	No	No	No		

Patient

Initials:	Sex: Female	Birth: privacy	Age: 43 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Toxic epidermal necrolysis			20 Day	1 Day	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 acetylsalicylic acid	Unknown	/	-	Drug use for unknown indication
acetylsalicylic acid			-	
2 fluconazole	Oral	1 Df dosage form/	-	Vaginal candidiasis
fluconazole		TABLET	1 Day	
3 PARACETAMOL	Unknown	/	-	Drug use for unknown indication
paracetamol			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Multiple sclerosis			Unknown	
Diabetes mellitus			Unknown	

Primary source

Qualification:	Other Health Professional	observestudytype:
Literature:	Lewerenz V, Bruch-Gerharz D, Ruzicka T, Kruse R. Toxic epidermal necrolysis (Toxische epidermale Nekrolyse). Der Hautarzt. 2006;57(4):322-4.	

Narratives (nur bei elektronischem Eingang)

SANOFI-SYNTHELABO, 19.07.2007 DE-SANOFI-SYNTHELABO-A01200707407

Sender diagnosis: not reported

Narrative

Reaction as reported: toxic epidermal necrolysis
 LLT: TOXIC EPIDERMAL NECROLYSIS / 10044223
 PT: TOXIC EPIDERMAL NECROLYSIS / 10044223

Narrative:
 Case received from a literature article in local language on 06-Jul-2007.

Anamnesis:
 A 43-year-old female patient was admitted as emergency case in condition with generalized skin changes, fever and severely reduced general condition, 1 day after intake of a tablet fluconazole for vaginal candidosis and 1 week after intake of acetylsalicylic acid and paracetamol. Secondary findings of insulin-dependent type I diabetes mellitus as well as multiple sclerosis were known. The patient stated not to take any other drugs.

Skin findings and other findings:
 Maculae and plaques of cockade-shaped configuration, partially with central blister or erosions, were found on the entire integument. On the lips, erosions and crusts were found. In the genital region, vesicles and erosions located in groups were found and the labia were erythematoid and showed oedematous swelling. Nikolsky's sign was positive on healthy skin.

Pathological laboratory values: CRP 6.6 mg/dL.
 Microbiological findings (vulva): mycological culture: Candida albicans, Candida glabrata. Virus culture: normal. HIV serology: negative.
 ENT examination: affection of auditory canals, nose, mouth and hypopharynx. No affection of the glottis.
 Ophthalmologic examination: no affection of the eyes.
 Internal-medical examination: no systemic affection.

Histopathology:

The epithelium consisted of necrotic keratinocytes in all layers. Between them, a mild infiltrate of lymphocytes and histiocytes as well as individual neutrophils was found. Focal findings of erythrocytes and serum infiltrations.

Diagnosis:

Toxic epidermal necrolysis.

Therapy and progress:

The patient refused to be transferred to an intensive-care unit. We performed an oral therapy with a massive dose of prednisolone (initially 100 mg/day) as well as a local therapy with antiseptics, triamcinolone acetonide cream and non adhesive wound dressings. The genital candidosis was treated locally with nystatin. A significant improvement of the skin findings occurred. The patient left the clinic on the 20th day against medical advice so that allergological diagnoses could not be completed.

This case was also captured in Aventis database with reference 200713339EU.

Sender's comment:

Based on the chronology and the absence of alternative explanation the causal role of fluconazole cannot be excluded in the occurrence of the event, however paracetamol and ASA may have also contributed to the adverse reaction.

Reporter's comment:

Dr. R. Pfleger Chemische Fabrik GmbH, 19.03.2009 DE-PFLEGER-000063

Sender diagnosis: not reported

Narrative

A 43-year-old female patient with a medical history of diabetes mellitus type 1 and multiple sclerosis developed toxic epidermal necrolysis during treatment with fluconazole, paracetamol and acetylsalicylic acid. The patient recovered on corrective treatment including drug discontinuation and corticosteroid therapy.

Sender's comment:

The patient developed toxic epidermal necrolysis during treatment with fluconazole, paracetamol and acetylsalicylic acid. Due to the temporal connection and the pharmacological relationship between suspected drugs and adverse reaction the causality is judged as possible.

Reporter's comment:

Molimin Arzneimittel GmbH, 20.03.2009 DE-MOLIMIN-000016

Sender diagnosis: not reported

Narrative

A 43-year-old female patient with a medical history of diabetes mellitus type 1 and multiple sclerosis developed toxic epidermal necrolysis during treatment with fluconazole, paracetamol and acetylsalicylic acid. The patient recovered on corrective treatment including drug discontinuation and corticosteroid therapy.

Sender's comment:

The patient developed toxic epidermal necrolysis during treatment with fluconazole, paracetamol and acetylsalicylic acid. Due to the temporal

connection and the pharmacological relationship between suspected drugs and adverse reaction the causality is judged as possible.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Report from study

case: DE-BFARM-08008191

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		Yes

Patient

Initials:	Sex: Female	Birth: privacy	Age: 80 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	20031110	20031207	28 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 fenistil		/	20031006-	
dimetindene maleate			-	
2 FUROSEMIDE		120 Mg milligram(s)/Day	20031014-	
furosemide			-	
3 gelonida	Oral	1 Df dosage form/Day	-	
acetylsalicylic acid; codeine phosphate; paracetamol			-	
4 PARACETAMOL		500 Mg milligram(s)/Day	-	
paracetamol			-	
5 PHENYTOIN		0.1 Mg milligram(s)/Day	-	
phenytoin			-	
6 RANITIDINE		300 Mg milligram(s)/Day	20031014-	
ranitidine			-	
7 voltaren emulgel	Topical	1 Df dosage form/Day	-	
diclofenac diethylamine		Gel	-	

Primary source

Qualification:	Physician	observestudytype:	Other studies
Literature:			

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 22.01.2008 DE-NOVNCHHQ-PHNU2008DE00541

Sender diagnosis: not reported

Narrative

Initial report received on 14 Jan 2008 (report from Intensive Monitoring Project, Schäpf-Mockenhaupt-SCAR Project = RegiSCAR project, interview number 9150097). This patient was treated with several drugs during summer 2003 and autumn 2003, for unstated reasons. Voltaren Emulgel (diclofenac) was used once daily, but therapy duration was not given. Fenistil (dimetinden-maleate) was started on 6 Oct 2003 (indication and therapy duration unknown). On 10 Nov 2003, patient suffered from Stevens Johnson syndrome. The event lasted until 7 Dec 2003, and the patient made a complete recovery. As suspect drugs, Voltaren Emulgel, Fenistil, furosemid, ranitidine, paracetamol, Gelonida (paracetamol, codeinphosphate), and phenytoin were mentioned. No further information with respect to other causative factors could be given.

Sender's comment:

Serious adverse event report [medically significant] "Stevens-Johnson syndrome" assessed as unlisted according to the Core Data Sheet. However other alternative causes [other drugs: co-suspect] provide a possible explanation for the reported adverse event. Reporting healthcare professional causality is suspected.

This is a serious adverse event (medically significant hazard). The event Steven-Johnson syndrome was assessed as unlabelled according to the product information. However other alternative cause (concomitant medications) provides a possible explanation for the reported adverse event. Reporting healthcare professional causality was suspected.

Reporter's comment:

NOVARTIS, 22.01.2008 DE-002147023-PHNU2008DE00541

Sender diagnosis: not reported

Narrative

Initial report received on 14 Jan 2008 (report from Intensive Monitoring Project, SchÃ¶pf-Mockenhaupt-SCAR Project = RegiSCAR project, interview number 9150097). This patient was treated with several drugs during summer 2003 and autumn 2003, for unstated reasons. Voltaren Emulgel (diclofenac) was used once daily, but therapy duration was not given. Fenistil (dimetinden-maleate) was started on 6 Oct 2003 (indication and therapy duration unknown). On 10 Nov 2003, patient suffered from Stevens Johnson syndrome. The event lasted until 7 Dec 2003, and the patient made a complete recovery. As suspect drugs, Voltaren Emulgel, Fenistil, furosemid, ranitidine, paracetamol, Gelonida (paracetamol, codeinphosphate), and phenytoin were mentioned. No further information with respect to other causative factors could be given.

Sender's comment:

Serious adverse event report [medically significant] "Stevens-Johnson syndrome" assessed as unlisted according to the Core Data Sheet. However other alternative causes [other drugs: co-suspect] provide a possible explanation for the reported adverse event. Reporting healthcare professional causality is suspected.

This is a serious adverse event (medically significant hazard). The event Steven-Johnson syndrome was assessed as unlabelled according to the product information. However other alternative cause (concomitant medications) provides a possible explanation for the reported adverse event. Reporting healthcare professional causality was suspected.

Reporter's comment:

NOVARTIS, 24.01.2008 DE-SZ09-PHNU2008DE00541

Sender diagnosis: not reported

Narrative

Initial report received on 14 Jan 2008 (report from Intensive Monitoring Project, SchÃ¶pf-Mockenhaupt-SCAR Project = RegiSCAR project, interview number 9150097). This patient was treated with several drugs during summer 2003 and autumn 2003, for unstated reasons. Voltaren Emulgel (diclofenac) was used once daily, but therapy duration was not given. Fenistil (dimetinden-maleate) was started on 6 Oct 2003 (indication and therapy duration unknown). On 10 Nov 2003, patient suffered from Stevens Johnson syndrome. The event lasted until 7 Dec 2003, and the patient made a complete recovery. As suspect drugs, Voltaren Emulgel, Fenistil, furosemid, ranitidine, paracetamol, Gelonida (paracetamol, codeinphosphate), and phenytoin were mentioned. No further information with respect to other causative factors could be given.

Sender's comment:

Serious adverse event report [medically significant] "Stevens-Johnson syndrome" assessed as unlisted according to the Core Data Sheet. However other alternative causes [other drugs: co-suspect] provide a possible explanation for the reported adverse event. Reporting healthcare professional causality is suspected.

This is a serious adverse event (medically significant hazard). The event Steven-Johnson syndrome was assessed as unlabelled according to the product information. However other alternative cause (concomitant medications) provides a possible explanation for the reported adverse event. Reporting healthcare professional causality was suspected.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08008881

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes	Yes	Yes	No	No	No		Yes

Patient

Initials:	Sex: Male	Birth: privacy	Age: 31 Year	weight (kg):	BMI_int:	Age fetus:
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Patientdeath

deathdate: 20040812	deathcause: Stevens-Johnson syndrome
deathdate: 20040812	deathcause: Toxic epidermal necrolysis

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens-Johnson syndrome	20040728	20040812	16 Day	21 Day	fatal
2 Mucosal erosion					Unknown
3 Rash papular					Unknown
4 Toxic epidermal necrolysis	20040812				fatal
5 Macule	20040728				Unknown
6 Fever	20040803				Unknown
7 Oral ulceration	20040728		16 Day		fatal
8 Nasal ulcer	20040730		16 Day		fatal
9 Itching	20040728		16 Day		fatal
10 Genital ulceration	20040730		16 Day		fatal
11 Urogenital disorder	200407			0 Month	Unknown
12 Pain					Unknown
13 Oral candidiasis					Unknown
14 Hypertension arterial					Unknown
15 Anxiety	200407			0 Month	Unknown
16 Oral mucosa erosion	20040728	20040812	16 Day	21 Day	Unknown
17 Genital erosion	20040730	20040812	14 Day	23 Day	Unknown
18 Nasal mucosal disorder	20040730	20040812	14 Day	23 Day	Unknown
19 Lip erosion	20040728	20040812	16 Day	21 Day	Unknown
20 Toxic epidermal necrolysis	20040728	20040812	16 Day	21 Day	fatal
21 Sepsis	20040728	20040812	16 Day	21 Day	Unknown
22 Restlessness	20040728	20040812	16 Day	21 Day	Unknown
23 Fever	20040728	20040812	16 Day	21 Day	Unknown
24 Infection	20040728	20040812	16 Day	21 Day	Unknown
25 Macule	20040728	20040812	16 Day	21 Day	Unknown
26 Nasal stuffiness	20040728	20040812	16 Day	21 Day	Unknown
27 Nausea	20040728	20040812	16 Day	21 Day	Unknown
28 Pain of skin	20040728	20040812	16 Day	21 Day	Unknown
29 Pruritus	20040729	20040812	15 Day	22 Day	Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
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1	ampho-moronal amphotericin b	Oral	6 Df dosage form/Day	20040624-20040727 Candidiasis of mouth 34 Day
2	beloc-zok mite metoprolol succinate	Oral	1 Df dosage form/	20040702-20040711 Hypertension arterial 10 Day
3	benuron paracetamol	Oral	/	20040720-20040720 Fever 1 Day
4	benuron paracetamol	Oral	/	20040716-20040717 Fever 2 Day
5	benuron paracetamol	Oral	20 ml millilitre(s)/Day	20040717-20040717 Fever -
6	benuron paracetamol	Oral	/	20040723-20040725 Fever 3 Day
7	Benuron Saft paracetamol	Oral	20 ml millilitre(s)/ 244	20040717-20040717 Fever 1 Day
8	Benzodiazepin nnb	Oral	5 Mg milligram(s)/ 231	20040719-20040719 Restlessness -
9	Calciumcarbonat calcium carbonate	Oral	/	20040702-20040727 Calcium supplementation 26 Day
10	decortin prednisone	Oral	/	20040625-20040727 Acute myelocytic leukemia 33 Day
11	DORMICUM midazolam	Intravenous (not otherwis	5 Mg milligram(s)/ 243	20040719-20040719 Restlessness -
12	DORMICUM midazolam	Oral	5 Mg milligram(s)/	20040719-20040719 Anxiety 1 Day
13	dridase oxybutynin hydrochloride	Oral	5 Mg milligram(s)/Day 245	20040726-20040727 Urinary tract pain 2 Day
14	eusaprim forte sulfamethoxazole; trimethoprim	Oral	/Day 245	20040707-20040727 Coxitis 21 Day
15	Eusaprim forte sulfamethoxazole; trimethoprim	Oral	/	20040707-20040727 Coxitis 21 Day
16	fraxiparin nadroparin calcium	Subcutaneous	0.4 lu international unit(s)/ 230	20040626-20040717 Thrombosis prophylaxis 22 Day
17	fraxiparin nadroparin calcium	Subcutaneous	0.4 ml millilitre(s)/Day 120	20040626-20040717 Thrombosis prophylaxis 22 Day
18	glucose glucose	Intravenous (not otherwis	500 ml millilitre(s)/Day 124	20040724-20040727 Glucose decreased 4 Day
19	kalium duriles potassium chloride	Oral	3* /2 Day	20040623-20040727 Potassium replacement 35 Day
20	meronem meropenem	Intravenous (not otherwis	3 G gram(s)/Day 243	20040725-20040727 Infection 3 Day
21	midazolam midazolam	Intravenous (not otherwis	5 Mg milligram(s)/ 120	20040719-20040719 Anxiety -
22	nacl sodium chloride	Intravenous (not otherwis	500 ml millilitre(s)/Day 124	20040723-20040727 Volume blood decreased 5 Day
23	novalgine metamizole	Intravenous (not otherwis	1 Df dosage form/Day	20040725-20040725 Pain 1 Day
24	NOVALGIN /00039501/ metamizole	Intravenous (not otherwis	/ 231	20040725-20040725 Pain -
25	otriven xylometazoline hydrochloride	Nasal	6 Df dosage form/Day 146	20040720-20040727 Nasal stuffiness 8 Day
26	PANTOZOL pantoprazole	Oral	/ 134	20040623-20040726 Prophylaxis 34 Day
27	PANTOZOL /01263202/ pantoprazole sodium	Oral	/	20040623-20040726 Prophylactic 34 Day
28	paspertin metoclopramide hydrochloride	Oral	/	20040721-20040721 Nausea 1 Day

29	paspartin metoclopramide hydrochloride	Oral	/	20040722-20040725 Nausea 4 Day
30	paspartin metoclopramide hydrochloride	Oral	/	20040718-20040719 Nausea 2 Day
31	rifampicin rifampicin	Intravenous (not otherwis	600 Mg milligram(s)/Day 120	20040725-20040726 Sepsis 2 Day
32	rulid roxithromycin	Oral	300 Mg milligram(s)/Day	20040719-20040726 Fever 8 Day
33	sobelin clindamycin hydrochloride	Oral	1800 Mg milligram(s)/Day	20040623-20040725 Arthritis infective 33 Day
34	tavanic levofloxacin	Oral	500 Mg milligram(s)/Day 245	20040623-20040725 Coxitis 33 Day
35	thomaejonin calcium chloride dihydrate; magnesium chloride hexahydrate; potassium chloride; sodium acetate; sodium chloride	Intravenous (not otherwis	1000 ml millilitre(s)/Day 124	20040724-20040727 Volume blood decreased 4 Day
36	THROMBOLYTIC AGENTS	Intravenous (not otherwis	/	20040723-20040723 Platelet transfusion -
37	tramal tramadol hydrochloride	Intravenous (not otherwis	1 Df dosage form/Day 120	20040725-20040725 Pain 1 Day
38	tramal long tramadol hydrochloride	Oral	/Day	20040623-20040726 Pain 34 Day
39	ursofalk ursodeoxycholic acid	Oral	/	20040707-20040727 Gastroesophageal reflux 21 Day prophylaxis
40	vergentan alizapride hydrochloride	Intravenous (not otherwis	111.6 Mg milligram(s)/Day 243	20040724-20040727 Nausea 4 Day

Drugs (concomitant)

41	BELOC /00376903/ metoprolol succinate	Oral	/	20040702-20040711 Hypertension arterial 10 Day
42	Beloc-Zok metoprolol succinate	Oral	/	20040724-20040727 Hypertension arterial 4 Day
43	Calciumbrause calcium carbonate	Oral	/	20040702-20040727 Calcium decreased 26 Day
44	diflucan fluconazole	Oral	/	20040625-20040714 Oral candidiasis 20 Day
45	kalium duriles potassium chloride	Oral	/	20040623-20040727 Electrolyte replacement 35 Day
46	MERONEM /01250501/ meropenem	Intravenous (not otherwis	3 G gram(s)/Day	20040725-20040727 Infection 3 Day
47	ospur calcium lactate	Oral	/	20040707-20040710 Vitamin supplementation 4 Day
48	OSPUR D3 colecalfiferol	Oral	/	20040707-20040710 Vitamin supplementation 4 Day
49	THOMAEJONIN /01119801/ calcium chloride dihydrate; magnesium chloride hexahydrate; potassium chloride; sodium acetate; sodium chloride	Intravenous (not otherwis	1000 ml millilitre(s)/Day 230	20040724-20040727 Fluid replacement 4 Day
50	Thrombocyte concentrate platelets, human blood	Intravenous (not otherwis	/	20040723-20040723 Platelet transfusion 1 Day
51	ulcogant sucralfate	Oral	3 Df dosage form/Day	20040624-20040710 Gastric ulcer prophylaxis 17 Day
52	vigantoletten colecalfiferol	Oral	/	20040703-20040706 Vitamin supplementation 4 Day

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Acute myeloid leukaemia	1998		Unknown	
Unevaluable event	20040623	20040728	Unknown	
Anxiety disorder			Unknown	
Hypertension arterial			Unknown	
Bile reflux (esophageal)			Unknown	
Calcium supplementation			Unknown	
Candidiasis			Unknown	
Fever of unknown origin			Unknown	
Urinary tract pain			Unknown	
Nasal stuffiness			Unknown	
Nausea			Unknown	
Oral candidiasis			Unknown	
Pain			Unknown	
Potassium replacement			Unknown	
Arthritis infective			Unknown	
Sepsis			Unknown	
Prophylaxis			Unknown	
Infection			Unknown	
Platelet transfusion			Unknown	
Thrombosis prophylaxis			Unknown	
Vitamin supplementation			Unknown	
Fluid replacement NOS			Unknown	
Coxitis			Yes	
Bone marrow transplant			Unknown	Relevant medical history
Agitation			k.A.	
Arthritis			k.A.	
Myeloid leukaemia			k.A.	
Septicaemia			k.A.	
Vomiting			k.A.	

Primary source

Qualification: Physician

observestudytype:

Literature:

Narratives (nur bei elektronischem Eingang)ROCHE, 28.01.2008 DE-ROCHE-541274

Sender diagnosis: not reported

Narrative

A 31 YEAR-OLD MALE PATIENT EXPERIENCED THE FATAL EVENT OF STEVENS-JOHNSON SYNDROME, WHILE RECEIVING MIDAZOLAM (DORMICUM) FOR THE TREATMENT OF ANXIETY AND RESTLESSNESS, IN THIS INTENSIVE MONITORING STUDY.

THE PATIENT HAD A MEDICAL HISTORY OF PURULENT COXITIS, ACUTE MYELOID LEUKAEMIA, BONE MARROW TRANSPLANTATION, STATIONARY CARE, ORAL CANDIDOSIS, THROMBOSIS PROPHYLAXIS, PAIN, CANDIDA MYCOSIS, POTASSIUM SUBSTITUTION, ARTERIAL HYPERTENSION, CALCIUM SUBSTITUTION, VITAMIN D SUBSTITUTION, BILE REFLUX PROPHYLAXIS, FEVER, NAUSEA, NASAL STUFFINESS, VOLUME SUBSTITUTION, GLUCOSE SUBSTITUTION, SEPSIS, SUSPECTED INFECTION, GENITOURINARY TRACT PAIN.

TREATMENT WITH ORAL SOBELIN (600MG DOSAGE AMOUNT), ORAL TAVANIC (500MG DOSAGE AMOUNT), ORAL TRAMAL LONG, ORAL PANTOZOL (20MG DOSAGE AMOUNT) AND ORAL KALIUM DURILES WAS COMMENCED ON 23 JUN 2004. THERAPIES LASTED FOR 3 DAYS (SOBELIN, TAVANIC), 4 DAYS (TRAMAL LONG, PANTOZOL) AND 5 DAYS (KALIUM DURILES). ON 24 JUN 2004 ORAL AMPHO-MORONAL THERAPY WAS COMMENCED. AMPHO-MORONAL TREATMENT LASTED 39 DAYS. ORAL DIFLUCAN (400MG DOSAGE AMOUNT) THERAPY BEGAN ON 25 JUN 2004 AS WELL AS ORAL DECORTIN TREATMENT. DIFLUCAN TREATMENT LASTED 20 DAYS AND DECORTIN 39 DAYS.

ORAL BELOC-ZOK THERAPY COMMENCED ON 2 JUL 2004 AS WELL AS ORAL CALCIUM CARBONATE. BELOC-ZOK THERAPY LASTED 9 DAYS AND CALCIUM CARBONATE LASTED 31 DAYS. ORAL URSOFALK AND ORAL EUSAPRIM FORTE COMMENCED ON 7TH JULY 2007. URSOFALK AND EUSAPRIM THERAPY LASTED 29 DAYS. ORAL BENURON WAS TAKEN BETWEEN THE 16TH AND 17TH JULY 2004, ON THE 20TH JULY 2004 AND BETWEEN THE 23RD AND 25TH JULY 2004. ORAL BENURON SOLUTION WAS TAKEN ON THE 17TH JULY 2004. ORAL PASPERTIN DROPS WERE TAKEN BETWEEN THE 18TH AND 19TH JULY 2004 AND ON THE 21ST JULY 2004. INTRAVENOUS MIDAZOLAM (DORMICUM) WAS ADMINISTERED ON THE 19TH JULY 2004 AND DISCONTINUED THE SAME DAY. ORAL RULID (300MG DOSAGE AMOUNT) WAS TAKEN BETWEEN THE 19TH AND 26TH JULY 2004 AND TOPICAL OTRIVEN BETWEEN THE 20TH AND 27TH JULY 2004. ORAL PASPERTIN LASTED BETWEEN THE 22ND AND 25TH JULY 2004. CONCENTRATED THROMBOCYTES WERE INTRAVENOUSLY ADMINISTERED ON 23RD JULY 2004. INTRAVENOUS NACL WAS COMMENCED ON 23RD JULY 2004 AND COMPLETED ON 27TH JULY 2004. INTRAVENOUS VERGENTAN, GLUCOSE, THOMAEJONIN WERE ALL COMMENCED ON 24TH JULY 2004 AND FINISHED ON 27TH JULY 2004. INTRAVENOUS NOVALGIN AND TRAMAL WERE ADMINISTERED ON 25TH JULY 2004. INTRAVENOUS RIFAMPICIN AND INTRAVENOUS MERONEM WERE COMMENCED ON 25TH JULY 2004. RIFAMPICIN THERAPY LASTED 2 DAYS AND MERONEM 3 DAYS. ORAL DRIDASE (5MG DOSAGE AMOUNT) WAS TAKEN BETWEEN THE 26TH AND 27TH JULY 2004.

ON THE 28TH JULY THE PATIENT EXPERIENCED THE EVENT OF STEVENS-JOHNSON SYNDROME/TOXIC EPIDERMAL NECROSIS WITH BLISTERS AND EROSIONS, PAIN OF SKIN, COCKADES AND SPOTS, AND EROSIONS OF LIPS AND ORAL MUCOSA (ALL CO-MANIFESTATIONS). DURING THE WEEK BEFORE ONSET OF THIS EVENT THE PATIENT WAS REPORTED TO HAVE BEEN ON 26 DIFFERENT CO-MEDICATIONS. ON 29TH JULY 2004 HE ADDITIONALLY DEVELOPED PRURITIS (CO-MANIFESTATION). SUBSEQUENTLY ON 30TH JULY 2004 HE DEVELOPED EROSIONS OF GENITAL AND NASAL MUCOSA (BOTH CO-MANIFESTATIONS). ON 3 AUG 2004 HE DEVELOPED FEVER UP TO 38 DEG CENTIGRADE (CO-MANIFESTATION), THE OUTCOME OF THE AE WAS DEATH ON THE 12TH AUG 2004.

THE REPORTER ASSESSED THE EVENT OF FATAL STEVENS-JOHNSON SYNDROME AS PROBABLY RELATED TO EUSAPRIM FORTE. THE EVENT WAS LISTED AS HAVING A GOOD POSSIBILITY OF BEING RELATED TO TAVANIC, TRAMAL LONG, RULID, TRAMAL. IT WAS ALSO REPORTED TO BE POSSIBLY RELATED TO MIDAZOLAM, PASPERTIN, BENURON, URSOFALK, DECORTIN, PANTOZOL, VERGENTAN, NOVALGIN, RIFAMPICIN AND SOBELIN THERAPY. FINALLY IT WAS DEEMED THAT FRAXIPARIN, KALIUM DURILES, BELOC-ZOK MITE, CALCIUM, CONCENTRATED THROMBOCYTE, NACL, GLUCOSE, THOMAEJONIN, MERONEM AND DRIDASE HAD QUESTIONABLE CAUSALITY TO THE EVENT.

NO FURTHER INFORMATION WAS AVAILABLE.

Sender's comment:

CIOMS TEXT: FATAL STEVENS-JOHNSON SYNDROME IS NOT LISTED IN THE COMPANY REFERENCE DOCUMENT (CDS) FOR MIDAZOLAM (DORMICUM). BASED ON THE INFORMATION RECEIVED FOR THIS CASE A CAUSAL RELATIONSHIP BETWEEN STEVENS-JOHNSON SYNDROME AND TREATMENT WITH MIDAZOLAM IS ASSESSED AS POSSIBLE. COMPANY CAUSALITY WAS ASSESSED BASED ON THE NARANJO ALGORITHM.

; ***LOCAL ASSESSMENT: MELDUNG VOM DOKUMENTATIONSZENTRUM FÜR SCHWERE HAUTREAKTIONEN MAJA MOCKENHAUPT
TÄDLICHES STEVEN JOHNSON SYNDROM IST NICHT ALS NEBENWIRKUNG VON DORMICUM GELISTET.
ZUSAMMENHANG MÖGLICH. BEURTEILUNG SCHWIERIG DA PATIENT NOCH WEITERE 24 MEDIKAMENTE ERHIELT

EINZELFALLBEOBACHTUNG = KEIN HANDLUNGSBEDARF

Reporter's comment:

GlaxoSmithKline, 05.02.2008 DE-GlaxoSmithKline-D0055996A

Sender diagnosis: not reported

Narrative

This case was reported by a physician and described the occurrence of toxic epidermal necrolysis in a 31-year-old male patient who received

Co-trimoxazole (Eusaprim forte) over a period of 21 days for purulent coxitis. This report was received from the Intensified Monitoring from the Documentation Centre for Severe Skin Reactions in Freiburg.

The patient's past medical history included bone marrow transplant in 1998 because of acute myeloid leukaemia (AML).

Concurrent medical conditions included arterial hypertension, oral candidiasis and pain. Co-suspect medication included levofloxacin (Tavanic), tramadol (Tramal long), roxithromycin (Rulid), Tramadol (Tramal), metamizol (Novalgin), Rifampicin (Rifampicin) and Multiple other medication. Concurrent medications included Multiple medication.

From 7 July 2004 to 27 July 2004 the patient took Co-trimoxazole (oral) daily. Approximately 21 days after starting Co-trimoxazole, on 28 July 2004, the patient experienced painful skin, macule, lip erosion and oral mucosa erosion. On the next day the patient developed itching and on 30 July 2004 erosion of genital mucosa and erosion of nasal mucosa. On 03 August 2004 the patient had fever up to 38.2 degC. The patient was diagnosed with toxic epidermal necrolysis and stevens johnson syndrome.

The physician considered the events to be life threatening.

Additional events included fever, nausea, restlessness, stuffed nose, sepsis and possible infection. These were treated between 16 and 27 July 2004, but it was not reported, if they were pre-existing.

The patient died on 12 August 2004 from toxic epidermal necrolysis and stevens johnson syndrome. It is unknown whether an autopsy was performed.

The reporting physician considered toxic epidermal necrolysis and stevens johnson syndrome were probably related to treatment with Co-trimoxazole, but also possibly related to co-suspect medication. Alternative explanations were purulent coxitis and AML.

Sender's comment:

Die Grunderkrankung des Patienten mit KM-transplantation und permanenter Immunsuppression sowie die antibiotische Begleitmedikation liefern eine

alternative Erklärung fuer das Auftreten des SJS und den letalen Verlauf .

Ein Kausalzusammenhang mit Eusaprim und SJS kann jedoch nicht ausgeschlossen werden.

Auf Stevens-Johnson Syndrom (SJS) weisen wir hin.

Aufgrund dieses Einzelfalles bleibt das Nutzen/Risikoverhaeltnis unverändert und eigenverantwortliche Massnahmen sind derzeit nicht notwendig.

Auf der Basis dieses Einzelfalles ergibt sich keine Änderung des wissenschaftlichen Kenntnisstandes und der Nutzen/Risiko-Relation. Weitergehende eigenverantwortliche Massnahmen halten wir derzeit nicht fuer erforderlich.

Reporter's comment:

NOVARTIS, 21.02.2008 DE-002147023-CHNY2008DE00863

Sender diagnosis: not reported

Narrative

Initial report received from an intensive monitoring project (German documentation center for severe skin disease dZh, Prof. Schäffpf, Freiburg) on 04 Feb 2008: A patient with pre-existing purulent coxitis, was hospitalised from 23 Jun 2004 to 28 Jul 2004. The patient started Otriven (xylometazoline hydrochloride) on 20 Jul 2004. He was concomitantly treated with Ulcogant oral (sucralfat), Diflucan oral (fluconazole), Fraxiparin subcutaneous (heparin), Sobelin oral (clindamycin), Tavenic oral (levofloxacin), Tramal long oral (tramadol), Pantozol oral (pantoprazole), Ampho-Moronal oral (amphotericin B), Decortin oral (prednisone), Kalium-Duriles (potassium chloride), Beloc-Zok oral (metoprolol), Calciumbrause oral (calcium carbonate), Vigantoletten oral (cholecalciferol), Ospur oral (cholecalciferol), Ursofalk oral (ursodeoxycholic), Eusaprim oral (trimethoprim, sulfamethoxazol), Benuron oral (paracetamol), with Beuron Saft oral (paracetamol), Paspertin drops (metoclopramide), Dormicum intravenous (midazolam), Rulid oral (roxithromycin), Paspertin oral (metoclopramide), "platelet concentrate" intravenous, with "sodium chloride" intravenous, Vergentan intravenous (alizaprid), glucose intravenous, Thomaesonin intravenous, Novalgin intravenous (metamizole), Tramal intravenous (tramadol), Rifampicin intravenous (rifampicin), Meronem intravenous (meropenem), and Dridase oral (oxybutynin). On 28 Jul 2004, the patient developed painful skin, "Kokardes" and skin spots as well as erosions of lips and oral mucosa which were regarded as onset date of Stevens-Johnson syndrome or toxic epidermal necrolysis (confirmed case). The patient developed itching on 29 Jul 2004, erosions of genital and nasal mucosa on 30 Jul 2004 and fever up to 38.2 C on 03 Aug 2004. For the duration of reaction the date 12 Aug 2004 was stated. The reaction was rated as life-threatening and the patient died. In Aug 2004 the intensive monitoring project assessed the causality with regard to the patient's multiple medication (using an algorithm with 8 different levels, ranging from definite to improbable) as follows: Eusaprim oral was rated as "probable"; Tavenic oral, Tramal long oral, Rulid oral, and Tramal intravenous were rated as "well possible"; Sobelin oral, Pantozol oral, Decortin oral, Ursofalk oral, Benuron oral, Beuron Saft oral, Paspertin Tropfen oral, Dormicum oral, Paspertin oral, Vergentan intravenous, Novalgin intravenous, and Rifampicin intravenous were rated as "possible". Fraxiparin subcutaneous, Ampho-Moronal oral, Kalium Duriles oral, Beloc-Zok oral, Calciumbrause oral, Otriven spray topical (stated daily dose 3x2 spray puffs), "platelet concentrate" intravenous, "sodium chloride" intravenous, Glucose intravenous, Thomaesonin intravenous, Meronem intravenous, and Dridase oral were rated as "questionable". The rest of compounds was rated as "no causal relationship". It was added that the patient had purulent coxitis and unspecified "KMT due to AML in 1998" as other possible risk factors. Outcome: death.

Sender's comment:

Otriven:

This is a serious adverse event (death). The events Stevens-Johnson syndrome and Toxic epidermal necrolysis (and the related symptoms) were assessed as unlabelled according to the product information. It is Novartis policy to assess a case with a fatal outcome as unlabelled unless the label specifically mentions death as possible outcome of the labelled risks. Investigator causality was not suspected.

Pantozole:

Serious adverse event report, TEN and SJS [death] assessed as unlisted according to the Basic Prescribing Information.

It is Novartis' policy to assess a case with a fatal outcome as unlisted unless the label specifically mentions death as a possible outcome of the listed risks. Reporting physician causality is suspected.

Reporter's comment:

Bene-Arzneimittel, 21.02.2008 DE-Bene-Arzneimittel-B213

Sender diagnosis: not reported

Narrative

No summary.

Sender's comment:

Dok.-Fall in 2004 aus Dok.-Zentrum f. schw. Hautreakt. in BRD (Freiburg); keine seriöse wiss. Beurteil. möglich, da insg. 33 Begleitmedik. und lebensbedrohl. Grunderkrankung (Ak. Myel. Leuk. + KMT); indiv. Bewert. durch Derm. Exp.-Gremium aufgr. spez. Bewert.-Algorhythmus: Kat. 5 = Zsh. möglich aufgr. engem zeitl. Zsh. mit oraler Einnahme von Paracetamol (P); P. zählt nicht zu gefährl. Auslösern der beobacht. ADR.; kein wiss. Erkenntnisgewinn hins. Sicherheit beteil. P.Prod. erkennbar!

Reporter's comment:

Bene-Arzneimittel, 21.02.2008 DE-Bene-Arzneimittel-B213

Sender diagnosis: not reported

Narrative

No summary.

Sender's comment:

Dok.-Fall in 2004 aus Dok.-Zentrum f. schw. Hautreakt. in BRD (Freiburg); keine seriöse wiss. Beurteil. möglich, da insg. 33 Begleitmedik. und lebensbedrohl. Grunderkrankung (Ak. Myel. Leuk. + KMT); indiv. Bewert. durch Derm. Exp.-Gremium aufgr. spez. Bewert.-Algorhythmus: Kat. 5 = Zsh. möglich aufgr. engem zeitl. Zsh. mit oraler Einnahme von Paracetamol (P); P. zählt nicht zu gefährl. Auslösern der beobacht. ADR.; kein wiss. Erkenntnisgewinn hins. Sicherheit beteil. P.Prod. erkennbar!

Reporter's comment:

Dr. Falk Pharma GmbH, 25.02.2008 DE-Dr. Falk Pharma GmbH-U-01/08

Sender diagnosis: not reported

Narrative

This report from a healthcare professional was received via BfArM (DE-BFARM-08008881) and pertains to a 31 year old male patient with bone marrow transplantation in 1998 following leucemia.

On 28 July 2004 the patient developed toxic epidermal necrolysis (TEN), while being hospitalized (since 23 June 2004). The patient died on the 12 August 2004.

Details on the concomitant medication (32 drugs) is captured under the according section. No information about the course of the event and treatments performed have been reported.

Sender's comment:

Dr. Falk Pharma GmbH assessed the reaction of TEN as serious and unexpected, due to the high number of concomitant medications the causality has not been assessed.

The "Dokumentationszentrum schwerer Hautreaktionen" assessed co-trimoxazole as probable cause of TEN; levofloxacin, tramadol and roxithromycin were related more closely to the event than UDCA (off label use: prophylaxis of bile reflux) and various other drugs.

In the light of the information that is currently available Dr. Falk Pharma GmbH, is of the opinion that the benefit risk balance of Ursofalk (UDCA) remains unchanged.

Reporter's comment:

GlaxoSmithKline, 26.02.2008 DE-GlaxoSmithKline-D0056191A

Sender diagnosis: not reported

Narrative

A physician reported the occurrence of Stevens Johnson syndrome in a male patient who received Co-trimoxazole (Eusaprim forte) tablet over a period of 21 days for coxitis and nadroparine calcium (Fraxiparin) injection over a period of 22 days for thrombosis prophylaxis.

This case was received from the Intensified Monitoring from the Documentation Centre for Severe Skin Reactions in Freiburg, via German regulatory authority (BfArM, number DE-BFARM-08008881) and another manufacturer (Sanofi Aventis) respectively.

Co-suspect medication included levofloxacin (Tavanic), tramadol hydrochloride (Tramal long, 1-0-1 0-0-1), roxithromycin (Rulid), tramadol hydrochloride (Tramal, 1 ampoule), metamizol (Novalgin, 1 ampoule), rifampicin (Rifampicin 0-1-0) and 21 other non-GSK suspect drugs. The patient also received 4 other non-GSK concurrent drugs.

The patient's past medical history included bone marrow transplantation under acute myeloid leukemia in 1998. Concurrent medical conditions included purulent coxitis.

On 26 June 2004 the patient started nadroparine calcium (subcutaneous) at 1x0.4IE. On 07 July 2004 the patient started Co-trimoxazole (oral) at 800 + 160 mg every Monday, Wednesday and Friday in the morning.

Treatment with nadroparine calcium was discontinued on 17 July 2004.

On 28 July 2004, 21 days after starting Co-trimoxazole and 33 days after starting nadroparine calcium, the patient experienced painful skin, cocardic macule, lip erosion and oral mucosa erosion. On 29 July 2004, 22 days after starting Co-trimoxazole and 34 days after starting nadroparine calcium, the patient experienced itching. On 30 July 2004, 23 days after starting Co-trimoxazole and 35 days after starting nadroparine calcium, the patient experienced erosion of genital mucosa and erosion of nasal mucosa.

In July 2004, at an unknown time after starting Co-trimoxazole, the patient was treated for fever, nausea, anxiety, stuffed nose, sepsis, possible infection and disorder of genitourinary tract. On 03 August 2004, body temperature was up to 38.2 degC.

Treatment with Co-trimoxazole was discontinued on 27 July 2004.

On 28 July 2004, post-treatment the patient experienced Stevens Johnson syndrome and toxic epidermal necrolysis.

Additionally the patient was treated for oral candidiasis, pain and hypertension arterial, but start date of these events was not reported.

The physician considered the events to be life threatening.

The reporting physician considered the events were probably related to treatment with Co-trimoxazole. The reporting physician considered the events were possible related to treatment with nadroparine calcium, levofloxacin, tramadol hydrochloride, roxithromycin, metamizol, rifampicin and 21 other non-GSK suspect drugs. Purulent coxitis and bone marrow transplantation under acute myeloid leukemia was also considered a possible cause of the events.

The patient died on 12 August 2004, cause of death is unknown. It is unknown

whether an autopsy was performed.

Sender's comment:

Auf Stevens-Johnson-Syndrom und Lyell-Syndrom weisen wir bei Eusaprim hin. Ein Kausalzusammenhang ist nicht auszuschliessen.

Bei Fraxiparin sind allergische Reaktionen bekannt, Stevens-Johnson Syndrom und Lyell-Syndrom sind jedoch nicht ausdruecklich in der Produktinformation erwaeht. Der letale Verlauf ist nicht zu beurteilen.

Aufgrund dieses Einzelfalles bleibt das Nutzen/Risikoverhaeltnis unveraendert und eigenverantwortliche Massnahmen sind derzeit nicht notwendig.

Auf der Basis dieses Einzelfalles ergibt sich keine Aenderung des wissenschaftlichen Kenntnisstandes und der Nutzen/Risiko-Relation. Weitergehende eigenverantwortliche Massnahmen halten wir derzeit nicht fuer erforderlich.

Reporter's comment:

Merck KGaA, 27.02.2008 DE-Merck KGaA-6040910

Sender diagnosis: not reported

Narrative

This is a case from the Schoepf registry / intensified monitoring programme being called RegiSCAR study (# 9150169) that is coordinated by the Dermatological University Hospital Freiburg, Germany, and that registers and evaluates all hospitalized cases of severe skin reactions such as toxic epidermal necrolysis (TEN), Stevens-Johnson syndrome (SJS), acute generalised exanthematous pustulosis (AGEP) and drug reaction with eosinophilia and systemic symptoms (DRESS) in 6 European countries, and that documents the demographic characteristics, the role of infection and drug intake in the past history of the severe skin reaction.

The report was received via German Health Authority (# 08008881) on 19 Feb 2008.

On 28 Jul 2004, this 31-year-old male patient developed life-threatening Stevens-Johnson syndrome (SJS) / toxic epidermal necrolysis (TEN), (transition form), ascertained case, from which he died on 12 Aug 2004 under Decortin treatment amongst 30 other drugs or preparations.

Other suspected drugs included Fraxiparin (heparin-calcium), Sobelin (clindamycin), Tavanic (levofloxacin), Tramal long (tramadol), Pantozol (pantoprazole), Ampho-Moronal (amphotericin B), Kalium Duriles (potassium chloride), Beloc-Zok mite (metoprolol), Calcium effervescent (calcium carbonate), Ursofalk (ursodeoxycholic acid), Eusaprim forte (trimethoprim/sulfamethoxazole), Benuron (paracetamol), Paspertin (metoclopramide), Dormicum (midazolam), Rulid (roxithromycin), Otriven NS (xylometazoline), thrombocytes concentrate (no brand stated), NaCl, Vergentan (alizapride), glucose (no brand stated), Thomaesonin (Na-acetate, Na-lactate, NaCl, CaCl₂, Glucose), Novalgin (metamizole), Tramal (tramadol), Rifampicin (rifampicin), Meronem (meropenem) and Dridase (oxybutynin).

As concomitant drugs were considered: Ulcogant (Sucralfate), Diflucan (fluconazole) Vigantoletten (colecalfiferol) and Ospan (colecalfiferol).

A 31-year-old male patient started treatment with Decortin (prednisone) on 25 Jun 2004 for acute myeloid leukaemia (AML) that was diagnosed in

1998 and for which he had received bone marrow transplantation. The patient was admitted to hospital from 23 Jun 2004 to 28 Jul 2004. On 28 Jul 2004, life-threatening SJS / TEN (transition form) occurred. He suffered from skin pain, cockades, spots, lip erosions and oral mucosa erosions as of 28 Jul 2004 (index day). On 29 Jul 2004, the patient developed pruritus. On 30 Jul 2004, he experienced erosions of genital mucosa and erosions of nasal mucosa. On 03 Aug 2004, the patient developed fever (up to 38.2°C). On 12 Aug 2004, the patient died from transition form of SJS / TEN.

The patient's medical history included concurrent purulent coxitis and bone marrow transplantation after acute myeloid leukaemia in 1998.

The documentation of the patient was then transferred to the expert panel of the Schoepf registry. This case was evaluated by the expert panel of Prof. Schoepf in Aug 2004. There the diagnosis of (transition form of) SJS / TEN was ascertained and suspected drugs and concomitant drugs were identified.

Suspected drugs were Decortin (prednisone) for acute myeloid leukaemia, Fraxiparin (heparin-calcium) for thrombosis prophylaxis, Sobelin (clindamycin) for purulent coxitis, Tavanic (levofloxacin) for purulent coxitis, Tramal long (tramadol) for pain not further specified, Pantozol (pantoprazole) for gastric ulcer prophylaxis, Ampho-Moronal (amphotericin B) for oral candidiasis, Kalium Duriles (potassium chloride) for potassium supplementation, Beloc-Zok mite (metoprolol) for arterial hypertension, Calcium effervescent (calcium carbonate) for calcium substitution, Ursosalk (ursodeoxycholic acid) for biliary reflux prophylaxis, Eusaprim forte (trimethoprim/sulfamethoxazole) for purulent coxitis, Benuron (paracetamol) for fever, Paspertin (metoclopramide) for nausea, Dormicum (midazolam) for restlessness/anxiety, Rulid (roxithromycin) for fever, Otriven NS (xylometazoline) for congested nose, thrombocytesconcentrate (no brand stated) for substitution, NaCl for volume substitution, Vergentan (alizapride) for nausea, Glucose (no brand stated) for glucose substitution, Thomaesonin (Na-acetate, Na-lactate, NaCl, CaCl₂, Glucose) for volume substitution, Novalgin (metamizole) for pain not further specified, Tramal (tramadol) for pain not further specified, Rifampicin (rifampicin) for sepsis, Meronem (meropenem) for suspected infection and Dridase (oxybutynin) for urogenital pain.

As concomitant drugs were identified: Ulcogant (Sucralfate) for gastric ulcer prophylaxis, Diflucan (fluconazole) for oral candidiasis, Vigantoletten (colecalciferol) for vitamin D substitution and Ospur (colecalciferol) for vitamine D3 substitution.

It was indicated that Decortin, Calcium effervescent, Benuron, thrombocyte concentrate, NaCl and Thomaesonin were given previously and were well tolerated.

The assessment of causal relationship between the suspected drugs and the occurrence of severe skin reaction is recorded in a rating scale from 1-8 scores (1= definite, 2= very probable, 3= probable, 4= likely; 5= possible; 6= doubtful; 7= no relationship, 8= unlikely).

The expert panel assessed the transition form of SJS /TEN as probable for Eusaprim, as likely for Tavanic, Tramal long / Tramal injection solution and Rulid, as possible for Sobelin, Pantozol, Decortin, Ursosalk, Benuron, Paspertin, Dormicum, Vergentan, Novalgin and Rifampicin and as doubtful for Fraxiparin, Ampho-Moronal, Kalium Duriles, Beloc-Zok mite, Calcium effervescent, Otriven, thrombocyte concentration, NaCl infusion, Glucose infusion, Thomaesonin infusion, Meronem and Dridase. No relationship was assumed by the expert panel for Ulcogant, Diflucan, Vigantoletten and Ospur and therefore, these drugs were not considered as suspected drugs but as concomitant drugs.

Furthermore, the expert panel indicated that purulent coxitis and acute myeloid leukaemia might have contributed to the transition form of SJS /TEN with fatal outcome as well.

Sender's comment:

C

Reporter's comment:

NOVARTIS, 27.02.2008 DE-002147023-CHNY2008DE00863

Sender diagnosis: not reported

Narrative

Initial report received from an intensive monitoring project (German documentation center for severe skin disease dZh, Prof. Schäffpf, Freiburg) on 04 Feb 2008: A patient with pre-existing purulent coxitis, was hospitalised from 23 Jun 2004 to 28 Jul 2004. The patient started Otriven (xylometazoline hydrochloride) on 20 Jul 2004. He was concomitantly treated with Ulcogant oral (sucralfat), Diflucan oral (fluconazole), Fraxiparin subcutaneous (heparin), Sobelin oral (clindamycin), Tavenic oral (levofloxacin), Tramal long oral (tramadol), Pantozol oral (pantoprazole), Ampho-Moronal oral (amphotericin B), Decortin oral (prednisone), Kalium-Duriles (potassium chloride), Beloc-Zok oral (metoprolol), Calciumbrause oral (calcium carbonate), Vigantoletten oral (colecalciferol), Ospur oral (colecalciferol), Ursosalk oral (ursodeoxycholic), Eusaprim oral (trimethoprim, sulfamethoxazole), Benuron oral (paracetamol), with Beuron Saft oral (paracetamol), Paspertin drops (metoclopramide), Dormicum intravenous (midazolam), Rulid oral (roxithromycin), Paspertin oral (metoclopramide), "platelet concentrate" intravenous, with "sodium chloride" intravenous, Vergentan intravenous (alizaprid), glucose intravenous, Thomaesonin intravenous, Novalgin intravenous (metamizole), Tramal intravenous (tramadol),

Rifampicin intravenous (rifampicin), Meronem intravenous (meropenem), and Dridase oral (oxybutynin). On 28 Jul 2004, the patient developed painful skin, "Kokardes" and skin spots as well as erosions of lips and oral mucosa which were regarded as onset date of Stevens-Johnson syndrome or toxic epidermal necrolysis (confirmed case). The patient developed itching on 29 Jul 2004, erosions of genital and nasal mucosa on 30 Jul 2004 and fever up to 38.2 C on 03 Aug 2004. For the duration of reaction the date 12 Aug 2004 was stated. The reaction was rated as life-threatening and the patient died. In Aug 2004 the intensive monitoring project assessed the causality with regard to the patient's multiple medication (using an algorithm with 8 different levels, ranging from definite to improbable) as follows: Eusaprim oral was rated as "probable"; Tavanic oral, Tramal long oral, Rulid oral, and Tramal intravenous were rated as "well possible"; Sobelin oral, Pantozol oral, Decortin oral, Ursofalk oral, Benuron oral, Beuron Saft oral, Paspertin Tropfen oral, Dormicum oral, Paspertin oral, Vergentan intravenous, Novalgin intravenous, and Rifampicin intravenous were rated as "possible". Fraxiparin subcutaneous, Ampho-Moronal oral, Kalium Duriles oral, Belok-Zok oral, Caliumbrause oral, Otriven spray topical (stated daily dose 3x2 spray puffs), "platelet concentrate" intravenous, "sodium chloride" intravenous, Glucose intravenous, Thomaesonin intravenous, Meronem intravenous, and Dridase oral were rated as "questionable". The rest of compounds was rated as "no causal relationship". It was added that the patient had purulent coxitis and unspecified "KMT due to AML in 1998" as other possible risk factors. Outcome: death.

Follow-up information received on 22 Feb 2008. This case was also reported from License Partner Nycomed (80336).

Sender's comment:

Pantozole:

Serious adverse event report, TEN and SJS [death] assessed as unlisted according to the Basic Prescribing Information.

It is Novartis' policy to assess a case with a fatal outcome as unlisted unless the label specifically mentions death as a possible outcome of the listed risks. Reporting physician causality is suspected.

Otriven:

This is a serious adverse event (death). The events Stevens-Johnson syndrome and Toxic epidermal necrolysis (and the related symptoms) were assessed as unlabelled according to the product information. It is Novartis policy to assess a case with a fatal outcome as unlabelled unless the label specifically mentions death as possible outcome of the labelled risks. Investigator causality was not suspected.

Reporter's comment:

Sanofi-Aventis Deutschland GmbH, 27.02.2008 DE-AVENTIS-200810480DE

Sender diagnosis: not reported

Narrative

Initial report:

This case is associated with another case (Nr.: A04200800205).

This serious case of a transitional form of Stevens Johnson syndrome / Toxic Epidermal Necrolysis (SJS/TEN) was received from the German health authority (DE-BFARM-08015238). It was reported by the German Documentation Centre for Severe Skin Reactions in Freiburg / Breisgau, Germany (code-no. 9150169). The centre is conducting a prospective study on the incidence and drug aetiology of severe skin reactions. Diagnosis was made by an expert panel who assessed it as a "definite case" of SJS/TEN. Causal relationship was assessed by the experts in Aug-2004 using an algorithm. For the following suspected drugs, causality was assessed as:

"probable": trimethoprim+sulfamethoxazole

"well possible": levofloxacin, tramadol hydrochloride, roxithromycin

"possible": alizapride hydrochloride, clindamycin hydrochloride, pantoprazole sodium, prednisone, ursodeoxycholic acid, paracetamol, metoclopramide hydrochloride, midazolam maleate, metoclopramide, metamizole sodium, and rifampicin

"conditional": heparin calcium

Not suspected were: sucralfate, fluconazole, amphotericin B, potassium chloride, metoprolol succinate, calcium carbonate, colecalciferol, xylometazoline hydrochloride, platelets, sodium chloride, glucose, sodium acetate+sodium lactate+calcium chloride+glucose, meropenem and oxybutynin hydrochloride.

This case concerns a male patient of 31 years with a medical history of bone marrow transplantation due to acute myelogenous leukemia in 1998. From 23-Jun-2004 to 28-Jul-2004, the patient was hospitalized to internal medicine, possibly because of purulent coxitis since treatment with levofloxacin and clindamycin was started for purulent coxitis on that day. This was amended by trimethoprim-sulfa on 7-Jul-2004. Antiinfective treatment was changed to rifampicin and meropenem on 25-Jul-2004 for suspected sepsis.

This patient received a wide spectrum of medication. This included (from 24-Jul-2004 to 27-Jul-2004) alizapride hydrochloride (Vergentan) at a dose of 111.6 mg daily for nausea and metamizole (Novalgin, 1 amp i.v. on 25-Jul-2004) for pain. With an index day (first appearance of bullae or erosions) of 28-Jul-2004, this patient developed SJS/TEN. Chronology of symptoms was as follows: On 28-Jul-2004: skin pain, target lesions, maculae and erosions of lips and oral mucous membrane. On 29-Jul-2004: pruritus. On 30-Jul-2004: erosions of nasal and genital mucous membrane. On 03-Aug-2004: fever to 38.2 degrees Celsius.

The outcome was reported as "death", date of end of event was reported as "12-Aug-2004".

Laboratory: no values provided.

The experts gave their assessment of causes of SJS/TEN (see above). Involvement of purulent coxitis and bone marrow transplantation in 1998 were additionally provided as possible alternative explanations by the experts.

Sender's comment:

21-FEB-2008 Sanofi-Aventis Company Comment: In this case, there was a wide spectrum of drugs given in a patient with a severe underlying disease (AML) and a long standing course of infection. Metamizole was only started 3 days prior to the index day which would be temporally incompatible with a primary sensitization. The experts assessed causality of levofloxacin and roxithromycin for SJS/TEN to be "well possible" and of metamizole to be "possible". However, they assessed trimethoprim-sulfamethoxazole to be the most likely cause of SJS/TEN in this patient ("probable").

21-FEB-2008 Sanofi Aventis Germany: STEVENS-JOHNSON SYNDROME/TOXIC EPIDERMAL NECROLYSIS is considered locally labelled for the suspected product levofloxacin and metamizole but fatal outcome is considered locally unlabelled for both drugs. SJS/TEN is considered unlabelled for the suspected product roxithromycin. Since the case concerns a patient with a severe underlying disease (AML) and a long standing course of infection and who also received a wide spectrum of medication, assessment of a causal relationship is difficult. The experts considered another drug to be the most likely explanation for the events. This case does not imply a change in the state of the scientific knowledge for the 3 products levofloxacin, roxithromycin and metamizole. Measures according to Para 29 German Drug Law (AMG), section (1) concerning the approval status are not deemed necessary.

Reporter's comment:

GRUNENTHAL, 28.02.2008 DE-GRUNENTHAL-2008 10689

Sender diagnosis: not reported

Narrative

Manufacturer Control Number: 2008-10689

This solicited case from special registry for severe skin reactions from GERMANY, that was reported by a physician via regulatory authority was received at GRT on 19-FEB-2008 and concerns a 31- year- old male patient (height unknown / weight unknown).

The relevant medical history included: Prophylaxis against gastrointestinal ulcer , Oral candidiasis , Thrombosis prophylaxis , Arthritis , Pain , Oral candidiasis , Acute myeloid leukaemia , Electrolyte substitution therapy , Hypertension , Mineral supplementation , Vitamin supplementation , Vitamin supplementation , Prophylaxis , Pyrexia , Nausea , Agitation , Anxiety , Nasal congestion , Platelet transfusion , Hypovolaemia , Medical diet , Sepsis , Infection , Pain , Bone marrow transplant.

The relevant concomitant medication included:
HEPARIN-FRACTION, CALCIUM SALT / FRAXIPARIN;
AMPHOTERICIN B / AMPHO-MORONAL;
POTASSIUM CHLORIDE / KALIUM-DURILES;
METOPROLOL SUCCINATE / BELOC-ZOK;
CALCIUM / CALCIUM;
XYLOMETAZOLINE HYDROCHLORIDE / OTRIVEN;

This case was reported by the Documentation Centre for Severe Skin Reactions (Freiburg/Germany) via BfArM and concerns a 21-year-old multimorbid male patient on multimedication (incl. tramadol). His medical history included purulent coxitis and acute myelocytic leukaemia with bone marrow transplantation in 1998.

On 28-JUL-2004, 36 days after the first administration of tramadol hydrochloride this patient experienced painful skin, cockades, maculae and erosions of the lips and the oral mucosa followed by pruritus on 29-JUL-2004 . On 30-JUL-2004, erosions of the genital mucosa and the nasal mucosa developed. On 03-AUG-2008, the patient experienced fever up to 38.2 degrees Celsius. The patient was hospitalized from 23-JUN-04 to 28-JUL-04 in the internal medicine department.

The reported symptoms were considered to be life-threatening. The patient died.

No further information was requested.

Third party case numbers

E2B Authority Number: DE-BFARM-08008881
 (GLS)-DE-GRT: DE-08-DS-0107
 Patient ID: 9150169

Sender's comment:

Stevens-Johnson syndrome is assessed as serious due to the criteria life-threatening and fatal outcome.

Stevens-Johnson syndrome is not listed in the reference safety information of tramadol hydrochloride.

The adverse event occurred in a reasonable temporal relationship with the administration of tramadol hydrochloride. However, this multimorbid patient, whose medical history included purulent coxitis and acute myelotic leukaemia with bone marrow transplantation in 1998 was treated with a multitude of other drugs, among them roxithromycin, metamizole sodium, rifampicin and especially trimetoprim/sulfamethoxazole that are all known to cause severe skin reactions. Under consideration of these facts presenting high risk for the patient to develop Stevens-Johnson syndrome it is unlikely that tramadol hydrochloride triggered or contributed to the adverse event.

In conclusion Stevens-Johnson syndrome is assessed to be unlikely related to the administration of tramadol hydrochloride.

The information in this single case does not appear to alter the favourable benefit to risk ratio of tramadol hydrochloride.

26-Feb-2008:

Reporter causality was corrected from 'not reported' to 'possible'.
 The company causality assessment was not changed.

Reporter's comment:

Wyeth-Pharma GmbH, 03.03.2008 DE-WYE-H02729408

Sender diagnosis: not reported

Narrative

Information was received from a registry via a business alliance (case # 80336) regarding a 31-year-old male participant in a study entitled "German documentation center for severe skin reactions" who received Pantozol (pantoprazole tablet, delayed release) therapy and experienced toxic epidermal necrolysis and Stevens-Johnson syndrome.

MEDICAL HISTORY:

The patient's concurrent illnesses include nasal congestion, arthritis (purulent), oral candidiasis, acute myeloid leukaemia, essential hypertension, nausea, vomiting, sepsis, pain, anxiety, pyrexia, restlessness, urogenital disorder, purulence and hospitalisation with a past history of bone marrow transplant (in 1998 due to AML).

PRODUCT DETAILS:

Indication for Pantozol was not provided. Therapy began on 23-Jun-2004 and was discontinued on 26-Jul-2004. Dose regimen was "2/day 1 DF" (oral).

Additional suspect medication included Otriven (xylometazoline hydrochloride), Decortin (prednisone), Kalium-Duriles (potassium chloride), Beloc Zok (metoprolol succinate), Paspertin (metoclopramide hydrochloride), Sodium Chloride (injection), Glucose, Rifampicin, Ulcogant (sucralfate), Diflucan (fluconazole), Fraxiparin (heparin-fraction, calcium salt), Sobelin "Upjohn" (clindamycin hydrochloride), Tavanic (levofloxacin), Tramal (tramadol hydrochloride), Ampho-Moronal (amphotericin b), Calcium Carbonate, Vigantoletten (colecalfiferol), Ospur D3 (colecalfiferol), Ursofalk (ursodeoxycholic acid), Eusaprim (sulfamethoxazole/trimethoprim), Ben-U-Ron (paracetamol), Dormicum "Roche" (midazolam maleate), Rulid (roxithromycin), Vergentan (alozapride hydrochloride), Thomaejonin (calcium chloride dihydrate/magnesium chloride hexahydrate/potassium chloride/sodium acetate/sodium chloride), Novalgin (metamizole sodium), Meronem (meropenem), platelets, human and Dridase (oxybutynin hydrochloride).

CONCOMITANT THERAPY:

Concomitant medications were not reported.

EVENT DETAILS:

The patient with pre-existing purulent coxitis was hospitalized from 23-Jun-2004 to 28-Jul-2004. On 28-Jul-2004, the patient developed painful skin, cockades and skin spots as well as erosions of the lips and oral mucosa, which were considered the onset of Stevens-Johnson syndrome (Stevens-Johnson syndrome) or toxic epidermal necrolysis (toxic epidermal necrolysis). On 29-Jul-2004, the patient developed itching. On 30-Jul-2004, the patient developed erosions of the genital and nasal mucosa. On 03-Aug-2004, the patient developed a fever up to 38.2 degrees Centigrade. The reaction was considered life-threatening and the patient died. The duration of the reaction was stated as 12-Aug-2004. An autopsy report was not

provided.

The cause of death was reported as Stevens-Johnson syndrome and toxic epidermal necrolysis.

RELATEDNESS ASSESSMENT:

The investigator(s) considered Stevens-Johnson syndrome and toxic epidermal necrolysis possibly related to the study product. The business partner's medical monitor(s) assessment for Stevens-Johnson syndrome and toxic epidermal necrolysis was unknown.

Sender's comment:

Wyeth comment:

INITIAL (v1) 03-Mar-2008

STEVENS-JOHNSON SYNDROME

Bericht aus dem "Dokumentationszentrum für schwere Hautreaktionen in Freiburg". Es geht um einen 31-jährigen Patienten, der im Zusammenhang mit der Einnahme zahlreicher Medikamente, darunter auch Pantozol SJS / TEN entwickelte, der Patient verstarb.

Es handelt sich nicht um eine bekannte NW (US case expectedness) von Pantozol.

Ein kausaler Zusammenhang kann nicht ausgeschlossen werden.

Basierend auf der aktuellen Information führt dieser Bericht nicht zu einer Änderung des Nutzen- Risiko-Verhältnisses. Zu diesem Zeitpunkt sind keine weiteren Massnahmen vorgesehen. Sobald Änderungen hierzu bekannt werden, werden wir darüber unverzüglich und unaufgefordert informieren.

Dr. R. Jöstingmeier

Reporter's comment:

SANOFI-SYNTHELABO, 03.03.2008 DE-SANOFI-SYNTHELABO-A04200800205

Sender diagnosis: not reported

Narrative

Reaction as reported: transitional form of Stevens Johnson syndrome / toxic epidermal necrolysis

LLT: TOXIC EPIDERMAL NECROLYSIS / 10044223

PT: TOXIC EPIDERMAL NECROLYSIS / 10044223

Reaction as reported: pain of skin

LLT: PAIN OF SKIN / 10033474

PT: PAIN OF SKIN / 10033474

Reaction as reported: target lesions and maculae

LLT: MACULE / 10025421

PT: RASH MACULAR / 10037867

Reaction as reported: erosions of oral mucosa

LLT: ORAL ULCERATION / 10031027

PT: MOUTH ULCERATION / 10028034

Reaction as reported: erosions of lips

LLT: LIP EROSION / 10051992

PT: LIP EROSION / 10051992

Reaction as reported: pruritus

LLT: PRURITUS / 10037087

PT: PRURITUS / 10037087

Reaction as reported: erosions of genital mucosa

LLT: GENITAL ULCERATION NOS / 10018181

PT: GENITAL ULCERATION / 10018180

Reaction as reported: erosions of nasal mucosa

LLT: NASAL ULCER / 10028780

PT: NASAL ULCER / 10028780

Reaction as reported: fever up to 38.2 degrees C

LLT: FEVER / 10016558

PT: PYREXIA / 10037660

Narrative:**Initial report:**

This case is associated with another case, created for the other suspect drugs metamizole (Novalgin), levofloxacin (Tavanic) and roxithromycin (Rulid) in the database of products from Aventis origin, with reference 200810480DE.

This serious case of a transitional form of Stevens Johnson syndrome (SJS) / Toxic Epidermal Necrolysis (TEN) was received from the German health authority (DE-BFARM-08015238). It was reported by the German Documentation Centre for Severe Skin Reactions in Freiburg / Breisgau, Germany (code-no. 9150169). The centre is conducting a prospective study on the incidence and drug aetiology of severe skin reactions. Diagnosis of both events was made by an expert panel who assessed it as a "definite case" of SJS/TEN. Causal relationship was assessed by the experts in Aug-2004 using an algorithm.

As for alizapride hydrochloride (Vergentan), the causality was assessed as "possible".

For the other suspected drugs, causality was assessed as:

"probable": trimethoprim+sulfamethoxazole

"well possible": levofloxacin, tramadol hydrochloride, roxithromycin

"possible": clindamycin hydrochloride, pantoprazole sodium, prednisone, ursodeoxycholic acid, paracetamol, metoclopramide hydrochloride, midazolam maleate, metoclopramide, metamizole sodium, and rifampicin

"conditional": heparin calcium

Not suspected were: sucralfate, fluconazole, amphotericin B, potassium chloride, metoprolol succinate, calcium carbonate, colecalciferol, xylometazoline hydrochloride, platelets, sodium chloride, glucose, sodium acetate+sodium lactate+calcium chloride+glucose, meropenem and oxybutynin hydrochloride.

This case concerns a male patient of 31 years with a medical history of bone marrow transplantation due to acute myelogenous leukemia in 1998. From 23-Jun-2004 to 28-Jul-2004, the patient was hospitalized to internal medicine, possibly because of purulent coxitis since treatment with levofloxacin and clindamycin was started for purulent coxitis on that day. This was amended by trimethoprim-sulfa on 7-Jul-2004. Antiinfective treatment was changed to rifampicin and meropenem on 25-Jul-2004 for suspected sepsis.

This patient received a wide spectrum of medication. This included (from 24-Jul-2004 to 27-Jul-2004) alizapride hydrochloride (Vergentan, i.v.) at a dose of 111.6 mg daily for nausea. With an index day (first appearance of bullae or erosions) of 28-Jul-2004, this patient developed Stevens Johnson syndrome/Toxic Epidermal Necrolysis. Chronology of symptoms was as follows: On 28-Jul-2004: skin pain, target lesions, maculae and erosions of lips and oral mucous membrane. On 29-Jul-2004: pruritus. On 30-Jul-2004: erosions of nasal and genital mucous membrane. On 03-Aug-2004: fever to 38.2 degrees Celsius.

In the questionnaire, exitus is ticked and a duration of SJS/TEN until 12-Aug-2004 is given. This is considered to indicate that the patient died on that day from SJS/TEN. However, no further information is given on the patient's course and the causes of death. Laboratory: no values provided.

The experts gave their assessment of causes of SJS/TEN (see above). Involvement of purulent coxitis and bone marrow transplantation in 1998 in the course of an acute myelocytic leukemia were additionally provided as possible alternative explanations by the experts.

Sender's comment:

In this case, there was a wide spectrum of drugs given in a patient with a severe underlying disease (AML) and a long standing course of infection. Alizapride was only started 4 days prior to the index day which would be temporally incompatible with a primary sensitization. The experts assessed causality of alizapride for SJS/TEN to be possible. However, they assessed trimethoprim-sulfamethoxazole (started 3 weeks before the index day) to be the most likely cause of SJS/TEN in this patient ("probable"). 20-Feb-2008 SA Germany - Pharmacovigilance: This case report includes clinical events, which are considered locally unlabelled for alizapride. In this case, a wide spectrum of other drugs was given in a severely ill patient. Other drugs were assessed by the experts to be more likely causes for the events. The evaluation does not imply a change in the state of scientific knowledge for the suspected product. Measures according to Paragraph 29 German Drug Law (AMG), section (1) concerning the approval status are not deemed necessary.

Reporter's comment:

NOVARTIS, 01.03.2010 DE-002147023-PHHY2008DE34233

Sender diagnosis: not reported

Narrative

Initial spontaneous report received on 20 Feb 2008 and forwarded by Nycomed (Reference number: 80336) on 27 Aug 2009. Concomitant diseases included nasal congestion, arthritis, oral candidiasis, acute myeloid leukaemia, hypertension, nausea, sepsis, pain, anxiety, pyrexia, restlessness and urogenital disorder. The patient started receiving oral Pantozol (pantoprazolesodium sesquihydrate) for an unknown indication (dose: 2/day, 1 DF) on 23 Jun 2004. Co suspect medications included Otriven (xylometazoline hydrochloride), Ulcogant oral (sucralfat), Diflucan oral (fluconazole), Fraxiparin subcutaneous (heparin), Sobelin oral (clindamycin), Tavenic oral (levofloxacin), Tramal long oral (tramadol), Pantozol oral (pantoprazole), Ampho-Moronal oral (amphotericin B), Decortin oral (prednisone), Kalium-Duriles (potassium chloride), Beloc-Zok oral (metoprolol), Calciumbrause oral (calcium carbonate), Vigantoletten oral (cholecalciferol), Ospur oral (cholecalciferol), Ursofalk oral (ursodeoxycholic), Eusaprim oral (trimethoprim, sulfamethoxazol), Benuron oral (paracetamol), with Beuron Saft oral (paracetamol), Paspertin drops (metoclopramide), Dormicum intravenous (midazolam), Rulid oral (roxithromycin), Paspertin oral (metoclopramide), "platelet concentrate" intravenous, with "sodium chloride" intravenous, Vergentan intravenous (alizaprid), glucose intravenous, Thomaejonin intravenous, Novalgin intravenous (metamizole), Tramal intravenous (tramadol), Rifampicin intravenous (rifampicin), Meronem intravenous (meropenem), and Dridase oral (oxybutynin). On 26 Jul 2004, therapy with Pantozol was discontinued. On 28 Jul 2004, the patient experienced Stevens-Johnson syndrome and toxic epidermal necrolysis. The patient with pre-existing purulent coxitis, was hospitalised from 23 Jun 2004 to 28 Jul 2004. On 28 Jul 2004, the patient developed painful skin, "cockades" and skin spots as well as erosions of lips and oral mucosa, which were regarded as onset date of Stevens-Johnson syndrome or toxic epidermal necrolysis. On 29 Jul 2004, the patient developed itching. On 30 Jul 2004, the patient developed erosions of genital and nasal mucosa. On 03 Aug 2004, the patient developed fever up to 38.2°C. The reaction was rated as life-threatening and the patient died. The duration of reaction was stated as 12 Aug 2004. No autopsy report was provided. The reporter assessed the causality as possible for Pantozol. The Intensive Monitoring Project assessed the causality with regard to the patient's multiple medication (using an algorithm with 8 different levels ranging from 1=definite to 8=improbable). Sulfamethoxazole/trimethoprim was rated as "probable"; Tavanic oral, Tramal long oral, roxithromycin oral and tramadol i.v. were rated as "well possible". Pantoprazole was rated as "possible" due to this algorithm. It was added that the patient had purulent coxitis and unspecified bone marrow transplantation due to AML in 1998 as other possible risk factors.

Sender's comment:

Serious spontaneous report:

Toxic epiderma necrolysis [Death], assessed as unlisted according to the Core Data Sheet. The information provided in this individual case does not warrant a change to the Core Data Sheet text. The topic will be monitored closely. All spontaneous reports are considered suspected for reporting purposes or in accordance with national regulatory requirements.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08009522

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes	Yes	Yes	No	No	No		Yes

Patient

Initials:	Sex: Male	Birth: privacy	Age: 62 Year	weight (kg):	BMI_int:	Age fetus:
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Patientdeath

deathdate: 20041022	deathcause: Toxic epidermal necrolysis
deathdate: 20041022	deathcause: Stevens Johnson syndrome

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Rash papular	20040806			1 Day	fatal
2 Stevens Johnson syndrome	20040806	20041022	78 Day	1 Day	fatal
3 Nikolsky's sign	20040810			5 Day	fatal
4 Fever	20040721				fatal
5 Blisters	20040808			3 Day	fatal
6 Toxic epidermal necrolysis	20040806	20041022	78 Day	1 Day	fatal

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
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1	acc acetylcysteine	Intravenous (not otherwis 3 Df dosage form/Day	Solution for injection	20040801-20040806 6 Day
2	ACC LONG acetylcysteine	Oral	/	20040711-20040728 18 Day
3	ACC LONG acetylcysteine	Oral	600 Mg milligram(s)/Day	20040420-20040728 3 Month
4	ACICLOVIR aciclovir	Oral	600 Mg milligram(s)/Day	20040720-20040722 Antiviral prophylaxis 3 Day
5	ACICLOVIR (NGX) aciclovir	Oral	600 Mg milligram(s)/Day	20040724-20040728 Antiviral prophylaxis 5 Day
6	atropine atropine	Subcutaneous	/	20040721-20040723 3 Day
7	BENURON paracetamol	Oral	1000 Mg milligram(s)/Day	20040808-20040808 Pain 1440 Minute
8	BENURON paracetamol	Oral	500 Mg milligram(s)/Day	20040806-20040806 Pain 1440 Minute
9	BENURON paracetamol	Rectal	500 Mg milligram(s)/	20040804-20040807 Pain 4 Day
10	BENURON paracetamol	Rectal	500 Mg milligram(s)/	20040802-20040802 Pain 1440 Minute
11	calcium carbonate calcium carbonate	Oral	1 G gram(s)/	20040807-20040808 Mineral supplementation 2 Day
12	ciclopirox ciclopirox	Topical	/	20040720-20040808 20 Day
13	ciprobay ciprofloxacin hydrochloride	Oral	1000 Mg milligram(s)/Day	20040728-20040801 Infection prophylaxis 5 Day
14	CIPROBAY ciprofloxacin hydrochloride	Intravenous (not otherwis 800 Mg milligram(s)/Day		20040806-20040808 Infection prophylaxis 3 Day
15	CIPROBAY ciprofloxacin hydrochloride	Oral	1000 Mg milligram(s)/Day	20040804-20040804 Infection prophylaxis 1440 Minute
16	CIPROBAY ciprofloxacin hydrochloride	Oral	1000 Mg milligram(s)/Day	20040717-20040717 Infection prophylaxis 1440 Minute
17	COMBACTAM sulbactam sodium	Intravenous (not otherwis /		20040730-20040806 Infection prophylaxis 8 Day
18	DELIX ramipril	Oral	0.5 Df dosage form/Day	20040717-20040717 Hypertension 1440 Minute
19	DELIX ramipril	Oral	10 Mg milligram(s)/Day	20040720-20040808 Hypertension 20 Day
20	diflucan fluconazole	Oral	/	20040728-20040804 Mycosis fungoides 8 Day
21	DORMICUM midazolam hydrochloride	Intravenous (not otherwis 3 Mg milligram(s)/		20040804-20040804 1440 Minute
22	fenistil dimetindene maleate	Intravenous (not otherwis 3 Df dosage form/Day	Solution for injection	20040806-20040806 Exanthema 1440 Minute
23	FENISTIL dimetindene maleate	Intravenous (not otherwis 3 Df dosage form/Day	Solution for injection	20040808-20040808 Exanthema 1440 Minute
24	FORTRAL pentazocine	Intravenous (not otherwis 15 Mg milligram(s)/		20040804-20040804 1440 Minute
25	fragmin p dalteparin sodium	Subcutaneous	2500 Iu international unit(s)	20040720-20040808 Thrombosis prophylaxis 20 Day
26	glucose glucose	Intravenous (not otherwis /		20040801-20040801 1440 Minute
27	kalinor-brausetabletten potassium carbonate; potassium citrate	Oral	1 Df dosage form/Day	20040723-20040724 Hypokalaemia 2 Day
28	KANAVIT phytomenadione	Oral	/	20040802-20040808 7 Day

29	KONAKION phytomenadione	Oral	40 Gtt drop(s)/Day	20040802-20040805 4 Day
30	LASIX furosemide	Oral	40 Mg milligram(s)/Day	20040803-20040803 Oedema 1440 Minute
31	MAXIPIME cefepime hydrochloride	Intravenous (not otherwis	4 G gram(s)/Day	20040806-20040806 Infection prophylaxis 1440 Minute
32	MERONEM meropenem	Intravenous (not otherwis /		20040803-20040806 4 Day
33	MERONEM meropenem	Intravenous (not otherwis /		20040718-20040722 5 Day
34	METOCLOPRAMIDE (NGX) metoclopramide	Oral	45 Gtt drop(s)/	20040801-20040801 Nausea 1440 Minute
35	METOPROLOL metoprolol	Oral	47.5 Mg milligram(s)/Day	20040720-20040721 Hypertension 2 Day
36	Natriumchlorid sodium chloride	Intravenous (not otherwis /		20040801-20040801 1440 Minute
37	Natriumchlorid sodium chloride	Intravenous (not otherwis /		20040805-20040808 4 Day
38	NEXIUM esomeprazole magnesium	Oral	40 Mg milligram(s)/Day	20040720-20040808 20 Day
39	NEXIUM esomeprazole magnesium	Oral	20 Mg milligram(s)/Day	20040717-20040717 1440 Minute
40	NOCTAMID lormetazepam	Oral	1 Mg milligram(s)/	20040725-20040726 Sleep disturbance 2 Day
41	NOCTAMID lormetazepam	Oral	1 Mg milligram(s)/	20040723-20040723 Sleep disturbance 1440 Minute
42	PANTOZOL pantoprazole	Oral	40 Mg milligram(s)/Day	20040724-20040724 1440 Minute
43	PANTOZOL pantoprazole	Oral	40 Mg milligram(s)/Day	20040420-20040921 5 Month
44	PIPRIL piperacillin sodium	Intravenous (not otherwis /		20040730-20040806 Infection prophylaxis 8 Day
45	PLACEBO (against sleep disorder)	Oral	1 Df dosage form/	20040728-20040728 Sleep disorder 1440 Minute
46	PREDNISOLONE prednisolone	Intravenous (not otherwis /		20040720-20040808 20 Day
47	prograf tacrolimus	Oral	/	20040717-20040808 23 Day
48	PSORCUTAN calcipotriol	Topical	2* /Day	20040720-20040808 20 Day
49	PSORCUTAN calcipotriol	Topical	2* /Day	20040720-20040808 20 Day
50	SAB SIMPLEX dimeticone	Oral	1 Df dosage form/Day	20040723-20040723 Stomach discomfort 1440 Minute
51	SIFROL pramipexole dihydrochloride	Oral	/	20040720-20040808 Extrapyrasidal symptoms 20 Day
52	SOLU-DECORTIN-H prednisolone sodium succinate	Intravenous (not otherwis /		20040723-20040726 4 Day
53	tavegil clemastine	Intravenous (not otherwis 1 Df dosage form/Day	Solution for injection	20040808-20040808 Exanthema 1440 Minute
54	TRAMAL tramadol hydrochloride	Oral	/	20040728- Pain -
55	TRAMAL tramadol hydrochloride	Oral	/	20040730-20040801 Pain 3 Day
56	tutofusin amino acids nos; electrolytes nos; vitamins nos	Intravenous (not otherwis /		20040805-20040808 4 Day

57 **URSOFALK**

Oral

/

20040717-20040718

ursodeoxycholic acid

2 Day

58 **vancomycin**

Intravenous (not otherwise 1 G gram(s)/

20040801-20040801 Infection prophylaxis

vancomycin

1440 Minute

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Prophylaxis against transplant rejection			Unknown	Prophylaxis of rejection reaction
Psoriasis			Unknown	Psoriasis
Transplant rejection			Unknown	Rejection reaction
Thrombosis prophylaxis			Unknown	Prophylaxis of thrombosis
Sputum expectoration difficult			Unknown	Mucolysis
Exanthem			Unknown	
Sleep disorder			Unknown	sleep disorder
Liver transplantation			Unknown	orthotopic liver transplantation
Mycosis fungoides NOS			Unknown	Mycosis
Restless legs syndrome			Unknown	Restless legs syndrome
Vitamin supplementation			Unknown	Vitamin K substitution
Calcium supplementation			Unknown	Calcium substitution
Enteral feeding			Unknown	Parenteral feeding
Gastric ulcer prophylaxis			Unknown	Gastric protection
Infection prophylaxis			Unknown	Infection prophylaxis
Infection prophylaxis			Unknown	Infection prophylaxis
Antiviral prophylaxis			Unknown	Antiviral prophylaxis
Endoscopic retrograde cholangiopancreatography			Unknown	Endoscopic retrograde
Extrapyramidal symptoms			Unknown	Extrapyramidal motoric disorder
Reflux gastritis			Unknown	Reflux
Hypertension			Unknown	Hypertension
Hypokalemia			Unknown	Hypokalaemia
Nausea			Unknown	Nausea
Oedema			Unknown	Oedema
Pain			Unknown	Pain
Volume depletion			Unknown	Volume substitution

Primary source

Qualification: Physician

observestudytype:

Literature:

Narratives (nur bei elektronischem Eingang)PFIZER INC, 28.01.2008 DE-PFIZER INC-2008005730

Sender diagnosis: not reported

Narrative

This is a report from the German Documentation Centre of Severe Skin Reactions, which is a sponsored project set up to collect suspected drug induced severe skin reactions leading to hospital admission.

This 62 year old male patient with psoriasis underwent orthotopic liver retransplantation on 18Jul2004. On 21Jul2004 he developed fever up to 40.0° C. On 04Aug2004 the patient underwent an ERCP (reason not provided). On 06Aug2007 he developed target lesions and spots on the skin. On 08Aug2004 he had blisters on the skin; Nikolsky's sign was positive on 10Aug2004. Definite diagnosis of Stevens-Johnson syndrome/toxic epidermal necrolysis (transition form) was made.

Duration of the adverse events was documented as "until 22Oct2004". The outcome was reported as "death" (date not provided).

Information on medication is available from 11Jul2004 until 08Aug2004. Between operation on 18Jul2004 and adverse events onset on 06Aug2004 the patient received following suspected Pfizer drugs: Combactam 4x1g/day intravenously from 30Jul2004 until 06Aug2004 for prophylaxis of infection, Fragmin P 2500 IU subcutaneously from 20Jul2004 for prophylaxis of thrombosis and Diflucan 3x100mg/day orally from 28Jul2004 due to mycosis. Furthermore he was treated with the following suspected drugs: Pantozol 40mg/day orally from 20Apr2004 until 21Jul2004 and on 24Jul2004

for stomach protection, ACC long 600mg/day orally from 20Apr2004 until 28Jul2004 due to mucolysis, Ciprobay 500mg/day orally on 17Jul2004 (the day prior to operation) and from 28Jul2004 until 01Aug2004 and additionally on 04Aug2004 for prophylaxis of infection, Delix on 17Jul2004 and from 20Jul2004 orally due to hypertension, Nexium 3x20mg/day orally on 17Jul2004 and from 20Jul2004 for stomach protection, Prograf orally from 17Jul2004 due to restless-legs-syndrome, Meronem 3x1g/day intravenously from 18Jul2004 until 22Jul2004 and from 03Aug2004 until 06Aug2004 for prophylaxis of infection, Aciclovir 3x200mg/day orally from 18Jul2004 until 22Jul2004 and from 03Aug2004 until 06Aug2004 for antiviral prophylaxis, Sebiprox 1.5% (ciclopirox olamine) topically from 20Jul2004 for hair wash, Prednisolon intravenously from 20Jul2004 due to rejection reaction, Psorcutan cream 2x1 application per day from 20Jul2004 and Psorcutan solution topically from 20Jul2004 both due to psoriasis, Sifrol 320mg/day orally due to extrapyramidal motor system, Kalinor on effervescent tablet from 23Jul2004 until 24Jul2004 due to hypokalemia, Solu-Decortin H 750mg/day intravenously from 23Jul2004 until 26Jul2004 for rejection prophylaxis, Noctamid 1mg/day on 23Jul2004 and from 25Jul2004 until 26Jul2004 due to sleep disorder, a placebo one tablet on 28Jul2004 also due to sleep disorder, Tramal (tramadol hydrochloride) drops on 28Jul2004 and from 30Jul2004 until 01Aug2004 due to pain, Pipril 4x2mg/day intravenously from 28Jul2004 until 06Aug2004 for prophylaxis of infection, Vancomycin 1g intravenously on 01Aug2004 for prophylaxis of infection, glucose 5% 1000ml intravenously on 01Aug2004, metoclopramide (MCP) 45 drops on 01Aug2004 due to nausea, ACC 3x300mg/day intravenously from 01Aug2004 until 06Aug2004 due to mucolysis, sodium chloride 0.9% on 01Aug2004 and from 05Aug2004 for volume replacement, Konakion 2x20 drops/day from 02Aug2004 until 05Aug2004 for replacement, Benuron 500mg/day rectally on 02Aug2004 and from 04Aug2004 until 07Aug2004 due to pain, Kanavit drops from 02Aug2004 for replacement, Lasix 40mg on 03Aug2004 due to edema, Fortral 15mg and Dormicum 3mg both intravenously on 04Aug2004 due to ERCP.

Furthermore the patient received the following non-suspected drugs: Sab Simplex one bottle suspension orally on 23Jul2004 due to gastrointestinal disorder. Apart from that he received Ursofalk 750mg/day orally from 17Jul2004 until 18Jul2004 due to reflux, Metoprolol 47.5mg/day orally from 20Jul2004 until 21Jul2004 due to hypertension, Atropin subcutaneously from 21Jul2004 until 23Jul2004 due to spasmodic and Tutofusin intravenously from 05Aug2004 for volume replacement.

The experts assessed the causal relationship as "very possible" with Combactam, Ciprobay, Tramal and Pipril, "possible" with Fragmin, Diflucan, ACC long, Delix, Nexium, Prograf, Meronem, Aciclovir, Prednisolon, Sifrol, Solu-Decortin H, Vancomycin, MCP drops, ACC and Benuron, "questionable" in view of the short latency time until manifestation of Stevens-Johnson syndrome/toxically epidermal necrolysis (transition form) with Pantozol, Sebiprox, Psorcutan, Kalinor, Noctamid, Placebo, Glucose, sodium chloride, Konakion drops, Kanavit, Lasix, Fortral and Dormicum. They assessed the causal relationship as "not related" with Sab Simplex, Ursofalk, Metoprolol 47.5mg, Atropin and Tutofusin.

Company Clinical Evaluation. Due to the several medications the patient was taking, a causal relationship for each of them in inducing the reported events can not be established.

Sender's comment:

Reporter's comment:

PFIZER INC, 01.02.2008 DE-PFIZER INC-2008005730

Sender diagnosis: not reported

Narrative

This is a report from the German Documentation Centre of Severe Skin Reactions, which is a sponsored project set up to collect suspected drug induced severe skin reactions leading to hospital admission.

This 62 year old male patient with psoriasis underwent orthotopic liver retransplantation on 18Jul2004. On 21Jul2004 he developed fever up to 40.0° C. On 04Aug2004 the patient underwent an ERCP (reason not provided). On 06Aug2007 he developed target lesions and spots on the skin.

On 08Aug2004 he had blisters on the skin; Nikolsky's sign was positive on 10Aug2004.

Definite diagnosis of Stevens-Johnson syndrome/toxic epidermal necrolysis (transition form) was made. Duration of the adverse events was documented as "until 22Oct2004".

The outcome was reported as "death" (date not provided).

Information on medication is available from 11Jul2004 until 08Aug2004. Between operation on 18Jul2004 and adverse events onset on 06Aug2004

the patient received following suspected Pfizer drugs: Combactam 4x1g/day intravenously from 30Jul2004 until 06Aug2004 for prophylaxis of infection, Fragmin P 2500 IU subcutaneously from 20Jul2004 for prophylaxis of thrombosis and Diflucan 3x100mg/day orally from 28Jul2004 due to mycosis. Furthermore he was treated with the following suspected drugs: Pantozol 40mg/day orally from 20Apr2004 until 21Jul2004 and on 24Jul2004 for stomach protection, ACC long 600mg/day orally from 20Apr2004 until 28Jul2004 due to mucolysis, Ciprobay 500mg/day orally on 17Jul2004 (the day prior to operation) and from 28Jul2004 until 01Aug2004 and additionally on 04Aug2004 for prophylaxis of infection, Delix on 17Jul2004 and from 20Jul2004 orally due to hypertension, Nexium 3x20mg/day orally on 17Jul2004 and from 20Jul2004 for stomach protection, Prograf orally from 17Jul2004 due to restless-legs-syndrome, Meronem 3x1g/day intravenously from 18Jul2004 until 22Jul2004 and from 03Aug2004 until 06Aug2004 for prophylaxis of infection, Aciclovir 3x200mg/day orally from 18Jul2004 until 22Jul2004 and from 03Aug2004 until 06Aug2004 for antiviral prophylaxis, Sebiprox 1.5% (ciclopirox olamine) topically from 20Jul2004 for hair wash, Prednisolon intravenously from 20Jul2004 due to rejection reaction, Psorcutan cream 2x1 application per day from 20Jul2004 and Psorcutan solution topically from 20Jul2004 both due to psoriasis, Sifrol 320mg/day orally due to extrapyramidal motor system, Kalinor on effervescent tablet from 23Jul2004 until 24Jul2004 due to hypokalemia, Solu-Decortin H 750mg/day intravenously from 23Jul2004 until 26Jul2004 for rejection prophylaxis, Noctamid 1mg/day on 23Jul2004 and from 25Jul2004 until 26Jul2004 due to sleep disorder, a placebo one tablet on 28Jul2004 also due to sleep disorder, Tramal (tramadol hydrochloride) drops on 28Jul2004 and from 30Jul2004 until 01Aug2004 due to pain, Pipril 4x2mg/day intravenously from 28Jul2004 until 06Aug2004 for prophylaxis of infection, Vancomycin 1g intravenously on 01Aug2004 for prophylaxis of infection, glucose 5% 1000ml intravenously on 01Aug2004, metoclopramide (MCP) 45 drops on 01Aug2004 due to nausea, ACC 3x300mg/day intravenously from 01Aug2004 until 06Aug2004 due to mucolysis, sodium chloride 0.9% on 01Aug2004 and from 05Aug2004 for volume replacement, Konakion 2x20 drops/day from 02Aug2004 until 05Aug2004 for replacement, Benuron 500mg/day rectally on 02Aug2004 and from 04Aug2004 until 07Aug2004 due to pain, Kanavit drops from 02Aug2004 for replacement, Lasix 40mg on 03Aug2004 due to edema, Fortral 15mg and Dormicum 3mg both intravenously on 04Aug2004 due to ERCP.

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The experts assessed the causal relationship as "very possible" with Combactam, Ciprobay, Tramal and Pipril, "possible" with Fragmin, Diflucan, ACC long, Delix, Nexium, Prograf, Meronem, Aciclovir, Prednisolon, Sifrol, Solu-Decortin H, Vancomycin, MCP drops, ACC and Benuron, "questionable" in view of the short latency time until manifestation of Stevens-Johnson syndrome/toxically epidermal necrolysis (transition form) with Pantozol, Sebiprox, Psorcutan, Kalinor, Noctamid, Placebo, Glucose, sodium chloride, Konakion drops, Kanavit, Lasix, Fortral and Dormicum. They assessed the causal relationship as "not related" with Sab Simplex, Ursofalk, Metoprolol 47.5mg, Atropin and Tutofusin.

Follow-up (15Jan2008):

This is a report from the German Documentation Centre of Severe Skin Reactions, a sponsored project set up to collect suspected drug induced severe skin reactions leading to hospital admission.

Pantozol 40mg/day was given from 20Apr2004 until 21Jul2004 and on 24Jul2004, ACC long from 20Apr2004 until 28Jul2004, Aciclovir from 20Jul2004 until 22Jul2004 and from 24Jul2004 until 28Jul2004, Pipril from 30Jul2004 until 06Aug2004.

Both "placebo" and "sodium chloride 0.9%" were considered suspect drugs by the project experts in the sense of "questionable" causal relationship.

Company Clinical Evaluation:

Based on the information provided in the case this report would not seem to modify the risk-benefit profile of the subject drug.

Sender's comment:

Reporter's comment:

NOVARTIS, 20.02.2008 DE-SZ09-CHNY2008DE00864

Sender diagnosis: not reported

Narrative

Report received from an Intensive Monitoring Project (German documentation center for severe skin disease) on 04 Feb 2008 via German Health Authority, BFARM (Ref: 9150171). This male patient was treated with Fensitil intravenous (dimetindene), Tavegil intravenous (clemastine), Pantozol oral (pantoprazole), ACC long oral (acetylcystein), Ursolfalk oral (acid ursodeoxycholic), Ciprobay oral (ciprofloxacin), Delix oral (ramipril), Nexium oral (esomeprazole), Prograf oral (tacrolimus), Meronem intravenous (meropenem), metoprolol oral, aciclovir oral, Fragmin P subcutaneous (dalteparin sodium), Sebiprox solution topical (ciclopirox), prednisolon intravenous, Psorcutan cream topical (calcipotriol), Psorcutan solution topical (calcipotriol), Sifrol oral (pramipexol), atropin subcutaneous, Sab Simplex oral (simethicone), Kalinor Brause oral (potassium citrate, potassium hydrogenocarbonate), Solu-Decortin H intravenous (prednisolone), Noctamid oral (lormetazepam), Placebo oral, Tramal Tropfen oral (tramadol), Diflucan oral (fluconazol), Pipril intravenous (piperacillin), Combactam intravenous (sulbactam), vancomycin intravenous, glucose intravenous, metoclopramide Tropfen oral, ACC intravenous (acetylcystein), sodium chloride intravenous, Konaktion Tropfen oral (phytomenadion), Benuron suppository (paracetamol), Kanavit Tropfen oral (phytomenadion), Lasix oral (furosemid), Fortral intravenous (pentazocin), Dormicum intravenous (midazolam), Tutofusin intravenous, Benuron oral (paracetamol), Ciprobay intravenous (ciprofloxacin), Maxipime intravenous (cefepim) and with Caliumbrause oral (calcium carbonate). On 18 Jul 2004, the patient underwent an orthotopic liver transplantation. On 21 Jul 2004, he experienced fever up to 40.0 C. On 06 Aug 2004, he developed "Kokardes" and skin spots which were regarded as onset date of Stevens-Johnson syndrome or toxic epidermal necrolysis (confirmed case). He developed skin blisters on 08 Aug 2004 and the Nikolsky's sign was rated as positive on 10 Aug 2004. For the duration of reaction, the date of 22 Oct 2004 was stated. The reaction was rated as life-threatening and the patient died. In Mar 2005, the Intensive Monitoring Project assessed the causality with regard to the patient's multiple medication (using an algorithm with 8 different levels, ranging from 1=definite to 8=improbable) as follows: Ciprobay oral, Tramal Tropfen oral, Pipril intravenous, and Combactam intravenous were rated as "4=well possible". ACC long oral, Delix oral, Nexium oral, Prograf oral, Meronem intravenous, Aciclovir oral, Fragmin P subcutaneous, Prednisolon intravenous, Sifrol oral, Solu-Decortin H intravenous, Diflucan oral, Vancomycin intravenous, MCP Tropfen oral, ACC intravenous, and Benuron suppository were rated as "5=possible". Pantozol oral, Sebiprox solution topical, Psorcutan cream topical, Psorcutan solution topical, Kalinor Brause oral, Noctamid oral, Placebo oral, Glucose intravenous, sodium chloride intravenous, Konaktion Tropfen oral, Kanavit Tropfen oral, Lasix oral, Fortral intravenous, and Dormicum intravenous were rated as "6=questionable". Ursolfalk oral, Metoprolol oral, Atropin subcutaneous, Sab Simplex oral, Tutofusin intravenous, Benuron oral, Fenistil intravenous, Ciprobay intravenous, Maxipime intravenous, Caliumbrause oral, and Tavegil intravenous were rated as "7=no causal relationship". The patient had been hospitalised in Intensive Care Unit from 18 Jul to 20 Jul 2004, in the department of Surgery from 20 Jul to 23 Jul 2004, and in the department of Internal Medicine from 23 Jul 2004 on until death. It was added that the patient had no infection or other possible risk factors. Outcome: death.

Sender's comment:

For Fensitil and Tavegil:

This is a serious adverse event (death). The events Stevens-Johnson syndrome and Toxic epidermal necrolysis (and related symptoms) were assessed as unlabelled according to the product information. It is Novartis policy to assess a case with a fatal outcome as unlabelled unless the label specifically mentions death as possible outcome of the labelled risks. Investigator causality was not-suspected.

For Acetylcystein, Aciclovir, and Metoclopramide.

Serious spontaneous report (death) assessed as unlisted according to the Core Data Sheet Information.

However other alternative causes (other drugs) provide a possible explanation for the reported adverse event.

All spontaneous reports are considered suspected for reporting purposes.

For Pantozol

Serious adverse event report, Stevens-Johnson syndrome and Toxic epidermal necrolysis [death] assessed as unlisted according to the Basic Prescribing Information.

It is Novartis' policy to assess a case with a fatal outcome as unlisted unless the label specifically mentions death as a possible outcome of the listed risks. Reporting physician causality is suspected.

Reporter's comment:

NOVARTIS, 21.02.2008 DE-002147023-CHNY2008DE00864

Sender diagnosis: not reported

Narrative

Report received from an Intensive Monitoring Project (German documentation center for severe skin disease) on 04 Feb 2008 via German Health Authority, BFARM (Ref: 9150171). This male patient was treated with Fensitil intravenous (dimetindene), Tavegil intravenous (clemastine), Pantozol oral (pantoprazole), ACC long oral (acetylcystein), Ursolfalk oral (acid ursodeoxycholic), Ciprobay oral (ciprofloxacin), Delix oral (ramipril), Nexium oral (esomeprazole), Prograf oral (tacrolimus), Meronem intravenous (meropenem), metoprolol oral, aciclovir oral, Fragmin P subcutaneous (dalteparin sodium), Sebiprox solution topical (ciclopirox), prednisolon intravenous, Psorcutan cream topical (calcipotriol), Psorcutan solution topical (calcipotriol),

Sifrol oral (pramipexol), atropin subcutaneous, Sab Simplex oral (simethicone), Kalinor Brause oral (potassium citrate, potassium hydrogenocarbonate), Solu-Decortin H intravenous (prednisolone), Noctamid oral (lorazepam), Placebo oral, Tramal Tropfen oral (tramadol), Diflucan oral (fluconazol), Pipril intravenous (piperacillin), Combactam intravenous (sulbactam), vancomycin intravenous, glucose intravenous, metoclopramide Tropfen oral, ACC intravenous (acetylcystein), sodium chloride intravenous, Konaktion Tropfen oral (phytomenadion), Benuron suppository (paracetamol), Kanavit Tropfen oral (phytomenadion), Lasix oral (furosemid), Fortral intravenous (pentazocin), Dormicum intravenous (midazolam), Tutofusin intravenous, Benuron oral (paracetamol), Ciprobay intravenous (ciprofloxacin), Maxipime intravenous (cefepim) and with Caliumbrause oral (calcium carbonate). On 18 Jul 2004, the patient underwent an orthotopic liver transplantation. On 21 Jul 2004, he experienced fever up to 40.0 C. On 06 Aug 2004, he developed "Kokardes" and skin spots which were regarded as onset date of Stevens-Johnson syndrome or toxic epidermal necrolysis (confirmed case). He developed skin blisters on 08 Aug 2004 and the Nikolsky's sign was rated as positive on 10 Aug 2004. For the duration of reaction, the date of 22 Oct 2004 was stated. The reaction was rated as life-threatening and the patient died. In Mar 2005, the Intensive Monitoring Project assessed the causality with regard to the patient's multiple medication (using an algorithm with 8 different levels, ranging from 1=definite to 8=improbable) as follows: Ciprobay oral, Tramal Tropfen oral, Pipril intravenous, and Combactam intravenous were rated as "4=well possible". ACC long oral, Delix oral, Nexium oral, Prograf oral, Meronem intravenous, Aciclovir oral, Fragmin P subcutaneous, Prednisolon intravenous, Sifrol oral, Solu-Decortin H intravenous, Diflucan oral, Vancomycin intravenous, MCP Tropfen oral, ACC intravenous, and Benuron suppository were rated as "5=possible". Pantozol oral, Sebiprox solution topical, Psorcutan cream topical, Psorcutan solution topical, Kalinor Brause oral, Noctamid oral, Placebo oral, Glucose intravenous, sodium chloride intravenous, Konaktion Tropfen oral, Kanavit Tropfen oral, Lasix oral, Fortral intravenous, and Dormicum intravenous were rated as "6=questionable". Ursofalk oral, Metoprolol oral, Atropin subcutaneous, Sab Simplex oral, Tutofusin intravenous, Benuron oral, Fenistil intravenous, Ciprobay intravenous, Maxipime intravenous, Caliumbrause oral, and Tavegil intravenous were rated as "7=no causal relationship". The patient had been hospitalised in Intensive Care Unit from 18 Jul to 20 Jul 2004, in the department of Surgery from 20 Jul to 23 Jul 2004, and in the department of Internal Medicine from 23 Jul 2004 on until death. It was added that the patient had no infection or other possible risk factors. Outcome: death.

Sender's comment:

For Fensitil and Tavegil:

This is a serious adverse event (death). The events Stevens-Johnson syndrome and Toxic epidermal necrolysis (and related symptoms) were assessed as unlabelled according to the product information. It is Novartis policy to assess a case with a fatal outcome as unlabelled unless the label specifically mentions death as possible outcome of the labelled risks. Investigator causality was not-suspected.

For Acetylcystein, Aciclovir, and Metoclopramide.

Serious spontaneous report (death) assessed as unlisted according to the Core Data Sheet Information.

However other alternative causes (other drugs) provide a possible explanation for the reported adverse event.

All spontaneous reports are considered suspected for reporting purposes.

For Pantozol

Serious adverse event report, Stevens-Johnson syndrome and Toxic epidermal necrolysis [death] assessed as unlisted according to the Basic Prescribing Information.

It is Novartis' policy to assess a case with a fatal outcome as unlisted unless the label specifically mentions death as a possible outcome of the listed risks. Reporting physician causality is suspected.

Reporter's comment:

NOVARTIS, 27.02.2008 DE-002147023-CHNY2008DE00864

Sender diagnosis: not reported

Narrative

Report received from an Intensive Monitoring Project (German documentation center for severe skin disease) on 04 Feb 2008 via German Health Authority, BFARM (Ref: 9150171). This male patient was treated with Fensitil intravenous (dimetindene), Tavegil intravenous (clemastine), Pantozol oral (pantoprazole), ACC long oral (acetylcystein), Ursofalk oral (acid ursodeoxycholic), Ciprobay oral (ciprofloxacin), Delix oral (ramipril), Nexium oral (esomeprazole), Prograf oral (tacrolimus), Meronem intravenous (meropenem), metoprolol oral, aciclovir oral, Fragmin P subcutaneous (dalteparin sodium), Sebiprox solution topical (ciclopirox), prednisolon intravenous, Psorcutan cream topical (calcipotriol), Psorcutan solution topical (calcipotriol), Sifrol oral (pramipexol), atropin subcutaneous, Sab Simplex oral (simethicone), Kalinor Brause oral (potassium citrate, potassium hydrogenocarbonate), Solu-Decortin H intravenous (prednisolone), Noctamid oral (lorazepam), Placebo oral, Tramal Tropfen oral (tramadol), Diflucan oral (fluconazol), Pipril intravenous (piperacillin), Combactam intravenous (sulbactam), vancomycin intravenous, glucose intravenous, metoclopramide Tropfen oral, ACC intravenous (acetylcystein), sodium chloride intravenous, Konaktion Tropfen oral (phytomenadion), Benuron suppository (paracetamol), Kanavit Tropfen oral (phytomenadion), Lasix oral (furosemid), Fortral intravenous (pentazocin), Dormicum intravenous (midazolam), Tutofusin intravenous, Benuron oral (paracetamol), Ciprobay intravenous (ciprofloxacin), Maxipime intravenous (cefepim) and with

Caliumbrause oral (cadium carbonate). On 18 Jul 2004, the patient underwent an orthotopic liver transplantation. On 21 Jul 2004, he experienced fever up to 40.0 C. On 06 Aug 2004, he developed "Kokardes" and skin spots which were regarded as onset date of Stevens-Johnson syndrome or toxic epidermal necrolysis (confirmed case). He developed skin blisters on 08 Aug 2004 and the Nikolsky's sign was rated as positive on 10 Aug 2004. For the duration of reaction, the date of 22 Oct 2004 was stated. The reaction was rated as life-threatening and the patient died. In Mar 2005, the Intensive Monitoring Project assessed the causality with regard to the patient's multiple medication (using an algorithm with 8 different levels, ranging from 1=definite to 8=improbable) as follows: Ciprobay oral, Tramal Tropfen oral, Pipril intravenous, and Combactam intravenous were rated as "4=well possible". ACC long oral, Delix oral, Nexium oral, Prograf oral, Meronem intravenous, Aciclovir oral, Fragmin P subcutaneous, Prednisolon intravenous, Sifrol oral, Solu-Decortin H intravenous, Diflucan oral, Vancomycin intravenous, MCP Tropfen oral, ACC intravenous, and Benuron suppository were rated as "5=possible". Pantozol oral, Sebiprox solution topical, Psorcutan cream topical, Psorcutan solution topical, Kalinor Brause oral, Noctamid oral, Placebo oral, Glucose intravenous, sodium chloride intravenous, Konakion Tropfen oral, Kanavit Tropfen oral, Lasix oral, Fortral intravenous, and Dormicum intravenous were rated as "6=questionable". Ursofalk oral, Metoprolol oral, Atropin subcutaneous, Sab Simplex oral, Tufotusin intravenous, Benuron oral, Fenistil intravenous, Ciprobay intravenous, Maxipime intravenous, Caliumbrause oral, and Tavegil intravenous were rated as "7=no causal relationship". The patient had been hospitalised in Intensive Care Unit from 18 Jul to 20 Jul 2004, in the department of Surgery from 20 Jul to 23 Jul 2004, and in the department of Internal Medicine from 23 Jul 2004 on until death. It was added that the patient had no infection or other possible risk factors. Outcome: death.

Follow-up information received on 22 Feb 2008. This case was also reported from License Partner Nycomed (80328). Pantozol treatment was recoded.

Sender's comment:

For Fenistil and Tavegil:

This is a serious adverse event (death). The events Stevens-Johnson syndrome and Toxic epidermal necrolysis (and related symptoms) were assessed as unlabelled according to the product information. It is Novartis policy to assess a case with a fatal outcome as unlabelled unless the label specifically mentions death as possible outcome of the labelled risks. Investigator causality was not-suspected.

For Acetylcystein, Aciclovir, and Metoclopramide:

Serious adverse event report [death] assessed as unlisted according to the Basic Prescribing Information.

However other alternative causes [other drugs] provide a possible explanation for the reported adverse event. Reporting physician causality is suspected.

For Pantozol

Serious adverse event report, Stevens-Johnson syndrome and Toxic epidermal necrolysis [death] assessed as unlisted according to the Basic Prescribing Information.

It is Novartis' policy to assess a case with a fatal outcome as unlisted unless the label specifically mentions death as a possible outcome of the listed risks. Reporting physician causality is suspected.

Reporter's comment:

GRUNENTHAL, 27.02.2008 DE-GRUNENTHAL-2008 10691

Sender diagnosis: not reported

Narrative

Manufacturer Control Number: 2008-10691

This solicited case from special registry for severe skin reactions from GERMANY, that was reported by a physician via regulatory authority was received at GRT on 19-FEB-2008 and concerns a 62- year- old male patient (height unknown / weight unknown).

The relevant medical history included: Prophylaxis against gastrointestinal ulcer , Sputum retention , Gastroesophageal reflux disease , Infection prophylaxis , Hypertension , Restless legs syndrome , Antiviral prophylaxis , Thrombosis prophylaxis , Transplant rejection , Psoriasis , Extrapryamidal disorder , Gastrointestinal disorder , Hypokalaemia , Prophylaxis against transplant rejection , Sleep disorder , Pain , Fungal infection , Infection prophylaxis , Enteral nutrition , Nausea , Hypovolaemia , Vitamin supplementation , Oedema , Endoscopic retrograde cholangiopancreatography , Mineral supplementation.

The relevant concomitant medication included:

PANTOPRAZOLE SODIUM / PANTOZOL;
 CICLOPIROX / CICLOPIROX;
 CALCIPOTRIOL / PSORCUTAN;
 CALCIPOTRIOL / PSORCUTAN;
 ATROPINE SULFATE / ATROPIN;
 DIMETICONE / SAB SIMPLEX;
 POTASSIUM BICARBONATE, POTASSIUM CITRATE / KALINOR;
 LORMETAZEPAM / NOCTAMID;
 PLACEBO;
 GLUCOSE / GLUCOSE;
 SODIUM CHLORIDE / SODIUM CHLORIDE;
 PHYTOMENDADIONE / KONAKION;
 PHYTOMENDADIONE / KANAVIT;
 FUROSEMIDE / LASIX;
 PENTAZOCINE / FORTRAL;
 MIDAZOLAM / DORMICUM;
 AMINO ACIDS NOS, ELECTROLYTES NOS, VITAMINS NOS / TUTOFUSIN;
 PARACETAMOL / BENURON;
 CIPROFLOXACIN / CIPROBAY;
 CEFEPIME HYDROCHLORIDE / MAXIPME;
 CALCIUM / CALCIUM

This case was reported by the Documentation Centre for Severe Skin Reactions (Freiburg/Germany) via BfArM and concerns a 62-year-old multimorbid patient on multimедication (incl. tramadol). On 18-JUL-2004, this patient underwent orthotopic liver re-transplantation. He developed fever up to 40.0 degrees Celsius on 21-JUL-2004.

On 06-AUG-2004, 10 days after administration of first administration of tramadol hydrochloride, cockades and maculae appeared followed by skin blisters on 08-AUG-2004. Nikolski + on 10-AUG-2004.

The patient was hospitalized from 18-JUL-2004 to 20-JUL-2004 in the intensive care unit of a hospital, from 20-JUL-2004 to 23-JUL-2004 in the surgery department and from 23-JUL-2004 in the internal medicine department.

The reported symptoms were considered to be life-threatening. The patient died.

No further information was requested.

Third party case numbers

E2B Authority Number: DE-BFARM-08009522
 (GLS)-DE-GRT: DE-08-DS-0109
 Patient ID: 9150171

Sender's comment:

The adverse event toxic epidermal necrolysis is assessed as serious due to the criteria life-threatening and fatal outcome.

Toxic epidermal necrolysis is not listed in the reference safety information of tramadol hydrochloride.

The adverse event occurred in a temporal relationship with the administration of tramadol hydrochloride, but therapy with tramadol hydrochloride was already terminated 5 days before the first symptoms occurred.

This multimorbid patient, who underwent orthotopic liver re-transplantation, was treated in consequence with a multitude of other drugs, among them ciprofloxacin, piperacillin sodium and sulbactam sodium that are all known to cause severe skin reactions. Under consideration of these facts presenting high risk for the patient to develop toxic epidermal necrolysis it is unlikely that tramadol hydrochloride contributed to the adverse event.

In conclusion toxic epidermal necrolysis is assessed to be unlikely related to the administration of tramadol hydrochloride.

The information in this single case does not appear to alter the favourable benefit to risk ratio of tramadol hydrochloride.

*** F/U Information ***

FU information from 26-02-2008: After an internal review the reporters causality was changed from -not reported- to -possible- in this case. The initial assessment not changed.

Reporter's comment:

GRUNENTHAL, 27.02.2008 DE-GRUNENTHAL-2008 10691

Sender diagnosis: not reported

Narrative

Manufacturer Control Number: 2008-10691

This solicited case from special registry for severe skin reactions from GERMANY, that was reported by a physician via regulatory authority was received at GRT on 19-FEB-2008 and concerns a 62- year- old male patient (height unknown / weight unknown).

The relevant medical history included: Prophylaxis against gastrointestinal ulcer , Sputum retention , Gastroesophageal reflux disease , Infection prophylaxis , Hypertension , Restless legs syndrome , Antiviral prophylaxis , Thrombosis prophylaxis , Transplant rejection , Psoriasis , Extrapramidal disorder , Gastrointestinal disorder , Hypokalaemia , Prophylaxis against transplant rejection , Sleep disorder , Pain , Fungal infection , Infection prophylaxis , Enteral nutrition , Nausea , Hypovolaemia , Vitamin supplementation , Oedema , Endoscopic retrograde cholangiopancreatography , Mineral supplementation.

The relevant concomitant medication included:

PANTOPRAZOLE SODIUM / PANTOZOL;
CICLOPIROX / CICLOPIROX;
CALCIPOTRIOL / PSORCUTAN;
CALCIPOTRIOL / PSORCUTAN;
ATROPINE SULFATE / ATROPIN;
DIMETICONE / SAB SIMPLEX;
POTASSIUM BICARBONATE, POTASSIUM CITRATE / KALINOR;
LORMETAZEPAM / NOCTAMID;
PLACEBO;
GLUCOSE / GLUCOSE;
SODIUM CHLORIDE / SODIUM CHLORIDE;
PHYTOMENDADIONE / KONAKION;
PHYTOMENDADIONE / KANAVIT;
FUROSEMIDE / LASIX;
PENTAZOCINE / FORTRAL;
MIDAZOLAM / DORMICUM;
AMINO ACIDS NOS, ELECTROLYTES NOS, VITAMINS NOS / TUTOFUSIN;
PARACETAMOL / BENURON;
CIPROFLOXACIN / CIPROBAY;
CEFEPIME HYDROCHLORIDE / MAXIPME;
CALCIUM / CALCIUM

This case was reported by the Documentation Centre for Severe Skin Reactions (Freiburg/Germany) via BfArM and concerns a 62-year-old multimorbid patient on multimedication (incl. tramadol). On 18-JUL-2004, this patient underwent orthotopic liver re-transplantation. He developed fever up to 40.0 degrees Celsius on 21-JUL-2004.

On 06-AUG-2004, 10 days after administration of tramadol hydrochloride, cockades and maculae appeared followed by skin blisters on 08-AUG-2004. Nikolski + on 10-AUG-2004.

The patient was hospitalized from 18-JUL-2004 to 20-JUL-2004 in the intensive care unit of a hospital, from 20-JUL-2004 to 23-JUL-2004 in the surgery department and from 23-JUL-2004 in the internal medicine department.

The reported symptoms were considered to be life-threatening. The patient died.

No further information was requested.

Third party case numbers

E2B Authority Number: DE-BFARM-08009522
(GLS)-DE-GRT: DE-08-DS-0109
Patient ID: 9150171

Sender's comment:

The adverse event toxic epidermal necrolysis is assessed as serious due to the criteria life-threatening and fatal outcome.

Toxic epidermal necrolysis is not listed in the reference safety information of tramadol hydrochloride.

The adverse event occurred in a temporal relationship with the administration of tramadol hydrochloride, but therapy with tramadol hydrochloride was already terminated 5 days before the first symptoms occurred.

This multimorbid patient, who underwent orthotopic liver re-transplantation, was treated in consequence with a multitude of other drugs, among them ciprofloxacin, piperacillin sodium and sulbactam sodium that are all known to cause severe skin reactions. Under consideration of these facts presenting high risk for the patient to develop toxic epidermal necrolysis it is unlikely that tramadol hydrochloride contributed to the adverse event.

In conclusion toxic epidermal necrolysis is assessed to be unlikely related to the administration of tramadol hydrochloride.

The information in this single case does not appear to alter the favourable benefit to risk ratio of tramadol hydrochloride.

*** F/U Information ***

FU information from 26-02-2008: After an internal review the reporters causality was changed from -not reported- to -possible- in this case. The initial assessment not changed.

Reporter's comment:

NOVARTIS, 28.02.2008 DE-SZ09-CHNY2008DE00864

Sender diagnosis: not reported

Narrative

Report received from an Intensive Monitoring Project (German documentation center for severe skin disease) on 04 Feb 2008 via German Health Authority, BFARM (Ref: 9150171). This male patient was treated with Fensitil intravenous (dimetindene), Tavegil intravenous (clemastine), Pantozol oral (pantoprazole), ACC long oral (acetylcystein), Ursofalk oral (acid ursodeoxycholic), Ciprobay oral (ciprofloxacin), Delix oral (ramipril), Nexium oral (esomeprazole), Prograf oral (tacrolimus), Meronem intravenous (meropenem), metoprolol oral, aciclovir oral, Fragmin P subcutaneous (dalteparin sodium), Sebiprox solution topical (ciclopirox), prednisolon intravenous, Psorcutan cream topical (calcipotriol), Psorcutan solution topical (calcipotriol), Sifrol oral (pramipexol), atropin subcutaneous, Sab Simplex oral (simethicone), Kalinor Brause oral (potassium citrate, potassium hydrogenocarbonate), Solu-Decortin H intravenous (prednisolone), Noctamid oral (lormetazepam), Placebo oral, Tramal Tropfen oral (tramadol), Diflucan oral (fluconazol), Pipril intravenous (piperacillin), Combactam intravenous (sulbactam), vancomycin intravenous, glucose intravenous, metoclopramide Tropfen oral, ACC intravenous (acetylcystein), sodium chloride intravenous, Konaktion Tropfen oral (phytomenadion), Benuron suppository (paracetamol), Kanavit Tropfen oral (phytomenadion), Lasix oral (furosemid), Fortral intravenous (pentazocin), Dormicum intravenous (midazolam), Tutofusin intravenous, Benuron oral (paracetamol), Ciprobay intravenous (ciprofloxacin), Maxipime intravenous (cefepim) and with Caliumbrause oral (calcium carbonate). On 18 Jul 2004, the patient underwent an orthotopic liver transplantation. On 21 Jul 2004, he experienced fever up to 40.0 C. On 06 Aug 2004, he developed "Kokardes" and skin spots which were regarded as onset date of Stevens-Johnson syndrome or toxic epidermal necrolysis (confirmed case). He developed skin blisters on 08 Aug 2004 and the Nikolsky's sign was rated as positive on 10 Aug 2004. For the duration of reaction, the date of 22 Oct 2004 was stated. The reaction was rated as life-threatening and the patient died. In Mar 2005, the Intensive Monitoring Project assessed the causality with regard to the patient's multiple medication (using an algorithm with 8 different levels, ranging from 1=definite to 8=improbable) as follows: Ciprobay oral, Tramal Tropfen oral, Pipril intravenous, and Combactam intravenous were rated as "4=well possible". ACC long oral, Delix oral, Nexium oral, Prograf oral, Meronem intravenous, Aciclovir oral, Fragmin P subcutaneous, Prednisolon intravenous, Sifrol oral, Solu-Decortin H intravenous, Diflucan oral, Vancomycin intravenous, MCP Tropfen oral, ACC intravenous, and Benuron suppository were rated as "5=possible". Pantozol oral, Sebiprox solution topical, Psorcutan cream topical, Psorcutan solution topical, Kalinor Brause oral, Noctamid oral, Placebo oral, Glucose intravenous, sodium chloride intravenous, Konaktion Tropfen oral, Kanavit Tropfen oral, Lasix oral, Fortral intravenous, and Dormicum intravenous were rated as "6=questionable". Ursofalk oral, Metoprolol oral, Atropin subcutaneous, Sab Simplex oral, Tutofusin intravenous, Benuron oral, Fenistil intravenous, Ciprobay intravenous, Maxipime intravenous, Caliumbrause oral, and Tavegil intravenous were rated as "7=no causal relationship". The patient had been hospitalised in Intensive Care Unit from 18 Jul to 20 Jul 2004, in the department of Surgery from 20 Jul to 23 Jul 2004, and in the department of Internal Medicine from 23 Jul 2004 on until death. It was added that the patient had no infection or other possible risk factors. Outcome: death.

Follow-up information received on 22 Feb 2008. This case was also reported from License Partner Nycomed (80328). Pantozol treatment was recorded.

Sender's comment:

For Fenistil and Tavegil:

This is a serious adverse event (death). The events Stevens-Johnson syndrome and Toxic epidermal necrolysis (and related symptoms) were assessed as unlabelled according to the product information. It is Novartis policy to assess a case with a fatal outcome as unlabelled unless the label specifically mentions death as possible outcome of the labelled risks. Investigator causality was not-suspected.

For Acetylcystein, Aciclovir, and Metoclopramide:

Serious adverse event report [death] assessed as unlisted according to the Basic Prescribing Information.

However other alternative causes [other drugs] provide a possible explanation for the reported adverse event. Reporting physician causality is suspected.

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For Pantozol

Serious adverse event report, Stevens-Johnson syndrome and Toxic epidermal necrolysis [death] assessed as unlisted according to the Basic Prescribing Information.

It is Novartis' policy to assess a case with a fatal outcome as unlisted unless the label specifically mentions death as a possible outcome of the listed risks. Reporting physician causality is suspected.

Reporter's comment:

Wyeth-Pharma GmbH, 04.03.2008 DE-WYE-G01094108

Sender diagnosis: not reported

Narrative

Additional information was received with the reference number of the business partner Nycomed, who is marketing authorisation holder for Pantozol in Germany.

Information was received from a healthcare professional via a registry regarding a 63-year-old male patient who received Pipril (piperacillin injection) therapy and experienced "Stevens-Johnson-Syndrom/Toxic epidermal necrolysis (transition form)" with fatal outcome.

MEDICAL HISTORY:

The patient's concurrent illnesses include hypertension, restless legs syndrome, psoriasis, extrapyramidal disorder, fungal infection and transplant rejection (Transplant rejection reaction) with a past history of liver transplant. Past therapies included Ursfolk (ursodeoxycholic acid), Metoprolol, Atropine and Sab Simplex (dimeticone).

PRODUCT DETAILS:

Indication for Pipril was infection prophylaxis. Therapy began on 30-Jul-2004 and was permanently discontinued on 06-Aug-2004. Dose regimen was 2 g 4 times per day (intravenous).

Additional suspect medication included Pantozol (pantoprazole tablet, delayed release), Acc Long (acetylcysteine), Ciprobay (ciprofloxacin hydrochloride), Delix (ramipril), Nexium (esomeprazole), Prograf (tacrolimus), Meronem (meropenem), Aciclovir, Fragmin (heparin-fraction, sodium salt), Ciclopirox, Prednisolon (prednisolone), Psorcutan (calcipotriol), Pramipexole, Kalinor-Brausetabletten (potassium carbonate/potassium citrate tablet), Solu-Decortin-H (prednisolone sodium succinate), Noctamid (lormetazepam), Tramadol, Diflucan (fluconazole), Combactam (sulbactam sodium), Vancomycin, Glucose, Metoclopramide, Acetylcysteine, Sodium Chloride (unspec), Konakion (phytomenadione), Ben-U-Ron (paracetamol), Kanavit (phytomenadione), Lasix (furosemide), Fortral (pentazocine), Dormicum (midazolam maleate), Tutofusin (amino acids nos/electrolytes nos/vitamins nos), Ben-U-Ron (paracetamol), Fenistil (dimetindene maleate unspec), Ciprobay (ciprofloxacin hydrochloride) and Maxipime (cefepime hydrochloride).

CONCOMITANT THERAPY:

Concomitant medications were not reported.

EVENT DETAILS:

The patient had already been hospitalized (ICU) from 18-Jul-2004 to 20-Jul-2004 because of an orthotopic liver-re-transplantation. Afterwards, the patient was transferred to the surgical ward. He experienced fever on 21-Jul-2004 and was transferred to the internal medicine ward on 23-Jul-2004. There, he experienced cockades and spots on 06-Aug-2007, blisters on 08-Jul-2004 and Nikolsky's sign on 10-Jul-2004. The adverse event was diagnosed as "Stevens-Johnson-Syndrom/Toxic epidermal necrolysis (transition form)" (toxic epidermal necrolysis). The patient died on 22-Oct-2004 due to the reported event.

TEST RESULTS:

Body temperature (results: 40 deg. C.) was done on 21-Jul-2004.

The cause of death was reported as toxic epidermal necrolysis.

RELATEDNESS ASSESSMENT:

The investigator considered toxic epidermal necrolysis possibly related to the study product. Pipril has an assessment of 4 (range 1 stands for certain relationship to event, 4 stands for very possible relationship, range 8 unlikely relationship). The dermatological review committee assessed Ciprobay tablets, Tramal drops and Combactam very possible related to the event, too. All other suspect drugs got a lower degree of probability

Sender's comment:

WYETH COMMENT

INITIAL (v2) 25-Feb-2008

TOXISCH EPIDERMAL NEKROLYSE unter PIPRIL

Version 2 wurde erstellt, um die Referenz-Nummer des Gesch?ftspartners Nycomed (Zulassungsinhaber f?r Pantozol in Deutschland) hinzuzuf?gen. Bericht vom Dokumentationszentrum f?r schwere Hautreaktionen in Freiburg.

Es wird ?ber einen 63-j?hrigen Patienten berichtet, der nach Pipril-Behandlung ein "Stevens-Johnson-Syndrome/Toxisch epidermale Nekrolyse-?bergangsform" entwickelte und verstarb. Der Patient war am 18.07.04 lebertransplantiert worden und wurde seitdem station?r behandelt. Vom 30.07.04 bis zum 06.08.04 erhielt er Pipril. Am 06.08.04 entwickelte er Kokarden u. Flecken, am 08.08.04 Hautblasen und am 10.08.04 Nikolski Zeichen. Es wurde ein "Stevens-Johnson-Syndrome/Toxische epidermale Nekrolyse-?bergangsform" diagnostiziert.

Der Patient verstarb schlie?lich am 22.10.2004. Der Zusammenhang zwischen dem Stevens Johnson Syndrom/der Toxisch epidermalen Nekrolyse und Pipril wurde nach dem Bewertungsalgorithmus des Dokumentationszentrums mit "very possible" beurteilt. Es werden allerdings auch zahlreiche andere Arzneimittel verd?chtigt.

Ein m?glicher Zusammenhang kann nicht ausgeschlossen werden.

Hautmanifestationen wie Erythema exsudativum multiforme und Stevens-Johnson-Syndrom sind in der Fachinformation aufgef?hrt, Toxische epidermale Nekrolyse jedoch nicht. Ausserdem wird nicht auf einen m?glicherweise t?dlichen Ausgang hingewiesen, daher wurde das Ereignis als unbekannt eingestuft.

Basierend auf der aktuellen Information f?hrt dieser Bericht nicht zu einer ?nderung des Nutzen-Risiko-Verh?ltnisses. Zu diesem Zeitpunkt sind keine weiteren Massnahmen vorgesehen. Sobald ?nderungen hierzu bekannt werden, werden wir dar?ber unverz?glich und unaufgefordert informieren.

Katrin Wiesmann

Reporter's comment:

Bayer Vital GmbH, 05.03.2008 DE-BAYER-200813772GPV

Sender diagnosis: not reported

Narrative

This post-marketing study report was received on 20-Feb-2008 by BFARM, the regulatory authority of Germany, referring to a 62 year-old male.

Postmarketing surveillance with Ciprofloxacin (tablet).

Patient had concomitant conditions of hypertension, extrapyramidal motoric disturbance, gastrointestinal disturbance, hypokalemia, sleep disorder, and mycosis. Patient's relevant medical history includes orthotopic liver transplantation. Concomitant drugs have been given. For details see section "Concomitant Drugs" at the end of this report.

Patient received:

- Ciprobay (ciprofloxacin) for infection prophylaxis. Drug was administered on 17-Jul-2004 at a total daily dose of 500 MG orally, on 28-Jul-2004, for a duration of 5 days at a total daily dose of 1000 MG orally, and also administered on 04-Aug-2004 at a total daily dose of 1000 MG orally.
- Vancomycin (vancomycin) for infection prophylaxis. Drug was administered on 01-Aug-2004 at a total daily dose of 1 G intravenous.
- ACC Long (acetylcysteine) for mucolysis. Administration began on 20-Apr-2004, for a duration of approximately 3 months at a total daily dose of 600 MG orally.
- ACC (acetylcysteine) for mucolysis. Administration began on 01-Aug-2004, for a duration of 6 days at a total daily dose of 900 MG intravenous.
- Delix (ramipril) for hypertension. Drug was administered on 17-Jul-2004 at a total daily dose of 2.5 MG orally, and also administered on 20-Jul-2004, for a duration of approximately 3 months at a total daily dose of 5 MG orally.
- Nexium (esomeprazole magnesium) for gastric protection. Drug was administered on 17-Jul-2004 at a total daily dose of 20 MG orally, and also administered on 20-Jul-2004, for a duration of approximately 3 months at a total daily dose of 40 MG orally.
- Prograf (tacrolimus) for restless leg syndrome. Administration began on 17-Jul-2004, for a duration of approximately 3 months, total daily dose was not reported.
- Meronem (meropenem) for infection prophylaxis. Administration began on 18-Jul-2004, for a duration of 5 days at a total daily dose of 3 G intravenous, and also administered on 03-Aug-2004, for a duration of 4 days at a total daily dose of 3 G intravenous.
- Aciclovir (aciclovir) for antiviral prophylaxis. Administration began on 20-Jul-2004, for a duration of 3 days at a total daily dose of 600 MG orally, and also administered on 24-Jul-2004, for a duration of 5 days at a total daily dose of 600 MG orally.
- Fragmin P (heparin-fraction, sodium salt) for thrombosis prophylaxis. Administration began on 20-Jul-2004, for a duration of approximately 3 months at a total daily dose of 2500 IU subcutaneous.
- Prednisolon (prednisolone) for rejection reaction (liver transplant). Administration began on 20-Jul-2004, for a duration of 10 weeks and 5 days, total

daily dose was not reported.

- Sifrol (pramipexole dihydrochloride) for endoscopic retrograde cholangiopancreatography. Administration began on 20-Jul-2004, for a duration of approximately 3 months at a total daily dose of 240 MG orally.
- Solu-Decortin-H (prednisolone sodium succinate) for transplant rejection prophylaxis. Administration began on 23-Jul-2004, for a duration of 4 days, total daily dose was not reported.
- Tramal (tramadol hydrochloride) for pain nos. Drug was administered on 28-Jul-2004, total daily dose was not reported, and also administered on 30-Jul-2004, for a duration of 3 days, total daily dose was not reported.
- Diflucan (fluconazole) for mycosis. Administration began on 28-Jul-2004, for a duration of 8 days at a total daily dose of 300 MG orally.
- Pipril (piperacillin sodium) for infection prophylaxis. Administration began on 30-Jul-2004, for a duration of 8 days at a total daily dose of 8 G intravenous.
- Combactam (sulbactam sodium) for infection prophylaxis. Administration began on 30-Jul-2004, for a duration of 8 days at a total daily dose of 4 G intravenous.
- MCP (metoclopramide) for nausea. Drug was administered on 01-Aug-2004 at a total daily dose of 45 GTT orally.
- Benuron (paracetamol) for pain nos. Drug was administered on 02-Aug-2004 at a total daily dose of 500 MG rectal, and also administered on 04-Aug-2004, for a duration of 4 days at a total daily dose of 500 MG rectal.

On 18-Jul-2004, the patient was hospitalized for orthotopic liver transplantation.

On 06-Aug-2004, 20 days after starting treatment with Ciprobay, 5 days after treatment with Vancomycin, approximately 3 months after starting treatment with ACC Long, 5 days after starting treatment with ACC, 20 days after starting treatment that was ongoing with Delix, 20 days after starting treatment that was ongoing with Nexium, 20 days after starting treatment that was ongoing with Prograf, 19 days after starting treatment with Meronem, 17 days after starting treatment with Aciclovir, 17 days after starting treatment that was ongoing with Fragmin P, 17 days after starting treatment that was ongoing with Prednisolon, 17 days after starting treatment that was ongoing with Sifrol, 14 days after starting treatment with Solu-Decortin-H, 9 days after starting treatment with Tramal, 9 days after starting treatment with Diflucan, 7 days after starting treatment with Pipril, 7 days after starting treatment with Combactam, 5 days after treatment with MCP, and 4 days after starting treatment that was ongoing with Benuron patient experienced FATAL STEVENS-JOHNSON SYNDROME (fever up to 40 C, target lesions, maculae, skin blisters, positive Nikolsky's sign), and FATAL TOXIC EPIDERMAL NECROLYSIS (target lesions, maculae, skin blisters, positive Nikolsky's sign) considered serious due to life-threatening nature, and death. The events were treated with Fenistil (dimetinden) and Tavegil (clemastin).

Patient died on 22-Oct-2004. Autopsy information is unknown. The (ultimate) cause of death was unknown, as stated by the reporter.

For the drugs Ciprobay and Vancomycin the investigator considered the diagnoses FATAL STEVENS-JOHNSON SYNDROME and FATAL TOXIC EPIDERMAL NECROLYSIS to be associated.

Sender's comment:

STEVENS-JOHNSON-SYNDROME and TOXIC EPIDERMAL NECROLYSIS, considered serious due to fatal outcome, are listed in the International Product Information for CIPROBAY (ciprofloxacin). Both events are unlisted for the second suspect drug, VANCOMYCIN (not a Bayer product in this case). Based on the information received for this case, a causal relation between the reported events and treatment with ciprofloxacin and vancomycin cannot be excluded due to a compatible temporal relationship. In this case, both events had the same start date. According to the data provided, a clinical progression from STEVENS-JOHNSON-SYNDROME to TOXIC EPIDERMAL NECROLYSIS can be assumed. The reported events have also been reported to occur after administration of several of the co-suspect drugs the patient received, including anti-infectives as meronem, sulbactam, piperacillin and fluconazole, as well as esoprenazole, tramadol, paracetamol and ramipril. Their causal or contributive effect on course and outcome of the reported events cannot be excluded either due to a compatible temporal relationship. The patient's compromised immune system following liver transplant and multi-drug therapy may also have contributed to the fatal outcome of the reported events.

Reporter's comment:

APHQWEB, 07.03.2008 DE-Nycomed GmbH-80328

Sender diagnosis: not reported

Narrative

This case report was forwarded to Nycomed (Germany) by Novartis Pharma AG Switzerland (case n°CHNY2008DE00864).

Novartis Pharma AG Switzerland received the case from an intensive monitoring project (German documentation center for severe skin disease, ref.no 9150171) via the German Health Authority (BFARM).

An elderly male patient experienced Stevens-Johnson syndrome and toxic epidermal necrolysis while receiving pantoprazole (Pantozol 40 mg once

daily) for an unknown indication for about three months.

Further 40 suspected drugs see list.

On 18-JUL-2004, the patient underwent an orthotopic liver transplantation.

On 21-JUL-2004, he experienced fever up to 40°C.

On 06-AUG-2004, he developed cockades and skin spots which were regarded as onset date of Stevens-Johnson-syndrome or toxic epidermal necrolysis (confirmed case).

On 08-AUG-2004 he developed skin blisters and the Nikolsky's sign was rated as positive on 10-AUG-2004.

The patient had been hospitalised in an intensive Care Unit from 18-JUL to 20-JUL-2004, in the department of surgery from 20-JUL to 23-JUL-2004, and in the department of Internal Medicine from 23-JUL-2004 on until death. It was added that the patient had no infection or other possible risk factor.

For the duration of reaction, the date of 22-Oct-2004 was stated. The reaction was rated as life-threatening and the patient died. No information regarding autopsy report was provided.

Outcome: death

Medical history and concomitant diseases included rash, nausea, hypertension, extrapyramidal disorder, stomach discomfort, hypokalaemia, sleep disorder, pain, mycosis fungoides, oedema, psoriasis, reflux gastritis.

The Intensive Monitoring Project assessed the causality with regard to the patient's multiple medication (using an algorithm with 8 different levels ranging from 1=definite to 8=improbable). Pantoprazole oral was rated as 6=questionable due to this algorithm. FOLLOW UP 28-FEB-2008 (information received on 25-FEB-2008):

A duplicate case was received from our licensee WYETH/US via the German Health authority (BfArM ref no. DE-BFARM-08009522).

MFR Control No° DE-WYE-G01094108.

Following additional information was received:

Indication for pantoprazole therapy was gastritis prophylaxis.

The adverse event was diagnosed as transition form of "Stevens-Johnson syndrome/Toxic epidermal necrolysis". Due to the information that this was a transition form it was coded as "toxic epidermal necrolysis". STEVENS-JOHNSON-SYNDROME/TOXICAL EPIDERMAL NECROLYSIS:

This patient developed Steven-Johnson-Syndrome/toxical epidermal necrolysis approximately 14 days after last intake of of pantoprazole, after having been on pantoprazole treatment for approximately three months.

Concomitantly, the patient received multiple other drugs (40 suspected drugs).

Steven-Johnson-Syndrome/toxical epidermal necrolysis are rare skin disorders, characterized by mucosal erosions, skin blisters, often accompanied by fever. They can be drug-induced or due to infectious diseases.

Several drugs have been observed to cause Steven-Johnson-Syndrome/toxical epidermal necrolysis. Toxic bullous skin reactions up to acute disseminated epidermal necrolysis have also been observed in very rare cases with pantoprazole and other proton pump inhibitors. Although Steven-Johnson-Syndrome and toxical epidermal necrolysis are considered labelled for pantoprazole, this event was considered to be unlabeled, since pantoprazole cases with "fatal outcome" are considered as unlabelled. However, the mortality is high in general for patients suffering from Steven-Johnson-Syndrome and toxic epidermal necrolysis (20-25%; Mockenhaupt et al 2008).

Sender's comment:

With the information on hand Nycomed is now going to assess the case in order to come to a conclusion.

Temporal relationship questionable. Mockenhaupt et al (2008) suggest, that Stevens-Johnson syndrome and toxical epidermal necrolysis often begin more than four and up to 28 days after the initiation of the responsible medication. The patient had been on treatment with pantoprazole for several month and pantoprazole had been discontinued two weeks before onset of the event. The patient received multiple other suspected drugs (ciprofloxacin, tramadol, piperacillin and sulbactam sodium had been rated as 4=well possible by the reporters algorithm). In addition, the patient had recently undergone a liver transplantation.

Due to the questionable temporal relationship to pantoprazole therapy and the more reasonable temporable relationship to several of the various concomitant medications, Nycomed assesses this case as unlikely related.

This report is not considered to have any influence on current knowledge regarding the safety profile of pantoprazole and has no effect on the status of the drug as reflected by the formal product characteristics.

FOLLOW UP 28-FEB-2008 (information received on 25-FEB-2008):

The a.m causality assessment remain unchanged.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08009364

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		Yes

Patient

Initials:	Sex: Male	Birth: privacy	Age: 70 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Nasal ulcer	20041030				Unknown
2 Macule	20041030				Unknown
3 Oral ulceration	20041030				Unknown
4 Abrasions	20041103			11 Day	not recovered/not resolved
5 Erythroderma	20041107			15 Day	not recovered/not resolved
6 Fever	20041028			5 Day	not recovered/not resolved
7 Stevens Johnson syndrome	20041030			7 Day	not recovered/not resolved
8 Oral mucosa erosion	20041103			11 Day	not recovered/not resolved
9 Rash	20041030			7 Day	not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
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1	acc acetylcysteine	Intravenous (not otherwis /	20041023- -	Hyperuricemia
2	aciclovir aciclovir	Intravenous (not otherwis /	20041025-20041027 3 Day	
3	ALLOPURINOL allopurinol	Oral /	20041024-20041028 5 Day	Acute renal failure
4	altinsulin insulin	Intravenous (not otherwis 50 lu international unit(s)/l	20041023- -	
5	ARTERENOL norepinephrine hydrochloride	Intravenous (not otherwis /	20041025-20041030 6 Day	
6	ass acetylsalicylic acid	Oral /	20041024-20041101 9 Day	
7	atrovent ipratropium bromide	Respiratory (inhalation) /	20041023- -	COPD
8	atrovent ipratropium bromide	Respiratory (inhalation) /	20041030- -	COPD
9	BENALAPRIL enalapril	Oral /	20020101-20041023 2 Year	Hypertension
10	CEFUROXIM cefuroxime sodium	Intravenous (not otherwis /	20041027-20041031 5 Day	Rash
11	clinomel amino acids nos; calcium chloride dihydrate; electrolytes nos; glucose; lipids nos	Intravenous (not otherwis /	20041030- -	Nutritional supplement
12	clinomel amino acids nos; calcium chloride dihydrate; electrolytes nos; glucose; lipids nos	Intravenous (not otherwis /	20041025-20041025 1440 Minute	Nutritional supplement
13	diclofenac diclofenac	Oral /	20020101- -	Pain
14	DOBUTAMIN dobutamine hydrochloride	Intravenous (not otherwis /	20041029- -	
15	EBRANTIL urapidil	Intravenous (not otherwis /	20041023-20041023 1440 Minute	Hypertension
16	EUPHYLONG theophylline	Oral /	20041025-20041025 1440 Minute	COPD
17	EUPHYLONG theophylline	Oral /	20041023-20041023 1440 Minute	COPD
18	fenta fentanyl	Intravenous (not otherwis /	20041023-20041023 1440 Minute	Pain
19	fresubin carbohydrates nos; fatty acids nos; minerals nos; proteins nos; vitamins nos	Oral /	20041026-20041030 5 Day	Nutritional supplement
20	FRISIUM clobazam	Oral /	20041023-20041023 1440 Minute	Sedation
21	FUROSEMIDE furosemide	Intravenous (not otherwis /	20041027-20071029 3 Year	Nausea
22	FUROSEMIDE furosemide	Intravenous (not otherwis 60 Mg milligram(s)/Day	20041024-20041025 2 Day	Nausea
23	glucose glucose	Intravenous (not otherwis /	20041020-20041101 13 Day	
24	haes-steril hetastarch; sodium chloride	Intravenous (not otherwis /	20041028-20041031 4 Day	
25	haes-steril hetastarch; sodium chloride	Intravenous (not otherwis /	20041023-20041023 1440 Minute	
26	HEPARIN heparin	Intravenous (not otherwis /	20041023- -	Thrombosis prophylaxis
27	Hydrokortison hydrocortisone	Intravenous (not otherwis /	20041030- -	

			-	
28	kcl potassium chloride	Intravenous (not otherwis /	20041023-20041028	Hypokalemia 6 Day
29	KLACID clarithromycin	Oral /	20041027-20041030	Rash 4 Day
30	klean-prep macrogol; potassium chloride; sodium bicarbonate; sodium chloride; sodium sulfate anhydrous	Intravenous (not otherwis /	20041103-20041103	Bowel discomfort 1440 Minute
31	Natriumbikarbonat sodium bicarbonate	Intravenous (not otherwis /	-	-
32	Natriumbikarbonat sodium bicarbonate	Intravenous (not otherwis /	20041102-20041103	2 Day
33	NORADRENALINE norepinephrine	Intravenous (not otherwis /	20041029-	-
34	NORADRENALINE norepinephrine	Intravenous (not otherwis /	20041023-20041023	1440 Minute
35	OPTIPECT KODEIN codeine	Oral /	20020101-	COPD -
36	ORFIRIL valproic acid	Intravenous (not otherwis /	20041023-20041023	Epilepsy 1440 Minute
37	panthenol panthenol	Intravenous (not otherwis /	20041029-	-
38	PANTOZOL pantoprazole	Oral 1 Df dosage form/	20041024-20041028	Prophylaxis 5 Day
39	PANTOZOL pantoprazole sodium	Intravenous (not otherwis / 231	20041023-20041023	Prophylaxis 1440 Minute
40	PARACETAMOL paracetamol	Rectal /	20041024-20041025	Pain 2 Day
41	PASPERTIN metoclopramide hydrochloride	Intravenous (not otherwis /	20041029-	Infection -
42	PHENYTOIN phenytoin	Intravenous (not otherwis /	20041028-20041102	Epilepsy 6 Day
43	PHENYTOIN phenytoin	Oral /	20041026-20041028	Epilepsy 3 Day
44	PHENYTOIN phenytoin	Intravenous (not otherwis /	20041025-20041025	Epilepsy 1440 Minute
45	PRES enalapril maleate	Oral /	20041023-20041023	Renal failure 1440 Minute
46	PROPOFOL propofol	Intravenous (not otherwis /	20041023-20041027	Sedation 5 Day
47	prostigmin neostigmine bromide	Intravenous (not otherwis /	20041029-	-
48	salbutamol salbutamol	/	20041023-	COPD -
49	SOLOSIN theophylline	Oral /	20041025-20041027	COPD 3 Day
50	SYNACTHEN tetracosactide	Intravenous (not otherwis /	20041030-20041030	Epilepsy 1440 Minute
51	tavor lorazepam	Intravenous (not otherwis 1 Mg milligram(s)/Day	20041023-20041025	Sedation 3 Day
52	tazobac piperacillin sodium; tazobactam sodium	Intravenous (not otherwis /	20041030-20041031	Rash 2 Day
53	thomaejonin calcium chloride dihydrate; magnesium chloride hexahydrate; potassium chloride; sodium acetate; sodium chloride	Intravenous (not otherwis /	20041023-20041029	7 Day

54	tracutil minerals nos	Intravenous (not otherwis /	20041030-	
55	trapanal thiopental sodium	Intravenous (not otherwis / 231	20041026-20041031	Sedation 6 Day
56	VALPROATE SODIUM valproate sodium	Intravenous (not otherwis /	20041023-	Epilepsy
57	vancomycin vancomycin	Intravenous (not otherwis /	20041029-20041029	Rash 1440 Minute
58	XANEF enalapril maleate	Oral /	20041024-20041027	Renal failure 4 Day

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Hypertension			Unknown	
Epilepsy			Unknown	
Status epilepticus			Unknown	
Blood sugar level fluctuation			Unknown	
Enteroclysis			Unknown	
Increased bronchial secretion			Unknown	
Volume depletion			Unknown	
Poor peripheral circulation			Unknown	
Hypokalemia			Unknown	
Hyperuricemia			Unknown	
Rhinitis			Unknown	
Renal insufficiency			Unknown	
Pruritus			Unknown	
Pneumonia			Unknown	
Pain			Unknown	
Nausea			Unknown	
Infection			Unknown	
Acute renal failure			Unknown	
Bacteraemia			Unknown	
Cardiac insufficiency			Unknown	
COPD			Unknown	
Exanthema			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:		
Qualification:	Other Health Professional	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

009507513, 29.01.2008 DE-MERCK-0801DEU00104

Sender diagnosis: not reported

Narrative

Information has been received from a registry, documenting at least 95% of all serious skin reactions that occur in Germany, concerning a 70-year-old hospitalized male patient with arterial hypertension, chronic obstructive pulmonary disease, pain, epilepsy, status epilepticus (reason for hospitalization on 23-OCT-2004), renal insufficiency, hypokalaemia, blood glucose fluctuation, cardiovascular instability, increased bronchial secretion, hyperuricaemia, acute renal failure, infection and nausea who, on 24-OCT-2004, was placed on therapy with enalapril maleate, 5 mg tablet, 5-10 mg daily for the treatment of renal insufficiency.

Prior and concomitant suspect secondary therapy included enalapril maleate (PRES, licensee), tablet, 1.5 mg once given on 23-OCT-2004 for the treatment of renal insufficiency, enalapril maleate (BENALAPRIL, NOD), 1 unit twice a day po for the treatment of arterial hypertension (01-JAN-2002 until 23-OCT-2004), diclofenac taken as needed for the treatment of pain (dose not reported; 01-JAN-2002 until 23-OCT-2004), valproate sodium

(ORFIRIL), injection, 600 mg iv given once on 23-OCT-2004, for the treatment of status epilepticus and valproate sodium injection 400 - 5000 mg/h for the treatment of epilepsy (started on 23-OCT-2004, stop date not reported), urapidil (EBRANTIL), injection, 250 mg/h iv for the treatment of arterial hypertension (on 23-OCT-2004), nitrendipine (BAYOTENSIN), injection, 1 unit iv once on 23-OCT-2004 for the treatment of arterial hypertension, fentanyl citrate (FENTA), injection, 2.5 mg/h iv on 23-OCT-2004 for the treatment of pain, clobazam (FRISIUM), injection, 1 unit iv once on 23-OCT-2004 for sedation, lorazepam (TAVOR), injection, 1 mg once iv on 23-OCT-2004 and on 25-OCT-2004 for sedation, propofol, injection, 1000 mg/h iv from 23-OCT-2004 until 27-OCT-2004 for sedation, pentastarch (HAES-STERIL), infusion, given iv on 23-OCT-2004 and from 28-OCT-2004 until 31-OCT-2004 for fluid replacement (dose not reported), insulin, injection, 60 IU/h iv for the treatment of blood glucose fluctuation (started on 23-OCT-2004, stop date not reported), norepinephrine hydrochloride, injection, 5-25 mg/h iv given on 23-OCT-2004 and from 29-OCT-2004 onwards (stop date not reported) for the treatment of cardiovascular instability, heparin, injection, 2500 IE/h iv as thrombosis prophylaxis (started on 23-OCT-2004, stop date not reported), acetylcysteine, injection, 300 mg iv given daily for the treatment of increased bronchial secretion (stop date not reported), ipratropium bromide (ATROVENT), inhalant, for the treatment of COPD (dose not reported) from 23-OCT-2004 until 27-OCT-2004 and from 30-OCT-2004 onwards (stop date not reported), albuterol, inhalant, for the treatment of COPD (dose not reported, started on 23-OCT-2004, stop date not reported), pantoprazole sodium (PANTOZOL), injection, given iv on 23-OCT-2004 and from 28-OCT-2004 onwards as gastric ulcer prophylaxis (dose and stop date not reported) and pantoprazole sodium (PANTOZOL), 1 unit daily po from 24-OCT-2004 until 28-OCT-2004 as gastric ulcer prophylaxis, theophylline (EUPHYLONG), tablet, 575 mg given po on 24-OCT-2004, 375 mg theophylline given po 25-OCT-2004 and theophylline (SOLOSIN), solution, 40 drops given po daily (from 26-OCT-2004 until 27-OCT-2004), all three for the treatment of COPD, acetaminophen, suppository, given from 24-OCT-2004 until 25-OCT-2004 for the treatment of pain (dose not reported), allopurinol, 300 mg daily po from 24-OCT-2004 until 28-OCT-2004 for the treatment of hyperuricaemia, furosemide (FURO), injection, 20 mg iv three times a day (from 24-OCT-2004 until 25-OCT-2004) and 500 mg/h iv given (from 27-OCT-2004 until 29-OCT-2004) for the treatment of acute renal failure, aspirin, 100 mg po daily from 24-OCT-2004 until 01-NOV-2004 (indication not reported), acyclovir, injection, 500 mg iv given from 25-OCT-2004 until 27-OCT-2004 (indication not reported), phenytoin, injection, 750 mg/h iv given on 25-OCT-2004 and from 28-OCT-2004 onwards for the treatment of epilepsy (stop date not reported and phenytoin, 350 mg daily po from 26-OCT-2004 until 28-OCT-2004 for the treatment of epilepsy and thiopental sodium (TRAPANAL), injection, 1000-2500 mg/h iv given from 26-OCT-2004 until 01-NOV-2004 for sedation.

Other concomitant therapy included codeine, potassium chloride, dextrose (+) sodium chloride (+) sodium lactate (+) sodium acetate (+) calcium chloride (THOMAEJONIN), amino acids (unspecified) (+) dextrose (+) electrolytes (unspecified) (+) lipids (unspecified) (CLINOMEL), nutritional supplements (FRESUBIN), clarithromycin (KLACID), cefuroxime, vancomycin, dobutamine, neostigmine bromide (PROSTIGMIN), metoclopramide hydrochloride (PASPERTIN), panthenol, cosyntropin (SYNACTHEN), piperacillin sodium (+) tazobactam sodium (TAZOBAC), dextrose, sodium bicarbonate, hydrocortisone, minerals (unspecified) (TRACUTIL) and human albumin human.

On 27-OCT-2004, therapy with enalapril maleate was stopped.

On 28-OCT-2004, the patient developed fever and, on 30-OCT-2004, spots, on 04-NOV-2004, tongue erosions, on 05-NOV-2004, erosions of the nasal mucosa, on 06-NOV-2004, erosions of the oral mucosa and on 07-NOV-2004, generalized erythrodermia. Final diagnosis was Stevens-Johnson syndrome. Further course of events was not provided. The patient was reported as not recovered, however, a stop date of the AE was specified with 21-DEC-2004.

The reporting physician felt that Stevens-Johnson syndrome was possibly related to therapy with enalapril maleate (MSD) and/or enalapril maleate (PRES, licencee) and/or enalapril maleate (BENALAPRIL, NOD) and/or diclofenac and/or valproate sodium and/or urapidil (EBRANTIL), and/or nitrendipine (BAYOTENSIN) and/or fentanyl citrate (FENTA) and/or clobazam (FRISIUM and/or lorazepam (TAVOR) and/or propofol, and/or pentastarch (HAES-STERIL), and/or insulin, and/or norepinephrine hydrochloride and/or heparin and/or acetylcysteine and/or ipratropium bromide (ATROVENT), and/or albuterol, and/or pantoprazole sodium iv and po and/or theophylline (EUPHYLONG), tablet and solution and/or furosemide (FURO) and/or aspirin, and/or acyclovir and/or thiopental sodium (TRAPANAL) and that Stevens-Johnson syndrome was probably related to allopurinol and/or phenytoin.

Additional information is not expected.

Upon internal review, Stevens-Johnson syndrome was determined to be an Other Important Medical Event.

Sender's comment:

Stevens-Johnson-syndrome is a serious labeled event for therapy with XANEF°. In this setting of severe underlying conditions and multiple drug therapy the role of enalapril cannot be rated. The safety profile of the drug remains unchanged. Self-initiated measures are not indicated.

Reporter's comment:

Sender diagnosis: not reported

Narrative

Initial report

This case of Stevens-Johnson syndrome (SJS) was received from the German health authority (DE-BFARM-08017209). It was reported by the German Documentation Centre for Severe Skin Reactions in Freiburg / Breisgau, Germany (code-no. 9160240). The centre is conducting a prospective study on the incidence and drug aetiology of severe skin reactions. Diagnosis of SJS was made by an expert panel who assessed it as a "probable case" of SJS. Causal relationship was assessed by the experts in Mar. 2006 using an algorithm.

As for norepinephrine (Arterenol), theophylline (Solosin) and clobazam (Frisium) the causality was assessed as "possible". For the other suspected drugs causality was assessed as:

"probable": allopurinol, phenytoin

"possible": diclofenac, valproinic acid, urapidil, nitrendipine, enalapril, fentanyl, lorazepam, propofol, polyhydroxyethyl starch + sodium chloride, insulin, heparin sodium, acetylcysteine, ipatropium bromide, salbutamol, pantoprazole, theophylline (from another company), paracetamol, furosemide, acetylsalicylic acid, aciclovir, thiopental

Not suspected were: enalapril, codeine, potassium chloride, electrolyte infusion, norepinephrine (of other company), soya oil + glucose + calcium chloride, proteins + fats + carbon hydrates + vitamins + trace elements, clarithromycin, cefuroxime, vancomycin, dobutamine, neostigmin, metoclopramide, dexpanthenol, tetracosactide, piperacillin + tazobactam, glucose, sodium, hydrocortisone, trace elements, retinol + colecalciferol + alpha-tocopherol, albumin, dimetindene, blood components nos, metamizole, amidotrizoate, imipenem + cilastatin, amiodarone, metoprolol, magnesium salt, levetiracetam, xylometazoline, electrolytes.

Pneumonia and bacteremia were considered possible cause of SJS.

Epicrisis:

Patient: male, 70 years

Medical history: arterial hypertension, epilepsy, renal insufficiency, chronic obstructive pulmonary disease, hyperuricemia, hypokalemia, pneumonia, bacteremia.

The patient was hospitalised on 23-Oct-2004 due to epileptic state.

He was treated with clobazam (one tablet) on 23-Oct-2004 for tranquillisation, with theophylline 40 drops/day from 25-Oct-2004 to 27-Oct-2004 for chronic obstructive pulmonary disease, and with norepinephrine 5mg/hour from 25-Oct-2004 to 30-Oct-2004 for stabilisation for circulation. Before and during hospitalisation the patient was also treated with a lot of other drugs.

On 28-Oct-2004 the patient developed fever (not considered onset of event).

On 30-Oct-2004 generalised maculae were detected.

On 03-Nov-2004 the patient developed erosions of his tongue, on 04-Nov-2004 erosions of his nasal mucosa and on 06-Nov-2004 erosions of his oral mucosa.

On 07-Nov-2004 generalised erythrodermia was diagnosed.

The patient had not yet recovered on 21-Dec-2004.

Sender's comment:

29-Feb-2008 Sanofi Aventis Company Comment:

The case report presented here is properly documented, and the temporal relationship between the event and administration of norepinephrine hydrochloride, theophylline and clobazam seems plausible. Dechallenge and rechallenge is not applicable as concomitant medications were altered at the same time. In addition, concomitant drugs as reported here and/or pneumonia and bacteremia may be considered an alternative cause to explain the event.

29-FEB-2008 Sanofi Aventis Germany: STEVENS-JOHNSON SYNDROME is considered locally unlabelled for the suspected products. The facts presented here, however, are not very suggestive of a causal relationship (please refer to the intl. evaluator's comment), and thus do not imply a change in the state of the scientific knowledge for the products. Measures according to Para 29 German Drug Law (AMG), section (1) concerning the approval status are not deemed necessary.

Reporter's comment:

Wyeth-Pharma GmbH, 07.03.2008 DE-WYE-G01144608

Sender diagnosis: not reported

Narrative

Information was received from a registry via a regulatory authority regarding a 70-year-old male patient who received Tavor (lorazepam injection) therapy and experienced Stevens-Johnson syndrome.

MEDICAL HISTORY:

The patient's concurrent illnesses include epilepsy, cardiac failure and chronic obstructive pulmonary disease.

PRODUCT DETAILS:

Indication for Tavor was restlessness. The dose regimen was 1 mg 1 time per day (intravenous) from 23-Oct-2004 to 23-Oct-2004 and then was 1 mg 1 time per day (intravenous) from 25-Oct-2004 to 25-Oct-2004. Therapy was permanently discontinued.

Additional suspect medication included enalapril, Optipect Kodein (codeine), diclofenac, Orfiril (valproate sodium), Ebrantil (urapidil), Bayotensin (nitrendipine), Pres (enalapril maleate), fentanyl, Frisium (clobazam), propofol, potassium chloride, Haes-Steril (hydroxyethylcellulose/sodium chloride), Insulin (unspec), valproate sodium, Noradrenaline (norepinephrine), Heparin Sodium, acetylcysteine, Atrovent (ipratropium bromide), salbutamol, Pantozol (pantoprazole injection), Euphyllong (theophylline), paracetamol, Xanef (enalapril maleate), allopurinol, Pantozol (pantoprazole tablet, delayed release), Furo (furosemide), acetylsalicylic acid, theophylline, aciclovir, Solosin (theophylline), Arterenol (norepinephrine hydrochloride), phenytoin, Clinomel (amino acids/carbohydrates/minerals/vitamins), Fresubin (carbohydrates nos/fatty acids nos/minerals nos/proteins nos/vitamins nos), Trapanal (thiopental sodium), Klacid (clarithromycin), cefuroxime, Thomaejonin (calcium chloride dihydrate/magnesium chloride hexahydrate/potassium chloride/sodium acetate/sodium chloride), vancomycin, dobutamine, Prostigmin (neostigmine metilsulfate injection), Paspertin (metoclopramide hydrochloride), panthenol and Synacthen (tetracosactide). Wyeth Pharma is not the marketing authorisation holder for Pantozol (pantoprazole) in Germany.

CONCOMITANT THERAPY:

Concomitant medications were not reported.

EVENT DETAILS:

The patient had already been hospitalized since 23-Oct-2004 because of status epilepticus. He experienced fever (non-serious) on 28-Oct-2004 and Stevens-Johnson syndrome (Stevens-Johnson syndrome) on 30-Oct-2004.

The event was described as follows: 'On 30-Oct-2004 the patient experienced generalized blotchy changes of skin. On 03-Nov-2004 he experienced erosions of tongue. On 04-Nov-2004 he experienced erosions of nasal mucous membrane. On 06-Nov-2004 the patient experienced erosions of oral mucous membrane. On 07-Nov-2004 the patient experienced generalized erythrodermia.'

The patient received antibiotic treatment with Tazobac (piperacillin/ tazobactam) from 30-Oct-2004 to 31-Oct-2004 because of an infection. On 01-Nov-2004 he developed pneumonia and he received Zienam (cilastatin sodium/ imipenem) therapy. His exanthema was treated with hydrocortisone starting on 30-Oct-2004 and additionally he received Fenistil (dimetindene maleate) one time on 31-Oct-2004 for pruritus. Metamizol was administered for pain relief on 01-Nov-2004. The patient had to be reanimated on 02-Nov-2004 wherefore he received Cordarex (amiodarone hydrochloride) and Beloc (metoprolol tartrate) for his heart insufficiency. His epilepsy was treated with Keppra (levetiracetam) from 02-Nov-2004 and Otriven (xylometazoline hydrochloride) was administered for rhinitis. An X-ray of the gastrointestinal tract was performed on 01-Nov-2004 wherefore he received Gastrografin (meglumine amidotrizoate/sodium amidotrizoate). On 03-Nov-2004 a colonic irrigation was carried out. Further treatment included nutritional supplement and electrolyte substitutional therapy.

The patient has not recovered yet.

TEST RESULTS:

Body temperature (results: Body temperature increased) was done on 28-Oct-2004. X-ray gastrointestinal tract (results: unknown) was done on 01-Nov-2004.

RELATEDNESS ASSESSMENT:

The dermatological review committee has categorized Tavor as possibly related to skin reactions. Tavor has an assessment of 5 (range 1 stands for certain relationship to event, range 8 unlikely relationship). The same assessment was provided for diclofenac, Orfiril (valproate sodium), Ebrantil (urapidil), Bayotensin (nitrendipine), Pres (enalapril maleate), fentanyl, Frisium (clobazam), propofol, Haes-Steril (hydroxyethylcellulose/sodium chloride), Insulin (unspec), valproate sodium, heparin sodium, acetylcysteine, Atrovent (ipratropium bromide), salbutamol, Pantozol (pantoprazole injection), Euphyllong (theophylline), paracetamol, Xanef (enalapril maleate), Pantozol (pantoprazole tablet, delayed release), Furo (furosemide), acetylsalicylic acid, theophylline, aciclovir, Solosin (theophylline), Arterenol (norepinephrine hydrochloride) and Trapanal (thiopental sodium). The dermatological review committee categorized phenytoin and allopurinol as likely related to Stevens-Johnson syndrome. All other suspect drugs got a lower degree of probability. According to dermatological review committee the drugs given for treatment since 30-Oct-2004 have no temporal relationship to reaction or are not known for causing Stevens-Johnson syndrome.

Sender's comment:

WYETH COMMENT

INITIAL (v1) 28-Feb-2008

STEVENS JOHNSON SYNDROME unter Tavor

Bericht vom Dokumentationszentrum f?r schwere Hautreaktionen in Freiburg.

Ein 70-j?hriger Patient wurde auf Grund eines Status epilepticus am 23-Oct-2004 station?r aufgenommen. Zur Beruhigung erhielt der Patient am 23-Oct-2004 und 25-Oct-2004 Tavor Injektionsl?sung. Am 30-Oct-2004 zeigte der Patient dann Stevens Johnson Syndrom, zun?chst durch generalisierte Flecken, dann mit Erosionen von Zunge, Nasen- und Mund-Schleimhaut und schliesslich durch generalisierte Erythrodermie. Der Patient wurde noch nicht wiederhergestellt. Der Zusammenhang zwischen dem Stevens Johnson Syndrom und Tavor wurde nach dem Bewertungsalgorithmus des Dokumentationszentrums als m?glich beurteilt.

Ein m?glicher Zusammenhang kann nicht ausgeschlossen werden.

Stevens Johnson Syndrom ist in der Fachinformation nicht aufgef?hrt und wurde daher als unbekannt eingestuft.

Basierend auf der aktuellen Information f?hrt dieser Bericht nicht zu einer ?nderung des Nutzen-Risiko-Verh?ltnisses. Zu diesem Zeitpunkt sind keine weiteren Massnahmen vorgesehen. Sobald ?nderungen hierzu bekannt werden, werden wir dar?ber unverz?glich und unaufgefordert informieren.

Birte Kruse

Michael Pl?ger

Reporter's comment:

ratiopharm GmbH, 07.03.2008 DE-RATIOPH-2008R021838

Sender diagnosis: not reported

Narrative

Reported terms: Stevens-Johnson-Syndrome(Stevens-Johnson syndrome)

Case reference number 2008R021838 is a spontaneous case report from the RegiSCAR-Studie, sent by the Dokumentationszentrum für schwere Hautkrankheiten (dZh), Uniklinik Freiburg, Germany. It refers to a male aged 70 years.

The patient's past medical history included Enterocolitis (Enterocolitis). There is no information about risk factors. Concomitant diseases include Pain (Pain), COPD (Chronic obstructive pulmonary disease), Hypertension arterial (Hypertension), Status epilepticus (Status epilepticus), Hypertension (Hypertension), Blood sugar level fluctuation (Blood glucose fluctuation), Poor peripheral circulation (Poor peripheral circulation), Increased bronchial secretion (Increased bronchial secretion), Pneumonia (Pneumonia), Rhinitis (Rhinitis), Hypokalemia (Hypokalaemia), Acute renal failure (Renal failure acute), Epilepsy (Epilepsy), Volume depletion (Hypovolaemia), Renal insufficiency (Renal failure), Hyperuricemia (Hyperuricaemia) and Infection (Infection). No prior drugs are known to have been taken by the patient.

The patient started taking Theophyllin retard-ratiopharm 375 mg (Retardkapseln) (THEOPHYLLINE), indication COPD on 25-Oct-2004 and Aciclovir-ratiopharm 500 mg p.i. (Durchstechflaschen) (ACICLOVIR), indication unknown on 25-Oct-2004. The patient has been exposed to the following concomitant drug(s): Orfiril InjLsg 600mg (VALPROIC ACID), indication Status epilepticus, Optipect Kodein forte (CODEINE), indication COPD, Diclofenac (DICLOFENAC), indication Pain, Ebrantil InjLsg 250mg/h (URAPIDIL), indication Hypertension, Bayotensin InjLsg Akut (NITRENDIPINE), indication Hypertension, Benalaprill (ENALAPRIL), indication Hypertension arterial, Pres 1.5mg (ENALAPRIL), indication Renal insufficiency, Fenta InjLsg 2.5mg/h (FENTANYL), indication Pain, Paspertin (METOCLOPRAMIDE), indication Nausea, Prostigmin (NEOSTIGMINE), indication unknown, ACC InjLsg 3000mg (ACETYLCYSTEINE), indication Increased bronchial secretion, Paracetamol Supp b.B. (PARACETAMOL), indication Pain, Allopurinol (ALLOPURINOL), indication Hyperuricemia, Cernevit (VITAMINS), indication Nutritional support, Albumin (ALBUMIN), indication Protein abnormal NOS, Zienam (IMIPENEM AND ENZYME INHIBITOR), indication Pneumonia, Metamizol (METAMIZOLE SODIUM), indication Pain, Blutproduktinf. (BLOOD AND RELATED PRODUCTS), indication Drug therapy, Mg-Mineralien (MAGNESIUM OROTATE), indication Drug therapy, Furo InjLsg 20mg (FUROSEMIDE), indication Acute renal failure, Otriven (XYLOMETAZOLINE), indication Rhinitis, ASS 100mg (ACETYLSALICYLIC ACID), indication Boost of blood flow, Solosin Tropfen (THEOPHYLLINE), indication COPD, KleanPrep (MACROGOL, COMBINATIONS), indication Enterocolitis, Frisium (CLOBAZAM), indication Sedation, Propofol InjLsg 1000mg/h (PROPOFOL), indication Sedation, Heparin InjLsg 2500 IE/h (HEPARIN), indication Thrombosis prophylaxis, HAES InfLsg 10% (BLOOD SUBSTITUTES AND PLASMA PROTEIN FRACTIONS), indication Volume depletion, Panthenol (DEXPANTHENOL), indication Drug therapy, Clacid 250mg (CLARITHROMYCIN), indication Infection, Atrovent Inhalat LSO 0.25% (IPRATROPIUM BROMIDE), indication COPD, Pantozol InjLsg (PANTOPRAZOLE), indication Gastritis prophylaxis, Clinomel InfLsg 2.2% 30ml (ALL OTHER NON-THERAPEUTIC PRODUCTS), indication Parenteral nutrition, Salbutamol Inhalat (SALBUTAMOL), indication COPD, Tazobac (PIPERACILLIN AND ENZYME INHIBITOR), indication Infection, Trapanal InjLsg 1000-2500 mg/h (THIOPENTAL), indication Sedation, Thomaejonin InfLsg HG5 (ELECTROLYTES WITH CARBOHYDRATES), indication Volume depletion, Glucose (CARBOHYDRATES), indication Drug therapy, Beloc (METOPROLOL), indication Cardiac insufficiency, KCl InfLsg 14.9% 100mmol/h (POTASSIUM CHLORIDE), indication Hypokalemia, Valproat InjLsg 400-5000 mg/h (VALPROIC ACID), indication Epilepsy, Tavor InjLsg 1mg (LORAZEPAM), indication Sedation, Altinsulin InjLsg 50IE/h (INSULIN (HUMAN)), indication Blood sugar level fluctuation, Noradrenalin InjLsg 5mg/h, 25 mg/h (NOREPINEPHRINE), indication Stabilisation of circulation, Dobutamin (DOBUTAMINE), indication Stabilisation of circulation, Vancomycin (VANCOMYCIN), indication Infection, Xanef (ENALAPRIL), indication Renal insufficiency, Synacthen (TETRACOSACTIDE), indication Epilepsy, Phenytoin InjLst 750mg/h (PHENYTOIN), indication Epilepsy, Cefuroxim InjLsg (CEFUROXIME), indication Infection, NaHCO3 (SODIUM BICARBONATE), indication Drug therapy, Hydrocortison (HYDROCORTISONE), indication Exanthema, Tracutil (COMBINATIONS OF ELECTROLYTES), indication Nutritional support, Arterenol InjLsg 5mg/h (NOREPINEPHRINE), indication Stabilisation of circulation, Fresubin plus Sonde (ALL OTHER NON-THERAPEUTIC PRODUCTS), indication Enteral nutrition, Gastrografin (DIATRIZOIC ACID), indication Scan with contrast, Cordarex (AMIODARONE), indication Resuscitation, Keppra (LEVETIRACETAM), indication Epilepsy, Fenistil (DIMETINDENE), indication Pruritus and Euphylong (THEOPHYLLINE), indication COPD. The patient developed Stevens-Johnson-Syndrome (Stevens-Johnson syndrome)(start date 30-Oct-2004).

The following action was taken: for Theophyllin retard-ratiopharm 375 mg (Retardkapseln) (THEOPHYLLINE) - Drug withdrawn for Aciclovir-ratiopharm 500 mg p.i. (Durchstechflaschen) (ACICLOVIR) - Drug withdrawn.

The outcome for Stevens-Johnson-Syndrome (Stevens-Johnson syndrome) is Recovered / Resolved.

Rechallenge information for Theophyllin retard-ratiopharm 375 mg (Retardkapseln) is not applicable. Rechallenge information for Aciclovir-ratiopharm 500 mg p.i. (Durchstechflaschen) is not applicable. Dechallenge information for Theophyllin retard-ratiopharm 375 mg (Retardkapseln) is not applicable. Dechallenge information for Aciclovir-ratiopharm 500 mg p.i. (Durchstechflaschen) is not applicable.

The reporter's assessment for drug Aciclovir-ratiopharm 500 mg p.i. (Durchstechflaschen) related with Stevens-Johnson-Syndrome is as follows: Possible. The reporter's assessment for drug Theophyllin retard-ratiopharm 375 mg (Retardkapseln) related with Stevens-Johnson-Syndrome is as follows: Possible. CLASSIFICATION: Adverse Event: No, Adverse Reaction: Yes, Drug / Drug Interaction: No, Abuse / Misuse: No, Pregnancy / Lactation tracking: No

Sender's comment:

Steven-Johnson-Syndrom (SJS) is not SPC/RSI-listed neither for Theophyllin retard-ratiopharm 375 mg (Retardkapseln) nor for Aciclovir-ratiopharm 500 mg (solution for injection), respectively.

Generally, it cannot be excluded that reported symptomatic is related to Theophyllin and/or Aciclovir, however, the patient used multiple drugs concomitantly, which cannot be excluded for causing SJS, respectively. This is the first case including SJS in association with Theophyllin as well for Aciclovir after parenteral administration, in our Pharmacovigilance database. For Aciclovir after oral administration one case including erythema exsudativum multiforme as well one case including SJS were reported to the MAH so far.

Based on this evidence i.e. the use of multiple drugs with known potentiation of induction of a SJS (e.g. valproic acid, phenytoin, clarithromycin...), at present no actions are considered necessary.

Note: Within the case the outcome of patient was reported to be "not recovered/not resolved" but on the other hand reaction end date of SJS was given to be 21.12.2004. Therefore, the outcome is assessed to be "recovered/resolved" (at the time of this report). No further informations are available.

Reporter's comment:

Fresenius Kabi, 10.03.2008 DE-Fresenius Kabi-2008030051

Sender diagnosis: not reported

Narrative

Verbatim of the Reporter:

Stevens-Johnson-Syndrome (SJS)

(Was assessed as probable case according to the assessment of experts March 2006).

Case narrative:

The case was reported by the BfArM (health authority Germany) with Worldwide Reference Number DE-BFARM-08017209 and was detected in an epidemiological study (RegiSCAR Studie) in Germany.

On October 23rd, 2004 a 70-year-old male patient was admitted to the hospital due to status epilepticus. HAES steril 10% (dosage unknown) was administered on the same day and also from 28.10.2004 until 30.10.2004. On 30.10.2004 a Stevens-Johnson-Syndrome occurred and 04.11.2004 the patient suffered from erosions of the tongue, on 06.11. and on 07.11.2004 erosions of mouth, oral and nasal mucosa and generalized erythrodermia.

Medical history: Status epilepticus.

Concomitant medication:

Assessed as probable related to reaction according to the assessment of experts 03/2006: Phenytoin iv. 750 mg/h 25.10.04 and 28.10. - 03.11.04 and 350 mg/day per oral on 26.10. - 28.10.04; Allopurinol 300 mg/day per oral on 24.10. - 28.10.04.

Assessed as possibly related to reaction according to the assessment of experts 03/2006:

Trapanal (thiopental), Diclofenac, Orfiril (valproic acid), Ebrantil (urapidil), Bayotensin (nitredipin), Pres (enalapril), Fenta (fentanyl), Frisium (clobazam), Propofol, Altinsulin (insulin), Valproat (valproic acid), Heparin, ACC (acetylcystein), Atrovent (ipratropiumbromid), Salbutamol, Pantozol (pantoprazole), Euphyllong (theophylline), Paracetamol, Xanef (enalapril), Furo (furosemide), ASS (acetylsalicyl acid), Theophylline, Aciclovir, Solosin (theophylline) and Arterenol (norepinephrine).

Due to the huge number of co-medication irrelevant to assessment of causality with HAES steril 10% we decided to include in our database only such co-medication that has been suspected as being probably causative (allopurinol and phenytoin).

Sender's comment:

According to the reporter's assessment the reaction is possibly related to the administration of HAES-steril 10%.

We agree to the reporter's assessment. The reaction occurred in plausible temporal relationship to the administration of HAES steril 10%. However, concomitant medication allopurinol and phenytoin are more likely causative.

It can not be excluded that HES may have caused the event, as the reason for the occurrence of the first symptoms and the worsening remains unclear. However no similar other cases from spontaneous reporting or clinical studies - not deriving from this special epidemiologic study - are known for starches.

Steven-Johnson-Syndrome is not labeled as a possible side effect of hydroxyethyl starch.

Reporter's comment:

001307602, 17.03.2008 DE-ABBOTT-08P-062-0441536-00

Sender diagnosis: not reported

Narrative

This report received from Nycomed; reference number: 80493.

Report from Germany of Stevens Johnson syndrome coincident with CLARITHROMYCIN (KLACID) therapy.

This report was received from the German Documentation centre of severe skin reactions (ref. no. 9160240) via the German Health authority (BfArM ref. no. DE-BFARM-08017209).

This is a serious case due to hospitalization.

An elderly male patient experienced Stevens-Johnson syndrome while receiving PANTOPRAZOLE (PANTOZOL i.v. and PANTOZOL unknown dose p.o) for ulcer prophylaxis, urapidil (EBRANTIL 250 mg/h i.v) for hypertension, THEOPHYLLINE for COPD and TRAPANAL for sedation. Further 61 suspected drugs were administered.

On 21-Oct-2004 patient had been admitted to hospital due to status epilepticus.

On 28-Oct-2004 patient developed fever.

On 30-Oct-2004 patient developed generalized skin spots.

On 03-Nov-2004 patient developed tongue erosions.

On 04-Nov-2004 patient developed erosions of nasal mucous membranes.

On 06-Nov-2004 patient experienced erosions of oral mucous membranes.

On 07-Nov-2004 patient experienced generalized erythrodermia.

The outcome was reported as still ongoing at the time of the report.

Concomitant diseases included status epilepticus, epilepsy, hypertension, COPD, renal insufficiency, hyperuricaemia, pain, acute renal failure, infection, hypokalemia, nausea, pruritus, pneumonia, cardiac insufficiency, rhinitis and bacteraemia.

Sender's comment:

Abbott Germany Assessment: 17.03.2008, Klacid(R) - 08P-062-0441536-00 (0), PA
Die in diesem Fallbericht geschilderte Symptomatik ist in den Angaben zum Arzneimittel in adaequater Form beschrieben.

Reporter's comment:

Wyeth-Pharma GmbH, 18.03.2008 DE-WYE-G01144608

Sender diagnosis: not reported

Narrative

New version created to add Business Partner (Nycomed) Ref No (80493).

Information was received from a registry via a regulatory authority regarding a 70-year-old male patient who received Tavor (lorazepam injection) therapy and experienced Stevens-Johnson syndrome.

MEDICAL HISTORY:

The patient's concurrent illnesses include epilepsy, cardiac failure and chronic obstructive pulmonary disease.

PRODUCT DETAILS:

Indication for Tavor was restlessness. The dose regimen was 1 mg 1 time per day (intravenous) from 23-Oct-2004 to 23-Oct-2004 and then was 1 mg 1 time per day (intravenous) from 25-Oct-2004 to 25-Oct-2004. Therapy was permanently discontinued.

Additional suspect medication included enalapril, Optipect Kodein (codeine), diclofenac, Orfiril (valproate sodium), Ebrantil (urapidil), Bayotensin (nitrendipine), Pres (enalapril maleate), fentanyl, Frisium (clobazam), propofol, potassium chloride, Haes-Steril (hydroxyethylcellulose/sodium chloride), Insulin (unspec), valproate sodium, Noradrenaline (norepinephrine), Heparin Sodium, acetylcysteine, Atrovent (ipratropium bromide), salbutamol, Pantozol (pantoprazole injection), Euphyllong (theophylline), paracetamol, Xanef (enalapril maleate), allopurinol, Pantozol (pantoprazole tablet, delayed release), Furo (furosemide), acetylsalicylic acid, theophylline, aciclovir, Solosin (theophylline), Arterenol (norepinephrine hydrochloride), phenytoin, Clinomel (amino acids/carbohydrates/minerals/vitamins), Fresubin (carbohydrates nos/fatty acids nos/minerals nos/proteins nos/vitamins nos), Trapanal (thiopental sodium), Klacid (clarithromycin), cefuroxime, Thomaesonin (calcium chloride dihydrate/magnesium chloride hexahydrate/potassium chloride/sodium acetate/sodium chloride), vancomycin, dobutamine, Prostigmin (neostigmine metilsulfate injection), Paspertin (metoclopramide hydrochloride), panthenol and Synacthen (tetracosactide). Wyeth Pharma is not the marketing authorisation holder for Pantozol (pantoprazole) in Germany.

CONCOMITANT THERAPY:

Concomitant medications were not reported.

EVENT DETAILS:

The patient had already been hospitalized since 23-Oct-2004 because of status epilepticus. He experienced fever (non-serious) on 28-Oct-2004 and Stevens-Johnson syndrome (Stevens-Johnson syndrome) on 30-Oct-2004.

The event was described as follows: 'On 30-Oct-2004 the patient experienced generalized blotchy changes of skin. On 03-Nov-2004 he experienced erosions of tongue. On 04-Nov-2004 he experienced erosions of nasal mucous membrane. On 06-Nov-2004 the patient experienced erosions of oral mucous membrane. On 07-Nov-2004 the patient experienced generalized erythrodermia.'

The patient received antibiotic treatment with Tazobac (piperacillin/ tazobactam) from 30-Oct-2004 to 31-Oct-2004 because of an infection. On 01-Nov-2004 he developed pneumonia and he received Zienam (cilastatin sodium/ imipenem) therapy. His exanthema was treated with hydrocortisone starting on 30-Oct-2004 and additionally he received Fenistil (dimetindene maleate) one time on 31-Oct-2004 for pruritus. Metamizole was administered for pain relief on 01-Nov-2004. The patient had to be reanimated on 02-Nov-2004 wherefore he received Cordarex (amiodarone hydrochloride) and Beloc (metoprolol tartrate) for his heart insufficiency. His epilepsy was treated with Keppra (levetiracetam) from 02-Nov-2004 and Otriven (xylometazoline hydrochloride) was administered for rhinitis. An X-ray of the gastrointestinal tract was performed on 01-Nov-2004 wherefore he received Gastrografin (meglumine amidotrizoate/sodium amidotrizoate). On 03-Nov-2004 a colonic irrigation was carried out. Further treatment included nutritional supplement and electrolyte substitutional therapy.

The patient has not recovered yet.

TEST RESULTS:

Body temperature (results: Body temperature increased) was done on 28-Oct-2004. X-ray gastrointestinal tract (results: unknown) was done on 01-Nov-2004.

RELATEDNESS ASSESSMENT:

The dermatological review committee has categorized Tavor as possibly related to skin reactions. Tavor has an assessment of 5 (range 1 stands for certain relationship to event, range 8 unlikely relationship). The same assessment was provided for diclofenac, Orfiril (valproate sodium), Ebrantil (urapidil), Bayotensin (nitrendipine), Pres (enalapril maleate), fentanyl, Frisium (clobazam), propofol, Haes-Steril (hydroxyethylcellulose/sodium chloride), Insulin (unspec), valproate sodium, heparin sodium, acetylcysteine, Atrovent (ipratropium bromide), salbutamol, Pantozol (pantoprazole injection), Euphyllong (theophylline), paracetamol, Xanef (enalapril maleate), Pantozol (pantoprazole tablet, delayed release), Furo (furosemide), acetylsalicylic acid, theophylline, aciclovir, Solosin (theophylline), Arterenol (norepinephrine hydrochloride) and Trapanal (thiopental sodium). The dermatological review committee categorized phenytoin and allopurinol as likely related to Stevens-Johnson syndrome. All other suspect drugs got a lower degree of probability. According to dermatological review committee the drugs given for treatment since 30-Oct-2004 have no temporal relationship to reaction or are not known for causing Stevens-Johnson syndrome.

Sender's comment:**WYETH COMMENT**

1. FOLLOW UP (v2) 10-MAR-2008

STEVENS JOHNSON SYNDROME unter Tavor

Neue Version wurde erstellt um die Business Partner (Nycomed) Ref No (80493) einzufügen.

Bericht vom Dokumentationszentrum für schwere Hautreaktionen in Freiburg.

Ein 70-jähriger Patient wurde auf Grund eines Status epilepticus am 23-Oct-2004 stationär aufgenommen. Zur Beruhigung erhielt der Patient am 23-Oct-2004 und 25-Oct-2004 Tavor Injektionslösung. Am 30-Oct-2004 zeigte der Patient dann Stevens Johnson Syndrom, zunächst durch generalisierte

Flecken, dann mit Erosionen von Zunge, Nasen- und Mund-Schleimhaut und schliesslich durch generalisierte Erythrodermie. Der Patient wurde noch nicht wiederhergestellt. Der Zusammenhang zwischen dem Stevens Johnson Syndrom und Tavor wurde nach dem Bewertungsalgorithmus des Dokumentationszentrums als möglich beurteilt.

Ein möglicher Zusammenhang kann nicht ausgeschlossen werden.

Stevens Johnson Syndrom ist in der Fachinformation nicht aufgeführt und wurde daher als unbekannt eingestuft.

Basierend auf der aktuellen Information führt dieser Bericht nicht zu einer Änderung des Nutzen-Risiko-Verhältnisses. Zu diesem Zeitpunkt sind keine weiteren Massnahmen vorgesehen. Sobald Änderungen hierzu bekannt werden, werden wir darüber unverzüglich und unaufgefordert informieren.

Dr. Peter Gores

Reporter's comment:

NOVARTIS, 19.03.2008 DE-002147023-PHBS2008DE03039

Sender diagnosis: not reported

Narrative

Initial report was received from German Health Authority (08017209) via Nycomed 80493) on 07 Mar 2008. This elderly patient with status epilepticus, epilepsy, hypertension, COPD, renal insufficiency, acute renal failure, infection, hypokalaemia, pneumonia, cardiac insufficiency and bacteremia started treatment with Pantozol (pantoprazole) for ulcer prophylaxis on 24 Oct 2004 and diclofenac (manufacturer name unknown) on 01 Jan 2002 for pain valporate (manufacturer name unknown) on 23 Oct 2004 for epilepsy, vancomycin (manufacturer name unknown) on 29 Oct 2004 for rash, Synacthen (tetracosactide) for epilepsy. There are 45 other suspected drug added in the system and 11 other suspected drug are not coded in the system due to space problem: Cernevit (vitamins), abumin, Fenistil (dimethindene), blutproduct, metamizol, Gastrogratin (meglumine aminotrizonate, sodium aminotrizolate), Zienam (cistastatin sodium), Cordarex (amiodarone), Beloc (metoprolol), Mg-Mineralien (mineral supplement), Keppa (levetiracetam), Otriven (xylometazoline). On 22 Oct 2004 he was admitted to hospital due to status epilepticus. On 30 Nov 2007 Steven Johnson syndrome was diagnosed with the symptoms of fever, generalized skin spots, tongue erosion, nasal mucous membrane, and erosion of oral mucous membrane and generalized erythrodermia. Pantozol treatment was discontinued on 28 Oct 2004. The outcome of the event was reported as unchanged. The reporter suspected a causal relationship between the event and Pantozol and the other 59 drugs.

Sender's comment:

For Pantozol and diclofenac:

Serious spontaneous report, Stevens-Johnson syndrome [hospitalisation], already listed in the Core Data Sheet. All spontaneous reports are considered suspected for reporting purposes or in accordance with national regulatory requirements.

For Synacthen:

Serious spontaneous report, Stevens-Johnson syndrome [hospitalisation], assessed as unlisted according to the Core Data Sheet.

However other alternative causes [other drugs] provide a possible explanation for the reported adverse event. All spontaneous reports are considered suspected for reporting purposes or in accordance with national regulatory requirements.

Reporter's comment:

APHQWEB, 25.03.2008 DE-Nycomed GmbH-80493

Sender diagnosis: not reported

Narrative

This report was received from the German Documentation centre of severe skin reactions (ref no. 9160240) via the German Health authority (BfArM ref. no. DE-BFARM-08017209).

This is a serious case due to hospitalisation.

A elderly male patient experienced Stevens-Johnson syndrome while receiving pantoprazole (Pantozol i.v. and Pantozol unknown dose p.o.) for ulcer prophylaxis, urapidil (Ebrantil 250mg/h i.v.) for hypertension, theophylline for COPD and Trapanal for sedation. Further 61 suspected drugs were administered.

On 22-OCT-2004 patient had been admitted to hospital due to status epilepticus.

On 28-OCT-2004 patient developed fever.

On 30-OCT-2004 patient developed generalized skin spots.

On 03-NOV-2004 patient developed tongue erosions.

On 04-NOV-2004 patient developed erosions of nasal mucous membranes.

On 06-NOV-2004 patient experienced erosions of oral mucous membranes.

On 07-NOV-2004 patient experienced generalized erythrodermia.

The outcome was reported as still ongoing at the time of the report.

Concomitant diseases included status epilepticus, epilepsy, hypertension, COPD, renal insufficiency, hyperuricaemia, pain, acute renal failure, infection, hypokalaemia, nausea, pruritus, pneumonia, cardiac insufficiency, rhinitis and bacteraemia. The Intensive Monitoring Project assessed the causality with regard to the patient's multiple medication (using an algorithm with 8 different levels ranging from 1=definite to 8=improbable). Pantoprazole, urapidil, theophylline (Theophyllin 375mg) and thiopental were rated as possible due to this algorithm. Allopurinol and phenytoin were considered to be highly suspected drugs and the causal relation was rated as probable due to this algorithm.

STEVENS-JOHNSON-SYNDROME:

This patient developed Stevens-Johnson syndrome approximately eight days after first injection of pantoprazole and urapidil, four days after introduction of thiopental, five days after first intake of theophylline and six days after first intake of pantoprazole p.o. Concomitantly, the patient received multiple other drugs (approximately further 60 suspected drugs).

Stevens-Johnson syndrome is a rare skin disorder, characterized by mucosal erosions, skin blisters, often accompanied by sore throat, malaise and fever. It can be drug-induced or due to infectious diseases.

Several drugs have been observed to cause Stevens-Johnson syndrome (SJS).

Toxic bullous skin reactions up to acute disseminated epidermal necrolysis have also been observed in very rare cases with pantoprazole and other proton pump inhibitors. SJS is not expected for urapidil, theophylline and thiopental.

Temporal relationship plausible, time to onset reasonable for pantoprazole, urapidil, thiopental and theophylline. Mockenhaupt et al (2008) suggest, that Stevens-Johnson syndrome often begins more than four and up to 28 days after the initiation of the responsible medication.

Sender's comment:

The patient received multiple other suspected drugs (allopurinol and phenytoin had been rated as probable by the reporter's algorithm). In addition, the patient concomitantly suffered from an infection (bacteraemia and pneumonia) as well as from renal failure, which could be a contributing factor due to impaired drug metabolism.

However, although temporal relationship to pantoprazole, urapidil, thiopental and theophylline therapy is reasonable, it is hard to determine a single event as trigger for the reported event in this severely ill patient treated with various medications.

As a causal relationship cannot be fully ruled out, Nycomed conservatively assesses this case as possibly related to pantoprazole, urapidil, thiopental and theophylline.

This report is not considered to have any influence on current knowledge regarding the safety profile of pantoprazole, urapidil, thiopental and theophylline and has no effect on the status of these drugs as reflected by the formal product characteristics.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08009502

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	Yes	No	No	No	No		

Patient

Initials:	Sex: Male	Birth: privacy	Age: 30 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens-Johnson syndrome	20050212				not recovered/not resolved
2 Toxic epidermal necrolysis	20050212				not recovered/not resolved
3 Stevens-Johnson syndrome	20050212				not recovered/not resolved
4 Constipation	20050206				Unknown
5 Dyspnoea	20050130				Unknown
6 Sore throat	20050130				Unknown
7 Meteorism	20050204				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
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1	ARCOXIA etoricoxib	Oral	/Day 245	20050127-20050213 Shoulder pain 18 Day
2	BENURON paracetamol	Oral	/	20050202-20050202 Pain 1 Day
3	BENURON paracetamol	Oral	/	20050204-20050206 Pain 3 Day
4	BENURON paracetamol	Oral	/	20050208-20050208 Pain 1 Day
5	BENURON paracetamol	Oral	/	20050213-20050213 Pain -
6	BENURON paracetamol	Oral	/	20050119-20050122 Pain 4 Day
7	decortin prednisone	Intravenous (not otherwis /	120	20050121-20050209 Systemic lupus erythematosus 20 Day
8	decortin prednisone	Oral	100 Mg milligram(s)/ Day	20050210-20050213 SLE 4 Day
9	decortin prednisone	Intravenous (not otherwis /		20050121-20050209 SLE 20 Day
10	DECORTIN prednisone	Oral	100 Mg milligram(s)/Day 245	20050119-20050122 SLE 4 Day
11	DIPIDOLOR piritramide	Intravenous (not otherwis /	120	20050118- Pain -
12	Dormicum midazolam	Intravenous (not otherwis	5 Mg milligram(s)/Day 120	20050202-20050202 Sedation 1 Day
13	DULCOLAX bisacodyl	Oral	10 Mg milligram(s)/Day 242	20050207-20050207 Constipation 1 Day
14	ENDOXAN cyclophosphamide	Oral	150 Mg milligram(s)/Day 245	20050126-20050126 SLE -
15	ESPUMISAN simeticone	Oral	/	20050207-20050207 Meteorism 1 Day
16	ESPUMISAN simeticone	Oral	/	20050204-20050204 Meteorism 1 Day
17	FENISTIL RETARD dimetindene	Oral	4 Mg milligram(s)/Day 5	20050122-20050123 SLE 2 Day
18	FENISTIL RETARD dimetindene	Oral	4 Mg milligram(s)/Day 5	20050126-20050213 SLE 19 Day
19	FLUNITRAZEPAM flunitrazepam	Oral	/ Day	20050207-20050208 Sleep disorder 2 Day
20	FLUNITRAZEPAM flunitrazepam	Oral	/	20050205-20050205 Sleep disorder -
21	FLUNITRAZEPAM flunitrazepam	Oral	/ Day	20050201-20050202 Sleep disorder 2 Day
22	IDEOS calcium carbonate; colecalciferol	Oral	800 Mg milligram(s)/Day 245	20050127-20050213 Reaction unevaluable 18 Day
23	ISOCILLIN phenoxymethylpenicillin potassium	Oral	3 Df dosage form/Day 245	20050118-20050207 Infection prophylaxis 21 Day
24	magnesium verla magnesium hydrogen aspartate	Oral	12 Df dosage form/Day 245	20050201-20050209 Reaction unevaluable 9 Day
25	microklist sodium citrate; sodium lauryl sulfoacetate; sorbitol	Oral	/ 242	20050208-20050208 Constipation -
26	MOVICOL polyethylene glycol 3350; potassium chloride; sodium bicarbonate; sodium chloride	Oral	1 Df dosage form/Day 99	20070207-20070207 Constipation 1 Day
27	Natriumcitrat, Natrium laurylsulfoacetat, Sorbito sodium citrate; sodium lauryl sulfoacetate; sorbitol	Rectal	1 /Day 242	20050211-20050211 Constipation 1 Day
28	Natriumcitrat, Natrium laurylsulfoacetat, Sorbito sodium citrate; sodium lauryl sulfoacetate; sorbitol	Rectal	1 /Day	20050208-20050208

		242	1 Day	
29	NICOTINELL nicotine	Transdermal 1 Df dosage form/Day	20050205-20050213	Tobacco abuse
		250	9 Day	
30	NOVALGIN metamizole	Oral /	20050122-20050122	Pain
			1 Day	
31	NOVALGIN metamizole	Oral /	20050127-20050127	Pain
			1 Day	
32	NOVALGIN metamizole	Oral /	20050130-20050130	Pain
			1 Day	
33	NOVALGIN metamizole	Oral /	20050110-20050110	Pain
			1 Day	
34	OXIS formoterol	Respiratory (inhalation) 48 µg microgram(s)/Day	20050130-20050213	Dyspnoea
		113	15 Day	
35	PANTOZOL pantoprazole	Oral 40 Mg milligram(s)/Day	20050119-20050213	Prophylaxis
		245	26 Day	
36	PLANUM temazepam	Oral / Day	20050131-20050131	Sleep disorder
			-	
37	PLANUM temazepam	Oral / Day	20050209-20050209	Sleep disorder
			-	
38	PROPOFOL propofol	Intravenous (not otherwis 100 Mg milligram(s)/Day	20050202-20050202	Sedation
		120	1 Day	
39	ROHYPNOL flunitrazepam	Oral 1 Df dosage form/Day	20050205-20050205	Sleep disorder
		245	1 Day	
40	ROHYPNOL flunitrazepam	Oral 1 Df dosage form/Day	20050201-20050202	Sleep disorder
		245	2 Day	
41	ROHYPNOL flunitrazepam	Oral 1 Df dosage form/Day	20050207-20050208	Sleep disorder
		245	2 Day	
42	sterofundin electrolytes nos	Intravenous (not otherwis 500 ml millilitre(s)/	20050201-20050209	Reaction unevaluable
			9 Day	
43	UROMITEXAN mesna	Oral 400 Mg milligram(s)/Day	20050130-20050213	Prophylaxis
		245	15 Day	
44	verdaechtigtes Arzneimittel nnb	Oral /	20050130-20050131	Sore throat
			2 Day	
45	verdaechtigtes Arzneimittel nnb	Oral /	20050206-20050206	Constipation
			-	

Drugs (concomitant)

46	dormicum midazolam maleate	Oral	/	20050118-20050118	Premedication
47	Elektrolyte nnb, Natriumlaktat sodium lactate	Intravenous (not otherwis	/ 120	20050201-20050209	Electrolyte substitution therapy
48	IMUREK azathioprine	Oral	3*50 Mg milligram(s)/ Day	20050116-20050125	SLE
49	SEVREDOL morphine	Oral	/	20050213-20050213	Pain
50	STILNOX zolpidem	Oral	/	20050219-20050219	Sleep disorder
51	STILNOX zolpidem	Oral	/	20050124-20050124	Sleep disorder
52	STILNOX zolpidem	Oral	/	20050127-20050127	Sleep disorder
53	STILNOX zolpidem	Oral	/	20050121-20050121	Sleep disorder
54	TRAMAL tramadol	Oral	/ 158	20050122-20050122	Pain
55	tranxilium clorazepate dipotassium	Oral	/	20050118-20050118	Premedication
56	voltaren dispers diclofenac sodium	Oral	1 Df dosage form/ 245	20050218-20050219	Pain

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Hospitalisation			Yes	
Intermittent pyrexia			Yes	
Pain			Yes	
Sleep disorder			Yes	
Surgery			No	
Sleep disorder			Unknown	
Tobacco abuse			Yes	
Constipation			Unknown	
Meteorism			Unknown	
Shoulder pain			Unknown	
SLE			Unknown	
Systemic lupus erythematosus			Yes	

Primary source

Qualification: Physician observestudytype:
Literature:

Narratives (nur bei elektronischem Eingang)

009507513, 30.01.2008 DE-MERCK-0801DEU00101

Sender diagnosis: not reported

Narrative

Information has been received from a registry (documenting at least 95% of all serious skin reaction that occur in Germany) concerning a 30-year-old hospitalized male patient with pain, systemic lupus erythematosus (SLE), intermittent fever, tobacco abuse and sleep disorder and a history of surgery (not further specified; on 18-JAN-2007) who, on 27-JAN-2005, was placed on therapy with etoricoxib, tablet, 90-120 mg, once a day for the treatment of shoulder pain (duration not reported). Concomitant therapy included penicillin V potassium (ISOCILLIN), tablet, 1 tablet, three times a day for infection prophylaxis (therapy start on 18-JAN-2005), piritramide (DIPIDOLOR), for the treatment of pain (therapy start on 18-JAN-2005; duration not reported, dose unclear), acetaminophen (BENURON), for the treatment of pain (therapy start on 19-JAN-2005; dose unclear), prednisone (DECORTIN), injection (form), IV for the treatment of SLE (therapy start on 21-JAN-2005; dose unclear), pantoprazole sodium (PANTOZOL), tablet, 40 mg, once a day for gastric ulcer prophylaxis (therapy start on 19-JAN-2005; duration not reported), dipyrone (NOVALGIN), solution, 50 drops daily for the treatment of pain (therapy start on 22-JAN-2005), dimethindene maleate (FENISTIL), capsule, 4 mg, once a day for the treatment of SLE (therapy start on 22-JAN-2005), cyclophosphamide (ENDOXAN), tablet, 50 mg, three times a day for the treatment of SLE (therapy start on 22-JAN-

2005; duration not reported). Other concomitant therapy included prednisone (DECORTIN), calcium carbonate (+) cholecalciferol (IDEOS), magnesium aspartate (+) magnesium citrate (+) magnesium glutamate (+) magnesium sulfate (MAGNESIUM VERLA) and electrolytes (unspecified) (+) sodium lactate.

On 22-JAN-2005, therapy with acetaminophen and dipyrone was stopped. On 23-JAN-2005, therapy with dimethindene maleate was stopped. On 26-JAN-2005, therapy with dimethindene maleate was restarted. On 27-JAN-2005, therapy with dipyrone was restarted for one day. On 30-JAN-2005, the patient was placed on therapy with mesna (UROMITEXAN), tablet, 400 mg, once a day for bladder protection (duration not reported) and formoterol fumarate (OXIS), inhalant, 24 mcg, twice a day for the treatment of dyspnea (duration not reported). That same day, therapy with dipyrone was restarted for one day. On 01-FEB-2005, the patient was placed on therapy with flunitrazepam (ROHYPNOL), tablet, 1 tablet, once a day for the treatment of sleep disorder. On 02-FEB-2005, he was placed on therapy with midazolam (DORMICUM), injection (form), IV, 5 mg, once a day as sedative therapy for one day and propofol, injection (form), IV, 100 mg, once a day as sedative therapy for one day. That same day, therapy with acetaminophen was restarted for one day and therapy with flunitrazepam was stopped. On 04-FEB-2005, the patient was placed on therapy with simethicone (ESPUMISAN), solution for the treatment of meteorism for one day (dose not reported). That same day, therapy with acetaminophen was restarted. On 05-FEB-2005, the patient was placed on therapy with nicotine (NICOTINELL), TDS, 1 application, once a day for the treatment of tobacco abuse (duration not reported). That same day, therapy with flunitrazepam was restarted for one day. On 06-FEB-2005, the patient was placed on therapy with unspecified laxative, solution, 10 drops daily for the treatment of constipation for one day. That same day, therapy with acetaminophen was stopped. On 07-FEB-2005, the patient was placed on therapy with bisacodyl (DULCOLAX), suppository, 10 mg, once a day, for the treatment of constipation for one day and polyethylene glycol 3350/KCl/NaHCO3/NaCl (MOVICOL), granules, 1 dose, once a day for the treatment of constipation for one day. That same day, therapy with penicillin V potassium was stopped and therapy with simethicon and flunitrazepam was restarted for one day. On 08-FEB-2005, the patient was placed on therapy with sodium citrate (+) sodium lauryl sulfoacetate (+) sorbitol, suppository, 1 application, once a day for the treatment of constipation for one day. That same day, therapy with flunitrazepam was stopped. On 09-FEB-2005, therapy with prednisone IV was stopped. On 10-FEB-2005, therapy with dipyrone was restarted for one day. On 11-FEB-2005, therapy with sodium citrate (+) sodium lauryl sulfoacetate (+) sorbitol was restarted for one day.

On 12-FEB-2005, the patient developed fever (until 15-FEB-2005), pruritus and pain of skin. That same day, therapy with acetaminophen was restarted. On 13-FEB-2005, the patient developed blisters on his skin and reddening of his genital mucosa. On 14-FEB-2005, he developed erosions of his oral mucosa. On 16-FEB-2005, he developed conjunctivitis and Nikolski signs were positive. On 17-FEB-2005, he developed erosions of his lips. He was diagnosed with transitional form of Stevens-Johnson syndrome/TEN. The patient's transitional form of Stevens-Johnson syndrome/TEN persisted (also reported as "duration: 15-MAR-2005"). Action taken with regard to etoricoxib was not reported.

The reporting physician felt that transitional form of Stevens-Johnson syndrome/TEN was possibly related to therapy with etoricoxib, pritramide, acetaminophen, pantoprazole sodium, prednisone IV, dipyrone, dimethindene maleate, cyclophosphamide, mesna, formoterol fumarate, flunitrazepam, midazolam, propofol, simethicone, nicotine, unspecified laxative, bisacodyl, polyethylene glycol 3350/KCl/NaHCO3/NaCl and sodium citrate (+) sodium lauryl sulfoacetate (+) sorbitol and probably related to penicillin V potassium. He also considered the patient's systemic lupus erythematosus as a possible cause.

Transitional form of Stevens-Johnson syndrome/TEN was considered to be immediately life-threatening.

Information received on 22-JAN-2008 contained the following adverse experiences: sore throat (30-JAN-2005), dyspnea (30-JAN-2005), meteorism (04-FEB-2005), constipation (06-FEB-2005).

Additional information is not expected.

Sender's comment:

Stevens-Johnson syndrome (SJS) is a serious labeled event for therapy with ARCOXIA°. This patient with underlying systemic lupus erythematosus (SLE) developed a severe skin reaction diagnosed as transitional form Stevens-Johnson syndrome/toxic epidermal necrolysis (TEN) after taking ARCOXIA° for 17 days. The causality evaluation of the skin reaction is confounded by several factors. Firstly the underlying autoimmune disease (SLE) is associated with a higher risk for SJS or TEN. Secondly, many concomitant medications were administered and one of them, methylpenicillin, was considered as primary suspect for the triggering of the skin reaction. Considering these facts the contribution of ARCOXIA° to the skin reaction cannot be assessed. Severe skin reactions including SJS and TEN are adequately described in the current labeling. Self initiated measures are not indicated.

Reporter's comment:

ROCHE, 13.02.2008 DE-ROCHE-545533

Sender diagnosis: not reported

Narrative**INTENSIVE MONITORING**

THIS CASE WAS REPORTED BY A PHYSICIAN AND CONCERNS A 33-YEAR-OLD MALE PATIENT WHO EXPERIENCED LIFE-THREATENING STEVENS-JOHNSON-SYNDROME AND LIFE-THREATENING TOXIC EPIDERMAL NECROLYSIS WHILE RECEIVING FLUNITRAZEPAM (ROHYPNOL) FOR SLEEPING DISORDER AND MIDAZOLAM (DORMICUM) FOR SEDATION, PHENOXYMETHYLPENICILLIN POTASSIUM (ISOCILLIN) FOR PROPHYLAXIS OF INFECTION, PIRITRAMIDE (DIPIDOLOR) FOR PAIN, PARACETAMOL (BENURON) FOR PAIN, PREDNISONE (DECORTIN) FOR SYSTEMIC LUPUS ERYTHROMATOSUS, PANTOPRAZOLE (PANTOZOL) FOR PROPHYLAXIS OF ULCER, PREDNISONE (DECORTIN INJ) FOR SYSTEMIC LUPUS ERYTHROMATOSUS, METAMIZOLE SODIUM (NOVALGIN TRPOFEN) FOR PAIN, DIMETINDENE (FENISTIL RETARD) FOR SYSTEMIC LUPUS ERYTHROMATOSUS, CYCLOPHOSPHAMIDE (ENDOXAN) FOR SYSTEMIC LUPUS ERYTHROMATOSUS, CALCIUM CARBONATE/COLECALCIFEROL (IDEOS) AS SUBSTITUTION THERAPY, ETORICOXIB (ARCOXIA) FOR SHOULDER PAIN, MESNA (UROMITEXAN) FOR BLADDER PROTECTION, PRIMULA ROOT (IPALAT) FOR SORE THROAT, FORMOTEROL (OXIS) FOR DYSPTNOEA, MAGNESIUM-HYDROGENASPARTATE (MAGNESIUM VERLA) AS SUBSTITUTION THERAPY, TEMAZEPAM (PLANUM) FOR SLEEPING DISORDER, ELECTROLYTES (STEROFUNDIN) FOR SUBSTITUTION, PROPOFOL (PROPOFOL) FOR SEDATION, SIMETHICON (ESPUMISAN) FOR METEORISM, NICOTIN (NICOTINELL) FOR NICOTIN ABUSUS, UNBEK/LAXANS (LAXOS TROPOFEN) FOR CONSTIPATION, BISACODYL (DULCOLAX) FOR CONSTIPATION, ELECTROLYTE/MACROGOL (MOVICOL) FOR CONSTIPATION AND SODIUM CITRATE/SODIUM LAURYL SULFATE/SORBITOL (MICROKLIST) FOR CONSTIPATION.

THE PATIENT HAD A HISTORY OF SYSTEMIC LUPUS ERYTHROMATOSUS. THE PATIENT'S CONCOMITANT MEDICATIONS INCLUDED DICLOFENAC (VOLTAREN DISPERS).

ON APPROXIMATELY 18 JANUARY 2005, THE PATIENT WAS HOSPITALISED (DEPARTMENT OF OTOLARYNGOLOGY) AND UNDERWENT SURGERY. THE SAME DAY, HE COMMENCED ORAL PHENOXYMETHYLPENICILLIN POTASSIUM (3X1, 2M) AND INTRAVENOUS PIRITRAMIDE "7.5 MG 15 MG 45 MG". THE FOLLOWING DAY, HE COMMENCED ORAL PARACETAMOL "1X1G 2X1G 3X1G", ORAL PREDNISONE 100 MG DAILY (GIVEN IN THE MORNING) AND PANTOPRAZOLE 40 MG DAILY (GIVEN IN THE MORNING). ON 21 JANUARY 2005, HE RECEIVED INTRAVENOUS PREDNISOLONE "130 MG 100MG 1-0-0 3/2-0-0". THE FOLLOWING DAY, HE RECEIVED ORAL METAMIZOLE SODIUM (1X20 GTT 1X30 GTT) ONCE AND ORAL DYPYRONE ONCE (GIVEN IN THE EVENING) WHICH WAS DISCONTINUED THE NEXT DAY. ON 25 JANUARY 2005, THE PATIENT WAS TRANSFERRED TO DEPARTMENT OF INTERNAL MEDICINES. ON 26 JANUARY 2005, THE PATIENT RE-COMMENCED ORAL DIMETINDENE ONCE DAILY (GIVEN IN THE EVENING) AND RECEIVED ORAL CYCLOPHOSPHAMIDE 50 MG THRICE DAILY (GIVEN IN THE MORNING, AFTERNOON AND EVENING). ON 27 JANUARY 2005, HE RECEIVED ORAL METAMIZOLE SODIUM ONCE, ORAL CALCIUM CARBONATE/COLECALCIFEROL 400 MG TWICE DAILY AND ORAL ETORICOXIB (120 MG 90 MG) ONCE DAILY (GIVEN IN THE MORNING). IT WAS REPORTED THAT THE PATIENT EXPERIENCED INTERMITTENT FEVER FROM 27 JANUARY 2007 UNTIL 9 FEBRUARY 2007. ON 30 JANUARY 2007, HE AGAIN RECEIVED ORAL METAMIZOL, ORAL MESNA, AND ORAL PRIMULA ROOT AND INHALATION FORMOTEROL. THE FOLLOWING DAY, PRIMULA ROOT WAS DISCONTINUED AND THE PATIENT RECEIVED ORAL MAGNESIUM-HYDROGENASPARTATE THRICE DAILY (TWO IN THE MORNING, TWO IN THE AFTERNOON AND TWO IN THE EVENING) AND ORAL TEMAZEPAM ONCE (GIVEN AT NIGHT). ON 1 FEBRUARY 2005, THE PATIENT COMMENCED ORAL FLUNITRAZEPAM ONCE DAILY (GIVEN IN THE NIGHT) AND INTRAVENOUS ELECTROLYTES (500 ML). THE FOLLOWING DAY, FLUNITRAZEPAM WAS DISCONTINUED AND HE RECEIVED PARACETAMOL ONCE, INTRAVENOUS MIDAZOLAM 5 MG (GIVEN IN THE MORNING) ONCE AND INTRAVENOUS PROPOFOL 100 MG ONCE (GIVEN IN THE MORNING). ON 4 FEBRUARY 2005, THE PATIENT COMMENCED ORAL PARACETAMOL AND ORAL SIMETHICON ONCE. THE FOLLOWING DAY, THE PATIENT RECEIVED ORAL FLUNITRAZEPAM ONCE AND TRANSDERMAL NICOTIN PATCHES ONCE DAILY (IN THE MORNING). ON 6 FEBRUARY 2005, PARACETAMOL WAS DISCONTINUED AND ORAL LAXANS WAS STARTED. THE FOLLOWING DAY, PHENOXYMETHYLPENICILLIN WAS DISCONTINUED AND ORAL FLUNITRAZEPAM, ORAL SIMETHICON, ORAL BISACODYL AND ORAL MACROGOL WAS STARTED. ON 8 FEBRUARY 2005, FLUNITRAZEPAM WAS DISCONTINUED AND CITRATE/SODIUM LAURYL SULFATE/SORBITOL WAS STARTED. THE FOLLOWING DAY, INTRAVENOUS PREDNISOLONE AND ELECTROLYTE WERE DISCONTINUED. ON 10 FEBRUARY 2005, HE RECOMMENCED ORAL PREDNISOLONE AND METAMIZOLE SODIUM ONCE. THE FOLLOWING DAY, THE PATIENT AGAIN RECEIVED NATRIUM CITRATE/SODIUM LAURYL SULFATE/SORBITOL AND HE DEVELOPED FEVER, PRURITUS AND SKIN PAIN (COMANIFESTATION). THE FOLLOWING DAY, HE DEVELOPED SKIN BLISTERS AND REDDENING OF GENITAL MUCOSA. HE WAS DIAGNOSED WITH STEVENS-JOHNSON-SYNDROME AND TOXIC EPIDERMAL NECROLYSIS. THE SAME DAY, ALL THE DRUGS WERE DISCONTINUED. ON 14 FEBRUARY 2005, HE DEVELOPED ORAL MUCOSA EROSIONS. HIS FEVER RESOLVED ON 15 JANUARY 2005. THE FOLLOWING DAY, HE HAD CONJUKTIVITIS. ON 17 FEBRUARY 2007, HE ALSO EXPERIENCED LIP EROSIONS AND SHIFTED TO CENTER OF BURN INJURY. AT THE TIME OF THIS REPORT, THE EVENTS OF STEVENS-JOHNSONS-SYNDROME AND TOXIC EPIDERMAL NECROLYSIS PERSISTED. THE PHYSICIAN ASSESSED THE DRUG-EVENT RELATIONSHIP FOR THE EVENTS AS POSSIBLY RELATED TO FLUNITRAZEPAM, MIDAZOLAM, PARACETAMOL, PANTOPRAZOLE, PREDNISONE, METAMIZOLE SODIUM, DYPYRONE, CYCLOPHOSPHAMIDE, ETORICOXIB, MESNA, FORMOTEROL, PROPOFOL, SIMETHICON, LAXANS, BISACODYL, ELECTROLYTE/MACROGOL, CITRATE/SODIUM LAURYL SULFATE/SORBITOL AND UNDERLYING DISEASE OF SYSTEMIC LUPUS ERYTHROMATOSUS. THE INVESTIGATOR ASSESSED THE DRUG-EVENT RELATIONSHIP FOR THE EVENT AS VERY WELL RELATED TO PHENOXYMETHYLPENICILLIN POTASSIUM. THE INVESTIGATOR ASSESSED THE DRUG-EVENT RELATIONSHIP FOR THE EVENTS AND ELECTROLYTES, PRIMULA ROOT, CALCIUM CARBONATE/COLECALCIFEROL AND PREDNISONE AS QUESTIONABLE.

NO FURTHER INFORMATION WAS AVAILABLE.

Sender's comment:

CIOMS TEXT: STEVENS JOHNSON SYNDROME AND TOXIC EPIDERMAL NECROLYSIS ARE NOT LISTED IN THE COMPANY REFERENCE

DOCUMENT (CDS) FOR FLUNITRAZEPAM AND MIDAZOLAM. BASED ON THE INFORMATION RECEIVED FOR THIS CASE, A CAUSAL RELATIONSHIP BETWEEN THE REPORTED EVENTS AND TREATMENT WITH FLUNITRAZEPAM AND MIDAZOLAM IS ASSESSED AS POSSIBLE. COMPANY CAUSALITY WAS ASSESSED BASED ON THE NARANJO ALGORITHM. HOWEVER ALTERNATIVE EXPLANATIONS INCLUDE PHENOXYMETHYLPENICILLIN AND DICLOFENAC (VOLTAREN DISPERS).; ***LOCAL ASSESSMENT: STEVENS JOHNSON SYNDROM UND EPIDERMAL NECROLYSE SIND NICHT IN DER FACHINFORMATION FÜR ROHYPNOL AUFGEFÜHRT.
ZUSAMMENHANG MÖGLICH
ALTERNATIVE URSACHE KÖNNTE PENICILLIN, DICLOFENAC, DORMICUM, ISOCILLIN, DIPIDOLOR ODER BEURON SEIN. MELDUNG DES DOKUMENTATIONSZENTRUMS FÜR SCHWERE HAUTKRANKHEITEN MAJA MOCKENHAUPT
EINZELFALLBEOBACHTUNG = KEIN HANDLUNGSBEDARF

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08013929

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	Yes	No	No	No	No		Yes

Patient

Initials:	Sex: Female	Birth: privacy	Age: 36 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Toxic epidermal necrolysis	20050317	20050414	29 Day	23 Day	not recovered/not resolved
2 Bronchopulmonary infection					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
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1	ACC acetylcysteine	Intravenous (not otherwis 300 Mg milligram(s)/	20050222-	Mechanical ventilation
2	Ambroxol ambroxol	Intravenous (not otherwis 60 Mg milligram(s)/Day	20050222-	Mechanical ventilation
3	Beloc-Zok metoprolol succinate	Intravenous (not otherwis 15 Mg milligram(s)/Day	20050310-20050311	Hypertension
4	Beloc-Zok mite metoprolol succinate	Oral / 245	-	Hypertension
5	Cefuroxim cefuroxime	4.5 G gram(s)/Day	20050225-20050225	Infection
6	Clonidin clonidine	Intravenous (not otherwis 0.25 Mg milligram(s)/ 120	20050310-20050310	Hypertension
7	Disoprivan propofol	Intravenous (not otherwis /	20050223-20050310	Sedation
8	Dormicum midazolam	Intravenous (not otherwis 90 Mg milligram(s)/	20050222-20050310	Sedation
9	Dormicum midazolam	90 Mg milligram(s)/	20050317-20050317	Sedation
10	fenistil dimetindene maleate	4 Mg milligram(s)/	20050304-20080312	Exanthema
11	fenistil dimetindene maleate	8 Mg milligram(s)/Day	-	Exanthema
12	Fentanyl fentanyl	Intravenous (not otherwis /	20050222-20050310	Sedation
13	gentamicin gentamicin	Intravenous (not otherwis 400 Mg milligram(s)/	20050304-20050306	Prophylaxis
14	HAES STERIL 10% hetastarch; sodium chloride	Intravenous (not otherwis 125 ml millilitre(s)/ 230	20050222-20050308	Volume depletion
15	HYDROCORTISONE hydrocortisone	Intravenous (not otherwis 300 Mg milligram(s)/	20050305-20050310	Exanthema
16	Hydrokortison hydrocortisone	Intravenous (not otherwis 100 Mg milligram(s)/	20050305-20050310	Exanthema
17	Laxoberal sodium picosulfate	20 Gtt drop(s)/ 158	20050303-20050908	Obstipation
18	LIPOVENOES 10% glycerol; lecithin; soya oil	Intravenous (not otherwis 500 ml millilitre(s)/ 61	20050310-20050310	Parenteral nutrition
19	MANNITOL mannitol	Intravenous (not otherwis 125 ml millilitre(s)/	20050222-20050311	Prophylaxis
20	mono-embolex certoparin sodium	Subcutaneous /	20050223-	Thrombosis prophylaxis
21	MOVICOL macrogol; potassium chloride; sodium bicarbonate; sodium chloride	Oral 1 Df dosage form/ 59	20050303-	Obstipation
22	Noradrenalin norepinephrine	5 Mg milligram(s)/	20050226-20050226	Hypovolaemia
23	Noradrenalin norepinephrine	5 Mg milligram(s)/	20050228-20050228	Hypovolaemia
24	Novaminsulfon metamizole	2.5 G gram(s)/	20050305-20050305	Pain
25	Novaminsulfon metamizole	1 G gram(s)/	20050228-20050228	Pain
26	Novaminsulfon metamizole	2.5 G gram(s)/	20050302-20050303	Pain
27	Novaminsulfon metamizole	Oral 5 G gram(s)/ 245	20050304-20050304	Pain
28	Pantozol pantoprazole sodium	Intravenous (not otherwis /	20050222-	

		194	-	
29	paracefan clonidine hydrochloride	Intravenous (not otherwis 1.5 Mg milligram(s)/	20050311-20050314	Hypertension
			4 Day	
30	Perfalgan paracetamol	1 G gram(s)/	20050310-20050312	Pain
			3 Day	
31	Perfalgan paracetamol	1 G gram(s)/	20050315-20050315	Pain
			1 Day	
32	Ranitidin ranitidine	Intravenous (not otherwis /	-	Exanthema
			-	
33	ringerloesung calcium chloride dihydrate; potassium chloride; sodium chloride	Intravenous (not otherwis 1000 ml millilitre(s)/	20050222-20050227	
			6 Day	
34	TAZOBAC piperacillin sodium; tazobactam sodium	Intravenous (not otherwis 13.5 Mg milligram(s)/Day	20050228-20050306	Infection
			7 Day	
35	Torem torasemide	Intravenous (not otherwis /	20050224-	Oedema
			-	
Drugs (concomitant)				
36	Dolantin pethidine hydrochloride	Intravenous (not otherwis 25 Mg milligram(s)/	20050304-20050304	Pain
			1 Day	
37	Espumisam simeticone	Oral 20 ml millilitre(s)/	20050312-20050314	Meteorism
			3 Day	
38	Glucose 10% glucose	Intravenous (not otherwis 500 ml millilitre(s)/	20050304-20050307	Parenteral nutrition
			4 Day	
39	Intrafusin 15% amino acids nos	Intravenous (not otherwis 500 ml millilitre(s)/	20050305-20050307	Electrolyte substitution therapy
			3 Day	
40	Jonosteril calcium acetate; magnesium acetate; potassium acetate; sodium acetate; sodium chloride	Intravenous (not otherwis 1000 ml millilitre(s)/	20050228-20050313	Electrolyte substitution therapy
			14 Day	
41	NaCL sodium chloride	Intravenous (not otherwis 500 ml millilitre(s)/	20050309-20050309	Electrolyte substitution therapy
			1 Day	
42	Prednisolone prednisolone	Intravenous (not otherwis 500 ml millilitre(s)/	20050305-20050307	Exanthema
			3 Day	
43	Propofol propofol	1 G gram(s)/	20050317-20050317	Anesthesia
			1 Day	
44	sufentanil sufentanil	Intravenous (not otherwis 1 Mg milligram(s)/	20050317-20050317	Anesthesia
			1 Day	
45	Tracutil minerals nos	Intravenous (not otherwis /Day 120	20050310-20050313	Therapeutic procedure
			4 Day	
46	Trapanalintrav thiopental sodium	Intravenous (not otherwis 1.5 G gram(s)/	20050226-20050304	Narcosis
			7 Day	
47	Urbason methylprednisolone	Intravenous (not otherwis /	20050317-20050317	Exanthema
			1 Day	
48	Voluven hetastarch	Intravenous (not otherwis 500 ml millilitre(s)/	20050304-20050304	Electrolyte substitution therapy
			1 Day	
49	Zienam cilastatin sodium; imipenem	Intravenous (not otherwis 3 G gram(s)/Day	20050313-20050316	Bronchial infection
			4 Day	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Oedema			Unknown	
Exanthema	20050306	20050310	No	makulopapuloeses exanthem
Hypertension			Unknown	
Hospitalisation			Yes	
Flatulence			Yes	
Fall			Yes	
Narcosis			Unknown	
Bronchial infection			Unknown	
Anaesthesia			Unknown	
Mechanical ventilation			Yes	
Bronchopulmonary infection			Yes	
Multiple fractures	20050222		Yes	
Brain edema			Yes	
Pain			Unknown	
Obstipation			Unknown	
Meteorism			Unknown	
Infection			Unknown	
Hypovolaemia			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:		
Qualification:	Other Health Professional	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

ROCHE, 13.02.2008 DE-ROCHE-545347

Sender diagnosis: not reported

Narrative**INTENSIVE MONITORING STUDY**

THIS CASE WAS REPORTED BY A PHYSICIAN AND CONCERNS A 36-YEAR-OLD FEMALE PATIENT WHO EXPERIENCED TOXIC EPIDERMAL NECROLYSIS WHILST RECEIVING HETASTARCH (HAES); CALCIUM ACETATE/MAGNESIUM ACETATE/POTASSIUM ACETATE/SODIUM ACETATE (JONOSTERIL), HETASTARCH (VOLUVEN), AMINO ACIDS (INTRAFUSIN) AND SODIUM CHLORIDE (NACL) ALL TAKEN FOR VOLUME SUBSTITUTION; FENTANYL, PROPOFOL (DISOPRIVAN) AND MIDAZOLAM (DORMICUM) FOR SEDATION; MANNITOL, ACETYLCYSTEINE (ACC), AMBROXOL ALL THREE TAKEN FOR PROPHYLAXIS FOR CEREBRAL EDEMA DUE TO VENTILATION; PANTOPRAZOLE (PANTOZOL) FOR GASTRIC PROTECTION; CERTOPARIN SODIUM (MONO-EMBOLEX) FOR THROMBOSIS PROPHYLAXIS; TORASEMID (TOREM) TAKEN FOR EDEMA; THIOPENTAL SODIUM (TRAPANAL) AS ANESTHESIA; NOREPINEPHRINE (NORADRENALIN) FOR HYPOVOLAEMIA; METAMIZOLE SODIUM (NOVAMINSULFON), PETHIDINE (DOLANTIN) AND PARACETAMOL (PERFALGAN) FOR PAIN; PIPERACILLIN/TAZOBACTAM (TAZOBAC) FOR SUSPECTED INFECTION; SODIUM PICOSULFATE (LAXOBERAL) AND ELECTROLYTES/MACROGOL (MOVICOL) BOTH TAKEN FOR OBSTIPATION; GLUCOSE AND GLYCINE MAX (LIPOVENOES) AS PARENTERAL FEEDING; GENTAMICIN FOR PROPHYLAXIS AGAINST INFECTION; CROMOGLICIC ACID/DIMETINDENE/HYDROCORTISONE/PENCICLOVIR/TRIPELENNAMINE HYDROCHLORIDE (FENISTIL), HYDROCORTISONE, RANITIDINE AND PREDNISOLON FOR EXANTHEMA; CLONIDIN AND METOPROLOL (BELOC-ZOK), CLONIDIN (PARACEFAN) FOR HYPERTENSION; MINERALS (TRACUTIL) FOR MINERAL SUBSTITUTION; SIMETICONE (ESPUMISAN) FOR METEORISM; CILASTATIN/IMIPENEM (ZIENAM) FOR BRONCHIAL INFECTION.

ON 22 FEBRUARY 2005, THE PATIENT HAD MULTIPLE FRACTURES DUE TO FALL OF STAIRCASE. THE SAME DAY, THE PATIENT WAS HOSPITALIZED AND SHE COMMENCED INTRAVENOUS HETASTARCH 1X125 ML, 4X125ML; INTRAVENOUS FENTANYL 1X1, INTRAVENOUS MANNITOL TWICE 125 ML AND FOUR TIMES 125ML, INTRAVENOUS ACETYLCYSTEINE THRICE 300MG AND TWICE 300MG, INTRAVENOUS AMBROXOL TWICE 30MG, INTRAVENOUS MIDAZOLAM ONCE 90 MG, INTRAVENOUS PANTOPRAZOLE 1X1. THE NEXT DAY, SHE COMMENCED INTRAVENOUS PROPOFOL 1X AND SUBCUTANEOUS CERTOPARIN SODIUM 1X. THE FOLLOWING DAY, SHE STARTED INTRAVENOUS TORASEMID ONCE 5MG, TWICE 5MG, THRICE 5MG, THRICE 10MG AND TWICE 10MG. ON 26 FEBRUARY 2005, THE PATIENT STARTED THIOPENTAL 1.5 GRAM AND RECEIVED NOREPINEPHRINE 1X5 MG. TWO DAYS LATER, THE PATIENT STARTED

INTRAVENOUS NOREPINEPHRIN ONCE 5MG AND RECEIVED METAMIZOLE SODIUM 1X1G, 1X2.5G, 1X2.5G, PIPERACILLIN/TAZOBACTAM THRICE 4.5 MG AND CALCIUM ACETATE/MAGNESIUM ACETATE/POTASSIUM ACETATE/SODIUM ACETATE ONCE 1000 ML. ON 2 MARCH 2005, THE PATIENT RECEIVED METAMIZOLE SODIUM ONCE 5 GRAM. THE NEXT DAY, THE PATIENT STARTED SODIUM PICOSULFATE 20 TR AND MACROGOL THRICE ONE DOSE, ONCE ONE DOSE. ON 4 MARCH 2005, SHE EXPERIENCED MACULOPAPULOSSES EXANTHEMA. THE SAME DAY, PATIENT RECEIVED METAMIZOLE SODIUM, HETASTARCH 1500ML AND PETHIDINE 1X25 MG, THIOPENTAL WAS DISCONTINUED AND THE PATIENT STARTED INTRAVENOUS GLUCOSE ONCE 500ML, GENTAMICIN ONCE 400MG AND CROMOGLICIC ACID/DIMETINDENE/HYDROCORTISONE/PENCICLOVIR/TRIPELENNAMINE HYDROCHLORIDE ONCE 4 MG AND TWICE 4 MG. THE PATIENT ALSO RECEIVED PREDNISOLONE THRICE 100 MG, ONCE 50 MG AND ONCE 100MG. ON 5 MARCH 2005, THE PATIENT RECEIVED AMINO ACIDS (INTRAFUSIN) ONCE 500ML AND INTRAVENOUS HYDROCORTISONE 100 MG AND 300 MG. ON 6 MARCH 2005, MACULOPAPULOSSES EXANTHEMA IMPROVED AND PIPERACILLIN/TAZOBACTAM, AMINO ACID AND GENTAMICIN WERE DISCONTINUED. ON 7 MARCH 2005, THE GLUCOSE WAS DISCONTINUED. ON 8 MARCH 2005, HETASTARCH AND SODIUMPICOSULFATE WAS DISCONTINUED. ON 9 MARCH 2005, THE PATIENT ALSO RECEIVED INTRAVENOUS SODIUM CHLORIDE ONCE 500 ML AND NEXT DAY RECEIVED CLONIDIN ONCE 1/4 AMP, GLYCINE MAX ONCE 500 ML. ON 10 MARCH 2005, MACULOPAPULOUS EXANTHEMA RESOLVED. THE SAME DAY, FANTANYL, MIDAZOLAM, PROPOFOL, NOREPINEPHRIN AND HYDROCORTISON WERE DISCONTINUED AND THE PATIENT RECEIVED METOPROLOL THRICE 5 MG, MINERALS ONCE ONE AMPULE AND PARACETAMOL ONCE 1 GRAM AND THRICE 1 GRAM. THE NEXT DAY, THE PATIENT STARTED CLONIDIN ONCE 1.5 MG. THE FOLLOWING DAY, PARACETAMOL WAS DISCONTINUED AND SHE STARTED ORAL SIMETICONE ONCE 20 ML. ON 13 MARCH 2005, MINERALS WERE DISCONTINUED AND THE PATIENT RECEIVED INTRAVENOUS CILASTATIN/IMIPENEM THRICE 1 GRAM AND ORAL METOPROLOL TWICE ONE DOSE AND GENTAMICIN FOR TWO DAYS. THE NEXT DAY, CLONIDIN AND SIMETICONE WERE DISCONTINUED. ON 15 MARCH 2005, THE PATIENT STARTED INTRAVENOUS RANITIDINE TWICE 1 AMPULE, ONCE 1 AMPULE. ON 16 MARCH 2005, THE PATIENT EXPERIENCED LIFE THREATENING TOXIC EPIDERMAL NECROLYSIS WITH MACULAE (COMANIFESTATION) AND RECEIVED PARACETAMOL. SHE EXPERIENCED HEBRA'S DISEASE AND INOCULATION. THE SAME DAY, CILASTATIN/IMIPENEM WERE DISCONTINUED. THE NEXT DAY, SHE EXPERIENCED BLADDERS OF SKIN. THE SAME DAY, ACETYLCYSTEIN, AMBROXOL, PANTOPRAZOL, CERTOPARIN SODIUM, TORASEMID, MACROGOL, CROMOGLICIC ACID/DIMETINDENE/HYDROCORTISONE/PENCICLOVIR/TRIPELENNAMINE HYDROCHLORIDE, METOPROLOL AND RANITIDINE WERE DISCONTINUED. ON 17 MARCH 2005 THE PATIENT ALSO RECEIVED TREATMENT WITH MIDAZOLAM, PREDNISOLONE, HYDROCORTISON. ON 19 MARCH 2005, SHE WAS NIKOLSKI POSITIVE. ON 14 APRIL 2005, THE EVENT IMPROVED AND THE PATIENT WAS DISCHARGED.

AT THE TIME OF THIS REPORT, THE EVENT WAS PERSISTING.

THE INVESTIGATOR ASSESSED THE EVENT AS POSSIBLY RELATED TO HETASTARCH, FENTANYL, MANNITOL, ACETYLCYSTEINE, AMBROXOL, MIDAZOLAM, PANTOPRAZOLE, PROPOFOL, CERTOPARIN SODIUM, TORASEMIDE, NOREPINEPHRIN, METAMIZOLE SODIUM, PIPERACILLIN/TAZOBACTAM, SODIUMPICOSULFATE, MACROGOL, METAMIZOL, CROMOGLICIC ACID/DIMETINDENE/HYDROCORTISONE/PENCICLOVIR/TRIPELENNAMINE HYDROCHLORIDE, GENTAMICIN, HYDROCORTISONE, CLONIDIN, GLYCINE MAX, METOPROLOL, PARACETAMOL AND QUESTIONABLY RELATED TO THIOPENTAL SODIUM, PETHIDINE, CALCIUM ACETATE/MAGNESIUM ACETATE/POTASSIUM ACETATE/SODIUM ACETATE, PREDNISOLONE, AMINO ACIDS, MINERALS, SIMETICONE, CILASTATIN/IMIPENEM, METOPROLOL, RANITIDINE, HETASTARCH, SODIUM CHLORIDE AND PETHIDIN GLUCOSE. SUSPECTED TRIGGER OF EVENT WERE REPORTED AS CEFUROXIM, METAMIZOL SODIUM, PIPERACILLIN/TAZOBACTAM, GENTAMICIN.

NO FURTHER INFORMATION WAS AVAILABLE.

Sender's comment:

CIOMS TEXT: TOXIC EPIDERMAL NECROLYSIS IS NOT LISTED IN THE COMPANY REFERENCE DOCUMENT (CDS) FOR MIDAZOLAM AND TORASEMIDE. BASED ON THE INFORMATION RECEIVED FOR THIS CASE, A CAUSAL RELATIONSHIP BETWEEN THE REPORTED EVENT AND TREATMENT WITH MIDAZOLAM AND TORASEMIDE IS ASSESSED AS POSSIBLE. COMPANY CAUSALITY WAS ASSESSED BASED ON THE NARANJO ALGORITHM. HOWEVER POSSIBLE ALTERNATIVE EXPLANATIONS INCLUDE PANTOPRAZOL, CEFUROXIME, PIPERACILLIN/TAZOBACTAM, RANITIDIN.; ***LOCAL ASSESSMENT: INTENSIVE MONITORING

TOXIC EPIDERMAL NECROLYSIS = NOT LABELED FOR MIDAZOLAM AND TORASEMIDE.

ALTERNATIVE EXPLANATIONS = PANTOPRAZOL, CEFUROXIME, PIPERACILLIN/TAZOBACTAM, RANITIDIN

EINZELFALLBEOBACHTUNG. KEIN HANDLUNGSBEDARF

Reporter's comment:

Sender diagnosis: not reported

Narrative

{TOXIC EPIDERMAL NECROLYSIS}

A report has been received from the Centre for Documentation of Severe Skin Reactions in Germany via the German Health Authority (BfArM) concerning an 36-year-old female patient who experienced an evidenced a toxic epidermal necrolysis (TEN). Suspected medication consisted of DISOPRIVAN "IMPERIAL" (propofol), BELOC ZOK (metoprolol succinate), haes, fentanyl, mannitol, ACC (acetylcysteine), ambroxol, DORMICUM (midazolam), PANTOZOL (pantoprazole sodium), MONO EMBOLEX (certoparin sodium), TOREM (torasemide), NORADRENALINE (norepinephrine), metamizole sodium, tazobac, sodium picosulfate, movicol, gentamicin, FENISTIL (dimetindene maleate), HYDROCORTISON (hydrocortisone), clonidine, lipovenoes, PERFALGAN (paracetamol), PARACEFAN (clonidine hydrochloride) and PARACEFAN (clonidine hydrochloride).

Concomitant drugs included thiopental sodium for anaesthesia, jonosteril for fluid substitution, pethidine hydrochloride for pain management, hetastarch for fluid substitution, glucose for parenteral nutrition, prednisolone for exanthema, amino acids nos for fluid substitution, sodium chloride for fluid substitution, minerals nos for mineral substitution, dimeticone for meteorism, zienam for bronchial infection, metoprolol succinate for hypertension, ranitidine for exanthema, sufentanil for anaesthesia, methylprednisolone for exanthem and propofol for anaesthesia.

Concomitantly she suffered from multiple fractures due to a fall from stairs, brain edema, suspected bronchopulmonary infection. The patient received mechanical ventilation.

The patient's medical history was not reported.

On 16-Mar-2005, an unspecified time after start of the suspected medication the patient experienced toxic epidermal necrolysis. The patient has not yet recovered.

The reporting experts from the documentation centre for severe skin reactions considered the event to be possibly related to therapy with the suspected medication.

Serious criteria were hospitalisation and life-threatening event.

Sender's comment:

Due to multiple suspected medications, some of which are recognized to precipitate TEN, a causality assessment is not possible. Diprivan/propofol is not recognized to precipitate TEN. Report according to article 63b AMG

Due to multiple suspected medications, some of which are recognized to precipitate TEN, a causality assessment is not possible. Diprivan/propofol is not recognized to precipitate TEN.

Causality assessment: unassessableNO

Reporter's comment:

Boehringer Ingelheim GmbH, Germany, 22.02.2008 DE-Boehringer Ingelheim GmbH, Germany-2008-DE-01034DE

Sender diagnosis: not reported

Narrative

This case was identified and registered by the "Centre of Documentation of severe skin diseases" in Freiburg, Germany and reported to the manufacturers as well as to the Federal Health Office. The "Centre" collects and files nationwide case reports of severe skin diseases.

Therefore, these reports must not be confused with documentations of suspected adverse drug reactions.

A 36-year-old female patient suffered from toxic-epidermal necrolysis with maculae (symptoms: maculopapular exanthem, cockades, spots and blisters) while on therapy with Laxoberal (sodium picosulfate) for obstipation, Clonidine (clonidine-hcl) for hypertension, Paracefan (clonidine-hcl) for hypertension, Haes for volume substitution, Fentanyl for sedation, Mannitol for prophylaxis of brain edema, ACC (at mechanical ventilation), Ambroxol (at mechanical ventilation), Dormicum for sedation, Pantozol fo stomach protection, Disoprivan for sedation, Mono-Embolex for prophylaxis of thrombosis, Torem for edema, Noradrenalin for hypovolaemia, Novaminsulfon for pain, Tazobac for infection, Movicol for constipation, Gentamicin for infection prophylaxis, Fenistil for exanthema, Hydrocortison for exanthem, Lipovenoes for parenteral nutrition, Beloc Zok for hypertension and

Perfalgan for pain.

The patient was admitted to hospital from 22Feb to 14Apr05.

The patient has not yet recovered.

Co-medication included Trapanal (thiopental sodium), jonosteril, Dolantin (pethidine hydrochloride), Voluven (hetastarch), glucose, prednisolon, intrafusin, NaCl, Zienam (primaxin), Ranitidin (ranitidine hydrochloride), sufentanil, Urbason (methylprednisolone) propofol, Tracutil (ZnCL₂, FECI₂, MnCl₂, CuCl₂, CrCl₃) and Espumisan (dimeticon).

Co-diseases included bronchial infection and meteorism. The patient had a narcosis and a short anaesthesia.

The reporting physician described the events as life-threatening.

Sender's comment:

Regional Centre Drug Safety Boehringer Ingelheim

Comment on case 2008-DE-01034DE / LAXOBERAL

The event in this case report is not a listed adverse drug reaction for the product in question. Based on the details currently available concerning this case there is no reasonable evidence in favour of a causal association between the reported adverse event and the product in question. Furthermore, the adverse event may be more plausibly explained by other characteristics of the case (e.g. multiple concomitant medication).

As with all unlisted events, the company will continue to monitor all respective reports received and, based on cumulative experience, will re-evaluate the available evidence on an ongoing basis. No change in the company core safety information or other measures are recommended at this time.

DUEMPELM

Case assessment created by DP on 21 Feb 2008

Reporter's comment:

NORGINECDs, 25.02.2008 DE-Norgine-20050001102

Sender diagnosis: not reported

Narrative

Regulatory Authority Report serious- LIFE THREATENING- Unlabelled.

Regulatory case received via GERMANY Regulatory Authority BfArM. Regulatory Authority Ref Number: DE-BFARM-08013929

Case 2005-0001102(0.0)

Case was reported to the BfArM by a dermatological clinic (study documentation centre) at Freiburg. The dermatological clinic is reported to be processing a retrospective observation which started in 2003 and will continue on. Physicians are required to report all cases with diagnosed Stevens-Johnson-Syndrome or Toxic Epidermal Necrosis and the centre evaluates all medication and other risk factors of these patients up to 6 weeks before the diagnosis. Observation stops in each case with the date of diagnosis.

A 36 year old female received MOVICOL (polyethylene glycol 3350 & electrolytes), at a daily dose of 1-3 sachets from 03-Mar-2005 for the treatment of constipation.

The next day (04-Mar-2005) she is reported to have experienced MACULO-PAPULOUS EXANTHEMA. 13 days later (16-Mar-2005) she experienced TOXIC EPIDERMAL NECROSIS WITH MACULAE and DERMAL SPOTS. On the 17-Mar-2005 (14 days later) she is reported to have experienced SKIN BULLAE.

It is known that the product was continued.

Other suspect drugs: None

Outcome: At the time of the reporting the dermal spots and skin bullae had not resolved. However, the maculo-papulous exanthema resolved 6 days later on 10-Mar-2005. There is conflicting information regarding the toxic epidermal necrosis with maculae which needs clarification. The toxic epidermal necrosis with maculae is reported as stopping a month later on 14-Apr-2005, however, it is also reported as outcome not resolved.

Concomitant medication:

It should be noted that it was not reported whether the comedication listed below was concomitant medication or event treatment drugs:

Ringerlösung (Ringers solution), HAES 10% (polyhydroxyethyl starch, sodium chloride), fentanyl, mannitol 15%, ACC (acetylcysteine), ambroxol, Dormicum (midazolam), Pantozol (pantoprazole sodium), Disoprivan (propofol), Mono-embolex (heparin fraction sodium salt), Torem (torasemide), Cefuroxim (cefuroxime), HyperHaes (polyhydroxyethyl starch), Trapanal (thiopental sodium), noradrenalin (norepinephrine), Novaminsulfon (metamizole), Tazobac (piperacillin & tazobactam), jonosteril, Laxoberal (sodium picosulphate), Dolantin (pethidine), Voluven (hetastarch), glucose 10%, gentamicin, Fenistil (dimetindene maleate), prednisolon (prednisolone), intrafusin 15%, hydrocortison (hydrocortisone), sodium chloride, clonidin (clonidine), Lipovenos 10% (intralipid), Beloc Zok (metoprolol), Tracutil (minerals), Peralgan (paracetamol), Paracefan (clonidine hydrochloride), Espusmisam (dimeticone), Zienam (primaxin), Beloc-zok mite (metoprolol succinate), ranitidin (ranitidine), sufentanil, Urbason (methylprednisolone), Propofol (propofol).

Relevant medical history:

Multiple fractures after falling down the staircase on 23-Feb-2005

Reporter comments:

Patient experienced toxic epidermal necrosis from 16-Mar-2005 until 14-Apr-2005. Event was life threatening. Outcome is not resolved.

Second reporter's comments (physician):

Patient was hospitalised from 22-Feb-2005 until 14-Apr-2005. Causality with Movicol is possible.

Company Comments continued:

The information on any resolution of the symptoms is conflicting; being said to have stopped the next month in one sentence, but also being reported as not resolved. However, it is noted that the patient was hospitalised until 14th April, which suggests that the acute symptoms had been treated.

These points require clarification, since if there was resolution of the symptoms with continued use of Movicol, it would be considered unlikely that Movicol was the causative agent. We have only one previous report of Stevens Johnson syndrome or toxic epidermal necrolysis in association with Movicol or any PEG based products. In this case another product was considered to be a more likely precipitant. This another case received recently from the Freiberg group. There are no other reports in the literature in association with Movicol or PEG products.

The accurate assessment of causality is confounded by the lack of clarity on whether the toxic epidermal necrolysis resolved, and whether the Movicol was continued. It would also be helpful to know whether the two other drugs which have been documented to precipitate this type of reaction were administered prior to its development or as treatment for it, and if the antibiotics used were to treat an established infection, or as prophylaxis, since infection is also known to be a precipitant of toxic epidermal necrolysis. If the reaction resolved whilst Movicol was continued or if a drug known to cause toxic epidermal necrolysis was used prior to the development of symptoms or there was an established infection, causality would be considered to be unlikely. However, were this not to be the case, the temporal relationship between the exposure to Movicol and the development of the reaction supports a causality assessment of possible. It is therefore considered that based on the information available at present, causality is not assessable.

Case status: Further information has been requested and will be made available upon receipt.

Sender's comment:**Company comments:**

This is the second of two cases reported by a group in Freiberg undertaking a retrospective review of cases of Stevens-Johnson syndrome, and the drugs consumed by each patient in the 6 weeks prior to the development of symptoms.

In this case patient was hospitalised after sustaining multiple fractures due to falling down a stair case. No other medical history is provided. Therefore although toxic epidermal necrolysis may be associated with infection, malignancy, or be drug induced or idiopathic, in the absence of any other history it would seem most likely that this case was drug induced, presumably with a drug in the management of her fractures. Unfortunately there is no differentiation between concomitant medication, presumably in the 6 weeks prior to developing symptoms, and medication used for treatment. Two of the extensive list of drugs given, pantoprazole and cefuroxime, have been associated with Stevens Johnson syndrome.

The fact that the first symptoms of maculo-papular exanthema occurred on the day after treatment with Movicol is noted, and that the florid symptoms of toxic epidermal necrolysis developed thirteen days later. Movicol is known to cause allergic reactions, which can involve a rash. This is commonly urticarial in origin rather than maculo-papular, and therefore the rash reported on the day after treatment with Movicol is considered likely to be an early symptom of the toxic epidermal necrolysis, rather than a separate reaction. However, it would appear that Movicol was not considered to be the causative agent since it was continued after the appearance of the maculo-papular rash and there is no indication that treatment with Movicol was stopped, although it is understood that "observation stops in each case with the date of diagnosis". Company comments continued in Case narrative.

Reporter's comment:

Wyeth-Pharma GmbH, 03.03.2008 DE-WYE-H02714308

Sender diagnosis: not reported

Narrative

Information was received from the German Documentation Centre of Severe Skin Reactions via a regulatory authority, via a business partner (case #

80329), regarding a 36-year-old female patient who received Pantozol (pantoprazole injection) therapy and experienced toxic epidermal necrolysis.

MEDICAL HISTORY:

The patient's concurrent illnesses include flatulence, hospitalisation, multiple fractures and fall.

PRODUCT DETAILS:

Indication for Pantozol was "stomach prophylaxis". Therapy began on 22-Feb-2005. It was unknown if therapy continued. Dose regimen was "daily dose 1 DF" (intravenous).

Additional suspect medication included Trapanal (thiopental sodium), Ringerlösung (calcium chloride dihydrate/potassium chloride/sodium chloride), Haes 10% (hyetellose/sodium chloride), Fentanyl, Mannitol 15%, Acc (acetylcysteine), Ambroxol, Dormicum (midazolam maleate), Disoprivan (propofol), Mono-Embolex (heparin-fraction, sodium salt), Torem (torasemide), Cefuroxim (cefuroxime), Noradrenalin (norepinephrine), "HyperHaes", Novaminsulfon (metamizole sodium), Tazobac (piperacillin/tazobactam injection), Jonosteril (calcium acetate/magnesium acetate/potassium acetate/sodium acetate/sodium chloride), Laxoberal (sodium picosulfate), Movicol (macrogol/potassium chloride/sodium bicarbonate/sodium chloride), Dolantin (pethidine hydrochloride), Voluven (hetastarch), Glucose 10%, Gentamicin, Fenistil (dimetindene maleate), Prednisolone, Intrafusin 15% (amino acids nos), Hydrocortison (hydrocortisone), NaCl (sodium chloride)(injection), Clonidin (clonidine), Beloc Zok and Beloc-Zok mite (metoprolol succinate), Tracutal (chromic chloride, cupric chloride dihydrate, ferrous chloride, manganese chloride tetrahydrate, potassium iodide, sodium fluoride, sodium molybdate, sodium selenite, zinc chloride), Perfalgan (paracetamol), Paracefan (clonidine hydrochloride), Espumisan, Zienam (cilastatin sodium/imipenem), Ranitidin (ranitidine), Sufentanyl (sufentanil), Urbason (methylprednisolone) and Propofol.

CONCOMITANT THERAPY:

Concomitant therapy included Lipovenoes (glycerol/lecithin/soya oil).

EVENT DETAILS:

The patient experienced toxic epidermal necrolysis (toxic epidermal necrolysis) with maculae while receiving pantoprazole therapy for stomach protection. On 22-Feb-2005, the patient was admitted to the hospital due to multiple fractures as a result of a fall. The patient was in the intensive care unit. On the same day, pantoprazole therapy was started. On 26-Feb-2005, thiopental therapy was started and was discontinued on 04-Mar-2005. On 04-Mar-2005, the patient was found to have a maculopapular exanthem, which was improved on 06-Mar-2005 and resolved on 10-Mar-2005. On 16-Mar-2005, the patient developed cockades and skin spots. On 17-Mar-2005, the patient was found to have skin blisters. On 19-Mar-2005, Nikolsky's sign was positive and the patient was transferred into a centre of burn injury until 14-Apr-2005. The event was reported as ongoing. The event was considered life-threatening.

RELATEDNESS ASSESSMENT:

The investigator(s) considered toxic epidermal necrolysis possibly related to the study product. The medical monitor(s) assessment for toxic epidermal necrolysis was unknown. The business partner considered the event possibly related to pantoprazole and thiopental therapies.

Sender's comment:

Wyeth comment:

INITIAL (v1) 03-Mar-2008

TOXIC EPIDERMAL NECROLYSIS

Bericht aus dem "Dokumentationszentrum für schwere Hautreaktionen in Freiburg". Es geht um eine 36-jährige Patientin, die im Zusammenhang mit der Einnahme zahlreicher Medikamente, darunter auch Pantozol und Tazobac ein TEN entwickelte, eingestuft als "lebensgefährlich". Gem. Bericht ist die Reaktion (aus dem Jahr 2005) noch nicht abgeklungen.

Es handelt sich um eine bekannte NW (US case expectedness) von Pantozol. TEN ist auch eine bekannte NW von Tazobac.

Ein kausaler Zusammenhang kann nicht ausgeschlossen werden.

Basierend auf der aktuellen Information führt dieser Bericht nicht zu einer Änderung des Nutzen- Risiko-Verhältnisses. Zu diesem Zeitpunkt sind keine weiteren Massnahmen vorgesehen. Sobald Änderungen hierzu bekannt werden, werden wir darüber unverzüglich und unaufgefordert informieren.

Dr. R. Jöstingmeier

Reporter's comment:

APHQWEB, 10.03.2008 DE-Nycomed GmbH-80329

Sender diagnosis: not reported

Narrative

This report was received from the German Documentation centre of severe skin reactions(ref no. 9160268) via the German Health authority (Bfarm ref. no. DE-BFARM-08013929).

A middle-aged female patient experienced Toxic epidermal necrolysis with maculae while receiving pantoprazole (Pantozol iv) for stomach protection

and thiopental-natrium (Trapanal 1.5 g iv) for anaesthesia.

Further 42 suspected drugs were administered.

On 22 FEB 2005 the patient was admitted to hospital due to multiples fractures effected by a fall. She was in an intensive care unit. On the same day pantoprazole therapy was started.

On 26 FEB 2005 trapanal therapy was started and was discontinued on 04 MAR 2008.

On 04 MAR 2005 the patient was found to have a maculopapular exanthem which was improved on 06 MAR 2005 and resolved on 10 MAR 2005.

On 16 MAR 2005 the patient developed cockades and skin spots.

On 17MAR 2005 patient was found to have skin blisters.

On 19 MAR 2005 nicholsky's sign was positive and patient was transferred into a centre of burn injury until 14 APR 2005.

outcome: still ongoing.

Concomitant diseases included oedema, hypovolaemia, pain, obstipation, exanthema, hypertension, meteorism and bronchial infection.

No further information was provided.

Sender's comment:

With the information on hand Nycomed is now going to assess the case in order to come to some conclusion.

TOXIC EPIDERMAL NECROLYSIS:

This patient developed toxic epidermal necrolysis in a plausible temporal relationship with intake of pantoprazole, approximately 4 weeks after introduction of pantoprazole. The same plausible temporal relationship with approximately 2 weeks exist with the application of Trapanal. Concomitantly, he received multiple other drugs (42 other suspected drugs).

Toxic epidermal necrolysis (TEN or Lyell syndrome) is a rare skin disorder characterized by extensive epidermal loss. In most cases it is drug-induced.

Several drugs have been observed to cause TEN. Toxic bullous skin reactions up to acute disseminated epidermal necrolysis have also been observed in very rare cases with pantoprazole and other proton pump inhibitors. There is no knowledge about cases in association with the application of Trapanal.

As time to onset was basically plausible and in the presence of multiple other suspected drugs, Nycomed assesses this case as "possible" to both pantoprazole and trapanal.

This report is not considered to have any influence on current knowledge regarding the safety profile of pantoprazole and Trapanal and has no effect on the status of the drugs as reflected by the formal product characteristics.

Reporter's comment:

Fresenius Kabi, 11.03.2008 DE-Fresenius Kabi-2008030053

Sender diagnosis: not reported

Narrative

Verbatim of Reporter:

Toxic epidermal necrolysis. Started on March 16th, 2005 until April 14th 2005

Narrative:

The case was reported by the BfArM (health authority Germany) with Worldwide Reference Number DE-BFARM-08013929 and was detected in an epidemiological study (RegiSCAR Studie) in Germany.

On February 22nd, 2005 a 36-year-old female patient was admitted to the hospital for multiple fractures after falling down the stairs. 125 ml HAES steril 10% was administered from 22.02.2005 until 08.03. 2005. 500 ml Lipovenoes was administered on 10.03.2005. On March 3rd, 2005 a maculopapular exantema occurred and reversed on March 6th, 2005 and abated on March 10th, 2005. On March 17th, 2005 blister occurred.

Medical history: multiple fractures

Concomitant medication:

Assessed as possibly related to reaction according to the assessment of experts 03/2006

Fentanyl, mannitol, ACC, ambroxol, Dormicum (midazolam), Pantozol (pantoprazol), Disoprivan (propofol), Mono-Embolex (heparin), Torem (torasemid, noradrenalin, Novaminsulfon (metamizol), Tazobac (tazobactam), Laxoberal (natriumpicosulfat), Movicol, Dolantin (pethidin), gentamicin, Fenistil (dimetinden), hydrocortisone, NaCL, clonidin, Beloc.Zok (metoprolol), Perfalgan (propacetamol), Paracefan (clonidin).

Sender's comment:

According to the reporter's assessment the symptoms are possibly related to the administration of Haessteril 10% and Lipovenoes 10%.

We agree to the reporter's assessment. The symptoms occurred in plausible temporal relationship to the administration of Haes steril 10% and Lipovenoes 10%. Concomitant medication may have caused the reported symptoms as well. Therefore we consider it as possible that Haes steril 10% and Lipovenoes have caused or contributed to the reported symptoms.

The symptoms are not labeled as possible side effects in the SmPC of Haes steril 10% and Lipovenoes 10%.

Reporter's comment:

APHQWEB, 12.03.2008 DE-Nycomed GmbH-80329

Sender diagnosis: not reported

Narrative

This report was received from the German Documentation centre of severe skin reactions(ref no. 9160268) via the German Health authority (Bfarm ref. no. DE-BFARM-08013929).

A middle-aged female patient experienced Toxic epidermal necrolysis with maculae while receiving pantoprazole (Pantozol iv) for stomach protection and thiopental-natrium (Trapanal 1.5 g iv) for anaesthesia.

Further 42 suspected drugs were administered.

On 22 FEB 2005 the patient was admitted to hospital due to multiples fractures effected by a fall. She was in an intensive care unit. On the same day pantoprazole therapy was started.

On 26 FEB 2005 trapanal therapy was started and was discontinued on 04 MAR 2008.

On 04 MAR 2005 the patient was found to have a maculopapular exanthem which was improved on 06 MAR 2005 and resolved on 10 MAR 2005.

On 16 MAR 2005 the patient developed cockades and skin spots.

On 17MAR 2005 patient was found to have skin blisters.

On 19 MAR 2005 nicholsky's sign was positive and patient was transferred into a centre of burn injury until 14 APR 2005.

outcome: still ongoing.

Concomitant diseases included oedema, hypovolaemia, pain, obstipation, exanthema, hypertension, meteorism and bronchial infection.

No further information was provided.

Sender's comment:

With the information on hand Nycomed is now going to assess the case in order to come to some conclusion.

TOXIC EPIDERMAL NECROLYSIS:

This patient developed toxic epidermal necrolysis in a plausible temporal relationship with intake of pantoprazole, approximately 4 weeks after introduction of pantoprazole. The same plausible temporal relationship with approximately 2 weeks exist with the application of Trapanal. Concomitantly, he received multiple other drugs (42 other suspected drugs).

Toxic epidermal necrolysis (TEN or Lyell syndrome) is a rare skin disorder characterized by extensive epidermal loss. In most cases it is drug-induced.

Several drugs have been observed to cause TEN. Toxic bullous skin reactions up to acute disseminated epidermal necrolysis have also been observed in very rare cases with pantoprazole and other proton pump inhibitors. There is no knowledge about cases in association with the application of Trapanal.

As time to onset was basically plausible and in the presence of multiple other suspected drugs, Nycomed assesses this case as "possible" to both pantoprazole and trapanal.

This report is not considered to have any influence on current knowledge regarding the safety profile of pantoprazole and Trapanal and has no effect on the status of the drugs as reflected by the formal product characteristics.

Reporter's comment:

APHQWEB, 12.03.2008 DE-Nycomed GmbH-80329

Sender diagnosis: not reported

Narrative

This report was received from the German Documentation centre of severe skin reactions(ref no. 9160268) via the German Health authority (Bfarm ref. no. DE-BFARM-08013929).

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On 19 MAR 2005 nicholsky's sign was positive and patient was transferred into a centre of burn injury until 14 APR 2005.

outcome: still ongoing.

Concomitant diseases included oedema, hypovolaemia, pain, obstipation, exanthema, hypertension, meteorism and bronchial infection.

No further information was provided.

Sender's comment:

With the information on hand Nycomed is now going to assess the case in order to come to some conclusion.

TOXIC EPIDERMAL NECROLYSIS:

This patient developed toxic epidermal necrolysis in a plausible temporal relationship with intake of pantoprazole, approximately 4 weeks after introduction of pantoprazole. The same plausible temporal relationship with approximately 2 weeks exist with the application of Trapanal. Concomitantly, he received multiple other drugs (42 other suspected drugs).

Toxic epidermal necrolysis (TEN or Lyell syndrome) is a rare skin disorder characterized by extensive epidermal loss. In most cases it is drug-induced.

Several drugs have been observed to cause TEN. Toxic bullous skin reactions up to acute disseminated epidermal necrolysis have also been observed in very rare cases with pantoprazole and other proton pump inhibitors. There is no knowledge about cases in association with the application of Trapanal.

As time to onset was basically plausible and in the presence of multiple other suspected drugs, Nycomed assesses this case as "possible" to both pantoprazole and trapanal.

This report is not considered to have any influence on current knowledge regarding the safety profile of pantoprazole and Trapanal and has no effect on the status of the drugs as reflected by the formal product characteristics.

Reporter's comment:

APHQWEB, 13.03.2008 DE-Nycomed GmbH-80329

Sender diagnosis: not reported

Narrative

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A middle-aged female patient experienced Toxic epidermal necrolysis with maculae while receiving pantoprazole (Pantozol iv) for stomach protection and thiopental-natrium (Trapanal 1.5 g iv) for anaesthesia.

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outcome: still ongoing.

Concomitant diseases included oedema, hypovolaemia, pain, obstipation, exanthema, hypertension, meteorism and bronchial infection.

No further information was provided.

Sender's comment:

With the information on hand Nycomed is now going to assess the case in order to come to some conclusion.

TOXIC EPIDERMAL NECROLYSIS:

This patient developed toxic epidermal necrolysis in a plausible temporal relationship with intake of pantoprazole, approximately 4 weeks after introduction of pantoprazole. The same plausible temporal relationship with approximately 2 weeks exist with the application of Trapanal. Concomitantly, he received multiple other drugs (42 other suspected drugs).

Toxic epidermal necrolysis (TEN or Lyell syndrome) is a rare skin disorder characterized by extensive epidermal loss. In most cases it is drug-induced.

Several drugs have been observed to cause TEN. Toxic bullous skin reactions up to acute disseminated epidermal necrolysis have also been observed in very rare cases with pantoprazole and other proton pump inhibitors. There is no knowledge about cases in association with the application of Trapanal.

As time to onset was basically plausible and in the presence of multiple other suspected drugs, Nycomed assesses this case as "possible" to both pantoprazole and trapanal.

This report is not considered to have any influence on current knowledge regarding the safety profile of pantoprazole and Trapanal and has no effect on the status of the drugs as reflected by the formal product characteristics.

Reporter's comment:

APHQWEB, 13.03.2008 DE-Nycomed GmbH-80329

Sender diagnosis: not reported

Narrative

This report was received from the German Documentation centre of severe skin reactions(ref no. 9160268) via the German Health authority (Bfarm ref. no. DE-BFARM-08013929).

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On 19 MAR 2005 nicholsky's sign was positive and patient was transferred into a centre of burn injury until 14 APR 2005.

outcome: still ongoing.

Concomitant diseases included oedema, hypovolaemia, pain, obstipation, exanthema, hypertension, meteorism and bronchial infection.

No further information was provided.

Sender's comment:

With the information on hand Nycomed is now going to assess the case in order to come to some conclusion.

TOXIC EPIDERMAL NECROLYSIS:

This patient developed toxic epidermal necrolysis in a plausible temporal relationship with intake of pantoprazole, approximately 4 weeks after introduction of pantoprazole. The same plausible temporal relationship with approximately 2 weeks exist with the application of Trapanal. Concomitantly, he received multiple other drugs (42 other suspected drugs).

Toxic epidermal necrolysis (TEN or Lyell syndrome) is a rare skin disorder characterized by extensive epidermal loss. In most cases it is drug-induced.

Several drugs have been observed to cause TEN. Toxic bullous skin reactions up to acute disseminated epidermal necrolysis have also been observed in very rare cases with pantoprazole and other proton pump inhibitors. There is no knowledge about cases in association with the application of Trapanal.

As time to onset was basically plausible and in the presence of multiple other suspected drugs, Nycomed assesses this case as "possible" to both pantoprazole and trapanal.

This report is not considered to have any influence on current knowledge regarding the safety profile of pantoprazole and Trapanal and has no effect on the status of the drugs as reflected by the formal product characteristics.

Reporter's comment:

APHQWEB, 18.03.2008 DE-Nycomed GmbH-80329

Sender diagnosis: not reported

Narrative

This report was received from the German Documentation centre of severe skin reactions(ref no. 9160268) via the German Health authority (Bfarm ref. no. DE-BFARM-08013929).

A middle-aged female patient experienced Toxic epidermal necrolysis with maculae while receiving pantoprazole (Pantozol iv) for stomach protection and thiopental-natrium (Trapanal 1.5 g iv) for anaesthesia.

Further 42 suspected drugs were administered.

On 22 FEB 2005 the patient was admitted to hospital due to multiples fractures effected by a fall. She was in an intensive care unit. On the same day pantoprazole therapy was started.

On 26 FEB 2005 trapanal therapy was started and was discontinued on 04 MAR 2008.

On 04 MAR 2005 the patient was found to have a maculopapular exanthem which was improved on 06 MAR 2005 and resolved on 10 MAR 2005.

On 16 MAR 2005 the patient developed cockades and skin spots.

On 17MAR 2005 patient was found to have skin blisters.

On 19 MAR 2005 nicholsky's sign was positive and patient was transferred into a centre of burn injury until 14 APR 2005.

outcome: still ongoing.

Concomitant diseases included oedema, hypovolaemia, pain, obstipation, exanthema, hypertension, meteorism and bronchial infection.

No further information was provided.

Sender's comment:

With the information on hand Nycomed is now going to assess the case in order to come to some conclusion.

TOXIC EPIDERMAL NECROLYSIS:

This patient developed toxic epidermal necrolysis in a plausible temporal relationship with intake of pantoprazole, approximately 4 weeks after introduction of pantoprazole. The same plausible temporal relationship with approximately 2 weeks exist with the application of Trapanal. Concomitantly, he received multiple other drugs (42 other suspected drugs).

Toxic epidermal necrolysis (TEN or Lyell syndrome) is a rare skin disorder characterized by extensive epidermal loss. In most cases it is drug-induced.

Several drugs have been observed to cause TEN. Toxic bullous skin reactions up to acute disseminated epidermal necrolysis have also been observed in very rare cases with pantoprazole and other proton pump inhibitors. There is no knowledge about cases in association with the application

of Trapanal.

As time to onset was basically plausible and in the presence of multiple other suspected drugs, Nycomed assesses this case as "possible" to both pantoprazole and trapanal.

This report is not considered to have any influence on current knowledge regarding the safety profile of pantoprazole and Trapanal and has no effect on the status of the drugs as reflected by the formal product characteristics.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Report from study

case: DE-BFARM-08014001

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes			Yes		Yes

Patient

Initials:	Sex: Male	Birth: privacy	Age: 26 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Erythema exsudativum multiforme	20050713	20050729	17 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Gripin caffeine; paracetamol	Oral	2 Df dosage form/Day Tablet	20050710-20050714 5 Day	Pain
2 Paracetamol-ratiopharm 500 paracetamol	Oral	1000 Mg milligram(s)/Day Tablet	20050701-20050710 10 Day	Pain
3 Talcid hydrotalcite	Oral	/	200101-20050712 -	Heartburn

Drugs (concomitant)

4 Alcon AT ciprofloxacin hydrochloride	Ophthalmic	/	20050713-	Conjunctivitis
5 Cromo AT cromoglicic acid	Ophthalmic	2 Gtt drop(s)/Day Eye drops	20050501-20050625 56 Day	Pollinosis
6 Cromo NS cromoglicic acid	Nasal	2* /Day Nasal drops	200505-20050625 -	Pollinosis

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Heartburn			Yes	
Pollinosis			Yes	
Rhinopharyngitis	20050701		Unknown	

Primary source

Qualification:	Physician	observestudytype:	Other studies
Literature:			

Narratives (nur bei elektronischem Eingang)

ratiopharm GmbH, 04.03.2008 DE-RATIOPH-2008R021594

Sender diagnosis: not reported

Narrative

Reported terms: Erythema exsudativum multiforme (majus)(Erythema multiforme)

Case reference number 2008R021594 is a spontaneous case report from the RegiSCAR-Studie, sent by the Dokumentationszentrum für schwere Hautkrankheiten (dZh), Uniklinik Freiburg. It refers to a male aged 26 years.

The patient's past medical history included no medical records. There is no information about risk factors. Concomitant diseases include Rhinopharyngitis (Nasopharyngitis), Heartburn (Dyspepsia) and Pollinosis (Seasonal allergy). No prior drugs are known to have been taken by the patient.

The patient started taking Paracetamol-ratiopharm 500 (PARACETAMOL), indication Pain on 01-Jul-2005, Gripin (SALICYLAMIDE, COMBINATIONS EXCL. PSYCHOLEPTICS), indication Pain on 10-Jul-2005 and Talcid (HYDROTALCITE), indication Heartburn on ??-Jan-2001. The patient has been exposed to the following concomitant drug(s): Alcon AT (ARTIFICIAL TEARS AND OTHER INDIFFERENT PREPARATIONS), indication Conjunctivitis, Cromo AT (CROMOGLICIC ACID), indication Pollinosis and Cromo NS (CROMOGLICIC ACID), indication Pollinosis. The patient developed Erythema exsudativum multiforme (majus) (Erythema multiforme)(start date 13-Jul-2005).

The following action was taken: for Paracetamol-ratiopharm 500 (PARACETAMOL) - Drug withdrawn, for Gripin (SALICYLAMIDE, COMBINATIONS EXCL. PSYCHOLEPTICS) - Drug withdrawn for Talcid (HYDROTALCITE) - Drug withdrawn.

The outcome for Erythema exsudativum multiforme (majus) (Erythema multiforme) is Recovered / Resolved.

Rechallenge information for Paracetamol-ratiopharm 500, for Gripin and for Talcid is not applicable. Dechallenge information for Paracetamol-ratiopharm 500, for Gripin and for Talcid is not applicable.

The reporter's assessment for drug Paracetamol-ratiopharm 500 related with Erythema exsudativum multiforme (majus) is as follows: Possible.
CLASSIFICATION: Adverse Event: No, Adverse Reaction: Yes, Drug / Drug Interaction: No, Abuse / Misuse: No, Pregnancy / Lactation tracking: No

Sender's comment:

Erythema exsudativum multiforme is unlisted in the SPC/RSI of Paracetamol-ratiopharm. We have three more cases including this symptom after paracetamol use in our data bank. In all cases causal relationship is considered as "possible". In the drugdex one case report is listed. Immediate actions are not regarded necessary, however this possible adverse drug reaction will be kept under close surveillance.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Report from study

case: DE-BFARM-08014009

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes	Yes	No	No	No	No		

Patient

Initials:	Sex: Female	Birth: privacy	Age: 54 Year	weight (kg):	BMI_int:	Age fetus:
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Patientdeath

deathdate: 20050420	deathcause: Toxic epidermal necrolysis
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Macule	20050317		35 Day		fatal
2 Blister	20050317		35 Day		fatal
3 Toxic epidermal necrolysis	20050317	20050420	35 Day	8 Day	fatal
4 Oral ulceration	20050317		35 Day		fatal
5 Lip erosion	20050317		35 Day		fatal
6 Mucosal erosion	20050317		35 Day		fatal
7 Eyes red	20050317		35 Day		fatal

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
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DE-BFARM-08014009

1	ALLOPURINOL allopurinol	Oral	150 Mg milligram(s)/ Day	20050314-20050314	Hyperuricemia
2	BELOC-ZOK MITE metoprolol	Oral	2*1 Df dosage form/ Day	20050203-20050319	Aortic incompetence 45 Day
3	ciprobay ciprofloxacin hydrochloride	Intravenous (not otherwis	2*1 Df dosage form/ Day	20050304-20050316	Infection prophylaxis 13 Day
4	CO-DIOVAN hydrochlorothiazide; valsartan	Oral	80 Mg milligram(s)/ Day	20050127-20050304	Aortic incompetence 37 Day
5	decortin prednisone	Oral	2 Df dosage form/ Day	20050203-20050316	Renal transplant 42 Day
6	Erythromycinum homeopatics nos	Intravenous (not otherwis	2 Df dosage form/ Day	20050312-20050312	Anaemia
7	FORADIL P formoterol	Respiratory (inhalation)	2*1 Df dosage form/ Day	20050127-20050317	COPD 50 Day
8	FRAXIPARIN nadroparin calcium	Subcutaneous	.4 ml millilitre(s)/ Day	20050127-20050319	Thrombosis prophylaxis 52 Day
9	gentamycin gentamicin sulfate	Intravenous (not otherwis	40 Mg milligram(s)/ Day	20050202-20050319	Endocarditis 46 Day
10	Hydrokortison hydrocortisone	Intravenous (not otherwis	100 Mg milligram(s)/24 Hc	20050304-20050304	Hypotension
11	IMUREK azathioprine	Oral	100 Mg milligram(s)/ Day	20050127-20050319	Renal transplant 52 Day
12	l-thyroxin levothyroxine sodium	Oral	100 Mg milligram(s)/ Day	20050127-20050319	Hypothyroidism 52 Day
13	nacl sodium chloride	Intravenous (not otherwis	500 ml millilitre(s)/	20050127-20050319	Fluid replacement NOS 52 Day
14	NEORECORMON epoetin beta	Intravenous (not otherwis	4000 lu international unit(s)	-20050319	Anaemia
15	NEUPOGEN - prefilled syringe	Intravenous (not otherwis	30 lu international unit(s)/l	20050309-20050309	Neutropenia
16	NOVALGIN metamizole	Unknown	1 G gram(s)/ Day	20050316-20050319	Pain 4 Day
17	NOVALGIN metamizole	Unknown	1 G gram(s)/ Day	20050311-20050311	Pain
18	novalgin tropfen metamizole sodium	Oral	40 Gtt drop(s)/Day	20050317-	Pain
19	OMEPI omeprazole	Oral	40 Mg milligram(s)/ Day	20050203-20050317	Prophylaxis 43 Day
20	OXIS formoterol	Respiratory (inhalation)	2*1 Df dosage form/ Day	20050127-20050317	COPD 50 Day
21	PARACETAMOL paracetamol	Oral	500 Mg milligram(s)/	20050304-20050304	Pain
22	ringer calcium chloride; potassium chloride; sodium chloride; sodium chloride compound injection	Intravenous (not otherwis	500 ml millilitre(s)/	20050127-20050319	Fluid replacement NOS 52 Day
23	tavegil clemastine	Unknown	1 Df dosage form/ Day	20050316-20051117	Pruritus 8 Month
24	tavegil clemastine	Unknown	2*1 Df dosage form/ Day	20050224-20050302	Pruritus 7 Day
25	TELFAS fexofenadine	Oral	180 Mg milligram(s)/Day	20050302-20050309	Pruritus 8 Day
26	UNAT torasemide	Oral	20 Mg milligram(s)/Day	20050213-20050215	Diuresis 3 Day
27	valcyte valganciclovir hydrochloride	Oral	450 Mg milligram(s)/	-	Infection
28	valganciclovir valganciclovir	Unknown	/	-	

29	vancomycin vancomycin	Intravenous (not otherwis /	-	20050127-20050319 Endocarditis
30	ZOLDEM zolpidem	Oral	1 Df dosage form/ Day	52 Day 20050210-20050222 Sleep disorder
31	ZOLDEM zolpidem	Oral	/	13 Day -20050319 Sleep disorder
32	ZOLDEM zolpidem	Oral	/	- 20050307-20050313 Sleep disorder 7 Day

Drugs (concomitant)					
33	acc acetylcysteine	Oral	3*600 Mg milligram(s)/	Da 20050217-20050224	Reaction unevaluable 8 Day
34	acyclovir aciclovir	Topical	/	20050204-20050223	Herpes infection 20 Day
35	ampho-moronal amphotericin b	Other	3*2 Df dosage form/	Day 20050203-20050224	Candidiasis 22 Day
36	AZATHIOPRINE azathioprine	Oral	100 Mg milligram(s)/	20050127-	-
37	BIFITERAL lactulose	Oral	10 ml millilitre(s)/	Day 20050223-20050223	Obstipation -
38	BIFITERAL lactulose	Oral	10 ml millilitre(s)/	Day 20050303-20050303	Obstipation -
39	broncho fertiginhalat salbutamol sulfate	Respiratory (inhalation)	3*1 Df dosage form/	Day -	COPD -
40	cymeven ganciclovir sodium	Oral	2*500 Mg milligram(s)/	Da 20050218-20050222	Infection 5 Day
41	DECOSTRIOL calcitriol	Oral	3*.5 Mg milligram(s)/	Wee 20050203-20050223	Reaction unevaluable 21 Day
42	DERMATOP prednicarbate	Topical	/	20050302-20050303	Exanthema 2 Day
43	HEPARIN CALCIUM heparin calcium	Subcutaneous	0.4 ml millilitre(s)/	20050127-	-
44	KLYSMA SORBIT potassium sorbate; sorbitol		1 Df dosage form/	Day 20050302-20050302	Obstipation 242 -
45	LAXOBERAL sodium picosulfate	Oral	1 Df dosage form/	Day -	Obstipation -
46	locol fluvastatin sodium	Oral	80 Mg milligram(s)/	Day 20050203-20050227	Hypercholesterolaemia 25 Day
47	magnesium verla magnesium hydrogen aspartate	Oral	5 Df dosage form/	Day 20050203-20050224	Reaction unevaluable 22 Day
48	METOPROLOL metoprolol	Oral	/	20050203-	-
49	nacl sodium chloride	Respiratory (inhalation)	1 Df dosage form/	Day 20050217-20050224	8 Day
50	Natriumchlorid sodium chloride	Intravenous (not otherwis	/	20050127-	-
51	paracodin dihydrocodeine bitartrate	Oral	/	Day 20050222-20050223	Pain 2 Day
52	PREDNISON prednisone	Oral	/	20050302-	-
53	thesit polidocanol	Topical	/	20050224-20050224	Pruritus -
54	thesit polidocanol	Topical	/	20050302-20050303	Pruritus 2 Day
55	zienam cilastatin sodium; imipenem	Intravenous (not otherwis	2*.5 Mg milligram(s)/	Day 20050217-20050224	Endocarditis 8 Day

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Exanthematic drug eruption	20050223		Unknown	
Sleep disorder			Unknown	
Renal transplant			Yes	
Pruritus			Unknown	
Pain			Unknown	
Obstipation			Unknown	
Neutropenia			Unknown	
Infection			Unknown	
Hypothyroidism			Unknown	
Hypotension			Unknown	
Hyperuricemia			Unknown	
Hypercholesterolaemia			Unknown	
Herpes infection			Unknown	
Endocarditis			Yes	
Diuresis			Unknown	
COPD			Yes	
Candidiasis			Unknown	
Aortic incompetence			Unknown	
Anaemia			Unknown	
Lip erosion	20050323		Unknown	
Exanthema		20050308	Unknown	
Fever			Unknown	

Primary source

Qualification: Physician observestudytype: Other studies
 Literature:

Narratives (nur bei elektronischem Eingang)

ROCHE, 13.02.2008 DE-ROCHE-545539

Sender diagnosis: not reported

Narrative

INTENSIVE MONITORING CASE

REGISCAR STUDY

THIS CASE WAS REPORTED BY A PHYSICIAN AND CONCERNS A 57-YEAR-OLD FEMALE PATIENT WHO EXPERIENCED FATAL TEN (TOXIC EPIDERMAL NECROLYSIS) DURING THE USE OF VALGANCICLOVIR (VALCYTE) FOR SUSPECTED INFECTION, TORASEMID (UNAT) FOR DIURESIS, EPOETIN BETA (NEORECORMONE) FOR ANAEMIA, VALSARTAN (CODIOVAN) FOR AORTIC VITUM, PREDNISONE (DECORTIN) FOR STATUS POST RENAL TRANSPLANT, FORMOTEROL (FORADIL) FOR CHRONIC OBSTRUCTIVE PULMONARY DISEASE (COPD), FORMOTEROL (OXIS) FOR CHRONIC OBSTRUCTIVE PULMONARY DISEASE (COPD), OMEPRAZOL (OMEP) FOR STOMACH PROTECTION, GENTAMYCIN FOR ENDOCARDITIS, VANCOMYCIN FOR ENDOCARDITIS, NADROPARIN CALCIUM FOR THROMBOSIS PROPHYLAXIS, METOPROLOL (BELOC-ZOK) FOR AORTIC VITRIUM, ZOLPIDEM (ZOLDEM) FOR SLEEPING DISORDER, AZATHIOPRIN (IMUREK) FOR STATUS POST RENAL TRANSPLANT, SODIUM CHLORIDE FOR FLUID SUBSTITUTION, LEVOTHYROXIN (L-THYROXIN) FOR HYPOTHYREOSIS, RINGER SOLUTION FOR FLUID SUBSTITUTION, FEXOFENADIN (TELFAST) FOR PRURITUS, PARACETAMOL FOR PAIN, HYDROCORTISONE FOR HYPOTENSION, CIPROFLOXACIN (CIPROBAY) FOR INFECTION PROPHYLAXIS, FILGRASTIM (NEUPOGEN) FOR NEUTROPENIA, METAMIZOL (NOVALGIN) FOR PAIN, ALLOPURINOL FOR HYPERURIKALAEMIA AND CLEMASTIN (TAVEGIL) FOR PRURITUS.

THE PATIENT HAD THE EARLIER EXPOSURE TO ZOLPIDEM, LEVOTHYROXIN AND PARACETAMOL.

ON 27 JANUARY 2005, THE PATIENT WAS HOSPITALISED FOR AN UNREPORTED REASON. THE SAME DAY, THE PATIENT STARTED RECEIVING ORAL VALSARTAN (80 MG ONCE DAILY), FORMOTEROL (TWICE DAILY), INTRAVENOUS VANCOMYCIN, SUBCUTANEOUS NADROPARIN CALCIUM (0.4ML ONCE DAILY), ORAL AZATHIOPRIN (100MG ONCE DAILY), INTRAVENOUS SODIUM CHLORIDE (500ML-1000ML), ORAL LEVOTHYROXIN (100 MG ONCE DAILY) AND INTRAVENOUS RINGER SOLUTION (500ML-1000ML). ON 2 FEBRUARY 2005, SHE STARTED RECEIVING GENTAMICIN (40MG ONCE DAILY). THE FOLLOWING DAY, SHE STARTED RECEIVING TREATMENT WITH ORAL

PREDNISONE (2 DOSE FORMS ONCE DAILY), ORAL OMEPRAZOLE (40MG ONCE DAILY) AND ORAL METOPROLOL (TWICE DAILY). ON 10 FEBRUARY 2005, SHE COMMENCED ORAL ZOLPIDEM (ONCE DAILY). ZOLPIDEM WAS INTERRUPTED ON 22 FEBRUARY 2005 AND THE FOLLOWING DAY, SHE EXPERIENCED DRUG ERUPTION. ON 4 MARCH 2005, VALSARTAN WAS DISCONTINUED. FROM 2 MARCH 2005 TO 9 MARCH THE PATIENT RECEIVED ORAL FEXOFENADIN (180MG ONCE DAILY). ON 5 MARCH 2005 AND ON 7 MARCH 2005 THE PATIENT RECEIVED VALGANCICLOVIR AT 450 MG DAILY. FROM 7 MARCH 2005 TO 13 MARCH 2005, SHE RESTARTED THE USE OF ORAL ZOLPIDEM. FROM 13 MARCH 2005 TO 15 MARCH 2005 SHE ADDITIONALLY RECEIVED TORASEMID AT 10 MG TWICE DAILY. ON 15 MARCH 2005, THE PATIENT EXPERIENCED FEVER FOLLOWED BY MACULAE AND SPOTS ON 17 MARCH 2005. ON 16 MARCH 2005, PREDNISONE WAS DISCONTINUED AND THE FOLLOWING DAY, FORMOTEROL, FORMOTEROL AND OMEPRAZOLE WERE DISCONTINUED. ON 16 MARCH 2005, SHE RECEIVED INTRAVENOUS METAMIZOL (1 GRAM ONCE DAILY) AND INTRAVENOUS CLEMASTIN (ONCE DAILY). CLEMASTIN WAS DISCONTINUED THE FOLLOWING DAY WHILE METAMIZOL WAS DISCONTINUED ON 19 MARCH 2005. THE PATIENT ALSO RECEIVED ZOLPIDEM ON 15 MARCH, 17 MARCH AND 19 MARCH 2005. IT WAS REPORTED THAT ON 4 MARCH 2005, SHE HAD ALSO RECEIVED ORAL PARACETAMOL (500MG) AND INTRAVENOUS HYDROCORTISONE (100MG PER 24 HOURS). SHE RECEIVED INTRAVENOUS FILGRASTIM (30IE ONCE DAILY) ON 9 MARCH 2005, INTRAVENOUS EPOETIN BETA (400IE) ON 12 MARCH 2005, 15 MARCH 2005, 17 MARCH 2005 AND 19 MARCH 2005 AND ORAL ALLOPURINOL (150MG ONCE DAILY) ON 16 MARCH 2005. ON 19 MARCH 2005, SHE EXPERIENCED BLISTERING OF THE SKIN. THE SAME DAY, ALL THE ONGOING DRUGS WERE DISCONTINUED. ON 22 MARCH 2005, SHE DEVELOPED EROSIONS ON LIPS FOLLOWED BY RED EYES AND TRACHEOBRONCHIAL EROSIONS ON 23 MARCH 2005. ON 20 APRIL 2005, THE PATIENT DIED OF TOXIC EPIDERMAL NECROLYSIS.

THE REPORTER ASSESSED THE EVENT AS POSSIBLY RELATED TO VALGANCICLOVIR, TORASEMID, FEXOFENADIN, FILGRASTIM, ZOLPIDEM AND METAMIZOL. THE REPORTER ASSESSED THE EVENT AS VERY POSSIBLY RELATED TO CIPROFLOXACIN, METAMIZOL AND ALLOPURINOL WHILE QUESTIONABLY RELATED TO VALSARTAN, PREDNISONE, FORMOTEROL, OMEPRAZOLE, GENTAMYCIN, VANCOMYCIN, NADROPARIN CALCIUM, METOPROLOL, AZATHIOPRIN, SODIUM CHLORIDE, LEVOTHYROXIN, RINGER SOLUTION, PARACETAMOL, HYDROCORTISONE, EPOETIN BETA AND CLEMASTIN.

NO FURTHER INFORMATION WAS AVAILABLE.

Sender's comment:

CIOMS TEXT: FATAL TOXIC EPIDERMAL NECROLYSIS IS NOT LISTED IN THE COMPANY REFERENCE DOCUMENT (CDS) FOR VALGANCICLOVIR, TORASEMID AND EPOETIN BETA. BASED ON THE INFORMATION RECEIVED FOR THIS CASE, A CAUSAL RELATIONSHIP BETWEEN THE REPORTED EVENT AND TREATMENT WITH VALGANCICLOVIR, TORASEMID AND EPOETIN BETA IS ASSESSED AS POSSIBLE. COMPANY CAUSALITY WAS ASSESSED BASED ON THE NARANJO ALGORITHM. HOWEVER, AN ALTERNATIVE EXPLANATION FOR THE EVENT INCLUDES CO-SUSPECT MEDICATION ALLOPURINOL.; ***LOCAL ASSESSMENT: BERICHT DES REGISCAR (#9160270) ÄßBER DAS AUFTRETEN EINER TOXISCH EPIDERMALEN NEKROLYSE (TEN) IM ZUSAMMENHANG MIT DER ANWENDUNG VON VALGANCICLOVIR (VALCYTEÄ®), TORASEMID (UNATÄ®) UND EPOETIN BETA (NEORECORMONÄ®) DAS AUFTRETEN EINER DERARTIGER BEGLEITERSCHINUNGEN ZÄ„HLT NICHT ZUM TYPISCHEN NEBENWIRKUNGSPROFIL EINER ANWENDUNG DER GENANNTEN PRÄ„PRATE UND WIRD IN DEN FACHINFORMATIONEN NICHT ENTSPRECHEND BESCHRIEBEN. DIE VIELZAHL WEITERER VERABREICHTER WIRKSTOFFE LÄ„SST KEINE EINDEUTIGE URSÄ„CHLICHE EINSCHÄ„TZUNG IM VORLIEGENDEN FALL ZU. KEIN HANDLUNGSBEDARF AUFGRUND DIESER EINZELFALLBEOBACHTUNG.

Reporter's comment:

Sanofi-Aventis Deutschland GmbH, 22.02.2008 DE-AVENTIS-200810462DE

Sender diagnosis: not reported

Narrative

Initial report

This case of toxic epidermal necrolysis (TEN) with maculae was received from the German health authority (DE-BFARM-08014009) . It was reported by the German Documentation Centre for Severe Skin Reactions in Freiburg / Breisgau, Germany (code-no. 9160270). The centre is conducting a prospective study on the incidence and drug aetiology of severe skin reactions. Diagnosis of TEN was made by an expert panel who assessed it as a "definite case" of TEN. Causal relationship was assessed by the experts in Sep. 2005 using an algorithm.

As for metamizole (Novalgin) the causality was assessed as "well possible" (solution for injection) and "possible" (drops), for fexofenadine (Telfast) it was assessed as "possible". For the other suspected drugs causality was assessed as :

"well possible": ciprofloxacin, allopurinol

"possible": zolpidem, valganciclovir, filgrastim, torasemide

Not suspected were: ganciclovir, aciclovir, calcitriol, amphotericin B, acetylcysteine, sodium chloride, potassium, valsartan + hydrochlorothiazide, fluvastatin, prednisone, formoterol, omeprazole, gentamicin, vancomycin, heparin calcium, metoprolol, azathioprine, levothyroxine, imipenem +

cilastatin, electrolyte solution, dihydrocodeine, lactulose, salbutamol, clemastine, polidocanol, sorbitol, prednicarbate, sodium picosulfate, paracetamol, hydrocortisone, erythrocytes, epoetin beta..

"endocarditis and state after renal transplantation" were considered possible causes of TEN.

Epicrisis:

Patient: female, 54 years

Medical history: state after renal transplantation, endocarditis, aortic valve defect, soor, renal cleft herpes, chronic obstructive pulmonary disease, hypercholesterolemia, hyperuricemia, hypotension, anemia, drug induced exanthema (until 08-Mar-2005).

The patient had suffered from drug-induced exanthema until 08-Mar-2005.

From 02- to 09-Mar-2005 she was treated with fexofenadine 1x180mg/day for pruritus, On 11-Mar-2005 she received 1g metamizole intravenously for pain, and on 16-Mar-2005 she took 40 drops of metamizole for pain; from 16-Mar-2005 to at least 19-Mar-2005 she again received intravenous injections of 1g of metamizole daily. Before and during hospitalisation the patient was also treated with a lot of other drugs

On 15-Mar-2005 the patient developed fever (not further specified).

On 17-Mar-2005 target lesions and maculae were detected.

On 19-Mar-2005 the patient was hospitalised due to skin blisters and erosions of oral mucosa.

On 22-Mar-2005 she developed erosions of her lips and, on 23-Mar-2005, red eyes and tracheobronchial erosions.

The outcome was reported as "death", date of end of event was reported as 20-Apr-2004.

Laboratory: no values provided.

Sender's comment:

20-FEB-2008 Sanofi-Aventis Company Comment: Based only on the compatible temporal relationship the causal role of metamizole and /or fexofenadine cannot be excluded. However, in this case the numerous other drugs as well as the underlying endocarditis and / or state after renal transplantation may be discussed as possible alternative causes of the reaction.

20-FEB-2008 Sanofi Aventis Germany: TOXIC EPIDERMAL NECROLYSIS - FATAL is considered locally unlabelled for fexofenadine (Telfast) as well as for metamizole sodium (Novalgin) (due to fatal outcome). The facts presented here however are not very suggestive of a causal relationship (please refer to the intl. evaluator's comment), and thus do not imply a change in the state of the scientific knowledge for the products. Measures according to Para 29 German Drug Law (AMG), section (1) concerning the approval status are not deemed necessary.

Reporter's comment:

039976196, 28.02.2008 DE-AMGEN-UK266181

Sender diagnosis: not reported

Narrative

Patient with a history of chronic obstructive pulmonary disease underwent a kidney transplant on an unspecified date. The patient also had endocarditis. The patient was receiving multiple immuno-suppressants and antibiotics. On 23-Feb-05 the patient developed an exanthem secondary to an unspecified drug. Treatment details were not provided. On 09-Mar-2005 the patient received Neupogen intravenously for neutropenia; lab values were not provided. Six days later the patient developed a fever; treatment details were not provided. Two days later the patient developed spots and "cockades" (rosettes) on the skin. On 19-Mar-2005 the patient presented with blisters on the skin. Three days later there was erosion of the lips. The next day the patient's eyes were red and she had tracheo-bronchial erosions. Treatment details were not provided. Subsequently the patient died. The physician reported the event to be toxic epidermal necrolysis with maculae. Additional information was not provided.

Sender's comment:

Toxic epidermal necrolysis has not been documented in the Amgen core safety data sheet for filgrastim.

A causal relationship between therapy and event is considered unlikely due to confounding by comorbidity or concurrent medication. Incomplete and lacking information does not permit a meaningful causality statement. The event of organ transplant, prior and concomitant use of antibiotics and drugs to suppress organ rejection are all known instigators of TEN (toxic epidermal necrolysis) and considered both confounders and co-suspects (in the case of the medications).

This individual case report does not change the safety profile of the product.

Reporter's comment:

039976196, 26.03.2008 DE-AMGEN-UK266181

Sender diagnosis: not reported

Narrative

Patient with a history of chronic obstructive pulmonary disease underwent a kidney transplant on an unspecified date. The patient also had endocarditis. The patient was receiving multiple immuno-suppressants and antibiotics. On 23-Feb-05 the patient developed an exanthem secondary to an unspecified drug. Treatment details were not provided. On 09-Mar-2005 the patient received Neupogen intravenously for neutropenia; lab values were not provided. Six days later the patient developed a fever; treatment details were not provided. Two days later the patient developed spots and "cockades" (rosettes) on the skin. On 19-Mar-2005 the patient presented with blisters on the skin. Three days later there was erosion of the lips. The next day the patient's eyes were red and she had tracheo-bronchial erosions. Treatment details were not provided. Subsequently the patient died. The physician reported the event to be toxic epidermal necrolysis with maculae. Additional information was not provided.

ADDITIONAL INFORMATION RECEIVED ON 13/MAR/2008: The German Centre for Severe Skin Toxicities commented that the cause of death in patients with severe skin toxicities like toxic epidermal necrolysis cannot be clearly determined and an autopsy is only performed in rare cases. Relying on statistical and epidemiological findings, a death that occurs within six weeks of the beginning of a severe skin toxicity is related to the severe skin toxicity and therefore an assumption is made that the severe skin toxicity is the cause of death if the patient dies within six weeks of the occurrence of a severe skin toxicity. The expert panel from the German Centre for Severe Skin Toxicities concluded that it is possible that the toxic epidermal necrolysis with maculae may have been caused by Neupogen.

Sender's comment:

Toxic epidermal necrolysis has not been documented in the Amgen core safety data sheet for filgrastim.

Incomplete and lacking information does not permit a meaningful causality statement. The event of organ transplant, prior and concomitant use of antibiotics and drugs to suppress organ rejection are all known instigators of TEN (toxic epidermal necrolysis) and considered both confounders and co-suspects (in the case of the medications).

This individual case report does not change the safety profile of the product.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08014010

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Male	Birth: privacy	Age: 10 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Erythema exsudativum multiforme	20041021	20041103	14 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 benuron paracetamol	Oral	/Day	20041014-20041016	Fever
2 mucosolvan ambroxol hydrochloride	Oral	/	20041014-20041020	Cough
			3 Day	
			7 Day	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Flu like symptoms			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08014056

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No						

Patient

Initials:	Sex: Male	Birth: privacy	Age: 14 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Skin erosion	20031126				Unknown
2 Conjunctivitis	20031126				Unknown
3 Lip erosion	20031125				Unknown
4 Itchy skin	20031125				Unknown
5 Skin erosion	20031125				Unknown
6 Herpes labialis	20031124				Unknown
7 Fever	20031124				Unknown
8 Infection	20031117				Unknown
9 Erythema exsudativum multiforme	20031124	20031213	20 Day		recovered/resolved
10 Pneumonia	20031125				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 acc acetylcysteine	Oral	/	-	Cough
2 benuron paracetamol	Oral	/	-	Flu
3 grippostad c caffeine; chlorphenamine maleate; paracetamol		/	-	Flu
4 roxithromycin roxithromycin		300 Mg milligram(s)/Day	-	Pyrexial cold

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08014064

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes	Yes	No	No	No	No		Yes

Patient

Initials:	Sex: Female	Birth: privacy	Age: 73 Year	weight (kg):	BMI_int:	Age fetus:
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Patientdeath

deathdate: 20050408	deathcause: Stevens Johnson syndrome
deathdate: 20050408	deathcause: Toxic epidermal necrolysis

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Blister	20050331				Unknown
2 Exanthema	20050317				Unknown
3 Nikolsky's sign	20050404				Unknown
4 Pain of skin	20050329				Unknown
5 Pruritus	20050328				Unknown
6 Skin lesion	20050323				Unknown
7 Stevens Johnson syndrome	20050329	20050408	11 Day	181 Day	fatal
8 Toxic epidermal necrolysis	20050329	20050408	11 Day	29 Day	fatal

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
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1	Benuron paracetamol	Rectal	/ Suppository	20050319-20050319	Fever 1 Day
2	Benuron paracetamol		500 Mg milligram(s)/	20050307-20050307	Fever 1 Day
3	Benuron paracetamol	Rectal	500 Mg milligram(s)/ Suppository	20050305-20050305	Fever 1 Day
4	Benuron paracetamol		500 Mg milligram(s)/	20050322-20050323	Fever 2 Day
5	Benuron paracetamol		500 Mg milligram(s)/	20050315-20050315	Fever 1 Day
6	clexane 0,6 ml enoxaparin		0.6 ml millilitre(s)/	20050222-20050307	Thrombosis prophylaxis 14 Day
7	Euglucon glibenclamide	Oral	1.75 Mg milligram(s)/Day	20050305-20050331	Diabetes mellitus 27 Day
8	Eusaprim forte sulfamethoxazole; trimethoprim		2* /Day	20050307-20050314	8 Day
9	Imodium loperamide hydrochloride		/	20050315-20050315	Diarrhea 1 Day
10	Imodium loperamide hydrochloride		/	20050308-20050308	Diarrhea 1 Day
11	Imodium loperamide hydrochloride		/	20050325-20050326	Diarrhea 2 Day
12	Konakion phytomenadione	Oral	10 Gtt drop(s)/ Oral drops	20050314-20050316	3 Day
13	Konakion phytomenadione	Oral	5 Gtt drop(s)/ Oral drops	20050309-20050310	2 Day
14	Myocholine 25 mg bethanechol		75 Mg milligram(s)/Day	-	Atonic bladder sphincter -
15	Novalgin metamizole		/	20050331-20050331	Pain 1 Day
16	Novalgin metamizole	Oral	80 Gtt drop(s)/Day Oral drops	20050309-20050314	Pain 6 Day
17	Pantozol i.v	Intravenous (not otherwis	80 Mg milligram(s)/Day Powder for injection	-	Prophylaxis -
18	pantozol 40 mg	Oral	40 Mg milligram(s)/ Gastro-resistant tablet	20050301-20050317	Prophylaxis 17 Day
19	Paspertin metoclopramide hydrochloride	Oral	60 Gtt drop(s)/Day Oral drops	20050310-20050318	Nausea 9 Day
20	Riopan magaldrate	Oral	/ Gel	20050307-20050307	Gastric pain 1 Day
21	Riopan magaldrate	Oral	/ Gel	-	Gastric pain -
22	Rocephin 2 g ceftriaxone sodium		2 G gram(s)/	20050322-20050325	Infection 4 Day
23	Saroten 50 mg amitriptyline hydrochloride		50 Mg milligram(s)/	20050301-20050318	Depression 18 Day

Drugs (concomitant)					
24	Actrapid insulin human	Subcutaneous	/	20050324-20050329	Diabetes mellitus
			Solution for injection	6 Day	
25	Aqua ad water for injection	Intravenous (not otherwis	/	20050323-20050323	Hypocalcemia
			Solution for injection	1 Day	
26	Avalox moxifloxacin hydrochloride	Intravenous (not otherwis	400 Mg milligram(s)/	20050331-20050331	Yersinia infection
			Injection	1 Day	
27	Ca-Gluconat calcium gluconate	Intravenous (not otherwis	/	20050323-20050323	Hypocalcemia
			Solution for injection	1 Day	
28	Calcium calcium		2* /Day	-	Hypocalcemia
				2 Day	
29	Calcium calcium		/	20050308-20050309	Hypocalcemia
				2 Day	
30	candio-hermal nystatin		/	20050317-20050317	Genital candida
				1 Day	
31	candio-hermal nystatin		/	20050326-20050326	Genital candida
				1 Day	
32	corangin nitrospray glyceryl trinitrate		/	20050327-20050327	Dyspnoea
				1 Day	
33	Decortin prednisone	Intravenous (not otherwis	100 Mg milligram(s)/Day	20050330-20050331	Skin disorder
				2 Day	
34	Electrolyt-153 calcium chloride hexahydrate	Intravenous (not otherwis	/	20050318-20050324	
			Solution for infusion	7 Day	
35	Fenistil dimetindene maleate	Intravenous (not otherwis	/ Day	20050328-20050328	Skin disorder
			Solution for injection	1 Day	
36	Fenistil dimetindene maleate	Oral	/	20050327-20050329	Skin disorder
			Tablet	3 Day	
37	GLIBENCLAMIDE glibenclamide	Oral	3.5 Mg milligram(s)/	20050305-20050331	Diabetes mellitus
				27 Day	
38	Haes 6% hetastarch; sodium chloride	Intravenous (not otherwis	/	20050304-20050305	Reaction unevaluable
			Solution for infusion	2 Day	
39	intramin acetylcysteine; amino acids nos; calcium chloride dihydrate; electrolytes nos; glucose; glucose monohydrate; potassium chloride; sodium glycerophosphate		500 ml millilitre(s)/	20050331-20050331	
				1 Day	
40	intramin acetylcysteine; amino acids nos; calcium chloride dihydrate; electrolytes nos; glucose; glucose monohydrate; potassium chloride; sodium glycerophosphate		500 ml millilitre(s)/	20050323-20050323	
				2 Day	
41	Maninil glibenclamide		1.75 Mg milligram(s)/	20050211-20050303	Diabetes mellitus
				21 Day	
42	Marcumar phenprocoumon		/	20050305-20050308	Thrombosis
				4 Day	
43	multibionta ascorbic acid; calcium pantothenate; cyanocobalamin; nicotinamide; pyridoxine hydrochloride; retinol palmitate; riboflavin; rutoside; thiamine hydrochloride; tocopheryl acetate	Intravenous (not otherwis	/	20050322-20050322	
			Solution for injection	1 Day	
44	nacl sodium chloride	Intravenous (not otherwis	500 ml millilitre(s)/	-	
			Solution for infusion	-	
45	sortis atorvastatin calcium		/	20040930-20050303	Hypercholesterolaemia
				5 Month	
46	Thioctacid thioctic acid		/	20050325-20050327	Polyneuropathy
				3 Day	
47	Urbason methylprednisolone		/	20050331-20050331	Collagenosis
				1 Day	

Drugs (concomitant)

48	Urbason	/	20050322-20050329
	methylprednisolone		8 Day
49	voltaren resinat	1 Df dosage form/Day	20050214-20050309 Pain
	diclofenac resinate		24 Day
50	xusal	/	- Pruritus
	levocetirizine dihydrochloride		-
51	Xylit	Intravenous (not otherwis 5 ml millilitre(s)/Hour	20050323-20050323
	xylitol	Solution for injection	1 Day

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Atonic bladder sphincter			Unknown	
Collagenosis			Unknown	
Depression			Unknown	
Diabetes mellitus			Unknown	
Diarrhoea			Unknown	
Dyspnoea			Unknown	
Emesis			Unknown	
Fever			Unknown	
Gastric pain			Unknown	
Genital candida			Unknown	
Hypercholesterolemia			Unknown	
Hypocalcaemia			Unknown	
Nausea			Unknown	
Pain			Unknown	
Polyneuropathy			Unknown	
Pruritus			Unknown	
Thrombosis			Unknown	
Yersinia infection			Unknown	

Primary source

Qualification: Physician observestudytype:
Literature:

Narratives (nur bei elektronischem Eingang)

[ROCHE, 18.02.2008 DE-ROCHE-545351](#)

Sender diagnosis: not reported

Narrative

INTENSIVE MONITORING STUDY

A 70 YEAR OLD FEMALE PATIENT EXPERIENCED FATAL STEVEN-JOHNSON SYNDROME AND FATAL TOXIC EPIDERMAL NECROLYSIS WHILE PARTICIPATING IN THE ABOVE STUDY.

ON 30 SEPTEMBER 2004, THE PATIENT COMMENCED ORAL AMITRIPTYLINE (SAROTEN) FOR DEPRESSION. ON 10 FEBRUARY 2005, THE PATIENT COMMENCED BETHANECHOL CHLORIDE (MYOCHOLINE) 25 MG THRICE DAILY FOR URINARY INCONTINENCE. TWELVE DAYS LATER, THE PATIENT COMMENCED ENOXAPARIN SODIUM (CLEXANE) 0.6 ML ONCE DAILY FOR THROMBOSIS PROPHYLAXIS. ON 1 MARCH 2005, THE PATIENT COMMENCED ORAL PANTOPRAZOLE (PANTOZOL) 40 MG ONCE DAILY FOR STOMACH PROTECTION. ON 5 MARCH 2005, SHE RECEIVED PARACETAMOL (BENURON) FOR FEVER AND ORAL GLIBENCLAMIDE FOR DIABETES MELLITUS. TWO DAYS LATER, SHE RECEIVED ORAL PARACETAMOL (BENURON) FOR FEVER, SULFAMETHOXAZOLE/TRIMETHOPRIM (EUSAPRIM) TWICE DAILY FOR SUSPECTED INFECTION AND ORAL MAGALDRATE (RIOPAN) FOR STOMACH PAIN. THE NEXT DAY, SHE RECEIVED LOPERAMIDE (IMODIUM) FOR DIARRHOEA. ON 9 MARCH 2005, THE PATIENT COMMENCED ORAL PHYTOMENADIONE (5-10 DROPS DAILY) FOR SUBSTITUTION AND ORAL METAMIZOLE SODIUM (NOVALGIN) FOR PAIN. THE FOLLOWING DAY, SHE RECEIVED ORAL METOCLOPRAMIDE (PASPERTIN) FOR NAUSEA. SEVEN DAYS LATER, THE PATIENT COMMENCED NYSTATIN (CANDIO-HERMAL) FOR SUSPECTED GENITAL YERSINIA INFECTION. THE FOLLOWING DAY, SHE RECEIVED INTRAVENOUS PANTOPRAZOLE (PANTOZOL) 40 MG TWICE DAILY FOR STOMACH PROTECTION, INTRAVENOUS SODIUM CHLORIDE AS CARRIER SOLUTION AND INTRAVENOUS ELECTROLYTES AS SUBSTITUTION. ON 22 MARCH 2005, THE PATIENT COMMENCED INTRAVENOUS CEFTRIAXONE 2 GRAMS DAILY FOR

INFECTION, INTRAVENOUS METOCLOPRAMIDE (PASPERTIN) ONCE DAILY FOR EMESIS, INTRAVENOUS MULTIVITAMINS AS SUBSTITUTION AND INTRAVENOUS METHYLPREDNISOLONE (URBASON) FOR SUSPECTED COLLAGEN EXACERBATION. THE PATIENT WAS PREVIOUSLY HOSPITALISED IN FEBRUARY 2005 AND MARCH 2005. ON 29 MARCH 2005, THE PATIENT WAS DIAGNOSED WITH STEVEN-JOHNSON SYNDROME AND TOXIC EPIDERMAL NECROLYSIS. CO-MANIFESTATION OF STEVEN-JOHNSON SYNDROME AND TOXIC EPIDERMAL NECROLYSIS OCCURRED ON 17 MARCH 2005 (EXANTHEMA), ON 24 MARCH 2005 (COCKADES AND STAINS), ON 28 MARCH 2005 (PRURITUS), ON 29 MARCH 2005 (PAIN OF THE SKIN). ON 31 MARCH 2005, THE PATIENT WAS TRANSFERRED TO INTENSIVE CARE UNITE AFTER THE FIRST OCCURRENCE OF BLISTERS AND EROSIONS. ON 4 APRIL 2005, NIKOLSKI TEST WAS POSITIVE AND THE PATIENT WAS SHIFTED TO THE CENTRE FOR BURN INJURIES. FOUR DAYS LATER, THE PATIENT DIED DUE TO STEVEN-JOHNSON SYNDROME AND TOXIC EPIDERMAL NECROLYSIS.

AT THE TIME OF THE PATIENT'S DEATH, ALL THE SUSPECT DRUGS HAD BEEN DISCONTINUED.

THE REPORTER ASSESSED THE EVENTS AS RELATED TO THE USE OF ALL THE SUSPECT DRUGS.

NO FURTHER INFORMATION WAS AVAILABLE.

Sender's comment:

CIOMS TEXT: FATAL STEVEN-JOHNSON SYNDROME AND TOXIC EPIDERMAL NECROLYSIS ARE LISTED IN THE COMPANY REFERENCE DOCUMENT (CDS) FOR CEFTRIAZONE WHILE NOT LISTED IN THE CDS OF GLIBENCLAMIDE AND ISPI OF PHENPROCOUMON. BASED ON THE INFORMATION RECEIVED FOR THIS CASE, A CAUSAL RELATIONSHIP BETWEEN THE REPORTED EVENTS AND TREATMENT WITH CEFTRIAZONE, PHENPROCOUMON AND GLIBENCLAMIDE IS ASSESSED AS POSSIBLE. COMPANY CAUSALITY WAS ASSESSED BASED ON THE NARANJO ALGORITHM.

HOWEVER, POSSIBLE ALTERNATIVE EXPLANATIONS INCLUDE USE OF PANTOPRAZOLE, PARACETAMOL FOR BOTH EVENTS. USE OF METAMIZOLE SODIUM IS AN ALTERNATIVE EXPLANATION FOR TOXIC EPIDERMAL NECROLYSIS AND NYSTATIN IS AN ALTERNATIVE EXPLANATION FOR STEVEN-JOHNSON SYNDROME.; ***LOCAL ASSESSMENT: SUBMITTED FOR CEFTRIAZONE

INTENSIVE MONITORING STUDY/ REGISCAR

LOCAL LABEL STATUS:

STEVEN-JOHNSON SYNDROME AND TOXIC EPIDERMAL NECROLYSIS LABELLED.

EINZELFALLBEOBACHTUNG/ KEIN HANDLUNGSBEDARF

Reporter's comment:

Sanofi-Aventis Deutschland GmbH, 21.02.2008 GB-AVENTIS-200811542GDDC

Sender diagnosis: not reported

Narrative

Initial report: This serious case from Germany was forwarded by our license partner Roche Products Ltd. (545351).

Intensive Monitoring Study

A 70-year-old female patient experienced fatal Steven-Johnson Syndrome and fatal toxic epidermal necrolysis while participating in the above study.

On 30-Sep-2004, the patient commenced oral amitriptyline (Saroten) for depression. On 10-Feb-05, the patient commenced bethanechol chloride (Myocholine) 25 mg thrice daily for urinary incontinence. Twelve days later, the patient commenced enoxaparin sodium (Clexane) 0.6 ml once daily for thrombosis prophylaxis. On 1-Mar-05, the patient commenced oral pantoprazole (Pantozol) 40 mg once daily for stomach protection. On 5-Mar-05, she received paracetamol (Benuron) for fever and oral glibenclamide for diabetes mellitus. Two days later, she received oral paracetamol (Benuron) for fever, sulfamethoxazole/trimethoprim (Eusaprim) twice daily for suspected infection and oral magaldrate (Riopan) for stomach pain. The next day, she received loperamide (Imodium) for diarrhea. On 9-Mar-05, the patient commenced oral phytomenadione (5-10 drops daily) for substitution and oral metamizole sodium (Novalgin) for pain. The following day, she received oral metoclopramide (Paspertin) for nausea. Seven days later, the patient commenced nystatin (Candio-Hermal) for suspected genital yersinia infection. The following day, she received intravenous pantoprazole (Pantozol) 40 mg twice daily for stomach protection, intravenous sodium chloride as carrier solution and intravenous electrolyte as substitution. On 22-Mar-05, the patient

intravenous ceftriaxone 2 grams daily for obscure infection, intravenous metoclopramide (Raspertin) once daily for emesis, intravenous multivitamins as substitution and intravenous methylprednisolone (Urbason) for suspected collagen exacerbation. The patient was previously hospitalized in February 2005 and March 2005. On 29-Mar-05, the patient was diagnosed with Steven-Johnson Syndrome and toxic epidermal necrolysis. Co-manifestation of

Steven-Johnson Syndrome and toxic epidermal necrolysis occurred on 17-Mar-05 (exanthema), on 24-Mar-05 (cockades and stains), on 28-Mar-05 (pruritus), on 29-Mar-05 (pain of the skin). On 31-Mar-05, the patient was transferred to intensive care unit after the first occurrence of blisters and erosions. On 4-April-05, the Nikolski test was positive and the patient was shifted to the centre for burn injuries. Four days later, the patient died due to Steven-Johnson Syndrome and toxic epidermal necrolysis.

At the time of the patient's death, all of the suspect drugs had been discontinued.

The reporter assessed the events as related to the use of all the suspect drugs.

No further information was available.

Based on the information received for this case, a causal relationship between the reported events and treatment with ceftriaxone, phenprocoumon and glibenclamide is assessed as possible. Company causality was assessed based on the Naranjo Algorithm. However, possible alternative explanations include use of pantoprazole, paracetamol for both events.

Use of metamizole sodium is an alternative explanation for toxic epidermal necrolysis and nystatin is an alternative explanation for Steven-Johnson Syndrome.

DRUG EVENT INFORMATION

Event: Steven-Johnson Syndrome/toxic epidermal necrolysis

Outcome: Death

Seriousness: Death

Onset date: 29-Mar-05

Relation to: glibenclamide/Clexane/Pantozol/myocholine/Riopan/Candio-hermal/NACL/electrolytes/Urbason/

Drug Continued: Insufficient info.

Drug Related (Roche): Yes

Drug Related (Rept): Yes

Relation to: ceftriaxone/phytomenadione/Pantozol/Saroten/Benuron/Eusaprim/Imodium/Novalgin/Paspertin/multibionta

Drug Continued: Therapy already completed

Drug Related (Roche): Yes

Drug Related (Rept): Yes

Sender's comment:

Sanofi-aventis Company Comment dated 19-FEB-2008: The development of Steven-Johnson Syndrome and toxic epidermal necrolysis with fatal outcome are alternatively explained by the concomitant use of any one of a number of medications used prior to the initial presentation of the events; these drugs include ceftriaxone, phytomenadione, pantoprazole, amitriptyline, enoxaparin, bethanechol, paracetamol, sulfamethoxazole+trimethoprim, loperamide, metamizole, metoclopramide and nystatin.

21-FEB-2008 Sanofi-Aventis Germany: This ICSR fulfils expedited reporting criteria, but the global validation is still in progress. The information presented here may therefore be incomplete or may be subject to modification. This non-approved version is being submitted to keep the regulatory timelines. A follow-up submission including the local evaluation will be made once the report has been finally validated.

Reporter's comment:

Sanofi-Aventis Deutschland GmbH, 25.02.2008 GB-AVENTIS-200811542GDDC

Sender diagnosis: not reported

Narrative

Initial report: This serious case from Germany was forwarded by our license partner Roche Products Ltd. (545351).

Intensive Monitoring Study

A 70-year-old female patient experienced fatal Steven-Johnson Syndrome and fatal toxic epidermal necrolysis while participating in the above study.

On 30-Sep-2004, the patient commenced oral amitriptyline (Saroten) for depression. On 10-Feb-05, the patient commenced bethanechol chloride (Myocholine) 25 mg thrice daily for urinary incontinence. Twelve days later, the patient commenced enoxaparin sodium (Clexane) 0.6 ml once daily for thrombosis prophylaxis. On 1-Mar-05, the patient commenced oral pantoprazole (Pantozol) 40 mg once daily for stomach protection. On 5-Mar-05, she received paracetamol (Benuron) for fever and oral glibenclamide for diabetes mellitus. Two days later, she received oral paracetamol (Benuron) for fever, sulfamethoxazole/trimethoprim (Eusaprim) twice daily for suspected infection and oral magaldrate (Riopan) for stomach pain. The next day, she received loperamide (Imodium) for diarrhea. On 9-Mar-05, the patient commenced oral phytomenadione (5-10 drops daily) for substitution and oral metamizole sodium (Novalgine) for pain. The following day, she received oral metoclopramide (Paspertin) for nausea. Seven days later, the patient commenced nystatin (Candio-Hermal) for suspected genital yersinia infection. The following day, she received intravenous pantoprazole (Pantozol) 40 mg twice daily for stomach protection, intravenous sodium chloride as carrier solution and intravenous electrolyte as substitution. On 22-Mar-05, the patient commenced intravenous ceftriaxone 2 grams daily for obscure infection, intravenous metoclopramide (Raspertin) once daily for emesis, intravenous multivitamins as substitution and intravenous methylprednisolone (Urbason) for suspected collagen exacerbation. The patient was previously hospitalized in February 2005 and March 2005. On 29-Mar-05, the patient was diagnosed with Steven-Johnson Syndrome and toxic epidermal necrolysis. Co-manifestation of Steven-Johnson Syndrome and toxic epidermal necrolysis occurred on 17-Mar-05 (exanthema), on 24-Mar-05 (cockades and stains), on 28-Mar-05 (pruritus), on 29-Mar-05 (pain of the skin). On 31-Mar-05, the patient was transferred to intensive care unit after the first occurrence of blisters and erosions. On 4-April-05, the Nikolski test was positive and the patient was shifted to the centre for burn injuries. Four days later, the patient died due to Steven-Johnson Syndrome and toxic epidermal necrolysis.

At the time of the patient's death, all of the suspect drugs had been discontinued.

The reporter assessed the events as related to the use of all the suspect drugs.

No further information was available.

Based on the information received for this case, a causal relationship between the reported events and treatment with ceftriaxone, phenprocoumon and glibenclamide is assessed as possible. Company causality was assessed based on the Naranjo Algorithm. However, possible alternative explanations include use of pantoprazole, paracetamol for both events.

Use of metamizole sodium is an alternative explanation for toxic epidermal necrolysis and nystatin is an alternative explanation for Steven-Johnson Syndrome.

DRUG EVENT INFORMATION

Event: Steven-Johnson Syndrome/toxic epidermal necrolysis

Outcome: Death

Seriousness: Death

Onset date: 29-Mar-05

Relation to: glibenclamide/Clexane/Pantozol/myocholine/Riopan/Candio-hermal/NACL/electrolytes/Urbason/

Drug Continued: Insufficient info.

Drug Related (Roche): Yes

Drug Related (Rept): Yes

Relation to: ceftriaxone/phytomenadione/Pantozol/Saroten/Benuron/Eusaprim/Imodium/Novalgine/Paspertin/multibionta

Drug Continued: Therapy already completed

Drug Related (Roche): Yes

Drug Related (Rept): Yes

Sender's comment:

Sanofi-aventis Company Comment dated 19-FEB-2008: The development of Steven-Johnson Syndrome and toxic epidermal necrolysis with fatal outcome are alternatively explained by the concomitant use of any one of a number of medications used prior to the initial presentation of the events; these drugs include ceftriaxone, phytomenadione, pantoprazole, amitriptyline, enoxaparin, bethanechol, paracetamol, sulfamethoxazole+trimethoprim, loperamide, metamizole, metoclopramide and nystatin.

25-FEB-2008 Sanofi Aventis Germany: FATAL STEVEN-JOHNSON SYNDROME and TOXIC EPIDERMAL NECROLYSIS are considered locally unlabelled for one or more suspected product(s). The facts presented here however are not very suggestive of a causal relationship (please refer to the intl. evaluator's comment), and thus do not imply a change in the state of the scientific knowledge for the product(s). Measures according to Para 29 German Drug Law (AMG), section (1) concerning the approval status are not deemed necessary.

Reporter's comment:

Janssen-Cilag, Germany, 26.02.2008 DE-JNJFOC-20080203703

Sender diagnosis: not reported

Narrative

This spontaneous report received from the European Registry of severe cutaneous adverse reactions via German Health Authority (BfArM) concerns a 70 year old female patient from Germany: A

The patient's medical history and concurrent conditions included: bladder disorder, suspected candida infection (genital), depression, diabetes mellitus, fever, hypercholesterolaemia, hypocalcaemia, nausea, stomach pain, suspected yersinia infection, polyneuropathy, dyspnoea, pyrexia and thrombosis. The patient's weight and height were not reported. The patient was treated with loperamide hydrochloride (unspecified, oral) 4 mg a day on 08-MAR-2005, 15-MAR-2005, and on 25-MAR-2005 to 26-MAR-2005 for diarrhoea. Non-company suspect drugs included: magaldrate for stomach pain, metamizole sodium for pain, phytomenadione for substitution, metoclopramide hydrochloride for nausea, nystatin for suspected genital candidal infection, pantoprazole sodium for stomach protection, ceftriaxone sodium for antibiotic prophylaxis, amitriptyline hydrochloride for depression, enoxaparin for thrombosis prophylaxis, bethanechol for bladder disorder, paracetamol for fever, glibenclamide for diabetes mellitus, and bactrim for suspected infection. Concomitant medications included calcium for hypocalcaemia, phenprocoumon for thrombosis, haes-steril for substitution, diclofenac sodium for pain, atorvastatin calcium for hypercholesterolaemia, glibenclamide for diabetes mellitus, sodium chloride for substitution, elektrolyt 153 for substitution, metoclopramide hydrochloride for emesis, multibionta for substitution, methylprednisolone for suspected exacerbation of collagenosis, xylitol for substitution, ca-gluconat for hypocalcaemia, aqua ad injectable, intramin for substitution, insulin human for diabetes mellitus, thioctic acid for polyneuropathy, isosorbide mononitrate for dyspnoea, dimetindene maleate for skin disorder, dimetindene maleate for skin disorder, levocetirizine dihydrochloride for pruritus, prednisone for skin disorder, metamizole sodium for pain, and moxifloxacin hydrochloride for suspected yersinia infection. On 17-MAR-2005, the patient developed an exanthema, on 23-MAR-2005, the patient developed maculae, on 28-MAR-2005, the patient developed pruritus, and on 29-MAR-2005, the patient developed stevens johnson syndrome and pain of the skin. On 04-APR-2005, Nikolsky's sign was positive. The patient died from Stevens Johnson Syndrome on 08-APR-2005. It was reported that the patient additionally suffered from a suspected yersinia infection.

It was unknown if an autopsy was performed. The reporter assessed the causality between the events and loperamide hydrochloride as possible, between pantoprazole sodium, amitriptyline hydrochloride, heparin-fraction, bethanechol, paracetamol, glibenclamide, magaldrate, phytomenadione, metoclopramide, ceftriaxone sodium, metamizole sodium and the events as possible, and between bactrim and the events as probable.

This report was serious (death).<dbtext>To_Char_NARRATIVE_2</dbtext>

Sender's comment:

Single case assessment:

STEVENS-JOHNSON SYNDROME: unexpected per RSI

RASH: expected per RSI

PRURITUS: expected per RSI

PAIN OF SKIN: unexpected per RSI

The benefit-risk-ratio remains unchanged based on the single case assessment of this case. No measures according to § 29 (1) AMG have to be taken.

Reporter's comment:

McNeil Germany, Local Safety Officers, 26.02.2008 DE-JNJFOC-20080203703

Sender diagnosis: not reported

Narrative

This spontaneous report received from the European Registry of severe cutaneous adverse reactions via German Health Authority (BfArM) concerns a 70 year old female patient from Germany: A

The patient's medical history and concurrent conditions included: bladder disorder, suspected candida infection (genital), depression, diabetes mellitus, fever, hypercholesterolaemia, hypocalcaemia, nausea, stomach pain, suspected yersinia infection, polyneuropathy, dyspnoea, pyrexia and thrombosis. The patient's weight and height were not reported. The patient was treated with loperamide hydrochloride (unspecified, oral) 4 mg a day on 08-MAR-2005, 15-MAR-2005, and on 25-MAR-2005 to 26-MAR-2005 for diarrhoea. Non-company suspect drugs included: magaldrate for stomach pain, metamizole sodium for pain, phytomenadione for substitution, metoclopramide hydrochloride for nausea, nystatin for suspected genital candidal infection, pantoprazole sodium for stomach protection, ceftriaxone sodium for antibiotic prophylaxis, amitriptyline hydrochloride for depression, enoxaparin for thrombosis prophylaxis, bethanechol for bladder disorder, paracetamol for fever, glibenclamide for diabetes mellitus, and

bactrim for suspected infection. Concomitant medications included calcium for hypocalcaemia, phenprocoumon for thrombosis, haes-steril for substitution, diclofenac sodium for pain, atorvastatin calcium for hypercholesterolaemia, glibenclamide for diabetes mellitus, sodium chloride for substitution, elektrolyt 153 for substitution, metoclopramide hydrochloride for emesis, multibionta for substitution, methylprednisolone for suspected exacerbation of collagenosis, xylitol for substitution, ca-gluconat for hypocalcaemia, aqua ad injectable, intramin for substitution, insulin human for diabetes mellitus, thioctic acid for polyneuropathy, isosorbide mononitrate for dyspnoea, dimetindene maleate for skin disorder, dimetindene maleate for skin disorder, levocetirizine dihydrochloride for pruritus, prednisone for skin disorder, metamizole sodium for pain, and moxifloxacin hydrochloride for suspected yersinia infection. On 17-MAR-2005, the patient developed an exanthema, on 23-MAR-2005, the patient developed maculae, on 28-MAR-2005, the patient developed pruritus, and on 29-MAR-2005, the patient developed stevens johnson syndrome and pain of the skin. On 04-APR-2005, Nikolsky's sign was positive. The patient died from Stevens Johnson Syndrome on 08-APR-2005. It was reported that the patient additionally suffered from a suspected yersinia infection.

It was unknown if an autopsy was performed. The reporter assessed the causality between the events and loperamide hydrochloride as possible, between pantoprazole sodium, amitriptyline hydrochloride, heparin-fraction, bethanechol, paracetamol, glibenclamide, magaldrate, phytomenadione, metoclopramide, ceftriaxone sodium, metamizole sodium and the events as possible, and between bactrim and the events as probable.

This report was serious (death).<dbtext>To_Char_NARRATIVE_2</dbtext>

Sender's comment:

Single case assessment (reported term/preferred term): LOPERAMIDE HYDROCHLORIDE
 STEVENS JOHNSON SYNDROME / STEVENS-JOHNSON SYNDROME: unexpected per local SmPc
 EXANTHEM / RASH: expected per local SmPc
 PRURITUS / PRURITUS: expected per local SmPc
 SKIN PAIN / PAIN OF SKIN: unexpected per local SmPc

The benefit-risk-ratio remains unchanged based on the single case assessment of this case. No measures according to § 29 (1) AMG have to be taken.

Reporter's comment:

Sanofi-Aventis Deutschland GmbH, 28.02.2008 DE-AVENTIS-200810471DE

Sender diagnosis: not reported

Narrative

Initial report

This case of a transitory form of Stevens-Johnson syndrome / toxic epidermal necrolysis (SJS/TEN) was received from the German health authority (DE-BFARM-08014063). It was reported by the German Documentation Centre for Severe Skin Reactions in Freiburg / Breisgau, Germany (code-no. 9160273). The centre is conducting a prospective study on the incidence and drug aetiology of severe skin reactions. Diagnosis of SJS/TEN was made by an expert panel who assessed it as a "definite case" of SJS/TEN. Causal relationship was assessed by the experts in Mar. 2006 using an algorithm.

As for metamizole (Novalgin) the causality was assessed as "well possible" (drops), for enoxaparin sodium (Clexane) and glibenclamide (Euglucon) it was assessed as "possible". For the other suspected drugs causality was assessed as :

"probable": trimethoprim + sulfamethoxazole,

"possible": pantoprazole, amitriptyline, bethanechol, paracetamol, magaldrate, loperamide, phytomenadione, metoclopramide, ceftriaxon.

Not suspected were: glibenclamide (Maninil, given until 14-Feb-2005), atorvastatin, diclofenac-colestyramine, polyethyl starch + sodium chloride, phenprocoumon, calcium, nystatin, sodium chloride, electrolytes, metoclopramide, multi-vitamins, methylprednisolone, xylitol, sodium gluconate, aqua ad injectabilia, amino acids+glucose+electrolytes, human insulin, alpha-liponic acid, glycerol trinitrate, dimetindene, levocetirizine, prednisone, metamizole (Novalgin, injection on 31-Mar-2005), moxifloxacin.

"Suspicion of acute Yersinia infection" was considered "possible" cause of SJS/TEN.

Epicrisis:

Patient: female, 70 years

Medical history: diabetes mellitus, hypercholesterolemia, depression, weakness of urinary bladder, state after thrombosis, suspicion of infection (acute Yersinia infection), stomach pain, hypocalcemia, suspicion of genital soor, polyneuropathy.

The patient was hospitalised on the internal ward from 10-Feb to 23-Feb-2005 and from 26-Feb to 01-Mar-2005. On 04-Mar-2005 she was hospitalised on the neurologic ward.

The patient was treated with enoxaparin 1x0.6ml/day from 22-Feb-2005 to 07-Mar-2005 and from 15-Mar-2005 to at least 31-Mar-2005 for prophylaxis of thrombosis, she took glibenclamide 1x1.75mg/day from 05-Mar-2005 to at least 31-Mar-2005 for diabetes mellitus. From 09-Mar-2005 to 14-Mar-2005 and again on 31-Mar-2005 (2 days after first symptoms of SJS/TEN) she was treated with metamizole (first 4x20 drops/day, on 31-Mar-2005 injection of 1 g). During hospitalisation the patient was also treated with a lot of other drugs.

On 17-Mar-2005 the patient developed exanthema (end unknown, not considered onset of SJS/TEN).

On 23-Mar-2005 target lesions and maculae were detected (not considered onset of SJS/TEN).

On 28-Mar-2005 the patient complained of pruritus.

On 29-Mar-2005 (reported as date of onset of SJS/TEN) the patient complained of skin pain. On 31-Mar-2005 skin blisters were detected. The patient was transferred to the intensive care ward.

On 04-Apr-2005 Nikolsky's sign was positive. The patient was transferred to the center for burn victims.

The outcome was reported as "death", date of end of event as "08-Apr-2005).

Laboratory: no values provided.

Sender's comment:

22-FEB-2008 Sanofi-Aventis Company Comment: Due to the compatible temporal relationship the causal role of metamizole sodium, glibenclamide and/or enoxaparin sodium cannot be excluded. However, the numerous concomitant medication as well as the suspected Yersinia infection may be discussed as possible alternative causes of the reported reaction.

22-FEB-2008 Sanofi Aventis Germany: TRANSIT.FORM STEVENS-JOHNSON SYND./TOX.EPID.NECROLYSIS-FATAL is considered locally unlabelled for metamizole-sodium (Novalgin) (due to fatal outcome) as well as for enoxaparin sodium and glibenclamide. The facts presented here however are not very suggestive of a causal relationship (please refer to the intl. evaluator's comment), and thus do not imply a change in the state of the scientific knowledge for the product. Measures according to Para 29 German Drug Law (AMG), section (1) concerning the approval status are not deemed necessary.

Reporter's comment:

Bayer Vital GmbH, 03.03.2008 DE-BAYER-200813641GPV

Sender diagnosis: not reported

Narrative

This solicited report was received on 19-Feb-2008 from a physician of the center of severe skin reactions in Germany via the regulatory health authority of Germany, referring to a 70 year-old female.

Another case reported by the same physician is documented at Bayer case number 200813705GPV.

Study description:

Post marketing observational study with Amitriptyline.

Study number: BAYO3211/PMS. Patient number: UNKNOWN.

Medical and drug history:

Concomitant conditions: diabetes mellitus, hypercholesterinemia, bladder atonia, fever, gastric pain, diarrhea, hypocalcemia, polyneuropathy, vomiting, dyspnea, thrombosis, suspicion exacerbating collagenosis, suspicion of genital candidiasis, and suspicion of Yersinia infection.

Concomitant drugs have been given. For details see section "Concomitant Drugs".

Suspect drugs, timing and conditions surrounding the onset of the reaction:

The patient received

- Saroten (amitriptyline hydrochloride) for depression from 30-Sep-2004 to 18-Mar-2005 at a total daily dose of 75 mg oral;
- Clexane (enoxaparin sodium) for thrombosis prophylaxis from 15-Feb-2005 to 31-Mar-2005, total daily dose was not reported, and from 22-Feb-2005 to 07-Mar-2005 at a total daily dose of 0.6 ML subcutaneous;
- Myocholine (bethanechol) for bladder atonia from 10-Feb-2005 to 31-Mar-2005 at a total daily dose of 75 mg oral;
- Euglucon (glibenclamide) for diabetes mellitus from 05-Mar-2005 to 31-Mar-2005 at a total daily dose of 1.625 mg oral;
- Eusaprim Forte for suspicion of infection from 07-Mar-2005 to 14-Mar-2005, total daily dose was not reported;
- Benuron (paracetamol) for fever on 05-Mar-2005 rectal, on 07-Mar-2005 at a total daily dose of 1000 mg oral, on 15-Mar-2005 at a total daily dose of 1000 mg oral, on 19-Mar-2005 rectal, and from 22-Mar-2005 to 23-Mar-2005 at a total daily dose of 1000 mg oral;
- Riopan (magaldrate) for gastric pain on 07-Mar-2005, and from 09-Mar-2005 to 31-Mar-2005, total daily dose was not reported;
- Pantozol (pantoprazole sodium) for gastric protection from 01-Mar-2005 to 17-Mar-2005 at a total daily dose of 40 mg oral, and from 18-Mar-2005 to 31-Mar-2005 at a total daily dose of 80 mg intravenous;
- IMODIUM [LOPERAMIDE HYDROCHLORIDE] (loperamide hydrochloride) for diarrhea on 08-Mar-2005, on 15-Mar-2005, and also administered from 25-Mar-2005 to 26-Mar-2005, total daily dose was not reported;
- Novalgin (metamizole sodium) for pain NOS from 09-Mar-2005 to 14-Mar-2005 at a total daily dose of 80 mg oral;
- Konakion (phytomenadione) for an unknown indication from 14-Mar-2005 to 16-Mar-2005, total daily dose was not reported;
- Rocephin (ceftriaxone) for an unknown indication from 02-Mar-2005 to 25-Mar-2005 at a total daily dose of 2 G intravenous;

- PASPERTIN [METOCLOPRAMIDE HYDROCHLORIDE] (metoclopramide hydrochloride) for nausea from 10-Mar-2005 to 18-Mar-2005 at a total daily dose of 60 mg oral.

The progression of the events and their outcome in the patient:

On 29-Mar-2005, the patient experienced TOXIC EPIDERMAL NECROLYSIS, which was considered serious due to life-threatening nature and death.

The event was treated with Fenistil, Decortin, Xusal, Novalgin.

During the same period patient experienced STEVENS-JOHNSON-SYNDROME (symptoms: exanthema, target lesions, pruritus, skin pain, skin blister, positive Nikolsky's sign), which was considered serious due to life-threatening nature and death.

Hospitalizations:

10-Feb-2005 to 23-Feb-2005: Internal Unit

26-Feb-2005 to 01-Mar-2005: Internal Unit

04-Mar-2005 to 31-Mar-2005: Neurology

31-Mar-2005 to 04-Apr-2005: Intensive Care Unit

Since 04-Apr-2005: Burn Center

Relevant details of fatal outcome:

Patient died on 08-Apr-2005. Autopsy information is unknown. The patient died from STEVENS-JOHNSON-SYNDROME, and TOXIC EPIDERMAL NECROLYSIS as stated by the reporter.

The original reporter's clinical assessment:

The investigator considered the diagnoses STEVENS-JOHNSON-SYNDROME and TOXIC EPIDERMAL NECROLYSIS probable for Eusaprim Forte, very possible for Novalgin oral and possible for all the other drugs.

No further information will be expected.

Sender's comment:

The reported events TOXIC EPIDERMAL NECROLYSIS and STEVENS-JOHNSON-SYNDROME, both considered serious as being of life-threatening condition and finally leading to patient's death, are not listed in the International product Information of Saroten (amitriptyline hydrochloride), Euglucon (glibenclamide), Eusaprim Forte, Benuron (paracetamol), Imodium (loperamide hydrochloride), Rocephin (ceftriaxone), Paspertin (metoclopramide hydrochloride). It is, however, to note that suspect drugs except Saroten (amitriptyline hydrochloride) are non-company drugs. The investigator considered the diagnoses STEVENS-JOHNSON-SYNDROME and TOXIC EPIDERMAL NECROLYSIS probable for Eusaprim Forte, very possible for Novalgin oral and possible for all the other drugs. This assessment is partially challenged by company. Suspect drugs Saroten (amitriptyline hydrochloride), Rocephin (ceftriaxone) and Paspertin (metoclopramide hydrochloride) have been withdrawn at least about 2 weeks prior to the event. Therefore, company considers a temporal and thus causal relationship to be highly unlikely. For the drugs Euglucon (glibenclamide), Eusaprim Forte, Benuron (paracetamol) and Imodium (loperamide hydrochloride) a causal relationship cannot be excluded as a temporal relationship is given. The concomitant suspected Yersinia infection is, however, a plausible alternative explanation for the event.

Reporter's comment:

Wyeth-Pharma GmbH, 03.03.2008 DE-WYE-H02714208

Sender diagnosis: not reported

Narrative

Information was received from the German Documentation Centre of Severe Skin Reactions via a regulatory authority, via a business partner (case # 80331), regarding a 71-year-old female who received Pantozol (pantoprazole tablet, delayed release) therapy and experienced overlapping form of Stevens-Johnson-syndrome/toxic epidermal necrolysis.

MEDICAL HISTORY:

The patient has a past history of hospitalisation (10-Feb-2005 until 23-Feb-2005 and from 26-Feb-2005 until 01-Mar-2005 - reason for admission unknown).

PRODUCT DETAILS:

Indication for Pantozol was "gastric protection". Therapy began on 01-Mar-2005 and was permanently discontinued on 17-Mar-2005. Dose regimen was 40 mg 1 time per day (oral).

Additional suspect medication included Pantozol (pantoprazole injection), Riopan (magaldrate), Maninil (glibenclamide), Sortis (atorvastatin calcium), Voltaren Resinat (diclofenac resinate), Saroten 50mg (amitriptyline hydrochloride), Clexane 0.6 ml (heparin-fraction, sodium salt), Myocholine 25 mg (bethanechol), Haes 6% (hyetellose/sodium chloride), Marcumar (phenprocoumon), Ben-U-Ron (paracetamol), Euglucon (glibenclamide), Eusaprim Forte (sulfamethoxazole/trimethoprim), Imodium (loperamide hydrochloride), Calcium, Novalgin (metamizole sodium), Konakion (phytomenadione),

Paspertin (metoclopramide hydrochloride), Candio-Hermal (nystatin), NaCl (sodium chloride) (injection), Elektrolyt-153 (calcium chloride hexahydrate/magnesium chloride anhydrous/potassium chloride/sodium acetate/sodium chloride), Multibionta (ascorbic acid/calcium pantothenate/cyanocobalamin/nicotinamide/pyridoxine hydrochloride/retinol palmitate/riboflavin/rutoside/thiamine hydrochloride/tocopheryl acetate), Rocephin 2g (ceftriaxone sodium), Urbason (methylprednisolone), Xylit (xylitol), Ca-Gluconat (calcium gluconate), Aqua ad (water for injection injection), Intramin, Actrapid Human (insulin human), Thioctacid (thioctic acid), Corangin Nitrospray (isosorbide mononitrate), Fenistil (dimetindene maleate), Xusal (levocetirizine dihydrochloride), Decortin (prednisone) and Avalox (moxifloxacin).

CONCOMITANT THERAPY:

Concomitant medications were not reported.

EVENT DETAILS:

The patient experienced overlapping form of Stevens-Johnson-syndrome/toxic epidermal necrolysis (toxic epidermal necrolysis) while receiving pantoprazole therapy. The patient was hospitalized from 10-Feb-2005 until 23-Feb-2005 and from 26-Feb-2005 until 01-Mar-2005 in a department for internal medicine (reason for admission unknown). On 01-Mar-2005, pantoprazole therapy was initiated. On 07-Mar-2005 magaldrate therapy was started. On 04-Mar-2005, the patient was transferred into the neurological department due to an unspecified reason. From 07-Mar-2005 to 14-Mar-2005, the patient received antibiotic treatment with sulfamethoxazole/trimethoprim due to suspected infection and on 22-Mar-2005, antibiotic treatment with ceftriaxone was initiated due to an infection of an unknown source. The patient repeatedly received paracetamol due to fever. On 17-Mar-2005, the patient started to experience exanthema. Oral pantoprazole was discontinued and IV pantoprazole was started. On 23-Mar-2005, the patient was found to have cockades and skin spots. On 28-Mar-2005, the patient developed pruritus. On 29-Mar-2005, the patient experienced skin pain. On 31-Mar-2005, the patient was found to have skin blisters. The patient was transferred to an intensive care unit. A yersinia infection was suspected. On 04-Apr-2005, Nicholsky's sign was positive and patient was transferred to a center of burn injury. The event was considered life-threatening. The patient died on an unspecified date. No autopsy was provided.

The cause of death was reported as toxic epidermal necrolysis.

RELATEDNESS ASSESSMENT:

The investigator(s) considered toxic epidermal necrolysis possibly related to the study product. The medical monitor(s) assessment for toxic epidermal necrolysis was unknown. The business partner considered toxic epidermal necrolysis possibly related to pantoprazole and magaldrate therapies.

Sender's comment:

Wyeth comment:

INITIAL (v1) 03-Mar-2008

TOXIC EPIDERMAL NECROLYSIS

Bericht aus dem "Dokumentationszentrum für schwere Hautreaktionen in Freiburg". Es geht um eine 71-jährige Patientin, die im Zusammenhang mit der Einnahme zahlreicher Medikamente, darunter auch Pantozol ein TEN entwickelte, eingestuft als "lebensgefährlich". Die Patientin ist zu einem unbekannten Zeitpunkt verstorben, als Todesursache wird TEN angegeben.

Es handelt sich um eine bekannte NW (US case expectedness) von Pantozol.

Ein kausaler Zusammenhang kann nicht ausgeschlossen werden.

Basierend auf der aktuellen Information führt dieser Bericht nicht zu einer Änderung des Nutzen- Risiko-Verhältnisses. Zu diesem Zeitpunkt sind keine weiteren Massnahmen vorgesehen. Sobald Änderungen hierzu bekannt werden, werden wir darüber unverzüglich und unaufgefordert informieren.

Dr. R. Jöstingmeier

Reporter's comment:

APHQWEB, 07.03.2008 DE-Nycomed GmbH-80331

Sender diagnosis: not reported

Narrative

This report was received from the German Documentation centre of severe skin reactions (ref no. 9160273) via the German Health authority (Bfarm ref. no. DE-BFARM-08014064).

An elderly female patient experienced overlapping form of Stevens-Johnson-syndrome/toxic epidermal necrolysis while receiving pantoprazole (Pantozol 40 mg po, Pantozol iv) and magaldrate (Riopan gel), a further Nycomed drug.

Further approximately 37 suspected drugs were administered.

Patient was hospitalized from 10-FEB-2005 until 23-FEB-2005 and from 26-FEB-2005 until 01-MAR-2005 in a department for internal medicine

(reason for admission unknown).

On 01-MAR-2005 pantoprazole (Pantozol 40 mg po) therapy was initiated. On 09-MAR-2005 magaldrate (Riopan) therapy started.

On 04-MAR-2005 patient was transferred into neurological department due to not reported reason.

From 7-MAR-2005 to 14-MAR the patient received antibiotic treatment with sulfamethoxazol/trimethoprim due to suspected infection and on 22-MAR-2005, antibiotic treatment with ceftriaxone was initiated due to infection of unknown source. The patient repeatedly received paracetamol due to fever. On 17-MAR-2005 patient started to experience exanthema. Pantoprazole po was discontinued and pantoprazole i.v. was started.

On 23-MAR-2005 patient was found to have cockades and skin spots.

On 28-MAR-2005 patient developed pruritus.

On 29-MAR-2005 patient experienced skin pain.

On 31-MAR-2005 patient was found to have skin blisters. The patient was transferred to a intensive care unit. A yersinia infection was suspected.

On 04-APR-2005 Nicholsky's sign was positive and patient was transferred to a center of burn injury.

Outcome: patient died (date of death not reported). No autopsy report was provided.

Concomitant diseases included gastric pain, hypercholesterinaemia, pain, depression, atonic bladder, thrombosis, fever, diabetes mellitus, diarrhoea, hypocalcaemia, nausea, suspicion of genital soor, emesis, polyneuropathy, dyspnoea, pruritus, suspicion of exacerbated collagenosis, suspicion of yersinia infection. With the information on hand Nycomed is now going to assess the case in order to come to a conclusion.

OVERLAPPING FORM OF STEVENS-JOHNSON-SYNDROME/TOXIC EPIDERMAL NECROLYSIS

This patient developed overlapping form of Stevens-Johnson-Syndrome/toxic epidermal necrolysis in a plausible temporal relationship with intake of pantoprazole and magaldrate (approximately 26 days after first reported intake of pantoprazole and about 20 days after first intake of magaldrate).

Concomitantly, he received multiple other drugs (37 further suspected drugs).

Trimethoprim/sulfamethoxazol treatment was "highly suspected". The reporter assessed causality to pantoprazole treatment (po. and i.v.) as possible and causality to magaldrate treatment as possible.

Stevens-Johnson-Syndrome/toxic epidermal necrolysis (overlapping form) is a rare skin disorder characterized by skin blisters, epidermal detachment of some part of body surface. It can be drug-induced or due to infectious diseases.

Several drugs have been observed to cause Stevens-Johnson-Syndrome and toxic epidermal necrolysis. Toxic bullous skin reactions up to acute disseminated epidermal necrolysis have also been observed in very rare cases with pantoprazole and other proton pump inhibitors. Although Stevens-Johnson-Syndrome and toxic epidermal necrolysis are considered labelled for pantoprazole, this event was considered to be unlabeled, since all pantoprazole cases with "fatal outcome" are unlabeled. However, the mortality is high in general for patients suffering under toxic epidermal necrolysis and SJS (20-25%; Mockenhaupt et al 2008).

For magaldrate, Stevens-Johnson-Syndrome and toxic epidermal necrolysis are not considered labelled.

However, this case is highly confounded as the patient received a various number of further suspected medications, including antibiotics for which the reported reactions have also been observed. This patient presented with fever without knowledge of focus some time before onset of the reported events, a yersinia infection was suspected. Pantoprazole therapy was switched from oral form to iv. presentation although first skin symptoms had already appeared.

Time to onset was basically plausible for pantoprazole po. and magaldrate, temporal relationship seems less plausible for pantoprazole iv; however in the presence of multiple other suspected drugs and a concomitant yersinia infection, a single trigger for the reported event is hard to determine. Therefore Nycomed assesses this case as "possibly related" for pantoprazole.

Magaldrate as an antacid containing magnesium and aluminium is not absorbed and therefore it has no systemic effect. Only very small amounts of magnesium and aluminium are released from magaldrate and only a small part of the released ions are absorbed. Therefore the causal relationship is considered to be "unlikely related" for magaldrate.

Sender's comment:

This report is not considered to have any influence on current knowledge regarding the safety profile of pantoprazole and magaldrate and has no effect on the status of those drugs as reflected by the formal product characteristics.

Reporter's comment:

APHQWEB, 10.03.2008 DE-Nycomed GmbH-80331

Sender diagnosis: not reported

Narrative

This report was received from the German Documentation centre of severe skin reactions (ref no. 9160273) via the German Health authority (Bfarm ref. no. DE-BFARM-08014064).

An elderly female patient experienced overlapping form of Stevens-Johnson-syndrome/toxic epidermal necrolysis while receiving pantoprazole (Pantozol 40 mg po, Pantozol iv) and magaldrate (Riopan gel), a further Nycomed drug.

Further approximately 37 suspected drugs were administered.

Patient was hospitalized from 10-FEB-2005 until 23-FEB-2005 and from 26-FEB-2005 until 01-MAR-2005 in a department for internal medicine (reason for admission unknown).

On 01-MAR-2005 pantoprazole (Pantozol 40 mg po) therapy was initiated. On 09-MAR-2005 magaldrate (Riopan) therapy started.

On 04-MAR-2005 patient was transferred into neurological department due to not reported reason.

From 7-MAR-2005 to 14-MAR the patient received antibiotic treatment with sulfamethoxazol/trimethoprim due to suspected infection and on 22-MAR-2005, antibiotic treatment with ceftriaxone was initiated due to infection of unknown source. The patient repeatedly received paracetamol due to fever. On 17-MAR-2005 patient started to experience exanthema. Pantoprazole po was discontinued and pantoprazole i.v. was started.

On 23-MAR-2005 patient was found to have cockades and skin spots.

On 28-MAR-2005 patient developed pruritus.

On 29-MAR-2005 patient experienced skin pain.

On 31-MAR-2005 patient was found to have skin blisters. The patient was transferred to a intensive care unit. A yersinia infection was suspected.

On 04-APR-2005 Nicholsky's sign was positive and patient was transferred to a center of burn injury.

Outcome: patient died (date of death not reported). No autopsy report was provided.

Concomitant diseases included gastric pain, hypercholesterinaemia, pain, depression, atonic bladder, thrombosis, fever, diabetes mellitus, diarrhoea, hypocalcaemia, nausea, suspicion of genital soor, emesis, polyneuropathy, dyspnoea, pruritus, suspicion of exacerbated collagenosis, suspicion of yersinia infection. With the information on hand Nycomed is now going to assess the case in order to come to a conclusion.

OVERLAPPING FORM OF STEVENS-JOHNSON-SYNDROME/TOXIC EPIDERMAL NECROLYSIS

This patient developed overlapping form of Stevens-Johnson-Syndrome/toxic epidermal necrolysis in a plausible temporal relationship with intake of pantoprazole and magaldrate (approximately 26 days after first reported intake of pantoprazole and about 20 days after first intake of magaldrate). Concomitantly, he received multiple other drugs (37 further suspected drugs).

Trimethoprim/sulfamethoxalol treatment was "highly suspected". The reporter assessed causality to pantoprazole treatment (po. and i.v.) as possible and causality to magaldrate treatment as possible.

Stevens-Johnson-Syndrome/toxic epidermal necrolysis (overlapping form) is a rare skin disorder characterized by skin blisters, epidermal detachment of some part of body surface. It can be drug-induced or due to infectious diseases.

Several drugs have been observed to cause Stevens-Johnson-Syndrome and toxic epidermal necrolysis. Toxic bullous skin reactions up to acute disseminated epidermal necrolysis have also been observed in very rare cases with pantoprazole and other proton pump inhibitors. Although Stevens-Johnson-Syndrome and toxic epidermal necrolysis are considered labelled for pantoprazole, this event was considered to be unlabeled, since all pantoprazole cases with "fatal outcome" are unlabeled. However, the mortality is high in general for patients suffering under toxic epidermal necrolysis and SJS (20-25%; Mockenhaupt et al 2008).

For magaldrate, Stevens-Johnson-Syndrome and toxic epidermal necrolysis are not considered labelled.

However, this case is highly confounded as the patient received a various number of further suspected medications, including antibiotics for which the reported reactions have also been observed. This patient presented with fever without knowledge of focus some time before onset of the reported events, a yersinia infection was suspected. Pantoprazole therapy was switched from oral form to iv. presentation although first skin symptoms had already appeared.

Time to onset was basically plausible for pantoprazole po. and magaldrate, temporal relationship seems less plausible for pantoprazole iv; however in the presence of multiple other suspected drugs and a concomitant yersinia infection, a single trigger for the reported event is hard to determine. Therefore Nycomed assesses this case as "possibly related" for pantoprazole.

Magaldrate as an antacid containing magnesium and aluminium is not absorbed and therefore it has no systemic effect. Only very small amounts of magnesium and aluminium are released from magaldrate and only a small part of the released ions are absorbed. Therefore the causal relationship is considered to be "unlikely related" for magaldrate.

Sender's comment:

This report is not considered to have any influence on current knowledge regarding the safety profile of pantoprazole and magaldrate and has no effect on the status of those drugs as reflected by the formal product characteristics.

Reporter's comment:

APHQWEB, 11.03.2008 DE-Nycomed GmbH-80331

Sender diagnosis: not reported

Narrative

This report was received from the German Documentation centre of severe skin reactions (ref no. 9160273) via the German Health authority (Bfarm ref. no. DE-BFARM-08014064).

An elderly female patient experienced overlapping form of Stevens-Johnson-syndrome/toxic epidermal necrolysis while receiving pantoprazole (Pantozol 40 mg po, Pantozol iv) and magaldrate (Riopan gel), a further Nycomed drug.

Further approximately 37 suspected drugs were administered.

Patient was hospitalized from 10-FEB-2005 until 23-FEB-2005 and from 26-FEB-2005 until 01-MAR-2005 in a department for internal medicine (reason for admission unknown).

On 01-MAR-2005 pantoprazole (Pantozol 40 mg po) therapy was initiated. On 09-MAR-2005 magaldrate (Riopan) therapy started.

On 04-MAR-2005 patient was transferred into neurological department due to not reported reason.

From 7-MAR-2005 to 14-MAR the patient received antibiotic treatment with sulfamethoxazol/trimethoprim due to suspected infection and on 22-MAR-2005, antibiotic treatment with ceftriaxone was initiated due to infection of unknown source. The patient repeatedly received paracetamol due to fever. On 17-MAR-2005 patient started to experience exanthema. Pantoprazole po was discontinued and pantoprazole i.v. was started.

On 23-MAR-2005 patient was found to have cockades and skin spots.

On 28-MAR-2005 patient developed pruritus.

On 29-MAR-2005 patient experienced skin pain.

On 31-MAR-2005 patient was found to have skin blisters. The patient was transferred to a intensive care unit. A yersinia infection was suspected.

On 04-APR-2005 Nicholsky's sign was positive and patient was transferred to a center of burn injury.

Outcome: patient died (date of death not reported). No autopsy report was provided.

Concomitant diseases included gastric pain, hypercholesterinaemia, pain, depression, atonic bladder, thrombosis, fever, diabetes mellitus, diarrhoea,

hypocalcaemia, nausea, suspicion of genital soor, emesis, polyneuropathy, dyspnoea, pruritus, suspicion of exacerbated collagenosis, suspicion of yersinia infection. With the information on hand Nycomed is now going to assess the case in order to come to a conclusion.

OVERLAPPING FORM OF STEVENS-JOHNSON-SYNDROME/TOXIC EPIDERMAL NECROLYSIS

This patient developed overlapping form of Stevens-Johnson-Syndrome/toxic epidermal necrolysis in a plausible temporal relationship with intake of pantoprazole and magaldrate (approximately 26 days after first reported intake of pantoprazole and about 20 days after first intake of magaldrate). Concomitantly, he received multiple other drugs (37 further suspected drugs).

Trimethoprim/sulfamethoxalol treatment was "highly suspected". The reporter assessed causality to pantoprazole treatment (po. and i.v.) as possible and causality to magaldrate treatment as possible.

Stevens-Johnson-Syndrome/toxic epidermal necrolysis (overlapping form) is a rare skin disorder characterized by skin blisters, epidermal detachment of some part of body surface. It can be drug-induced or due to infectious diseases.

Several drugs have been observed to cause Stevens-Johnson-Syndrome and toxic epidermal necrolysis. Toxic bullous skin reactions up to acute disseminated epidermal necrolysis have also been observed in very rare cases with pantoprazole and other proton pump inhibitors. Although Stevens-Johnson-Syndrome and toxic epidermal necrolysis are considered labelled for pantoprazole, this event was considered to be unlabeled, since all pantoprazole cases with "fatal outcome" are unlabeled. However, the mortality is high in general for patients suffering under toxic epidermal necrolysis and SJS (20-25%; Mockenhaupt et al 2008).

For magaldrate, Stevens-Johnson-Syndrome and toxic epidermal necrolysis are not considered labelled.

However, this case is highly confounded as the patient received a various number of further suspected medications, including antibiotics for which the reported reactions have also been observed. This patient presented with fever without knowledge of focus some time before onset of the reported events, a yersinia infection was suspected. Pantoprazole therapy was switched from oral form to iv. presentation although first skin symptoms had already appeared.

Time to onset was basically plausible for pantoprazole po. and magaldrate, temporal relationship seems less plausible for pantoprazole iv; however in the presence of multiple other suspected drugs and a concomitant yersinia infection, a single trigger for the reported event is hard to determine. Therefore Nycomed assesses this case as "possibly related" for pantoprazole.

Magaldrate as an antacid containing magnesium and aluminium is not absorbed and therefore it has no systemic effect. Only very small amounts of magnesium and aluminium are released from magaldrate and only a small part of the released ions are absorbed. Therefore the causal relationship is considered to be "unlikely related" for magaldrate.

Sender's comment:

This report is not considered to have any influence on current knowledge regarding the safety profile of pantoprazole and magaldrate and has no effect on the status of those drugs as reflected by the formal product characteristics.

Reporter's comment:

APHQWEB, 12.03.2008 DE-Nycomed GmbH-80331

Sender diagnosis: not reported

Narrative

This report was received from the German Documentation centre of severe skin reactions (ref no. 9160273) via the German Health authority (Bfarm ref. no. DE-BFARM-08014064).

An elderly female patient experienced overlapping form of Stevens-Johnson-syndrome/toxic epidermal necrolysis while receiving pantoprazole (Pantozol 40 mg po, Pantozol iv) and magaldrate (Riopan gel), a further Nycomed drug.

Further approximately 37 suspected drugs were administered.

Patient was hospitalized from 10-FEB-2005 until 23-FEB-2005 and from 26-FEB-2005 until 01-MAR-2005 in a department for internal medicine (reason for admission unknown).

On 01-MAR-2005 pantoprazole (Pantozol 40 mg po) therapy was initiated. On 09-MAR-2005 magaldrate (Riopan) therapy started.

On 04-MAR-2005 patient was transferred into neurological department due to not reported reason.

From 7-MAR-2005 to 14-MAR the patient received antibiotic treatment with sulfamethoxazol/trimethoprim due to suspected infection and on 22-MAR-2005, antibiotic treatment with ceftriaxone was initiated due to infection of unknown source. The patient repeatedly received paracetamol due to fever. On 17-MAR-2005 patient started to experience exanthema. Pantoprazole po was discontinued and pantoprazole i.v. was started.

On 23-MAR-2005 patient was found to have cockades and skin spots.

On 28-MAR-2005 patient developed pruritus.

On 29-MAR-2005 patient experienced skin pain.

On 31-MAR-2005 patient was found to have skin blisters. The patient was transferred to a intensive care unit. A yersinia infection was suspected.

On 04-APR-2005 Nicholsky's sign was positive and patient was transferred to a center of burn injury.

Outcome: patient died (date of death not reported). No autopsy report was provided.

Concomitant diseases included gastric pain, hypercholesterinaemia, pain, depression, atonic bladder, thrombosis, fever, diabetes mellitus, diarrhoea, hypocalcaemia, nausea, suspicion of genital soor, emesis, polyneuropathy, dyspnoea, pruritus, suspicion of exacerbated collagenosis, suspicion of yersinia infection. With the information on hand Nycomed is now going to assess the case in order to come to a conclusion.

OVERLAPPING FORM OF STEVENS-JOHNSON-SYNDROME/TOXIC EPIDERMAL NECROLYSIS

This patient developed overlapping form of Stevens-Johnson-Syndrome/toxic epidermal necrolysis in a plausible temporal relationship with intake of pantoprazole and magaldrate (approximately 26 days after first reported intake of pantoprazole and about 20 days after first intake of magaldrate). Concomitantly, he received multiple other drugs (37 further suspected drugs). Trimethoprim/sulfamethoxalol treatment was "highly suspected". The reporter assessed causality to pantoprazole treatment (po. and i.v.) as possible and causality to magaldrate treatment as possible.

Stevens-Johnson-Syndrome/toxic epidermal necrolysis (overlapping form) is a rare skin disorder characterized by skin blisters, epidermal detachment of some part of body surface. It can be drug-induced or due to infectious diseases.

Several drugs have been observed to cause Stevens-Johnson-Syndrome and toxic epidermal necrolysis. Toxic bullous skin reactions up to acute disseminated epidermal necrolysis have also been observed in very rare cases with pantoprazole and other proton pump inhibitors. Although Stevens-Johnson-Syndrome and toxic epidermal necrolysis are considered labelled for pantoprazole, this event was considered to be unlabeled, since all pantoprazole cases with "fatal outcome" are unlabeled. However, the mortality is high in general for patients suffering under toxic epidermal necrolysis and SJS (20-25%; Mockenhaupt et al 2008).

For magaldrate, Stevens-Johnson-Syndrome and toxic epidermal necrolysis are not considered labelled.

However, this case is highly confounded as the patient received a various number of further suspected medications, including antibiotics for which the reported reactions have also been observed. This patient presented with fever without knowledge of focus some time before onset of the reported events, a yersinia infection was suspected. Pantoprazole therapy was switched from oral form to iv. presentation although first skin symptoms had already appeared.

Time to onset was basically plausible for pantoprazole po. and magaldrate, temporal relationship seems less plausible for pantoprazole iv; however in the presence of multiple other suspected drugs and a concomitant yersinia infection, a single trigger for the reported event is hard to determine. Therefore Nycomed assesses this case as "possibly related" for pantoprazole.

Magaldrate as an antacid containing magnesium and aluminium is not absorbed and therefore it has no systemic effect. Only very small amounts of magnesium and aluminium are released from magaldrate and only a small part of the released ions are absorbed. Therefore the causal relationship is considered to be "unlikely related" for magaldrate.

Sender's comment:

This report is not considered to have any influence on current knowledge regarding the safety profile of pantoprazole and magaldrate and has no effect on the status of those drugs as reflected by the formal product characteristics.

Reporter's comment:

APHQWEB, 13.03.2008 DE-Nycomed GmbH-80331

Sender diagnosis: not reported

Narrative

This report was received from the German Documentation centre of severe skin reactions (ref no. 9160273) via the German Health authority (Bfarm ref. no. DE-BFARM-08014064).

An elderly female patient experienced overlapping form of Stevens-Johnson-syndrome/toxic epidermal necrolysis while receiving pantoprazole (Pantozol 40 mg po, Pantozol iv) and magaldrate (Riopan gel), a further Nycomed drug.

Further approximately 37 suspected drugs were administered.

Patient was hospitalized from 10-FEB-2005 until 23-FEB-2005 and from 26-FEB-2005 until 01-MAR-2005 in a department for internal medicine (reason for admission unknown).

On 01-MAR-2005 pantoprazole (Pantozol 40 mg po) therapy was initiated. On 09-MAR-2005 magaldrate (Riopan) therapy started.

On 04-MAR-2005 patient was transferred into neurological department due to not reported reason.

From 7-MAR-2005 to 14-MAR the patient received antibiotic treatment with sulfamethoxazol/trimethoprim due to suspected infection and on 22-MAR-2005, antibiotic treatment with ceftriaxone was initiated due to infection of unknown source. The patient repeatedly received paracetamol due to fever. On 17-MAR-2005 patient started to experience exanthema. Pantoprazole po was discontinued and pantoprazole i.v. was started.

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On 31-MAR-2005 patient was found to have skin blisters. The patient was transferred to a intensive care unit. A yersinia infection was suspected.

On 04-APR-2005 Nicholsky's sign was positive and patient was transferred to a center of burn injury.

Outcome: patient died (date of death not reported). No autopsy report was provided.

Concomitant diseases included gastric pain, hypercholesterinaemia, pain, depression, atonic bladder, thrombosis, fever, diabetes mellitus, diarrhoea, hypocalcaemia, nausea, suspicion of genital soor, emesis, polyneuropathy, dyspnoea, pruritus, suspicion of exacerbated collagenosis, suspicion of yersinia infection. With the information on hand Nycomed is now going to assess the case in order to come to a conclusion.

OVERLAPPING FORM OF STEVENS-JOHNSON-SYNDROME/TOXIC EPIDERMAL NECROLYSIS

This patient developed overlapping form of Stevens-Johnson-Syndrome/toxic epidermal necrolysis in a plausible temporal relationship with intake of pantoprazole and magaldrate (approximately 26 days after first reported intake of pantoprazole and about 20 days after first intake of magaldrate). Concomitantly, he received multiple other drugs (37 further suspected drugs).

Trimethoprim/sulfamethoxalol treatment was "highly suspected". The reporter assessed causality to pantoprazole treatment (po. and i.v.) as possible and causality to magaldrate treatment as possible.

Stevens-Johnson-Syndrome/toxic epidermal necrolysis (overlapping form) is a rare skin disorder characterized by skin blisters, epidermal detachment of some part of body surface. It can be drug-induced or due to infectious diseases.

Several drugs have been observed to cause Stevens-Johnson-Syndrome and toxic epidermal necrolysis. Toxic bullous skin reactions up to acute disseminated epidermal necrolysis have also been observed in very rare cases with pantoprazole and other proton pump inhibitors. Although Stevens-Johnson-Syndrome and toxic epidermal necrolysis are considered labelled for pantoprazole, this event was considered to be unlabeled, since all pantoprazole cases with "fatal outcome" are unlabeled. However, the mortality is high in general for patients suffering under toxic epidermal necrolysis and SJS (20-25%; Mockenhaupt et al 2008).

For magaldrate, Stevens-Johnson-Syndrome and toxic epidermal necrolysis are not considered labelled.

However, this case is highly confounded as the patient received a various number of further suspected medications, including antibiotics for which the reported reactions have also been observed. This patient presented with fever without knowledge of focus some time before onset of the reported

events, a yersinia infection was suspected. Pantoprazole therapy was switched from oral form to iv. presentation although first skin symptoms had already appeared.

Time to onset was basically plausible for pantoprazole po. and magaldrate, temporal relationship seems less plausible for pantoprazole iv; however in the presence of multiple other suspected drugs and a concomitant yersinia infection, a single trigger for the reported event is hard to determine. Therefore Nycomed assesses this case as "possibly related" for pantoprazole.

Magaldrate as an antacid containing magnesium and aluminium is not absorbed and therefore it has no systemic effect. Only very small amounts of magnesium and aluminium are released from magaldrate and only a small part of the released ions are absorbed. Therefore the causal relationship is considered to be "unlikely related" for magaldrate.

Sender's comment:

This report is not considered to have any influence on current knowledge regarding the safety profile of pantoprazole and magaldrate and has no effect on the status of those drugs as reflected by the formal product characteristics.

Reporter's comment:

H Lundbeck A/S, 14.03.2008 DE-LUNDBECK-DKLU1031200

Sender diagnosis: not reported

Narrative

Case reference number DKLU1031200 , is a clinical trial case Non-HLu Study : Post marketing observational study with Amitriptyline. Study number: BAYO3211. Evaluated by Bayer and submitted by H. Lundbeck A/S based on a pharmacovigilance agreement signed for a co-development programme.

This case concerns a 70 year old Female administered Amitriptyline hydrochloride 75 mg po daily from 30/Sep/2004 to 18/Mar/2005 for depression, Co-suspected drugs included Clexane (enoxaparin sodium), Myocholine (bethanechol), Euglucon (glibenclamide), Eusaprim Forte, Senuron (paracetamol), Riopan (magaldrate), - Pantozol (pantoprazole sodium), IMODIUM (loperamide hydrochloride), Novalgin (metamizole sodium), Konakion (phytomenadione), Rooephin (oeftriaxone) Paspertin (netoclopramide hydrochloride)

On 29/Mar/2005, the patient experienced Toxic epidermal necrolysis, which was considered serious due to life-threatening nature and death. The event was treated with Fenistil, Decortin, Xusal, Nowaigin.

During the same period patient experienced Stevens Johnson syndrome (symptoms: exanthema, target lesions, pruritus, skin pain, skin blister, positive Nikolsky's sign), which was considered serious due to life-threatening nature and death.

The patient was Hospitalized 10/Feb/2005 to 23/Feb/2005: Internal Unit, 26/Feb/2005 to 01/Mar/2005: Internal Unit, 04/Mar/2005 to 31/Mar/2005: Neurology, 31/Mar/2005 to 04/Apr/2005: Intensive Care Unit, Since 04/Apr/2005: Burn Center

On 08/Apr/2005 The patient died. Autopsy information is unknown. The patient died from STEVENS-JOHNSON-SYNDROME, and TOXIC EPIDERMAL NECROLYSIS as stated by the reporter.

The investigator considered the diagnoses STEVENS-JOHNSON-SYNDROME and TOXIC EPIDERMAL NECROLYSIS

The patient's medical history comprised Concomitant Diseases: diabetes mellitus, hypercholesterolemia, bladder atony, fever, gastric pain, diarrhea, hypocalcemia, polyneuropathy, vomiting, dyspnea, thrombosis, suspicion exacerbating collagenosis, suspicion of; genital candidiasis, and suspicion of Yersinia infection.

Concomitant medication was not reported.

Additional information has been requested and if new relevant information is received the case will be re-submitted.

Sender's comment:

Stevens-Johnson-Syndrome, and Toxic epidermal necrolysis are not listed according to core SPC for Amitriptyline. Although Steven Johnson Synd. may appear as late as six days after dechallenge of drug a time relationship with amitriptyline is not suggestive as the event occurred 11 days after last dose. Severe, potentially fatal, skin reactions including toxic epidermal necrolysis and the Stevens-Johnson syndrome have occurred in patients

treated with suspected drug sulfonamides (eusaprim forte). Other susp. drug Novalgin has been considered responsible for a case of drug-induced toxic epidermal necrolysis (Micromedex). The investigator considered the events Stevens-Johnson-Syndrome, and Toxic epidermal necrolysis probable for eusaprim forte, very possible for Novalgin Oral and possible for all other drugs. Medical history of various diseases and susp. Med included 11 drugs. Confounding factors include suspicion of Yersenia infection. Caus. relat. to amitrypt. not suggestive. . Company causality: Cannot be ruled out. Listedness equals expectedness. Causal relationship cannot be ruled out. This single, individual case report does not change the benefit-risk ratio of the drug. No measures are currently considered necessary by the MAH

Reporter's comment:

APHQWEB, 17.03.2008 DE-Nycomed GmbH-80331

Sender diagnosis: not reported

Narrative

This report was received from the German Documentation centre of severe skin reactions (ref no. 9160273) via the German Health authority (Bfarm ref. no. DE-BFARM-08014064).

An elderly female patient experienced overlapping form of Stevens-Johnson-syndrome/toxic epidermal necrolysis while receiving pantoprazole (Pantozol 40 mg po, Pantozol iv) and magaldrate (Riopan gel), a further Nycomed drug.

Further approximately 37 suspected drugs were administered.

Patient was hospitalized from 10-FEB-2005 until 23-FEB-2005 and from 26-FEB-2005 until 01-MAR-2005 in a department for internal medicine (reason for admission unknown).

On 01-MAR-2005 pantoprazole (Pantozol 40 mg po) therapy was initiated. On 09-MAR-2005 magaldrate (Riopan) therapy started.

On 04-MAR-2005 patient was transferred into neurological department due to not reported reason.

From 7-MAR-2005 to 14-MAR the patient received antibiotic treatment with sulfamethoxazol/trimethoprim due to suspected infection and on 22-MAR-2005, antibiotic treatment with ceftriaxone was initiated due to infection of unknown source. The patient repeatedly received paracetamol due to fever. On 17-MAR-2005 patient started to experience exanthema. Pantoprazole po was discontinued and pantoprazole i.v. was started.

On 23-MAR-2005 patient was found to have cockades and skin spots.

On 28-MAR-2005 patient developed pruritus.

On 29-MAR-2005 patient experienced skin pain.

On 31-MAR-2005 patient was found to have skin blisters. The patient was transferred to a intensive care unit. A yersinia infection was suspected.

On 04-APR-2005 Nicholsky's sign was positive and patient was transferred to a center of burn injury.

Outcome: patient died (date of death not reported). No autopsy report was provided.

Concomitant diseases included gastric pain, hypercholesterinaemia, pain, depression, atonic bladder, thrombosis, fever, diabetes mellitus, diarrhoea, hypocalcaemia, nausea, suspicion of genital soor, emesis, polyneuropathy, dyspnoea, pruritus, suspicion of exacerbated collagenosis, suspicion of yersinia infection. With the information on hand Nycomed is now going to assess the case in order to come to a conclusion.

OVERLAPPING FORM OF STEVENS-JOHNSON-SYNDROME/TOXIC EPIDERMAL NECROLYSIS

This patient developed overlapping form of Stevens-Johnson-Syndrome/toxic epidermal necrolysis in a plausible temporal relationship with intake of pantoprazole and magaldrate (approximately 26 days after first reported intake of pantoprazole and about 20 days after first intake of magaldrate). Concomitantly, he received multiple other drugs (37 further suspected drugs).

Trimethoprim/sulfamethoxalol treatment was "highly suspected". The reporter assessed causality to pantoprazole treatment (po. and i.v.) as possible and causality to magaldrate treatment as possible.

Stevens-Johnson-Syndrome/toxic epidermal necrolysis (overlapping form) is a rare skin disorder characterized by skin blisters, epidermal detachment of some part of body surface. It can be drug-induced or due to infectious diseases.

Several drugs have been observed to cause Stevens-Johnson-Syndrome and toxic epidermal necrolysis. Toxic bullous skin reactions up to acute disseminated epidermal necrolysis have also been observed in very rare cases with pantoprazole and other proton pump inhibitors. Although Stevens-Johnson-Syndrome and toxic epidermal necrolysis are considered labelled for pantoprazole, this event was considered to be unlabeled, since all pantoprazole cases with "fatal outcome" are unlabeled. However, the mortality is high in general for patients suffering under toxic epidermal necrolysis and SJS (20-25%; Mockenhaupt et al 2008).

For magaldrate, Stevens-Johnson-Syndrome and toxic epidermal necrolysis are not considered labelled.

However, this case is highly confounded as the patient received a various number of further suspected medications, including antibiotics for which the reported reactions have also been observed. This patient presented with fever without knowledge of focus some time before onset of the reported events, a yersinia infection was suspected. Pantoprazole therapy was switched from oral form to iv. presentation although first skin symptoms had already appeared.

Time to onset was basically plausible for pantoprazole po. and magaldrate, temporal relationship seems less plausible for pantoprazole iv; however in the presence of multiple other suspected drugs and a concomitant yersinia infection, a single trigger for the reported event is hard to determine. Therefore Nycomed assesses this case as "possibly related" for pantoprazole.

Magaldrate as an antacid containing magnesium and aluminium is not absorbed and therefore it has no systemic effect. Only very small amounts of magnesium and aluminium are released from magaldrate and only a small part of the released ions are absorbed. Therefore the causal relationship is considered to be "unlikely related" for magaldrate.

Sender's comment:

This report is not considered to have any influence on current knowledge regarding the safety profile of pantoprazole and magaldrate and has no effect on the status of those drugs as reflected by the formal product characteristics.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08014070

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes		Yes			Yes		Yes

Patient

Initials:	Sex: Female	Birth: privacy	Age: 81 Year	weight (kg):	BMI_int:	Age fetus:
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Patientdeath

deathdate: 20050804	deathcause: Stevens Johnson syndrome
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Bullae	20050723				fatal
2 Fever	20050723			37 Day	fatal
3 Oral pain	20050724			38 Day	fatal
4 Pimples	20050722				fatal
5 Pruritis	20050722			36 Day	fatal
6 Stevens Johnson syndrome	20050722	20050804	14 Day	36 Day	fatal
7 Skin rash	20050722			36 Day	fatal
8 Blisters	20050723		14 Day	37 Day	fatal
9 Itching	20050722		14 Day		fatal
10 Macule	20050722		14 Day		fatal
11 Pyrexia NOS	20050722		14 Day		fatal
12 Lip erosion	20050724		14 Day	38 Day	fatal
13 Lip disorder	20050724				fatal

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
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DE-BFARM-08014070

1	ACERBON lisinopril	Oral	5 Mg milligram(s)/Day	20050628- -	Heart failure
2	AGGRENOX acetylsalicylic acid; dipyridamole	Oral	2 Df dosage form/Day	20050723-20050723 1440 Minute	Atrial fibrillation
3	ALLOPURINOL allopurinol	Oral	150 Mg milligram(s)/ Day	20050701- -	Hyperuricaemia
4	ASS acetylsalicylic acid	Oral	100 Mg milligram(s)/Day	20050617-20050722 36 Day	Atrial fibrillation
5	BELOC-ZOK metoprolol succinate	Oral	1.5 Df dosage form/	- -	Heart failure
6	BELOC-ZOK metoprolol succinate	Oral	2 Df dosage form/Day	20040101-20050723 18 Month	Heart failure
7	BIFITERAL lactulose	Oral	20 ml millilitre(s)/Day Syrup	20050701-20050715 15 Day	Constipation
8	CIPRAMIL citalopram hydrochloride	Oral	20 Mg milligram(s)/ Day	20050712- -	Depression
9	DIGIMERCK digitoxin	Oral	0.07 Mg milligram(s)/Day	20040101-20050722 18 Month	Heart failure
10	FAVISTAN thiamazole	Oral	20 Mg milligram(s)/	- -	Hyperthyroidism
11	FAVISTAN thiamazole	Oral	40 Mg milligram(s)/	20050706- -	Hyperthyroidism
12	HEPARIN heparin	Subcutaneous	15000 lu international unit	20050628- -	Thrombosis prophylaxis
13	Isosorbiddinitrat isosorbide dinitrate	Oral	2 Df dosage form/Day	20050722-20050722 1440 Minute	Hypertension
14	MCP metoclopramide hydrochloride	Rectal	/	20050717-20050718 2 Day	Nausea
15	MOVICOL macrogol; potassium chloride; sodium bicarbonate; sodium chloride	Unknown	/	20050716- -	Constipation
16	NATRIUMFLUORID "ACO LAKEMEDEL" sodium fluoride	Oral	6 Df dosage form/Day	20050720- -	Electrolyte and fluid balance disorder NOS
17	NOVALGIN metamizole	Oral	30 Gtt drop(s)/Day	20050711-20050711 1440 Minute	Pain
18	NOVALGIN metamizole	Oral	30 Gtt drop(s)/Day	20050713-20050713 1440 Minute	Pain
19	PANTOZOL pantoprazole	Oral	80 Mg milligram(s)/Day	20050714- -	Prophylaxis of NSAID gastric ulceration
20	PANTOZOL pantoprazole	Oral	80 Mg milligram(s)/Day Gastro-resistant tablet	20050617-20050628 12 Day	Prophylaxis of NSAID gastric ulceration
21	PARACETAMOL paracetamol	Rectal	1000 Mg milligram(s)/Day Suppository	20050723-20050723 1440 Minute	Fever
22	stangyl trimipramine maleate	Oral	25 Mg milligram(s)/Day	20050718-20050721 4 Day	Depression
23	sterofundin electrolytes nos	Intravenous (not otherwis /		20050717-20050717 1440 Minute	Volume depletion
24	sterofundin electrolytes nos	Intravenous (not otherwis /		20050719-20050720 2 Day	Volume depletion
25	sterofundin electrolytes nos	Intravenous (not otherwis /		20050722-20050722 1440 Minute	Volume depletion
26	TOREM torasemide	Oral	10 Mg milligram(s)/	20040101- -	Heart failure
27	TOREM torasemide	Oral	40 Mg milligram(s)/Day	- -	Heart failure
28	TRENTAL pentoxifylline	Oral	600 Mg milligram(s)/	20050609-20050722	

44 Day

Drugs (concomitant)

29	Natriumchlorid sodium chloride	Oral	3* /2 Day	20050720-20050723 Hyponatremia
30	Nitrospray glyceryl trinitrate	Sublingual	/	4 Day 20050722-20050722 Hypertension
				1 Day

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
TIA			Unknown	
Cardiac flutter			Unknown	
Nausea			Unknown	
Hyponatraemia			Unknown	
Hyperuricaemia			Unknown	
Hyperthyroidism			Unknown	
Hypertension			Unknown	
Fever			Unknown	
Depression			Unknown	
Constipation			Unknown	
Cardiac insufficiency			Unknown	
Atrial fibrillation			Unknown	
Disorder circulatory system			Unknown	
Heart failure			Unknown	unspecified
Hypernatraemia			Unknown	
Osmolality increased			Unknown	
Vomiting			Unknown	
Hospitalization NOS			Unknown	in a rehabilitation center
Pain			Unknown	

Primary source

Qualification: Physician observestudytype:

Literature:

Narratives (nur bei elektronischem Eingang)Sanofi-Aventis Deutschland GmbH, 22.02.2008 DE-AVENTIS-200810473DE

Sender diagnosis: not reported

Narrative

Initial report

This case of toxic Stevens-Johnson syndrome (SJS) was received from the German health authority (DE-BFARM-08014070) . It was reported by the German Documentation Centre for Severe Skin Reactions in Freiburg / Breisgau, Germany (code-no. 9170316). The centre is conducting a prospective study on the incidence and drug aetiology of severe skin reactions. Diagnosis of SJS was made by an expert panel who assessed it as a "definite case" of SJS. Causal relationship was assessed by the experts in Sep. 2005 using an algorithm.

As for metamizole (Novalgin) the causality was assessed as "well possible", for pentoxifylline (Trental) it was assessed as "possible". For the other suspected drugs causality was assessed as :

"highly probable": allopurinol

"possible": lisinopril, pantoprazole, macrogol + electrolytes, heparin sodium, acetylsalicylic acid, lactulose, thiamazole, citalopram, metoclopramide, Not suspected were: digitoxin, metoprolol, torasemide, electrolyte solution, trimipramine, sodium chloride, glycerol trinitrate, paracetamol, dipyridamol + acetylsalicylic acid.

Epicrisis:

Patient: female, 81 years

Medical history: cardiac insufficiency, atrial fibrillation, hypertension, hyperthyroidism, hyperuricemia, depression.

From 17-Jun-2005 to 28-Jun-2006 hospitalisation on internal ward.

From 28-Jun-2005 to 24-Jul-2005 hospitalisation for rehabilitation.

The patient was treated with pentoxifylline 600mg orally from 09-Jun-2005 to 22-Jul-2005 for state after transient ischemic attack, and took 30 drops of metamizole on 11-Jul-2005 and 13-Jul-2005 for pain. Before and during hospitalisation the patient was also treated with a lot of other drugs (among others allopurinol 150 mg/day from 01-Jul-2005 to at least 23-Jul-2005 for hyperuricemia).

On 22-Jul-2005 the patient developed pruritus, target lesions and maculae, on 23-Jul-2005 fever and skin blisters.

On 24-Jul-2005 she developed erosions of her lips and complained of oral pain. The patient was transferred to the dermatologic wards on the same day where she stayed until 04-Aug-2005.

The outcome was reported as "death", date of end of event was reported as 04-Aug-2005.

Laboratory: no values provided.

Sender's comment:

20-FEB-2008 Sanofi-Aventis Company Comment: Based only on the compatible temporal relationship the causal role of metamizole and /or pentoxifylline cannot be excluded. However, in this case the numerous other drugs may be discussed as possible alternative cause of the reaction.

20-FEB-2008 Sanofi Aventis Germany: STEVENS-JOHNSON SYNDROME - FATAL is considered locally unlabelled (due to fatal outcome) for metamizole sodium and pentoxifylline. The facts presented here however are not very suggestive of a causal relationship (please refer to the intl. evaluator's comment), and thus do not imply a change in the state of the scientific knowledge for the products. Measures according to Para 29 German Drug Law (AMG), section (1) concerning the approval status are not deemed necessary.

Reporter's comment:

NORGINECDs, 25.02.2008 DE-Norgine-2005-0001101

Sender diagnosis: not reported

Narrative

Regulatory Authority Report FATAL - Serious - Unlabelled

Regulatory case received via GERMANY Regulatory Authority BfArM. Regulatory Authority Ref Number: DE-BFARM-08014070

Case 2005-0001101 (0.0)

An 81 year old female received MOVICOL (polyethylene glycol 3350 & electrolytes) at an unknown daily dose from 16-Jul-2005 for constipation.

6 days later on 22-Jul-2005 she is reported to have experienced STEVENS JOHNSON SYNDROME, PRURITUS, SPOTS. On the 23-Jul-2005 she experienced FEVER, SKIN BULLAE, and then on 24-Jul-2005 EROSIVE LESIONS AT THE LIPS and ORAL PAIN. The events resulted in DEATH at an unknown time later.

The reporter has marked Movicol as 'continued', this refers to the drug treatment of when the events occurred. It is however not known if they were continued or withdrawn during the events.

Other suspect drugs: none

The events were treated with: Paracetamol

Outcome: The patient died at an unknown time later

Concomitant medication: It should be noted that except for the drug paracetamol, it is not reported whether the comedication listed below was concomitant drugs or event treatment drugs.

Digimerck (digitoxin)

Acerbon (lisinopril)

Beloc-Zok (metoprolol)

Torem (torasemide)

Trental (pentoxifylline)

Pantozol (pantoprazol)

Heparin

ASS (acetylsalicylic acid)

Allopurinol

Favistan (thiamazole)

Novalgin (metamizole sodium)

Cipramil (citalopram hydrobromide)

MCP (metoclopramide)

Sterofundin

Stangyl (trimipramine maleate)

Natriumchlorid (sodium chloride)
 Nitrospray (glycerol nitrate)
 Aggrenox (asasantin)

Relevant medical history:

Cardiac insufficiency status after TIA, constipation, arterial fibrillation, hyperuricemia, hyperthyreosis, pain (not specified), depression, nausea, hyponatremia, hypertension, fever

Reporter comments:

Patient was hospitalised from 17-Jun-2005 until 28-Jun-2005 in the internal medicine, from 28-Jun until 24-Jul-2005 in rehabilitation and from 24-Jul-2005 until 04-Aug-2005 in dermatological clinic. On 22-Jul-2005 she experienced Stevens-Johnson-Syndrome. Outcome was fatal.

Another reporter's (physician) comments: Causality with Movicol is possible. There might be more relationship to the comedication Allopurinol. Therapy data after the 23-Jul-2005 are not reported, only the period before a reaction occurred is observed.

Sender's comment:

Stevens - Johnson syndrome may be associated with infection, malignancy, or be drug induced or idiopathic. Other than the fact that the patient had a fever, which may have pre-dated the event or be a symptom of it, no information is given on potential non-drug causes. Despite the fact that the patient was receiving a large number of drugs, none of them are commonly recognised as causing Stevens - Johnson syndrome, although it is noted that the reporter considered that allopurinol, which is known to rarely cause Stevens Johnson syndrome, may be a precipitant. The dates of treatment with allopurinol are not given. However it is known that Stevens Johnson syndrome may occur some weeks after treatment is initiated.

We have not had any previous reports of Stevens Johnson syndrome in association with Movicol or any PEG based products, nor are there any such reports in the literature. Macrogol does not undergo any receptor site interactions, and therefore despite the relatively close temporal relationship, causality is considered unlikely.

Case status: Further information has been requested and will be made available upon receipt.

Reporter's comment:

NOVARTIS, 26.02.2008 DE-002147023-PHBS2008DE02560

Sender diagnosis: not reported

Narrative

Health Authority report via the German Documentation centre of severe skin reaction, via Altana Pharma (80327) received on 22 Feb 2008: This patient experienced Stevens Johnson syndrome, whilst receiving Pantozol (pantoprazole 40 mg) for gastric protection for about five weeks. A further 20 drugs were suspected. From 17 Jun 2005 to 28 Jun 2008, the patient was hospitalized in a department for internal medicine for a not reported reason. This day, pantoprazole therapy was started.

From 28 Jun 2005 to 24 Jul 2005 the patient was staying in a rehabilitation centre. Pantoprazole therapy was interrupted from date of admission and re-started on 14 Jul 2005. On 22 Jul 2005, approximately eight days after the re-introduction of pantoprazole, the patient presented with pruritus, cockades and skin spots. On 23 Jul 2005, the patient developed a fever and skin blisters. On 24 Jul 2005, the patient was found to have lip erosions and oral pain. On this day she was transferred to a dermatology clinic. No concomitant infection was mentioned.

The patient died (exact date of death was not provided). No autopsy report was provided. Medical history and concomitant diseases included cardiac insufficiency, state after TIA, constipation, atrial fibrillation and flutter, hyperuricaemia, hyperthyreosis, pain, depression, nausea, hyponatraemia and hypertension. The reporter causality was possibly related.

Sender's comment:

Serious spontaneous report, SJS [death] assessed as unlisted according to the Basic Prescribing Information.

It is Novartis' policy to assess a case with a fatal outcome as unlisted unless the label specifically mentions death as a possible outcome of the listed risks. All spontaneous reports are considered suspected for reporting purposes.

Reporter's comment:

NOVARTIS, 26.02.2008 DE-002147023-PHBS2008DE02560

Sender diagnosis: not reported

Narrative

Health Authority report via the German Documentation centre of severe skin reaction, via Altana Pharma (80327) received on 22 Feb 2008: This patient experienced Stevens Johnson syndrome, whilst receiving Pantozol (pantoprazole 40 mg) for gastric protection for about five weeks. A further 20 drugs were suspected. From 17 Jun 2005 to 28 Jun 2008, the patient was hospitalized in a department for internal medicine for a not reported reason. This day, pantoprazole therapy was started.

From 28 Jun 2005 to 24 Jul 2005 the patient was staying in a rehabilitation centre. Pantoprazole therapy was interrupted from date of admission and re-started on 14 Jul 2005. On 22 Jul 2005, approximately eight days after the re-introduction of pantoprazole, the patient presented with pruritus, cockades and skin spots. On 23 Jul 2005, the patient developed a fever and skin blisters. On 24 Jul 2005, the patient was found to have lip erosions and oral pain. On this day she was transferred to a dermatology clinic. No concomitant infection was mentioned.

The patient died (exact date of death was not provided). No autopsy report was provided. Medical history and concomitant diseases included cardiac insufficiency, state after TIA, constipation, atrial fibrillation and flutter, hypericuriaemia, hyperthyreosis, pain, depression, nausea, hyponatraemia and hypertension. The reporter causality was possibly related.

Sender's comment:

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It is Novartis' policy to assess a case with a fatal outcome as unlisted unless the label specifically mentions death as a possible outcome of the listed risks. All spontaneous reports are considered suspected for reporting purposes.

Reporter's comment:NOVARTIS, 26.02.2008 DE-002147023-PHBS2008DE02560

Sender diagnosis: not reported

Narrative

Health Authority report via the German Documentation centre of severe skin reaction, via Altana Pharma (80327) received on 22 Feb 2008: This patient experienced Stevens Johnson syndrome, whilst receiving Pantozol (pantoprazole 40 mg) for gastric protection for about five weeks. A further 20 drugs were suspected. From 17 Jun 2005 to 28 Jun 2008, the patient was hospitalized in a department for internal medicine for a not reported reason. This day, pantoprazole therapy was started.

From 28 Jun 2005 to 24 Jul 2005 the patient was staying in a rehabilitation centre. Pantoprazole therapy was interrupted from date of admission and re-started on 14 Jul 2005. On 22 Jul 2005, approximately eight days after the re-introduction of pantoprazole, the patient presented with pruritus, cockades and skin spots. On 23 Jul 2005, the patient developed a fever and skin blisters. On 24 Jul 2005, the patient was found to have lip erosions and oral pain. On this day she was transferred to a dermatology clinic. No concomitant infection was mentioned.

The patient died (exact date of death was not provided). No autopsy report was provided. Medical history and concomitant diseases included cardiac insufficiency, state after TIA, constipation, atrial fibrillation and flutter, hypericuriaemia, hyperthyreosis, pain, depression, nausea, hyponatraemia and hypertension. The reporter causality was possibly related.

Sender's comment:

Serious spontaneous report, SJS [death] assessed as unlisted according to the Basic Prescribing Information.

It is Novartis' policy to assess a case with a fatal outcome as unlisted unless the label specifically mentions death as a possible outcome of the listed risks. All spontaneous reports are considered suspected for reporting purposes.

Reporter's comment:

NOVARTIS, 26.02.2008 DE-002147023-PHBS2008DE02560

Sender diagnosis: not reported

Narrative

Health Authority report via the German Documentation centre of severe skin reaction, via Altana Pharma (80327) received on 22 Feb 2008: This patient experienced Stevens Johnson syndrome, whilst receiving Pantozol (pantoprazole 40 mg) for gastric protection for about five weeks. A further 20 drugs were suspected. From 17 Jun 2005 to 28 Jun 2008, the patient was hospitalized in a department for internal medicine for a not reported reason. This day, pantoprazole therapy was started.

From 28 Jun 2005 to 24 Jul 2005 the patient was staying in a rehabilitation centre. Pantoprazole therapy was interrupted from date of admission and re-started on 14 Jul 2005. On 22 Jul 2005, approximately eight days after the re-introduction of pantoprazole, the patient presented with pruritus, cockades and skin spots. On 23 Jul 2005, the patient developed a fever and skin blisters. On 24 Jul 2005, the patient was found to have lip erosions and oral pain. On this day she was transferred to a dermatology clinic. No concomitant infection was mentioned.

The patient died (exact date of death was not provided). No autopsy report was provided. Medical history and concomitant diseases included cardiac insufficiency, state after TIA, constipation, atrial fibrillation and flutter, hypericuriaemia, hyperthyreosis, pain, depression, nausea, hyponatraemia and hypertension. The reporter causality was possibly related.

Sender's comment:

Serious spontaneous report, SJS [death] assessed as unlisted according to the Basic Prescribing Information.

It is Novartis' policy to assess a case with a fatal outcome as unlisted unless the label specifically mentions death as a possible outcome of the listed risks. All spontaneous reports are considered suspected for reporting purposes.

Reporter's comment:NORGINECDS, 27.02.2008 DE-Norgine-2005-0001101

Sender diagnosis: not reported

Narrative

Regulatory Authority Report FATAL - Serious - Unlabelled

Regulatory case received via GERMANY Regulatory Authority BfArM. Regulatory Authority Ref Number: DE-BFARM-08014070

Case 2005-0001101 (0.0)

An 81 year old female received MOVICOL (polyethylene glycol 3350 & electrolytes) at an unknown daily dose from 16-Jul-2005 for constipation.

6 days later on 22-Jul-2005 she is reported to have experienced STEVENS JOHNSON SYNDROME, PRURITUS, SPOTS. On the 23-Jul-2005 she experienced FEVER, SKIN BULLAE, and then on 24-Jul-2005 EROSIVE LESIONS AT THE LIPS and ORAL PAIN. The events resulted in DEATH at an unknown time later.

The reporter has marked Movicol as 'continued', this refers to the drug treatment of when the events occurred. It is however not known if they were continued or withdrawn during the events.

Other suspect drugs: none

The events were treated with: Paracetamol

Outcome: The patient died at an unknown time later

Concomitant medication: It should be noted that except for the drug paracetamol, it is not reported whether the comedication listed below was concomitant drugs or event treatment drugs.

Digimerck (digitoxin)

Acerbon (lisinopril)

Beloc-Zok (metoprolol)

Torem (torasemide)

Trental (pentoxifylline)

Pantazol (pantoprazol)

Heparin

ASS (acetylsalicylic acid)

Allopurinol

Favistan (thiamazole)
 Novalgin (metamizole sodium)
 Cipramil (citalopram hydrobromide)
 MCP (metoclopramide)
 Sterofundin
 Stangyl (trimipramine maleate)
 Natriumchlorid (sodium chloride)
 Nitrospray (glycerol nitrate)
 Aggrenox (asasantin)

Relevant medical history:

Cardiac insufficiency status after TIA, constipation, atrial fibrillation, hyperuricemia, hyperthyreosis, pain (not specified), depression, nausea, hyponatremia, hypertension, fever

Reporter comments:

Patient was hospitalised from 17-Jun-2005 until 28-Jun-2005 in the internal medicine, from 28-Jun until 24-Jul-2005 in rehabilitation and from 24-Jul-2005 until 04-Aug-2005 in dermatological clinic. On 22-Jul-2005 she experienced Stevens-Johnson-Syndrome. Outcome was fatal.

Another reporter's (physician) comments: Causality with Movicol is possible. There might be more relationship to the comedication Allopurinol. Therapy data after the 23-Jul-2005 are not reported, only the period before a reaction occurred is observed.

Sender's comment:

Corrected report: Atrial fibrillation amended in Relevant medical history.

Stevens - Johnson syndrome may be associated with infection, malignancy, or be drug induced or idiopathic. Other than the fact that the patient had a fever, which may have pre-dated the event or be a symptom of it, no information is given on potential non-drug causes. Despite the fact that the patient was receiving a large number of drugs, none of them are commonly recognised as causing Stevens - Johnson syndrome, although it is noted that the reporter considered that allopurinol, which is known to rarely cause Stevens Johnson syndrome, may be a precipitant. The dates of treatment with allopurinol are not given. However it is known that Stevens Johnson syndrome may occur some weeks after treatment is initiated.

We have not had any previous reports of Stevens Johnson syndrome in association with Movicol or any PEG based products, nor are there any such reports in the literature. Macrogol does not undergo any receptor site interactions, and therefore despite the relatively close temporal relationship, causality is considered unlikely.

Case status: Further information has been requested and will be made available upon receipt.

Reporter's comment:

Temmler Pharma GmbH & Co. KG, 03.03.2008 DE-TEMMLER06-000061

Sender diagnosis: not reported

Narrative

This case was received on 18 February 2008 from BfArM. It was reported by "Dokumentationszentrum für schwere Hautreaktionen in der BRD".

A 81-year-old female patient developed a Stevens-Johnson-Syndrom (SJS) on 22 July 2005 (=Indexday). The attending physician assessed the reaction as "not life threatening".

The patient died 14 days after the indexday. The patient had started a medication with Favistan (thiamazol) due to hyperthyreosis 16 days before the indexday. A treatment of hyperuricemia was started with allopurinol 22 days before indexday. Furthermore the drug history revealed the intake of several drugs within 4 weeks before the indexday: against cardiac insufficiency, atrial fibrillation, as stomach protection, prophylaxis of thrombosis, metamizol against pain, metoclopramid against nausea, and citalopram against depression.

Sender's comment:

Reporter's comment:

The reporters assessed the causal relationship between SJS and Favistan as possible.

The reporters assume, that the patient died due to SJS because of a short interval between the indexday and day of death.

APHQWEB, 04.03.2008 DE-Nycomed GmbH-80327

Sender diagnosis: not reported

Narrative

This case was received by the German Documentation centre of Severe skin reaction via the German Health authority (Bfarm ref.no. DE-BFARM-08014070).

A senior female patient experienced Stevens-Johnson-syndrom (SJS) while receiving pantoprazole (Pantozol 40 mg) for gastric protection for about five weeks.

Further 20 drugs were suspected - see list.

On from 17-JUN-2005 to 28-JUN-2008, the patient was hospitalized in a department for internal medicine for not reported reason. This day, pantoprazole therapy was started.

From 28-JUN-2005 to 24-JUL-2005, the patient was staying in a rehabilitation centre. Pantoprazole therapy was interrupted from date of admission and re-started on 14-JUL-2005.

On 22-JUL-2005, approximately eight days after re-introduction of pantoprazole, the patient presented pruritus, cockades and skin spots.

On 23-JUL-2005 patient developed fever and skin blisters.

On 24-JUL-2005 patient was found to have lip erosions and oral pain. On this day she was transferred to a dermatology clinic.

No concomitant infection was mentioned.

Outcome: patient died. Exact date of death was not provided. No autopsy report was provided.

Medical history and concomitant diseases included cardiac insufficiency, state after TIA, constipation, atrial fibrillation, hyperuricaemia, hyperthyreosis, pain, depression, nausea, hyponatraemia and hypertension.

Sender's comment:

The patient received further 20 suspected drugs. Allopurinol treatment in this case was considered to be "highly suspected". The causality to pantoprazole treatment was assessed as possible by the reporter.

With the information on hand Nycomed is now going to assess the case in order to come to some conclusion.

STEVEN-JOHNSON-SYNDROME:

This patient developed Steven-Johnson-Syndrome in a plausible temporal relationship with intake of pantoprazole, approximately 8 days after re-introduction of pantoprazole.

Concomitantly, she received multiple other drugs (20 suspected drugs).

Steven-Johnson-Syndrome is a rare skin disorder characterized by mucosal erosions, skin blisters, in many cases accompanied by fever. It can be drug-induced or due to infectious diseases.

Several drugs have been observed to cause Steven-Johnson-Syndrome. Toxic bullous skin reactions up to acute disseminated epidermal necrolysis have also been observed in very rare cases with pantoprazole and other proton pump inhibitors. Although Steven-Johnson-Syndrome is considered labelled for pantoprazole, this event was considered to be unlabeled, since pantoprazole cases with "fatal outcome" are considered as unlabeled. However, the mortality is high in general for patients suffering from Steven-Johnson-Syndrome and toxic epidermal necrolysis (20-25%; Mockenhaupt et al 2008).

Time to onset was basically plausible. In the presence of multiple other suspected drugs, it remains difficult to determine a single trigger for the reported event. The reporter considered allopurinol as highly suspected drug in this case.

Nycomed agrees with the reporter and assesses this case as "possible".

This report is not considered to have any influence on current knowledge regarding the safety profile of pantoprazole and has no effect on the status of

the drug as reflected by the formal product characteristics.

Reporter's comment:

APHQWEB, 05.03.2008 DE-Nycomed GmbH-80327

Sender diagnosis: not reported

Narrative

This case was received by the German Documentation centre of Severe skin reaction via the German Health authority (Bfarm ref.no. DE-BFARM-08014070).

A senior female patient experienced Stevens-Johnson-syndrom (SJS) while receiving pantoprazole (Pantozol 40 mg) for gastric protection for about five weeks.

Further 20 drugs were suspected - see list.

On from 17-JUN-2005 to 28-JUN-2008, the patient was hospitalized in a department for internal medicine for not reported reason. This day, pantoprazole therapy was started.

From 28-JUN-2005 to 24-JUL-2005, the patient was staying in a rehabilitation centre. Pantoprazole therapy was interrupted from date of admission and re-started on 14-JUL-2005.

On 22-JUL-2005, approximately eight days after re-introduction of pantoprazole, the patient presented pruritus, cockades and skin spots.

On 23-JUL-2005 patient developed fever and skin blisters.

On 24-JUL-2005 patient was found to have lip erosions and oral pain. On this day she was transferred to a dermatology clinic.

No concomitant infection was mentioned.

Outcome: patient died. Exact date of death was not provided. No autopsy report was provided.

Medical history and concomitant diseases included cardiac insufficiency, state after TIA, constipation, atrial fibrillation, hyperuricaemia, hyperthyreosis, pain, depression, nausea, hyponatraemia and hypertension.

Sender's comment:

The patient received further 20 suspected drugs. Allopurinol treatment in this case was considered to be "highly suspected". The causality to pantoprazole treatment was assessed as possible by the reporter.

With the information on hand Nycomed is now going to assess the case in order to come to some conclusion.

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This patient developed Steven-Johnson-Syndrome in a plausible temporal relationship with intake of pantoprazole, approximately 8 days after re-introduction of pantoprazole.

Concomitantly, she received multiple other drugs (20 suspected drugs).

Steven-Johnson-Syndrome is a rare skin disorder characterized by mucosal erosions, skin blisters, in many cases accompanied by fever. It can be drug-induced or due to infectious diseases.

Several drugs have been observed to cause Steven-Johnson-Syndrome. Toxic bullous skin reactions up to acute disseminated epidermal necrolysis have also been observed in very rare cases with pantoprazole and other proton pump inhibitors. Although Steven-Johnson-Syndrome is considered labelled for pantoprazole, this event was considered to be unlabeled, since pantoprazole cases with "fatal outcome" are considered as unlabeled. However, the mortality is high in general for patients suffering from Steven-Johnson-Syndrome and toxic epidermal necrolysis (20-25%; Mockenhaupt et al 2008).

Time to onset was basically plausible. In the presence of multiple other suspected drugs, it remains difficult to determine a single trigger for the reported event. The reporter considered allopurinol as highly suspected drug in this case.

Nycomed agrees with the reporter and assesses this case as "possible".

This report is not considered to have any influence on current knowledge regarding the safety profile of pantoprazole and has no effect on the status of the drug as reflected by the formal product characteristics.

Reporter's comment:

APHQWEB, 05.03.2008 DE-Nycomed GmbH-80327

Sender diagnosis: not reported

Narrative

This case was received by the German Documentation centre of Severe skin reaction via the German Health authority (Bfarm ref.no. DE-BFARM-08014070).

A senior female patient experienced Stevens-Johnson-syndrom (SJS) while receiving pantoprazole (Pantozol 40 mg) for gastric protection for about five weeks.

Further 20 drugs were suspected - see list.

On from 17-JUN-2005 to 28-JUN-2008, the patient was hospitalized in a department for internal medicine for not reported reason. This day, pantoprazole therapy was started.

From 28-JUN-2005 to 24-JUL-2005, the patient was staying in a rehabilitation centre. Pantoprazole therapy was interrupted from date of admission and re-started on 14-JUL-2005.

On 22-JUL-2005, approximately eight days after re-introduction of pantoprazole, the patient presented pruritus, cockades and skin spots.

On 23-JUL-2005 patient developed fever and skin blisters.

On 24-JUL-2005 patient was found to have lip erosions and oral pain. On this day she was transferred to a dermatology clinic.

No concomitant infection was mentioned.

Outcome: patient died. Exact date of death was not provided. No autopsy report was provided.

Medical history and concomitant diseases included cardiac insufficiency, state after TIA, constipation, atrial fibrillation, hyperuricaemia, hyperthyreosis, pain, depression, nausea, hyponatraemia and hypertension.

Sender's comment:

The patient received further 20 suspected drugs. Allopurinol treatment in this case was considered to be "highly suspected". The causality to pantoprazole treatment was assessed as possible by the reporter.

With the information on hand Nycomed is now going to assess the case in order to come to some conclusion.

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This patient developed Steven-Johnson-Syndrome in a plausible temporal relationship with intake of pantoprazole, approximately 8 days after re-introduction of pantoprazole.

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Steven-Johnson-Syndrome is a rare skin disorder characterized by mucosal erosions, skin blisters, in many cases accompanied by fever. It can be drug-induced or due to infectious diseases.

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have also been observed in very rare cases with pantoprazole and other proton pump inhibitors. Although Steven-Johnson-Syndrome is considered labelled for pantoprazole, this event was considered to be unlabeled, since pantoprazole cases with "fatal outcome" are considered as unlabeled. However, the mortality is high in general for patients suffering from Steven-Johnson-Syndrome and toxic epidermal necrolysis (20-25%; Mockenhaupt et al 2008).

Time to onset was basically plausible. In the presence of multiple other suspected drugs, it remains difficult to determine a single trigger for the reported event. The reporter considered allopurinol as highly suspected drug in this case. Nycomed agrees with the reporter and assesses this case as "possible".

This report is not considered to have any influence on current knowledge regarding the safety profile of pantoprazole and has no effect on the status of the drug as reflected by the formal product characteristics.

Reporter's comment:

APHQWEB, 06.03.2008 DE-Nycomed GmbH-80327

Sender diagnosis: not reported

Narrative

This case was received by the German Documentation centre of Severe skin reaction via the German Health authority (Bfarm ref.no. DE-BFARM-08014070).

A senior female patient experienced Stevens-Johnson-syndrom (SJS) while receiving pantoprazole (Pantozol 40 mg) for gastric protection for about five weeks.

Further 20 drugs were suspected - see list.

On from 17-JUN-2005 to 28-JUN-2008, the patient was hospitalized in a department for internal medicine for not reported reason. This day, pantoprazole therapy was started.

From 28-JUN-2005 to 24-JUL-2005, the patient was staying in a rehabilitation centre. Pantoprazole therapy was interrupted from date of admission and re-started on 14-JUL-2005.

On 22-JUL-2005, approximately eight days after re-introduction of pantoprazole, the patient presented pruritus, cockades and skin spots.

On 23-JUL-2005 patient developed fever and skin blisters.

On 24-JUL-2005 patient was found to have lip erosions and oral pain. On this day she was transferred to a dermatology clinic.

No concomitant infection was mentioned.

Outcome: patient died. Exact date of death was not provided. No autopsy report was provided.

Medical history and concomitant diseases included cardiac insufficiency, state after TIA, constipation, atrial fibrillation, hyperuricaemia, hyperthyreosis, pain, depression, nausea, hyponatraemia and hypertension.

Sender's comment:

The patient received further 20 suspected drugs. Allopurinol treatment in this case was considered to be "highly suspected". The causality to pantoprazole treatment was assessed as possible by the reporter.

With the information on hand Nycomed is now going to assess the case in order to come to some conclusion.

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This patient developed Steven-Johnson-Syndrome in a plausible temporal relationship with intake of pantoprazole, approximately 8 days after re-introduction of pantoprazole.

Concomitantly, she received multiple other drugs (20 suspected drugs).

Steven-Johnson-Syndrome is a rare skin disorder characterized by mucosal erosions, skin blisters, in many cases accompanied by fever. It can be drug-induced or due to infectious diseases.

Several drugs have been observed to cause Steven-Johnson-Syndrome. Toxic bullous skin reactions up to acute disseminated epidermal necrolysis have also been observed in very rare cases with pantoprazole and other proton pump inhibitors. Although Steven-Johnson-Syndrome is considered labelled for pantoprazole, this event was considered to be unlabeled, since pantoprazole cases with "fatal outcome" are considered as unlabeled. However, the mortality is high in general for patients suffering from Steven-Johnson-Syndrome and toxic epidermal necrolysis (20-25%; Mockenhaupt et al 2008).

Time to onset was basically plausible. In the presence of multiple other suspected drugs, it remains difficult to determine a single trigger for the reported event. The reporter considered allopurinol as highly suspected drug in this case. Nycomed agrees with the reporter and assesses this case as "possible".

This report is not considered to have any influence on current knowledge regarding the safety profile of pantoprazole and has no effect on the status of the drug as reflected by the formal product characteristics.

Reporter's comment:

APHQWEB, 10.03.2008 DE-Nycomed GmbH-80327

Sender diagnosis: not reported

Narrative

This case was received by the German Documentation centre of Severe skin reaction via the German Health authority (Bfarm ref.no. DE-BFARM-08014070).

A senior female patient experienced Stevens-Johnson-syndrom (SJS) while receiving pantoprazole (Pantozol 40 mg) for gastric protection for about five weeks.

Further 20 drugs were suspected - see list.

On from 17-JUN-2005 to 28-JUN-2008, the patient was hospitalized in a department for internal medicine for not reported reason. This day, pantoprazole therapy was started.

From 28-JUN-2005 to 24-JUL-2005, the patient was staying in a rehabilitation centre. Pantoprazole therapy was interrupted from date of admission and re-started on 14-JUL-2005.

On 22-JUL-2005, approximately eight days after re-introduction of pantoprazole, the patient presented pruritus, cockades and skin spots.

On 23-JUL-2005 patient developed fever and skin blisters.

On 24-JUL-2005 patient was found to have lip erosions and oral pain. On this day she was transferred to a dermatology clinic.

No concomitant infection was mentioned.

Outcome: patient died. Exact date of death was not provided. No autopsy report was provided.

Medical history and concomitant diseases included cardiac insufficiency, state after TIA, constipation, atrial fibrillation, hyperuricaemia, hyperthyreosis, pain, depression, nausea, hyponatraemia and hypertension.

Sender's comment:

The patient received further 20 suspected drugs. Allopurinol treatment in this case was considered to be "highly suspected". The causality to pantoprazole treatment was assessed as possible by the reporter.

With the information on hand Nycomed is now going to assess the case in order to come to some conclusion.

STEVEN-JOHNSON-SYNDROME:

This patient developed Steven-Johnson-Syndrome in a plausible temporal relationship with intake of pantoprazole, approximately 8 days after re-introduction of pantoprazole.

Concomitantly, she received multiple other drugs (20 suspected drugs).

Steven-Johnson-Syndrome is a rare skin disorder characterized by mucosal erosions, skin blisters, in many cases accompanied by fever. It can be drug-induced or due to infectious diseases.

Several drugs have been observed to cause Steven-Johnson-Syndrome. Toxic bullous skin reactions up to acute disseminated epidermal necrolysis have also been observed in very rare cases with pantoprazole and other proton pump inhibitors. Although Steven-Johnson-Syndrome is considered labelled for pantoprazole, this event was considered to be unlabeled, since pantoprazole cases with "fatal outcome" are considered as unlabeled. However, the mortality is high in general for patients suffering from Steven-Johnson-Syndrome and toxic epidermal necrolysis (20-25%; Mockenhaupt et al 2008).

Time to onset was basically plausible. In the presence of multiple other suspected drugs, it remains difficult to determine a single trigger for the reported event. The reporter considered allopurinol as highly suspected drug in this case.

Nycomed agrees with the reporter and assesses this case as "possible".

This report is not considered to have any influence on current knowledge regarding the safety profile of pantoprazole and has no effect on the status of the drug as reflected by the formal product characteristics.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08014082

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		Yes

Patient

Initials:	Sex: Female	Birth: privacy	Age: 88 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Dyspnea	20050308				Unknown
2 Stevens-Johnson syndrome	20050316	20050408	23 Day	8 Day	not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
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1	ACETYLCYSTEIN BT acetylcysteine	Oral	200 Mg milligram(s)/	20050308-20050317 COPD 10 Day
2	ambroxol ambroxol	Oral	/	20050314-20050317 Chronic obstructive pulmonary 4 Day disease
3	amphotericin b amphotericin b		/	20050317-20050317 Lip sore -
4	ARLEVERT cinnarizine; dimenhydrinate	Oral	/	20050212-20050316 Dizziness 33 Day
5	atrovent ipratropium bromide	Respiratory (inhalation)	/	20050308- COPD -
6	azithromycin azithromycin	Oral	/	20050308-20050310 Bronchitis 3 Day
7	BELOC-ZOK MITE metoprolol succinate	Oral	/	20050303- Cardiac insufficiency -
8	BERLOSIN metamizole	Oral	/	20050306-20050309 Pain 4 Day
9	cetirizine cetirizine	Oral	/	20050316-20050317 Pruritus 2 Day
10	CHLORALHYDRAT chloral hydrate	Oral	500 Mg milligram(s)/	20050225- Sleep disturbance -
11	CLARITHROMYCIN clarithromycin	Oral	/	20050316-20050316 Bronchitis -
12	digoxin digoxin	Oral	/	20050213-20050225 Cardiac insufficiency 13 Day
13	esomeprazole magnesium esomeprazole magnesium	Oral	20 Mg milligram(s)/	20050225-20050317 H.pylori gastrointestinal disease 21 Day
14	ferro sanol duodenal ferroglycine sulfate complex	Oral	/	20050212-20050314 Therapeutic procedure 31 Day
15	heparin-fraction, calcium salt heparin-fraction, calcium salt	Subcutaneous	0.4 ml millilitre(s)/	20050212-20050224 Thrombosis prophylaxis 13 Day
16	heparin-fraction, sodium salt heparin-fraction, sodium salt	Subcutaneous	0.5 ml millilitre(s)/	20050225-20050317 Thrombosis prophylaxis 21 Day
17	ipratropium bromide ipratropium bromide	Respiratory (inhalation)	/	20050308-20050317 Chronic obstructive pulmonary 10 Day disease
18	KLACID clarithromycin	Oral	/	20050315-20050316 Bronchitis 2 Day
19	MCP metoclopramide	Oral	/	20050306-20050307 Nausea 2 Day
20	megalac almasilate; oxetacaine	Oral	/	20050305-20050305 Heartburn -
21	metamizole sodium monohydrate metamizole	Oral	/	20050222-20050223 Pain 2 Day
22	metoclopramide metoclopramide	Oral	/	20050306-20050307 Nausea 2 Day
23	metoprolol succinate metoprolol succinate	Oral	/	20050303-20050317 Cardiac insufficiency 15 Day
24	mono-embolex certoparin sodium	Subcutaneous	/	20050225- Thrombosis prophylaxis -
25	NEXIUM esomeprazole magnesium	Oral	20 Mg milligram(s)/	20050225- Prophylaxis -
26	nitrazepam nitrazepam	Oral	/	20050217-20050224 Sleep disorder 8 Day
27	paracetamol paracetamol		500 Mg milligram(s)/	20050223-20050224 Fever 2 Day
28	paracetamol paracetamol		500 Mg milligram(s)/	20050217-20050217 -

29	PENTALONG pentaerithrityl tetranitrate	Oral	50 Mg milligram(s)/	20050212-20050317	Cardiac insufficiency 34 Day
30	salbutamol sulfate salbutamol sulfate	Respiratory (inhalation)	/	20050308-20050317	Chronic obstructive pulmonary disease
31	SULTANOL salbutamol	Respiratory (inhalation)	/	20050308-	COPD
32	TRAMAL tramadol hydrochloride	Oral	/	20050212-20050317	Pain 34 Day
33	TRAMAL LONG 100 tramadol hydrochloride	Oral	/	20050212-	Pain
34	vomex a dimenhydrinate		/	20050316-20050317	Nausea 2 Day
35	ZITHROMAX azithromycin	Oral	/	20050308-20050310	Bronchitis 3 Day
Drugs (concomitant)					
36	kalinor potassium chloride	Oral	/	20050314-	Hypokalemia
37	KAMILLENMUNDSPUELUNG chamomile		/	20050311-	Mouth irritation
38	MULTIBIONTA ascorbic acid; calcium pantothenate; cyanocobalamin; nicotinamide; pyridoxine hydrochloride; retinol palmitate; riboflavin; rutoside; thiamine hydrochloride; tocopheryl acetate	Oral	/	20050212-20050224	Therapeutic procedure 13 Day
39	MULTIBIONTA ascorbic acid; calcium pantothenate; cyanocobalamin; nicotinamide; pyridoxine hydrochloride; retinol palmitate; riboflavin; rutoside; thiamine hydrochloride; tocopheryl acetate	Oral	/	20050226-	
40	PARACETAMOL NON-BI DRUG paracetamol	Rectal	/	20050310-20050317	Fever 8 Day
41	THEOPHYLLIN theophylline	Oral	200 Mg milligram(s)/	20050310-20050316	COPD 7 Day
42	TORASEMID torasemide	Oral	10 Mg milligram(s)/	20050311-20050317	Cardiac insufficiency 7 Day

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Trouble falling asleep			Unknown	
Thrombosis prophylaxis			Unknown	
Pruritus			Unknown	
Pain			Unknown	
Nausea			Unknown	
Hypokalaemia			Unknown	
Heartburn			Unknown	
Dizziness			Unknown	
COPD			Unknown	
Cardiac insufficiency			Unknown	
Candidiasis			Unknown	
Bronchitis			Unknown	
Gastric ulcer prophylaxis			Unknown	
Iron supplementation			Unknown	
Vitamin supplementation			Unknown	

Primary source

Qualification: Physician observestudytype:
Literature:

Narratives (nur bei elektronischem Eingang)PFIZER INC, 19.02.2008 DE-PFIZER INC-2008011473

Sender diagnosis: not reported

Narrative

This is a report from the German Documentation Centre of Severe Skin Reactions a sponsored project set up to collect suspected drug induced severe skin reactions leading to hospital admission.

This physician reports that a 88-year-old female patient was treated with Zithromax (Azithromycin) twice daily from 08Mar2005 until 10Mar2005 for bronchitis with symptoms of dyspnea and fever. The subject had a history of cardiac insufficiency, dizziness, pain, disturbance to fall asleep, heartburn, nausea, bronchitis, chronic obstructive pulmonary disease, hypokalemia, candidosis and dyspnea. She was hospitalized on 10Feb2005 in an orthopedic department for unknown reason. On 25Feb2005 she was transferred to a geriatric department of the hospital. On 16Mar2005 the patient developed pruritus treated with Cetirizin since 16Mar2005, cockardes and spots on skin. Diagnosis of Stevens-Johnson syndrome was made on 16Mar2005. On 17Mar2005 the patient developed dysphagia and erosions of the oral mucosa. On 18Mar2005 blistering of the skin and erosions of the lips occurred. On 18Mar2005 the patient was transferred to the dermatologic department of the hospital. The outcome of the adverse event was reported as not recovered but also a recovery date 08Apr2005 was documented. The experts assessed the causal relationship as possible with Tramal (given since 12Feb2005), Berlosin (given from 06Mar2005 to 09Mar2005), as probable with Zithromax, Ferro sanol duodenal (given from 12Feb2005 to 14Mar2005), Arlevert (given from 12Mar2005 to 16Mar2005), Pentalong (given since 12Feb2005), Monoembolex (given since 25Feb2005), Chloralhydrat (given since 25Feb2005), Nexiun (given since 25Feb2005), Belok Zok (given since 03Mar2005), Megalac (given on 05Mar2005), MCP (given from 06Mar2005 to 07Mar2005), Acetylcysteine (given since 08Mar2005), Atrovent (given since 08Mar2005), Sultanol (given since 08Mar2005) and Klacid (given from 15Mar2005 to 16Mar2005), as questionable with Multibionta (given from 12Mar2005 to 24Feb2005 and then since 26Feb2005), Theophillin (given from 10Mar2005 to 16Mar2005), Paracetamol (given since 10Mar2005), Camomile Solution (given since 11Mar2005), Torasemide (given since 11Mar2005), Kalinor (given since 14Mar2005).

Company Clinical Evaluation

In light of the massive polypharmacy the patient was exposed to a separate causality assessment is impossible.

Sender's comment:Reporter's comment:AZPROD0000, 25.02.2008 DE-AstraZeneca-2008PK00365

Sender diagnosis: not reported

Narrative

{STEVENS-JOHNSON-SYNDROME}

A report was received from the Centre for Documentation of Severe Skin Reactions in Germany via the German Health Authority (BfArM) concerning a 88-year-old female patient. The patient was being treated with BELOC-ZOC MITE (metoprolol succinate) daily for cardiac insufficiency, NEXIUM ORAL (esomeprazole magnesium), ferroglycine sulfate complex, cinnarizine, dimenhydrinate, TRAMAL (tramadol hydrochloride), PENTALONG (pentaerithryl tetranitrate), MONO EMBOLEX (certoparin sodium), chloral hydrate, almasilate, metoclopramide hydrochloride, BERLOSIN (metamizole sodium), azithromycin, acetylcysteine, ATROVENT (ipratropium bromide), SULTANOL FOR INHALATION (salbutamol sulfate) and KLACID (clarithromycin).

Concomitant drugs included nadroparin calcium for thrombosis prophylaxis, digoxin and torasemide for cardiac insufficiency, multibionta for prophylaxis, nitrazepam for sleep disturbance, paracetamol and metamizole sodium for pain, theophylline, ambroxol for chronic obstructive pulmonary disease, herbal oil nos for irritation, potassium chloride for hypokalaemia, clarithromycin for bronchitis, cetirizine for pruritis, dimenhydrinate for nausea and amphotericin b for soor.

On 08-Mar-2005 she suffered from dyspnoea and fever and bronchitis and COPD was diagnosed. On 16-Mar-2005 she suffered from pruritis, pupura

and diffuse redness. On the next day dysphagia and oral mucosal erosions were found and blisters of the skin and lip erosions occurred on 18-Mar-2005. Stevens-Johnson Syndrome was diagnosed. The patient has not yet recovered.

The event was considered serious due to hospitalization.

The reporter considered the event possibly related to metoprolol succinate, esomeprazole magnesium, ferroglycine sulfate complex, cinnarizine/dimenhydrinate, tramadol hydrochloride, pentaerythrityl tetranitrate, certoparin sodium, chloral hydrate, almasilate, metoclopramide hydrochloride, metamizole sodium, azithromycin, acetylcysteine, ipratropium bromide, salbutamol sulfate and clarithromycin.

Sender's comment:

Stevens-Johnson Syndrome is not listed for metoprolol but has been associated with use of esomeprazole. It is difficult to assess causality based on the information provided because several other co-suspected drugs may also have been causative for the event. Report according to article 42 and 63b AMG. Stevens-Johnson Syndrome is not listed for metoprolol but has been associated with use of esomeprazole. It is difficult to assess causality based on the information provided because several other co-suspected drugs may also have been causative for the event.

Causality assessment: unassessable
NO

Reporter's comment:

Boehringer Ingelheim GmbH, Germany, 26.02.2008 DE-Boehringer Ingelheim GmbH, Germany-2008-DE-01033DE

Sender diagnosis: not reported

Narrative

Initial information has been received from the 'centre of documentation of severe skin diseases' in Freiburg, Germany via the German health authority. This case was identified and registered by the "centre of documentation of severe skin diseases" in Freiburg, Germany and reported to the federal health office. The "centre" collects and files nationwide case reports of severe skin diseases.

Therefore, these reports must not be confused with documentation of suspected adverse drug reactions.

While on therapy with Atrovent (ipratropium-BR) 3x1/d for COPD, Ferro Sanol Duodenal (ferric-glycine-sulphate-complex) 2x1/d for substitution, Arlevert (cinnarizin, dimenhydrinat) 1x1/d for dizziness, Tramal (tramadol-HCl) 1x50mg/d and 1xlong 100mg/d for pain, Pentalong (pentaerythrityltetranitrat) 2x1/d for cardiac insufficiency, Mono-Embolex (certoparin-sodium) 1x0,5ml/d for thrombosis prophylaxis, chlorhydrate 1x1/d for sleep disorder, Nexium (esomeprazol) 1x1/d for stomach protection, Beloc-Zok mite (metoprololsuccinat) 1x1/d for cardiac insufficiency, Megalac (almasilat) 1x1 table spoon for heartburn, MCP (metoclopramid-HCl 1H₂O) 1x20 drops/d, Berlosin (metamizol-sodium 1H₂O) 1x1/d for pain, Zithromax (azithromycin 2H₂O) 2x1/d for bronchitis, acetylcystein 3x1/d for COPD, Sultanol (salbutamolsulphate) 3x1/d for COPD and Klacid (clarithromycin) 2x2/d and 1x1/d for bronchitis a 88-year-old female patient experienced from 16Mar2005 to 08Apr2005 a Stevens-Johnson-syndrome (course: 08Mar2005: dyspnoea and fever; 16Mar2005: pruritus, cockades and spots; 17Mar2005: dysphagia and MSH-erosion; 18Mar2005: blisters on skin and erosion of the lips). The patient was hospitalised from 10Feb2005 until 25Feb2005 in the orthopaedy, 25Feb2005 until 18Mar2005 in the geriatrics and from 18Mar2005 until 08Apr2005 in the dermatology. The patient was at the onset time of the event in hospital. The patient has not recovered.

Co-medication included Multibionta drops (retinolpalmitat, thiaminchlorid-HCl, riboflavin-5'-phosphate-sodium 2H₂O, nicotinamid, dexpanthenol, pyridoxin-HCl, ascorbic acid, colecalciferol, a-tocopherolacetat), theophyllin, paracetamol, Kamillenmundspülung, torasemid and Kalinor (potassium chloride).

Co-disease included fever, irritation and hypokalemia.

The reporting "centre" assumed the event not to be life-threatening and rated the causal relationship between the Stevens-Johnson-Syndrome and Atrovent, Ferro Sanol Duodenal, Arlevert, Tramal, Pentalong, Mono-Embolex, chlorhydrate, Nexium, Beloc-Zok mite, Megalac, MCP, Berlosin, Zithromax, acetylcystein, Sultanol and Klacid as possible.

Sender's comment:

Regional Centre Drug Safety Boehringer Ingelheim

Comment on case 2008-DE-01033DE / ATROVENT

The event STEVENS-JOHNSON SYNDROME in this case report is a not listed adverse drug reaction for the product in question. Based on the details currently available concerning this case there is no reasonable evidence in favour of a causal association between the reported adverse event and the product in question. Furthermore, the adverse event may be more plausibly explained by other characteristics of the case: co-suspect drugs.

As with all unlisted events, the company will continue to monitor all respective reports received and, based on cumulative experience, will re-evaluate the available evidence on an ongoing basis. No change in the company core safety information or other measures are recommended at this time.

EICHLER

Case assessment created by EI on 25 Feb 2008

Reporter's comment:

AZPROD0000, 27.02.2008 DE-AstraZeneca-2008PK00365

Sender diagnosis: not reported

Narrative

{STEVENS-JOHNSON-SYNDROME}

A report was received from the Centre for Documentation of Severe Skin Reactions in Germany via the German Health Authority (BfArM) concerning a 88-year-old female patient. The patient was being treated with BELOC-ZOC MITE (metoprolol succinate) daily for cardiac insufficiency, NEXIUM ORAL (esomeprazole magnesium), ferroglycine sulfate complex, cinnarizine, dimenhydrinate, TRAMAL (tramadol hydrochloride), PENTALONG (pentaerithryl tetranitrate), MONO EMBOLEX (certoparin sodium), chloral hydrate, almasilate, metoclopramide hydrochloride, BERLOSIN (metamizole sodium), azithromycin, acetylcysteine, ATROVENT (ipratropium bromide), SULTANOL FOR INHALATION (salbutamol sulfate) and KLACID (clarithromycin).

Concomitant drugs included nadroparin calcium for thrombosis prophylaxis, digoxin and torasemide for cardiac insufficiency, multibionta for prophylaxis, nitrazepam for sleep disturbance, paracetamol and metamizole sodium for pain, theophylline, ambroxol for chronic obstructive pulmonary disease, herbal oil nos for irritation, potassium chloride for hypokalaemia, clarithromycin for bronchitis, cetirizine for pruritis, dimenhydrinate for nausea and amphotericin b for soor.

On 08-Mar-2005 she suffered from dyspnoea and fever and bronchitis and COPD was diagnosed. On 16-Mar-2005 she suffered from pruritis, pupura and diffuse redness. On the next day dysphagia and oral mucosal erosions were found and blisters of the skin and lip erosions occurred on 18-Mar-2005. Stevens-Johnson Syndrome was diagnosed. The patient has not yet recovered.

The event was considered serious due to hospitalization.

The reporter considered the event possibly related to metoprolol succinate, esomeprazole magnesium, ferroglycine sulfate complex, cinnarizine/ dimenhydrinate, tramadol hydrochloride, pentaerithryl tetranitrate, certoparin sodium, chloral hydrate, almasilate, metoclopramide hydrochloride, metamizole sodium, azithromycin, acetylcysteine, ipratropium bromide, salbutamol sulfate and clarithromycin.

Sender's comment:

Stevens-Johnson Syndrome is not listed for metoprolol but has been associated with use of esomeprazole. It is difficult to assess causality based on the information provided because several other co-suspected drugs may also have been causative for the event.

Report according to article 42 and 63b AMG. Stevens-Johnson Syndrome is not listed for metoprolol but has been associated with use of esomeprazole. It is difficult to assess causality based on the information provided because several other co-suspected drugs may also have been causative for the event.

Causality assessment: unassessable
NO

Reporter's comment:

001307602, 27.02.2008 DE-ABBOTT-08P-062-0438842-00

Sender diagnosis: not reported

Narrative

Report from Germany of Stevens-Johnson syndrome and dyspnea coincident with CLARITHROMYCIN therapy. On 10 Feb 2005, the patient was hospitalized in the orthopedic unit. On 12 Feb 2005, the patient began HEPARIN-FRACTION, CALCIUM SALT for thrombosis prophylaxis, FERRO SANOL for substitution, ARLEVERT for vertigo, TRAMADOL HYDROCHLORIDE for pain, and PENTAERITHRITYL TETRANITRATE for cardiac insufficiency, and MULTIBIONTA for roborierung. On 13 Feb 2005, the patient began DIGOXIN for cardiac insufficiency. On 17 Feb 2005, the patient began a one time dose of NITRAZEPAM for sleep disorder, and a one time dose of PARACETAMOL for pain. On 21 Feb 2005, the patient restarted NITRAZEPAM for sleep disorder. On 22 Feb 2005, the patient began METAMIZOLE SODIUM MONOHYDRATE for pain. On 23 Feb 2005, HEPARIN-FRACTION, SODIUM SALT was discontinued. On 23 Feb 2005, the patient restarted PARACETAMOL. On 24 Feb 2005, HEPARIN-FRACTION, CALCIUM SALT, PENTAERITHRITYL TETRANITRATE, MULTIBIONTA, NITRAZEPAM and PARACETAMOL were discontinued. On 25 Feb 2005, DIGOXIN was discontinued. On 25 Feb 2005, the patient was discharged from the orthopedic unit and transferred to the geriatric unit. On 25 Feb 2005, the patient began HEPARIN-FRACTION, SODIUM SALT for thrombosis prophylaxis, CHLORAL HYDRATE for sleep disorder, and ESOMEPRAZOLE MAGNESIUM for H+ pump inhibition. On 26 Feb 2006, MULTIBIONTA was restarted. On 03 Mar 2005, the patient began METOPROLOL SUCCINATE for cardiac insufficiency. On 05 Mar 2005, the patient began a one time dose MEGALAC for gastroesophageal reflux disease. On 06 Mar 2005, the patient began METOCLOPRAMIDE for nausea and BERIOSIN for pain. On 07 Mar 2005, METOCLOPRAMIDE was discontinued. On 08 Mar 2005, the patient experienced dyspnea and fever. On 08 Mar 2005, the patient began AZITHROMYCIN, for bronchitis, ACETYLCYSTEINE, IPRATROPIUM BROMIDE, and SALBUTAMOL SULFATE for chronic obstructive pulmonary disease. On 09 Mar 2005, the patient discontinued BERIOSIN. On 10 Mar 2005, the patient discontinued AZITHROMYCIN. On 10 Mar 2005, the patient began THEOPHYLLINE for chronic obstructive pulmonary disease and PARACETAMOL suppository for fever. On 11 Mar 2005, the patient began KAMILLENMUNDSPULUNG for irritation of the oral mucosa and TORASEMID for cardiac insufficiency. On 14 Mar 2005, the patient began AMBROXOL for chronic obstructive pulmonary disease and KALINOR-RET for hypokalemia. On 14 Mar 2005, FERRO SANOL was discontinued. On 16 Mar 2005, the patient experienced pruritus, cockades, and spots on the skin. On 16 Mar 2005, the patient began CLARITHROMYCIN for bronchitis, CETRIZIN for pruritus and VOMEX for nausea. On 16 Mar 2005, ARLEVERT and THEOPHYLLINE were discontinued. On 16 Mar 2005, the patient experienced Stevens-Johnson syndrome. On 17 Mar 2005, the patient experienced erosions on the lips and blisters on the skin. On 17 Mar 2005, the patient discontinued, TRAMADOL HYDROCHLORIDE, PENTAERITHRITYL TETRANITRATE, MULTIBIONTA, HEPARIN-FRACTION, SODIUM SALT, CHLORAL HYDRATE, ESOMEPRAZOLE MAGNESIUM, METOPROLOL SUCCINATE, ACETYLCYSTEINE, IPRATROPIUM BROMIDE, SALBUTAMOL SULFATE, PARACETAMOL, KAMILLENMUNDSPULUNG, TORASEMID, AMBROXOL, KALINOR-RET, CLARITHROMYCIN, CETRIZIN, and VOMEX. On 17 Mar 2005, the patient began a one time dose of AMPHOTERICIN B for soor. On 18 Mar 2005, the patient was discharged from the geriatric unit and transferred to the dermatology unit. On 08 Apr 2005, the patient was discharged from the dermatology unit. The patient had not recovered from the Stevens-Johnson syndrome. The reporting physician believed CLARITHROMYCIN, FERRO SANOL, ARLEVERT, TRAMADOL HYDROCHLORIDE, PENTAERITHRITYL TETRANITRATE, HEPARIN-FRACTION, SODIUM SALT, CHLORAL HYDRATE, ESOMEPRAZOLE MAGNESIUM, METOPROLOL SUCCINATE, MEGALAC, METOCLOPRAMIDE, BERIOSIN, AZITHROMYCIN, ACETYLCYSTEINE, IPRATROPIUM BROMIDE, and SALBUTAMOL SULFATE were possibly related to Stevens-Johnson syndrome. The reporting physician believed the MULTIBIONTA, PARACETAMOL suppository, KAMILLENMUNDSPULUNG, TORASEMID, and KALINOR-RET were probably not related to Stevens-Johnson syndrome. The reporting physician believed the HEPARIN-FRACTION, CALCIUM SALT, DIGOXIN, NITRAZEPAM, PARACETAMOL, HEPARIN-FRACTION, SODIUM SALT, AMBROXOL, CETRIZIN, VOMEX, and AMPHOTERICIN B were not related to the Stevens-Johnson syndrome. The Stevens-Johnson syndrome was considered life threatening, medically important, and involved or prolonged hospitalization. PARACETAMOL, AMBROXOL, HEPARIN-FRACTION, SODIUM SALT, DIGOXIN, FERRO SANOL, ARLEVERT, NITRAZEPAM, METAMIZOLE SODIUM MONOHYDRATE, MEGALAC, BERIOSIN, AZITHROMYCIN, THEOPHYLLINE, AMPHOTERICIN B, TRAMADOL, HYDROCHLORIDE, PENTAERITHRITYL TETRANITRATE, MULTIBIONTA, CHLORAL HYDRATE, ESOMEPRAZOLE MAGNESIUM, METOPROLOL SUCCINATE, ACETYLCYSTEINE, IPRATROPIUM BROMIDE, SALBUTAMOL SULFATE, KAMILLENMUNDSPULUNG, TORASEMID, KALINOR-RET, P, CETRIZIN, and VOMEX were also considered suspect.

Sender's comment:

Abbott Germany Assessment: 27.02.2008, Klacid(R) - 8976 / 08P-062-0438842-00, MK

Die in diesem Fallbericht geschilderte Symptomatik eines Stevens-Johnson-Syndrom ist in den Angaben zum Arzneimittel in adaequater Form beschrieben.

Reporter's comment:

SCHWARZ, 29.02.2008 DE-SCHWARZ-013#0#2008-00023

Sender diagnosis: not reported

Narrative

CASE received on 18-feb-2008 by special registry, transmitted by national authority:

A female patient experienced 'Stevens-Johnson-syndrome (SJS)' under therapy with amounts of medications inter alia ferro sanol. Therapy with ferro sanol administered for substitution started on 12-feb-2005 and was stopped 14-mar-2005. The ADR onset date - classified as so called index day - was 16-mar-2005 when symptoms like pruritus, cockades and spots appeared. On 17-mar-2005, dysphagia and oral mucosa erosion occurred additionally (first appearance of erosion at all) and on 18-mar-2005 cutaneous vesicles and lip erosion. Previously, on 8-mar-2005, dyspnoea and fever was present. The patient was at hospital from 10-feb-2005 until 25-feb-2005 at orthopedic department, from 25-feb-2005 until 18-mar-2005 at geriatric department and from 18-mar-2005 until 8-apr-2005 at dermatologic department. The ADRs' outcome was stated as not yet recovered. The reporter assessed the ADR as not life-threatening.

The causal relationship to the suspect drugs was assessed on a scale with "definite (1)", "highly probable (2)", "probable (3)", "quite possible (4)", "possibly related (5)", "questionable (6)" and "not given (7)".

The following drugs were classified as "not given (7)" by the reporter: heparin-fraction, calcium salt; digoxin, nitrazepam, paracetamol, metamizole sodium, ambroxol, cetirizine-hydrochloride, dimenhydrinate, and amphotericin B. According to the reporter's definition "not given (7)" means a drug without time relationship to skin reaction onset, i.e. either intake was after occurrence of skin reaction or last intake was at least 14 days or more prior to skin reaction onset.

The reporter assessed the following drugs as "questionably (6)" related: multivitamins, theophyllin, paracetamol suppositories, chamomile, torasemide, and potassium chloride. "Questionably (6)" related was defined as a drug that was not given in a close time relationship and is not known as causal agent for SJS so far.

The reporter stated a "quite possible (4)" relationship to tramadol and metamizole. "Quite possible (4)" means either a drug that was given in a close time relationship and belongs to the substance group of suspected drugs, without an indication of other probable triggers of SJS. Or it means a drug that was not given in a close time relationship and belongs to the substance group of highly suspected drugs, but another drug from this group was given in a close time relationship.

Among other drugs ferro sanol was assessed as "possibly (5)" related by the reporter. The following drugs were assessed as "possibly (5)" related, too: cinnarazine/dimenhydrinate, pentaerythritol-tetranitrate, heparin-sodium, chloral-hydrate, esomeprazole, metoprolol succinate, almasilate, metoclopramide, azithromycin, acetylcysteine, ipratropium-bromide, salbutamol, and clarithromycin. "Possibly (5)" related was defined as a drug that either was not given in a close time relationship and belongs to the substance group of suspected drugs, or that was given in a close time relationship and is not known as causal agent for SJS so far.

No further information is expected, case closed.

Sender's comment:

The event Stevens-Johnson-syndrome (SJS) is assessed as unexpected according to the German product information.

Based on the currently available information and the information in the enclosed report, we do not perceive a change in the risk/benefit analysis as stated in the German product information. No safety measures are required.

The serious event of "Stevens-Johnson-Syndrome (SJS)" was assessed as unlisted according to the Core Summary of Product Characteristics (Core SPC).

Based on the currently available information and the information in the enclosed report, we do not perceive a change in the risk/benefit analysis as stated in the Core SPC. No safety measures are required.

Reporter's comment:

Assessment of Relationship of STEVENS-JOHNSON-SYNDROME (SJS) to: Most suspect drug: not reported

GRUNENTHAL, 29.02.2008 DE-GRUNENTHAL-2008 10639

Sender diagnosis: not reported

Narrative

SUBJECT/PATIENT ID

Manufacturer Control Number: 2008-10639

ADVERSE EVENT DETAILS

This solicited case from special registry for severe skin reactions from GERMANY, that was reported by a physician via regulatory authority was received at GRT on 18-FEB-2008 and concerns a 88 years old female subject/patient (height unknown / weight unknown).

The relevant medical history included: Pain , Thrombosis prophylaxis , Cardiac failure , Mineral supplementation , Dizziness , Vitamin supplementation , Initial insomnia , Prophylaxis against gastrointestinal ulcer , Dyspepsia , Nausea , Bronchitis , Chronic obstructive pulmonary disease , Pyrexia , Stomatitis , Hypokalaemia , Pruritus , Candidiasis

This case was reported by the Documentation Centre for Severe Skin Reactions (Freiburg/Germany) via BfArM and concerns an 89-year-old multimorbid female patient on multimedications (incl. tramadol). On 08-Mar-2005, the patient experienced dyspnoea and fever. On 16-Mar-2005, pruritus occurred and cockades and maculae appeared followed by dysphagia and erosions of the oral mucosa on 17-Mar-2005. On 18-Mar-2005, the patient experienced blisters of the skin and erosions of the lips. The patient was hospitalized from 10-Feb-2005 to 25-Feb-2005 in the orthopaedic department of a hospital, from 25-Feb-2005 to 18-Mar-2005 in the geriatric department and from 18-Mar-2005 to 08-Apr-2005 in the dermatologic department. The reported symptoms were considered not to be life-threatening. Outcome: Not yet recovered.

No further information was requested.

The relevant concomitant medication included:

HEPARIN-FRACTION, CALCIUM SALT / FRAXIPARIN;
DIGOXIN / DILANACIN;
FERROGLYCINE SULFATE COMPLEX / FERRO DUODENAL;
CINNARIZINE, DIMENHYDRINATE / ARLEVERT;
PENTAERITHRITYL TETRANITRATE / PENTALONG 50;
ASCORBIC ACID, CALCIUM PANTOTHENATE, CYANOCOBALAMIN, NICOTINAMIDE, PYRIDOXINE HYDROCHLORIDE, RETINOL PALMITATE,
RIBOFLAVIN, RUTOSIDE, THIAMINE HYDROCHLORIDE, TOCOPHERYL ACETATE / MULTIBIONTA;
NITRAZEPAM / RADEDORM;
PARACETAMOL / PARACETAMOL;
METAMIZOLE SODIUM / NOVALGIN;
HEPARIN-FRACTION, SODIUM SALT / MONO-EMBOLEX;
CHLORAL HYDRATE / CHLORAL HYDRATE;
ESOMEPRAZOLE MAGNESIUM / NEXIUM;
METOPROLOL SUCCINATE / BELOC-ZOK;
ALMASILATE, OXETACAIN / MEGALAC;
METOCLOPRAMIDE / MCP;
AZITHROMYCIN / ZITHROMAX;
ACETYLCYSTEINE / ACETYLCYSTEIN;
IPRATROPIUM BROMIDE / ATROVENT;
SALBUTAMOL / SULTANOL;
THEOPHYLLINE / THEOPHYLLIN;
PARACETAMOL / PARACETAMOL;
Camomile / ALL OTHER THERAPEUTIC PRODUCTS;

Sender's comment:

Stevens-Johnson syndrome is assessed as serious due to the criterion hospitalization.

Stevens-Johnson syndrome is not listed in the reference safety information of tramadol hydrochloride.

The adverse event occurred in a reasonable temporal relationship with the administration of tramadol hydrochloride. It is of note that tramadol hydrochloride was not discontinued due to the event.

However, this multimorbid patient, whose medical history included acute bronchitis 9 days prior to onset of first skin lesions was treated with a multitude of other drugs, among them esomeprazole, metamizole sodium and azithromycin that are all known to cause severe skin reactions. Under consideration of these facts presenting high risk for the patient to develop Stevens-Johnson syndrome it is unlikely that tramadol hydrochloride contributed to the adverse event.

In conclusion Stevens-Johnson syndrome is assessed to be unlikely related to the administration of tramadol hydrochloride.

The information in this single case does not appear to alter the favourable benefit to risk ratio of tramadol hydrochloride.

Reporter's comment:

001307602, 17.03.2008 DE-ABBOTT-08P-062-0438842-00

Sender diagnosis: not reported

Narrative

Report from Germany of Stevens-Johnson syndrome and dyspnea coincident with CLARITHROMYCIN therapy. On 10 Feb 2005, the patient was hospitalized in the orthopedic unit. On 12 Feb 2005, the patient began HEPARIN-FRACTION, CALCIUM SALT for thrombosis prophylaxis, FERRO SANOL for substitution, ARLEVERT for vertigo, TRAMADOL HYDROCHLORIDE for pain, and PENTAERITHRITYL TETRANITRATE for cardiac insufficiency, and MULTIBIONTA for roborierung. On 13 Feb 2005, the patient began DIGOXIN for cardiac insufficiency. On 17 Feb 2005, the patient began a one time dose of NITRAZEPAM for sleep disorder, and a one time dose of PARACETAMOL for pain. On 21 Feb 2005, the patient restarted NITRAZEPAM for sleep disorder. On 22 Feb 2005, the patient began METAMIZOLE SODIUM MONOHYDRATE for pain. On 23 Feb 2005, HEPARIN-FRACTION, SODIUM SALT was discontinued. On 23 Feb 2005, the patient restarted PARACETAMOL. On 24 Feb 2005, HEPARIN-FRACTION, CALCIUM SALT, PENTAERITHRITYL TETRANITRATE, MULTIBIONTA, NITRAZEPAM and PARACETAMOL were discontinued. On 25 Feb 2005, DIGOXIN was discontinued. On 25 Feb 2005, the patient was discharged from the orthopedic unit and transferred to the geriatric unit. On 25 Feb 2005, the patient began HEPARIN-FRACTION, SODIUM SALT for thrombosis prophylaxis, CHLORAL HYDRATE for sleep disorder, and ESOMEPRAZOLE MAGNESIUM for H+ pump inhibition. On 26 Feb 2006, MULTIBIONTA was restarted. On 03 Mar 2005, the patient began METOPROLOL SUCCINATE for cardiac insufficiency. On 05 Mar 2005, the patient began a one time dose MEGALAC for gastroesophageal reflux disease. On 06 Mar 2005, the patient began METOCLOPRAMIDE for nausea and BERIOSIN for pain. On 07 Mar 2005, METOCLOPRAMIDE was discontinued. On 08 Mar 2005, the patient experienced dyspnea and fever. On 08 Mar 2005, the patient began AZITHROMYCIN, for bronchitis, ACETYLCYSTEINE, IPRATROPIUM BROMIDE, and SALBUTAMOL SULFATE for chronic obstructive pulmonary disease. On 09 Mar 2005, the patient discontinued BERIOSIN. On 10 Mar 2005, the patient discontinued AZITHROMYCIN. On 10 Mar 2005, the patient began THEOPHYLLINE for chronic obstructive pulmonary disease and PARACETAMOL suppository for fever. On 11 Mar 2005, the patient began KAMILLENMUNDSPULUNG for irritation of the oral mucosa and TORASEMID for cardiac insufficiency. On 14 Mar 2005, the patient began AMBROXOL for chronic obstructive pulmonary disease and KALINOR-RET for hypokalemia. On 14 Mar 2005, FERRO SANOL was discontinued. On 16 Mar 2005, the patient experienced pruritus, cockades, and spots on the skin. On 16 Mar 2005, the patient began CLARITHROMYCIN for bronchitis, CETRIZIN for pruritus and VOMEX for nausea. On 16 Mar 2005, ARLEVERT and THEOPHYLLINE were discontinued. On 16 Mar 2005, the patient experienced Stevens-Johnson syndrome. On 17 Mar 2005, the patient experienced erosions on the lips and blisters on the skin. On 17 Mar 2005, the patient discontinued, TRAMADOL HYDROCHLORIDE, PENTAERITHRITYL TETRANITRATE, MULTIBIONTA, HEPARIN-FRACTION, SODIUM SALT, CHLORAL HYDRATE, ESOMEPRAZOLE MAGNESIUM, METOPROLOL SUCCINATE, ACETYLCYSTEINE, IPRATROPIUM BROMIDE, SALBUTAMOL SULFATE, PARACETAMOL, KAMILLENMUNDSPULUNG, TORASEMID, AMBROXOL, KALINOR-RET, CLARITHROMYCIN, CETRIZIN, and VOMEX. On 17 Mar 2005, the patient began a one time dose of AMPHOTERICIN B for soor. On 18 Mar 2005, the patient was discharged from the geriatric unit and transferred to the dermatology unit. On 08 Apr 2005, the patient was discharged from the dermatology unit. The patient had not recovered from the Stevens-Johnson syndrome. The reporting physician believed CLARITHROMYCIN, FERRO SANOL, ARLEVERT, TRAMADOL HYDROCHLORIDE, PENTAERITHRITYL TETRANITRATE, HEPARIN-FRACTION, SODIUM SALT, CHLORAL HYDRATE, ESOMEPRAZOLE MAGNESIUM, METOPROLOL SUCCINATE, MEGALAC, METOCLOPRAMIDE, BERIOSIN, AZITHROMYCIN, ACETYLCYSTEINE, IPRATROPIUM BROMIDE, and SALBUTAMOL SULFATE were possibly related to Stevens-Johnson syndrome. The reporting physician believed the MULTIBIONTA, PARACETAMOL suppository, KAMILLENMUNDSPULUNG, TORASEMID, and KALINOR-RET were probably not related to Stevens-Johnson syndrome. The reporting physician believed the HEPARIN-FRACTION, CALCIUM SALT, DIGOXIN, NITRAZEPAM, PARACETAMOL, HEPARIN-FRACTION, SODIUM SALT, AMBROXOL, CETRIZIN, VOMEX, and AMPHOTERICIN B were not related to the Stevens-Johnson syndrome. The Stevens-Johnson syndrome was considered life threatening, medically important, and involved or prolonged hospitalization. PARACETAMOL, AMBROXOL, HEPARIN-FRACTION, SODIUM SALT, DIGOXIN, FERRO SANOL, ARLEVERT, NITRAZEPAM, METAMIZOLE SODIUM MONOHYDRATE, MEGALAC, BERIOSIN, AZITHROMYCIN, THEOPHYLLINE, AMPHOTERICIN B, TRAMADOL, HYDROCHLORIDE, PENTAERITHRITYL TETRANITRATE, MULTIBIONTA, CHLORAL HYDRATE, ESOMEPRAZOLE MAGNESIUM, METOPROLOL SUCCINATE, ACETYLCYSTEINE, IPRATROPIUM BROMIDE, SALBUTAMOL SULFATE, KAMILLENMUNDSPULUNG, TORASEMID, KALINOR-RET. P, CETRIZIN, and VOMEX were also considered suspect.

Follow-up information received on 06 Mar 2008: It was clarified that the severe symptoms of Stevens-Johnson syndrome resolved and the patient was discharged. At date of the report the patient had some negative after effects, thus the event was stated as "not recovered" by the physician.

Sender's comment:

Abbott Germany Assessment: 27.02.2008, Klacid(R) - 8976 / 08P-062-0438842-00, MK

Die in diesem Fallbericht geschilderte Symptomatik eines Stevens-Johnson-Syndrom ist in den Angaben zum Arzneimittel in adaequater Form beschrieben.

Reporter's comment:

001307602, 03.04.2008 DE-ABBOTT-08P-062-0438842-00

Sender diagnosis: not reported

Narrative

Report from Germany of Stevens-Johnson syndrome and dyspnea coincident with CLARITHROMYCIN therapy. On 10 Feb 2005, the patient was hospitalized in the orthopedic unit. On 12 Feb 2005, the patient began HEPARIN-FRACTION, CALCIUM SALT for thrombosis prophylaxis, FERRO SANOL for substitution, ARLEVERT for vertigo, TRAMADOL HYDROCHLORIDE for pain, and PENTAERITHRITYL TETRANITRATE for cardiac insufficiency, and MULTIBIONTA for roborierung. On 13 Feb 2005, the patient began DIGOXIN for cardiac insufficiency. On 17 Feb 2005, the patient began a one time dose of NITRAZEPAM for sleep disorder, and a one time dose of PARACETAMOL for pain. On 21 Feb 2005, the patient restarted NITRAZEPAM for sleep disorder. On 22 Feb 2005, the patient began METAMIZOLE SODIUM MONOHYDRATE for pain. On 23 Feb 2005, HEPARIN-FRACTION, SODIUM SALT was discontinued. On 23 Feb 2005, the patient restarted PARACETAMOL. On 24 Feb 2005, HEPARIN-FRACTION, CALCIUM SALT, PENTAERITHRITYL TETRANITRATE, MULTIBIONTA, NITRAZEPAM and PARACETAMOL were discontinued. On 25 Feb 2005, DIGOXIN was discontinued. On 25 Feb 2005, the patient was discharged from the orthopedic unit and transferred to the geriatric unit. On 25 Feb 2005, the patient began HEPARIN-FRACTION, SODIUM SALT for thrombosis prophylaxis, CHLORAL HYDRATE for sleep disorder, and ESOMEPRAZOLE MAGNESIUM for H+ pump inhibition. On 26 Feb 2006, MULTIBIONTA was restarted. On 03 Mar 2005, the patient began METOPROLOL SUCCINATE for cardiac insufficiency. On 05 Mar 2005, the patient began a one time dose MEGALAC for gastroesophageal reflux disease. On 06 Mar 2005, the patient began METOCLOPRAMIDE for nausea and BERIOSIN for pain. On 07 Mar 2005, METOCLOPRAMIDE was discontinued. On 08 Mar 2005, the patient experienced dyspnea and fever. On 08 Mar 2005, the patient began AZITHROMYCIN, for bronchitis, ACETYLCYSTEINE, IPRATROPIUM BROMIDE, and SALBUTAMOL SULFATE for chronic obstructive pulmonary disease. On 09 Mar 2005, the patient discontinued BERIOSIN. On 10 Mar 2005, the patient discontinued AZITHROMYCIN. On 10 Mar 2005, the patient began THEOPHYLLINE for chronic obstructive pulmonary disease and PARACETAMOL suppository for fever. On 11 Mar 2005, the patient began KAMILLENMUNDSPULUNG for irritation of the oral mucosa and TORASEMID for cardiac insufficiency. On 14 Mar 2005, the patient began AMBROXOL for chronic obstructive pulmonary disease and KALINOR-RET for hypokalemia. On 14 Mar 2005, FERRO SANOL was discontinued. On 16 Mar 2005, the patient experienced pruritus, cockades, and spots on the skin. On 16 Mar 2005, the patient began CLARITHROMYCIN for bronchitis, CETRIZIN for pruritus and VOMEX for nausea. On 16 Mar 2005, ARLEVERT and THEOPHYLLINE were discontinued. On 16 Mar 2005, the patient experienced Stevens-Johnson syndrome. On 17 Mar 2005, the patient experienced erosions on the lips and blisters on the skin. On 17 Mar 2005, the patient discontinued, TRAMADOL HYDROCHLORIDE, PENTAERITHRITYL TETRANITRATE, MULTIBIONTA, HEPARIN-FRACTION, SODIUM SALT, CHLORAL HYDRATE, ESOMEPRAZOLE MAGNESIUM, METOPROLOL SUCCINATE, ACETYLCYSTEINE, IPRATROPIUM BROMIDE, SALBUTAMOL SULFATE, PARACETAMOL, KAMILLENMUNDSPULUNG, TORASEMID, AMBROXOL, KALINOR-RET, CLARITHROMYCIN, CETRIZIN, and VOMEX. On 17 Mar 2005, the patient began a one time dose of AMPHOTERICIN B for soor. On 18 Mar 2005, the patient was discharged from the geriatric unit and transferred to the dermatology unit. On 08 Apr 2005, the patient was discharged from the dermatology unit. The patient had not recovered from the Stevens-Johnson syndrome. The reporting physician believed CLARITHROMYCIN, FERRO SANOL, ARLEVERT, TRAMADOL HYDROCHLORIDE, PENTAERITHRITYL TETRANITRATE, HEPARIN-FRACTION, SODIUM SALT, CHLORAL HYDRATE, ESOMEPRAZOLE MAGNESIUM, METOPROLOL SUCCINATE, MEGALAC, METOCLOPRAMIDE, BERIOSIN, AZITHROMYCIN, ACETYLCYSTEINE, IPRATROPIUM BROMIDE, and SALBUTAMOL SULFATE were possibly related to Stevens-Johnson syndrome. The reporting physician believed the MULTIBIONTA, PARACETAMOL suppository, KAMILLENMUNDSPULUNG, TORASEMID, and KALINOR-RET were probably not related to Stevens-Johnson syndrome. The reporting physician believed the HEPARIN-FRACTION, CALCIUM SALT, DIGOXIN, NITRAZEPAM, PARACETAMOL, HEPARIN-FRACTION, SODIUM SALT, AMBROXOL, CETRIZIN, VOMEX, and AMPHOTERICIN B were not related to the Stevens-Johnson syndrome. The Stevens-Johnson syndrome was considered life threatening, medically important, and involved or prolonged hospitalization. PARACETAMOL, AMBROXOL, HEPARIN-FRACTION, SODIUM SALT, DIGOXIN, FERRO SANOL, ARLEVERT, NITRAZEPAM, METAMIZOLE SODIUM MONOHYDRATE, MEGALAC, BERIOSIN, AZITHROMYCIN, THEOPHYLLINE, AMPHOTERICIN B, TRAMADOL, HYDROCHLORIDE, PENTAERITHRITYL TETRANITRATE, MULTIBIONTA, CHLORAL HYDRATE, ESOMEPRAZOLE MAGNESIUM, METOPROLOL SUCCINATE, ACETYLCYSTEINE, IPRATROPIUM BROMIDE, SALBUTAMOL SULFATE, KAMILLENMUNDSPULUNG, TORASEMID, KALINOR-RET. P, CETRIZIN, and VOMEX were also considered suspect.

Follow-up information received on 06 Mar 2008: It was clarified that the severe symptoms of Stevens-Johnson syndrome resolved and the patient was discharged. At date of the report the patient had some negative after effects, thus the event was stated as "not recovered" by the physician.

Follow-up information received on 26 Mar 2008: Adverse event information was added or revised. The Meddra code was updated for roborierung from strength loss of, to vitamin supplementation and soor from lip sore, to fungal infection.

Sender's comment:

Abbott Germany Assessment: 27.02.2008, Klacid(R) - 8976 / 08P-062-0438842-00, MK

Die in diesem Fallbericht geschilderte Symptomatik eines Stevens-Johnson-Syndrom ist in den Angaben zum Arzneimittel in adaequater Form beschrieben.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08014087

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		Yes

Patient

Initials:	Sex: Female	Birth: privacy	Age: 33 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Mucosal erosion NOS	20041014	20041222	70 Day		Unknown
2 Conjunctivitis	20041020				not recovered/not resolved
3 Oral mucosa erosion	20041019				not recovered/not resolved
4 Lip erosion	20041019				not recovered/not resolved
5 Arteriovenous shunt placement	20041012			19 Day	Unknown
6 Skin disorder	20041014				not recovered/not resolved
7 Stevens-Johnson syndrome	20041014	20041222	70 Day		not recovered/not resolved
8 Skin disorder	20041014	20041222	70 Day		Unknown
9 Stevens Johnson syndrome	20041014	20041222	70 Day	26 Day	not recovered/not resolved
10 Blisters	20041015				not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
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DE-BFARM-08014087

1	ATOSIL /00133602/ isopromethazine hydrochloride	Oral	10 Gtt drop(s)/Day Oral drops	20040926-20041003 Sedation 8 Day
2	cernevit	Intravenous (not otherwise)	1 Df dosage form/Day Solution for injection	20040914-20041002 Hypovitaminosis 19 Day
3	claxane enoxaparin	Subcutaneous	0.4 ml millilitre(s)/Day Injection	20040911-20041015 Thrombosis prophylaxis 35 Day
4	depo-clinovir medroxyprogesterone acetate	Intramuscular	1 Df dosage form/3 Month Solution for injection	20040601-20040601 Contraception 1 Day
5	ENDOXAN cyclophosphamide	Oral	200 Mg milligram(s)/Day	20040910-20040926 Wegener's granulomatosis 17 Day
6	ENDOXAN cyclophosphamide	Oral	150 Mg milligram(s)/Day	20040928-20041002 Wegener's granulomatosis 5 Day
7	Euthyrox levothyroxine sodium	Oral	25 µg microgram(s)/Day Tablet	20040918-20041015 Hypothyroidism 28 Day
8	kalinor-brausetabletten potassium carbonate; potassium citrate	Oral	1 Df dosage form/Day Tablet	20040928-20041013 Hypokalemia 16 Day
9	Orfiril valproate sodium	Oral	1200 Mg milligram(s)/Day	20040912-20041015 Epilepsy 34 Day
10	ORFIRIL valproate sodium	Oral	20 ml millilitre(s)/Day	20040909-20041015 Epilepsy 37 Day
11	Pantozol pantoprazole sodium	Oral	40 Mg milligram(s)/Day	20040927-20041015 Prophylaxis against 19 Day gastrointestinal ulcer
12	PANTOZOL pantoprazole	Intravenous (not otherwise)	1 Df dosage form/Day Injection	20040911-20040926 Prophylaxis against 16 Day gastrointestinal ulcer
13	PANTOZOL pantoprazole	Oral	40 Mg milligram(s)/ Modified-release tablet	20040927- - Gastric ulcer prophylaxis
14	Paracetamol paracetamol	Rectal	1000 Mg milligram(s)/Day Suppository	20041001-20041001 Fever 1 Day
15	prednisolon prednisolone	Intravenous (not otherwise)	300 Mg milligram(s)/ Solution for injection	20040908-20041003 Wegener's granulomatosis 26 Day
16	tavor lorazepam	Intravenous (not otherwise)	1 Mg milligram(s)/Day Injection	20040923-20041015 Sedation 23 Day
17	TAVOR /00273201/ lorazepam	Intravenous (not otherwise)	1 Mg milligram(s)/Day Solution for injection	20040923- - Sedation
18	Uromitexan mesna	Oral	200 Mg milligram(s)/Day	20040910-20041011 Wegener's granulomatosis 32 Day

Drugs (concomitant)

19	ampho-moronal amphotericin b	Oral	8 ml millilitre(s)/Day Oral Solution	20040911-20040917 Candidiasis 7 Day
20	ARTERENOL norepinephrine hydrochloride	Intravenous (not otherwis	1 Df dosage form/Day Solution for injection	20040914-20040925 Intubation 12 Day
21	Benzodiazepin nnb	Intravenous (not otherwis	/ Solution for injection	20040917-20040923 Intubation 7 Day
22	Ceftriaxon ceftriaxone	Intravenous (not otherwis	2 G gram(s)/Day Solution for injection	20040911-20040926 Sepsis 16 Day
23	clont metronidazole	Intravenous (not otherwis	500 Mg milligram(s)/Day Solution for injection	20040924-20040930 Staphylococcus aureus 7 Day septicemia
24	Cyclophosphamid cyclophosphamide	Oral	150 Mg milligram(s)/Day	20040928-20041002 Wegener's granulomatosis 5 Day
25	Cyclophosphamid cyclophosphamide	Oral	200 Mg milligram(s)/Day	20040910-20040926 Wegener's granulomatosis 17 Day
26	DISOPRIVAN propofol	Intravenous (not otherwis	50 ml millilitre(s)/Day Solution for injection	20040913-20040918 Sedation 6 Day
27	Fentanyl fentanyl	Intravenous (not otherwis	1 Df dosage form/Day Solution for injection	20040917-20040923 Respiratory insufficiency 7 Day
28	Flucloxacillin flucloxacillin	Intravenous (not otherwis	2 G gram(s)/Day Solution for injection	20040915-20040927 Sepsis 13 Day
29	Furosemid furosemide	Intravenous (not otherwis	40 Mg milligram(s)/Day Solution for injection	20040921-20040922 Edema 2 Day
30	GLUKOSE INJEKTION glucose injection	Intravenous (not otherwis	500 ml millilitre(s)/Day Injection	20040902-20040930 Glucose low 29 Day
31	INSUMAN insulin human injection, isophane; insulin zinc protamine injection	Subcutaneous	/	20040918-20040920 Hyperglycaemia 3 Day
32	INSUMAN insulin human injection, isophane; insulin zinc protamine injection	Subcutaneous	/	20040923-20040928 Hyperglycaemia 6 Day
33	ISOPROMETHAZINE HYDROCHLORIDE isopromethazine hydrochloride	Oral	10 Gtt drop(s)/Day	20040926-20041003 Sedation 8 Day
34	Kenephro	Intravenous (not otherwis	250 ml millilitre(s)/Day Solution for injection	20040911-20041015 Supplementation therapy 35 Day
35	KONAKION phytomenadione	Intravenous (not otherwis	/ Solution for injection	20040919-20040928 Coagulation disorder 10 Day
36	LASIX SPECIAL furosemide	Intravenous (not otherwis	8 ml millilitre(s)/ Solution for infusion	20040917-20040917 Edema 1 Day
37	medroxyprogesterone acetate medroxyprogesterone acetate	Intramuscular	1 Df dosage form/	20040601-20040601 Contraception 1 Day
38	METOCLOPRAMIDE HYDROCHLORIDE metoclopramide hydrochloride	Intravenous (not otherwis	3 Df dosage form/Day	20040914-20040919 Nausea 6 Day
39	METOCLOPRAMIDE HYDROCHLORIDE metoclopramide hydrochloride	Intravenous (not otherwis	3 Df dosage form/Day	20040914-20040919 Vomiting 6 Day
40	metronidazole metronidazole	Intravenous (not otherwis	500 Mg milligram(s)/Day	20040924-20040930 Septicemia 7 Day
41	Morphin morphine	Intravenous (not otherwis	10 Mg milligram(s)/Day Solution for injection	20040923-20040929 Pain 7 Day
42	MULTITRACE chromic chloride; copper chloride; manganese chloride; zinc chloride	Intravenous (not otherwis	/	20040914-20040926 Electrolyte substitution therapy 13 Day
43	NITRAZEPAM nitrazepam	Intravenous (not otherwis	1 Df dosage form/	20040917-20040923 Intubation 7 Day
44	NOREPINEPHRINE HYDROCHLORIDE norepinephrine hydrochloride	Intravenous (not otherwis	1 Df dosage form/Day	20040914-20040925 Intubation 12 Day

Drugs (concomitant)

45	Paspertin metoclopramide	Intravenous (not otherwise 3 Df dosage form/Day Solution for injection	20040914-20040919 Nausea 6 Day
46	PHYTOMENADIONE phytomenadione	Intravenous (not otherwise 1 Df dosage form/Day	20040919-20040928 Coagulation defects, other and 10 Day unspecified
47	PROPOFOL propofol	Intravenous (not otherwise 50 ml millilitre(s)/Day	20040913-20040918 Sedation 6 Day
48	TRACUTIL /01597201/	Intravenous (not otherwise 1 Df dosage form/Day Solution for injection	20040914-20040926 Supplementation therapy 13 Day

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Thrush			Unknown	
Oedema			Unknown	
Wegener's granulomatosis			Yes	
Epilepsy			Yes	
Upper abdominal discomfort			Unknown	Relevant medical history
Inappetence			Unknown	Relevant medical history
Pulmonary congestion			Unknown	Concomitant disease
Fever			Unknown	
Hypokalaemia			Unknown	
Staphylococcal sepsis			Unknown	
Clotting disorder			Unknown	
Hyperglycaemia			Unknown	
Nausea			Unknown	
Intubation			No	
Pulmonary incompetence			Unknown	
Sepsis			Unknown	
Hypovitaminosis			Unknown	
Contraception			Unknown	

Primary source

Qualification: Physician observestudytype:
Literature:

Narratives (nur bei elektronischem Eingang)

Merck KGaA, 21.02.2008 DE-Merck KGaA-6040849

Sender diagnosis: not reported

Narrative

This is a case from the Schoepf registry at the Dermatological University Hospital Freiburg, Germany which registers and evaluates all cases of hospitalisation for SJS/TEN in Germany and the drugs given in temporal relationship with the adverse event received via German Health Authority (# 08014087) on 15 Feb 2008.

**

A 33-year-old female patient was treated with Euthyrox since 18 Sep 2004 for hypothyroidism. On 01 Sep 2004 she was hospitalized since she experienced anorexia and unclear disorders on the upper abdomen. From 11 Sep 2004 to 27 Sep 2004 she suffered from ITS (infective toxic shock) since pulmonary congestion. On 12 Oct 2004 she had a shunt implantation.

On 14 Oct 2004 Steven-Johnson Syndrome was diagnosed with cockades and spots. On 15 Oct 2004 she experienced vesicles and erosion on the skin. On 19 Oct 2004 lip- and MSH (oral mucosa) erosion was diagnosed and conjunctivitis on 20 Oct 2004.

Medical history: Morbus Wegener, Hypovitaminosis, Epilepsy and Hypokalaemia.

Other suspected drugs:

Prednisolon for Morbus Wegener (Wegener's granulomatosis), paracetamol for fever, Pantozol i.v. and oral for ulcer prophylaxis, Endoxan for Morbus Wegener, Cernevit for hypovitaminosis, Uromitexan for Morbus Wegener, Orfiril for epilepsy, Clexane for thrombosis prophylaxis, Depo-Clinovir for contraception, Kenephro for substitution, Tavor for sedation, Atosil for sedation, Kalinor for hypokalaemia. The patient was treated formerly with Prednisolon, paracetamol, Depo-Clinovir and Kalinor without experiencing any complaints.

Outcome: Recovering

The reporter assessed Steven-Johnson-Syndrome as possibly related to Euthyrox, prednisolon, paracetamol, Pantozol, Endoxan, Cernevit, Uromitexan, Orfiril, Clexane, Depo-Clinovir, Kenephro, Tavor, Atosil and Kalinor.

Sender's comment:

On 14 Oct 2004 a 33-year old female patient experienced Stevens-Johnson syndrome 26 days after starting levothyroxine treatment (dose unknown, start date: 18 Sep 2004) for hypothyroidism. On 01 Sep 2004 she was hospitalized for anorexia and unclear disorders of upper abdomen. From 11 Sep 2004 to 27 Sep 2004 she suffered from infective toxic shock (ITS) since pulmonary congestion. On 12 Oct 2004 she had a shunt implantation. On 14 Oct 2004 Steven-Johnson syndrome (SJS) was diagnosed. In the following 6 days she developed cockades, spots, vesicles and erosions of skin, oral mucosa erosion and conjunctivitis. Outcome: recovering. Action taken regarding levothyroxine: unknown. Medical history: M. Wegener, hypovitaminosis, epilepsy, hypokalemia. Co-suspect drugs: prednisolone*, paracetamol*, pantoprazol, cyclophosphamide, vitamins, mesna, valproic acid, enoxaparin, medroxyprogesteron*, lorazepam, enoxaparin, Kenephro, isopromethazine, potassium* (*former administered drugs without complaints). Causality by reporter: possibly related to levothyroxine and all other co-suspect drugs.

Company comment for levothyroxine: The reported serious Stevens-Johnson syndrome in compatible temporal relationship to levothyroxine intake. Latency: 26 days. Dechallenge: unknown. Stevens-Johnson syndrome is unlisted in the CCSI. Alternative explanations: can occur in response to infections (such as e.g. recent ITS) and illnesses but can also present with initial symptoms of respiratory infection (e.g. ITS) over 1-14 days, so it is unclear if ITS might have been a cause or symptom of SJS (in case of symptom of SJS temporal relationship would be incompatible); co-suspect pantoprazol, cyclophosphamide, valproic acid, mesna; all these co-suspect drugs were recently introduced. Causality: unlikely.

Company comment for prednisolone: The reported serious Stevens-Johnson syndrome in compatible temporal relationship to prednisolone intake. Latency: 1st dose: 36 days, last dose: 11 days. Dechallenge: n

Reporter's comment:

Sanofi-Aventis Deutschland GmbH, 21.02.2008 DE-AVENTIS-200810436DE

Sender diagnosis: not reported

Narrative

Initial

This case of Stevens-Johnson-Syndrome (SJS) was reported to us from the documentation centre for severe skin reactions in Freiburg im Breisgau, Germany, Prof. Schoepf via the German Health Authority.

The centre is conducting a prospective study on the incidence and drug etiology of severe skin diseases. Diagnosis of the SJS was made by an expert panel and was assessed as "definite". Drug causality was assessed by the experts using an algorithm. As for enoxaparin (Clexane) the causality was assessed as "possible", for other concomitantly given drugs causality was assessed as:

Further suspected drugs were assessed as follows:

pantoprazole, mesna, valproic acid, levothyroxine, lorazepam: possible

unknown active ingredients (Kenephro): possible/questionable

Non-suspected concomitant drugs (questionable, unrelated): amphotericine B, furosemide, propofol, metoclopramide, midazolam, fentanyl, norepinephrine, ceftriaxone, Tracutal (multiple ingredients), flucloxacillin, glucose, cyclophosphamide, retinol + colecalciferol + a-tocopherol, prednisolone, medroxyprogesteron, insulin human, phytomenadione, morphine, metronidazole, promethazine, potassium, paracetamol

Epicrisis:

A 33-year-old female patient with a medical history of Wegener's disease and epilepsy was hospitalised on 01-Sep-2004 because of inappetence and upper abdominal discomfort of unknown origin. From 11-Sep-2004 to 27-Sep-2004 she was in an intensive care unit due to pulmonary congestion.

From 17-Sep to 15-Oct-2004 the above reported medication was given for thrush, edemas, sedation, nausea, intubation, pulmonary insufficiency, ulcer prophylaxis, Wegener's disease, epilepsy, contraception, hyperglycemia, hypothyroidism, coagulation disorder, pain, staphylococcus aureus sepsis, hypokalemia and fever.

On 12-Oct-2004 the patient underwent implantation of shunt due to dialysis dependency.

On 14-Oct-2004 the patient developed cockades and spots on skin and on 15-Oct-2004 first skin blisters appeared and SJS was diagnosed. On 19-Oct-2004 she developed lip and oral mucous membrane erosions and on 20-Oct-2004 conjunctivitis. Further clinical course was not reported.

As stop date for the event of SJS 22-Dec-2004 was given but the patient was not considered as recovered at time of report.

As possible alternative explanation besides the concomitant medication Wegener's disease was given.

Sender's comment:

19-Feb-2008 Sanofi-Aventis Company : A 33-year-old female patient suffered from STEVENS JOHNSON SYNDROME in connection with treatment with multiple drugs including enoxaparin. The temporal relationship may imply a causal relationship between enoxaparin and the event. However, considering the adverse drug reaction profile of enoxaparin it's more likely that the event was the consequence of therapy with the concomitant drugs, in particular valproic acid.

19-FEB-2008 Sanofi Aventis Germany: STEVENS JOHNSON SYNDROME is considered locally unlabelled for the suspected product. The facts presented here however are not very suggestive of a causal relationship (please refer to the intl. evaluator's comment), and thus do not imply a change in the state of the scientific knowledge for the product. Measures according to Para 29 German Drug Law (AMG), section (1) concerning the approval status are not deemed necessary.

Reporter's comment:

Baxter, 21.02.2008 DE-BAXTER-2008BH001367

Sender diagnosis: not reported

Narrative

This is a spontaneous report of Stevens-Johnson syndrome and congestive lung in a 33 year old female patient from Germany after therapy with Uromitexan, Endoxan and Cernevit which was received by Baxter Germany (country operations reference DE2008-0064) from a German Documentation Centre for Severe Skin Diseases (Ref.no.: 9160212) via German regulatory authority (BfArM, Ref.no.: DE-BFARM-08014087).

On 01Sep2004, the patient was hospitalized due to loss of appetite and unclear upper abdominal complaints. On 10Sep2004 therapy with Endoxan (cyclophosphamide, 200 mg/150 mg, PO) together with Uromitexan (mesna, 200 mg, PO) for the treatment of Morbus Wegener was started. Concomitantly she was also administered Prednisolon (prednisolone, IV) from 08Sep2004 until 03Oct2004 for the Morbus Wegener.

From 11Sep2004 until 27Sep2004 the patient received intensified care in intensive care unit due to congested lung which was not reported as an adverse event by the German Documentation Centre for Severe Skin Diseases. On 14Sep2004 therapy with Cernevit injection solution at 1 ampoule intravenously was started due to a hypovitaminosis. Therapy with Endoxan and Uromitexan was discontinued on 02Oct2004. Therapy with Cernevit was discontinued on 11Oct2004. On 12Oct2004 the patient received a shunt implant due to a dialysis obligation. On 14Oct2004 the patient developed skin cockades and skin spots. The start of Stevens-Johnson syndrome was assessed on 14Oct2004. On 15Oct2004 the patient developed vesicular eruptions. On 19Oct2004 the patient developed lip and mouth mucosal skin erosion and on 20Oct2004 conjunctivitis.

According to a compiled drug anamnesis report, the patient received multiple medications prior to or during hospitalization for multiple indications. The causality assessment for these multiple drugs were also provided in the drug anamnesis report. The following listed drugs containing the number 5 in the parenthesis were reported as possibly related to the reported Stevens-Johnson syndrome. Those with the number 6 were reported as questionable and the drugs with the number 7 in the parenthesis were reported as not related to the Stevens-Johnson syndrome.

The patient received Depo-Clinovir (6, medroxyprogesteron, IM) for contraception from 01Jun2004. Glucose (7, IV) was administered for substitution from 02Sep2004 until 30Sep2004. On 11Sep2004, the patient received Ampho-Moronal (7, amphotericin B, PO) for canker. From 11Sep2004 Kenephro (5/6) was administered for substitution, Ceftriaxon (7, ceftriaxone, IV) for a sepsis and Pantozol (5, pantoprazol, IV) for ulcers prophylaxis until 26Sep2004 and Clexane (5, enoxaparin-sodium, IV) for thrombosis prophylaxis.

From 12Sep2004 she received Orfiril (5, valproic acid) for epilepsy. From 13Sep2004 she received Disoprivan (7, propofol, IV) for sedation until 18Sep2004, from 14Sep2004 she received Paspertin (7, metoclopramide, IV) for nausea until 19Sep2004, Arterenol (7, norepinephrin, IV) for intubation until 25Sep2004 and Tracutil (7, ZnCl₂, FeCl₂, MnCl₂, CuCl₂, CrCl₃) for substitution until 26Sep2004.

From 15Sep2004 until 27Sep2004 she received Flucloxacillin (7, flucloxacillin, IV) for sepsis. On 17Sep2004 she received Lasix spez. (7, furosemide, IV) for oedema. From 17Sep2004 until 23Sep2004 she received Dormicum (7, midazolam, IV) for intubation and Fentanyl (fentanyl, IV) for pulmonary insufficiency.

Further concomitant medications included Insuman Rapid (7, Insulin human) administered for hyperglycemia on 18 to 20 Sep2004 and from 23Sep2004 to 28Sep2004, Euthyrox (5, levothyroxin, PO) administered for hypothyreose from 18Sep2004, Konakion (7, phytomenadion, IV) for abnormal coagulation from 19Sep2004 until 28Sep2004, Furosemide (7, furosemide) for oedema from 21 to 22Sep2004, Morphine (7, morphine) for pain from 23Sep until 29Sep2004, Tavor (5, lorazepam, IV) for sedation from 23Sep2004, Atosil (6, promethazin, PO) for sedation from 26Sep until 03Oct2004, Clont (7, metronidazol, IV) for Staphylococcus aureus sepsis from 24Sep until 30Sep2004, Pantozol (5, pantoprazol, PO) for ulcer prophylaxis from 27Sep2004, Kalinor (6, potassium citrate, KHCO₃) for a decreased potassium level administered from 28Sep until 13Oct2004. Paracetamol suppository (6, paracetamol) was administered for fever on 01Oct2004.

The dates when the indications for these drugs were first noticed or diagnosed were not available.

The causalities with Endoxan and Cernevit were assessed as questionable, the Uromitexan as possibly related. The causality assessment for Prednisolon, reported as a concomitant medication, was reported as questionable.

The Stevens-Johnson syndrome persisted until 22Dec2004 and the patient had not recovered at the time of reporting. The adverse event was not considered as life-threatening by the reporting documentation centre. The reporting documentation centre assessed the causality of the event as unrelated to infection and possibly related to the Morbus Wegener.

Sender's comment:

Local expectedness for Germany.

Uromitexan:

Stephen-Johnson-Syndrome - expected,

Congestive lung " unexpected.

Cernevit:

Stephen-Johnson-Syndrome - unexpected,

Congestive lung " unexpected.

Endoxan:

Stephen-Johnson-Syndrome - expected,

Congestive lung " expected.

This case has been assessed locally as unexpected.

The reported case does not change the current state of present scientific knowledge. No changes regarding the product statements and documentation of the marketing authorisation or approval notification are necessary (paragraph 29 AMG, I.1.).

This is a spontaneous report of pulmonary congestion and Stevens-Johnson syndrome (SJS) occurring in a 33 year old female after administration of mesna, cyclophosphamide, and Cernevit for the treatment of "Morbus Wegener". This case is serious. The temporal sequence of events supports causality for pulmonary congestion on the part of mesna and cyclophosphamide, but not Cernevit (congestion predated Cernevit administration). The temporal sequence of events does not support causality for SJS on the part of cyclophosphamide or mesna (12 days elapsed between exposure to these 2 drugs and the onset of SJS). The patient received many medications in the days immediately preceding SJS (including Cernevit) and this is a more plausible explanation. Baxter is unable to exclude causality on the part of Cernevit for the event of SJS. The overall case assessment is serious and related (pulmonary congestion-cyclophosphamide and mesna, SJS-Cernevit).

19Feb2008

Reporter's comment:

Baxter, 26.02.2008 DE-BAXTER-2008BH001367

Sender diagnosis: not reported

Narrative

This is a spontaneous report of Stevens-Johnson syndrome and congestive lung in a 33 year old female patient from Germany after therapy with Uromitexan, Endoxan and Cernevit which was received by Baxter Germany (country operations reference DE2008-0064) from a German Documentation Centre for Severe Skin Diseases (Ref.no.: 9160212) via German regulatory authority (BfArM, Ref.no.: DE-BFARM-08014087).

On 01Sep2004, the patient was hospitalized due to loss of appetite and unclear upper abdominal complaints. On 10Sep2004 therapy with Endoxan (cyclophosphamide, 200 mg/150 mg, PO) together with Uromitexan (mesna, 200 mg, PO) for the treatment of Morbus Wegener was started. Concomitantly she was also administered Prednisolon (prednisolone, IV) from 08Sep2004 until 03Oct2004 for the Morbus Wegener.

From 11Sep2004 until 27Sep2004 the patient received intensified care in intensive care unit due to congested lung which was not reported as an adverse event by the German Documentation Centre for Severe Skin Diseases. On 14Sep2004 therapy with Cernevit injection solution at 1 ampoule intravenously was started due to a hypovitaminosis. Therapy with Endoxan and Uromitexan was discontinued on 02Oct2004. Therapy with Cernevit was discontinued on 11Oct2004. On 12Oct2004 the patient received a shunt implant due to a dialysis obligation. On 14Oct2004 the patient developed skin cockades and skin spots. The start of Stevens-Johnson syndrome was assessed on 14Oct2004. On 15Oct2004 the patient developed vesicular eruptions. On 19Oct2004 the patient developed lip and mouth mucosal skin erosion and on 20Oct2004 conjunctivitis.

According to a compiled drug anamnesis report, the patient received multiple medications prior to or during hospitalization for multiple indications. The

causality assessment for these multiple drugs were also provided in the drug anamnesis report. The following listed drugs containing the number 5 in the parenthesis were reported as possibly related to the reported Stevens-Johnson syndrome. Those with the number 6 were reported as questionable and the drugs with the number 7 in the parenthesis were reported as not related to the Stevens-Johnson syndrome.

The patient received Depo-Clinovir (6, medroxyprogesteron, IM) for contraception from 01Jun2004. Glucose (7, IV) was administered for substitution from 02Sep2004 until 30Sep2004. On 11Sep2004, the patient received Ampho-Moronal (7, amphotericin B, PO) for canker. From 11Sep2004 Kenephro (5/6) was administered for substitution, Ceftriaxon (7, ceftriaxone, IV) for a sepsis and Pantozol (5, pantoprazol, IV) for ulcers prophylaxis until 26Sep2004 and Clexane (5, enoxaparin-sodium, IV) for thrombosis prophylaxis.

From 12Sep2004 she received Orfiril (5, valproic acid) for epilepsy. From 13Sep2004 she received Disoprivan (7, propofol, IV) for sedation until 18Sep2004, from 14Sep2004 she received Paspertin (7, metoclopramide, IV) for nausea until 19Sep2004, Arterenol (7, norepinephrin, IV) for intubation until 25Sep2004 and Tracutil (7, ZnCl₂, FeCl₂, MnCl₂, CuCl₂, CrCl₃) for substitution until 26Sep2004.

From 15Sep2004 until 27Sep2004 she received Flucloxacillin (7, flucloxacillin, IV) for sepsis. On 17Sep2004 she received Lasix spez. (7, furosemide, IV) for oedema. From 17Sep2004 until 23Sep2004 she received Dormicum (7, midazolam, IV) for intubation and Fentanyl (fentanyl, IV) for pulmonary insufficiency.

Further concomitant medications included Insuman Rapid (7, Insulin human) administered for hyperglycemia on 18 to 20 Sep2004 and from 23Sep2004 to 28Sep2004, Euthyrox (5, levothyroxin, PO) administered for hypothyreose from 18Sep2004, Konakion (7, phytomenadion, IV) for abnormal coagulation from 19Sep2004 until 28Sep2004, Furosemide (7, furosemide) for oedema from 21 to 22Sep2004, Morphine (7, morphine) for pain from 23Sep until 29Sep2004, Tavor (5, lorazepam, IV) for sedation from 23Sep2004, Atosil (6, promethazin, PO) for sedation from 26Sep until 03Oct2004, Clont (7, metronidazol, IV) for Staphylococcus aureus sepsis from 24Sep until 30Sep2004, Pantozol (5, pantoprazol, PO) for ulcer prophylaxis from 27Sep2004, Kalinor (6, potassium citrate, KHCO₃) for a decreased potassium level administered from 28Sep until 13Oct2004. Paracetamol suppository (6, paracetamol) was administered for fever on 01Oct2004.

The dates when the indications for these drugs were first noticed or diagnosed were not available.

The causalities with Endoxan and Cernevit were assessed as questionable, the Uromitexan as possibly related. The causality assessment for Prednisolon, reported as a concomitant medication, was reported as questionable.

The Stevens-Johnson syndrome persisted until 22Dec2004 and the patient had not recovered at the time of reporting. The adverse event was not considered as life-threatening by the reporting documentation centre. The reporting documentation centre assessed the causality of the event as unrelated to infection and possibly related to the Morbus Wegener.

FOLLOW-UP INFORMATION (19Feb2008):

The Documentation Centre for Severe Skin Diseases reported that the patient had not previously received Uromitexan and Endoxan.

Sender's comment:

Local expectedness for Germany.

Uromitexan:

Stephen-Johnson-Syndrome - expected,

Congestive lung "unexpected".

Cernevit:

Stephen-Johnson-Syndrome - unexpected,

Congestive lung "unexpected".

Endoxan:

Stephen-Johnson-Syndrome - expected,

Congestive lung "expected".

This case has been assessed locally as unexpected.

The reported case does not change the current state of present scientific knowledge. No changes regarding the product statements and documentation of the marketing authorisation or approval notification are necessary (paragraph 29 AMG, I.1.).

Follow-up information (19Feb2008) does not change the previous overall assessment. The case remains serious and related.
26Feb2008

This is a spontaneous report of pulmonary congestion and Stevens-Johnson syndrome (SJS) occurring in a 33 year old female after administration of mesna, cyclophosphamide, and Cernevit for the treatment of "Morbus Wegener". This case is serious. The temporal sequence of events supports causality for pulmonary congestion on the part of mesna and cyclophosphamide, but not Cernevit (congestion predated Cernevit administration). The

temporal sequence of events does not support causality for SJS on the part of cyclophosphamide or mesna (12 days elapsed between exposure to these 2 drugs and the onset of SJS). The patient received many medications in the days immediately preceding SJS (including Cernevit) and this is a more plausible explanation. Baxter is unable to exclude causality on the part of Cernevit for the event of SJS. The overall case assessment is serious and related (pulmonary congestion-cyclophosphamide and mesna, SJS-Cernevit).

19Feb2008

Reporter's comment:

Wyeth-Pharma GmbH, 29.02.2008 DE-WYE-G01066808

Sender diagnosis: not reported

Narrative

New version was created to change clock start date to 15-Feb-2008.

Additional information was received regarding business partner reference number.

Information was received from a registry via a regulatory authority regarding a 33-year-old female patient who received Tavor (lorazepam injection) therapy and experienced hemodialysis shunt placement and Stevens-Johnson syndrome.

MEDICAL HISTORY:

The patient's concurrent illnesses include Wegener's granulomatosis and epilepsy. Past therapies included Ampho-Moronal (amphotericin b), Lasix Special (furosemide), Disoprivan (propofol), Paspertin (metoclopramide hydrochloride), Dormicum (midazolam maleate), Fentanyl and Furosemide.

PRODUCT DETAILS:

Indication for Tavor was sedation. Therapy began on 23-Sep-2004. It was unknown if therapy continued. Dose regimen was 1 mg 1 time per day (intravenous).

Additional suspect medication included Pantozol (pantoprazole injection), Endoxan (cyclophosphamide), Cernevit (vitamins nos), Prednisolon (prednisolone), Uromitexan (mesna), Orfiril (valproate sodium), Clexane (heparin-fraction, sodium salt), Depo-Clinovir (medroxyprogesterone acetate), Aminosteril Ke Nephro (amino acids nos/vitamins nos), Euthyrox (levothyroxine sodium), Atosil (isopromethazine hydrochloride), Pantozol (pantoprazole tablet, delayed release), Kalinor-Brausetabletten (potassium carbonate/potassium citrate tablet) and Paracetamol. Wyeth Pharma is not the marketing authorisation holder for Pantozol (pantoprazole) in Germany.

CONCOMITANT THERAPY:

Concomitant therapy included Arterenol (norepinephrine hydrochloride), Ceftriaxone, Tracutal (chromic chloride/copper chloride/manganese chloride/zinc chloride), Flucloxacillin, Glucose (glucose injection injection), Insuman (insulin human injection, isophane/insulin zinc protamine injection), Konakion (phytomenadione), Morphine and Clont (metronidazole).

EVENT DETAILS:

The patient had already been hospitalized since 01-Sep-2004 because of anorexia and vague upper abdominal problems. She was moved to the intensive care unit on 11-Sep-2004 because of pulmonary edema and stayed there until 27-Sep-2004. The patient received a hemodialysis shunt placement (arteriovenous shunt operation) on 12-Oct-2004. The patient experienced Stevens-Johnson syndrome (Stevens-Johnson syndrome) on 14-Oct-2004.

The event was described as follows: 'On 14-Oct-2004 the patient experienced blotchy changes of skin. On 15-Oct-2004 she experienced blisters. On 19-Oct-2004 she experienced erosions of oral mucous membrane and lips. On 20-Oct-2004 the patient experienced a conjunctivitis.'

The patient has not recovered yet.

RELATEDNESS ASSESSMENT:

The dermatological review committee has categorized Tavor as possibly related to skin reactions. Tavor has an assessment of 5 (range 1 stands for certain relationship to event, range 8 unlikely relationship). The dermatological review committee categorized Pantozol, Uromitexan, Orfiril, Clexane, Aminosteril Ke Nephro and Euthyrox as possibly related to Stevens-Johnson syndrome. All other suspect drugs got a lower degree of probability. According to dermatological review committee the drugs of the concomitant therapy have no temporal relationship to reaction or are not known for causing Stevens-Johnson syndrome.

The assessments of the investigator for arteriovenous shunt operation were unknown.

Sender's comment:

WYETH COMMENT

INITIAL (v5) 28-Feb-2008

STEVENS JOHNSON SYNDROME unter Tavor

Neue Informationen wurden erhalten bezüglich der 'business partner reference number'.

Bericht vom Dokumentationszentrum für schwere Hautreaktionen in Freiburg.

Eine 33-jährige Patientin wurde auf Grund von Appetitlosigkeit und unklarer Oberbauchbeschwerden seit 01-Sep-2004 stationär behandelt. Vom 11-Sep-2004 bis zum 27-Sep-2004 wurde sie wegen pulmonaler Stauung auf der Intensivstation versorgt. Zur Sedierung erhielt die Patientin seit dem 23

Sep-2004 Tavor Injektionslösung. Eine Shuntimplantation wegen Dialysepflichtigkeit wurde am 12-Oct-2004 durchgeführt. Am 14-Oct-2004 zeigte die Patientin dann Stevens Johnson Syndrom, zunächst durch Kokarden und Flecken, dann mit Hautblasen und es folgten Lippen- und Mundschleimhaut-Erosionen und Konjunktivitis. Die Patientin wurde noch nicht wiederhergestellt. Der Zusammenhang zwischen dem Stevens Johnson Syndrom und Tavor wurde nach dem Bewertungsalgorithmus des Dokumentationszentrums als möglich beurteilt.

Ein möglicher Zusammenhang kann nicht ausgeschlossen werden.

Stevens Johnson Syndrom ist in der Fachinformation nicht aufgeführt und wurde daher als unbekannt eingestuft.

Basierend auf der aktuellen Information führt dieser Bericht nicht zu einer Änderung des Nutzen-Risiko-Verhältnisses. Zu diesem Zeitpunkt sind keine weiteren Maßnahmen vorgesehen. Sobald Änderungen hierzu bekannt werden, werden wir darüber unverzüglich und unaufgefordert informieren.

Birte Kruse

Michael Pfleger

Reporter's comment:

APHQWEB, 29.02.2008 DE-Nycomed GmbH-80334

Sender diagnosis: not reported

Narrative

This report was received from the German Documentation centre of severe skin reactions via the German Health authority (BfArM ref. no. DE-BFARM-08014087).

This is a serious case due to medically important condition.

A middle-aged female patient experienced Stevens-Johnson-syndrome while receiving pantoprazole (Pantozol i.v. and Pantozol 40 mg p.o.) for ulcer prophylaxis. Further 30 suspected drugs were administered.

On 01-SEP-2004 patient had been admitted to hospital due to loss of appetite and epigastric complaints of unknown origin. From 11-SEP until 27-SEP-2004, the patient was in an intensive care unit due to pulmonary congestion, where she received pantoprazole (Pantozol i.v.). On 27-SEP, the patient was discharged from the intensive care unit and Pantozol i.v. was switched to Pantozol 40 mg po.

In intensive care unit, the patient received antibiotic treatment with ceftriaxon and flucloxacilline for sepsis. Starting 24-SEP, the patient received treatment with metronidazole for Staphylococcus aureus sepsis.

On 12-OCT-2004 patient had a shunt implantation due to requirement of dialysis.

On 14-OCT-2004 patient developed cockades and skin spots.

On 15-OCT-2004 patient developed skin blisters.

On 19-OCT-2004 patient developed erosions of lips and oral mucous membranes.

On 20-OCT-2004 patient experienced conjunctivitis.

The outcome was reported as still ongoing at the time of the report.

Concomitant diseases included soor, oedema, nausea, pulmonary insufficiency, sepsis, Wegener's granulomatosis, hypovitaminosis, epilepsy, hyperglycaemia, hypothyreodism, coagulation disorder, pain, hypokalaemia, fever and sepsis due to staphylococcus aureus infection.

Sender's comment:

The Monitoring Project assessed the causality with regard to the patient's multiple medication (using an algorithm with 8 different levels ranging from 1 =definite to 8=improbable). Causality to pantoprazole p.o. and i.v. was rated as possible.

STEVENS-JOHNSON-SYNDROME

Patient developed Stevens-Johnson-Syndrome approximately four weeks after start of pantoprazole treatment.

Concomitantly, the patient received multiple other drugs (further 30 suspected drugs).

Stevens-Johnson-Syndrome (SJS) is a rare skin disorder, characterized by mucosal erosions, skin blisters, often accompanied by sore throat and fever. SJS can be drug-induced or due to infectious diseases.

Several drugs have been observed to cause Stevens-Johnson-Syndrome. Toxic bullous skin reactions up to acute disseminated epidermal necrolysis have also been observed in very rare cases with pantoprazole and other proton pump inhibitors. Temporal relationship basically plausible. Mockenhaupt et al (2008) suggest, that SJS often begins more than four and up to 28 days after the initiation of the responsible medication. Patient's pantoprazole treatment had been initiated approximately four weeks before onset of the reported events. However, the patient received multiple other suspected drugs, some of which are known to be associated with severe skin reactions (i.e. ceftriaxon, flucloxacillin, mesna, valproic acid and others). In addition, treatment for (Staphylococcus aureus) sepsis and Wegener's granulomatosis were reported for this patient. In the presence of multiple other suspected drugs and a bacterial infection, it remains difficult to determine a single trigger for the reported event. Therefore Nycomed agrees with the reporter and conservatively assesses this case as possible. This report is not considered to have any influence on current knowledge regarding the safety profile of pantoprazole and has no effect on the status of the drug as reflected by the formal product characteristics.

Reporter's comment:

APHQWEB, 04.03.2008 DE-Nycomed GmbH- 80334

Sender diagnosis: not reported

Narrative

This report was received from the German Documentation centre of severe skin reactions via the German Health authority (BfArM ref. no. DE-BFARM-08014087).

This is a serious case due to medically important condition.

A middle-aged female patient experienced Stevens-Johnson-syndrome while receiving pantoprazole (Pantozol i.v. and Pantozol 40 mg p.o.) for ulcer prophylaxis. Further 30 suspected drugs were administered.

On 01-SEP-2004 patient had been admitted to hospital due to loss of appetite and epigastric complaints of unknown origin. From 11-SEP until 27-SEP-2004, the patient was in an intensive care unit due to pulmonary congestion, where she received pantoprazole (Pantozol i.v.). On 27-SEP, the patient was discharged from the intensive care unit and Pantozol i.v. was switched to Pantozol 40 mg po.

In intensive care unit, the patient received antibiotic treatment with ceftriaxon and flucloxacilline for sepsis. Starting 24-SEP, the patient received treatment with metronidazole for Staphylococcus aureus sepsis.

On 12-OCT-2004 patient had a shunt implantation due to requirement of dialysis.

On 14-OCT-2004 patient developed cockades and skin spots.

On 15-OCT-2004 patient developed skin blisters.

On 19-OCT-2004 patient developed erosions of lips and oral mucous membranes.

On 20-OCT-2004 patient experienced conjunctivitis.

The outcome was reported as still ongoing at the time of the report.

Concomitant diseases included soor, oedema, nausea, pulmonary insufficiency, sepsis, Wegener's granulomatosis, hypovitaminosis, epilepsy, hyperglycaemia, hypothyreodism, coagulation disorder, pain, hypokalaemia, fever and sepsis due to staphylococcus aureus infection. STEVENS-JOHNSON-SYNDROME

This patient developed Stevens-Johnson-Syndrome approximately four weeks after start of pantoprazole treatment. Concomitantly, the patient received multiple other drugs (further 30 suspected drugs).

Stevens-Johnson-Syndrome (SJS) is a rare skin disorder, characterized by mucosal erosions, skin blisters, often accompanied by sore throat and fever. SJS can be drug-induced or due to infectious diseases.

Several drugs have been observed to cause Stevens-Johnson-Syndrome. Toxic bullous skin reactions up to acute disseminated epidermal necrolysis have also been observed in very rare cases with pantoprazole and other proton pump inhibitors.

Temporal relationship basically plausible. Mockenhaupt et al (2008) suggest, that Stevens-Johnson syndrome often begins more than four and up to 28 days after the initiation of the responsible medication. The patient's pantoprazole treatment had been initiated approximately four weeks before onset of the reported events. However, the patient received multiple other suspected drugs, some of which are known to be associated with severe skin reactions (i.e. ceftriaxon, flucloxacillin, mesna, valproic acid and others).

In addition, treatment for (Staphylococcus aureus) sepsis and Wegener's granulomatosis were reported for this patient.

Sender's comment:

The Monitoring Project assessed the causality with regard to the patient's multiple medication (using an algorithm with 8 different levels ranging from 1 =definite to 8=improbable). Causality to pantoprazole p.o. and i.v. was rated as "possible".

In the presence of multiple other suspected drugs and a bacterial infection, it remains difficult to determine a single trigger for the reported event. Therefore Nycomed agrees with the reporter and conservatively assesses this case as "possible".

This report is not considered to have any influence on current knowledge regarding the safety profile of pantoprazole and has no effect on the status of the drug as reflected by the formal product characteristics.

Reporter's comment:

SANOFI-SYNTHELABO, 19.03.2008 DE-SANOFI-SYNTHELABO-A01200802779

Sender diagnosis: not reported

Narrative

Reaction as reported: stevens johnson syndrome

LLT: STEVENS JOHNSON SYNDROME / 10042030

PT: STEVENS-JOHNSON SYNDROME / 10042033

Reaction as reported: cockades and skin spots

LLT: SKIN DISORDER / 10040831

PT: SKIN DISORDER / 10040831

Reaction as reported: skin blisters

LLT: BLISTERS / 10005216

PT: BLISTER / 10005191

Reaction as reported: erosions of lips and oral mucous membranes

LLT: LIP EROSION / 10051992

PT: LIP EROSION / 10051992

Reaction as reported: erosions of lips and oral mucous membranes

LLT: ORAL MUCOSA EROSION / 10064594

PT: ORAL MUCOSA EROSION / 10064594

Reaction as reported: conjunctivitis

LLT: CONJUNCTIVITIS / 10010741

PT: CONJUNCTIVITIS / 10010741

Narrative:

Initial information received from Abbott Laboratories (08P-062-0439710-00) on 06-Mar-2008.

This report of Stevens Johnson Syndrome coincident with pantoprazole IV (Pantozol IV) therapy (Study ID XRP4563ZSCHOEPPF) was received by Nycomed (80334) from the German Documentation Centre for severe skin reactions via the German Health Authority.

A 33-year-old female patient had a medical history of Wegener's granulomatosis, pain, nausea and vomiting, hypothyroidism, candidiasis, pulmonary insufficiency, hypovitaminosis, coagulation disorder, hyperglycaemia, edema and hypokalemia.

On 01-Sep-2004, the patient was admitted to hospital because of loss of appetite and epigastric complaints of unknown origin. From 11-Sep-2004 to 27-Sep-2004, the patient was in intensive care unit due to pulmonary congestion.

The patient was receiving oral valproate (Orfiril), 10 ml twice daily since 09-Sep-2004 and 600 mg twice daily since 12-Sep-2004 for epilepsy. Co

suspect medication included pantoprazole, levothyroxine, lorazepam and uromitexan.

From 17-Sep to 15-Oct-2004, she had also been treated with amphotericin B, furosemide, propofol, metoclopramide, nitrazepam, fentanyl, norepinephrine, ceftriaxone, Tracutal, flucloxacillin, glucose, cyclophosphamide, prednisolone, medroxyprogesteron, insulin, phytomeniadone, morphine, metronidazole, isopromethazine, potassium, multivitamin preparation, enoxaparin, protein supplements and paracetamol.

On 12-Oct-2004, the patient had a shunt implantation due to requirement of dialysis.

On 14-Oct-2004, he developed cockades and skin spots.

On 15-Oct-2004, he developed skin blisters.

On 19-Oct-2004, he developed erosions of lips and oral mucous membranes.

On 20-Oct-2004, he experienced conjunctivitis.

Action taken regarding valproate was not provided.

At the time of reporting, the events were still ongoing.

No more information was available.

Sender's comment:

Reporter's comment:

SANOFI-SYNTHELABO, 16.06.2008 DE-SANOFI-SYNTHELABO-A01200802779

Sender diagnosis: not reported

Narrative

Reaction as reported: stevens johnson syndrome

LLT: STEVENS JOHNSON SYNDROME / 10042030

PT: STEVENS-JOHNSON SYNDROME / 10042033

Reaction as reported: cockades and skin spots

LLT: SKIN DISORDER / 10040831

PT: SKIN DISORDER / 10040831

Reaction as reported: skin blisters

LLT: BLISTERS / 10005216

PT: BLISTER / 10005191

Reaction as reported: erosions of lips and oral mucous membranes

LLT: LIP EROSION / 10051992

PT: LIP EROSION / 10051992

Reaction as reported: erosions of lips and oral mucous membranes

LLT: ORAL MUCOSA EROSION / 10064594

PT: ORAL MUCOSA EROSION / 10064594

Reaction as reported: conjunctivitis

LLT: CONJUNCTIVITIS / 10010741

PT: CONJUNCTIVITIS / 10010741

Narrative:

Initial information received from Abbott Laboratories (08P-062-0439710-00) on 06-Mar-2008.

This report of Stevens Johnson Syndrome coincident with pantoprazole IV (Pantozol IV) therapy (Study ID XRP4563ZSCHOEPPF) was received by Nycomed (80334) from the German Documentation Centre for severe skin reactions via the German Health Authority.

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27-Sep-2004, the patient was in intensive care unit due to pulmonary congestion.

The patient was receiving oral valproate (Orfiril), 10 ml twice daily since 09-Sep-2004 and 600 mg twice daily since 12-Sep-2004 for epilepsy. Co-suspect medication included pantoprazole, levothyroxine, lorazepam and uromitexan.

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On 12-Oct-2004, the patient had a shunt implantation due to requirement of dialysis.

On 14-Oct-2004, he developed cockades and skin spots.

On 15-Oct-2004, he developed skin blisters.

On 19-Oct-2004, he developed erosions of lips and oral mucous membranes.

On 20-Oct-2004, he experienced conjunctivitis.

Action taken regarding valproate was not provided.

At the time of reporting, the events were still ongoing.

No more information was available.

Corrected CIOMS on 12-Jun-08.

After internal review, valproate (Orfiril) is not a sanofi-aventis drug. Therefore, this case will be deleted.

Sender's comment:

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Report from study

case: DE-BFARM-08014089

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	Yes		

Patient

Initials:	Sex: Male	Birth: privacy	Age: 12 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Erythema exudativum multiforme	20050724	20050804	12 Day	6 Day	not recovered/not resolved
2 Oral lesion	20050724	20050804	12 Day		Unknown
3 Blister	20050724	20050804	12 Day		Unknown
4 Burning eyes	20050724	20050804	12 Day		Unknown
5 Conjunctivitis	20050724	20050804	12 Day		Unknown
6 Fever	20050724	20050804	12 Day		Unknown
7 Mucosal erosion NOS	20050724	20050804	12 Day		Unknown
8 Mucous membrane disorder	20050724	20050804	12 Day		Unknown
9 Skin lesion	20050724	20050804	12 Day		Unknown
10 Lip erosion	20050724	20050804	12 Day		Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 bronchicum primula rhizome; thyme extract	Oral	/	20050718-20050726	Cough 9 Day
2 bronchicum primula rhizome; thyme extract	Oral	/Day	20050718-20050726	Coughing 9 Day
3 CEFUROXIM cefuroxime sodium	Oral	/	20050718-20050723	Cough 6 Day
4 Paracetamol paracetamol	Rectal	/ Suppository	20050717-20050726	Fever 10 Day
5 PARACETAMOL (NON-BI) paracetamol	Rectal	/ Suppository	20050717-20050726	Fever 10 Day
6 silomat clobutinol hydrochloride	Oral	/	20050718-20050726	Cough 9 Day
7 SILOMAT /00096702/ clobutinol hydrochloride	Oral	/	20050718-20050726	Coughing 9 Day

Drugs (concomitant)

8 berberil tetryzoline hydrochloride	Ophthalmic	/	20050724-20050726	Eye pain 3 Day
9 centrum minerals nos; vitamins nos	Oral	/	20050301-	Nutritional support -
10 CENTRUM A-Z minerals nos; vitamins nos	Oral	/	20050301-	Therapeutic procedure -
11 COTRIM sulfamethoxazole; trimethoprim	Oral	/	20050724-20050726	Coughing 3 Day
12 TANNOSYNT urea-cresol-sulfonate sodium	Topical	/Day	20050726-	Itching -

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Flu		20050720	Unknown	grippaler Infekt

Primary source

Qualification:	Physician	observestudytype:	Other studies
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Literature:

Narratives (nur bei elektronischem Eingang)

Boehringer Ingelheim GmbH, Germany, 22.02.2008 DE-Boehringer Ingelheim GmbH, Germany-2008-DE-01032DE

Sender diagnosis: not reported

Narrative

Initial information has been received from the 'centre of documentation of severe skin diseases' in Freiburg, Germany via the German health authority.

This case was identified and registered by the "centre of documentation of severe skin diseases" in Freiburg, Germany and reported to the federal health office. The "centre" collects and files nationwide case reports of severe skin diseases.

Therefore, these reports must not be confused with documentation of suspected adverse drug reactions.

While on therapy with Silomat (clobutinol) 1x1/d for cough, paracetamol 5 supp. all in all for fever, Cefuroxim 2x1/d for cough and Bronchicum (herbal ingredients) for cough a 12-year-old male patient experienced on 24Jul2005 an erythema exsudativum multiforme majus (course: 24Jul2005: fever, burning eyes, lip erosions; 25Jul2005: cockades and spots, blisters of the skin, erosions of oral mucosa; 26Jul2005: redness of genital mucosa, erosions of nasal mucosa; 27Jul2005: conjunctivitis).

An erythema exsudativum multiforme majus was diagnosed on 24Jul2005 and the patient was hospitalised on 27Jul2005.

Co-medication included Centrum A-Z (vitamin B1, B6, E riboflavine, nicotinamid).

Co-diagnoses included flu

The reporting "centre" assumed the event not to be life-threatening and rated the causal relationship between the erythema exsudativum multiforme majus and Silomat, paracetamol, cefuroxim and Bronchicum as possible.

Sender's comment:

Reporter's comment:

Sanofi-Aventis Deutschland GmbH, 26.02.2008 DE-AVENTIS-200810484DE

Sender diagnosis: not reported

Narrative

Initial report: This case of erythema exsudativum multiforme majus (EEMM) was received via the German health authority (DE-BFARM-08014089). It was reported by the German Documentation Centre for Severe Skin Reactions in Freiburg / Breisgau, Germany (code-no. 9170318). The centre is conducting a prospective study on the incidence and drug aetiology of severe skin reactions. Diagnosis of EEMM was made by an expert panel who assessed it as a "definite case" of EEMM. Causal relationship was assessed by the experts in September 2005 using an internal standardized algorithm.

As for primula w/thyme (Bronchicum drops) the causality was assessed as "possible". For the other suspected drug causality was assessed as: "possible": paracetamol, cefuroxime and clobutinol. Not considered as suspected were: a vitamin preparation (Centrum A-Z), tetrazoline, trimethoprim + sulfamethoxazole and a synthetic tannin.

Patient demography: male, 12 years. Medical history reported at the time of event: influenza like infection, cough and fever until 20-Jul-2005.

From 18-Jul to 26-Jul-2005, the patient was treated with primula w/thyme once per day (exact dose not reported) for his cough.

On 24-Jul-2005, the patient experienced again fever, and eye burning. The day after, on 25-Jul-2005, he presented with cockades and spots, skin blisters and erosions of oral mucous membrane. On 26-Jul-2005 redness of genital mucous membrane and erosions of nasal mucous membrane were present.

On 27-Jul-2005 the patient developed conjunctivitis, and he was hospitalised. The patient had not recovered at time when the report was made. Concomitant medication and influenza like infection was considered by the experts as possible alternative causes to explain the occurrences. No further information was provided with this report.

Sender's comment:

21-FEB-2008 sanofi-aventis company: Due to the compatible temporal relationship the causal role of primula / thyme extract cannot be excluded. However, based on the information presented in this case the concomitant medication as well as the intercurrent influenzal infection may provide a plausible alternative cause to explain the occurrence of EEMM.

21-FEB-2008 Sanofi-Aventis Germany: Marketing Authorization Holder for Bronchicum Tropfen (R) (primula / thyme extract) is represented by Cassella-med GmbH & Co. KG. Please refer to their evaluator's comment.

Reporter's comment:

Boehringer Ingelheim GmbH, Germany, 28.02.2008 DE-Boehringer Ingelheim GmbH, Germany-2008-DE-01032DE

Sender diagnosis: not reported

Narrative

Initial information has been received from the 'centre of documentation of severe skin diseases' in Freiburg, Germany via the German health authority.

This case was identified and registered by the "centre of documentation of severe skin diseases" in Freiburg, Germany and reported to the federal health office. The "centre" collects and files nationwide case reports of severe skin diseases.

Therefore, these reports must not be confused with documentation of suspected adverse drug reactions.

While on therapy with Silomat (clobutinol) 1x1/d for cough, paracetamol 5 supp. all in all for fever, Cefuroxim 2x1/d for cough and Bronchicum (herbal ingredients) for cough a 12-year-old male patient experienced on 24Jul2005 an erythema exsudativum multiforme majus (course: 24Jul2005: fever, burning eyes, lip erosions; 25Jul2005: cockades and spots, blisters of the skin, erosions of oral mucosa; 26Jul2005: redness of genital mucosa, erosions of nasal mucosa; 27Jul2005: conjunctivitis).

An erythema exsudativum multiforme majus was diagnosed on 24Jul2005 and the patient was hospitalised on 27Jul2005.

Co-medication included Centrum A-Z (vitamin B1, B6, E riboflavine, nicotinamid).

Co-diagnoses included flu

The reporting "centre" assumed the event not to be life-threatening and rated the causal relationship between the erythema exsudativum multiforme majus and Silomat, paracetamol, cefuroxim and Bronchicum as possible.

Sender's comment:

Regional Centre Drug Safety Boehringer Ingelheim

Comment on case 2008-DE-01032DE / SILOMAT

Erythema exsudativum multiforme majus (EEMM) is unlisted for clobutinol. Other factors have been reported in this case which may have caused the events, such as the other suspect drugs paracetamol, cefuroxim and Bronchicum (herbal medication containing thyme and primula root) or a flu-like infection.

As with all unlisted events, the company will continue to monitor all respective reports received and, based on cumulative experience, will re-evaluate the available evidence on an ongoing basis. No change in the company core safety information or other measures are recommended at this time.

PILZDEYN Regional Centre Drug Safety Boehringer Ingelheim

Case assessment created by PL on 27 Feb 2008

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Report from study

case: DE-BFARM-08014095

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	Yes		Yes

Patient

Initials:	Sex: Male	Birth: privacy	Age: 13 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Erythema exudativum multiforme	20050728	20050810	14 Day	7 Day	not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 BENURON paracetamol	Oral	/ Syrup	20050721-	Headache
2 BENURON paracetamol		/	-	Fever
3 DOLO-DOBENDAN benzocaine; cetylpyridinium chloride	Sublingual	/ Lozenge	20050725-	Sore throat
4 mucosolvan hustensaft 30 mg/5 ml ambroxol hydrochloride	Oral	/ Syrup	20050721-	Cough
5 mucosolvan lutschpastillen 15 mg ambroxol hydrochloride	Sublingual	/ Lozenge	20050721-	Cough

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Sore throat			Unknown	
Cold sores			Unknown	
Cold sores			Unknown	
Headache	20050721		Unknown	
Flu			Unknown	grippaler Infekt
Pneumonia	20050730		Unknown	

Primary source

Qualification:	Physician	observestudytype:	Other studies
Literature:			

Narratives (nur bei elektronischem Eingang)

Boehringer Ingelheim GmbH, Germany, 21.02.2008 DE-Boehringer Ingelheim GmbH, Germany-2008-DE-01031DE

Sender diagnosis: not reported

Narrative

This case was identified and registered by the "Centre of Documentation of severe skin diseases" in Freiburg, Germany and reported to the manufacturers as well as to the Federal Health Office. The "Centre" collects and files nationwide case reports of severe skin diseases.

Therefore, these reports must not be confused with documentations of suspected adverse drug reactions.

It was reported that a 13 year-old male patient developed erythema exsudativum multiforme majus on 28Jul2005 and was hospitalized on 30Jul2005. The boy took Mucosolvan (ambroxol) syrup and lozenges tablets for cough from 21Jul 2005 for an unknown time period, Benuron (paracetamol) syrup for headache and fever from 21Jul 2005 for unknown time period and Dolo-Dobendan (cetylpyridinium, benzocain) for sore throat from 25Jul 2005 for unknown time period.

Additionally, pneumonia was treated with Zithromax (azithromycin) from 28Jul2005 and also Erythromycin iv. from 30Jul2005 , suspected herpes labialis treated with Zovirax (aciclovir) from 28Jul2005 and secretolysis was performed with NAC (acetylcystein).

These events were formally marked as not life-threatening and the patient is not yet recovered.

The reporting source rated causal relationship between Mucosolvan and erythema exsudativum multiforme majus as possible.

Sender's comment:

Reporter's comment:

Boehringer Ingelheim GmbH, Germany, 28.02.2008 DE-Boehringer Ingelheim GmbH, Germany-2008-DE-01031DE

Sender diagnosis: not reported

Narrative

This case was identified and registered by the "Centre of Documentation of severe skin diseases" in Freiburg, Germany and reported to the manufacturers as well as to the Federal Health Office. The "Centre" collects and files nationwide case reports of severe skin diseases.

Therefore, these reports must not be confused with documentations of suspected adverse drug reactions.

It was reported that a 13 year-old male patient developed erythema exsudativum multiforme majus on 28Jul2005 and was hospitalized on 30Jul2005. The boy took Mucosolvan (ambroxol) syrup and lozenges tablets for cough from 21Jul 2005 for an unknown time period, Benuron (paracetamol) syrup for headache and fever from 21Jul 2005 for unknown time period and Dolo-Dobendan (cetylpyridinium, benzocain) for sore throat from 25Jul 2005 for unknown time period.

Additionally, pneumonia was treated with Zithromax (azithromycin) from 28Jul2005 and also Erythromycin iv. from 30Jul2005, suspected herpes labialis treated with Zovirax (aciclovir) from 28Jul2005 and secretolysis was performed with NAC (acetylcystein).

These events were formally marked as not life-threatening and the patient is not yet recovered.

The reporting source rated causal relationship between Mucosolvan and erythema exsudativum multiforme majus as possible.

Sender's comment:

Regional Centre Drug Safety Boehringer Ingelheim

Comment on case 2008-DE-01031DE / MUCOSOLVAN SAFT 30 MG/5 ML

Erythema exsudativum multiforme majus (EEMM) is unlisted for Mucosolvan syrup and Mucosolvan lozenges. Other factors have been reported in this case which may have caused the events, such as the other suspect drugs paracetamol and Dolo-Dobendan lozenges (cetylpyridinium chloride and benzocaine) or the flu-like infection and pneumonia.

As with all unlisted events, the company will continue to monitor all respective reports received and, based on cumulative experience, will re-evaluate the available evidence on an ongoing basis.

No change in the company core safety information or other measures are recommended at this time.

PILZDEYN

Reporter's comment:Bene-Arzneimittel, 04.03.2008 DE-Bene-Arzneimittel-B218

Sender diagnosis: not reported

Narrative

see also "patient's history";
oral treatment with paracetamol because patient suffered from headache/fever

Sender's comment:

Dok.-Fall in 2005 aus Dok.-Zentrum für schwere Hautreaktionen in BRD (Freiburg); insgesamt wurde an 9 Tagen ben-u-ron Saft, jeweils 1 TL (vermutlich 1 Meßbecher) bei Bedarf gegeben, Angaben über die Gesamtmenge liegen nicht vor. beschriebene Nebenwirkung zeigte sich 7 Tage nach der Erstbehandlung. Begleitmedikation waren Ambroxol, Cetylpyridiniumchlorid, Bencocain, Azithromycin, NAC, Aciclovir und Erythromycin. Ein Zusammenhang mit ben-u-ron-Saft ist trotz der zahlreichen Begleitmedikation nicht auszuschließen. Beobachtete Nebenwirkung nicht unbekannt; keine Änderung hins. wissenschaftl. Erkenntnisstand bei Anwendung von Paracetamol/Benuron-Saft am Menschen

Reporter's comment:

individuelle Bewertung durch Dermatologisches Expertengremium aufgrund spezifischem Bewertungsalgorithmus: Kat. "5" = "Zusammenhang möglich" aufgrund engem zeitlichen Zusammenhang mit oraler Einnahme von Paracetamol(P); P. zählt laut Exp.-gremium nicht zu gefährlichen Auslösern der beobachteten ADR

Bene-Arzneimittel, 04.03.2008 DE-Bene-Arzneimittel-B218

Sender diagnosis: not reported

Narrative

see also "patient's history";
oral treatment with paracetamol because patient suffered from headache/fever

Sender's comment:

Dok.-Fall in 2005 aus Dok.-Zentrum für schwere Hautreaktionen in BRD (Freiburg); insgesamt wurde an 9 Tagen ben-u-ron Saft, jeweils 1 TL (vermutlich 1 Meßbecher) bei Bedarf gegeben, Angaben über die Gesamtmenge liegen nicht vor. beschriebene Nebenwirkung zeigte sich 7 Tage nach der Erstbehandlung. Begleitmedikation waren Ambroxol, Cetylpyridiniumchlorid, Bencocain, Azithromycin, NAC, Aciclovir und Erythromycin. Ein Zusammenhang mit ben-u-ron-Saft ist trotz der zahlreichen Begleitmedikation nicht auszuschließen. Beobachtete Nebenwirkung nicht unbekannt; keine Änderung hins. wissenschaftl. Erkenntnisstand bei Anwendung von Paracetamol/Benuron-Saft am Menschen

Reporter's comment:

individuelle Bewertung durch Dermatologisches Expertengremium aufgrund spezifischem Bewertungsalgorithmus: Kat. "5" = "Zusammenhang möglich" aufgrund engem zeitlichen Zusammenhang mit oraler Einnahme von Paracetamol(P); P. zählt laut Exp.-gremium nicht zu gefährlichen Auslösern der beobachteten ADR

FIRST LOOK

Country: Deutschland

Type: Report from study

case: DE-BFARM-08014097

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes					

Patient

Initials:	Sex: Male	Birth: privacy	Age: 14 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens-Johnson syndrome	20050825	20050915	22 Day		not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 amoxicillin amoxicillin	Oral	2 Df dosage form/Day	20050820-20050821	Infection
2 infectocel cefaclor monohydrate	Oral	2 Df dosage form/Day	20050823-20050826	Tonsillitis
3 paracetamol Supp paracetamol	Rectal	1 Df dosage form/Day	20050819-20050823	Fever

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Mycoplasma infection			Unknown	
Fever			Unknown	
Tonsillitis			Unknown	
Gastroenteritis			Unknown	

Primary source

Qualification:	Physician	observestudytype:	Other studies
Literature:			

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08015238

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Male	Birth: privacy	Age: 44 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Mucous membrane disorder	20030109	20030122	14 Day		recovered/resolved
2 Erythema exudativum multiforme	20030109	20030122	14 Day		recovered/resolved
3 Erythema	20030109	20030122	14 Day		recovered/resolved
4 Chills	20030109	20030122	14 Day		recovered/resolved
5 Blister	20030109	20030122	14 Day		recovered/resolved
6 Asthenia	20030109	20030122	14 Day		recovered/resolved
7 Erythema exsudativum multiforme	20030109	20030122	14 Day		recovered/resolved
8 Lip erosion	20030109	20030122	14 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 paracetamol paracetamol	Oral	/	20021201-	Headache
2 Urbason methylprednisolone	Oral	20 Mg milligram(s)/	20031216-20031231 16 Day	Lupoid dermatitis

Drugs (concomitant)

3 INSIDON opipramol hydrochloride	Oral	/	20021101-20021231 61 Day	Sleep disturbance
4 TRUSOPT dorzolamide hydrochloride	Ophthalmic	9 Gtt drop(s)/Day Irrigation solution	20001101-20021231 2 Year	Cataract

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Sleep disorder			Unknown	
Cataract			Unknown	
Discoid lupus erythematosus			k.A.	Concomitant disease

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

Sanofi-Aventis Deutschland GmbH, 21.02.2008 DE-AVENTIS-200810453DE

Sender diagnosis: not reported

Narrative

Initial report:

This case of erythema exsudativum multiforme majus (EEMM) was received from the German health authority (DE-BFARM-08015238). It was reported by the German Documentation Centre for Severe Skin Reactions in Freiburg / Breisgau, Germany (code-no. 9140004). The centre is conducting a prospective study on the incidence and drug aetiology of severe skin reactions. Diagnosis of EEMM was made by an expert panel who assessed it as a "definite case" of EEMM. Causal relationship was assessed by the experts in Jul-2003 using an algorithm.

As for methylprednisolone (Urbason) the causality was assessed as "possible". For the other suspected drug causality was assessed as: "possible": paracetamole

Not suspected were: dorzolamide hydrochloride and opipramol hydrochloride

Patient: male, 44 years

Medical history: chronic discoidal lupus erythematosus (cutaneous lupus), headache, sleep disturbance and cataract.

From 15-Dec-2002 to 31-Dec-2002, the patient was treated with methylprednisolone (Urbason) at dose of 20 mg daily for cutaneous lupus.

On 03-Jan-2003, the patient experienced asthenia and chills.

On 09-Jan-2003, the patient target lesions, skin blisters and erosions of lips and erosions of nasal and oral mucous membrane.

On 13-Jan-2003 the patient was admitted to a hospital because of these symptoms.

Laboratory: no values provided.

At that time of the report, the patient had recovered.

Chronic discoidal lupus erythematosus was additionally provided as possible alternative explanation by the experts.

Sender's comment:

19-Feb-2008 Sanofi Aventis Company Comment:

The case report presented here is properly documented, and the temporal relationship between the event and administration of methylprednisolone (Urbason) seems plausible. Dechallenge and rechallenge is not applicable as the patient was hospitalized and the concomitant medications were altered at the same time. In addition, concomitant drugs as reported here and chronic discoidal lupus erythematosus may be considered an alternative cause to explain this event.

19-FEB-2008 Sanofi Aventis Germany: ERYTHEMA EXSUDATIVUM MULTIFORME MAJUS is considered locally unlabelled for the suspected product. The facts presented here, however, are not very suggestive of a causal relationship (please refer to the intl. evaluator's comment), and thus do not imply a change in the state of the scientific knowledge for the product. Measures according to Para 29 German Drug Law (AMG), section (1) concerning the approval status are not deemed necessary.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08016537

Serious Yes	Death	Lifethr.	Hospital.	Disabling	Congenital	Other No	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Female	Birth: privacy	Age: 10 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens-Johnson syndrome	20041115	20041208	24 Day		not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ambroxol ambroxol	Oral	5 Df dosage form/Day	-	-
2 klacid clarithromycin	Oral	250 Mg milligram(s)/Day	20041113-20041115	Mycoplasmal pneumonia
3 paracetamol paracetamol		1000 Mg milligram(s)/Day	-	-

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Mycoplasma infection NOS			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

001307602, 14.03.2008 DE-ABBOTT-08P-062-0441104-00

Sender diagnosis: not reported

Narrative

Report from Germany of Stevens-Johnson syndrome and red eyes coincident with CLARITHROMYCIN (KLACID) therapy. On 11 Nov 2004, the patient began AMBROXOL for cough and PARACETAMOL for fever. On 12 Nov 2004, the patient developed an infection with fever and a cough. On 13 Nov 2004, the patient began KLACID therapy for mycoplasma pneumonia. On 15 Nov 2004, the patient experienced red eyes. PARACETAMOL, AMBROXOL and KLACID were discontinued on 15 Nov 2004. On 16 Nov 2004, the patient developed pain of the skin with erosions of the lips and oral mucosa. The patient was hospitalized on 16 Nov 2004 for Stevens-Johnson syndrome, which was also considered medically significant. On 17 Nov 2004, the patient developed pruritus and skin blisters. On 26 Nov 2004, conjunctivitis and erosions of the genital mucosa developed. The Stevens-Johnson syndrome resolved on 08 Dec 2004. In Mar 2005, the patient was not yet fully restored to health. The reporter believed that the Stevens-Johnson syndrome was possibly related to KLACID, PARACETAMOL and AMBROXOL therapies. PARACETAMOL and AMBROXOL were also considered suspect.

Sender's comment:

Abbott Germany Assessment: 14.03.2008, Klacid(R) - 9034 / 08P-062-0441104-00, MK
Die in diesem Fallbericht geschilderte Symptomatik ist in den Angaben zum Arzneimittel in adaequater Form beschrieben.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08016572

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes		Yes					

Patient

Initials:	Sex: Male	Birth: privacy	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Necrolysis epidermal toxic (Lyell type)	20050112	20050202	22 Day		fatal
2 Stevens-Johnson syndrome	20050112	20050202	22 Day		fatal

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Amitriptylin amitriptyline hydrochloride	Oral	25 Mg milligram(s)/	-	Depression
2 combactam sulbactam sodium	Intravenous (not otherwis /		-	Infection
3 imeson nitrazepam	Oral	/	-	Sleep disorder NOS
4 insulin insulin	Intravenous (not otherwis /		-	
5 paracetamol paracetamol	Unknown	/	-	Pyrexia
6 pipril piperacillin sodium	Intravenous (not otherwis /		-	Infection
7 sterofundin electrolytes nos	Intravenous (not otherwis /		-	Volume blood
8 unacid ampicillin sodium; sulbactam sodium	Intravenous (not otherwis /		-	Infection

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Catheter infection			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08017073

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	Yes	Yes	No	No	No		Yes

Patient

Initials:	Sex: Female	Birth: privacy	Age: 56 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Mucosal erosion	20041221			6 Day	recovering/resolving
2 Lip swelling	20041219			4 Day	recovering/resolving
3 Mucosal inflammation	20041221			6 Day	recovering/resolving
4 Pain of skin	20041220			5 Day	recovering/resolving
5 Toxic epidermal necrolysis	20041221			6 Day	recovering/resolving
6 Lip erosion	20041220			5 Day	recovering/resolving
7 Erythema	20041220			5 Day	recovering/resolving
8 Conjunctivitis	20041221			6 Day	recovering/resolving
9 Blister	20041221			6 Day	recovering/resolving
10 Fever	20041220			5 Day	recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 allopurinol	Oral	300 Mg milligram(s)/Day	20041125-	Hyperuricaemia
allopurinol			-	
2 diclac	Oral	100 Mg milligram(s)/Day	20041216-	Coxitis
diclofenac			-	
3 GLUCOPHAGE	Oral	/	20030601-	Diabetes mellitus
metformin			-	
4 levothyroxine	Oral	200 µg microgram(s)/Day	20030501-	
levothyroxine			-	
5 paracetamol comp	Oral	1 Df dosage form/Day	20041115-	Pain
paracetamol			-	
6 zolpidem	Oral	10 Mg milligram(s)/Day	20041201-	Disorder sleep
zolpidem			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Diabetes mellitus non-insulin-dependent			Yes	
Hypothyroidism			Yes	
Cold			Unknown	
Hyperuricaemia			Yes	
Sleep disorder			Yes	
Coxitis			Yes	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 29.02.2008 DE-SZ09-GXKR2008DE01894

Sender diagnosis: not reported

Narrative

Initial registry report from the Freiburg Documentation Centre for Severe Skin Reactions received on 25 Feb 2008 from the German health authority

(BfArM case number: DE-BFARM-08017073): This patient with medical history of diabetes mellitus, hypothyroidism, hyperuricaemia and sleep disorder developed Toxic epidermal necrolysis (TEN) under treatment with Diclac (diclofenac) taken for coxitis pain at a daily dosage of 100 mg since 16 Dec 2004. Concomitantly several other drugs were taken which were considered causative: Among others L-Thyrox (levothyroxine, MAH unknown), allopurinol (MAH unknown), zolpidem (MAH unknown), Glucophage (metformine) and Paracetamol Comp (paracetamol/codeine) were considered causative. The patient had a common cold on 18 Dec 2004 and developed on 19 Dec 2004 a TEN with the first symptom of lip swelling. On the next day fever up to 39.5 degree Celsius, pain of skin, cockades and maculae as well as lip erosions occurred. On 21 Dec 2004 the patient was hospitalized with blisters, Nikolsky's sign, conjunctivitis and oral mucosal erosions. On the next day the genital mucosa was reddened. The adverse event lasted until 17 Feb 2005, but at the time of the report the patient had not yet recovered. The reporter assessed the causality between onset of adverse event and administration of diclofenac and zolpidem as possible, for levothyroxine as questionable and for allopurinol as very probable. The causal relationship between adverse event and cold assessed the reporter as possible.

Sender's comment:

For diclofenac and allopurinol:

Serious adverse event report (life-threatening) of Toxic epidermal necrolysis, assessed as listed according to the Core Data Sheet Information. Reporting physician causality is suspected.

For levothyroxine and zolpidem:

Serious adverse event report (life-threatening) of Toxic epidermal necrolysis, assessed as unlisted according to the Core Data Sheet Information. However, other alternative causes (other drugs, common cold) provide a possible explanation for the reported adverse event. Reporting physician causality is suspected.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08017576

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes	Yes	Yes					

Patient

Initials:	Sex: Female	Birth: privacy	Age: 87 Year	weight (kg):	BMI_int:	Age fetus:
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Patientdeath

deathdate:	deathcause: Stevens Johnson syndrome
deathdate:	deathcause: Toxic epidermal necrolysis
deathdate:	deathcause: Unknown cause of death

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	20030829	20030927	30 Day		fatal
2 Necrolysis epidermal toxic (Lyell type)	20030820	20030927	39 Day		fatal
3 Oral mucosa erosion	20030908				Unknown
4 Oral mucosa erosion	20030908				Unknown
5 Blistering	20030908				Unknown
6 Blistering	20030908				Unknown
7 Exanthema	20030830				Unknown
8 Exanthema	20030830				Unknown
9 Fever	20030829				Unknown
10 Pruritus	20030906				Unknown
11 Pruritus	20030906				Unknown
12 Lip erosion	20030908				Unknown
13 Lip erosion	20030908				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Benuron paracetamol	Rectal	3 G gram(s)/ 242	20030904-20030907 4 Day	Fever
2 Benuron paracetamol	Rectal	1 G gram(s)/ 242	20030822-20030822 1 Day	Fever
3 Benuron paracetamol	Rectal	1.5 G gram(s)/ 242	20030829-20030829 1 Day	Fever
4 Ceftriaxon ceftriaxone sodium	Intravenous (not otherwis	/Day 231	20030822-20030829 8 Day	Cystitis
5 Ciprobay ciprofloxacin hydrochloride	Oral	500 Mg milligram(s)/Day 245	20030825-20030825 1 Day	Cystitis
6 Ciprobay ciprofloxacin hydrochloride	Oral	250 Mg milligram(s)/Day 245	20030820-20030823 4 Day	Cystitis
7 Corvaton retard molsidomine	Oral	/	20030816-20030908 24 Day	Coronary disease
8 Furo furosemide	Intravenous (not otherwis	2* /Day 231	20030817-20030817 1 Day	Edema
9 Furosemid furosemide	Oral	/	20030823-20030908 17 Day	Edema
10 Furosemid furosemide	Intravenous (not otherwis	/ 231	20030818-20030823 6 Day	Edema
11 Klacid clarithromycin	Oral	250 Mg milligram(s)/ 245	20030822-20030830 9 Day	Pneumonia
12 Novalgin metamizole	Oral	4* /Day	20030817-20030818 2 Day	Pain NOS

Drugs (concomitant)					
13	Aquaphor xipamide	Oral	20 Mg milligram(s)/Day	20030817- -	Edema
14	ass acetylsalicylic acid	Oral	100 Mg milligram(s)/Day	20000101- -	Thrombosis prophylaxis
15	Augmentan amoxicillin sodium; clavulanate potassium	Oral	875 Mg milligram(s)/	20030903-20030905 3 Day	Pneumonia
16	Bezabeta bezafibrate	Oral	/Day	20000101-20030815 3 Year	Hypercholesteremia
17	Calcium calcium	Oral	/	20030825- -	Calcium supplementation
18	Calcium calcium	Intravenous (not otherwis	/ 230	20030817-20030820 4 Day	Calcium supplementation
19	Diazepam diazepam	Oral	5 Mg milligram(s)/	20030907-20030907 1 Day	Sleep disorder
20	Diclofenac-ratiopharm diclofenac sodium	Sublingual	75 Mg milligram(s)/	20000101-20030815 3 Year	Pain NOS
21	diflucan fluconazole	Oral	5 ml millilitre(s)/ 169	20030907-20030907 1 Day	Candidiasis
22	Eatan methaqualone; promazine phosphate	Oral	/Day	20000101-20030815 3 Year	Sleep disorder
23	Ecural Salbe mometasone furoate	Topical	/	20030831-20030907 8 Day	Exanthema
24	hydergin dihydroergocornine mesilate; dihydroergocristine mesilate; dihydroergocryptine mesilate	Oral	2* /Day	20000101-20030815 3 Year	Central nervous system disorder
25	isoket isosorbide dinitrate	Oral	30 Mg milligram(s)/Day	20000101-20030901 3 Year	Coronary disease
26	kalinor potassium chloride	Oral	2* /Day 59	20030816-20030820 5 Day	Potassium supplementation
27	Lavasept polihexanide	Topical	/Day	20030817- -	Ulcus cruris
28	Loperamid loperamide hydrochloride	Oral	/ 245	20030906-20030906 1 Day	Diarrhea
29	medivitan n folate sodium; hydroxocobalamin; lidocaine hydrochloride; pyridoxine hydrochloride	Intramuscular	1 Df dosage form/Day 231	20030815-20030815 1 Day	Multi-vitamin maintenance
30	Molsidomin ratiopharm molsidomine	Oral	16 Mg milligram(s)/Day	20000101-20030815 3 Year	Coronary disease
31	Morphin morphine sulfate	Oral	20 Mg milligram(s)/ 99	20030908-20030908 1 Day	Pain NOS
32	Morphin morphine sulfate	Subcutaneous	/ 231	20030908-20030908 1 Day	Pain NOS
33	nacl sodium chloride	Intravenous (not otherwis	/ 230	20030820-20030821 2 Day	Volume depletion
34	nacl sodium chloride	Intravenous (not otherwis	/ 230	20030816-20030818 3 Day	Volume depletion
35	Nitrolingual N-Spray glyceryl trinitrate	Sublingual	/	20000101-20030815 3 Year	Coronary disease
36	Normofundin calcium chloride; magnesium chloride anhydrous; potassium chloride; sodium acetate; sodium chloride; sorbitol; xylitol	Intravenous (not otherwis	1 l litre(s)/ 230	20030908-20030908 1 Day	Volume depletion
37	Novalgin metamizole	Oral	/ 158	20030904-20030904 1 Day	Pain NOS
38	Novalgin metamizole	Intravenous (not otherwis	/ 231	20030908-20030908 1 Day	Pain NOS

Drugs (concomitant)

39	Ranitidin ratiopharm ranitidine hydrochloride	Oral	/Day	20000101-20030117 Prophylaxis 3 Year
40	Scandicain mepivacaine hydrochloride	Subcutaneous	/ 231	20030829-20030829 Anesthesia 1 Day
41	tavegil clemastine		3* /Day	20030907-20030908 Itching 2 Day

Primary source

Qualification: Physician observestudytype:

Literature:

Narratives (nur bei elektronischem Eingang)001307602, 03.03.2008 DE-ABBOTT-08P-062-0439924-00

Sender diagnosis: not reported

Narrative

Report from Germany of acute gastroenteritis, Stevens Johnson syndrome, toxic epidermal necrolysis (TEN), pneumonia, and dysphagia coincident with CLARITHROMYCIN (KLACID) therapy. On 11 Aug 2003, the patient began DICLOFENAC SL therapy for pain, HYDERGIN FORT therapy for ZNS blood circulation, EATAN N therapy for sleep disorder, and BEZABETA RETARD therapy for hypercholesterolemia. On 11 Aug, the patient began NITROLINGUAL N Spray, MOLISDOMIN TATIOPHARM, ISOKET, and RANITIDAN RATIOPHARM therapies for coronary artery disease. On 15 Sep 2003, DICLOFENAC SL, HYDERGIN FORT, EATAN, NITROLINGUAL N SPRAY, MOLISDOMIN TARIOPPHARM, BEZABETA RETARD, and RANITIDAN RATIOPHARM therapies were discontinued. On 16 Aug 2003, the patient began CORVATON RETARD therapy for coronary artery disease, KALINOR therapy for potassium substitution, and sodium chloride (NACL) therapy for volume substitution. On 17 Aug 2003, the patient began AQUAPHOR therapy for edema and LAVASEPT therapy for ulcer cruris. On 17 Aug 2003, for one day, the patient received NOVALGIN therapy for pain and CALCIUM therapy for calcium substitution. On 17 Aug 2003, for one day, the patient received FURO therapy for edema. On Aug 18 2003, the patient began FUROSEMIDE INJECTION for edema. On 18 Aug 2003, NOVAGLIN and NACL therapies were discontinued. On 20 Aug 2003, the patient experienced acute cystitis. On 20 Aug 2003, the patient began CIPROBAY therapy for cystitis. On 20 Aug 2003, KALINOR therapy was discontinued. On 21 Aug 2003, NACL therapy was discontinued. On 22 Aug 2007, the patient began KLACID and CEFTRIAXON therapies for cystitis. On 22 Aug 2003, for one day, the patient received BENURON therapy for fever. On 23 Aug 2003, FUROSEMIDE and CIPROBAY therapies were discontinued. On 24 Aug 2003, she developed fever up to 38 degrees Celsius and on 25 Aug 2003, she additionally experienced pneumonia. On 25 Aug 2003, for one day, the patient received CIPROBAY for cystitis. On Aug 29, the patient began SCANDICAIN therapy for anesthesia. On 29 Aug 2003, CEFTRIAXON therapy was discontinued. On 30 Aug 2003, for one day, the patient received BENURON therapy for pain. The fever increased up to 40.2 degrees Celsius on 29 Aug 2003. On 31 Aug 2003, the patient experienced exanthema and KLACID therapy was discontinued. On 31 Aug 2003, the patient began ECURAL therapy for exanthema. On 01 Sep 2003, ISOKET therapy was discontinued. On 03 Sep 2003, the patient began AUGMENTIN therapy for pneumonia. On 04 Sep 2003, for one day, the patient received NOVALGIN therapy for pain. On 05 Sep 2003, the patient developed acute gastroenteritis. On 05 Sep 2003, the patient began BENURON therapy for fever. On 05 Sep 2003, AUGMENTIN therapy was discontinued. On 06 Sep 2003, the patient experienced pruritus. On 06 Sep 2003, for one day, the patient received LOPERAMID therapy for diarrhea. On 07 Sep 2008, the patient experienced dysphagia. On 07 Sep 2003, the patient began TAVEGIL therapy for itching and urticaria. On 07 Sep 2003, for one day, the patient received DIFLUCAN therapy for candidiasis and DIAZEPAM therapy for sleep disorder. On 07 Sep 2003, BENURON and ECURAL therapies were discontinued. On 08 Sep 2003 erosions and blisters on the lips and oral mucosa appeared. A form of Stevens-Johnson syndrome or toxic epidermal necrolysis (TEN) was diagnosed. On 08 Sep 2003, for one day, the patient received SOLU-DECORTIN H therapy for exanthema, NORPHIN therapy for pain, and NOVALGIN therapy for pain. On 08 Sep 2003, CALCIUM, AQUAPHOR, TAVEGIL, CORVATON and LAVACEPT therapies were discontinued. The patient passed away on 27 Sep 2008. The acute gastroenteritis and toxic epidermal necrolysis were considered medically significant. DICLOFENAC, HYDERGIN, AQUAPHOR, LAVASEPT, SOLU-DECORTIN H, NORMOFUNDIN, MORPHIN, NOVALGIN, TAVEGIL, DIFLUCAN, DIAZEPAM, LOPERAMID, ECURAL, AUGMENTIN, CALCIUM, SCANDICAIN, CEFTRAIXON, BENURON, CALCIUM, FUROSEMID, CIPROBAY, EATAN N, NITROLINGUAL N SPRAY, MOLSIDOMIN RATIOPHARM, BEZABETA RETARD, RANITIDIN RATIOPHARM, ISOKET, KALINOR, FURO, and NaCl therapies were also considered suspect.

Sender's comment:

Abbott Germany Assessment: 03.03.2008, Klacid(R) - 9004 / 08P-062-0439924-00, MK

Die in diesem Fallbericht geschilderte Symptomatik ist in den Angaben zum Arzneimittel in adaequater Form beschrieben. Eine begründete Beurteilung des Kausalzusammenhangs ist anhand der uns bislang zur Verfügung gestellten Informationen nicht möglich.

Reporter's comment:

Sender diagnosis: not reported

Narrative

Initial report:

This serious case of a transitional form of Stevens Johnson syndrome / Toxic Epidermal Necrolysis (SJS/TEN) was received from the German health authority (DE-BFARM-08017576). It was reported by the German Documentation Centre for Severe Skin Reactions in Freiburg / Breisgau, Germany (code-no. 9150073). The centre is conducting a prospective study on the incidence and drug aetiology of severe skin reactions. Diagnosis was made by an expert panel who assessed it as a "definite case" of SJS/TEN. Causal relationship was assessed by the experts in January 2004 using an algorithm.

For the following suspected drugs, causality was assessed as:

"well possible": ciprofloxacin, ceftriaxone, clarithromycin

"possible": molsidomine (Corvaton retard) at a dose of 0-0-1 or 1-0-0 since 16-Aug-2003, metamizole (Novalgin) at a dose of 1-1-1-1 on 17-Aug and 18-Aug-2003, furosemide (3x), paracetamol.

Not suspected were: diclofenac, digydroergotamine, nitrazepam, glycerol trinitrate, molsidomine (other manufacturer), bezafibrate, ranitidine, isosorbide dinitrate, acetylsalicylic acid, cyanocobalamin + folic acid, potassium, NaCl, calcium (2x), polihexanide, xipamide, mepivacaine, mometasone, amoxicillin + clavulanic acid, metamizole (drops, solution for injection, different time period), loperamide, fluconazole, diazepam, clemastine, morphine (2x), prednisolone, infusion solution (Normofundin).

The patient's medical history included pain, sleep disorder, coronary heart disease, hypercholesterolemia, edemas and suspicion of candidiasis.

The patient had been hospitalised on 16-Aug-2003 for unreported reason. On 20-Aug-2003 she developed acute cystitis; from 24-Aug to 25-Aug-2003 she suffered from fever up to 38 Celsius; on 25-Aug-2003 she presented with pneumonia; on 29-Aug-2003 fever was at 40.2 Celsius (start date of SJS/TEN), on 30-Aug she developed exanthema.

On 05-Sep-2003 she experienced an acute gastroenteritis. The next day she developed pruritus and on 07-Sep-2003 dysphagia. On 08-Sep-2003 the first skin blister occurred and lip as well as oral mucous membrane erosions became visible.

Further course and treatment was not reported. As stop date for the event SJS/TEN 27-Sep-2003 was reported but outcome was the death of the patient. Therefore, this date should be considered as the date of the death of the patient.

Besides the above mentioned concomitant medication the pneumonia was assessed as a "possible" alternative explanation.

Sender's comment:

29-Feb-2008 Sanofi Aventis Company Comment:

The case report presented here is properly documented, and the temporal relationship between the event and administration of molsidomine (Corvaton retard) and metamizole sodium (Novalgin) seems plausible. Dechallenge and rechallenge is not applicable as concomitant medications were altered at the same time. In addition, concomitant drugs as reported here and/or pneumonia may be considered an alternative cause to explain the event.

29-FEB-2008 Sanofi Aventis Germany: TRANSIT.FORM STEVENS-JOHNSON SYND./TOX.EPID.NECROLYSIS-FATAL is considered locally unlabelled for the suspected products. The facts presented here, however, are not very suggestive of a causal relationship (please refer to the intl. evaluator's comment), and thus do not imply a change in the state of the scientific knowledge for the products. Measures according to Para 29 German Drug Law (AMG), section (1) concerning the approval status are not deemed necessary.

Reporter's comment:

Sender diagnosis: not reported

Narrative

This post-marketing study report was received on 28-Feb-2008 by BFARM, the regulatory authority of Germany, referring to an 87 year-old female.

Postmarketing surveillance with Ciprofloxacin (tablet).

Study number: BAYO9867/PMS.

Concomitant drugs have been given. For details see section "Concomitant Drugs" at the end of this report.

Patient received:

- Ciprobay (ciprofloxacin hydrochloride) for cystitis. Administration began on 20-Aug-2003, for a duration of 4 days at a total daily dose of 250 MG orally, and also administered on 25-Aug-2003 at a total daily dose of 500 MG orally.
- Furosemid (furosemide) for edema. Administration began on 18-Aug-2003, for a duration of 6 days, total daily dose was not reported, and also administered on 23-Aug-2003, for a duration of 17 days, total daily dose was not reported.
- Corvaton retard (molsidomine) for coronary heart disease. Administration began on 16-Aug-2003, for a duration of 24 days, total daily dose was not reported.
- Furo (furosemide) for edema. Drug was administered on 17-Aug-2003, total daily dose was not reported.
- Novalgin (metamizole sodium) for pain nos. Administration began on 17-Aug-2003, for a duration of 2 days, total daily dose was not reported.
- Ceftriaxon (ceftriaxone sodium) for cystitis. Administration began on 22-Aug-2003, for a duration of 8 days, total daily dose was not reported.
- Klacid (clarithromycin) for pneumonia. Administration began on 22-Aug-2003, for a duration of 9 days, total daily dose was not reported.
- Benuron (paracetamol) for fever. Drug was administered on 22-Aug-2003, total daily dose was not reported, on 29-Aug-2003, total daily dose was not reported, and also administered on 04-Sep-2003, for a duration of 4 days, total daily dose was not reported.

On 29-Aug-2003, 9 days after starting treatment with Ciprobay, 11 days after starting treatment that was ongoing with Furosemid, 13 days after starting treatment that was ongoing with Corvaton retard, 12 days after treatment with Furo, 12 days after starting treatment with Novalgin, 7 days after starting treatment with Ceftriaxon, 7 days after starting treatment that was ongoing with Klacid, and 7 days after starting treatment that was ongoing with Benuron patient experienced STEVENS-JOHNSON SYNDROME (exanthema, pruritus, first occurrence of skin blistering, erosive lips, erosive oral mucosa, fever), and TOXIC EPIDERMAL NECROLYSIS (exanthema, pruritus, first occurrence of skin blistering, erosive lips, erosive oral mucosa) considered serious due to life-threatening nature, and death.

Patient died on an unknown date. Autopsy information is unknown. The patient died from unknown cause of death as stated by the reporter.

For the drugs Ciprobay and Furosemid the investigator considered the diagnoses STEVENS-JOHNSON SYNDROME and TOXIC EPIDERMAL NECROLYSIS to be associated.

Sender's comment:

FATAL STEVENS-JOHNSON-SYNDROME and FATAL TOXIC EPIDERMOLYSIS, considered serious due to fatal outcome, is listed in the International Product Information for CIPROBAY (ciprofloxacin), however unlisted for FUROSEMIDE. Based on the information received for this case, a causal relation between the reported events and treatment with ciprofloxacin and furosemide cannot be excluded due to a compatible temporal relationship. STEVENS-JOHNSON SYNDROME has also been reported for the co-suspect drugs NOVALGIN (metamizol) and BENURON (paracetamol), as well as for cephalosporins (CEFTRIAXON), macrolides (KLACID (clarithromycin)), and furosemide. A temporal relation between treatment and occurrence of the first clinical signs of STEVENS-JOHNSON-SYNDROME cannot be excluded for all mentioned co-suspect drugs. In the reported case, STEVENS-JOHNSON SYNDROME and TOXIC EPIDERMAL NECROLYSIS were classified as transitional forms. An additive effect of several drugs known to cause the reported events, may have contributed to the clinical progression and fatal outcome of the events.

Reporter's comment:

001307602, 12.03.2008 DE-ABBOTT-08P-062-0439924-00

Sender diagnosis: not reported

Narrative

Report from Germany of acute gastroenteritis, Stevens Johnson syndrome, toxic epidermal necrolysis (TEN), pneumonia, and dysphagia coincident with CLARITHROMYCIN (KLACID) therapy. On 11 Aug 2003, the patient began DICLOFENAC SL therapy for pain, HYDERGIN FORT therapy for ZNS blood circulation, EATAN N therapy for sleep disorder, and BEZABETA RETARD therapy for hypercholesterolemia. On 11 Aug, the patient began NITROLINGUAL N Spray, MOLISDOMIN TATIOPHARM, ISOKET, and RANITIDAN RATIOPHARM therapies for coronary artery disease. On 15 Sep 2003, DICLOFENAC SL, HYDERGIN FORT, EATAN, NITROLINGUAL N SPRAY, MOLISDOMIN TARIOPPHARM, BEZABETA RETARD, and RANITIDAN RATIOPHARM therapies were discontinued. On 16 Aug 2003, the patient began CORVATON RETARD therapy for coronary artery disease, KALINOR therapy for potassium substitution, and sodium chloride (NACL) therapy for volume substitution. On 17 Aug 2003, the patient began AQUAPHOR therapy for edema and LAVASEPT therapy for ulcer cruris. On 17 Aug 2003, for one day, the patient received NOVALGIN therapy for pain and CALCIUM therapy for calcium substitution. On 17 Aug 2003, for one day, the patient received FURO therapy for edema. On Aug 18 2003, the patient began FUROSEMIDE INJECTION for edema. On 18 Aug 2003, NOVAGLIN and NACL therapies were discontinued. On 20 Aug 2003, the patient experienced acute cystitis. On 20 Aug 2003, the patient began CIPROBAY therapy for cystitis. On 20 Aug 2003, KALINOR therapy was discontinued. On 21 Aug 2003, NACL therapy was discontinued. On 22 Aug 2007, the patient began KLACID and CEFTRIAXON therapies for cystitis. On 22 Aug 2003, for one day, the patient received BENURON therapy for fever. On 23 Aug 2003, FUROSEMIDE and CIPROBAY therapies were discontinued. On 24 Aug 2003, she developed fever up to 38 degrees Celsius and on 25 Aug 2003, she additionally experienced pneumonia. On 25 Aug 2003, for one day, the patient received CIPROBAY for cystitis. On Aug 29, the patient began SCANDICAIN therapy for anesthesia. On 29

Aug 2003, CEFTRIAXON therapy was discontinued. On 30 Aug 2003, for one day, the patient received BENURON therapy for pain. The fever increased up to 40.2 degrees Celsius on 29 Aug 2003. On 31 Aug 2003, the patient experienced exanthema and KLACID therapy was discontinued. On 31 Aug 2003, the patient began ECURAL therapy for exanthema. On 01 Sep 2003, ISOKET therapy was discontinued. On 03 Sep 2003, the patient began AUGMENTIN therapy for pneumonia. On 04 Sep 2003, for one day, the patient received NOVALGIN therapy for pain. On 05 Sep 2003, the patient developed acute gastroenteritis. On 05 Sep 2003, the patient began BENURON therapy for fever. On 05 Sep 2003, AUGMENTIN therapy was discontinued. On 06 Sep 2003, the patient experienced pruritus. On 06 Sep 2003, for one day, the patient received LOPERAMID therapy for diarrhea. On 07 Sep 2003, the patient experienced dysphagia. On 07 Sep 2003, the patient began TAVEGIL therapy for itching and urticaria. On 07 Sep 2003, for one day, the patient received DIFLUCAN therapy for candidiasis and DIAZEPAM therapy for sleep disorder. On 07 Sep 2003, BENURON and ECURAL therapies were discontinued. On 08 Sep 2003 erosions and blisters on the lips and oral mucosa appeared. A form of Stevens-Johnson syndrome or toxic epidermal necrolysis (TEN) was diagnosed. On 08 Sep 2003, for one day, the patient received SOLU-DECORTIN H therapy for exanthema, NORPHIN therapy for pain, and NOVALGIN therapy for pain. On 08 Sep 2003, CALCIUM, AQUAPHOR, TAVEGIL, CORVATON and LAVACEPT therapies were discontinued. The patient passed away on 27 Sep 2008. The acute gastroenteritis and toxic epidermal necrolysis were considered medically significant. DICLOFENAC, HYDERGIN, AQUAPHOR, LAVASEPT, SOLU-DECORTIN H, NORMOFUNDIN, MORPHIN, NOVALGIN, TAVEGIL, DIFLUCAN, DIAZEPAM, LOPERAMID, ECURAL, AUGMENTIN, CALCIUM, SCANDICAIN, CEFTRIAISON, BENURON, CALCIUM, FUROSEMID, CIPROBAY, EATAN N, NITROLINGUAL N SPRAY, MOLSIDOMIN RATIOPHARM, BEZABETA RETARD, RANITIDIN RATIOPHARM, ISOKET, KALINOR, FURO, and NaCL therapies were also considered suspect.

Follow-up information was clarified on 03 Mar 2008: This version was created to clarify that there was no drug interaction related to CLARITHROMYCIN and acute gastroenteritis.

Sender's comment:

Abbott Germany Assessment: 03.03.2008, Klacid(R) - 9004 / 08P-062-0439924-00, MK

Die in diesem Fallbericht geschilderte Symptomatik ist in den Angaben zum Arzneimittel in adaequater Form beschrieben. Eine begründete Beurteilung des Kausalzusammenhangs ist anhand der uns bislang zur Verfügung gestellten Informationen nicht möglich.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-08017695

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		Yes

Patient

Initials:	Sex: Female	Birth: privacy	Age: 6 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Erythema exudativum multiforme	20041231	20050114	15 Day	7 Day	not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 AMOXYPEN SAFT amoxicillin trihydrate		3*5 ml millilitre(s)/ Syrup	20041227-20050102	Infection
2 BENURON paracetamol	Oral	500 Mg milligram(s)/	20041225-20041226	Fever
3 DOLORMIN SAFT ibuprofen	Oral	100 Mg milligram(s)/ Syrup	20041224-20041224	Flu
4 MUCOSOLVAN SAFT 30 MG/5 ML ambroxol hydrochloride	Oral	3*5 ml millilitre(s)/ Syrup	20041224-20041226	Flu
5 PARACETAMOL paracetamol	Oral	500 Mg milligram(s)/ Day	20041225-	Mycoplasma infection

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Fever			Unknown	
Influenza			Unknown	
Mycoplasma infection NOS			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:	Primary Source auf zugesandtem BFARM-Meldebogen leider geschwÃ¤rzt; warum?	

Narratives (nur bei elektronischem Eingang)

Boehringer Ingelheim GmbH, Germany, 03.03.2008 DE-Boehringer Ingelheim GmbH, Germany-2008-DE-01172DE

Sender diagnosis: not reported

Narrative

This case was identified and registered by the "Centre of Documentation of severe skin diseases" in Freiburg, Germany and reported to the manufacturers as well as to the Federal Health Office. The "Centre" collects and files nationwide case reports of severe skin diseases.

Therefore, these reports must not be confused with documentations of suspected adverse drug reactions.

It was reported that a 6 year-old female child developed erythema exsudativum multiforme majus (EEMM) from 31Dec2004 until 14Jan2005 (but not recovered).

Flu with Fever up to 38.5 C started on 23Dec2004. On 24Dec2004 treatment with Mucosolvan (ambroxol) syrup 3x5 ml and Dolormin (ibuprofen) syrup 1x5 ml was initiated.

In addition the child received Benuron (paracetamol) 1x500 mg per os from 25Dec2004 until 26Dec2004. Mycoplasma infection was diagnosed treated with Amoxypen (amoxicillin) syrup 3x5 ml from 27Dec2004 until 01Jan2005.

On 01Jan2005 burning and redness of mouth mucosa with burning and swelling lips occurred.

Erythema exsudativum multiforme majus with skin pain, eye redness and erosions of mouth mucosa was diagnosed. The child was hospitalised on 02Jan2005 with pruritus, cockades and spots followed by lip erosions and skin blisters.

These adverse event was formally not marked as life-threatening.

The reporting source rated causal relationship between Mucosolvan and erythema exsudativum multiforme majus as possible.

Sender's comment:

Regional Centre Drug Safety Boehringer Ingelheim

Comment on case 2008-DE-01172DE / MUCOSOLVAN SAFT 30 MG/5 ML

Erythema exsudativum multiforme majus (EEMM) is unlisted for Mucosolvan syrup. Other factors have been reported in this case which may have caused the events, such as the other suspect drugs paracetamol, ibuprofen and, with even more likelihood, amoxicillin or the underlying mycoplasma infection.

As with all unlisted events, the company will continue to monitor all respective reports received and, based on cumulative experience, will re-evaluate the available evidence on an ongoing basis.

No change in the company core safety information or other measures are recommended at this time

PILZDEYN

Case assessment created by PL on 29 Feb 2008

Reporter's comment:

GRUNENTHAL, 06.03.2008 DE-GRUNENTHAL-2008 10811

Sender diagnosis: not reported

Narrative

SUBJECT/PATIENT ID

Manufacturer Control Number: 2008-10811

ADVERSE EVENT DETAILS

This solicited case from special registry for severe skin reactions from GERMANY, that was reported by a other health professional via regulatory authority was received at GRT on 25-FEB-2008 and concerns a 6 years old female patient (height unknown / weight unknown).

The relevant medical history included: Pyrexia, Influenza, Mycoplasma infection.

This case was reported by the Documentation Centre for Severe Skin reactions (Freiburg/Germany) via BfArM and concerns a 6 year old female patient. The patient was treated with Dolormin syrup (ibuprofen) and Mucosolvan syrup (ambroxol) for grippal infection and with Benuron

(paracetamol) for fever. In addition she was treated with Amoxypen syrup (amoxicillin) for infection from 27-Dec-2004 to 01-Jan-2005. The patient was suffering from mycoplasma infection. On 23-Dec-2004 the patient experienced fever up to 38.5 degrees Celsius and grippal infection. On 31-Dec-2004, 5 days after first administration of AMOXYPEN UNK burning/reddening of the oral mucosa and burning/swelling of the lips occurred followed by painful skin, reddening of the eyes and erosions of the oral mucosa on 01-Jan-2005. On 02-Jan-2005, pruritus, cockades and maculae appeared. The patient was hospitalized. On 03-Jan-2005, she experienced skin blisters and erosions of the lips. The reported symptoms were considered not to be life-threatening. Outcome: not yet recovered.

The relevant concomitant medication included:

IBUPROFEN / DOLORMIN;

AMBROXOL HYDROCHLORIDE / MUCOSOLVAN;

PARACETAMOL / BENURON;

Third party case numbers

E2B Authority Number: DE-BFARM-08017695

(GLS)-DE-GRT: DE-08-DS-0122

Sender's comment:

Stevens-Johnson syndrome is assessed as serious due to the criterion hospitalization.

Stevens-Johnson syndrome is listed in the reference safety information of amoxicillin trihydrate.

The adverse event occurred in a reasonable temporal relationship to the administration of amoxicillin trihydrate. In addition the patient suffered from a mycoplasma infection presenting a cause for the development of Stevens-Johnson syndrome as well.

In conclusion Stevens-Johnson syndrome is assessed to be possibly related to the administration of amoxicillin trihydrate.

The information in this single case does not appear to alter the favourable benefit to risk ratio of amoxicillin trihydrate.

Reporter's comment:

McNeil Germany, Local Safety Officers, 07.03.2008 DE-JNJFOC-20080206506

Sender diagnosis: not reported

Narrative

This spontaneous report from a physician concerns a 6-year-old female from Germany: 00025-08.

The patient's weight, height and medical history were unknown. The patient was treated with ibuprofen (100 mg/5 ml/suspension, oral) 100 mg once total, on 24-DEC-2004 for common cold. Non-company suspect drugs included ambroxol hydrochloride (5 ml, 3 times total, 24-DEC-2004 to 25-DEC-2004) for common cold, amoxicillin trihydrate (5 ml, 3 times total, 27-DEC-2004 to 01-JAN-2005) for infection, and paracetamol (500 mg once total on 25-DEC-2004) for fever. Concomitant medications were not reported. On 31-DEC-2004, the patient experienced erythema exsudativum multiforme major. On 03-JAN-2005, the patient was admitted to the hospital. The physician considered the event of erythema exsudativum multiforme major most suspected to amoxicillin, and also considered the other suspect medications as possibly related. Additionally, an infection with mycoplasma was assessed as probable. Treatment with ibuprofen, ambroxol hydrochloride, amoxicillin trihydrate and paracetamol was withdrawn. As of MAR-2005, the patient was not fully recovered.

This report was serious (hospitalization).<dbtext>To_Char_NARRATIVE_2</dbtext>

Sender's comment:

Single case assessment (reported term/preferred term): IBUPROFEN

ERYTHEMA MULTIFORME MAJOR /STEVENS-JOHNSON SYNDROME: expected per local SmPc

The benefit-risk-ratio remains unchanged based on the single case assessment of this case. No measures according to § 29 (1) AMG have to be taken.

Reporter's comment:

Bene-Arzneimittel, 10.03.2008 DE-Bene-Arzneimittel-B220

Sender diagnosis: not reported

Narrative

siehe auch Patient's History;

Behandlung u.a. mit Ben-u-ron p.o. erfolgte aufgrund Fieber bis 38,5Â° und grippalem Infekt ("Mykoplasmeninfektion: ja, wahrscheinlich"); 5 Tage nach einmaliger Einnahme von Ben-u-ron (sowie unmittelbar nach 4- bis 5-tägiger Behandlung mit Amoxicillin) wurden folgende Hauterscheinungen beobachtet: "Brennen/Rötung MSH, Brennen/Schwellung Lippen, Schmerzen der Haut, Augen gerötet, MSH-Erosionen, Pruritus, Kokarden und Flecken, schließlich Hautblasen und Lippenerosionen". 7 Tage nach Einnahme von Ben-u-ron p.o. stationäre Aufnahme, wobei klin. Situation nicht als lebensbedrohlich eingestuft wurde.

Sender's comment:

Hauterscheinungen traten erst 5 Tage nach einmaliger Einnahme von Ben-u-ron p.o. auf.

Laut Dokumentation ist der Patient "noch nicht wiederhergestellt".....entspricht vermutlich einem Dokumentationsdefizit, da die Hauterscheinungen sicherlich nicht mehrere Jahre andauerten.....eine valide Stellungnahme ist nur auf Basis vollständiger Unterlagen möglich. Neue wissenschaftliche Erkenntnisse liegen nicht vor bzw. können durch diesen Fall nicht gewonnen werden.

Reporter's comment:

individuelle Bewertung durch Dermatologisches Expertengremium aufgrund spezifischem Bewertungsalgorithmus: Kat."5" = "Zusammenhang möglich" aufgrund engem zeitlichen Zusammenhang mit Einnahme von ben-u-ron. Begleitmedikationen waren Ibuprofen, Ambroxol und Amoxicillin; letzteres wurde vom Exp.-gremium als "verdächtiger Auslöser" eingestuft (Kat."4" = "Zusammenhang gut möglich")

Bene-Arzneimittel, 10.03.2008 DE-Bene-Arzneimittel-B220

Sender diagnosis: not reported

Narrative

siehe auch Patient's History;

Behandlung u.a. mit Ben-u-ron p.o. erfolgte aufgrund Fieber bis 38,5Â° und grippalem Infekt ("Mykoplasmeninfektion: ja, wahrscheinlich"); 5 Tage nach einmaliger Einnahme von Ben-u-ron (sowie unmittelbar nach 4- bis 5-tägiger Behandlung mit Amoxicillin) wurden folgende Hauterscheinungen beobachtet: "Brennen/Rötung MSH, Brennen/Schwellung Lippen, Schmerzen der Haut, Augen gerötet, MSH-Erosionen, Pruritus, Kokarden und Flecken, schließlich Hautblasen und Lippenerosionen". 7 Tage nach Einnahme von Ben-u-ron p.o. stationäre Aufnahme, wobei klin. Situation nicht als lebensbedrohlich eingestuft wurde.

Sender's comment:

Hauterscheinungen traten erst 5 Tage nach einmaliger Einnahme von Ben-u-ron p.o. auf.

Laut Dokumentation ist der Patient "noch nicht wiederhergestellt".....entspricht vermutlich einem Dokumentationsdefizit, da die Hauterscheinungen sicherlich nicht mehrere Jahre andauerten.....eine valide Stellungnahme ist nur auf Basis vollständiger Unterlagen möglich. Neue wissenschaftliche Erkenntnisse liegen nicht vor bzw. können durch diesen Fall nicht gewonnen werden.

Reporter's comment:

individuelle Bewertung durch Dermatologisches Expertengremium aufgrund spezifischem Bewertungsalgorithmus: Kat."5" = "Zusammenhang möglich" aufgrund engem zeitlichen Zusammenhang mit Einnahme von ben-u-ron. Begleitmedikationen waren Ibuprofen, Ambroxol und Amoxicillin; letzteres wurde vom Exp.-gremium als "verdächtiger Auslöser" eingestuft (Kat."4" = "Zusammenhang gut möglich")

McNeil Germany, Local Safety Officers, 20.03.2008 DE-JNJFOC-20080206506

Sender diagnosis: not reported

Narrative

This spontaneous report from a physician concerns a 6-year-old female from Germany: 00025-08.

The patient's weight, height and medical history were unknown. The patient was treated with ibuprofen (100 mg/5 ml/suspension, oral) 100 mg once total, on 24-DEC-2004 for common cold. Non-company suspect drugs included ambroxol hydrochloride (5 ml, 3 times total, 24-DEC-2004 to 25-DEC-2004) for common cold, amoxicillin trihydrate (5 ml, 3 times total, 27-DEC-2004 to 01-JAN-2005) for infection, and paracetamol (500 mg once total on 25-DEC-2004) for fever. Concomitant medications were not reported. On 31-DEC-2004, the patient experienced erythema exsudativum multiforme major. On 03-JAN-2005, the patient was admitted to the hospital. The physician considered the event of erythema exsudativum multiforme major most suspected to amoxicillin, and also considered the other suspect medications as possibly related. Additionally, an infection with mycoplasma was assessed as probable. Treatment with ibuprofen, ambroxol hydrochloride, amoxicillin trihydrate and paracetamol was withdrawn. As of MAR-2005, the patient was not fully recovered.

This report was serious (hospitalization).

Additional information received on 12-MAR-2008 indicated the body surface area involved was not calculated, and no testing regarding culprit drug or infection was done. No leukocyte stimulation testing was done. There were no concomitant diseases. It was reported that co-suspect drugs ambroxol hydrochloride and paracetamol were possibly responsible for the erythema exsudativum multiforme major and amoxicillin trihydrate was even more than possibly considered to be responsible. <dbtext>To_Char_NARRATIVE_2</dbtext>

Sender's comment:

Single case assessment (reported term/preferred term):

ERYTHEMA MULTIFORME MAJOR /STEVENS-JOHNSON SYNDROME: unexpected per local SmPc

The benefit-risk-ratio remains unchanged based on the single case assessment of this case. No measures according to § 29 (1) AMG have to be taken.

Reporter's comment:

Bene-Arzneimittel, 02.10.2009 DE-Bene-Arzneimittel-B220

Sender diagnosis: not reported

Narrative

siehe auch Patient's History;

Behandlung u.a. mit Ben-u-ron p.o. erfolgte aufgrund Fieber bis 38,5Â° und grippalem Infekt ("Mykoplasmeninfektion: ja, wahrscheinlich"); 5 Tage nach einmaliger Einnahme von Ben-u-ron (sowie unmittelbar nach 4- bis 5-tägiger Behandlung mit Amoxicillin) wurden folgende Hauterscheinungen beobachtet: "Brennen/Rötung MSH, Brennen/Schwellung Lippen, Schmerzen der Haut, Augen gerötet, MSH-Erosionen, Pruritus, Kokarden und Flecken, schließlich Hautblasen und Lippenerosionen". 7 Tage nach Einnahme von Ben-u-ron p.o. stationäre Aufnahme, wobei klin. Situation nicht als lebensbedrohlich eingestuft wurde.

Sender's comment:

Hauterscheinungen traten erst 5 Tage nach einmaliger Einnahme von Ben-u-ron p.o. auf.

Laut Dokumentation ist der Patient "noch nicht wiederhergestellt".....entspricht vermutlich einem Dokumentationsdefizit, da die Hauterscheinungen sicherlich nicht mehrere Jahre andauerten.....eine valide Stellungnahme ist nur auf Basis vollständiger Unterlagen möglich. Neue wissenschaftliche Erkenntnisse liegen nicht vor bzw. können durch diesen Fall nicht gewonnen werden.

Reporter's comment:

individuelle Bewertung durch Dermatologisches Expertengremium aufgrund spezifischem Bewertungsalgorithmus: Kat."5" = "Zusammenhang möglich" aufgrund engem zeitlichen Zusammenhang mit Einnahme von ben-u-ron. Begleitmedikationen waren Ibuprofen, Ambroxol und Amoxicillin; letzteres wurde vom Exp.-gremium als "verdächtigster Auslöser" eingestuft (Kat."4" = "Zusammenhang gut möglich")

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08017844

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes	Yes	Yes	No	No	No		

Patient

Initials:	Sex: Male	Birth: privacy	Age: 59 Year	weight (kg):	BMI_int:	Age fetus:
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Patientdeath

deathdate: 20050119	deathcause: Stevens Johnson syndrome
deathdate: 20050119	deathcause: Toxic epidermal necrolysis

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens-Johnson syndrome	20041215				fatal
2 Toxic epidermal necrolysis	20041215				fatal

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
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1	Aldactone		50 Mg milligram(s)/Day	-
	spironolactone			-
2	Arterenol		/	-
	norepinephrine hydrochloride		231	-
3	Beloc	Intravenous (not otherwis	/	-
	metoprolol tartrate		231	-
4	Beloc-Zok		/	-
	metoprolol succinate			-
5	Benuron	Rectal	1000 Mg milligram(s)/	-
	paracetamol		242	-
6	bepanthen		/	-
	dexpantenol			-
7	Calcium		/	-
	calcium		231	-
8	ciprobay		500 Mg milligram(s)/	-
	ciprofloxacin hydrochloride			-
9	ciprobay		500 Mg milligram(s)/Day	-
	ciprofloxacin hydrochloride			-
10	clexane		60 ml millilitre(s)/	-
	enoxaparin		231	-
11	clexane		80 ml millilitre(s)/Day	-
	enoxaparin		231	-
12	clont		400 Mg milligram(s)/	-
	metronidazole			-
13	Cordarex		600 Mg milligram(s)/Day	-
	amiodarone hydrochloride			-
14	Cordarex		6 Df dosage form/	-
	amiodarone hydrochloride			-
15	decortin		/	-
	prednisone			-
16	decortin		100 Mg milligram(s)/	-
	prednisone		231	-
17	decortin h		/	-
	prednisolone			-
18	Dormicum		90 ml millilitre(s)/	-
	midazolam hydrochloride			-
19	fenistil	Topical	/	-
	dimetindene maleate		95	-
20	Fentanyl	Intravenous (not otherwis	/	-
	fentanyl		231	-
21	Fresubin		/	-
	carbohydrates nos; fatty acids nos; minerals nos; proteins nos; vitamins nos			-
22	Furosemide		/	-
	furosemide			-
23	glucose		/	-
	glucose			-
24	humanalbumin 5%		5 % percent/	-
	albumin human		231	-
25	Imodium		/	-
	loperamide hydrochloride			-
26	kalium		/	-
	potassium		231	-
27	Kaliumchlorid		40 Mg milligram(s)/	-
	potassium chloride			-
28	Konakion	Intravenous (not otherwis	/	-
	phytomenadione			-

			231	-
29	Lasix		40 Mg milligram(s)/	-
	furosemide sodium			-
30	Marcumar		/	-
	phenprocoumon			-
31	Meropenem		1 G gram(s)/Day	-
	meropenem			-
32	Natriumbikarbonat		250 ml millilitre(s)/	-
	sodium bicarbonate			-
33	Natriumchlorid		/	-
	sodium chloride			-
34	Nexium		40 Mg milligram(s)/	-
	esomeprazole			-
35	Nexium		40 Mg milligram(s)/	-
	esomeprazole		231	-
36	novodigal		0.1 Mg milligram(s)/	-
	beta-acetyldigoxin			-
37	pantozol	Oral	2 Df dosage form/Day	-
	pantoprazole sodium		94	-
38	paracodin	Oral	25 Gtt drop(s)/	-
	dihydrocodeine bitartrate		158	-
39	Paspertin	Oral	20 Gtt drop(s)/	-
	metoclopramide hydrochloride		158	-
40	Red blood cells, concentrated		/	-
	red blood cells			-
41	ringers solution		/	-
	calcium chloride; potassium chloride; sodium chloride; sodium chloride compound injection			-
42	Rocephin	Intravenous (not otherwis	2 G gram(s)/	-
	ceftriaxone sodium		231	-
43	tavegil	Oral	/	-
	clemastine			-
44	tavegil	Oral	/	-
	clemastine			-
45	tavegil	Topical	/	-
	clemastine		95	-
46	tavegil		3 Df dosage form/Day	-
	clemastine			-
47	tavor		3 Mg milligram(s)/Day	-
	lorazepam			-
48	tavor		1 Mg milligram(s)/	-
	lorazepam			-
49	Torem		20 Mg milligram(s)/Day	-
	torasemide			-
50	Torem		30 Mg milligram(s)/Day	-
	torasemide			-
51	VANCOMYCIN		2 G gram(s)/Day	-
	vancomycin			-

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Stevens-Johnson syndrome			Unknown	

Primary source

Qualification:	Other Health Professional	observestudytype:	Other studies
Literature:			

Qualification: Physician

observestudytype:

Literature:

Narratives (nur bei elektronischem Eingang)ROCHE, 14.02.2008 DE-ROCHE-545524

Sender diagnosis: not reported

Narrative

INTENSIVE MONITORING (REGISCAR)

THIS CASE CONCERNS A 59 YEARS OLD MALE PATIENT WHO EXPERIENCED FATAL EVENTS OF STEVENS-JOHNSONS-SYNDROME AND TOXIC EPIDERMAL NECROLYSIS WHILST RECEIVING TORASEMIDE FOR RENAL INSUFFICIENCY, PHENPROCOUMON FOR HEART FOLDING REPLACEMENT, PHYTOMENADIONE AS SUBSTITUTION, CLEMASTINE (CLEMASIN) FOR PRURITUS, SODIUM CHLORIDE FOR SOLUTION FOR MEDIUM, METOPROLOL (BELOC-ZOK FORTE) FOR CARDIAC INSUFFICIENCY, PREDNISOLONE (DECORTIN H), FLUCONAZOLE/LORAZEPAM/OXYBUTYNIN/SIMVASTATIN (TAVOR) FOR SLEEPING AIDS, CIPROFLOXACIN (CIPROBAY) FOR INFECTION, FUROSEMIDE (LASIX) FOR OEDEMA, PARACETAMOL (BENURON) FOR FEVER, LOPERAMIDE (IMODIUM) FOR DIARRHOEA, PREDNISONE (DECORTIN) FOR NEPHROT. SYNDROME, ENOXAPARIN SODIUM (CLEXANE) FOR THROMBOSEPROPH., METRONIDAZOLE (CLONT) FOR DIARRHOEA, CALCIUM (CA²⁺) AS SUBSTITUTION, ESOMEPRAZOLE (MEDIUM) FOR STOMACH PROTECTION, SODIUM BICARBONATE (NABIC), ALBUMIN HUMAN (HUMANALBUMIN), EPOETIN NOS (ERYTHROZYTEN), CALCIUM CHLORIDE/POTASSIUM CHLORIDE/SODIUM CHLORIDE (RINGERSLOSUNG), POTASSIUM CHLORIDE (KCL), POTASSIUM (KALIUM) AS SUBSTITUTION AND METOCHLOPRAMIDE (PASPERTIN) FOR STOMACH DISORDER.

THE PATIENT WAS RECEIVING MULTIPLE CONCOMITANT MEDICATIONS.

ON 6 NOVEMBER 2004, THE PATIENT WAS HOSPITALISED FOR THORACIC SURGERY. THE NEXT DAY, HE COMMENCED TREATMENT WITH TORASEMIDE (15 MG TO 20 MG DAILY) AND ORAL METOPROLOL (ONCE DAILY). ON 25 NOVEMBER 2004, HE RECEIVED ORAL PHENPROCOUMON AND INTRAVENOUS SODIUM CHLORIDE. ON 15 DECEMBER 2004, HE RECEIVED ORAL CLEMASTINE (1 DOSE FORM TWICE DAILY TO 1 DOSE FORM THRICE DAILY). THE SAME DAY, HE EXPERIENCED SPOTTED, CONFLUENCING ERYTHEMATOSUS EXANTHEMA, PARTLY WITH PETECHIAL BLEEDINGS WHICH WAS DIAGNOSED AS LIFE-THREATENING EVENTS OF STEVENS-JOHNSONS-SYNDROME AND TOXIC EPIDERMAL NECROLYSIS. TWO DAYS LATER, TORASEMIDE WAS DISCONTINUED AND ORAL PREDNISOLONE WAS STARTED. THE NEXT DAY, HE STARTED ORAL FLUCONAZOLE/LORAZEPAM/OXYBUTYNIN/SIMVASTATIN (1 MG TO 1.5 MG DAILY); THE NEXT DAY, ON 20 DECEMBER 2004, HE STARTED ORAL CIPROFLOXACIN (250 MG TWICE DAILY TO 500 MG DAILY) AND INTRAVENOUS FUROSEMIDE (40 MG ONCE DAILY) AND SODIUM CHLORIDE WAS DISCONTINUED. TWO DAYS LATER, CLEMASTIN WAS DISCONTINUED. THE NEXT DAY, PREDNISOLONE WAS DISCONTINUED. ON 26 DECEMBER 2004, PARACETAMOL (1000 MG SINGLE DOSE) WAS RECEIVED. THE NEXT DAY, ORAL LOPERAMIDE (SINGLE DOSE), ORAL PREDNISONE, SUBCUTANEOUS ENOXAPARIN SODIUM (40 ML TWICE DAILY TO 60 ML ONCE DAILY), ORAL CLEMASTINE, INTRAVENOUS SODIUM CHLORIDE WERE STARTED. THE NEXT DAY, PHENPROCOUMON, CIPROFLOXACIN, ENOXAPARIN SODIUM AND METRONIDAZOLE WERE DISCONTINUED. THE NEXT DAY, CLEMASTINE AND SODIUM CHLORIDE WERE DISCONTINUED AND INTRAVENOUS CALCIUM (SINGLE DOSE) WAS STARTED. ON 30 DECEMBER 2004, ORAL ESOMEPRAZOLE (40 MG ONCE DAILY) WAS STARTED. THE NEXT DAY, HE RECEIVED INTRAVENOUS PHYTOMENADIONE (SINGLE DOSE), ENOXAPARIN SODIUM, INTRAVENOUS SODIUM BICARBONATE (SINGLE DOSE), INTRAVENOUS ALBUMIN HUMAN (250 ML SINGLE DOSE), INTRAVENOUS EPOETIN NOS (SINGLE DOSE). ON 1 JANUARY 2005, HE STARTED INTRAVENOUS CALCIUM CHLORIDE/POTASSIUM CHLORIDE/SODIUM CHLORIDE (1000 ML TO 2000 ML) AND PHENPROCOUMON WAS RESTARTED. THE NEXT DAY, HE RECEIVED INTRAVENOUS POTASSIUM CHLORIDE (40 MG SINGLE DOSE) AND FUROSEMIDE WAS DISCONTINUED. THE NEXT DAY, ORAL PREDNISONE WAS DISCONTINUED AND ORAL POTASSIUM (1 DOSE FORM ONCE DAILY TO 1 DOSE FORM THRICE DAILY WAS STARTED. THE NEXT DAY, ORAL PREDNISOLONE WAS RESTARTED. ORAL ESOMEPRAZOLE AND INTRAVENOUS CALCIUM CHLORIDE/POTASSIUM CHLORIDE/SODIUM CHLORIDE WERE DISCONTINUED. THE NEXT DAY, ON 5 JANUARY 2005, ORAL POTASSIUM WAS DISCONTINUED. THE NEXT DAY, TORASEMIDE WAS RESTARTED AND PHENPROCOUMON AND ENOXAPARIN SODIUM WERE DISCONTINUED. THE SAME DAY, ON 6 JANUARY 2005, SPOTS WERE VISIBLE. THE NEXT DAY, ORAL METOCLOPRAMIDE WAS STARTED AND TORASEMIDE, METOPROLOL AND PREDNISOLONE WERE DISCONTINUED. ON 13 JANUARY 2005, SKIN BLISTERS APPEARED. SPOTS AND BLISTERS WERE THE COMANIFESTATION OF THE EVENTS. ON 19 JANUARY 2005, THE PATIENT DIED OF STEVENS-JOHNSONS-SYNDROME AND TOXIC EPIDERMAL NECROLYSIS.

THE REPORTER ASSESSED THE EVENTS OF FATAL STEVENS-JOHNSONS-SYNDROME AND TOXIC EPIDERMAL NECROLYSIS AS VERY WELL POSSIBLY RELATED TO CIPROFLOXACIN, POSSIBLY RELATED TO PHENPROCOUMON, METOPROLOL, FUROSEMIDE, PREDNISONE, ENOXAPARIN SODIUM, ESOMEPRAZOL AND CLEMASTIN. THE RELATIONSHIP BETWEEN THE EVENTS AND OTHER SUSPECT DRUGS WAS QUESTIONABLE.

NO FURTHER INFORMATION WAS AVAILABLE.

Sender's comment:

CIOMS TEXT: FATAL STEVENS-JOHNSONS-SYNDROME AND TOXIC EPIDERMAL NECROLYSIS ARE NOT LISTED IN THE COMPANY REFERENCE DOCUMENT (CDS) FOR TORASEMIDE AND ISPI FOR PHENPROCOUMON AND PHYTOMENADIONE. BASED ON THE INFORMATION RECEIVED FOR THIS CASE, A CAUSAL RELATIONSHIP BETWEEN THE REPORTED EVENTS AND TREATMENT WITH

TORASEMIDE, PHENPROCOUMON AND PHYTOMENADIONE IS ASSESSED AS POSSIBLE. COMPANY CAUSALITY WAS ASSESSED BASED ON THE NARANJO ALGORITHM.

HOWEVER, POSSIBLE CONFOUNDERS FOR BOTH THE EVENTS INCLUDE PANTOPRAZOLE.

CONFOUNDER FOR STEVENS-JOHNSON SYNDROME INCLUDE FUROSEMIDE.; ***LOCAL ASSESSMENT: STEVENS JOHNSON SYNDROME, TOXIC EPIDERMAL NECROLYSIS = NOT LABELED

EINZELFALLBEOBACHTUNG. KEIN HANDLUNGSBEDARF!

Reporter's comment:

AZPROD0000, 29.02.2008 DE-AstraZeneca-2008PK00414

Sender diagnosis: not reported

Narrative

{STEVENS-JOHNSON SYNDROME (EVIDENTED CASE), TOXIC EPIDERMAL NECROLYSIS TRANSITION FORM (EVIDENTED CASE)}

A report was received from the Centre for Documentation of Severe Skin Reactions in Germany via the German Health Authority (BfArM) concerning a 59 year old male patient. The patient was being treated with NEXIUM ORAL (esomeprazole magnesium) QD oral, BELOC-ZOK (metoprolol succinate), TAVEGIL (clemastine), MARCUMAR (phenprocoumon), CIPROBAY (ciprofloxacin hydrochloride), LASIX (furosemide), DECORTIN (prednisone), CLEXANE (enoxaparin sodium) and CLONT (metronidazole).

Concomitant drugs included sodium chloride, prednisolone, lorazepam, paracetamol, loperamide hydrochloride, calcium, sodium bicarbonate, albumin human, red blood cells, concentrated, phytomenadione, electrolytes, potassium chloride, potassium and metoclopramide hydrochloride.

Medical history was not provided.

The patient started metoprolol succinate on 07-Nov-2004 and it was not reported when the patient started esomeprazole magnesium. On 19-Dec-2004 the patient suffered from stevens-johnson syndrome and toxic epidermal necrolysis transition. Further course was not reported. The patient died.

The events were considered serious due to life threatening and death.

The reporting experts from the documentation centre for severe skin reactions considered (during September 2005) the event possible related to esomeprazole magnesium and metoprolol succinate and possibly related to the other suspected drugs.

Sender's comment:

Stevens-Johnson-Syndrome and toxic epidermal necrolysis are not listed for metoprolol but have been associated rarely with esomeprazole. It is difficult to assess causality based on the information provided because several other co-suspected drugs may also have been causative for the event.

Report according to article 63b AMG. Stevens-Johnson-Syndrome and toxic epidermal necrolysis are not listed for metoprolol but have been associated rarely with esomeprazole. It is difficult to assess causality based on the information provided because several other co-suspected drugs may also have been causative for the event.

Causality assessment: unassessable

NO

Reporter's comment:

Sanofi-Aventis Deutschland GmbH, 04.03.2008 DE-AVENTIS-200810578DE

Sender diagnosis: not reported

Narrative

Initial report:

This case of Stevens-Johnson syndrome (SJS)-fatal and Toxic Epidermal Necrolysis (TEN)-fatal was received from the German health authority (DE-BFARM-08017844). It was reported by the German Documentation Centre for Severe Skin Reactions in Freiburg / Breisgau, Germany (code-no. 9160244). The centre is conducting a prospective study on the incidence and drug aetiology of severe skin reactions. Diagnosis of SJS and TEN were made by an expert panel, who assessed it as a "definitive case". Causal relationship was assessed by the experts in September 2005 using an algorithm.

This case concerned a 59-year-old male patient, who was treated amongst others with furosemide (Lasix 40) and enoxaparin sodium (Clexane). Treatment dates, dosage and indication were not provided. The patient developed SJS and TEN, diagnosed on 15-Dec-2004. Furosemide and enoxaparin sodium were assessed by this centre as suspected drugs.

Further suspected drugs: Clemastine, metoprolol succinate, phenprocoumon, ciprofloxacin HCl, prednisone, metronidazole, and esomeprazole, each given for unspecified indication.

Non-suspected drugs: Spironolactone, vancomycin HCl, dimetindene maleate, sodium chloride, torasemide, prednisolone, ceftriaxone, dihydrocodeine, lorazepam, dexamphenol, pantoprazole sodium, clemastine, paracetamol, loperamide HCl, glucose, calcium, lovastatin, albumin, concentrated red blood cells, phytomenadione, sodium chloride compound injection, potassium chloride, metoclopramide HCl, furosemide, ambroxol, clemastine, amiodarone HCl, meropenem, norepinephrine HCl, metoprolol tartrate, prednisone, nitrazepam, esomeprazole, fentanyl citrate, fresubin, digoxin, each given for unspecified indication.

Medical history was not provided.

The patient was hospitalized on 06-Nov-2004. On 15-Dec-2005, a Stevens-Johnson-Syndrome and TEN were diagnosed, symptoms were not provided.

The reporter ticked as outcome of the events "exitus". Stop date was specified with 19-Jan-2005.

Causality assessment for company drugs furosemide (Lasix 40) and enoxaparin sodium (Clexane) was given with "possible".

Laboratory: No values provided.

Sender's comment:

29-Feb-2008 Sanofi Aventis Company Comment:

The temporal relationship between the events and administration of furosemide and enoxaparin seems possible. Dechallenge and rechallenge is not applicable as concomitant medications were altered at the same time. In addition, concomitant drugs as reported here may be considered an alternative cause to explain the events.

29-FEB-2008 Sanofi Aventis Germany: STEVENS-JOHNSON-SYNDROME - FATAL and TOXIC EPIDERMAL NECROLYSIS - FATAL are considered locally unlabelled for the suspected product. The facts presented here, however, are not very suggestive of a causal relationship (please refer to the intl. evaluator's comment), and thus do not imply a change in the state of the scientific knowledge for the products. Measures according to Para 29 German Drug Law (AMG), section (1) concerning the approval status are not deemed necessary.

Reporter's comment:

Merck KGaA, 05.03.2008 DE-Merck KGaA-6041150

Sender diagnosis: not reported

Narrative

This is a case from the Schoepf registry / intensified monitoring programme being called RegiSCAR study (# 9160244) that is coordinated by the Dermatological University Hospital Freiburg, Germany, and that registers and evaluates all hospitalized cases of severe skin reactions such as toxic epidermal necrolysis (TEN), Stevens-Johnson syndrome (SJS), acute generalised exanthematous pustulosis (AGEP) and drug reaction with eosinophilia and systemic symptoms (DRESS) in 6 European countries, and that documents the demographic characteristics, the role of infection and drug intake in the past history of the severe skin reaction.

The report was received via the German Health Authority (# 08017844) on 29 Feb 2008.

On 15 Dec 2004, this 59-year-old male patient developed life-threatening transition form of Stevens-Johnson syndrome (SJS) / toxic epidermal necrolysis (TEN), ascertained case, from which he died on 19 Jan 2005 under Decortin and Decortin H treatment amongst 43 other drugs or preparations.

Other suspected drugs included Tavegil (clemastine), NaCl 0.9%, Beloc-Zok forte (metoprolol), Torem (torasemide), Marcumar (phenprocoumon),

Tavor (lorazepam), Ciprobay (ciprofloxacin), Lasix (furosemide), Benuron (paracetamol), Imodium (loperamide), Clexane (enoxaparin sodium), Clont (metronidazole), Ca²⁺, Nexium (esomeprazole), Nabic (NaHCO₃), Humanalbumin, erythrocytes concentrate, Konakion (phytomenadion), Ringer's solution, KCL, Potassium effervescent and Paspertin (metoclopramide).

As concomitant drugs were considered: Aldactone (spironolactone), vancomycin, Fenistil (dimetinden), Rocephin (ceftriaxone), Paracodin (dihydrocodeine), Bepanthen (dexpanthenol), Pantozol (pantoprazole), Tavegil (clemastin), glucose solution for injection, Furosemid (furosemide), Mucosolvan (ambroxol), Cordarex (amiodarone), Meronem (meropenem), Arterenol (norepinephrin), Beloc (metoprolol), Decortin (prednison), Dormicum (midazolam), Nexium (esomeprazole), fentanyl, Fresubin (proteins, lipids, carbohydrates, vitamins, micro elements) and Novodigal (digoxin).

The patient started treatment with Decortin H (prednisone) and Decortin (prednisone) on an unknown date for unknown indication.

On 15 Dec 2004, the patient experienced life-threatening TEN (transition form). On 19 Jan 2005, he died from transition form of TEN. No concurrent infection or other concurrent condition was present in the patient.

The documentation of the patient was then transferred to the expert panel of the Schoepf registry. This case was evaluated by the expert panel of Prof. Schoepf in Sep 2005. There the diagnosis of transition form of SJS / TEN was ascertained and suspected drugs and concomitant drugs were identified.

The assessment of causal relationship between the suspected drugs and the occurrence of severe skin reaction is recorded in a rating scale from 1-8 scores (1= definite, 2= very probable, 3= probable, 4= likely; 5= possible; 6= doubtful; 7= no relationship, 8= unlikely).

The expert panel assessed the transition form of SJS / TEN as likely for Ciprobay, as possible for Tavegil tab., Beloc-Zok forte, Marcumar, Lasix, Decortin, Clexane, Clont, Nexium, and as doubtful for NaCl, Torem, Decortin-H, Tavor, Benuron, Imodium, Ca²⁺, Nabic, human albumin, erythrocytes concentrate, Konakion, Ringer's solution, KCl, potassium effervescent and Paspertin.

Sender's comment:

Case from the Schoepf Registry / Intensified Monitoring Programme for hospitalized cases of severe skin reactions at the University of Freiburg in Germany (RegiSCAR study) being reported by Health Authority.

Ascertained case of transition form of Stevens-Johnson syndrome (SJS) / toxic epidermal necrolysis (TEN), with fatal outcome reported in a 59-year old male patient taking Decortin (prednisone) and Decortin H (prednisolone) among 43 further drugs.

The documentation of the patient was then transferred to the expert panel of the Schoepf registry for evaluation in Sep 2005. There the diagnosis of transition form of SJS / TEN was ascertained. No concurrent infection or other concurrent condition was present in the patient.

Co-suspected drugs included Tavegil (clemastine), NaCl 0.9%, Beloc-Zok forte (metoprolol), Torem (torasemide), Marcumar (phenprocoumon), Tavor (lorazepam), Ciprobay (ciprofloxacin), Lasix (furosemide), Benuron (paracetamol), Imodium (loperamide), Clexane (enoxaparin sodium), Clont (metronidazole), Ca²⁺, Nexium (esomeprazole), Nabic (NaHCO₃), Humanalbumin, erythrocytes concentrate, Konakion (phytomenadion), Ringer's solution, KCL, Potassium effervescent and Paspertin (metoclopramide).

As concomitant drugs were considered: Aldactone (spironolactone), vancomycin, Fenistil (dimetinden), Rocephin (ceftriaxone), Paracodin (dihydrocodeine), Bepanthen (dexpanthenol), Pantozol (pantoprazole), Tavegil (clemastin), glucose solution for injection, Furosemid (furosemide), Mucosolvan (ambroxol), Cordarex (amiodarone), Meronem (meropenem), Arterenol (norepinephrin), Beloc (metoprolol), Decortin (prednison), Dormicum (midazolam), Nexium (esomeprazole), fentanyl, Fresubin (proteins, lipids, carbohydrates, vitamins, micro elements) and Novodigal (digoxin).

Relevant medical history: not provided.

The expert panel assessed the causality for transition form of SJS / TEN as likely for Ciprobay, as possible for Tavegil tab., Beloc-Zok forte, Marcumar, Lasix, Decortin, C

Reporter's comment:

NOVARTIS, 07.03.2008 DE-NOVNCHHQ-CHNY2008DE01075

Sender diagnosis: not reported

Narrative

This report combines initial report received from an intensive monitoring project (German documentation centre for severe skin disease dZh, Prof. Schärf, Freiburg) via the German Health Authority, BfArM (Ref: DE-BFARM-08017844) on 27 Feb 2008 and follow-up information received on 27 Feb 2008 from German Health Authority. A 59-year-old hospitalised patient was treated with Aldactone (spironolactone) 25mg from 07 Nov 2004, Vancomycin injection solution 1g from 14 Nov 2004, Fenistil Gel (dimetindene maleate) topical from 15 Dec 2004, Tavegil oral(clemastine fumarate)

from 15 Dec 2004, sodium chloride injection solution 0.9% from 25 Nov 2004, Beloc-Zok forte (metoprolol) from 07 Nov 2004, Torem (torasemid) 10mg from 07 Nov 2004, Marcumar from 25 Nov 2004, Decortin H (prednisolon) 20 mg and then 10 mg, Rocephin (ceftriaxon) injection solution 2g, Paracodin Tropfen (dihydrocodein), Tavor 1mg (lorazepam), Bepanthen Crème (dexpantenol), Ciprobay 500mg (ciprofloxacin), Lasix injection solution 40mg (furosemid), Pantozol (pantoprazol) oral, Tavegil Gel (clemastine fumarate), Benuron (paracetamol) suppositories 1000mg, Imodium (loperamid), Decortin 100mg (prednison), Clexane (enoxaparin natrium) injection solution subcutaneous, glucose injection solution, Clont 400mg (metronidazol), Calcium injection solution (unknown salt), Nexium 40mg (esomeprazol), Nabic (sodium bicarbonate) injection solution 250ml, human albumin injection solution 5%, red blood cell concentrate, Konakion (phytomenadione) injection solution, Ringer solution, potassium chloride injection solution 40mg, Kaliumbrause (kalium unknown salt), Paspertin Tropfen (metoclopramid), Furosemid 40mg, Mucosolvan injection solution (ambroxol), Tavegil injection (clemastine fumarate), Cordarex 200mg (amiodaron), Cordarex injection solution (amiodaron), Meronem injection solution 0.5g (meropenem), Arterenol injection solution 10mg (norepinephrine), Beloc injection solution 50mg (metoprolol), Decortin injection solution 100mg (predison), Dormicum injection solution 90ml (midazolam), Nexium injection solution 40mg (esomeprazol), Fentanyl injection solution 1.5ml (fentanyl), Fresubin solution (carbohydrates NOS, fatty acids NOS, minerals NOS, protein NOS, vitamins), Novodigal 0.1mg (digoxin). The patient was diagnosed with "transition form" of Stevens-Johnson syndrome / Toxic epidermal necrolysis on 15 Dec 2004. For the duration of reaction, the date 19 Jan 2005 was stated. The reaction was rated as life-threatening and the patient died. The reaction was rated as life-threatening and the patient died. The intensive monitoring project assessed the causality with regard to the patient's multiple medication (using an algorithm with 8 different levels, ranging from definite to improbable) as follows: Ciprobay was rated as "well possible". Tavegil oral, Beloc-Zok forte, Marcumar, Lasix injection solution, Decortin, Clexane injection solution, Clont, and Nexium were rated as "possible". Sodium chloride injection solution, Torem 10mg, Decortin H, Tavor, Benuron suppositories, Imodium, Calcium injection solution, Nabic injection solution, human albumin injection solution 5%, red blood cell concentrate, Konakion injection solution, Ringer solution, potassium chloride injection solution, Kaliumbrause, and Paspertin Tropfen were rated as "questionable". Aldactone, Vancomycin injection solution, Fensitil Gel topical, Rocephin injection solution, Paracodin Tropfen, Bepanthen Creme, Pantozol oral, Tavegil Gel, Glucose injection solution, Furosemid, Mucosolvan injection solution, Tavegil injection solution, Cordarex, Cordarex injection solution, Meronem injection solution, Arterenol injection solution, Beloc injection solution, Decortin injection solution, Dormicum injection solution, Nexium injection solution, Fentanyl injection solution, Fresubin solution, and Novodigal were rated as "no causal relationship". It was added that the patient had no infection and no other pre-existing condition as other possible risk factors. Final outcome: death.

Sender's comment:

Reporter's comment:

Merck KGaA, 12.03.2008 DE-Merck KGaA-6041150

Sender diagnosis: not reported

Narrative

This is a case from the Schoepf registry / intensified monitoring programme being called RegiSCAR study (# 9160244) that is coordinated by the Dermatological University Hospital Freiburg, Germany, and that registers and evaluates all hospitalized cases of severe skin reactions such as toxic epidermal necrolysis (TEN), Stevens-Johnson syndrome (SJS), acute generalised exanthematous pustulosis (AGEP) and drug reaction with eosinophilia and systemic symptoms (DRESS) in 6 European countries, and that documents the demographic characteristics, the role of infection and drug intake in the past history of the severe skin reaction.

The report was received via the German Health Authority (# 08017844) on 29 Feb 2008.

On 15 Dec 2004, this 59-year-old male patient developed life-threatening transition form of Stevens-Johnson syndrome (SJS) / toxic epidermal necrolysis (TEN), ascertained case, from which he died on 19 Jan 2005 under Decortin and Decortin H treatment amongst 43 other drugs or preparations.

Other suspected drugs included Tavegil (clemastine), NaCl 0.9%, Beloc-Zok forte (metoprolol), Torem (torasemide), Marcumar (phenprocoumon), Tavor (lorazepam), Ciprobay (ciprofloxacin), Lasix (furosemide), Benuron (paracetamol), Imodium (loperamide), Clexane (enoxaparin sodium), Clont (metronidazole), Ca²⁺, Nexium (esomeprazole), Nabic (NaHCO₃), Humanalbumin, erythrocytes concentrate, Konakion (phytomenadion), Ringer's solution, KCL, Potassium effervescent and Paspertin (metoclopramide).

As concomitant drugs were considered: Aldactone (spironolactone), vancomycin, Fenistil (dimetinden), Rocephin (ceftriaxone), Paracodin (dihydrocodeine), Bepanthen (dexpantenol), Pantozol (pantoprazole), Tavegil (clemastin), glucose solution for injection, Furosemid (furosemide), Mucosolvan (ambroxol), Cordarex (amiodarone), Meronem (meropenem), Arterenol (norepinephrin), Beloc (metoprolol), Decortin (prednison), Dormicum (midazolam), Nexium (esomeprazole), fentanyl, Fresubin (proteins, lipids, carbohydrates, vitamins, micro elements) and Novodigal (digoxin).

The patient started treatment with Decortin H (prednisone) and Decortin (prednisone) on an unknown date for unknown indication.

On 15 Dec 2004, the patient experienced life-threatening transition form of SJS / TEN. On 19 Jan 2005, he died from transition form of SJS / TEN. No concurrent infection or other concurrent condition was present in the patient.

The documentation of the patient was then transferred to the expert panel of the Schoepf registry. This case was evaluated by the expert panel of Prof. Schoepf in Sep 2005. There the diagnosis of transition form of SJS / TEN was ascertained and suspected drugs and concomitant drugs were identified.

The assessment of causal relationship between the suspected drugs and the occurrence of severe skin reaction is recorded in a rating scale from 1-8 scores (1= definite, 2= very probable, 3= probable, 4= likely; 5= possible; 6= doubtful; 7= no relationship, 8= unlikely).

The expert panel assessed the transition form of SJS / TEN as likely for Ciprobay, as possible for Tavegil tab., Beloc-Zok forte, Marcumar, Lasix, Decortin, Clexane, Clont, Nexium, and as doubtful for NaCl, Torem, Decortin-H, Tavor, Benuron, Imodium, Ca²⁺, Nabic, human albumin, erythrocytes concentrate, Konakion, Ringer's solution, KCl, potassium effervescent and Paspertin.

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On 12 Mar 2008 new version was created for correction of event description (term of diagnosis: transition form of SJS / TEN).

Sender's comment:

Case from the Schoepf Registry / Intensified Monitoring Programme for hospitalized cases of severe skin reactions at the University of Freiburg in Germany (RegiSCAR study) being reported by Health Authority.

Ascertained case of transition form of Stevens-Johnson syndrome (SJS) / toxic epidermal necrolysis (TEN), with fatal outcome reported in a 59-year old male patient taking Decortin (prednisone) and Decortin H (prednisolone) among 43 further drugs.

The documentation of the patient was then transferred to the expert panel of the Schoepf registry for evaluation in Sep 2005. There the diagnosis of transition form of SJS / TEN was ascertained. No concurrent infection or other concurrent condition was present in the patient.

Co-suspected drugs included Tavegil (clemastine), NaCl 0.9%, Beloc-Zok forte (metoprolol), Torem (torasemide), Marcumar (phenprocoumon), Tavor (lorazepam), Ciprobay (ciprofloxacin), Lasix (furosemide), Benuron (paracetamol), Imodium (loperamide), Clexane (enoxaparin sodium), Clont (metronidazole), Ca²⁺, Nexium (esomeprazole), Nabic (NaHCO₃), Humanalbumin, erythrocytes concentrate, Konakion (phytomenadion), Ringer's solution, KCL, Potassium effervescent and Paspertin (metoclopramide).

As concomitant drugs were considered: Aldactone (spironolactone), vancomycin, Fenistil (dimetinden), Rocephin (ceftriaxone), Paracodin (dihydrocodeine), Bepanthen (dexpantenol), Pantozol (pantoprazole), Tavegil (clemastin), glucose solution for injection, Furosemid (furosemide), Mucosolvan (ambroxol), Cordarex (amiodarone), Meronem (meropenem), Arterenol (norepinephrin), Beloc (metoprolol), Decortin (prednison), Dormicum (midazolam), Nexium (esomeprazole), fentanyl, Fresubin (proteins, lipids, carbohydrates, vitamins, micro elements) and Novodigal (digoxin).

Relevant medical history: not provided.

The expert panel assessed the causality for transition form of SJS / TEN as likely for Ciprobay, as possible for Tavegil tab., Beloc-Zok forte, Marcumar, Lasix, Decortin, C

Reporter's comment:

Sanofi-Aventis Deutschland GmbH, 19.03.2008 DE-AVENTIS-200810578DE

Sender diagnosis: not reported

Narrative

Initial report:

This case of Stevens-Johnson syndrome (SJS)-fatal and Toxic Epidermal Necrolysis (TEN)-fatal was received from the German health authority (DE-BFARM-08017844). It was reported by the German Documentation Centre for Severe Skin Reactions in Freiburg / Breisgau, Germany (code-no. 9160244). The centre is conducting a prospective study on the incidence and drug aetiology of severe skin reactions. Diagnosis of SJS and TEN were made by an expert panel, who assessed it as a "definitive case". Causal relationship was assessed by the experts in September 2005 using an algorithm.

This case concerned a 59-year-old male patient, who was treated amongst others with furosemide (Lasix 40) and enoxaparin sodium (Clexane). Treatment dates, dosage and indication were not provided. The patient developed SJS and TEN, diagnosed on 15-Dec-2004. Furosemide and enoxaparin sodium were assessed by this centre as suspected drugs.

Further suspected drugs: Clemastine, metoprolol succinate, phenprocoumon, ciprofoxacin HCl, prednisone, metronidazole, and esomeprazole, each

given for unspecified indication.

Non-suspected drugs: Spironolactone, vancomycin HCl, dimetindene maleate, sodium chloride, torasemide, prednisolone, ceftriaxone, dihydrocodeine, lorazepam, dexamethasone, pantoprazole sodium, clemastine, paracetamol, loperamide HCl, glucose, calcium, lovastatin, albumin, concentrated red blood cells, phytomenadione, sodium chloride compound injection, potassium chloride, metoclopramide HCl, furosemide, ambroxol, clemastine, amiodarone HCl, meropenem, norepinephrine HCl, metoprolol tartrate, prednisone, nitrazepam, esomeprazole, fentanyl citrate, fresubin, digoxin, each given for unspecified indication.

Medical history was not provided.

The patient was hospitalized on 06-Nov-2004. On 15-Dec-2005, a Stevens-Johnson-Syndrome and TEN were diagnosed, symptoms were not provided.

The reporter ticked as outcome of the events "exitus". Stop date was specified with 19-Jan-2005.

Causality assessment for company drugs furosemide (Lasix 40) and enoxaparin sodium (Clexane) was given with "possible".

Laboratory: No values provided.

Addendum on 17-Mar-2008

After internal review at the time of addendum preparation, the association between AE and drug enoxaparin sodium (Clexane) was added.

Sender's comment:

29-Feb-2008 Sanofi Aventis Company Comment:

The temporal relationship between the events and administration of furosemide and enoxaparin seems possible. Dechallenge and rechallenge is not applicable as concomitant medications were altered at the same time. In addition, concomitant drugs as reported here may be considered an alternative cause to explain the events.

17-MAR-2008 Sanofi Aventis Germany: STEVENS-JOHNSON-SYNDROME - FATAL and TOXIC EPIDERMAL NECROLYSIS - FATAL are considered locally unlabelled for one or more of the suspected products. The facts presented here, however, are not very suggestive of a causal relationship (please refer to the intl. evaluator's comment), and thus do not imply a change in the state of the scientific knowledge for the products. Measures according to Para 29 German Drug Law (AMG), section (1) concerning the approval status are not deemed necessary.

Reporter's comment:

APHQWEB, 19.03.2008 DE-Nycomed GmbH-80592

Sender diagnosis: not reported

Narrative

This case report was forwarded to Nycomed (Germany) by Novartis Pharma AG Switzerland (case n°CHNY2008DE01075) and also received directly from the German Health Authority (Ref. DE-BFARM-08017844).

Novartis Pharma AG Switzerland received the case from an intensive monitoring project (German documentation center for severe skin disease) via the German Health authority (Ref.: DE-BFARM-08017844).

An elderly male patient experienced a transition form of Stevens-Johnson syndrome/ toxic epidermal necrolysis (diagnosed on 15-Dec-2004) while receiving pantoprazole (indication, dose regimen and therapy dates unknown). Further multiple suspected drugs see list.

No infections or other risk factors were reported.

The reaction was rated as life-threatening and the patient died.

Neither concomitant medication nor medical history were reported.

TRANSITION FORM OF STEVENS-JOHNSON SYNDROME/TOXIC EPIDERMAL NECROLYSIS:

This patient developed transition form of Stevens-Johnson-Syndrome after intake of of pantoprazole. Concomitantly, the patient received multiple other drugs (approximately further 45 drugs were suspected).

Stevens-Johnson-Syndrome/toxic epidermal necrolysis are rare skin disorders, characterized by mucosal erosions, skin blisters, often accompanied by fever. They can be drug-induced or due to infectious diseases.

Several drugs have been observed to cause Steven-Johnson Syndrome and toxic epidermal necrolysis. Toxic bullous skin reactions up to acute disseminated epidermal necrolysis have also been observed in very rare cases with pantoprazole and other proton pump inhibitors. Although Stevens-Johnson syndrome and toxic epidermal necrolysis are considered labelled for pantoprazole, this event was considered to be unlabeled, since pantoprazole cases with "fatal outcome" are considered as unlabelled. However, the mortality is high in general for patients suffering from Stevens-Johnson syndrome and toxic epidermal necrolysis (20-25%; Mockenhaupt et al 2008).

Temporal relationship not evaluable. Detailed therapy dates for pantoprazole were not reported. Mockenhaupt et al (2008) suggest, that Stevens-Johnson syndrome and toxic epidermal necrolysis often begin more than four and up to 28 days after the initiation of the responsible medication. The patient received multiple other suspected drugs. Although it was stated that the patient did not suffer from any infection, it remains unclear why the patient received antibiotic treatment with vancomycin, ceftriaxon, metronidazol and ciprofloxacin.

Little information was provided, therapy dates for pantoprazole were not provided. However, the reporter stated a quite probable relationship for treatment with ciprofloxacin. Without having medical details of the reported events, Nycomed is not in possession of essential data to assess the events in a responsible way, and therefore gives a causality assessment of "not evaluable" due to insufficient information.

Sender's comment:

The Intensive Monitoring Project assessed the causality with regard to the patient's multiple medication (using an algorithm with 8 different levels ranging from 1=definite to 8=improbable). Pantoprazole was rated as "not applicable". Ciprofloxacin (indication not reported) was rated as "quite probable" according to the algorithm.

This report is not considered to have any influence on current knowledge regarding the safety profile of pantoprazole and has no effect on the status of the drug as reflected by the formal product characteristics.

Reporter's comment:

Wyeth-Pharma GmbH, 26.03.2008 DE-WYE-H03080308

Sender diagnosis: not reported

Narrative

Information was received from a regulatory authority via a business partner (case # 80592) regarding a 59-year-old male patient who received Pantozol (pantoprazole tablet, delayed release) therapy and experienced transition form of Stevens-Johnson syndrome/toxic epidermal necrolysis.

MEDICAL HISTORY:

Relevant medical history was not provided.

PRODUCT DETAILS:

Indication for Pantozol was not provided. Duration of therapy was not provided. Dose regimen was 1 dose 2 times per day (oral).

Additional suspect medication included Tavegil (clemastine), Fenistil (dimetindene maleate), Tavor (lorazepam unsp), Calcium, Vancomycin, Sodium Chloride, Glucose, Furosemide, Fentanyl, Albumin Human, Potassium Chloride, Arterenol (norepinephrine hydrochloride), Aldactone (spironolactone), Beloc Zok (metoprolol succinate), Decortin (prednisone), Paspertin (metoclopramide hydrochloride), Beloc (metoprolol tartrate), Torem (torasemide), Marcumar (phenprocoumon), Decortin-H (prednisolone), Paracodin (dihydrocodeine bitartrate), Bepanthen (dexpantenol), Ciprobay (ciprofloxacin hydrochloride), Lasix (furosemide), Ben-U-Ron (paracetamol), Imodium (loperamide hydrochloride), Clexane (heparin-fraction, sodium salt), Clont (metronidazole), Nexium (esomeprazole), Sodium Bicarbonate, Red Blood Cells, Konakion (phytomenadione), Ringers (sodium chloride compound injection solution), Kalium (potassium chloride), Cordarex (amiodarone unsp), Meronem (meropenem), Midazolam Hydrochloride, Fresubin (carbohydrates nos/fatty acids nos/minerals nos/proteins nos/vitamins nos) and Novodigal (digoxin).

CONCOMITANT THERAPY:

Concomitant medications were not reported.

EVENT DETAILS:

The patient experienced transition form of Stevens-Johnson syndrome/toxic epidermal necrolysis (toxic epidermal necrolysis) diagnosed on 15-Dec-2004, while receiving pantoprazole therapy. No infections or other risk factors were reported. According to the reporter, the patient was hospitalized and the event was considered life threatening. The patient died on an unspecified date. No further information was provided.

The cause of death was reported as toxic epidermal necrolysis.

RELATEDNESS ASSESSMENT:

The reporter assessed toxic epidermal necrolysis as not related to pantoprazole therapy. However, the reporter rated a "quite probable" relationship to the treatment with ciprofloxacin. The business partner assessed the event of toxic epidermal necrolysis as not evaluable due to insufficient information.

Sender's comment:

WYETH COMMENT

INITIAL (v1) 25-Mar-2008

TOXIC EPIDERMAL NECROLYSIS unter Tavor

Es handelt sich um ein nicht in der Fachinformation aufgeführtes unerwünschtes Ereignis.

Ein möglicher Zusammenhang kann nicht ausgeschlossen werden.

Basierend auf der aktuellen Information führt dieser Bericht nicht zu einer Änderung des Nutzen-Risiko-Verhältnisses. Zu diesem Zeitpunkt sind keine weiteren Maßnahmen vorgesehen. Sobald Änderungen hierzu bekannt werden, werden wir darüber unverzüglich und unaufgefordert informieren.

Birte Kruse

Reporter's comment:

NOVARTIS, 21.01.2011 DE-NOVNCHHQ-CHNY2008DE01075

Sender diagnosis: not reported

Narrative

This report combines initial report received from an intensive monitoring project (German documentation centre for severe skin disease dZh, Prof. Schäffler, Freiburg) via the German Health Authority, BfArM (Ref: DE-BFARM-08017844) on 27 Feb 2008 and follow-up information received on 27 Feb 2008 from German Health Authority. A 59-year-old hospitalised patient was treated with Aldactone (spironolactone) 25mg from 07 Nov 2004, Vancomycin injection solution 1g from 14 Nov 2004, Fenistil Gel (dimetindene maleate) topical from 15 Dec 2004, Tavegil oral (clemastine fumarate) from 15 Dec 2004, sodium chloride injection solution 0.9% from 25 Nov 2004, Beloc-Zok forte (metoprolol) from 07 Nov 2004, Torem (torasemid) 10mg from 07 Nov 2004, Macurmar from 25 Nov 2004, Decortin H (prednisolon) 20 mg and then 10 mg, Rocephin (ceftriaxon) injection solution 2g, Paracodin Tropfen (dihydrocodein), Tavor 1mg (lorazepam), Bepanthen Crème (dexpantenol), Ciprobay 500mg (ciprofloxacin), Lasix injection solution 40mg (furosemid), Pantozol (pantoprazol) oral, Tavegil Gel (clemastine fumarate), Benuron (paracetamol) suppositories 1000mg, Immodium (loperamid), Decortin 100mg (prednison), Clexane (enoxaparin natrium) injection solution subcutaneous, glucose injection solution, Clont 400mg (metronidazol), Calcium injection solution (unknown salt), Nexium 40mg (esomeprazol), Nabic (sodium bicarbonate) injection solution 250ml, human albumin injection solution 5%, red blood cell concentrate, Konakion (phytomenadione) injection solution, Ringer solution, potassium chloride injection solution 40mg, Kaliumbrause (kalium unknown salt), Paspertin Tropfen (metoclopramid), Furosemid 40mg, Mucosolvan injection solution (ambroxol), Tavegil injection (clemastine fumarate), Cordarex 200mg (amiodaron), Cordarex injection solution (amiodaron), Meronem injection solution 0.5g (meropenem), Arterenol injection solution 10mg (norepinephrine), Beloc injection solution 50mg (metoprolol), Decortin injection solution 100mg (predison), Dormicum injection solution 90ml (midazolam), Nexium injection solution 40mg (esomeprazol), Fentanyl injection solution 1.5ml (fentanyl), Fresubin solution (carbohydrates NOS, fatty acids NOS, minerals NOS, protein NOS, vitamins), Novodigal 0.1mg (digoxin). The patient was diagnosed with "transition form" of Stevens-Johnson syndrome / Toxic epidermal necrolysis on 15 Dec 2004. For the duration of reaction, the date 19 Jan 2005 was stated. The reaction was rated as life-threatening and the patient died. The reaction was rated as life-threatening and the patient died. The intensive monitoring project assessed the causality with regard to the patient's multiple medication (using an algorithm with 8 different levels, ranging from definite to improbable) as follows: Ciprobay was rated as "well possible". Tavegil oral, Beloc-Zok forte, Marcumar, Lasix injection solution, Decortin, Clexane injection solution, Clont, and Nexium were rated as "possible". Sodium chloride injection solution, Torem 10mg, Decortin H, Tavor, Benuron suppositories, Imodium, Calcium injection solution, Nabic injection solution, human albumin injection solution 5%, red blood cell concentrate, Konakion injection solution, Ringer solution, potassium chloride injection solution, Kaliumbrause, and Paspertin Tropfen were rated as "questionable". Aldactone, Vancomycin injection solution, Fenistil Gel topical, Rocephin injection solution, Paracodin Tropfen, Bepanthen Creme, Pantozol oral, Tavegil Gel, Glucose injection solution, Furosemid, Mucosolvan injection solution, Tavegil injection solution, Cordarex, Cordarex injection solution, Meronem injection solution, Arterenol injection solution, Beloc injection solution, Decortin injection solution, Dormicum injection solution, Nexium injection solution, Fentanyl injection solution, Fresubin solution, and Novodigal were rated as "no causal relationship". It was added that the patient had no infection and no other pre-existing condition as other possible risk factors. Final outcome: death.

Upon internal review done on 19 Jan 2011 of the data received on 27 Feb 2008, the suspected drug Tavor (lorazepam) is not a NCH suspected drug and therefore was encoded with the WHO code.

Sender's comment:

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-08020550

Serious Yes	Death No	Lifethr. No	Hospital. Yes	Disabling No	Congenital No	Other No	Pregnancy	Med. conf. Yes
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Patient

Initials:	Sex: Male	Birth: privacy	Age: 10 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Erythema exudativum multiforme	20041021	20041103	14 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 BENURON paracetamol	Oral	/ Syrup	20041014-20041016	Fever
2 mucosolvan ambroxol hydrochloride	Respiratory (inhalation)	/ Syrup	20041014-20041020	Cough
			3 Day	
			7 Day	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Flu			Unknown	
Common cold			Unknown	

Primary source

Qualification: Consumer or other non health professional observestudytype:

Literature:

Narratives (nur bei elektronischem Eingang)

Boehringer Ingelheim GmbH, Germany, 22.02.2008 DE-Boehringer Ingelheim GmbH, Germany-2008-DE-01049DE

Sender diagnosis: not reported

Narrative

This case was identified and registered by the "Centre of Documentation of severe skin diseases" in Freiburg, Germany and reported to the manufacturers as well as to the Federal Health Office. The "Centre" collects and files nationwide case reports of severe skin diseases.

Therefore, these reports must not be confused with documentations of suspected adverse drug reactions.

It was reported that a 10 year-old male child was treated with Mucosolvan (ambroxol) sirup for cough due to flu from 14Oct2004 until 20Oct2004 and in addition with Benuron (paracetamol) from 14Oct2004 until 16Oct2004 against fever.

An infection was suspected and treated with Zithromax (azithromycin) 500 mg/day.

On 21Oct2004 symptoms of red eyes started, following mouth mucosa erosion, dysphagia, conjunctivitis oedema of face. Erythema exsudativum multiforme major was diagnosed which led to hospitalization on 24Oct2004. On 25Oct2004 in addition symptoms pruritus, cockades, blisters and lip erosions occurred.

The patient recovered.

The event was not marked as life-threatening but possibly related to Mucosolvan.

Sender's comment:

Reporter's comment:

Boehringer Ingelheim GmbH, Germany, 03.03.2008 DE-Boehringer Ingelheim GmbH, Germany-2008-DE-01049DE

Sender diagnosis: not reported

Narrative

This case was identified and registered by the "Centre of Documentation of severe skin diseases" in Freiburg, Germany and reported to the manufacturers as well as to the Federal Health Office. The "Centre" collects and files nationwide case reports of severe skin diseases.

Therefore, these reports must not be confused with documentations of suspected adverse drug reactions.

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On 21Oct2004 symptoms of red eyes started, following mouth mucosa erosion, dysphagia, conjunctivitis oedema of face. Erythema exsudativum multiforme major was diagnosed which led to hospitalization on 24Oct2004. On 25Oct2004 in addition symptoms pruritus, cockades, blisters and lip erosions occurred.

The patient recovered.

The event was not marked as life-threatening but possibly related to Mucosolvan.

Sender's comment:

Regional Centre Drug Safety Boehringer Ingelheim

Comment on case 2008-DE-01049DE / MUCOSOLVAN

Erythema exsudativum multiforme majus (EEMM) is unlisted for Mucosolvan syrup. Other factors have been reported in this case which may have caused the events, such as the other suspect drug paracetamol and the underlying flu-like infection.

As with all unlisted events, the company will continue to monitor all respective reports received and, based on cumulative experience, will re-evaluate the available evidence on an ongoing basis.

No change in the company core safety information or other measures are recommended at this time

PILZDEYN

Case assessment created by PL on 29 Feb 2008

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08024983

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes	Yes	No	No	No	No		

Patient

Initials:	Sex: Male	Birth: privacy	Age: 77 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Toxic epidermal necrolysis	20041118			10 Day	fatal
2 Fever	20041109			1 Day	fatal
3 Pruritus					fatal
4 Rash papular	20041116			8 Day	fatal
5 Nikolsky's sign	20041118			10 Day	fatal
6 Itching	20041116			8 Day	fatal
7 Conjunctivitis	20041122			14 Day	fatal
8 Lip erosion	20041122			14 Day	fatal
9 Oral mucosa erosion	20041122			14 Day	fatal
10 Genital erosion	20041122			14 Day	fatal
11 Blister	20041118			10 Day	fatal

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
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DE-BFARM-08024983

1	BALDRIAN-DISPERT valeriana officinalis root	Oral	10 Gtt drop(s)/Day	20041021-20041021	Restlessness 1440 Minute
2	BAYOTENSIN nitrendipine	Oral	1 Df dosage form/Day	20041023-20041023	Hypertension 1440 Minute
3	CIPROBAY ciprofloxacin hydrochloride	Oral	500 Mg milligram(s)/Day	20041021-20041023	Cystitis 3 Day
4	CIPROBAY ciprofloxacin hydrochloride	Oral	500 Mg milligram(s)/Day	20041017-20041023	Cystitis 7 Day
5	DECORTIN-H prednisolone	Oral	20 Mg milligram(s)/Day	20041113-20041114	2 Day
6	DIAZEPAM diazepam	Intravenous (not otherwis / Injection		20041027-20041027	Restlessness 1440 Minute
7	EUNERPAN melperone hydrochloride	Oral	/	20041028-20041116	Depression 20 Day
8	FENISTIL (NCH) dimetindene maleate		60 Gtt drop(s)/Day Oral drops	-200411	Itching -
9	FENISTIL (NCH) dimetindene maleate	Oral	40 Gtt drop(s)/Day Oral drops	20041109-	Itching -
10	GENTAMICIN (NGX) gentamicin	Intravenous (not otherwis / Solution for injection		20041112-20041118	Sepsis 7 Day
11	HEPARIN SODIUM heparin sodium	Intravenous (not otherwis / Injection		20041115-20041118	Thrombosis prophylaxis 4 Day
12	IMESON nitrazepam	Oral	1 Df dosage form/Day	20041021-20041022	Restlessness 2 Day
13	IMESON nitrazepam	Oral	1 Df dosage form/Day	20041028-20041030	Restlessness 3 Day
14	IMESON nitrazepam	Oral	1 Df dosage form/Day	20041108-20041109	Restlessness 2 Day
15	IMESON nitrazepam	Oral	1 Df dosage form/Day	20041102-20041102	Restlessness 1440 Minute
16	Kaliumchloride, Natriumchlorid potassium chloride	Intravenous (not otherwis / Injection		20041109-20041110	Dialysis 2 Day
17	Kortikosteroid nnb corticosteroid nos	Topical	1 Df dosage form/Day	20041117-20041118	Exanthema 2 Day
18	LASIX furosemide	Oral	375 Mg milligram(s)/Day	20041026-20041115	Renal failure 21 Day
19	lipofundin glycerol; lecithin; soya oil	Intravenous (not otherwis 500 ml millilitre(s)/ Injection		20041118-20071118	Parenteral nutrition 3 Year
20	LOPERAMIDE (NGX) loperamide	Oral	2 Df dosage form/Day	20041114-20041114	Diarrhea 1440 Minute
21	LOPERAMIDE (NGX) loperamide	Oral	2 Df dosage form/Day	20041116-20041118	Diarrhea 3 Day
22	MUCOSOLVAN ambroxol hydrochloride		3 Df dosage form/Day Prolonged-release capsule	20041023-20041110	19 Day
23	multifib.e plasma protein fraction (human)	Intravenous (not otherwis / Injection		20041116-20041118	Supplementation therapy 3 Day
24	NAC 1A PHARMA (NGX) acetylcysteine	Oral	3 Df dosage form/Day	20041023-20041105	14 Day
25	Natriumchlorid sodium chloride	Intravenous (not otherwis / Injection		20041109-20041110	Volume depletion 2 Day
26	Natriumchlorid Injektion sodium chloride	Intravenous (not otherwis / Injection		20041021-20041101	Volume depletion 12 Day
27	Natriumchlorid Injektion sodium chloride	Intravenous (not otherwis / Injection		20041018-20041101	Volume depletion 15 Day
28	NOVALGIN metamizole	Oral	20 Gtt drop(s)/Day	20041023-20041023	Pain 1440 Minute

29	oxygen oxygen		2 l litre(s)/Day Prolonged-release capsule	20041116-20041118 Dyspnea 3 Day
30	PANTOZOL pantoprazole	Oral	/	20041013-20041116 Gastritis prophylaxis 35 Day
31	PANTOZOL pantoprazole	Oral	40 Mg milligram(s)/Day	20041021-20041116 27 Day
32	PARACETAMOL (NGX) paracetamol	Rectal	3 Df dosage form/Day Suppository	20041023-20041023 Pain 1440 Minute
33	PARACETAMOL (NGX) paracetamol	Rectal	1 Df dosage form/Day Suppository	20041115-20041115 Pain 1440 Minute
34	parenteral calcium chloride dihydrate; magnesium chloride hexahydrate; potassium chloride; sodium acetate; sodium chloride	Intravenous (not otherwise)	500 ml millilitre(s)/ Injection	20041118-20041118 Parenteral nutrition 1440 Minute
35	renilon minerals nos; vitamins nos	Oral	3 Df dosage form/Day	20041103-20041116 Depression 14 Day
36	RIFAMPICIN rifampicin	Oral	600 Mg milligram(s)/Day	20041104-20041112 Sepsis 9 Day
37	SOLU-DECORTIN-H prednisolone sodium succinate	Intravenous (not otherwise)	1 Df dosage form/Day Injection	20041117-20041118 Exanthema 2 Day
38	TAVEGIL (NCH) clemastine	Oral	2 Df dosage form/Day	200410-20041105 Itching 16 Day
39	TEMGESIC buprenorphine	Subcutaneous	12 Df dosage form/Day Injection	20041024-20041024 Pain 1440 Minute
40	Tilidine, Naloxone tilidine	Oral	10 Gtt drop(s)/Day	20041025-20041025 Pain 1440 Minute
41	Tilidine, Naloxone tilidine	Oral	/	20041020-20041020 Pain 1440 Minute
42	Tilidine, Naloxone tilidine	Oral	/	20041021-20041021 Pain 1440 Minute
43	TRAMAL tramadol hydrochloride	Oral	20 Gtt drop(s)/Day	20041022-20041022 Pain 1440 Minute
44	TRAMAL tramadol hydrochloride	Oral	20 Gtt drop(s)/Day	20041024-20041024 Pain 1440 Minute
45	TURIXIN mupirocin calcium	Nasal	3 Df dosage form/Day	20041028-20041105 MRSA wound infection 9 Day
46	UREA urea	Topical	2 Df dosage form/Day Ointment	20041107-20041116 Itching 10 Day
47	VANCOMYCIN (NGX) vancomycin	Intravenous (not otherwise)	/ Injection	20041028-20041029 Sepsis 2 Day
48	VANCOMYCIN (NGX) vancomycin	Intravenous (not otherwise)	/ Injection	20041115-20041116 Sepsis 2 Day
49	VANCOMYCIN (NGX) vancomycin	Intravenous (not otherwise)	/ Injection	20041112-20041112 Sepsis 1440 Minute
50	VANCOMYCIN (NGX) vancomycin	Intravenous (not otherwise)	/ Injection	20041101-20041102 Sepsis 2 Day
51	ZIENAM cilastatin sodium; imipenem	Intravenous (not otherwise)	1500 Mg milligram(s)/Day Injection	20041023-20041104 Sepsis 13 Day

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Exanthema			Unknown	
Sepsis			Unknown	
Methicillin-resistant Staphylococcus aureus infection			Unknown	
Essential hypertension			Unknown	
Dyspnoea			Unknown	
Diarrhoea			Unknown	
Pruritus			Unknown	
Pain			Unknown	
Cystitis			Unknown	
Renal failure			Unknown	
Septicaemia			Unknown	
Restlessness			Unknown	
Depression			Unknown	

Primary source

Qualification: Physician observestudytype:
Literature:

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 25.02.2008 DE-002147023-PHBS2008DE02442

Sender diagnosis: not reported

Narrative

Initial report received on 20 Feb 2008 from the German documentation centre of severe skin reactions via Health Authority (DE-BFARM-08014078) and Nycomed (80325). This patient had a relevant history of restlessness, cystitis, pain, pruritus, arterial hypertension, sepsis, renal insufficiency, MRSA (methicillin-resistenter staphylococcus aureus) infection, depression, diarrhoea, dyspnoea and exanthema. The patient was treated with Pantozol (pantoprazole) for gastric protection since 13 Nov 2004 for about 4 weeks. Further 34 suspected drugs were administered. The patient experienced toxic epidermal necrolysis with maculae while receiving Pantozol. About four weeks after start date of pantoprazole the patient experienced fever up to 39.5 degrees Celsius and was hospitalized. In the hospital he experienced pruritus, cockades and skin spots. Further two days later the patient was found to have positive Nikolsky's sign as well as skin blisters. Four days later the patient experienced conjunctivitis, erosion of lips, oral and genital mucosa. On the same day he was transferred into a burns unit. The patient died at an unknown date. The causality was assessed as "possible" by the reporter.

Sender's comment:

Serious spontaneous report, Toxic epidermal necrolysis [death] assessed as unlisted according to the Basic Prescribing Information. It is Novartis' policy to assess a case with a fatal outcome as unlisted unless the label specifically mentions death as a possible outcome of the listed risks. All spontaneous reports are considered suspected for reporting purposes.

Reporter's comment:

NOVARTIS, 25.02.2008 DE-002147023-PHBS2008DE02442

Sender diagnosis: not reported

Narrative

Initial report received on 20 Feb 2008 from the German documentation centre of severe skin reactions via Health Authority (DE-BFARM-08014078) and Nycomed (80325). This patient had a relevant history of restlessness, cystitis, pain, pruritus, arterial hypertension, sepsis, renal insufficiency, MRSA (methicillin-resistenter staphylococcus aureus) infection, depression, diarrhoea, dyspnoea and exanthema. The patient was treated with Pantozol (pantoprazole) for gastric protection since 13 Nov 2004 for about 4 weeks. Further 34 suspected drugs were administered. The patient experienced toxic epidermal necrolysis with maculae while receiving Pantozol. About four weeks after start date of pantoprazole the patient

experienced fever up to 39.5 degrees Celsius and was hospitalized. In the hospital he experienced pruritus, cockades and skin spots. Further two days later the patient was found to have positive Nicholsky's sign as well as skin blisters. Four days later the patient experienced conjunctivitis, erosion of lips, oral and genital mucosa. On the same day he was transferred into a burns unit. The patient died at an unknown date. The causality was assessed as "possible" by the reporter.

Sender's comment:

Serious spontaneous report, Toxic epidermal necrolysis [death] assessed as unlisted according to the Basic Prescribing Information. It is Novartis' policy to assess a case with a fatal outcome as unlisted unless the label specifically mentions death as a possible outcome of the listed risks. All spontaneous reports are considered suspected for reporting purposes.

Reporter's comment:

NOVARTIS, 25.02.2008 DE-002147023-PHBS2008DE02442

Sender diagnosis: not reported

Narrative

Initial report received on 20 Feb 2008 from the German documentation centre of severe skin reactions via Health Authority (DE-BFARM-08014078) and Nycomed (80325). This patient had a relevant history of restlessness, cystitis, pain, pruritus, arterial hypertension, sepsis, renal insufficiency, MRSA (methicillin-resister staphylococcus aureus) infection, depression, diarrhoea, dyspnoea and exanthema. The patient was treated with Pantozol (pantoprazole) for gastric protection since 13 Nov 2004 for about 4 weeks. Further 34 suspected drugs were administered. The patient experienced toxic epidermal necrolysis with maculae while receiving Pantozol. About four weeks after start date of pantoprazole the patient experienced fever up to 39.5 degrees Celsius and was hospitalized. In the hospital he experienced pruritus, cockades and skin spots. Further two days later the patient was found to have positive Nicholsky's sign as well as skin blisters. Four days later the patient experienced conjunctivitis, erosion of lips, oral and genital mucosa. On the same day he was transferred into a burns unit. The patient died at an unknown date. The causality was assessed as "possible" by the reporter.

Sender's comment:

Serious spontaneous report, Toxic epidermal necrolysis [death] assessed as unlisted according to the Basic Prescribing Information. It is Novartis' policy to assess a case with a fatal outcome as unlisted unless the label specifically mentions death as a possible outcome of the listed risks. All spontaneous reports are considered suspected for reporting purposes.

Reporter's comment:

NOVARTIS, 26.02.2008 DE-002147023-PHBS2008DE02442

Sender diagnosis: not reported

Narrative

Initial report received on 20 Feb 2008 from the German documentation centre of severe skin reactions via Health Authority (DE-BFARM-08014078) and Nycomed (80325). This patient had a relevant history of restlessness, cystitis, pain, pruritus, arterial hypertension, sepsis, renal insufficiency, MRSA (methicillin-resister staphylococcus aureus) infection, depression, diarrhoea, dyspnoea and exanthema. The patient was treated with Pantozol (pantoprazole) for gastric protection since 13 Nov 2004 for about 4 weeks. Further 34 suspected drugs were administered. The patient experienced toxic epidermal necrolysis with maculae while receiving Pantozol. About four weeks after start date of pantoprazole the patient experienced fever up to 39.5 degrees Celsius and was hospitalized. In the hospital he experienced pruritus, cockades and skin spots. Further two days later the patient was found to have positive Nicholsky's sign as well as skin blisters. Four days later the patient experienced conjunctivitis, erosion of lips, oral and genital mucosa. On the same day he was transferred into a burns unit. The patient died at an unknown date. The causality was assessed as "possible" by the reporter.

Sender's comment:

Serious spontaneous report, Toxic epidermal necrolysis [death] assessed as unlisted according to the Basic Prescribing Information. It is Novartis' policy to assess a case with a fatal outcome as unlisted unless the label specifically mentions death as a possible outcome of the listed risks. All spontaneous reports are considered suspected for reporting purposes.

Reporter's comment:

NOVARTIS, 27.02.2008 DE-002147023-CHNY2008DE00905

Sender diagnosis: not reported

Narrative

Initial report received from BfArM (BfArM-Nr.: DE-BFARM-0814078 for Fenistil, no BfArM Nr for Tavegil oral) via Intensive Monitoring Project (German documentation centre for severe skin disease) on 14 Feb 2008. A patient with pre-existing sepsis, hospitalised at the Internal Department from 12 Oct 2004 to 22 Nov 2004 and on Burn Injury Department from 22 Nov 2004 to 24 Nov 2004, was treated with Tavegil oral (clemastine), Fensitil drops oral (dimetindene), Baldrian tropfen oral (valeriana root), Ciprobay oral (ciprofloxacin), Tilidin oral (tilidine, naloxone), Imeson oral (nitrazepam), Pantozol oral (pantoprazol), sodium chloride intravenous, Tramal oral (tramadol), Novalgin oral (metamizole), Bayotensin oral (nitrendipine), Zienam intravenous (imipenem, cilastatin), NAC oral (acetylcysteine), Mucosolvan inhaler (ambroxol), paracetamol suppositories, Temgesic subcutaneous (buprenorphine), Lasix oral (furosemide), diazepam intravenous, Turixin NS topical (mupirocin calcium), vancomycin intravenous, Eunerpan oral (melperone), Renilon oral, rifampicin oral, Harnstoffsalbe topical (urea), sodium chloride and potassium chloride intravenous, gentamycin intravenous, Decortin H oral (prednisolone), loperamid oral, heparin intravenous, oxygen gas inhalation, Multifib.e intravenous (unknown composition), Solu-Decortin H intravenous (prednisolone), Corticoid Salbe topical (cortisone), Lipofundin intravenous (parenteral nutrition) and AP alpha intravenous (parenteral nutrition). The patient developed fever up to 39.5°C on 09 Nov 2004. He developed itching, Kokarden and skin spots on 16 Nov 2004. He was diagnosed with positive Nikolsky's sign and developed first blisters and erosions on 18 Nov 2004 which were regarded as onset date of toxic epidermal necrolysis (confirmed case). The patient developed conjunctivitis and erosions of lip, oral mucosa and genital mucosa on 22 Nov 2004. For the duration of reaction, the date 24 Nov 2004 was stated. The reaction was rated as life-threatening and the patient died. In Mar 2005, the Intensive Monitoring Project assessed the causality with regard to the patient's multiple medication (using an algorithm with 8 different levels, ranging from definite to improbable) as follows: Rifampicin oral was rated as 4= well possible. Imeson oral, Pantozol oral, Mucosolvan inhaler, Lasix oral, Vancomycin intravenous, Eunerpan oral, and Fenistil drops oral were rated as 5= possible. Tavegil oral, sodium chloride intravenous, Zienam intravenous, NAC oral, Paracetamol suppositories, Turixin NS topical, Renilon oral, Harnstoffsalbe topical, sodium chloride and potassium chloride intravenous, Gentamycin intravenous, Decortin H oral, Loperamid oral, and Heparin intravenous were rated as 6=questionable. Baldrian tropfen oral, Ciprobay oral, Tilidin oral, Tramal oral, Novalgin oral, Temgesic subcutaneous, Diazepam intravenous, oxygen gas inhalation, Multifib.e intravenous, Solu-Decortin H intravenous, Corticoid Salbe topical, Lipofundin intravenous and AP alpha intravenous were rated as '7=no causal relationship'. It was added that the patient had sepsis as other possible risk factors. Outcome: death.

Sender's comment:

For Fenistil:

This is a serious adverse event (death). The event Toxic epidermal necrolysis and related symptoms was assessed as unlabelled according to the product information. It is Novartis policy to assess a case with a fatal outcome as unlabelled unless the label specifically mentions death as possible outcome of the labelled risks. Investigator causality was suspected.

For Tavegil,

This is a serious adverse event (death). The event Toxic epidermal necrolysis and related symptoms was assessed as unlabelled according to the product information. It is Novartis policy to assess a case with a fatal outcome as unlabelled unless the label specifically mentions death as possible outcome of the labelled risks. Investigator causality was not suspected.

For Pantozol:

Serious adverse event report [death] assessed as unlisted according to the Basic Prescribing Information. It is Novartis' policy to assess a case with a fatal outcome as unlisted unless the label specifically mentions death as a possible outcome of the listed risks. Reporting physician causality is suspected.

For loperamide and vancomycin:

Serious report (death) assessed as unlisted according to the Core Data Sheet Information. It is Sandoz' policy to assess a case with a fatal outcome as unlisted unless the label specifically mentions death as a possible outcome of the listed risks.

For acetylcysteine, gentamycin, paracetamol, and tilidine:

Serious spontaneous report (death) assessed as unlisted according to the Core Data Sheet Information. However other alternative causes (other drugs) provide a possible explanation for the reported adverse event.

All spontaneous reports are considered suspected for reporting purposes.

Reporter's comment:

NOVARTIS, 28.02.2008 DE-SZ09-CHNY2008DE00905

Sender diagnosis: not reported

Narrative

Initial report received from BfArM (BfArM-Nr.: DE-BFARM-0814078 for Fenistil, no BfArM Nr for Tavegil oral) via Intensive Monitoring Project (German documentation centre for severe skin disease) on 14 Feb 2008. A patient with pre-existing sepsis, hospitalised at the Internal Department from 12 Oct 2004 to 22 Nov 2004 and on Burn Injury Department from 22 Nov 2004 to 24 Nov 2004, was treated with Tavegil oral (clemastine), Fensitil drops oral (dimetindene), Baldrian tropfen oral (valeriana root), Ciprobay oral (ciprofloxacin), Tilidin oral (tilidine, naloxone), Imeson oral (nitrazepam), Pantozol oral (pantoprazol), sodium chloride intravenous, Tramal oral (tramadol), Novalgin oral (metamizole), Bayotensin oral (nitrendipine), Zienam intravenous (imipenem, cilastatin), NAC oral (acetylcysteine), Mucosolvan inhaler (ambroxol), paracetamol suppositories, Temgesic subcutaneous (buprenorphine), Lasix oral (furosemide), diazepam intravenous, Turixin NS topical (mupirocin calcium), vancomycin intravenous, Eunerpan oral (melperone), Renilon oral, rifampicin oral, Harnstoffsalbe topical (urea), sodium chloride and potassium chloride intravenous, gentamycin intravenous, Decortin H oral (prednisolone), loperamid oral, heparin intravenous, oxygen gas inhalation, Multifib.e intravenous (unknown composition), Solu-Decortin H intravenous (prednisolone), Corticoid Salbe topical (cortisone), Lipofundin intravenous (parenteral nutrition) and AP alpha intravenous (parenteral nutrition). The patient developed fever up to 39.5°C on 09 Nov 2004. He developed itching, Kokarden and skin spots on 16 Nov 2004. He was diagnosed with positive Nikolsky's sign and developed first blisters and erosions on 18 Nov 2004 which were regarded as onset date of toxic epidermal necrolysis (confirmed case). The patient developed conjunctivitis and erosions of lip, oral mucosa and genital mucosa on 22 Nov 2004. For the duration of reaction, the date 24 Nov 2004 was stated. The reaction was rated as life-threatening and the patient died. In Mar 2005, the Intensive Monitoring Project assessed the causality with regard to the patient's multiple medication (using an algorithm with 8 different levels, ranging from definite to improbable) as follows: Rifampicin oral was rated as 4= well possible. Imeson oral, Pantozol oral, Mucosolvan inhaler, Lasix oral, Vancomycin intravenous, Eunerpan oral, and Fenistil drops oral were rated as 5= possible. Tavegil oral, sodium chloride intravenous, Zienam intravenous, NAC oral, Paracetamol suppositories, Turixin NS topical, Renilon oral, Harnstoffsalbe topical, sodium chloride and potassium chloride intravenous, Gentamycin intravenous, Decortin H oral, Loperamid oral, and Heparin intravenous were rated as 6=questionable. Baldrian tropfen oral, Ciprobay oral, Tilidin oral, Tramal oral, Novalgin oral, Temgesic subcutaneous, Diazepam intravenous, oxygen gas inhalation, Multifib.e intravenous, Solu-Decortin H intravenous, Corticoid Salbe topical, Lipofundin intravenous and AP alpha intravenous were rated as 7=no causal relationship'. It was added that the patient had sepsis as other possible risk factors. Outcome: death.

Sender's comment:

For Fenistil:

This is a serious adverse event (death). The event Toxic epidermal necrolysis and related symptoms was assessed as unlabelled according to the product information. It is Novartis policy to assess a case with a fatal outcome as unlabelled unless the label specifically mentions death as possible outcome of the labelled risks. Investigator causality was suspected.

For Tavegil,

This is a serious adverse event (death). The event Toxic epidermal necrolysis and related symptoms was assessed as unlabelled according to the product information. It is Novartis policy to assess a case with a fatal outcome as unlabelled unless the label specifically mentions death as possible outcome of the labelled risks. Investigator causality was not suspected.

For Pantozol:

Serious adverse event report [death] assessed as unlisted according to the Basic Prescribing Information. It is Novartis' policy to assess a case with a fatal outcome as unlisted unless the label specifically mentions death as a possible outcome of the listed risks. Reporting physician causality is suspected.

For loperamide and vancomycin:

Serious report (death) assessed as unlisted according to the Core Data Sheet Information. It is Sandoz' policy to assess a case with a fatal outcome as unlisted unless the label specifically mentions death as a possible outcome of the listed risks.

For acetylcysteine, gentamycin, paracetamol, and tilidine:

Serious spontaneous report (death) assessed as unlisted according to the Core Data Sheet Information. However other alternative causes (other drugs) provide a possible explanation for the reported adverse event.

All spontaneous reports are considered suspected for reporting purposes.

Reporter's comment:

NOVARTIS, 28.02.2008 DE-NOVNCHHQ-CHNY2008DE00905

Sender diagnosis: not reported

Narrative

Initial report received from BfArM (BfArM-Nr.: DE-BFARM-0814078 for Fenistil, no BfArM Nr for Tavegil oral) via Intensive Monitoring Project (German documentation centre for severe skin disease) on 14 Feb 2008. A patient with pre-existing sepsis, hospitalised at the Internal Department from 12 Oct 2004 to 22 Nov 2004 and on Burn Injury Department from 22 Nov 2004 to 24 Nov 2004, was treated with Tavegil oral (clemastine), Fensitil drops oral (dimetindene), Baldrian tropfen oral (valeriana root), Ciprobay oral (ciprofloxacin), Tilidin oral (tilidine, naloxone), Imeson oral (nitrazepam), Pantozol oral (pantoprazol), sodium chloride intravenous, Tramal oral (tramadol), Novalgin oral (metamizole), Bayotensin oral (nitrendipine), Zienam intravenous (imipenem, cilastatin), NAC oral (acetylcysteine), Mucosolvan inhaler (ambroxol), paracetamol suppositories, Temgesic subcutaneous (buprenorphine), Lasix oral (furosemide), diazepam intravenous, Turixin NS topical (mupirocin calcium), vancomycin intravenous, Eunerpan oral (melperone), Renilon oral, rifampicin oral, Harnstoffsalbe topical (urea), sodium chloride and potassium chloride intravenous, gentamycin intravenous, Decortin H oral (prednisolone), loperamid oral, heparin intravenous, oxygen gas inhalation, Multifib.e intravenous (unknown composition), Solu-Decortin H intravenous (prednisolone), Corticoid Salbe topical (cortisone), Lipofundin intravenous (parenteral nutrition) and AP alpha intravenous (parenteral nutrition). The patient developed fever up to 39.5°C on 09 Nov 2004. He developed itching, Koppen and skin spots on 16 Nov 2004. He was diagnosed with positive Nikolsky's sign and developed first blisters and erosions on 18 Nov 2004 which were regarded as onset date of toxic epidermal necrolysis (confirmed case). The patient developed conjunctivitis and erosions of lip, oral mucosa and genital mucosa on 22 Nov 2004. For the duration of reaction, the date 24 Nov 2004 was stated. The reaction was rated as life-threatening and the patient died. In Mar 2005, the Intensive Monitoring Project assessed the causality with regard to the patient's multiple medication (using an algorithm with 8 different levels, ranging from definite to improbable) as follows: Rifampicin oral was rated as 4= well possible. Imeson oral, Pantozol oral, Mucosolvan inhaler, Lasix oral, Vancomycin intravenous, Eunerpan oral, and Fenistil drops oral were rated as 5= possible. Tavegil oral, sodium chloride intravenous, Zienam intravenous, NAC oral, Paracetamol suppositories, Turixin NS topical, Renilon oral, Harnstoffsalbe topical, sodium chloride and potassium chloride intravenous, Gentamycin intravenous, Decortin H oral, Loperamid oral, and Heparin intravenous were rated as 6=questionable. Baldrian tropfen oral, Ciprobay oral, Tilidin oral, Tramal oral, Novalgin oral, Temgesic subcutaneous, Diazepam intravenous, oxygen gas inhalation, Multifib.e intravenous, Solu-Decortin H intravenous, Corticoid Salbe topical, Lipofundin intravenous and AP alpha intravenous were rated as '7=no causal relationship'. It was added that the patient had sepsis as other possible risk factors. Outcome: death.

Sender's comment:

For Fenistil:

This is a serious adverse event (death). The event Toxic epidermal necrolysis and related symptoms was assessed as unlabelled according to the product information. It is Novartis policy to assess a case with a fatal outcome as unlabelled unless the label specifically mentions death as possible outcome of the labelled risks. Investigator causality was suspected.

For Tavegil,

This is a serious adverse event (death). The event Toxic epidermal necrolysis and related symptoms was assessed as unlabelled according to the product information. It is Novartis policy to assess a case with a fatal outcome as unlabelled unless the label specifically mentions death as possible outcome of the labelled risks. Investigator causality was not suspected.

For Pantozol:

Serious adverse event report [death] assessed as unlisted according to the Basic Prescribing Information. It is Novartis' policy to assess a case with a fatal outcome as unlisted unless the label specifically mentions death as a possible outcome of the listed risks. Reporting physician causality is suspected.

For loperamide and vancomycin:

Serious report (death) assessed as unlisted according to the Core Data Sheet Information. It is Sandoz' policy to assess a case with a fatal outcome as unlisted unless the label specifically mentions death as a possible outcome of the listed risks.

For acetylcysteine, gentamycin, paracetamol, and tilidine:

Serious spontaneous report (death) assessed as unlisted according to the Core Data Sheet Information. However other alternative causes (other drugs) provide a possible explanation for the reported adverse event.

All spontaneous reports are considered suspected for reporting purposes.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08027908

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes	Yes	Yes	No	No	No		Yes

Patient

Initials:	Sex: Female	Birth: privacy	Age: 70 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Skin blotches	20050323			23 Day	fatal
2 Exanthema	20070317			747 Day	fatal
3 Stevens Johnson syndrome	20050329			29 Day	fatal
4 Yersinia infection NOS	20050331			31 Day	fatal
5 Nikolsky's sign	20050404			35 Day	fatal
6 Toxic epidermal necrolysis	20050329			29 Day	fatal
7 Pruritus	20050328			28 Day	fatal
8 Pain of skin	20050329			29 Day	fatal
9 Blisters	20050331			31 Day	fatal

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
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DE-BFARM-08027908				
1	ACTRAPID HUMAN insulin human	Subcutaneous	6 G gram(s)/	20050324-20050329 Diabetes mellitus 6 Day
2	AVALOX moxifloxacin hydrochloride	Intravenous (not otherwis	400 Mg milligram(s)/	20050331-20050331 1440 Minute
3	BENURON paracetamol	Oral	500 Mg milligram(s)/	20050322-20050323 Fever 2 Day
4	BENURON paracetamol	Oral	500 Mg milligram(s)/	20050307-20050307 Fever 1440 Minute
5	BENURON paracetamol	Oral	500 Mg milligram(s)/	20050315-20050315 Fever 1440 Minute
6	BENURON paracetamol	Rectal	1 Df dosage form/	20050319-20050319 Fever 1440 Minute
7	BENURON paracetamol	Rectal	1 Df dosage form/	20050305-20050305 Fever 1440 Minute
8	CALCIUM calcium	Oral	2 Df dosage form/Day	20050324- -
9	CALCIUM calcium		2 Df dosage form/Day	20050308-20050309 2 Day
10	CALCIUM GLUCONATE calcium gluconate	Intravenous (not otherwis	1 Df dosage form/	20050323-20050323 1440 Minute
11	CANDIO-HERMAL nystatin	Topical	1 Df dosage form/	20050326-20050326 Candidiasis 1440 Minute
12	CANDIO-HERMAL nystatin	Topical	/	20050317-20050317 Candidiasis 1440 Minute
13	CORANGIN isosorbide mononitrate		1 Df dosage form/	20050327-20050702 3 Month
14	DECORTIN prednisone	Intravenous (not otherwis	/	20050330- -
15	EUGLUCON glibenclamide	Oral	1.75 Mg milligram(s)/	- -
16	EUSAPRIM sulfamethoxazole; trimethoprim	Oral	2 Df dosage form/Day	20050307-20050314 Infection 8 Day
17	FENISTIL (NCH) dimetindene maleate	Intravenous (not otherwis	/	20050328-20050328 Skin disorder 1440 Minute
18	HAES-STERIL hetastarch; sodium chloride	Intravenous (not otherwis	/	20050304-20050305 2 Day
19	HEPARIN heparin	Subcutaneous	150305 Df dosage form/	- -
20	HEPARIN heparin	Subcutaneous	1 Df dosage form/	20050222-20050307 Thrombosis prophylaxis 14 Day
21	IMODIUM loperamide hydrochloride	Oral	/	20050308-20050308 Diarrhea 1440 Minute
22	IMODIUM loperamide hydrochloride	Oral	2 Df dosage form/	20050315-20050315 Diarrhea 1440 Minute
23	IMODIUM loperamide hydrochloride	Oral	2 Df dosage form/	20050325-20050326 Diarrhea 2 Day
24	INTRAMIN G acetylcysteine; amino acids nos; calcium chloride dihydrate; electrolytes nos; glucose; glucose monohydrate; potassium chloride; sodium glycerophosphate	Intravenous (not otherwis	500 ml millilitre(s)/	20050331-20050331 1440 Minute
25	INTRAMIN G acetylcysteine; amino acids nos; calcium chloride dihydrate; electrolytes nos; glucose; glucose monohydrate; potassium chloride; sodium glycerophosphate	Intravenous (not otherwis	500 ml millilitre(s)/	20050323-20050324 2 Day
26	KONAKION phytomenadione	Oral	5 Gtt drop(s)/	20050309-20050310

27	KONAKION phytomenadione	Oral	10 Gtt drop(s)/	2 Day 20050314-20050316
28	MANINIL glibenclamide	Oral	/	3 Day 20050211-20050303 Diabetes mellitus
29	MARCUMAR phenprocoumon	Oral	/	21 Day 20050305-20050308
30	MULTIBIONTA ascorbic acid; calcium pantothenate; cyanocobalamin; nicotinamide; pyridoxine hydrochloride; retinol palmitate; riboflavin; rutoside; thiamine hydrochloride; tocopheryl acetate	Intravenous (not otherwis 1 Df dosage form/		4 Day 20000322-20050322 5 Year
31	MYOCHOLINE bethanechol	Oral	75 Mg milligram(s)/Day	20050210- -
32	NACL sodium chloride	Intravenous (not otherwis /		20050318-20050324
33	NACL sodium chloride	Intravenous (not otherwis /		7 Day 20050329- -
34	NACL sodium chloride	Intravenous (not otherwis 500 ml millilitre(s)/		20050318- -
35	NOVALGIN metamizole	Oral	80 Gtt drop(s)/Day	20050309-20050314 Pain
36	NOVALGIN metamizole	Intravenous (not otherwis 1 Df dosage form/		6 Day 20050131-20050531 Pain
37	PANTOPRAZOLE SODIUM pantoprazole sodium	Intravenous (not otherwis 80 Mg milligram(s)/Day		4 Month 20050318- -
38	PANTOZOL pantoprazole	Oral	/	Gastritis prophylaxis 20050301- -
39	PASPERTIN metoclopramide hydrochloride	Intravenous (not otherwis 1 Df dosage form/		20050322-20050322 Nausea
40	PASPERTIN metoclopramide hydrochloride	Oral	60 Gtt drop(s)/Day	1440 Minute 20050310-20050318 Nausea
41	RIOPAN magaldrate	Oral	/	9 Day 20070309- -
42	RIOPAN magaldrate		/	Abdominal pain 20050307-20050307
43	ROCEPHIN ceftriaxone sodium	Intravenous (not otherwis 2 G gram(s)/		1440 Minute 20050322-20050325 Infection
44	SAROTEN amitriptyline hydrochloride	Oral	/	4 Day 20050301-20050318 Depression
45	sortis atorvastatin calcium	Oral	1 Df dosage form/	18 Day 20040930-20050303 Hypercholesterolaemia
46	THIOCTACID thioctic acid	Intravenous (not otherwis 600 Mg milligram(s)/		5 Month 20050325-20050327 Polyneuropathy
47	Urbason methylprednisolone	Intravenous (not otherwis /		3 Day 20050322-20050329
48	Urbason methylprednisolone	Intravenous (not otherwis /		8 Day 20050331-20050331
49	VOLTARENE RESINATE diclofenac resinate	Oral	2 Df dosage form/	1440 Minute 20050214-20050309 Pain
50	WATER FOR INJECTION water for injection	Intravenous (not otherwis 100 ml millilitre(s)/		24 Day 20050323-20050323
51	XUSAL levocetirizine dihydrochloride	Oral	/	1440 Minute 20050329- -
52	XYLIT xylitol	Intravenous (not otherwis /		Pruritus 20050323-20050323 1440 Minute

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Collagenosis			Unknown	
Gastric pain			Unknown	
Hypercholesterolaemia			Unknown	
Pain			Unknown	
Depression			Unknown	
Pruritus			Unknown	
Dyspnoea			Unknown	
Polyneuropathy			Unknown	
Yersinia infection			Unknown	
Emesis			Unknown	
Nausea			Unknown	
Hypocalcemia			Unknown	
Diarrhea			Unknown	
Diabetes mellitus			Unknown	
Fever			Unknown	
Thrombosis			Unknown	
Atonic bladder (neurogenic)			Unknown	

Primary source

Qualification: Other Health Professional observestudytype:

Literature:

Qualification: observestudytype:

Literature:

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 27.02.2008 DE-002147023-PHBS2008DE02561

Sender diagnosis: not reported

Narrative

The patient was hospitalized from 10 Feb 2005 until 23 Feb 2005 and from 26 Feb 2005 to 01 Mar 2005 in a department for internal medicine (reason for admission unknown). On 01 Mar 2005 Pantozol therapy was initiated. On 09 Mar 2005 Riopan therapy was started. On 04 Mar 2005 she was transferred into neurological department due to not reported reason. From 07 Mar 2007 to 14 Mar 2007 she received antibiotic treatment with sulfamethoxazol/trimethoprim due to suspected infection and on 22 Mar 2005, antibiotic treatment with ceftriaxone was initiated due to infection of unknown source. The patient repeatedly received paracetamol due to fever. On 17 Mar 2007 she experienced exanthema. Pantozol treatment was discontinued and pantoprazole was started. On 23 Mar 2005 she experienced cockades and skin spots. On 28 Mar 2005 she developed pruritus on 29 Mar 2005 she experienced skin pain. On 31 Mar 2005 she experienced skin blisters. The patient was transferred to an intensive care unit. A Yesinia infection was suspected. On 04 Apr 2005 Nikolsky's sign was positive and she was transferred to a center of burn injury. The patient died on an unknown date. No autopsy reported was provided. The reporter suspected a causal relationship between the event and Pantozol. Diagnosis of overlapping Stevens-Johnson / Toxic Epidermal Necrolysis was made.

Sender's comment:

For Pantozol and Voltaren Resinate:

Serious spontaneous report, T.E.N. [death] assessed as unlisted according to the Basic Prescribing Information. It is Novartis' policy to assess a case with a fatal outcome as unlisted unless the label specifically mentions death as a possible outcome of the listed risks. All spontaneous reports are considered suspected for reporting purposes.

For Corangin:

Serious spontaneous report, T.E.N. [death] assessed as unlisted according to the Basic Prescribing Information. However other alternative causes [other drugs] provide a possible explanation for the reported adverse event. All spontaneous reports are considered suspected for reporting purposes.

For Fenistil:

Serious spontaneous report, T.E.N. [death] assessed as unlisted according to the Basic Prescribing Information.

However other alternative causes [other drugs] provide a possible explanation for the reported adverse event. All spontaneous reports are considered suspected for reporting purposes.

Reporter's comment:

NOVARTIS, 28.02.2008 DE-NOVNCHHQ-PHBS2008DE02561

Sender diagnosis: not reported

Narrative

The patient was hospitalized from 10 Feb 2005 until 23 Feb 2005 and from 26 Feb 2005 to 01 Mar 2005 in a department for internal medicine (reason for admission unknown). On 01 Mar 2005 Pantozol therapy was initiated. On 09 Mar 2005 Riopan therapy was started. On 04 Mar 2005 she was transferred into neurological department due to not reported reason. From 07 Mar 2007 to 14 Mar 2007 she received antibiotic treatment with sulfamethoxazol/trimethoprim due to suspected infection and on 22 Mar 2005, antibiotic treatment with ceftriaxone was initiated due to infection of unknown source. The patient repeatedly received paracetamol due to fever. On 17 Mar 2007 she experienced exanthema. Pantozol treatment was discontinued and pantoprazole was started. On 23 Mar 2005 she experienced cockades and skin spots. On 28 Mar 2005 she developed pruritus on 29 Mar 2005 she experienced skin pain. On 31 Mar 2005 she experienced skin blisters. The patient was transferred to an intensive care unit. A Yesinia infection was suspected. On 04 Apr 2005 Nikolsky's sign was positive and she was transferred to a center of burn injury. The patient died on an unknown date. No autopsy reported was provided. The reporter suspected a causal relationship between the event and Pantozol. Diagnosis of overlapping Stevens-Johnson / Toxic Epidermal Necrolysis was made.

Sender's comment:

For Pantozol and Voltaren Resinate:

Serious spontaneous report, T.E.N. [death] assessed as unlisted according to the Basic Prescribing Information.

It is Novartis' policy to assess a case with a fatal outcome as unlisted unless the label specifically mentions death as a possible outcome of the listed risks. All spontaneous reports are considered suspected for reporting purposes.

For Corangin:

Serious spontaneous report, T.E.N. [death] assessed as unlisted according to the Basic Prescribing Information.

However other alternative causes [other drugs] provide a possible explanation for the reported adverse event. All spontaneous reports are considered suspected for reporting purposes.

For Fenistil:

Serious spontaneous report, T.E.N. [death] assessed as unlisted according to the Basic Prescribing Information.

However other alternative causes [other drugs] provide a possible explanation for the reported adverse event. All spontaneous reports are considered suspected for reporting purposes.

Reporter's comment:

NOVARTIS, 03.03.2008 DE-002147023-PHBS2008DE02561

Sender diagnosis: not reported

Narrative

The patient was hospitalized from 10 Feb 2005 until 23 Feb 2005 and from 26 Feb 2005 to 01 Mar 2005 in a department for internal medicine (reason for admission unknown). On 01 Mar 2005 Pantozol therapy was initiated. On 09 Mar 2005 Riopan therapy was started. On 04 Mar 2005 she was transferred into neurological department due to not reported reason. From 07 Mar 2007 to 14 Mar 2007 she received antibiotic treatment with

sulfamethoxazol/trimethoprim due to suspected infection and on 22 Mar 2005, antibiotic treatment with ceftriaxone was initiated due to infection of unknown source. The patient repeatedly received paracetamol due to fever. On 17 Mar 2007 she experienced exanthema. Pantozol treatment was discontinued and pantoprazole was started. On 23 Mar 2005 she experienced cockades and skin spots. On 28 Mar 2005 she developed pruritus on 29 Mar 2005 she experienced skin pain. On 31 Mar 2005 she experienced skin blisters. The patient was transferred to an intensive care unit. A Yesinia infection was suspected. On 04 Apr 2005 Nikolsky's sign was positive and she was transferred to a center of burn injury. The patient died on an unknown date. No autopsy reported was provided. The reporter suspected a causal relationship between the event and Pantozol. Diagnosis of overlapping Stevens-Johnson / Toxic Epidermal Necrolysis was made.

Significant correction on narrative and suspected drugs was done after internal revue on 22 Feb 2008. Initial report received from Health Authority (08014064) via Nycomed (8331) on 22 Feb 2008. This elderly patient was hospitalized from 10 Feb 2005 until 23 Feb 2005 and from 26 Feb 2005 to 01 Mar 2005 in a department for internal medicine (reason for admission unknown). On 01 Mar 2005 Pantozol (pantoprazole) therapy was initiated and on 14 Mar 2007 Voltaren Resinate (diclofenac resinate) (Other suspected drugs are: pantoprazole sodium, Maninil (glibenclamide), Sortis (atrovastatin), Saroten (amitriptyline), Clexane (heparin) Myocholine (bethanechol), Haes (hyetellose), Marcumar (phenprocoumon), Benuron (paracetamol), Euglucon (glibenclamide), Imodium (loperamide), calcium, Novalgin (metamizole sodium monohydrate), Konakion (phytomenadione), Paspertin (metoclopramide), Candio-Hermal (nystatin), NaCl (sodium chloride), Multibionta (ascorbic acid, calcium pantothenate, cyanocobalamin, nicotinamide, ...), Rocephin (ceftriaxone), Urbason (methylprednisolone), Xylit (xylitol), Ca-Gluconat (calcium gluconate), Aqua ad (water for injection), Intramin (cetylcysteine, amino acids, electrolytes, glucose, sodium ...), Actrapid (insulin), Thioctacid (thioctic acid), Corangin (isosorbide mononitrate), Fenistil (dimetindene), Xusal (levocetirizine dihydrochloride), Decortin (prednisone), Avalox (moxifloxacin). On 09 Mar 2005 Riopan therapy was started. On 04 Mar 2005 she was transferred into neurological department due to not reported reason. From 07 Mar 2007 to 14 Mar 2007 she received antibiotic treatment with sulfamethoxazol/trimethoprim due to suspected infection and on 22 Mar 2005, antibiotic treatment with ceftriaxone was initiated due to infection of unknown source. The patient repeatedly received paracetamol due to fever. On 17 Mar 2007 she experienced exanthema. Pantozol treatment was discontinued and pantoprazole was started. On 23 Mar 2005 she experienced cockades and skin spots. On 28 Mar 2005 she developed pruritus on 29 Mar 2005 she experienced skin pain. On 31 Mar 2005 she experienced skin blisters. The patient was transferred to an intensive care unit. A Yesinia infection was suspected. On 04 Apr 2005 Nikolsky's sign was positive and she was transferred to a center of burn injury. The patient died on an unknown date. No autopsy reported was provided. The reporter suspected a causal relationship between the event and Pantozol. Diagnosis of overlapping Stevens-Johnson / Toxic Epidermal Necrolysis was made.

Sender's comment:

For Pantozol and Voltaren Resinate:

Serious spontaneous report, T.E.N. [death] assessed as unlisted according to the Basic Prescribing Information.

It is Novartis' policy to assess a case with a fatal outcome as unlisted unless the label specifically mentions death as a possible outcome of the listed risks. All spontaneous reports are considered suspected for reporting purposes.

For Corangin:

Serious spontaneous report, T.E.N. [death] assessed as unlisted according to the Basic Prescribing Information.

However other alternative causes [other drugs] provide a possible explanation for the reported adverse event. All spontaneous reports are considered suspected for reporting purposes.

For Fenistil:

Serious spontaneous report, T.E.N. [death] assessed as unlisted according to the Basic Prescribing Information.

However other alternative causes [other drugs] provide a possible explanation for the reported adverse event. All spontaneous reports are considered suspected for reporting purposes.

Reporter's comment:

NOVARTIS, 04.03.2008 DE-SZ09-PHBS2008DE02561

Sender diagnosis: not reported

Narrative

The patient was hospitalized from 10 Feb 2005 until 23 Feb 2005 and from 26 Feb 2005 to 01 Mar 2005 in a department for internal medicine (reason for admission unknown). On 01 Mar 2005 Pantozol therapy was initiated. On 09 Mar 2005 Riopan therapy was started. On 04 Mar 2005 she was transferred into neurological department due to not reported reason. From 07 Mar 2007 to 14 Mar 2007 she received antibiotic treatment with sulfamethoxazol/trimethoprim due to suspected infection and on 22 Mar 2005, antibiotic treatment with ceftriaxone was initiated due to infection of

unknown source. The patient repeatedly received paracetamol due to fever. On 17 Mar 2007 she experienced exanthema. Pantozol treatment was discontinued and pantoprazole was started. On 23 Mar 2005 she experienced cockades and skin spots. On 28 Mar 2005 she developed pruritus on 29 Mar 2005 she experienced skin pain. On 31 Mar 2005 she experienced skin blisters. The patient was transferred to an intensive care unit. A Yesinia infection was suspected. On 04 Apr 2005 Nikolsky's sign was positive and she was transferred to a center of burn injury. The patient died on an unknown date. No autopsy reported was provided. The reporter suspected a causal relationship between the event and Pantozol. Diagnosis of overlapping Stevens-Johnson / Toxic Epidermal Necrolysis was made.

Significant correction on narrative and suspected drugs was done after internal revue on 22 Feb 2008. Initial report received from Health Authority (08014064) via Nycomed (8331) on 22 Feb 2008. This elderly patient was hospitalized from 10 Feb 2005 until 23 Feb 2005 and from 26 Feb 2005 to 01 Mar 2005 in a department for internal medicine (reason for admission unknown). On 01 Mar 2005 Pantozol (pantoprazole) therapy was initiated and on 14 Mar 2007 Voltaren Resinate (diclofenac resinate) (Other suspected drugs are: pantoprazole sodium, Maninil (glibenclamide), Sortis (atrovastatin), Saroten (amitriptyline), Clexane (heparin) Myocholine (bethanechol), Haes (hyetellose), Marcumar (phenprocoumon), Benuron (paracetamol), Euglucon (glibenclamide), Imodium (loperamide), calcium, Novalgin (metamizole sodium monohydrate), Konakion (phytomenadione), Paspertin (metoclopramide), Candio-Hermal (nystatin), NaCl (sodium chloride), Multibionta (ascorbic acid, calcium pantothenate, cyanocobalamin, nicotinamide, ...), Rocephin (ceftriaxone), Urbason (methylprednisolone), Xylit (xylitol), Ca-Gluconat (calcium gluconate), Aqua ad (water for injection), Intramin (cetylcysteine, amino acids, electrolytes, glucose, sodium ...), Actrapid (insulin), Thioctacid (thioctic acid), Corangin (isosorbide mononitrate), Fenistil (dimetindene), Xusal (levocetirizine dihydrochloride), Decortin (prednisone), Avalox (moxifloxacin). On 09 Mar 2005 Riopan therapy was started. On 04 Mar 2005 she was transferred into neurological department due to not reported reason. From 07 Mar 2007 to 14 Mar 2007 she received antibiotic treatment with sulfamethoxazol/trimethoprim due to suspected infection and on 22 Mar 2005, antibiotic treatment with ceftriaxone was initiated due to infection of unknown source. The patient repeatedly received paracetamol due to fever. On 17 Mar 2007 she experienced exanthema. Pantozol treatment was discontinued and pantoprazole was started. On 23 Mar 2005 she experienced cockades and skin spots. On 28 Mar 2005 she developed pruritus on 29 Mar 2005 she experienced skin pain. On 31 Mar 2005 she experienced skin blisters. The patient was transferred to an intensive care unit. A Yesinia infection was suspected. On 04 Apr 2005 Nikolsky's sign was positive and she was transferred to a center of burn injury. The patient died on an unknown date. No autopsy reported was provided. The reporter suspected a causal relationship between the event and Pantozol. Diagnosis of overlapping Stevens-Johnson / Toxic Epidermal Necrolysis was made.

Sender's comment:

For Pantozol and Voltaren Resinate:

Serious spontaneous report, T.E.N. [death] assessed as unlisted according to the Basic Prescribing Information.

It is Novartis' policy to assess a case with a fatal outcome as unlisted unless the label specifically mentions death as a possible outcome of the listed risks. All spontaneous reports are considered suspected for reporting purposes.

For Corangin:

Serious spontaneous report, T.E.N. [death] assessed as unlisted according to the Basic Prescribing Information.

However other alternative causes [other drugs] provide a possible explanation for the reported adverse event. All spontaneous reports are considered suspected for reporting purposes.

For Fenistil:

Serious spontaneous report, T.E.N. [death] assessed as unlisted according to the Basic Prescribing Information.

However other alternative causes [other drugs] provide a possible explanation for the reported adverse event. All spontaneous reports are considered suspected for reporting purposes.

Reporter's comment:

NOVARTIS, 04.03.2008 DE-NOVNCHHQ-PHBS2008DE02561

Sender diagnosis: not reported

Narrative

The patient was hospitalized from 10 Feb 2005 until 23 Feb 2005 and from 26 Feb 2005 to 01 Mar 2005 in a department for internal medicine (reason for admission unknown). On 01 Mar 2005 Pantozol therapy was initiated. On 09 Mar 2005 Riopan therapy was started. On 04 Mar 2005 she was transferred into neurological department due to not reported reason. From 07 Mar 2007 to 14 Mar 2007 she received antibiotic treatment with sulfamethoxazol/trimethoprim due to suspected infection and on 22 Mar 2005, antibiotic treatment with ceftriaxone was initiated due to infection of unknown source. The patient repeatedly received paracetamol due to fever. On 17 Mar 2007 she experienced exanthema. Pantozol treatment was

discontinued and pantoprazole was started. On 23 Mar 2005 she experienced cockades and skin spots. On 28 Mar 2005 she developed pruritus on 29 Mar 2005 she experienced skin pain. On 31 Mar 2005 she experienced skin blisters. The patient was transferred to an intensive care unit. A Yesinia infection was suspected. On 04 Apr 2005 Nikolsky's sign was positive and she was transferred to a center of burn injury. The patient died on an unknown date. No autopsy reported was provided. The reporter suspected a causal relationship between the event and Pantozol. Diagnosis of overlapping Stevens-Johnson / Toxic Epidermal Necrolysis was made.

Significant correction on narrative and suspected drugs was done after internal revue on 22 Feb 2008. Initial report received from Health Authority (08014064) via Nycomed (8331) on 22 Feb 2008. This elderly patient was hospitalized from 10 Feb 2005 until 23 Feb 2005 and from 26 Feb 2005 to 01 Mar 2005 in a department for internal medicine (reason for admission unknown). On 01 Mar 2005 Pantozol (pantoprazole) therapy was initiated and on 14 Mar 2007 Voltaren Resinate (diclofenac resinate) (Other suspected drugs are: pantoprazole sodium, Maninil (glibenclamide), Sortis (atrovastatin), Saroten (amitriptyline), Clexane (heparin) Myocholine (bethanechol), Haes (hyetellose), Marcumar (phenprocoumon), Benuron (paracetamol), Euglucon (glibenclamide), Imodium (loperamide), calcium, Novalgin (metamizole sodium monohydrate), Konakion (phytomenadione), Paspertin (metoclopramide), Candio-Hermal (nystatin), NaCl (sodium chloride), Multibionta (ascorbic acid, calcium pantothenate, cyanocobalamin, nicotinamide, ...), Rocephin (ceftriaxone), Urbason (methylprednisolone), Xylit (xylitol), Ca-Gluconat (calcium gluconate), Aqua ad (water for injection), Intramin (cetylcysteine, amino acids, electrolytes, glucose, sodium ...), Actrapid (insulin), Thioctacid (thioctic acid), Corangin (isosorbide mononitrate), Fenistil (dimetindene), Xusal (levocetirizine dihydrochloride), Decortin (prednisone), Avalox (moxifloxacin). On 09 Mar 2005 Riopan therapy was started. On 04 Mar 2005 she was transferred into neurological department due to not reported reason. From 07 Mar 2007 to 14 Mar 2007 she received antibiotic treatment with sulfamethoxazol/trimethoprim due to suspected infection and on 22 Mar 2005, antibiotic treatment with ceftriaxone was initiated due to infection of unknown source. The patient repeatedly received paracetamol due to fever. On 17 Mar 2007 she experienced exanthema. Pantozol treatment was discontinued and pantoprazole was started. On 23 Mar 2005 she experienced cockades and skin spots. On 28 Mar 2005 she developed pruritus on 29 Mar 2005 she experienced skin pain. On 31 Mar 2005 she experienced skin blisters. The patient was transferred to an intensive care unit. A Yesinia infection was suspected. On 04 Apr 2005 Nikolsky's sign was positive and she was transferred to a center of burn injury. The patient died on an unknown date. No autopsy reported was provided. The reporter suspected a causal relationship between the event and Pantozol. Diagnosis of overlapping Stevens-Johnson / Toxic Epidermal Necrolysis was made.

Sender's comment:

For Pantozol and Voltaren Resinate:

Serious spontaneous report, T.E.N. [death] assessed as unlisted according to the Basic Prescribing Information.

It is Novartis' policy to assess a case with a fatal outcome as unlisted unless the label specifically mentions death as a possible outcome of the listed risks. All spontaneous reports are considered suspected for reporting purposes.

For Corangin:

Serious spontaneous report, T.E.N. [death] assessed as unlisted according to the Basic Prescribing Information.

However other alternative causes [other drugs] provide a possible explanation for the reported adverse event. All spontaneous reports are considered suspected for reporting purposes.

For Fenistil:

Serious spontaneous report, T.E.N. [death] assessed as unlisted according to the Basic Prescribing Information.

However other alternative causes [other drugs] provide a possible explanation for the reported adverse event. All spontaneous reports are considered suspected for reporting purposes.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08021152

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	Yes	No	Yes	No	No		Yes

Patient

Initials:	Sex: Female	Birth: privacy	Age: 00075 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Toxic epidermal necrolysis	20040913				not recovered/not resolved
2 Stevens Johnson syndrome	20040913				not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
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DE-BFARM-08021152					
1	actrapid insulin human	Subcutaneous	/	-	Hyperglycemia
2	atropine atropine	Intravenous (not otherwise)	00000001 Df dosage form	-	Arrhythmia
3	CAPTOPRIL captopril	Oral	/001 Day	-20040913	Hypertension
4	CEFTRIAZONE ceftriaxone sodium	Intravenous (not otherwise)	00000002 G gram(s)/	-	Infection
5	ciprobay ciprofloxacin hydrochloride		1000 Mg milligram(s)/Day	-	Pyrexia
6	CONCOR bisoprolol fumarate	Oral	2 Mg milligram(s)/Day	-	Hypertension
7	delix plus hydrochlorothiazide; ramipril	Oral	2 Mg milligram(s)/Day	-	Hypertension
8	DIAZEPAM diazepam		Df dosage form/	-	Insomnia
9	dormicum midazolam maleate	Intravenous (not otherwise)	Df dosage form/	-	Premedication
10	FURORESE furosemide	Intravenous (not otherwise)	00000040 Mg milligram(s)	-	Pleural effusion
11	LOPIRIN COR captopril	Oral	00000001 Df dosage form	20040819-20040911 24 Day	Hypertension
12	METOHEXAL metoprolol tartrate	Oral	75 Mg milligram(s)/Day	-	Hypertension
13	midazolam midazolam	Intravenous (not otherwise)	00000005 Mg milligram(s)	-	Premedication
14	nitro glyceryl trinitrate	Intravenous (not otherwise)	00000050 Mg milligram(s)	-	Hypertension
15	nitro glyceryl trinitrate	Intravenous (not otherwise)	00000050 Mg milligram(s)	-	Hypertension
16	novalgin metamizole	Oral	00000020 Df dosage form	-	Pain
17	OMEPRAZOL omeprazole	Oral	/	-	Gastric ulcer prophylaxis
18	OXAZEPAM oxazepam	Oral	00000010 Mg milligram(s)	-	Sedation
19	paracetamol paracetamol		00000015 ml millilitre(s)/	-	Pain
20	PERENTEROL saccharomyces boulardii		00000002 Df dosage form	-	Diarrhoea
21	PROPOFOL propofol	Intravenous (not otherwise)	00000030 Mg milligram(s)	-	Arrhythmia
22	refobacin gentamicin sulfate	Intravenous (not otherwise)	00000080 Mg milligram(s)	-	Endocarditis
23	ROCEPHIN ceftriaxone sodium	Intravenous (not otherwise)	00000002 G gram(s)/	-	Infection
24	vancomycin vancomycin	Intravenous (not otherwise)	00000001 G gram(s)/	-	Endocarditis

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Acute gastroenteritis	20040816		Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:		
Qualification:	Other Health Professional	observestudytype:

Literature:

Qualification: Consumer or other non health professional observestudytype:

Literature:

Narratives (nur bei elektronischem Eingang)Bristol-Myers Squibb Company, 27.02.2008 DE-Bristol-Myers Squibb Company-14082812

Sender diagnosis: not reported

Narrative

A physician reported via German health authority (local file number BFARM E08008860) and BMS Germany (local file number 2008021902) that a 75-year-old female patient experienced a life threatening and persistently disabling Stevens-Johnson-Syndrome/toxic epidermal necrolysis - transition form, while under therapy with oral captopril tablet between 19-Aug-2004 and 11-Sep-2004 for hypertension, oral paracetamol 15ml for pain, oral saccharomyces boulardii for diarrhea, oral diazepam as sleeping medication, oral oxazepam 10mg for sedation, intravenous midazolam as premedication, oral captopril for hypertension, oral ciprofloxacin 500 mg for pyrexia and oral bisoprolol fumarate for hypertension, oral ramipril + hydrochlorothiazide 2.5mg for hypertension, oral omeprazole for gastric protection, metamizole drops for pain, gentamicin 80mg for suspected endocarditis, intravenous vancomycin 1 gram for suspected endocarditis, intravenous midazolam 5 mg as premedication, intravenous propofol 1 x 30mg for arrhythmia, atropine intravenously one ampule for arrhythmia, subcutaneous human insulin for hyperglycemia, intravenous furosemide 1 x 40mg for pleural effusion, isosorbide dinitrate 50mg for hypertension and ceftriaxone 2 gram for suspected infection, oral metoprolol 25 mg for hypertension. The acute phase lasted from 13-Sep-2004 until 18-Oct-2004 and follow-up after one year revealed that the event had not yet resolved. The reporter considered the event as possibly related to oxazepam, captopril (Loprin), captopril, ciprofloxacin, ramipril + hydrochlorothiazide, omeprazole, metamizole, vancomycin, human insulin, isosorbide dinitrate, ceftriaxone (Rocephin), and ceftriaxone. The events were questionably related to diazepam, midazolam (Dormicum), midazolam, bisoprolol fumarate, gentamicin, propofol, atropine, furosemide, and metoprolol. The reporter's causality was not assessed for paracetamol and saccharomyces boulardii.

BMS Medical Evaluation Comment:

This 75-year-old female experienced Stevens-Johnson-Syndrome/Toxic Epidermal Necrolysis while on multiple drugs including captopril and paracetamol. The event of Stevens-Johnson-Syndrome was known to be associated with multiple drugs and a causal role to any one drug cannot be ascertained.

Not assessed for paracetamol and perenterol.

Sender's comment:

This report is submitted to you in the name of the marketing authorisation holder Bristol-Myers Squibb GmbH & Co. KGaA.

captopril

Event number; label*; causality**

#1; expected; PSB

#2; unexpected; PSB

* according to current Summary of Product Characteristics

** DEF=definite, PRB=probable, PSB=possible, NOL=not likely, NOR=not related

Change of current scientific knowledge: no

Comments: Stevens-Johnson-Syndrome is expected according to the current SPC of LOPIRIN COR but TEN is not explicitly mentioned.

Reporter's comment:Bristol-Myers Squibb Company, 07.03.2008 DE-Bristol-Myers Squibb Company-14082812

Sender diagnosis: not reported

Narrative

A physician reported via German health authority (local file number BFARM E08008860) and BMS Germany (local file number 2008021902) that a 75

year-old female patient experienced a life threatening and persistently disabling Stevens-Johnson-Syndrome/toxic epidermal necrolysis - transition form, while under therapy with oral captopril tablet between 19-Aug-2004 and 11-Sep-2004 for hypertension, oral paracetamol 15ml for pain, oral saccharomyces boulardii for diarrhea, oral diazepam as sleeping medication, oral oxazepam 10mg for sedation, intravenous midazolam as premedication, oral captopril for hypertension, oral ciprofloxacin 500 mg for pyrexia and oral bisoprolol fumarate for hypertension, oral ramipril + hydrochlorothiazide 2.5mg for hypertension, oral omeprazole for gastric protection, metamizole drops for pain, gentamicin 80mg for suspected endocarditis, intravenous vancomycin 1 gram for suspected endocarditis, intravenous midazolam 5 mg as premedication, intravenous propofol 1 x 30mg for arrhythmia, atropine intravenously one ampule for arrhythmia, subcutaneous human insulin for hyperglycemia, intravenous furosemide 1 x 40mg for pleural effusion, isosorbide dinitrate 50mg for hypertension and ceftriaxone 2 gram for suspected infection, oral metoprolol 25 mg for hypertension. The acute phase lasted from 13-Sep-2004 until 18-Oct-2004 and follow-up after one year revealed that the event had not yet resolved. The reporter considered the event as possibly related to oxazepam, captopril(Loprin), captopril, ciprofloxacin, ramipril + hydrochlorothiazide, omeprazole, metamizole, vancomycin, human insulin, isosorbide dinitrate, ceftriaxone (Rocephin), and ceftriaxone. The events were questionably related to diazepam, midazolam (Dormicum), midazolam, bisoprolol fumarate, gentamicin, propofol, atropine, furosemide, and metoprolol. The reporter's causality was not assessed for paracetamol and saccharomyces boulardii.

BMS Medical Evaluation Comment:

This 75-year-old female experienced Stevens-Johnson-Syndrome\Toxic Epidermal Necrolysis while on multiple drugs including captopril and paracetamol. The event of Stevens-Johnson-Syndrome was known to be associated with multiple drugs and a causal role to any one drug cannot be ascertained.

Not assessed for paracetamol and perenterol.

Supplemental information received on 03-Mar-2008.

The patient was admitted to the hospital on 16-Aug-2004 due to acute gastroenteritis. The patient had a fever (39°C) on 19-Aug-2004 and on 08-Sep-2004, fever was up to 39.4°C. On 11-Sep-2004, pain of skin occurred and first appearance of blisters of skin as well as spots occurred on 13-Sep-2004 and confluent blisters occurred on 16-Sep-2004. On 17-Sep-2004, Nikolsky sign was positive, erosions of genital mucosa and perianal and back erosions occurred. The drug captopril was given until 13-Sep-2004 (generic) and only Lopirin (captopril) was last administered on 11-Sep-2004.

The reporter causality for furosemide was amended from questionable to possibly related.

BMS Medical Evaluation Comment:

This female patient experienced Stevens-Johnson-Syndrome\Toxic Epidermal Necrolysis-transition form while on multiple drugs. The event of Stevens-Johnson-Syndrome is known to be associated with multiple drugs and a causal role to any one drug cannot be ascertained.

Sender's comment:

This report is submitted to you in the name of the marketing authorisation holder Bristol-Myers Squibb GmbH & Co. KGaA.

captopril

Event number; label*; causality**

#1; expected; PSB

#2; unexpected; PSB

* according to current Summary of Product Characteristics

** DEF=definite, PRB=probable, PSB=possible, NOL=not likely, NOR=not related

Change of current scientific knowledge: no

Comments: Stevens-Johnson-Syndrome is expected according to the current SPC of LOPIRIN COR but TEN is not explicitly mentioned.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08030335

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	Yes		

Patient

Initials:	Sex: Female	Birth: privacy	Age: 33 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	20041014			34 Day	not recovered/not resolved
2 Pulmonary congestion	20040911			1 Day	not recovered/not resolved
3 Sepsis					not recovered/not resolved
4 Staphylococcus aureus septicemia					not recovered/not resolved
5 Surgical vascular shunt	20041012			32 Day	not recovered/not resolved
6 Dialysis	20041012			32 Day	Unknown
7 Blisters	20041015			35 Day	not recovered/not resolved
8 Lip erosion	20041019			39 Day	not recovered/not resolved
9 Oral mucosal disorder	20041019			39 Day	not recovered/not resolved
10 Conjunctivitis	20041020			40 Day	not recovered/not resolved
11 Skin rash	20041014			34 Day	not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
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1	aminosteril amino acids nos; vitamins nos	Intravenous (not otherwis / Injection	20040911- -	Supplementation therapy
2	ampho-moronal amphotericin b	Oral 6 ml millilitre(s)/Day	20040911-20040917 7 Day	Candidiasis
3	ARTERENOL norepinephrine hydrochloride	Intravenous (not otherwis / Injection	20040914-20040925 12 Day	Intubation
4	ATOSIL isopromethazine hydrochloride	Oral /	20040926-20041003 8 Day	Sedation
5	CEFTRIAZONE ceftriaxone	Intravenous (not otherwis / Injection	20040911-20040926 16 Day	Septicaemia
6	cernevit	Intravenous (not otherwis / Injection	20040914-20041002 19 Day	Vitamin deficiency
7	clexane enoxaparin	Subcutaneous /	20040911- -	Thrombosis prophylaxis
8	clont metronidazole	Intravenous (not otherwis / Injection	20040924-20040930 7 Day	Sepsis
9	depo-clinovir medroxyprogesterone acetate	Intramuscular 1 Df dosage form/Day Injection	20040601- -	Contraception
10	DISOPRIVAN propofol	Intravenous (not otherwis / Injection	20040913-20040918 6 Day	Sedation
11	DORMICUM midazolam hydrochloride	Intravenous (not otherwis 1 Df dosage form/ Injection	20040917-20040923 7 Day	Intubation
12	ENDOXAN cyclophosphamide	Oral /	20040910-20040926 17 Day	Wegener's granulomatosis
13	ENDOXAN cyclophosphamide	Oral /	20040928-20041002 5 Day	Wegener's granulomatosis
14	EUTHYROX levothyroxine sodium	Oral /	20040918- -	Hypothyroidism
15	FENTANYL fentanyl	Intravenous (not otherwis / Injection	20040917-20040923 7 Day	Respiratory failure
16	FLUCLOXACILLIN flucloxacillin	Intravenous (not otherwis / Injection	20040915-20040927 13 Day	Sepsis
17	FUROSEMIDE furosemide	Intravenous (not otherwis / Injection	20040921-20040922 2 Day	Oedema
18	glucose glucose	Intravenous (not otherwis / Injection	20040902-20040930 29 Day	
19	insuman rapid insulin human; isophane insulin	Subcutaneous /	20040918-20040920 3 Day	Hyperglycaemia
20	insuman rapid insulin human; isophane insulin	Subcutaneous /	20040923-20040928 6 Day	Hyperglycaemia
21	kalinor potassium chloride	Oral /	20040928-20041013 16 Day	Hypokalaemia
22	KONAKION phytomenadione	Intravenous (not otherwis / Injection	20040919-20040928 10 Day	Coagulation defects, other and unspecified
23	LASIX SPECIAL furosemide	Intravenous (not otherwis 192 ml millilitre(s)/Day Injection	20040917-20040917 1440 Minute	Oedema
24	Morphin morphine	Intravenous (not otherwis / Injection	20040923-20040929 7 Day	Pain
25	ORFIRIL valproic acid	Oral 20 ml millilitre(s)/Day	20040923- -	Epilepsy
26	ORFIRIL valproic acid	Oral 1200 Mg milligram(s)/Day	20040912- -	Epilepsy
27	PANTOZOL pantoprazole	Oral /	20040927- -	Prophylaxis against gastrointestinal ulcer
28	PANTOZOL pantoprazole	Intravenous (not otherwis /	20040911-20040926 16 Day	Prophylaxis against gastrointestinal ulcer

29	PARACETAMOL paracetamol	Rectal	1000 Mg milligram(s)/Day	20041001-20041001	Fever 1440 Minute
30	PASPERTIN metoclopramide hydrochloride	Intravenous (not otherwise specified)	3 Df dosage form/Day Injection	20040914-20040919	Nausea 6 Day
31	prednisolon prednisolone	Intravenous (not otherwise specified)	/ Injection	20040908-20041003	Wegeners granulomatosis 26 Day
32	tavor lorazepam	Intravenous (not otherwise specified)	/ Injection	20040923-	Sedation -
33	tracutil minerals nos	Intravenous (not otherwise specified)	/ Injection	20040914-20040926	Supplementation therapy 13 Day

Drugs (concomitant)

34	uromitexan mesna	Oral	/	20040910-20041011	Wegener's granulomatosis 32 Day
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Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Coagulation defects, other and unspecified			Unknown	
Vitamin deficiency			Unknown	
Oedema			Unknown	
Hypokalaemia			Unknown	
Wegener's granulomatosis			Unknown	
Staphylococcus aureus septicemia			Unknown	
Appetite lost			Unknown	On 01 Sep 2004, the patient had been admitted to hospital due to loss of appetite.
Epigastric discomfort			Unknown	On 01 Sep 2004, the patient had been admitted to hospital due to epigastric complaints.
Septicaemia			Unknown	
Fever			Unknown	
Pain			Unknown	
Nausea			Unknown	
Vomiting			Unknown	
Hypothyroidism			Unknown	
Candidiasis			Unknown	
Epilepsy			Unknown	
Respiratory failure			Unknown	
Hyperglycaemia			Unknown	

Primary source

Qualification: Physician observestudytype:
Literature:

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 05.03.2008 DE-002147023-PHBS2008DE02601

Sender diagnosis: not reported

Narrative

Initial report received from the German documentation centre of severe skin reactions via Health Authority (DE-BFARM-08014087) and Nycomed (80334). This patient had a relevant history of soor, oedema, nausea, pulmonary insufficiency, sepsis, Wegener's granulomatosis, hypovitaminosis, epilepsy, hyperglycaemia, hypothyroidism, coagulation disorder, pain, hypokalaemia, fever and sepsis due to staphylococcus aureus infection. This patient was treated with Pantozol (pantoprazole) i.v. since 11 Sep 2004 and Pantozol (pantoprazole) 40 mg p.o since 27 Sep 2004 for ulcer prophylaxis. Further 30 suspected drugs were administered (Ampho-Moronal (amphotericin B), Lasix special (furosemide), Disoprivan (propofol), Paspertin (metoclopramide hydrochloride), Dormicum (midazolam hydrochloride), Fentanyl (fentanyl), Arterenol (norepinephrine hydrochloride), Ceftriaxone (ceftriaxone), Tracutil (chromic chloride, cupric chloride dihydrate, ferrous chloride), Flucloxacillin (flucloxacillin), Glucose (glucose), Endoxan (cyclophosphamide, Cernevit (vitamins nos), Prednisolon (prednisolone), Orfiril (valproic acid), Orfiril (valproic acid, Clexane (enoxaparin sodium, heparin-fraction, sodium salt), Depo-clinovir (medroxyprogesterone acetate), Aminosteril (amino acids nos, vitamins nos), Insuman rapid (insulin human), Euthyrox (levothyroxine sodium), Konakion (phytomenadione), Furosemide (furosemide), Morphine (morphine), Tavor (lorazepam), Clont (metronidazole), Atosil (isopromethazine hydrochloride), Kalinor (potassium bicarbonate, potassium citrate, Paracetamol (paracetamol)). The patient experienced Stevens-Johnson-syndrome while receiving Pantozol. On 01 Sep 2004, the patient had been admitted to hospital due to loss of

appetite and epigastric complaints of unknown origin. From 11 Sep until 27 Sep 2004, the patient was in an intensive care unit due to pulmonary congestion, where she received Pantozol i.v. On 27 Sep 2004, the patient was discharged from the intensive care unit and Pantozol i.v. was switched to Pantozol 40 mg p.o. At the intensive care unit, the patient received antibiotic treatment with ceftriaxon and flucloxacilline for sepsis. Starting on 24 Sep 2004, the patient received treatment with metronidazole for staphylococcus aureus sepsis. On 12 Oct 2004, the patient had a shunt implantation due to requirement of dialysis. On 14 Oct 2004, patient developed cockades and skin spots. The patient developed skin blisters on 15 Oct 2004. On 19 Oct 2004, the patient experienced erosions of lips and oral mucous membranes. On 20 Oct 2004, the patient developed conjunctivitis. The outcome was reported as still ongoing at the time of the report. The reporter assessed the Steven Johnson syndrome possible related to Pantozol i.v. and Pantozol p.o..

Sender's comment:

The available information was considered inadequate to fully assess the case.

Serious spontaneous report

Stevens-Johnson syndrome [medically significant], already listed in the Core Data Sheet.

Sepsis, Dialysis [medically significant], Pulmonary congestion [hospitalization], assessed as unlisted according to the Core Data Sheet. However other alternative causes [other drugs, underlying conditions] provide a possible explanation for the reported adverse event.

All spontaneous reports are considered suspected for reporting purposes.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08042815

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		

Patient

Initials:	Sex: Female	Birth: privacy	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens-Johnson syndrome					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 clexane enoxaparin	Subcutaneous	/	-	-
2 COVERSUM perindopril erbumine	Unknown	/	-	-
3 EUTHYROX levothyroxine sodium	Unknown	/	-	-
4 LOPIRIN captopril	Unknown	/	-	-
5 MOVICOL polyethylene glycol 3350; potassium chloride; sodium bicarbonate; sodium chloride	Unknown	/	-	-
6 Natriumchlorid sodium chloride	Intravenous (not otherwis	/	-	-
7 Natriumchlorid sodium chloride	Oral	/	-	-
8 norvasc amlodipine besilate	Oral	/	-	-
9 Paracetamol paracetamol	Oral	/	-	-
10 PHENHYDAN phenytoin	Oral	/	-	-
11 REMERGIL mirtazapine	Unknown	/	-	-
12 THROMBOPHOB heparin sodium	Topical	/	-	-
13 VIOXX rofecoxib	Oral	/ Tablet	-	-
14 XIMOVAN zopiclone	Unknown	/	-	-

Primary source

Qualification: Physician

observestudytype:

Literature:

Narratives (nur bei elektronischem Eingang)

009507513, 16.04.2008 DE-MERCK-0804DEU00041

Sender diagnosis: not reported

Narrative

This report was identified from a line listing obtained on request by the Company from the FDA under the Freedom of Information Act. A female patient treated with rofecoxib, amlodipine besylate, sodium chloride, levothyroxine sodium, enoxaparin sodium, phenytoin, captopril, mirtazapine, heparin sodium, acetaminophen, polyethylene glycol 3350/KCl/NaHCO3/NaCl, zopiclone and perindopril erbumine experienced Stevens-johnson syndrome.

The listing indicated that the event required hospitalization. Action taken and outcome were not provided.

Amlodipine was listed as primary suspect therapy, all other medications were considered secondary suspect. No further information is available.

This was reported through Pfizer (Mfr. number 2008011464). The original reporting source was not provided.

Sender's comment:

Stevens-Johnson syndrome is a serious labeled event for therapy with VIOXX°. This report was identified from a line listing obtained on request by the company from the FDA under the Freedom of Information Act. This patient received numerous medications and developed a Stevens-Johnson syndrome. The primary suspect drug was amlodipine. VIOXX° was considered among others a secondary suspect. The available information is scant and a causality evaluation is not possible. VIOXX° has been withdrawn from the market worldwide on 30th September 2004. Self-initiated measures are not indicated.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08121710

Serious No	Death	Lifethr. No	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Male	Birth: privacy	Age: 35 Year	weight (kg): 87	BMI_int: 24.88	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Reaction to drug excipients	20080924		3 Hour	1 Day	recovered/resolved
2 Pruritus	20080924		3 Hour	1 Day	recovered/resolved
3 Exanthematic drug eruption	20080924		3 Hour	1 Day	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Paracetamol 500 Hexal paracetamol	Oral	1000 Mg milligram(s)/Day	20080924- -	Lumbago

Drugs (concomitant)

2 mydocalm tolperisone hydrochloride	Oral	100 Mg milligram(s)/Day	20080924- -	Lumbago
3 solu-decortin-h prednisolone sodium succinate	Intravenous (not otherwis	50 Mg milligram(s)/Day	20080924- -	Exanthem
4 tramal tropfen tramadol hydrochloride	Oral	40 Gtt drop(s)/Day	20080924- -	Lumbago
5 xusal levocetirizine dihydrochloride	Oral	50 Mg milligram(s)/Day	20080924- -	Pruritus

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08146362

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	Yes	No	No	No	No		Yes

Patient

Initials:	Sex: Male	Birth: privacy	Age: 34 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens-Johnson syndrome	20070610	20070629	20 Day		recovered/resolved
2 Multi-organ failure	20070523				Unknown
3 Toxic epidermal necrolysis	20070610	20070629	20 Day	18 Day	recovered/resolved
4 Stevens-Johnson syndrome	20070610	20070629	20 Day	18 Day	recovered/resolved
5 Fever	20070521				Unknown
6 Nausea	20070523				Unknown
7 Redness	20070523				Unknown
8 Swelling of feet	20070523				Unknown
9 Restlessness	20070531				Unknown
10 Respiratory insufficiency	20070521				Unknown
11 Pain	20070523				Unknown
12 Hypertension arterial	20070525				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
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1	albuterol salbutamol		3* /Day 113	20070523- -	Dyspnoea
2	broncho inhalat salbutamol sulfate	Respiratory (inhalation)	3 Df dosage form/Day	20070523- -	Dyspnoea
3	CATAPRESAN clonidine	Intravenous (not otherwis /		20070523-20070604 13 Day	Withdrawal syndrome
4	CATAPRESAN clonidine	Intravenous drip	/	20070606-20070610 5 Day	
5	CIPROBAY ciprofloxacin hydrochloride	Intravenous drip	400 Mg milligram(s)/Day	20070517-20070522 6 Day	Pancreatitis
6	clexane enoxaparin	Subcutaneous	40 Mg milligram(s)/Day 120	20070603- -	Thrombosis prophylaxis
7	fluconazole fluconazole	Intravenous drip	800 Mg milligram(s)/Day	20070523-20070603 12 Day	Pancreatitis
8	HALDOL haloperidol	Intravenous drip	/	20070526-20070605 11 Day	Alcohol withdrawal syndrome
9	HEPARIN SODIUM heparin sodium	Subcutaneous	/	20070514-20070515 2 Day	Thrombosis prophylaxis
10	HEPARIN SODIUM heparin	Intravenous drip	/	20070515-20070602 19 Day	Thrombosis prophylaxis
11	HYDROCORTISONE hydrocortisone	Intravenous drip	100 Mg milligram(s)/	20070523-20070602 11 Day	Cardiovascular disorder
12	INSULIN HUMAN insulin	Parenteral	/	20070514- -	Hyperglycaemia
13	meropenem meropenem	Intravenous drip	3 G gram(s)/Day	20070604- -	Pancreatitis
14	METOCLOPRAMIDE metoclopramide	Oral	30 Df dosage form/Day	20070606-20070609 4 Day	Nausea
15	METOCLOPRAMIDE metoclopramide	Intravenous drip	3* /Day	20070523-20070605 14 Day	Nausea
16	NEPRESOL dihydralazine sulfate	Intravenous drip	/	20070529-20070531 3 Day	Hypertension
17	NOVALGIN metamizole	Intravenous drip	/	20070523-20070524 2 Day	Pain
18	NOVALGIN metamizole	Intravenous (not otherwis /		20070513-20070514 2 Day	Pain
19	NOVALGIN metamizole	Intravenous drip	/	20070530-20070530 5 Day	Pain
20	PANTOZOL I.V.	Intravenous drip	80 Mg milligram(s)/Day	20070523-20070605 14 Day	Gastric ulcer prophylaxis
21	PANTOZOL 40 MG	Oral	40 Mg milligram(s)/Day	20070606- -	Gastric ulcer prophylaxis
22	PERFALGAN paracetamol	Intravenous drip	1 G gram(s)/	20070529-20070529 4 Day	Fever
23	PERFALGAN paracetamol	Intravenous drip	1 G gram(s)/	20070603-20070603 4 Day	Fever
24	PERFALGAN paracetamol	Intravenous drip	1 G gram(s)/Day	20070526-20070527 2 Day	Pyrexia
25	Radiokontrastmittel	Intravenous (not otherwis /		20070531-20070531 1 Day	
26	Radiokontrastmittel	Intravenous (not otherwis /		20070523-20070523 1 Day	
27	Radiokontrastmittel	Intravenous drip	/	20070513-20070522 10 Day	
28	RIVOTRIL clonazepam	Intravenous drip	1 Df dosage form/	20070531-20070531 1 Day	Restlessness

29	ROCEPHIN ceftriaxone sodium	Intravenous drip	2 G gram(s)/Day	20070513-20070516 Pancreatitis 4 Day
30	SUFENTANIL sufentanil	Intravenous drip	/	20070523-20070602 Sedative therapy 11 Day
31	SUFENTANIL sufentanil	Intravenous drip	/	20070521-20070521 Sedative therapy 14 Day
32	SUFENTANIL sufentanil	Intravenous drip	/	20070515-20070516 Sedation 2 Day
33	TAVOR lorazepam	Intravenous drip	/	20070603- Alcohol withdrawal syndrome -
34	TAZOBAC piperacillin sodium; tazobactam sodium	Intravenous drip	13.5 G gram(s)/Day 120	20070530-20070603 Pancreatitis 5 Day
35	vancomycin vancomycin	Intravenous drip	/	20070523-20070603 Pancreatitis 12 Day
36	ZIENAM cilastatin sodium; imipenem	Intravenous drip	/	20070517-20070531 Pancreatitis 15 Day
37	zithromax azithromycin	Intravenous drip	500 Mg milligram(s)/Day	20070530-20070601 Pancreatitis 3 Day

Drugs (concomitant)

38	ascorbic acid ascorbic acid	1 Df dosage form/Day	20060601-	-
39	atosil promethazine hydrochloride	Intravenous (not otherwis 1 Df dosage form/Day	20070609-20070610	Alcohol withdrawal syndrome
40	calcium carbonate calcium carbonate	1 Df dosage form/Day	20060601-	2 Day
41	calcium folinate calcium folinate	Intravenous (not otherwis 1 Df dosage form/Day	20070523-	-
42	cernevit	Intravenous drip /	20070523-	-
43	clinomel amino acids nos; calcium chloride dihydrate; electrolytes nos; glucose; lipids nos	Intravenous (not otherwis /	20070603-	-
44	DORMICUM midazolam	Intravenous drip /	20070516-20070529	14 Day
45	electrolytes	Intravenous (not otherwis /	20070602-20070602	Electrolyte substitution therapy
46	electrolytes	Intravenous drip /	20070513-20070513	Electrolyte substitution therapy
47	electrolytes	Intravenous (not otherwis /	20070523-20070530	Electrolyte substitution therapy
48	Folsaeure folic acid	Intravenous drip /	20070523-	8 Day
49	furosemide furosemide	Intravenous drip /	20070515-20070520	Diuretic therapy
50	furosemide furosemide	Intravenous drip /	20070523-20070525	Diuretic therapy
51	furosemide furosemide	Intravenous drip /	20070527-20070530	Diuretic therapy
52	gastrografin meglumine amidotrizoate; sodium amidotrizoate	Intravenous drip 40 ml millilitre(s)/Day	20070524-20070524	1 Day
53	gastrografin meglumine amidotrizoate; sodium amidotrizoate	Intravenous drip 40 ml millilitre(s)/Day	20070529-20070529	1 Day
54	inzolen aspartate zinc; cobalt hydrogenaspartate; copper hydrogenaspartate; magnesium aspartate; manganese hydrogenaspartate; potassium hydrogen aspartate	Intravenous (not otherwis 1 Df dosage form/Day	20070523-	Electrolyte substitution therapy
55	kalinor potassium chloride	Oral /	20070603-20070605	3 Day
56	Kaliumchlorid potassium chloride	Intravenous drip /	20070515-20070520	Electrolyte substitution therapy
57	Kaliumchlorid potassium chloride	Intravenous drip /	20070523-20070602	Electrolyte substitution therapy
58	ketamine hydrochloride ketamine hydrochloride	Intravenous drip /	20070515-20070521	Sedative therapy
59	ketamine hydrochloride ketamine hydrochloride	Intravenous drip /	20070523-20070529	Sedative therapy
60	magnesium stoffangabe (teilweise) unklar	1 Df dosage form/Day	20060601-	7 Day
61	MOVICOL polyethylene glycol 3350; potassium chloride; sodium bicarbonate; sodium chloride	Oral 1 Df dosage form/Day	20070518-20070523	Constipation
62	MOVICOL polyethylene glycol 3350; potassium chloride; sodium bicarbonate; sodium chloride	Oral 1 Df dosage form/Day	20070530-20070604	6 Day

Drugs (concomitant)				
63	Natriumchlorid sodium chloride	Intravenous drip	/	20070514-20070515 Electrolyte substitution therapy 2 Day
64	Natriumchlorid sodium chloride	Intravenous (not otherwis	/	20070602-20070602 Electrolyte substitution therapy 1 Day
65	Natriumchlorid sodium chloride	Intravenous (not otherwis	/	20070523-20070530 Electrolyte substitution therapy 8 Day
66	oliclinomel amino acids nos; glucose; soya oil	Intravenous drip	/	20070603- -
67	Pyridoxin pyridoxine	Intravenous drip	/	20070523- Electrolyte substitution therapy -
68	Pyridoxin pyridoxine	Intravenous drip	/	20070513-20070513 Electrolyte substitution therapy 1 Day
69	ringer calcium chloride dihydrate; potassium chloride; sodium chloride		/	20070523-20070530 8 Day
70	RINGER calcium chloride dihydrate; potassium chloride; sodium chloride		/	20070513-20070514 2 Day
71	RINGER calcium chloride dihydrate; potassium chloride; sodium chloride		/	20070602-20070602 -
72	unizink zinc dl-aspartate	Intravenous drip	1 Df dosage form/Day	20070523- -
73	vitamin b1 thiamine hydrochloride		200 Mg milligram(s)/	20070515-20070515 -
74	vitamin b1 thiamine hydrochloride		/	20070516- -
75	vitamin b6 pyridoxine hydrochloride		1 Df dosage form/Day	20070513-20070514 2 Day
76	vitamin b6 pyridoxine hydrochloride		1 Df dosage form/Day	20070523- -
77	vitamin c ascorbic acid		1 Df dosage form/Day	20060601- -

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Alcoholic withdrawal symptoms			Yes	
Swelling of feet	20070523		Unknown	
Hyperglycaemia			Yes	
Asystolia			No	23-May-2007
Fever	20070521		Unknown	
Hypertension			Yes	
Multiorgan failure	20070523		Unknown	23-May-2007
Pain	20070523		Unknown	
Pancreatitis	20070513		Unknown	hospitalisation
Respiratory insufficiency	20070521		Unknown	
Restlessness			Yes	
Alcohol withdrawal syndrome			Yes	
Hospitalisation			Yes	since 13-May-2007 for pancreatitis
Disorder circulatory system			k.A.	
Dyspnoea			k.A.	
Nausea			k.A.	
Redness	20070523		Unknown	
Hospitalisation			Yes	

Primary source

Qualification: Physician

observestudytype:

Literature:

Narratives (nur bei elektronischem Eingang)

009507513, 15.12.2008 DE-MERCK-0812DEU00187

Sender diagnosis: not reported

Narrative

Information has been received from a registry (documenting serious skin reactions that occur in Germany) concerning a 34-year-old male hospitalized patient with hyperglycaemia and alcoholic withdrawal symptoms who, on 17-MAY-2007, was placed on therapy with imipenem/cilastatin sodium, 500 mg injection (form) for the treatment of pancreatitis (total daily dose not reported).

Concomitant therapy included ceftriaxone sodium (ROCEPHIN), 2g daily i.v. (therapy initiated on 13-MAY-2007 - discontinued on 16-MAY-2007) for the treatment of pancreatitis; dipyrone (NOVALGIN), i.v.; dose unclear, (therapy initiated on 23-MAY-2007 until 24-MAY-2007 and one time dose on 30-MAY-2007) for the treatment of pain; radiographic contrast medium (unspecified), i.v.; dose unknown, (initiated on 13-MAY-2007 until 22-MAY-2007, one time dose on 23-MAY-2007 and on 31-MAY-2007) for CT examination; heparin, i.v.; dose unknown (therapy initiated on 15-MAY-2007 until 02-JUN-2007) for prophylaxis of thrombosis; sufentanil, i.v., dose unknown (therapy initiated on 15-MAY-2007 until 16-MAY-2007, one time dose on 21-MAY-2007 and from 23-MAY-2007 until 02-JUN-2007) for sedation; insulin, injection, dose unknown, for the treatment of hyperglycaemia (therapy initiated on 15-MAY-2007); ciprofloxacin hydrochloride (CIPROBAY), 200 mg twice daily, i.v., for the treatment of pancreatitis (therapy initiated on 17-MAY-2007 until 22-MAY-2007); hydrocortisone, 200 mg, i.v., total daily dose and indication unknown (therapy initiated on 23-MAY-2007 until 02-JUN-2007); vancomycin, i.v., dose unknown, for the treatment of pancreatitis (therapy initiated on 23-MAY-2007 until 03-JUN-2007); fluconazole, 800 mg daily i.v. for the treatment of pancreatitis (therapy initiated on 23-MAY-2007 until 03-JUN-2007); metoclopramide, i.v., dose unknown, (initiated on 23-MAY-2007 until 05-JUN-2007) for the treatment of nausea and 30 drops daily, oral intake, from 06-JUN-2007 until 09-JUN-2007; pantoprazole sodium (PANTOZOL), 40 mg twice daily, i.v., (therapy initiated on 23-MAY-2007 until 05-JUN-2007) for stomach protection and 40 mg twice daily, oral intake (therapy initiated on 06-JUN-2007); clonidine HCl (CATAPRESAN), i.v., dose and indication unknown, (therapy initiated on 23-MAY-2007 until 04-JUN-2007 and from 06-JUN-2007 until 10-JUN-2007); albuterol, inhalant, dose unknown, (therapy initiated on 23-MAY-2007) for the treatment of dyspnoe in the context of respiratory insufficiency; acetaminophen (PERFALGAN), 1 gram, i.v., total daily dose unknown, (therapy initiated on 26-MAY-2007 until 27-MAY-2007, one time dose on 29-MAY-2007 and on 03-JUN-2007) for the treatment of fever; haloperidol (HALDOL), 5 mg, i.v., total daily dose unknown, (therapy initiated on 26-MAY-2007 until 05-JUN-2007) for the treatment of alcoholic withdrawal symptoms; dihydralazineSO4 (NEPRESOL (dihydralazine sulfate)), i.v.; dose unknown, (therapy initiated on 29-MAY-2007 until 31-MAY-2007) for the treatment of arterial hypertension; azithromycin (ZITHROMAX), 500 mg daily, i.v., (therapy initiated on 30-MAY-2007 until 01-JUN-2007) for the treatment of pancreatitis; clonazepam (RIVOTRIL), i.v.; dose unknown, (one time dose on 31-MAY-2007) for the treatment of restlessness; piperacillin sodium (+) tazobactam sodium (TAZOBAC), 4.5 gram three times daily, i.v., (therapy initiated on 31-MAY-2007 until 03-JUN-2007) for the treatment of pancreatitis; enoxaparin sodium (CLEXANE (enoxaparin sodium)), 40 mg daily, s.c. injection, (therapy initiated on 03-JUN-2007) for prophylaxis of thrombosis; lorazepam (TAVOR (lorazepam)), i.v., dose unknown, (therapy initiated on 03-JUN-2007) for the treatment of alcoholic withdrawal symptoms; meropenem, 1 gram three times daily, i.v., (therapy initiated on 04-JUN-2007) for the treatment of pancreatitis.

Other concomitant therapy included: promethazine HCl (ATOSIL); vitamins (unspecified) (CERNEVIT); midazolam (DORMICUM (midazolam)); diatrizoate meglumine (+) diatrizoate sodium (GASTROGRAFIN); electrolytes (unspecified) (INZOLEN); KHCO₃/K citrate (KALINOR); polyethylene glycol 3350/KCl/NaHCO₃/NaCl (MOVICOL); amino acids (unspecified) (+) dextrose (+) electrolytes (unspecified) (+) lipids (unspecified) (OLICLINOMEL); zinc aspartate (UNIZINK); TARUS [therapy unspecified]; electrolytes (unspecified); folic acid; furosemide; ketamine HCl; potassium chloride; pyridoxine; sodium chloride and thiamine.

On 10-JUN-2007, the patient developed spots on the skin (first symptoms of SJS). On 11-JUN-2007, he developed blisters of the skin. The patient was diagnosed with transitional form of Steven-Johnson-syndrom/TEN. On 29-JUN-2007, he recovered from transitional form of Steven-Johnson-syndrom/TEN.

Information received on 10-DEC-2008 contained the following adverse experiences: fever (21-MAY-2007); respiratory insufficiency (21-MAY-2007); swollen feet (23-MAY-2007); redness (23-MAY-2007); pain (23-MAY-2007); multi-organ failure (23-MAY-2007) and nausea (23-MAY-2007); arterial hypertension (25-MAY-2007); restlessness (31-MAY-2007).

The reporting physician felt that transitional form of Steven-Johnson-syndrom/TEN was possibly related to therapy with cilastatin sodium (+) imipenem and/or ceftriaxone sodium (ROCEPHIN) and/or dipyrone (NOVALGIN), and/or radiographic contrast medium (unspecified) and/or heparin and/or sufentanil and/or insulin and/or ciprofloxacin hydrochloride (CIPROBAY) and/or hydrocortisone and/or vancomycin and/or fluconazole and/or metoclopramide and/or pantoprazole sodium (PANTOZOL) and/or clonidine HCl (CATAPRESAN) and/or albuterol and/or acetaminophen (PERFALGAN) and/or haloperidol (HALDOL) and/or dihydralazineSO₄ (NEPRESOL (dihydralazine sulfate)) and/or azithromycin (ZITHROMAX) and/or clonazepam (RIVOTRIL) and/or piperacillin sodium (+) tazobactam sodium (TAZOBAC) and/or enoxaparin sodium (CLEXANE (enoxaparin sodium)) and/or lorazepam (TAVOR (lorazepam)) and/or meropenem..

Transitional form of Steven-Johnson-syndrom/TEN was considered to be immediately life-threatening by the reporting physician.

Upon internal review, multi-organ failure was determined to be an Other Important Medical Event.

Additional information is not expected.

Sender's comment:

Stevens-Johnson-syndrome (SJS) is a serious labeled event for therapy with ZIENAM°. Multi-organ failure is a serious unlabeled event. Nausea is non-serious and labeled; respiratory insufficiency, swollen feet, erythema, pain, agitation and arterial hypertension are non-serious and unlabeled. This 34 year old male received ZIENAM° for the treatment of pancreatitis. 11 days after completion of the course of ZIENAM°, he developed SJS. The other symptoms can be considered as complications of the underlying disease or of SJS and a direct link with ZIENAM° is not suspected. The causality evaluation regarding SJS is confounded by the multiple concomitant medication including drugs that have been related to the induction of severe skin reactions. Severe skin reaction including SJS and TEN are also adequately described in ZIENAM° labeling. Self initiated measures are not indicated.

Reporter's comment:

Wyeth-Pharma GmbH, 01.03.2010 DE-WYE-G05633610

Sender diagnosis: not reported

Narrative

This case has been received from Pfizer (Pfizer Case ID #2010019339) and contains a Pfizer co-suspect product. The Pfizer case may qualify for regulatory reporting for the Pfizer co-suspect product. This case may therefore be a duplicate report.

Information was received from a healthcare professional via a registry entitled "European Registry of Severe Cutaneous Adverse Reactions to Drugs (RegiSCAR)" regarding a 34-year-old male patient who received Tazobac (piperacillin/tazobactam injection) therapy and experienced toxic epidermal necrosis, intermediate state, and Stevens-Johnson syndrome.

MEDICAL HISTORY:

The patient has been hospitalized since 13-May-2007 for pancreatitis. His concurrent illnesses included respiratory failure, pain, hyperglycaemia, alcohol withdrawal syndrome, hypertension, restlessness and pyrexia with a past history of multi-organ failure with asystolia (on 23-May-2007). Past therapies included Xanef (enalapril maleate), Beloc Zok (metoprolol succinate), Ebrantil (urapidil), Gastrografin (meglumine amidotrizoate/sodium amidotrizoate unsp), Platelets, Human Blood, Vitamin B12 (cyanocobalamin), Etomidate, Bricanyl (terbutaline sulfate), Pancuronium, Sodium Chloride (unspec), Red Blood Cells, Concentrated, Suprarenin (epinephrine), Morphine, Intrafusin (amino acids nos), Neostigmine, Lipofundin (soya oil/xylitol), Dormicum (midazolam maleate), Midazolam, Furosemide, Ketamine, Calcium, Lactulose, Laxoberal (sodium picosulfate), Propofol, Arterenol (norepinephrine hydrochloride), Glucose, Glucose W/Electrolytes (electrolytes nos/glucose), Nexium (esomeprazole), Clonidine, Clont (metronidazole), Haes-Steril (hydroxyethylcellulose/sodium chloride), Distraneurin (clomethiazole edisilate), Dipidolor (piritramide), Sodium Bicarbonate and Calcium Polystyrene Sulfonate. The patient has received Tavor (lorazepam injection) for an unspecified period of time in the past without having experienced any adverse events.

PRODUCT DETAILS:

Indication for Tazobac was pancreatitis. Therapy began on 30-May-2007 and was permanently discontinued on 03-Jun-2007. Dose regimen was 4.5 g 3 times per day (intravenous).

Additional suspect medication included Insulin Human, Heparin Sodium, Rocephin (ceftriaxone sodium), Sufentanil, Ciprobay (ciprofloxacin hydrochloride), Zienam (cilastatin sodium/imipenem), Tavor (lorazepam injection), Pantozol (pantoprazole unsp), Metoclopramide, Meropenem, Clexane (heparin-fraction, sodium salt), Rivotril (clonazepam), Zithromax (azithromycin), Nepresol (dihydralazine sulfate), Haldol (haloperidol), Perfalgan (paracetamol), Broncho (salbutamol spray), Catapresan (clonidine), Fluconazole, Vancomycin, Hydrocortisone, Novalgin (metamizole sodium) and an unspecified contrast medium.

CONCOMITANT THERAPY:

Concomitant therapy included Magnesium, Calcium Carbonate, Vitamine C (ascorbic acid), Sodium Chloride (unspec), Ringer (calcium chloride dihydrate/potassium chloride/sodium chloride), Vitamin B6 (pyridoxine hydrochloride), Vitamin B1 (thiamine hydrochloride), Potassium Chloride, Movicol (macrogol/potassium chloride/sodium bicarbonate/sodium chloride), Atosil (promethazine hydrochloride), Clinomel (amino acids/carbohydrates/minerals/vitamins), Kalinor (potassium chloride), Cernevit (vitamins nos), Inzolen (aspartate zinc/cobalt hydrogenaspartate/copper hydrogenaspartate/magnesium aspartate/manganese hydrogenaspartate/potassium hydrogen aspartate), Unizink (zinc dl-aspartate) and Calcium Folate.

EVENT DETAILS:

The patient experienced erythema from 10-Jun-2007 and vesiculation of skin from 11-Jun-2007 and he was diagnosed with toxic epidermal necrosis, intermediate state (toxic epidermal necrolysis) and Stevens-Johnson syndrome (Stevens-Johnson syndrome). These events were considered life-threatening. No information about treatment was provided. The patient has recovered by 29-Jun-2007.

RELATEDNESS ASSESSMENT:

The investigators considered Stevens-Johnson syndrome and toxic epidermal necrolysis possibly related to Tazobac, Zithromax, Novalgin, contrast medium, Heparin, Sufentanil, Insulin human, Zienam, Hydrocortisone, Vancomycin, Fluconazol, Metoclopramide, Pantozol, Catapresan, BronchoPerfalgan, Haldol, Nepresol, Rivotril, Clexane, Tavor and Meropenem, but more likely related to Tazobac and Zithromax. The investigator's

causality assessment regarding Ciprobay and Rocephin were unknown. The medical monitor(s) assessment for Stevens-Johnson syndrome and toxic epidermal necrolysis was unknown for all products.

Sender's comment:

Wyeth comment:

Initial (v2) 26-Feb-2010

STEVENS-JOHNSON-SYNDROM unter Tazobac

Dieser Fall wurde von Pfizer erhalten (Ref. 2010019339) und enthält ein verdächtiges Pfizer-Produkt. Möglicherweise könnte dadurch eine Doppelmeldung entstehen.

Fall aus dem Dokumentationszentrum schwerer Hautreaktionen Freiburg:

Es wurde über einen 34-jährigen Patienten mit komplexer Anamnese berichtet, der unter Einnahme von u.a. Tazobac (seit 31.05.07) und Tavor (seit 03.06.07) ab dem 10.06.07 ein lebensbedrohliches Stevens-Johnson-Syndrom mit toxisch-epidermaler Nekrolyse entwickelte. Neben Tazobac und Tavor wurden ausserdem Rocephin, Novalgin, ein nicht näher bekanntes Kontrastmittel, Heparin, Sufentanil, Insulin, Ciprobay, Zienam, Hydrocortison, Vancomycin, Fluconazol, MCP, Pantozol, Catapresan, Broncho, Perfalgan, Haldol, Nepresol, Zithromax, Rivotril, Clexane und Meropenem als ursächlich für die schwere Hautreaktion verdächtigt, wobei am ehesten ein Zusammenhang für Tazobac und Zithromax vermutet worden ist.

Informationen über die Behandlung wurden nicht übermittelt, es kam jedoch zur vollständigen Heilung.

Unter Anwendung von Tazobac sind die berichteten schwerwiegenden Ereignisse bekannt. Unter Tavor-Anwendung sind die Ereignisse unbekannt. Basierend auf der aktuellen Information führt dieser Bericht nicht zu einer Änderung des Nutzen-Risiko-Verhältnisses. Zu diesem Zeitpunkt sind keine weiteren Maßnahmen vorgesehen. Sobald Änderungen hierzu bekannt werden, werden wir darüber unverzüglich und unaufgefordert informieren.

Dr. D. Kracht

Reporter's comment:

Nycomed, 01.03.2010 DE-Nycomed-0210214

Sender diagnosis: not reported

Narrative

This case was received by our licensee Wyeth (ref. no. DE-WYE-G05633610) on 15-FEB-2010 from Pfizer (ref. no. 2010019339) and forwarded to Nycomed on 26-FEB-2010. This case derives from a non interventional study entitled "European Registry of Severe Cutaneous Adverse Reactions".

It concerned a 34-year-old male patient started therapy treated with Pantozol IV (pantoprazole), 40 mg twice daily, intravenous, on 23-MAY-2007, for gastric ulcer prophylaxis. On 06-JUN-2007, IV pantoprazole was switched to oral pantoprazole at a dosage of 40 mg once daily. Additional suspect medications were piperacillin/tazobactam, human insulin, heparin Sodium, ceftriaxone, sufentanil, ciprofloxacin, cilastatin sodium/imipenem, lorazepam metoclopramide, meropenem, enoxaparin sodium, clonazepam, azithromycin, dihydralazine, haloperidol, paracetamol, salbutamol, clonidine, fluconazole, vancomycin, hydrocortisone, metamizole sodium and an unspecified contrast medium; nine of which were introduced on the same day as pantoprazole and the others within 3 to 18 days of pantoprazole introduction. The patient had a medical history of pancreatitis (for which he was hospitalised on 13-MAY-2007), respiratory insufficiency, hyperglycaemia, fever), pain, alcohol withdrawal syndrome, hypertension, restlessness, circulatory system disorder, dyspnoea, nausea and a past history of multi-organ failure with asystolia (on 23-MAY-2007). He was concomitantly treated with promethazine, multivitamins, minerals and aminoacids. The patient had numerous past medications. On 10-JUN-2007 (18 days after the first dose and 5 days after the last of pantoprazole IV; 4 days after the first dose of oral pantoprazole), the patient experienced erythema and on 11-JUN-2007, the patient developed vesiculation of skin. He was diagnosed with toxic epidermal necrosis, intermediate state and Stevens-Johnson syndrome. These events were considered life-threatening. No information about treatment was provided.

Action taken concerning therapy with pantoprazole was unknown.

The patient had recovered by 29-JUN-2007.

The investigators considered Stevens-Johnson syndrome and toxic epidermal necrolysis as possibly related to pantoprazole and all other suspect medications, except ceftriaxone and ciprofloxacin, for which no assessment was provided. The investigators however stated that the Stevens-Johnson syndrome was more likely related to piperacillin/tazobactam and azithromycin.

Sender's comment:

Stevens-Johnson-syndrome and toxic epidermal necrolysis are listed in the current reference safety information on pantoprazole.

Stevens - Johnson syndrome (SJS) is a rare but severe dermatological condition, considered as a severe type of erythema exsudativum multiforme. It is characterized by erythema with bullous and eroded lesions of skin and mucous membranes. Additional symptoms include fever, general ill feeling and joint aches. SJS is an immune-complex-mediated hypersensitivity reaction that occurs in response to medications (e.g., sulfonamides, penicillins, barbiturates, and phenytoin), infections (e.g., herpes simplex, Mycoplasma and malaria infections), or malignancies (various carcinomas and lymphomas). Stevens-Johnson syndrome (SJS) is idiopathic in 25-50% of cases.

Lyell's syndrome or toxic epidermal necrolysis (TEN) is an acute severe bullous cutaneous disease characterized by extensive areas of skin necrosis accompanied by a systemic toxic condition. It is characterized by the detachment of the epidermis from the dermis and may affect the whole body. It usually develops in the form of blistered erythematous patches and plaques which evolve within hours to extensive areas of skin necrosis with severe epidermolysis. TEN usually results from severe adverse reaction to certain drugs. The drugs most often implicated in TEN are antibiotics such as sulfonamides; NSAIDs, allopurinol, antiretroviral drugs, corticosteroids and anticonvulsants such as phenobarbital, carbamazepine and valproic acid. The condition might also result from immunizations, infection with agents such as Mycoplasma pneumoniae, herpes virus, transplants of bone marrow and organs.

This patient had other risk factors that could have caused or contributed to the development of the reactions e.g. co-suspect medications.

This single case report is not considered to change the current knowledge of the safety profile of pantoprazole.

Reporter's comment:

PFIZERINC, 02.03.2010 DE-PFIZER INC-2010019339

Sender diagnosis: not reported

Narrative

This is a spontaneous report from a project, sponsored by the German Regulatory Authority, set up to collect suspected drug induced severe skin reactions leading to hospital admission. Reference number from the documentation center for severe skin reactions (dZh): 9170540.

A contactable physician reported about a 34 year old male patient who was admitted to hospital due to pancreatitis on 13May2007.

On 21May2007 the patient experienced fever and respiratory insufficiency, On 23May2007 he experienced swelling of feet, redness, pain, multiorgan failure and pancreatitis (as reported).

The patient received the following treatments: 500 mg/day intravenous azithromycin (ZITHROMAX) due to pancreatitis from 30May2007 to 01Jun2007, intravenous ketamine hydrochloride for sedation from 15May2007 to 21May2007 and from 23May2007 to 29May2007, intravenous furosemide for diuresis from 15May2007 to 20May2007, 23May2007 to 25May2007 and 27May2007 to 30May2007, intravenous heparin sodium for thrombosis prophylaxis from 15May2007 to 02Jun2007, 100 mg intravenous hydrocortisone sodium succinate (HYDROCORTISONE) for circulatory stabilisation from 23May2007 to 02Jun2007, 800 mg/day intravenous fluconazole due to pancreatitis from 23May2007 to 03Jun2007, 2.5 g in the morning and 1 g in the evening intravenous metamizole sodium (NOVALGIN) due to pain on 14May2007, from 23May2007 to 24May2007 and on 30May2007, intravenous contrast media for computerized tomography from 13May2007 to 22May2007, on 23May2007 and on 31May2007, intravenous 0.9 % sodium chloride for substitution from 14May2007 to 15May2007, 23May2007 to 30May2007 and on 02Jun2007, intravenous RINGER LOSUNG for substitution on 14May2007, from 23May2007 to 30May2007 and on 02Jun2007, intravenous pyridoxine hydrochloride (VITAMIN B6) for substitution on 14May2007 and from 23May2007 onwards, intravenous potassium chloride for substitution from 15May2007 to 20May2007 and from 23May 2007 to 02Jun2007, intravenous sufentanil for sedation from 15May2007 to 16May2007, on 21May2007 and from 23May2007 to 02Jun2007, intravenous Insulin-H due to hyperglycemia from 15May2007 onwards, intravenous midazolam hydrochloride (DORMICUM) for substitution (as reported) from 16May2007 to 29May2007, intravenous thiamine hydrochloride (VITAMIN B1) for substitution from 16May2007 onwards, intravenous 500 mg, 4x/day cilastatin sodium, imipenem (ZIENAM) due to pancreatitis from 16May2007 to 31May2007, 1 sachet every evening oral macrogol, potassium chloride, sodium bicarbonate, sodium chloride (MOVICOL) due to constipation from 18May2007 to 23May2007 and from 30May2007 to 04Jun2007, intravenous vancomycin due to pancreatitis from 23May2007 to 03Jun2007, intravenous metoclopramide due to nausea from 23May2007 to 05Jun2007, 40 mg, 2x/day intravenous pantoprazole (PANTOZOL) for stomach protection from 23May2007 to 05Jun2007, intravenous clonidine (CATAPRESAN) due to withdrawal (as reported) from 23May2007 to 04Jun2007 and from 06Jun2007 to 10Jun2007, intravenous folic acid (FOLATE) for substitution from 23May2007 onwards, thrice daily respiratory salbutamol (BRONCHO SPRAY) due to dyspnea from 23May2007 onwards, once daily intravenous zinc DL-aspartate (UNIZINK) for substitution from 23May2007 onwards, once daily aspartate zinc, cobalt hydrogenaspartate, copper hydrogenaspartate, magnesium aspartate, manganese hydrogenaspartate, potassium hydrogen aspartate (INZOLEN) for substitution from 23May2007 onwards, once daily intravenous vitamins NOS (CERNEVIT) for substitution from 23May2007 onwards, intravenous meglumine amidotrizoate, sodium amidotrizoate (GASTROGRAFIN) for gastrography on 24May2007 and on 29May2007, 1 g intravenous propacetamol (PERFALGAN) due to pyrexia from 26May2007 to 27May2007, on 29May2007 and on 03Jun2007, intravenous haloperidol (HALDOL) due to alcohol withdrawal from 26May2007 to 05Jun2007 starting with 5 mg, thrice daily and continued on unspiced day with 5 mg twice daily, intravenous dihydralazine sulfate (NEPRESOL) due to hypertension from 29May2007 to 31May2007, intravenous unknown ingredients (TARUS) for an unknown indication on 30May2007, intravenous clonazepam (RIVOTRIL) due to restlessness on 31May2007, 4.5 g, 3x/day intravenous piperacillin sodium, tazobactam sodium (TAZOBAC) due to pancreatitis from 31May2007 to 03Jun2007, subcutaneous fructose, insulin zinc suspension (ALT-INSULIN) due to hyperglycemia from 03Jun2007 to 04Jun2007, oral potassium carbonate,

(KALINOR-BRAUSETABLETTEN) for substitution from 03Jun2007 to 05Jun2007, 40 mg, 1x/day subcutaneous heparin-fraction, sodium salt (CLEXANE) for thrombosis prophylaxis from 03Jun2007 onwards, intravenous lorazepam (TAVOR, Wyeth Co-suspect) due to alcohol withdrawal, intravenous soybean oil, glucose, calcium chloride, diverse amino acids (OLICLINOMEL) for substitution from 03Jun2007 onwards, 1 g, 3x/day intravenous meropenem due to pancreatitis from 04Jun2007 onwards, 30 drops/day oral metoclopramide due to nausea from 06Jun2007 to 09Jun2007, 40 mg, 2x/day oral pantoprazole sodium (PANTOZOL) for stomach protection from 06Jun2007 onwards.

Concomitant medication included: intravenous fructose, insulin zinc suspension (ALT-INSULIN) on 14May2007, 2 sachets oral calcium polystyrene sulfonate on 14May2007, 8.4% intravenous sodium bicarbonate from 14May2007 to 15May2007, 5 mg, 4x/day intravenous piritramide (DIPIDOLOR) from 14May2007 to 15May2007, subcutaneous heparin from 14May2007 to 15May2007, 20 mg, 2x/day oral furosemide from 14May2007 to 15May2007, every 4 hours clomethiazole edisilate (DISTRANEURIN) from 14May2007 to 15May2007, 6% intravenous hyetellose, sodium chloride (HAES-STERIL) from 14May2007 to 15May2007, 2 g, 1x/day intravenous ceftriaxone sodium (ROCEPHIN) from 13May2007 to 16May2007, 500 mg, 2x/day intravenous metronidazole (CLONT) from 13May2007 to 16May2007, intravenous clonidine on 14May2007 and from 16May2007 to 21May2007, starting with 40 mg, 2x/day, continued with 40 mg/, 1x/day intravenous esomeprazole (NEXIUM) from 13May2007 to 22May2007, 500 mg intravenous glucose monohydrate, magnesium chloride hexahydrate, potassium chloride, potassium phosphate monobasic, sodium acetate trihydrate, sodium chloride (TUTOFUSIN BG) on 13May2007 and from 18May2007 to 22May2007, once daily oral magnesium salt not spec. (MAGNESIUM SHERBET) from 01Jun2007 onwards, once daily oral calcium carbonate (CALCIUM SHERBET) from 01Jun2007 onwards, once daily ascorbic acid (VITAMIN C) from 01Jun2007 onwards, intravenous norepinephrine hydrochloride (ARTERENOL) 14May2007 to 19May2007 and 23May2007 to 24May2007, 5 % intravenous glucose from 14May2007 to 22May 2007, 20% intravenous glucose on 24May2007, intravenous midazolam on 15May2007, 200 mg, 1x/day oral thiamine hydrochloride (VITAMIN B1) on 15May2007, intravenous propofol from 15May2007 to 16May2007, 20 drops 1x/day oral sodium picosulfate (LAXOBERAL) from 15May2007 to 22May2007, 20 ml, 3x/day oral lactulose from 15May2007 to 22May2007, intravenous calcium salt (CALCIUM MINERALS) from 15May2007 to 22May2007, intravenous unknown ingredients (HG inf. sol.) on 17May2007, 200 mg, 2x/day intravenous ciprofloxacin hydrochloride (CIPROBAY), intravenous soya oil, xylitol (LIPOFUNDIN) from 18May2007 to 21May2007, intravenous neostigmine from 18May2007 to 22May2007, intravenous amino acids NOS (INTRAFUSIN) from 18May2007 to 22May2007, 10 mg intravenous morphine sulfate (MORPHIN) on 20May2007, intravenous epinephrine (SUPRARENIN) on 21May2007, 10 % intravenous sodium chloride on 22May2007, intravenous pancuronium on 22May2007, once daily subcutaneous terbutaline sulfate (BRICANYL) on 23May2007, 5 mg, 4x/day intravenous etomidate on 23May2007, intravenous cyanocobalamin (VITAMIN B12) from 23May2007 to 24May2007, intravenous urapidil (EBRANTIL) on 25MAY2007, 5 mg, 1x/day oral metoprolol succinate (BELOC ZOK) on 25May2007, intravenous enalapril maleate (XANEF) on 26May2007, intravenous isopromethazine hydrochloride (ATOSIL) from 09Jun2007 to 10Jun2007, intravenous red blood cell concentrate on 21May2007 and on 24May2007 and intravenous thrombocyte concentrate on 24May2007.

On 10Jun2007 the patient experienced spots (NOS), on 11Jun2007 he experienced blistering of skin and on 19Jun2007 he experienced redness and pain of oral mucosa. Diagnosis of Stevens-Johnson syndrome and toxic epidermal necrolysis transitional form was made on 10Jun2007 (or on 11Jun2007, contradicting reporting). The events were assessed as life-threatening. As per experts the patient recovered from the events on 29Jun2007. Information on medication was available from 14May2007 until diagnosis of the severe skin reaction on 11Jun2007 (as reported). On 10Jun2007 the project experts assessed the causal relationship as "very possible" for azithromycin and piperacillin sodium/tazobactam sodium, as "possible" for hydrocortisone sodium succinate, fluconazole, heparin sodium, metamizole sodium, contrast media, sufentanil, Insulin-H, cilastan sodium/imipenem, vancomycin, metoclopramide (intravenous), pantoprazole sodium (intravenous), clonidine, salbutamol, propacetamol, haloperidol, dihydralazine sulfate, clonazepam, fructose/insulin zinc suspension, heparin-fraction/sodium salt, lorazepam, meropenem, metoclopramide (oral) and pantoprazole sodium (oral), as "dubios" for ketamine hydrochloride, furosemide, sodium chloride, ringer losung, pyridoxine hydrochloride, potassium chloride, midazolam hydrochloride, thiamine hydrochloride, macrogol/potassium chloride/sodium bicarbonate/sodium chloride, folic acid, zinc DL-aspartate, aspartate zinc/cobalt hydrogenaspartate/copper hydrogenaspartate/magnesium aspartate/manganese hydrogenaspartate/potassium hydrogen aspartate, vitamins nos, meglumine amidotrizoate/sodium amidotrizoate, Tarus, potassium carbonate/ potassium citrate and soybean oil/glucose/calcium chloride/diverse amino acids. The project experts assessed the following drugs as "suspicious" (including drugs without causal relationship to the events as per experts): azithromycin, metamizole sodium, piperacillin sodium/tazobactam sodium, ceftriaxone sodium and ciprofloxacin hydrochloride. As alternative causes of the reported events the following was documented: The category "infection" was documented as "yes, possible" for necrotizing pancreatitis and the category "other" was documented as "yes, possible" for hepatitis.

Follow-up status: case closed (15Feb2010).

Sender's comment:

Bei Stevens-Johnson-Syndrom und Toxisch-epidermaler Nekrolyse handelt es sich um bekannte, in den Produktinformationen von Azithromycin und Fluconazol ausreichend praezise deklarierte Nebenwirkungen und in der Produktinformation von Hydrocortison nicht deklarierte moegliche Nebenwirkungen. Wir sehen zum gegenwaertigen Zeitpunkt keinen Anlass fuer eine geaenderte Bewertung des Nutzen-/Risikoverhaeltnisses.

Reporter's comment:

PFIZERINC, 27.01.2011 DE-PFIZER INC-2010019339

Sender diagnosis: not reported

Narrative

This is a spontaneous report from a project, sponsored by the German Regulatory Authority, set up to collect suspected drug induced severe skin reactions leading to hospital admission. Reference number from the documentation center for severe skin reactions (dZh): 9170540.

A contactable physician reported about a 34 year old male patient who was admitted to hospital due to pancreatitis on 13May2007.

On 21May2007 the patient experienced fever and respiratory insufficiency, On 23May2007 he experienced swelling of feet, redness, pain, multiorgan failure and pancreatitis (as reported).

The patient received the following treatments: 500 mg/day intravenous azithromycin (ZITHROMAX) due to pancreatitis from 30May2007 to 01Jun2007, intravenous ketamine hydrochloride for sedation from 15May2007 to 21May2007 and from 23May2007 to 29May2007, intravenous furosemide for diuresis from 15May2007 to 20May2007, 23May2007 to 25May2007 and 27May2007 to 30May2007, intravenous heparin sodium for thrombosis prophylaxis from 15May2007 to 02Jun2007, 100 mg intravenous hydrocortisone sodium succinate (HYDROCORTISONE) for circulatory stabilisation from 23May2007 to 02Jun2007, 800 mg/day intravenous fluconazole due to pancreatitis from 23May2007 to 03Jun2007, 2.5 g in the morning and 1 g in the evening intravenous metamizole sodium (NOVALGIN) due to pain on 14May2007, from 23May2007 to 24May2007 and on 30May2007, intravenous contrast media for computerized tomography from 13May2007 to 22May2007, on 23May2007 and on 31May2007, intravenous 0.9 % sodium chloride for substitution from 14May2007 to 15May2007, 23May2007 to 30May2007 and on 02Jun2007, intravenous RINGER LOSUNG for substitution on 14May2007, from 23May2007 to 30May2007 and on 02Jun2007, intravenous pyridoxine hydrochloride (VITAMIN B6) for substitution on 14May2007 and from 23May2007 onwards, intravenous potassium chloride for substitution from 15May2007 to 20May2007 and from 23May 2007 to 02Jun2007, intravenous sufentanil for sedation from 15May2007 to 16May2007, on 21May2007 and from 23May2007 to 02Jun2007, intravenous Insulin-H due to hyperglycemia from 15May2007 onwards, intravenous midazolam hydrochloride (DORMICUM) for substitution (as reported) from 16May2007 to 29May2007, intravenous thiamine hydrochloride (VITAMIN B1) for substitution from 16May2007 onwards, intravenous 500 mg, 4x/day cilastatin sodium, imipenem (ZIENAM) due to pancreatitis from 16May2007 to 31May2007, 1 sachet every evening oral macrogol, potassium chloride, sodium bicarbonate, sodium chloride (MOVICOL) due to constipation from 18May2007 to 23May2007 and from 30May2007 to 04Jun2007, intravenous vancomycin due to pancreatitis from 23May2007 to 03Jun2007, intravenous metoclopramide due to nausea from 23May2007 to 05Jun2007, 40 mg, 2x/day intravenous pantoprazole (PANTOZOL) for stomach protection from 23May2007 to 05Jun2007, intravenous clonidine (CATAPRESAN) due to withdrawal (as reported) from 23May2007 to 04Jun2007 and from 06Jun2007 to 10Jun2007, intravenous folic acid (FOLATE) for substitution from 23May2007 onwards, thrice daily respiratory salbutamol (BRONCHO SPRAY) due to dyspnea from 23May2007 onwards, once daily intravenous zinc DL-aspartate (UNIZINK) for substitution from 23May2007 onwards, once daily aspartate zinc, cobalt hydrogenaspartate, copper hydrogenaspartate, magnesium aspartate, manganese hydrogenaspartate, potassium hydrogen aspartate (INZOLEN) for substitution from 23May2007 onwards, once daily intravenous vitamins NOS (CERNEVIT) for substitution from 23May2007 onwards, intravenous meglumine amidotrizoate, sodium amidotrizoate (GASTROGRAFIN) for gastrography on 24May2007 and on 29May2007, 1 g intravenous propacetamol (PERFALGAN) due to pyrexia from 26May2007 to 27May2007, on 29May2007 and on 03Jun2007, intravenous haloperidol (HALDOL) due to alcohol withdrawal from 26May2007 to 05Jun2007 starting with 5 mg, thrice daily and continued on unspecied day with 5 mg twice daily, intravenous dihydralazine sulfate (NEPRESOL) due to hypertension from 29May2007 to 31May2007, intravenous unknown ingredients (TARUS) for an unknown indication on 30May2007, intravenous clonazepam (RIVOTRIL) due to restlessness on 31May2007, 4.5 g, 3x/day intravenous piperacillin sodium, tazobactam sodium (TAZOBAC) due to pancreatitis from 31May2007 to 03Jun2007, subcutaneous fructose, insulin zinc suspension (ALT-INSULIN) due to hyperglycemia from 03Jun2007 to 04Jun2007, oral potassium carbonate, potassium citrate (KALINOR-BRAUSETABLETTEN) for substitution from 03Jun2007 to 05Jun2007, 40 mg, 1x/day subcutaneous heparin-fraction, sodium salt (CLEXANE) for thrombosis prophylaxis from 03Jun2007 onwards, intravenous lorazepam (TAVOR, Wyeth Co-suspect) due to alcohol withdrawal, intravenous soybean oil, glucose, calcium chloride, diverse amino acids (OLICLINOMEL) for substitution from 03Jun2007 onwards, 1 g, 3x/day intravenous meropenem due to pancreatitis from 04Jun2007 onwards, 30 drops/day oral metoclopramide due to nausea from 06Jun2007 to 09Jun2007, 40 mg, 2x/day oral pantoprazole sodium (PANTOZOL) for stomach protection from 06Jun2007 onwards.

Concomitant medication included: intravenous fructose, insulin zinc suspension (ALT-INSULIN) on 14May2007, 2 sachets oral calcium polystyrene sulfonate on 14May2007, 8.4% intravenous sodium bicarbonate from 14May2007 to 15May2007, 5 mg, 4x/day intravenous piritramide (DIPIDOLOR) from 14May2007 to 15May2007, subcutaneous heparin from 14May2007 to 15May2007, 20 mg, 2x/day oral furosemide from 14May2007 to 15May2007, every 4 hours clomethiazole edisilate (DISTRANEURIN) from 14May2007 to 15May2007, 6% intravenous hyetellose, sodium chloride (HAES-STERIL) from 14May2007 to 15May2007, 2 g, 1x/day intravenous ceftriaxone sodium (ROCEPHIN) from 13May2007 to 16May2007, 500 mg, 2x/day intravenous metronidazole (CLONT) from 13May2007 to 16May2007, intravenous clonidine on 14May2007 and from 16May2007 to 21May2007, starting with 40 mg, 2x/day, continued with 40 mg/, 1x/day intravenous esomeprazole (NEXIUM) from 13May2007 to 22May2007, 500 mg intravenous glucose monohydrate, magnesium chloride hexahydrate, potassium chloride, potassium phosphate monobasic, sodium acetate trihydrate, sodium chloride (TUTOFUSIN BG) on 13May2007 and from 18May2007 to 22May2007, once daily oral magnesium salt not spec. (MAGNESIUM SHERBET) from 01Jun2007 onwards, once daily oral calcium carbonate (CALCIUM SHERBET) from 01Jun2007 onwards, once daily ascorbic acid (VITAMIN C) from 01Jun2007 onwards, intravenous norepinephrine hydrochloride (ARTERENOL) 14May2007 to 19May2007 and 23May2007 to 24May2007, 5 % intravenous glucose from 14May2007 to 22May 2007, 20% intravenous glucose on 24May2007, intravenous midazolam on 15May2007, 200 mg, 1x/day oral thiamine hydrochloride (VITAMIN B1) on 15May2007, intravenous propofol from 15May2007 to 16May2007, 20 drops 1x/day oral sodium picosulfate (LAXOBERAL) from 15May2007 to 22May2007, 20 ml, 3x/day oral lactulose from 15May2007 to 22May2007, intravenous calcium salt (CALCIUM MINERALS) from 15May2007 to 22May2007, intravenous unknown ingredients (HG inf. sol.) on 17May2007, 200 mg, 2x/day intravenous ciprofloxacin hydrochloride (CIPROBAY), intravenous soya oil, xylitol (LIPOFUNDIN) from 18May2007 to 21May2007, intravenous neostigmine from 18May2007 to 22May2007, intravenous amino acids NOS (INTRAFUSIN) from 18May2007 to 22May2007, 10 mg intravenous morphine sulfate (MORPHIN) on 20May2007, intravenous epinephrine (SUPRARENIN) on 21May2007, 10 % intravenous sodium chloride on 22May2007, intravenous pancuronium on 22May2007, once daily subcutaneous terbutaline sulfate (BRICANYL) on 23May2007, 5 mg, 4x/day intravenous etomidate on 23May2007, intravenous cyanocobalamin (VITAMIN B12) from 23May2007 to 24May2007, intravenous urapidil (EBRANTIL) on 25MAY2007, 5 mg, 1x/day oral metoprolol succinate (BELOC ZOK) on 25May2007, intravenous enalapril maleate (XANEF) on 26May2007, intravenous isopromethazine hydrochloride (ATOSIL) from 09Jun2007 to 10Jun2007, intravenous red blood cell concentrate on 21May2007 and on 24May2007 and intravenous thrombocyte concentrate on 24May2007.

On 10Jun2007 the patient experienced spots (NOS), on 11Jun2007 he experienced blistering of skin and on 19Jun2007 he experienced redness and pain of oral mucosa. Diagnosis of Stevens-Johnson syndrome and toxic epidermal necrolysis transitional form was made on 10Jun2007 (or on 11Jun2007, contradicting reporting). The events were assessed as life-threatening. As per experts the patient recovered from the events on 29Jun2007. Information on medication was available from 14May2007 until diagnosis of the severe skin reaction on 11Jun2007 (as reported). On 10Jun2007 the project experts assessed the causal relationship as "very possible" for azithromycin and piperacillin sodium/tazobactam sodium, as "possible" for hydrocortisone sodium succinate, fluconazole, heparin sodium, metamizole sodium, contrast media, sufentanil, Insulin-H, cilastan sodium/imipenem, vancomycin, metoclopramide (intravenous), pantoprazole sodium (intravenous), clonidine, salbutamol, propacetamol, haloperidol, dihydralazine sulfate, clonazepam, fructose/insulin zinc suspension, heparin-fraction/sodium salt, lorazepam, meropenem, metoclopramide (oral) and pantoprazole sodium (oral), as "dubios" for ketamine hydrochloride, furosemide, sodium chloride, ringer losung, pyridoxine hydrochloride, potassium chloride, midazolam hydrochloride, thiamine hydrochloride, macrogol/potassium chloride/sodium bicarbonate/sodium chloride, folic acid, zinc DL-aspartate, aspartate zinc/cobalt hydrogenaspartate/copper hydrogenaspartate/magnesium aspartate/manganese hydrogenaspartate/potassium hydrogen aspartate, vitamins nos, meglumine amidotrizoate/sodium amidotrizoate, Tarus, potassium carbonate/ potassium citrate and soybean oil/glucose/calcium chloride/diverse amino acids. The project experts assessed the following drugs as "suspicious" (including drugs without causal relationship to the events as per experts): azithromycin, metamizole sodium, piperacillin sodium/tazobactam sodium, ceftriaxone sodium and ciprofloxacin hydrochloride. As alternative causes of the reported events the following was documented: The category "infection" was documented as "yes, possible" for necrotizing pancreatitis and the category "other" was documented as "yes, possible" for hepatosis. Follow-up status: case closed (15Feb2010).

Follow up (11Jan2011): This is a follow-up report to notify that case 2010019339 and case DE-WYE-G05633610 are found to be duplicates resulting from case sharing between Pfizer and Wyeth after the merger in Oct2009. All subsequent follow-up information will be reported under 2010019339 which will be the retained case. Case DE-WYE-G05633610 is invalid and will be deleted from the database.

Sender's comment:

Bei Stevens-Johnson-Syndrom und Toxisch-epidermaler Nekrolyse handelt es sich um bekannte, in den Produktinformationen von Azithromycin und Fluconazol ausreichend praezise deklarierte Nebenwirkungen und in der Produktinformation von Hydrocortison nicht deklarierte moegliche Nebenwirkungen. Wir sehen zum gegenwaertigen Zeitpunkt keinen Anlass fuer eine geaenderte Bewertung des Nutzen-/Risikoverhaeltnisses.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08148338

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	Yes		

Patient

Initials:	Sex: Female	Birth: privacy	Age: 48 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Brain neoplasm	20051216				Unknown
2 Stevens-Johnson syndrome	20060106	20060130	25 Day		recovered/resolved
3 Convulsion	200512				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 DOLORMIN (ibuprofen lysine) ibuprofen lysinate	Oral	/ 245	20051201-20051231 31 Day	Back pain
2 DORMICUM (midazolam) midazolam	Intravenous (not otherwis	7.5 Mg milligram(s)/Day 120	20051230-20051230 -	Sedation
3 fortecortin dexamethasone	Oral	/	20051216- -	Prophylaxis
4 fortecortin dexamethasone	Oral	/	20051216- -	Prophylaxis
5 iodine I 125 iodinated (125 i) human serum albumin	Unknown	/	20051230- -	Brain neoplasm
6 LAXOBERAL sodium picosulfate	Oral	12 Gtt drop(s)/Day	20051231-20051231 -	Constipation
7 NEXIUM esomeprazole magnesium	Oral	1 Df dosage form/Day	20051229-20060103 6 Day	Gastric ulcer prophylaxis
8 perfalgan paracetamol	Intravenous (not otherwis	1 G gram(s)/Day 120	20051230-20051230 -	Pain
9 TAVOR (lorazepam) lorazepam	Oral	1 Df dosage form/Day	20051229-20051229 1 Day	Sleep disorder
10 Valeriana valeriana officinalis root	Oral	1 Df dosage form/Day	20051230-20060101 3 Day	Sleep disorder
11 ZENTROPIL phenytoin	Oral	300 Mg milligram(s)/Day	20060104- -	Convulsion
12 ZENTROPIL phenytoin	Oral	300 Mg milligram(s)/Day	20051216-20060103 19 Day	Convulsion
13 ZOCOR simvastatin	Oral	20 Mg milligram(s)/Day 245	20051212- -	Hypercholesterolaemia

Drugs (concomitant)

14	ADUMBRAN oxazepam	Oral	1 Df dosage form/Day	20051217-20051222 6 Day	
15	aspirin acetylsalicylic acid	Oral	100 Mg milligram(s)/Day	20051122-20051216	Thrombosis prophylaxis
16	BIFITERAL lactulose	Oral	15 ml millilitre(s)/Day 244	20051219-20051219	Constipation
17	CONCOR bisoprolol fumarate	Oral	5 Mg milligram(s)/Day	20050101- -	Hypertension arterial
18	DELIX PLUS hydrochlorothiazide; ramipril	Oral	Mg milligram(s)/Day	20000101- -	Hypertension arterial
19	PANTOZOL pantoprazole sodium	Oral	40 Mg milligram(s)/Day	20051216-20051222 7 Day	
20	sortis atorvastatin calcium	Oral	1 Df dosage form/Day 245	20050101-20051211	Hypercholesterolaemia

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Back pain			Yes	
Surgery			Yes	
Sleep disorder			Yes	
Pain			Yes	
Hypertension arterial			Yes	
Hypercholesterolaemia			Yes	
Constipation			Yes	

Primary source

Qualification: Physician observestudytype:
Literature:

Narratives (nur bei elektronischem Eingang)

009507513, 18.12.2008 DE-MERCK-0812DEU00197

Sender diagnosis: not reported

Narrative

Information has been received from a registry, documenting at least 95% of all serious skin reactions that occur in Germany, concerning a 48-year-old female patient with back pain, sleep disorder, pain, arterial hypertension and constipation who, on 12-DEC-2005, was placed on therapy with simvastatin, 20 mg tablet, 20 mg, once a day for the treatment of hypercholesterolaemia (duration not reported).

In December 2005, the patient developed seizure for the first time. On 15-DEC-2005, CT with unspecified radiographic contrast medium, injection, given iv (dose not reported) was performed (results not provided). On 16-DEC-2005, the patient was hospitalized and diagnosed with brain neoplasm (not further specified).

Concomitant suspect secondary therapy included ibuprofen lysine (DOLORMIN), tablet, for the treatment of back pain (5 tablets between 01-DEC-2005 and 31-DEC-2005), radiographic contrast medium (unspecified) iv given (dose not reported, applied between 16-DEC-2005 and 22-DEC-2005 and from 29-DEC-2005 until 30-DEC-2005), phenytoin (ZENTROPIL), po, 100 mg three times a day for the treatment of seizure (from 16-DEC-2005 until 03-JAN-2006, and from 04-JAN-2006 - stop date not reported), dexamethasone sodium phosphate (FORTECORTIN), 4 mg and/or 40 mg (start date 16-DEC-2005) as brain edema prophylaxis, lorazepam (TAVOR), 1 dose unit once on 29-DEC-2005 for the treatment of sleep disorder, esomeprazole magnesium (NEXIUM), 1 dose unit once a day, po, as gastric ulcer prophylaxis (from 29-DEC-2005 until 03-JAN-2006), midazolam (DORMICUM), 7.5 mg iv once on 30-DEC-2005 for sedation, unspecified anesthetic medication, subcutaneously given on 30-DEC-2005 for unspecified surgery, acetaminophen (PERFALGAN), iv, 1 gram given on 30-DEC-2005 for the treatment of pain, valerian (SEDONIUM), po, 1 dose daily for the treatment of sleep disorder (from 30-DEC-2005 until 01-JAN-2006), iodine I 125 SEED implant (placed on 30-DEC-2005) for the treatment of brain neoplasm and sodium picosulfate (LAXOBERAL), drops, po, 12 drops once on 31-DEC-2005 for the treatment of constipation.

Other prior and concomitant therapy included atorvastatin calcium (SORTIS), aspirin, bisoprolol fumarate (CONCOR), hydrochlorothiazide (+) ramipril (DELIX PLUS), pantoprazole sodium (PANTOZOL), oxazepam (ADUMBRAN) and lactulose (BIFITERAL).

On 06-JAN-2006, the patient developed pruritus, spots, erosions of lips and oral mucosa and, on 08-JAN-2006, erosions of the nasal mucosa, on 10-JAN-2006, blisters and, on 20-JAN-2006, erosions of genital mucosa. Final diagnosis was Stevens-Johnson syndrome. Action taken regarding therapy with simvastatin was not reported. On 30-JAN-2006, the patient recovered from Stevens-Johnson syndrome, outcome regarding seizure and brain

neoplasm was not provided.

The reporting physician felt that Stevens-Johnson syndrome was possibly related to therapy with simvastatin and/or ibuprofen lysine and/or radiographic contrast medium (unspecified) and/or dexamethasone sodium phosphate and/or lorazepam and/or esomeprazole magnesium and/or midazolam and/or anesthetic (unspecified) and/or acetaminophen and/or valerian and/or iodine I 125 and/or sodium picosulfate. The reporting physician felt that Stevens-Johnson syndrome was very likely related to therapy with phenytoin (ZENTROPIL). Drug relationship regarding seizure and brain neoplasm was not reported.

Additional information is not expected.

Upon internal review, Stevens-Johnson syndrome was determined to be an Other Important Medical Event.

Sender's comment:

Steven-Johnson syndrome, brain neoplasm and convulsion are serious unlabeled events for therapy with ZOCOR°. The same month she was started on ZOCOR a 48-year-old female patient developed convulsions and was diagnosed with brain neoplasm. The following month she developed a SJS. Given the time relation and pharmacologic action of ZOCOR° a link is rated as not probable for convulsions and brain neoplasm. The data are too poor to assess a link for SJS. The safety profile remains unchanged. Self-initiated measures are not indicated.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Report from study

case: DE-BFARM-08148882

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes	No	No	No	No	No		Yes

Patient

Initials:	Sex: Male	Birth: privacy	Age: 64 Year	weight (kg):	BMI_int:	Age fetus:
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Patientdeath

deathdate: 20060610	deathcause: Stevens Johnson syndrome
deathdate: 20060610	deathcause: Sepsis
deathdate: 20060610	deathcause: Hepatic failure

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	20060514			29 Day	fatal
2 Liver failure					fatal
3 Esophageal erosions	20060519			34 Day	fatal
4 Mucosal erosion	20060516			31 Day	fatal
5 Blister	20060515			30 Day	fatal
6 Eye redness	20060516			31 Day	fatal
7 Fever	20060508			25 Day	fatal
8 Sepsis					fatal
9 Toxic epidermal necrolysis	20060514			29 Day	fatal

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
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1	acc acetylcysteine	Oral	/	20060416-20060513 Respiratory disorder 28 Day
2	ACERBON lisinopril	Oral	/	20060424-20060513 Hypertension 20 Day
3	ANAESTHETIKA, Lokal	Subcutaneous	/	20060508- -
4	augmentan i.v. amoxicillin sodium; clavulanate potassium		/	20060508- Prophylaxis -
5	BISOHEXAL bisoprolol hemifumarate	Oral	/	20060424-20060513 Hypertension 20 Day
6	BONDIOL alfacalcidol	Oral	/	20060416-20060513 Hyperparathyroidism secondary 28 Day
7	calcimagon calcium carbonate	Oral	/	20060416-20060513 Hyperparathyroidism secondary 28 Day
8	LASIX furosemide	Oral	/	20060416-20060513 Renal failure 28 Day
9	lonolox minoxidil	Oral	/	20060416-20060422 Hypertension 7 Day
10	OXAZEPAM oxazepam	Oral	/	20060508- Sedation -
11	PANTOZOL pantoprazole sodium	Oral	/	20060416-20060513 Prophylaxis 28 Day
12	PERFALGAN paracetamol		/	20060508- Fever -
13	SANDOSTATIN octreotide acetate	Subcutaneous	/	20060420-20060503 Pancreatitis 14 Day
14	thyronajod levothyroxine sodium; potassium iodide	Oral	/	20060416-20060513 28 Day
15	TRANXILIUM clorazepate dipotassium	Oral	/	20060508- Sedation -
16	XIPAMIDE xipamide	Oral	/	20060417-20060513 Hypertension 27 Day

Drugs (concomitant)

17	AQUAPHOR xipamide	/CAN/	Oral	/	20060416-20060417	Hypertension arterial
					2 Day	
18	BICANORM sodium carbonate anhydrous	/01764101/	Oral	/	20060416-20060514	
					29 Day	
19	clont metronidazole			/	20060411-20060425	Sepsis NOS
					15 Day	
20	CONCOR bisoprolol fumarate		Oral	/	20060416-20060423	Hypertension arterial
					8 Day	
21	dreisavit ascorbic acid; calcium pantothenate; folic acid; nicotinamide; pyridoxine hydrochloride; riboflavin; thiamine hydrochloride		Oral	/	20060417-20060514	
					28 Day	
22	HYDROCORTISON hydrocortisone	/CZE/		/	20060412-20060430	
					19 Day	
23	LISIHEXAL lisinopril		Oral	/	20060416-20060423	Hypertension arterial
					8 Day	
24	MERONEM meropenem			/	20060411-20060424	Sepsis NOS
					14 Day	
25	nacl sodium chloride			/	20060411-20060417	
					7 Day	
26	nephro-vite ascorbic acid; folic acid; vitamin b nos			/	20060424-20060503	
					10 Day	
27	VANCOMYCIN vancomycin			/	20060420-	Sepsis NOS
					-	
28	VANCOMYCIN vancomycin			/	20060425-	
					-	
29	VANCOMYCIN vancomycin			/	20060430-	
					-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Hyperparathyroidism secondary			Unknown	
Renal insufficiency			Unknown	
Fever			Unknown	
Prophylaxis			Unknown	
Respiratory disorder			Yes	
Sepsis	200804	200804	No	
Difficulty breathing			Unknown	
Hypertension arterial			Yes	
Renal failure			Unknown	

Primary source

Qualification: Physician observestudytype: Other studies
Literature:

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 18.12.2008 DE-002147023-PHHY2008DE31655

Sender diagnosis: not reported

Narrative

Physician report originating from a non-Novartis registry, "RegiScar Project, Documentation Centre for Severe Skin Reactions in Germany", case ref-no. 9170408, received on 11 Dec 2008: This patient with a history of arterial hypertension and renal insufficiency had been treated with multiple drugs. He had been hospitalized in a surgery ward since 10 Apr 2006. On 08 May 2006 and 11 May 2006 he underwent surgery for shunt placements. He had received Lonolox 5 mg daily (minoxidil) from 16 Apr 2006 to 22 Apr 2006, acetylcysteine 600 mg daily from 16 Apr to 13 May 2006, Lasix (furosemide) 40 mg daily from 16 Apr to 13 May 2006, Pantozol (pantoprazole) 80 mg daily from 16 Apr to 13 May 2006, Calcimagon (calcium carbonate) 1500 mg daily from 16 Apr to 13 May 2006, Bondiol (alfacalcidol) 0.25 mg daily from 16 Apr to 13 May 2006, Thyronajod (levothyroxine, potassium iodide) from 16 Apr to 13 May 2006, xipamid 20 mg daily from 17 Apr to 13 May 2006, Sandostatin s.c. injections 3 times daily for treatment

of a pancreatitis from 20 Apr 2006 to 03 May 2006, Acerbon (lisinopril) 10 mg daily from 24 Apr to 13 May 2006, Bisohexal (bisoprolol) 5 mg daily from 24 Apr to 13 May 2006, an unknown local anaesthetic substance on 08 May 2006, oxazepam 10 mg on 08 May 2006, Tranxilium (dipotassium chlorazepate) 10 mg on 08 May 2006, Perfalgan (propacetamol) 1 g injection on 08 May 2006 and Augmentan (amoxicilline, clavulanic acid) 2.2 g injection on 08 May 2006. On 08 and 09 May 2006 he showed fever. On 14 May 2006, he again experienced fever, on 15 May 2006 he showed spots and cocards on his skin, as well as skin blistering. On this day, he was transferred from surgery to an intensive care unit. On 16 May 2006 he showed eye redness and erosions at lips and oral mucosa. On 18 May 2006, he also showed erosions at genital mucosa. On 19 May 2006, Nikolski's sign was positive, there were also erosive oesophagus lesions. In the further course he developed Stephens-Johnson-syndrome / Toxic epidermal necrolysis, transitional form. He developed sepsis and liver failure and finally passed away due to the events. End date of the events (presumably date of death) was provided as 10 Jun 2006. For Lonolox, acetylcysteine, Lasix, Pantozol, Calcimagon, Bondiol, Thyronajod, xipamid, Sandostatin, Acerbon, Bisohexal, the unknown local anaesthetic substance, oxazepam, tranxilium and Perfalgan, causality was reported as possible. For Augmantan causality was reported as probable.

Sender's comment:

Serious adverse drug reaction report [death], Steven Johnson Syndrome and hepatic failure assessed as unlisted according to the Core Data Sheet. However other alternative causes [multiple suspected drugs] provide a possible explanation for the reported adverse event.

Reporting healthcare professional causality is suspected.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08150138

Serious Yes	Death	Lifethr.	Hospital.	Disabling	Congenital	Other Yes	Pregnancy	Med. conf. Yes
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Patient

Initials:	Sex: Male	Birth: privacy	Age: 34 Year	weight (kg): 75	BMI_int: 27.22	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hot flushes	20081215		2 Day	0.25 Hour	recovered/resolved
2 Restlessness	20081215			31 Day	recovered/resolved
3 Nausea	20081215			31 Day	recovered/resolved
4 Malaise	20081215			31 Day	recovered/resolved
5 Hot flush	20081215			31 Day	recovered/resolved
6 Bullous rash	20081215			31 Day	recovered/resolved
7 Eruption	20081215		2 Day	0.25 Hour	recovered/resolved
8 Swelling of face	20081215		2 Day	0.25 Hour	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Paracetamol-ratiopharm 500 mg Tabletten paracetamol	Oral	500 Mg milligram(s)/Day Tablet	20081115- -	Headache

Drugs (concomitant)

2 Cefuroxim 250 AL Tabletten cefuroxime	Oral	250 Mg milligram(s)/Day Tablet	20081208-20081213 6 Day	Sore throat
3 Roxithromycin 300 ct (Filmtabletten) roxithromycin	Oral	300 Mg milligram(s)/Day Film-coated tablet	20081216- -	Sore throat

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Allergy to animal dander			Unknown	
House dust allergy			Unknown	
Pollen allergy			Unknown	(trees)

Primary source

Qualification: Pharmacist

observestudytype:

Literature:

Narratives (nur bei elektronischem Eingang)

[ratiopharm GmbH, 23.12.2008 DE-RATIOPH-2008R129129](#)

Sender diagnosis: not reported

Narrative

Reported terms: Bullous rash in face and upper part of body(Dermatitis bullous) Nausea(Nausea) Malaise(Malaise) Restlessness(Restlessness) Hot flush(Hot flush)

Case reference number 2008R129129 is a spontaneous case report sent by a pharmacist which refers to a male aged 34 years.

The patient's past medical history included no medical records. There is no information about risk factors. There is no information about concomitant diseases. No prior drugs are known to have been taken by the patient.

The patient started taking Paracetamol-ratiopharm 500 mg Tabletten (PARACETAMOL), indication Headache on 15-Nov-2008. The patient has been exposed to the following concomitant drug(s): Cefuroxim 250 AL Tabletten (CEFUROXIME), indication Sore throat and Roxithromycin 300 ct (Filmtabletten) (ROXITHROMYCIN), indication Sore throat. The patient developed Bullous rash in face and upper part of body (Dermatitis bullous) (start date 15-Dec-2008), Nausea (Nausea)(start date 15-Dec-2008), Malaise (Malaise)(start date 15-Dec-2008), Restlessness (Restlessness)(start date 15-Dec-2008) and Hot flush (Hot flush)(start date 15-Dec-2008).

The following action was taken: for Paracetamol-ratiopharm 500 mg Tabletten (PARACETAMOL) - Drug withdrawn

The outcome for Bullous rash in face and upper part of body (Dermatitis bullous), for Nausea (Nausea), for Malaise (Malaise), for Restlessness (Restlessness) and for Hot flush (Hot flush) is Recovered / Resolved.

Rechallenge information for Paracetamol-ratiopharm 500 mg Tabletten is not applicable. Paracetamol-ratiopharm 500 mg Tabletten was subsequently dechallenged and patient recovered.

The reporter's assessment for drug Paracetamol-ratiopharm 500 mg Tabletten related with Malaise, with Restlessness, with Hot flush, with Bullous rash in face and upper part of body and with Nausea is as follows: Unknown. CLASSIFICATION: Adverse Event: No, Adverse Reaction: Yes, Drug / Drug Interaction: No, Abuse / Misuse: No, Pregnancy / Lactation tracking: No

Sender's comment:

Bullous rash is not exactly worded in our official documents, however it could be summarized under the listed "urticaria". Restlessness, malaise, nausea and hot flush are unlisted in the RSI of Paracetamol-ratiopharm. These symptoms may be part of an allergic reaction. "Hypersensitivity reactions, ranging from skin reddening to urticaria and anaphylactic shock" are listed. Due to this actions with regard to the labelling are not necessary. The causal relationship is rated as "possible".

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09000968

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes					

Patient

Initials:	Sex: Male	Birth: privacy	Age: 19 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Erythema multiforme major	20051204	20051223	20 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 acc akut acetylcysteine	Oral	/	-	Cough
2 cefpodoxim 200 mg cefpodoxime	Oral	2 Df dosage form/Day	-	Infection NOS
3 paracetamol paracetamol	Oral	2 Df dosage form/Day	-	Fever
4 tonsilgon achillea millefolium; althaea extract; chamomile; equisetum arvense; oak bark extract; rosa canina; taraxacum officinale; walnut leaf extract	Oral	/	-	Throat pain

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09000991

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes					

Patient

Initials:	Sex: Male	Birth: privacy	Age: 11 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Erythema exudativum multiforme	20070915	20070930	16 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 benuron Saft paracetamol	Oral	/	20070908-	Fever
2 cefixim cefixime	Oral	0.5 Df dosage form/Day	20070912-20070914	Infection
3 erythromycin Saft erythromycin	Oral	0.5 Df dosage form/Day	20070915-20070916	Infection
4 Hustenstiller pentoxifyverine	Oral	/	20070910-	Cough

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Mycoplasmal pneumonia			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09001001

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes					

Patient

Initials:	Sex: Male	Birth: privacy	Age: 8 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Erythema exsudativum multiforme	20051007	20051019	13 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 benuron paracetamol	Rectal	1 Df dosage form/2 Hour	20051001-20051007	Fever 7 Day
2 capval-saft noscaphine hydrochloride	Oral	1 Df dosage form/Day	20051002-20051007	Cough 6 Day
3 Infectomycin erythromycin estolate	Oral	2 Df dosage form/Day	20051006-20051007	Bronchitis 2 Day
4 sinuc saft hedera helix	Oral	1 Df dosage form/Day	20051002-20051007	 6 Day

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Mycoplasma infection			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09001075

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes					

Patient

Initials:	Sex: Female	Birth: privacy	Age: 15 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Erythema exsudativum multiforme	20060428	20060508	11 Day		recovered/resolved
2 Stevens Johnson syndrome	20060428	20060508	11 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 amoxicillin amoxicillin	Oral	2 Df dosage form/Day	20060422-20060426 5 Day	Infection
2 doxycyclin doxycycline	Oral	/	20060426-20060429 4 Day	Bronchopneumonia
3 ibuprofen ibuprofen	Oral	2 Df dosage form/Day	20060422-20060426 5 Day	Fever
4 paracetamol paracetamol	Oral	2 Df dosage form/Day	20060422-20060426 5 Day	Fever

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09001968

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes					

Patient

Initials:	Sex: Male	Birth: privacy	Age: 16 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Erythema multiforme major	20060103	20060117	15 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ACC akut Brause 200 mg acetylcysteine	Oral	2 Df dosage form/Day	-	Cough
2 Hustagil-Thymian thyme extract	Oral	2 Df dosage form/Day	-	Cough
3 klacid clarithromycin	Oral	/	-	
4 paracetamol 500 paracetamol		/	-	Fever

Drugs (concomitant)

5 bricanyl elixier terbutaline sulfate	Oral	/	-	Cough
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Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09002015

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes					

Patient

Initials:	Sex: Female	Birth: privacy	Age: 8 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Erythema exsudativum multiforme	20071105	20071117	13 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 buscopan hyoscine butylbromide	Intravenous (not otherwis	7.5 Mg milligram(s)/Day	20071023-20071024	Pain
2 cotrim sulfamethoxazole; trimethoprim	Intravenous (not otherwis /		2 Day	20071023-20071023 Prophylaxis
3 cotrim sulfamethoxazole; trimethoprim	Oral	2 Df dosage form/Day	20071024-	Prophylaxis
4 PCM Saft Hexal paracetamol	Oral	2000 Mg milligram(s)/Day	20071024-20071026	Headache
5 tramal tramadol hydrochloride	Intravenous (not otherwis	78 Mg milligram(s)/Day	3 Day	20071023-20071027 Pain
			5 Day	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Dry eyes			Unknown	
Headache			Unknown	
Pain			Unknown	
Sore throat			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09002805

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.
 Yes Yes

Patient

Initials: Sex: Female Birth: privacy Age: 46 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	20070522	20070801	72 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 amoxypen amoxicillin trihydrate	Oral	3* /Day	20070514-20070522	Pneumonia 9 Day
2 diazepam diazepam	Oral	5 Mg milligram(s)/	20070424-20070514	Restlessness 21 Day
3 Distra clomethiazole	Oral	5 ml millilitre(s)/	20070426-	Behaviour disorder -
4 lamictal lamotrigine	Oral	12.5 Mg milligram(s)/	20070425-20070523	Epilepsy 29 Day
5 novalgin tropfen metamizole sodium	Oral	10 Df dosage form/Day	20070513-20070513	Fever -
6 orfiril valproate sodium	Oral	/	20070425-	Epilepsy -
7 paracetamol paracetamol	Rectal	500 Mg milligram(s)/Day Suppository	20070525-20070525	Fever -
8 paracetamol paracetamol	Rectal	500 Mg milligram(s)/Day Suppository	20070513-20070515	Fever 3 Day
9 paracetamol paracetamol	Rectal	2000 Mg milligram(s)/Day Suppository	20070521-20070521	Fever -

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Skin lesion			Yes	
Allergic reaction NOS			Unknown	

Primary source

Qualification: Physician observestudytype:
 Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09002808

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes	Yes	Yes					

Patient

Initials:	Sex: Male	Birth: privacy	Age: 53 Year	weight (kg):	BMI_int:	Age fetus:
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Patientdeath

deathdate:	deathcause: Multiple organ failure
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Toxic epidermal necrolysis	20050911	20051005	25 Day		fatal

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 clexane enoxaparin	Subcutaneous	1 Df dosage form/Day	20050821-20050907	Thrombosis prophylaxis
2 eugalac lactulose	Oral	30 ml millilitre(s)/Day	20050821-20050907	Constipation
3 nexium esomeprazole	Oral	20 Mg milligram(s)/Day	20050818-20050914	Gastric ulcer prophylaxis
4 novalgine metamizole	Oral	60 Gtt drop(s)/Day	20050817-	Headache
5 paracetamol paracetamol	Oral	500 Mg milligram(s)/Day	20050910-20050913	Pain
6 phenhydan phenytoin	Oral	3 Df dosage form/Day	20050818-20050912	Prophylaxis

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Hypokalaemia			Unknown	
Sleep disorder			Unknown	
Exanthema			Unknown	
Meningitis			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09002826

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Female	Birth: privacy	Age: 14 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Erythema exsudativum multiforme	20050914	20051004	21 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 benuron paracetamol	Oral	1 Df dosage form/Day	20050908-20050912	Fever
2 klacid clarithromycin	Oral	2 Df dosage form/Day	20050912-20050914	Bronchitis

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Cough			Unknown	
Throat secretion increased			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09002855

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Female	Birth: privacy	Age: 9 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Erythema exsudativum multiforme	20051102	20051110	9 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 belladonna belladonna alkaloids	Oral	30 Df dosage form/Day	20051026-20051027 2 Day	Infection
2 dolormin ibuprofen lysinate	Oral	1 Df dosage form/Day	- -	Fever
3 klacid clarithromycin	Oral	15 ml millilitre(s)/Day	20051028-20051101 5 Day	Bronchitis
4 paracetamol paracetamol	Oral	/	- -	Fever
5 spasmo-mucosolvan ambroxol hydrochloride; clenbuterol hydrochloride	Oral	3 Df dosage form/Day	20051027-20051101 6 Day	Cough

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Throat secretion increased			Unknown	
Desensitization injection			Unknown	
Conjunctivitis			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

case: DE-BFARM-09002956

Patient

Reactions

Drugs (suspected or interacting)

Primary source

561 / 678

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09003225

Serious Yes	Death	Lifethr.	Hospital.	Disabling	Congenital	Other Yes	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Female	Birth: privacy	Age: 48 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	20060106	20060130	25 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 dolormin ibuprofen lysinate	Oral	5 Df dosage form/	-	Back pain
2 dormicum 7,5 midazolam maleate	Intravenous (not otherwis	7.5 Mg milligram(s)/	-	Sedation
3 fortecortin 4 mg dexamethasone	Oral	4 Mg milligram(s)/	-	
4 kontrastmittel/MRT	Intravenous (not otherwis	/	-	
5 laxoberal sodium picosulfate	Oral	12 Gtt drop(s)/	-	Obstipation
6 lod-125-seed-implantat iodine (125 i)		/	-	
7 nexium esomeprazole	Oral	1 Df dosage form/	-	
8 perfalgan paracetamol	Intravenous (not otherwis	1 G gram(s)/	-	Ache
9 sedonium valeriana officinalis root	Oral	1 Df dosage form/	-	Disorder sleep
10 tavor lorazepam	Oral	1 Df dosage form/	-	Disorder sleep
11 zentropil phenytoin	Oral	3*100 Mg milligram(s)/	-	Seizure
12 zocor 20 mg simvastatin	Oral	20 Mg milligram(s)/	-	Hypercholesteremia

Primary source

Qualification: Physician

observestudytype:

Literature:

case: DE-BFARM-09003289

Patient

Reactions

Drugs (suspected or interacting)

Medical History and concurrent conditions

Primary source

563 / 678

case: DE-BFARM-09003356

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other	Pregnancy	Med. conf.
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Initials: Sex: Male Birth: privacy Age: 27 Month weight (kg): BMI int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens-Johnson syndrome	20061224	20070118	26 Day	23 Month	not recovered/not resolved
2 Necrolysis epidermal toxic (Lyell type)	20061224	20070118	26 Day	23 Month	not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 cotrimox-wolff forte saft sulfamethoxazole; trimethoprim	Oral	/	20061212-20061224 13 Day	Otitis media
2 nurofen fuer kinder fiebersaft ibuprofen	Oral	/	20061201-20061224 24 Day	Fever
3 paracetamol saft paracetamol	Oral	/	20061128-20061224 27 Day	Fever

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Common cold			k.A.	
Rhinitis			k.A.	
Otitis media			k.A.	
Fever			k.A.	
Hematoma			k.A.	
Cough			k.A.	

Primary source

Qualification:	Pharmacist	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09003393

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		Yes	Yes					

Patient

Initials:	Sex: Female	Birth: privacy	Age: 24 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	20060608	20060630	23 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 paracetamol paracetamol	Oral	/	20060501-	Head pain
2 sulfasalazin 500 mg sulfasalazine	Oral	6* /Day	-	Crohn's
			-	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09003976

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes					

Patient

Initials:	Sex: Male	Birth: privacy	Age: 13 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Erythema exsudativum multiforme	20060622	20060710	19 Day	7 Day	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 pcm supp 500 mg paracetamol	Rectal	2* /Day	20060616-20060618	Fever
			3 Day	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09003987

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes					

Patient

Initials:	Sex: Male	Birth: privacy	Age: 18 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Erythema exsudativum multiforme	20060617	20060707	21 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 belladonna globuli belladonna alkaloids	Oral	/	20060612-20060612	Throat pain
2 dolo-dobendan lt benzocaine; cetylpyridinium chloride	Oral	/	20060612-20060612	Throat pain
3 megacillin 1,5 mega phenoxymethylpenicillin potassium	Oral	2* /Day	-	Angina tonsillaris
4 paracetamol 500 mg paracetamol	Oral	/	20060613-20060613	Fever

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Asthma			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09056489

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
No		No						

Patient

Initials:	Sex:	Birth: privacy	Age: 17 Year	weight (kg): 78	BMI_int: 26.99	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Urticaria	20090310				recovered/resolved
2 Erythroderma	20090310				recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Grippostad caffeine; diphenhydramine hydrochloride; hexethylamine citrate; propyphenazone; salicylamide	Oral	1 Df dosage form/Day	20090310- -	Headache
2 Grippostad caffeine; diphenhydramine hydrochloride; hexethylamine citrate; propyphenazone; salicylamide	Oral	1 Df dosage form/Day	20090310- -	Headache
3 novaminsulfon stada metamizole	Oral	1 Df dosage form/Day	20090310- -	Headache
4 PARACETAMOL-RATIOPHARM paracetamol	Oral	1 Df dosage form/Day	20090310- -	Headache

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Limb discomfort			Unknown	
Chills			Unknown	

Primary source

Qualification: Physician observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-09074418

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		Yes

Patient

Initials:	Sex: Male	Birth: privacy	Age: 48 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Acute generalized exanthematous pustulosis	2009		6 Week	5 Day	recovered/resolved
2 Elevated liver enzyme levels	2009		6 Week	3 Day	recovered/resolved
3 Acute generalised exanthematous pustulosis			6 Week	3 Day	recovered/resolved
4 GGTP increase					recovered/resolved
5 ALT increased					recovered/resolved
6 Hepatomegaly	2009		6 Week	3 Day	recovered/resolved
7 Fever					recovered/resolved
8 Mucosal erosion			6 Week	3 Day	recovered/resolved
9 C-reactive protein increased	2009			3 Day	recovered/resolved
10 Exanthema				3 Day	recovered/resolved
11 Leukocytosis	2009			3 Day	recovered/resolved
12 Neutrophilia				3 Day	recovered/resolved
13 Hypersensitivity reaction					recovered/resolved
14 Liver disorder					recovered/resolved
15 Alanine aminotransferase increased	2009			3 Day	recovered/resolved
16 GGT increased	2009				recovered/resolved
17 Acanthosis				3 Day	Unknown
18 Gamma-glutamyltransferase increased				3 Day	Unknown
19 Pustule				3 Day	Unknown
20 Skin lesion				3 Day	Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 AMOXICILLIN amoxicillin	Unknown	/	- 3 Day	Otitis media
2 Paracetamol paracetamol	Unknown	/	- 5 Day	Otitis media

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Psoriasis			Unknown	
Otitis media			Unknown	

Primary source

Qualification:	Other Health Professional	observestudytype:
Literature:	Treudler R, Grunewald S, Gebhardt C, Simon JC. Prolonged course of acute generalized exanthematous pustulosis with liver involvement due to sensitization to amoxicillin and paracetamol. Acta Dermato-Venereologica 2009 Jan 1;89:314-5.	
Qualification:	Other Health Professional	observestudytype:
Literature:	Treudler R, Grunewald S, Gebhardt C, Simon JC. Prolonged Course of Acute Generalized Exanthematous Pustulosis with Liver Involvement due to Sensitization to Amoxicillin and Paracetamol. Acta Derm Venereol 2009; 89: 314-5	
Qualification:	Physician	observestudytype:
Literature:	Treudler R. et al. Prolonged Course of Acute Generalised Exanthematous Pustulosis with Liver Involvement due to Sensitization to Amoxicillin and Paracetamol. Acta Dermato-Venereologica 2009; 89: 314-315.	
Qualification:	Other Health Professional	observestudytype:

Literature:

Qualification: Other Health Professional observestudytype:

Literature: Treudler R, Grunewald S, Gebhardt C, Simon JC. Prolonged course of acute generalized exanthematous pustulosis with liver involvement due to sensitization to amoxicillin and paracetamol. Acta Derm Venereol 2009 Jan 1;89:314-5.

Narratives (nur bei elektronischem Eingang)JUTA, 11.06.2009 DE-JUTA-1103-09

Sender diagnosis: not reported

Narrative

A 48-year-old male patient developed acute generalized exanthematous pustulosis and raised liver function tests during treatment with amoxicillin and paracetamol. 5 days after the start of paracetamol and 3 days after the start of amoxicillin the patient developed exanthema, which was progressive despite immediate cessation of both drugs. Histology revealed slight acanthosis and subcorneal pustules filled with neutrophils. In the papillary dermis there was a mixed perivascular and interstitial infiltrate. Skin lesions were accompanied by fever, leukocytosis, neutrophilia and increased C-reactive protein. After further 3 days GGT and ALT increased two- to three-fold. Ultrasound revealed mild hepatomegaly. Patch tests for amoxicillin and paracetamol were positive. With medical treatment skin lesions, together with hepatomegaly and laboratory abnormalities improved.

Sender's comment:

Allergic skin reactions and raised liver function tests are expected adverse drug reactions for amoxicillin. The comedication is an alternative explanation. No further measures are considered to be necessary.

Reporter's comment:BETA, 16.06.2009 DE-BETA-19465-09L

Sender diagnosis: not reported

Narrative

A 48-year-old male patient with otitis media was treated with paracetamol and amoxicillin and developed a prolonged course of acute generalised exanthematous pustulosis (AGEP) with liver involvement. The patient was treated with paracetamol for otitis media, and in addition, amoxicillin was started 2 days later. Within 3 days he developed exanthema, which was progressive despite immediate cessation of both drugs. The patient was hospitalised with pustular suberythrodermic exanthema and oral erosions. Skin lesions were accompanied by fever > 39°C, leukocytosis (25/nl), neutrophilia (87%) and increased C-reactive protein level (250 mg/l). The skin lesions healed with desquamation. Hepatomegaly and laboratory abnormalities as well as the skin lesions showed a prolonged clinical course over a period of 6 weeks on administration of corticosteroid treatment. Skin testing performed 4 months later revealed positive patch tests for both amoxicillin and paracetamol. The authors state that they ascribe the patient's symptoms to both amoxicillin and paracetamol because of the clear history in combination with positive skin tests.

Sender's comment:

ASSESSMENT for Amoxicillin: Serious / Acute generalised exanthematous pustulosis: unexpected / Elevated liver enzyme levels: expected / Hepatomegaly: expected / ASSESSMENT for Paracetamol: Serious / Acute generalised exanthematous pustulosis: unexpected / Elevated liver enzyme levels: expected / Hepatomegaly: expected

Reporter's comment:Dr. R. Pfleger Chemische Fabrik GmbH, 16.06.2009 DE-PFLEGER-000067

Sender diagnosis: not reported

Narrative

A 48-year-old male patient with otitis media was treated with paracetamol and amoxicillin and developed a prolonged course of acute generalised exanthematous pustulosis (AGEP) with liver involvement. The patient was treated with paracetamol for otitis media, and in addition, amoxicillin was started 2 days later. Within 3 days he developed exanthema, which was progressive despite immediate cessation of both drugs. The patient was hospitalised with pustular suberythrodermic exanthema and oral erosions. Skin lesions were accompanied by fever > 39°C, leukocytosis (25/nl), neutrophilia (87%) and increased C-reactive protein level (250 mg/l). The skin lesions healed with desquamation. Hepatomegaly and laboratory abnormalities as well as the skin lesions showed a prolonged clinical course over a period of 6 weeks on administration of corticosteroid treatment. Skin testing performed 4 months later revealed positive patch tests for both amoxicillin and paracetamol. The authors state that they ascribe the patient's symptoms to both amoxicillin and paracetamol because of the clear history in combination with positive skin tests.

CAUSALITY ASSESSMENT for paracetamol: Temporal Relationship: plausible, Pharmacological Relationship: plausible, Observed reaction is a Sign/Symptom of Underlying Disease: not probable, History of Observed Event without drug involvement: no, Other Risk Factors: probable, Sufficiency of Documentation: sufficient, OVERALL CAUSALITY ASSESSMENT: possible, NOTE: Other risk factor: co-suspect drug amoxicillin, REACTION ABATE AFTER STOPPING DRUG: Not applicable.<dbtext>To_Char_NARRATIVE_2</dbtext>

Sender's comment:

In agreement with the reporter the causal connection of the observed reaction to paracetamol is judged as possible. The other risk factor is the co-suspect drug amoxicillin.

Reporter's comment:

According to the reporter this is a case of a patient for whom they have good evidence (i.e. by skin testing) that AGEP was elicited by a combination of amoxicillin and paracetamol.

Dr. R. Pfleger Chemische Fabrik GmbH, 16.06.2009 DE-PFLEGER-000067

Sender diagnosis: not reported

Narrative

A 48-year-old male patient with otitis media was treated with paracetamol and amoxicillin and developed a prolonged course of acute generalised exanthematous pustulosis (AGEP) with liver involvement. The patient was treated with paracetamol for otitis media, and in addition, amoxicillin was started 2 days later. Within 3 days he developed exanthema, which was progressive despite immediate cessation of both drugs. The patient was hospitalised with pustular suberythrodermic exanthema and oral erosions. Skin lesions were accompanied by fever > 39°C, leukocytosis (25/nl), neutrophilia (87%) and increased C-reactive protein level (250 mg/l). The skin lesions healed with desquamation. Hepatomegaly and laboratory abnormalities as well as the skin lesions showed a prolonged clinical course over a period of 6 weeks on administration of corticosteroid treatment. Skin testing performed 4 months later revealed positive patch tests for both amoxicillin and paracetamol. The authors state that they ascribe the patient's symptoms to both amoxicillin and paracetamol because of the clear history in combination with positive skin tests.

CAUSALITY ASSESSMENT for paracetamol: Temporal Relationship: plausible, Pharmacological Relationship: plausible, Observed reaction is a Sign/Symptom of Underlying Disease: not probable, History of Observed Event without drug involvement: no, Other Risk Factors: probable, Sufficiency of Documentation: sufficient, OVERALL CAUSALITY ASSESSMENT: possible, NOTE: Other risk factor: co-suspect drug amoxicillin, REACTION ABATE AFTER STOPPING DRUG: Not applicable.<dbtext>To_Char_NARRATIVE_2</dbtext>

Sender's comment:

In agreement with the reporter the causal connection of the observed reaction to paracetamol is judged as possible. The other risk factor is the co-suspect drug amoxicillin.

Reporter's comment:

According to the reporter this is a case of a patient for whom they have good evidence (i.e. by skin testing) that AGEP was elicited by a combination of amoxicillin and paracetamol.

CT-Arzneimittel GmbH, 17.06.2009 DE-CT-2009CT0605704

Sender diagnosis: not reported

Narrative

A 48-year-old male patient with otitis media was treated with paracetamol and amoxicillin and developed a prolonged course of acute generalised exanthematous pustulosis (AGEP) with liver involvement.

The patient was treated with paracetamol for otitis media, and in addition, amoxicillin was started 2 days later. Within 3 days he developed exanthema, which was progressive despite immediate cessation of both drugs. The patient was hospitalised with pustular suberythrodermic exanthema and oral erosions. Skin lesions were accompanied by fever > 39°C, leukocytosis (25/nl), neutrophilia (87%) and increased C-reactive protein level (250 mg/l). The skin lesions healed with desquamation. Hepatomegaly and laboratory abnormalities as well as the skin lesions showed a prolonged clinical course over a period of 6 weeks on administration of corticosteroid treatment. Skin testing performed 4 months later revealed positive patch tests for both amoxicillin and paracetamol. The authors state that they ascribe the patient's symptoms to both amoxicillin and paracetamol because of the clear history in combination with positive skin tests. CLASSIFICATION: Adverse Event: No, Adverse Reaction: Yes, Drug / Drug Interaction: No, Abuse / Misuse: No, Pregnancy / Lactation tracking: No

Sender's comment:

The authors conclude that the patient's symptoms were triggered by both Amoxicillin and Paracetamol, because of the "clear history in combination with positive skin tests". However, only positive oral rechallenge with both drugs separately would confirm this assumption.

AGEP is not exactly worded as an undesirable effect in the Amoxicillin RSI. As various skin reactions up to serious reactions like Lyell syndrome are listed, this information is considered to be sufficient and covering also the ADR AGEP.

AGEP is not worded as an undesirable effect in the Paracetamol RSI. Despite the very widespread use of Paracetamol over many years, only single cases concerning AGEP associated with Paracetamol use are available in the literature. In most of these cases, Paracetamol wasn't the only suspected drug. AGEP is characterized by fever and generalized scarlatiniform erythema with small, sterile, nonfollicular pustules. "Hypersensitivity reactions, ranging from erythema to urticaria and anaphylactic shock" is listed as adverse effect of Paracetamol in the RSI. No actions concerning the wording of the RSI are regarded to be necessary based on current data.

Reporter's comment:

MENARINIP, 17.06.2009 DE-MEN-14305

Sender diagnosis: not reported

Narrative

This case, identified within the international literature, involved a 48-year-old male patient with otitis media, who was treated with paracetamol and amoxicillin and developed a prolonged course of acute generalised exanthematous pustulosis (AGEP) with liver involvement. The patient was treated with paracetamol for otitis media, and in addition, amoxicillin was started 2 days later. Within 3 days he developed exanthema, which was progressive despite immediate cessation of both drugs. The patient was hospitalised with pustular suberythrodermic exanthema and oral erosions. Skin lesions were accompanied by fever > 39°C, leukocytosis (25/nl), neutrophilia (87%) and increased C-reactive protein level (250 mg/l). The skin lesions healed with desquamation. Hepatomegaly and laboratory abnormalities as well as the skin lesions showed a prolonged clinical course over a period of 6 weeks on administration of corticosteroid treatment. Skin testing performed 4 months later revealed positive patch tests for both amoxicillin and paracetamol. The authors state that they ascribe the patient's symptoms to both amoxicillin and paracetamol because of the clear history in combination with positive skin tests.

Sender's comment:

Due to the close time relation and the fact that a skin test revealed a positive result for paracetamol and amoxicillin, we consider all of the events as possible related to the administration of paracetamol and amoxicillin. Acute generalised exanthematous pustulosis is not listed for paracetamol. Only hypersensitivity reactions are listed. Elevated liver enzyme levels are listed for both substances and hepatomegaly neither listed for paracetamol nor for amoxicillin. Presently no measures are deemed necessary.

Reporter's comment:

Molimin Arzneimittel GmbH, 18.06.2009 DE-MOLIMIN-000017

Sender diagnosis: not reported

Narrative

A 48-year-old male patient with otitis media was treated with paracetamol and amoxicillin and developed a prolonged course of acute generalised exanthematous pustulosis (AGEP) with liver involvement. The patient was treated with paracetamol for otitis media, and in addition, amoxicillin was started 2 days later. Within 3 days he developed exanthema, which was progressive despite immediate cessation of both drugs. The patient was hospitalised with pustular suberythrodermic exanthema and oral erosions. Skin lesions were accompanied by fever > 39°C, leukocytosis (25/nl), neutrophilia (87%) and increased C-reactive protein level (250 mg/l). The skin lesions healed with desquamation. Hepatomegaly and laboratory abnormalities as well as the skin lesions showed a prolonged clinical course over a period of 6 weeks on administration of corticosteroid treatment. Skin testing performed 4 months later revealed positive patch tests for both amoxicillin and paracetamol. The authors state that they ascribe the patient's symptoms to both amoxicillin and paracetamol because of the clear history in combination with positive skin tests.

CAUSALITY ASSESSMENT for paracetamol: Temporal Relationship: plausible, Pharmacological Relationship: plausible, Observed reaction is a Sign/Symptom of Underlying Disease: not probable, History of Observed Event without drug involvement: no, Other Risk Factors: probable, Sufficiency of Documentation: sufficient, OVERALL CAUSALITY ASSESSMENT: possible, NOTE: Other risk factor: co-suspect drug amoxicillin, REACTION ABATE AFTER STOPPING DRUG: Not applicable.<dbtext>To_Char_NARRATIVE_2</dbtext>

Sender's comment:

In agreement with the reporter the causal connection of the observed reaction to paracetamol is judged as possible. The other risk factor is the co-suspect drug amoxicillin.

Reporter's comment:

According to the reporter this is a case of a patient for whom they have good evidence (i.e. by skin testing) that AGEP was elicited by a combination of amoxicillin and paracetamol.

Krewel Meuselbach GmbH, 18.06.2009 DE-KREWEL-001510

Sender diagnosis: not reported

Narrative

A 48-year-old male patient with otitis media was treated with paracetamol and amoxicillin and developed a prolonged course of acute generalised exanthematous pustulosis (AGEP) with liver involvement. The patient was treated with paracetamol for otitis media, and in addition, amoxicillin was started 2 days later. Within 3 days he developed exanthema, which was progressive despite immediate cessation of both drugs. The patient was hospitalised with pustular suberythrodermic exanthema and oral erosions. Skin lesions were accompanied by fever > 39°C, leukocytosis (25/nl), neutrophilia (87%) and increased C-reactive protein level (250 mg/l). The skin lesions healed with desquamation. Hepatomegaly and laboratory abnormalities as well as the skin lesions showed a prolonged clinical course over a period of 6 weeks on administration of corticosteroid treatment. Skin testing performed 4 months later revealed positive patch tests for both amoxicillin and paracetamol. The authors state that they ascribe the patient's symptoms to both amoxicillin and paracetamol because of the clear history in combination with positive skin tests.

NOTE: Other risk factor: co-suspect drug amoxicillin.<dbtext>To_Char_NARRATIVE_2</dbtext>

Sender's comment:

This German literature case report describes generalised exanthematous pustulosis after treatment with paracetamol and amoxicillin. The leading adverse reaction is not expressis verbis listed but hypersensitivity reactions including skin reactions and anaphylactic shock are listed for Captin our paracetamol containing medicinal products. Hepatomegaly is not listed but could be seen as a consequence of the hypersensitivity reaction. Because both suspected drugs showed a positive rechallenge the causality for both drugs is assessed as possible. Further measures are not necessary. The brand name of the medicinal products in this case report are not mentioned by the authors.

Reporter's comment:

MEDA, 19.06.2009 DE-MEDA-V-DE-2009-5216

Sender diagnosis: not reported

Narrative

A literature report was received by YES Services (DE-VIGILIT-401590)- literature search project VIGILIT on 05-Jun-2009.

A 48-year-old male patient with otitis media was treated with paracetamol and amoxicillin and developed a prolonged course of acute generalised exanthematous pustulosis (AGEP) with liver involvement. The patient was treated with paracetamol for otitis media, and in addition, amoxicillin was started 2 days later. Within 3 days he developed exanthema, which was progressive despite immediate cessation of both drugs. The patient was hospitalised with pustular suberythrodermic exanthema and oral erosions. Skin lesions were accompanied by fever > 39°C, leukocytosis (25/nl), neutrophilia (87%) and increased C-reactive protein level (250 mg/l). The skin lesions healed with desquamation. Hepatomegaly and laboratory abnormalities as well as the skin lesions showed a prolonged clinical course over a period of 6 weeks on administration of corticosteroid treatment. Skin testing performed 4 months later revealed positive patch tests for both amoxicillin and paracetamol. The authors state that they ascribe the patient's symptoms to both amoxicillin and paracetamol because of the clear history in combination with positive skin tests.

Treudler R, Grunewald S, Gebhardt C, Simon JC. Prolonged Course of Acute Generalized Exanthematous Pustulosis with Liver Involvement due to Sensitization to Amoxicillin and Paracetamol. Acta Derm Venereol 2009; 89: 314-5

Sender's comment:

MEDA comment:

Serious report from literature search project VIGILIT. The case is serious due to the hospitalization. Acute generalised exanthematous pustulosis, elevated liver enzyme levels and hepatomegaly are considered listed for amoxicillin. The event occurred with a plausible temporal relationship to the administration of drug. Furthermore, patient was recovering after corticosteroid treatment. Skin testing performed 4 months later revealed positive patch tests for amoxicillin but also for the concomitant drug paracetamol. It is considered possible that each of the products individually, or in combination, have led to the reported reaction. Causality assessed as possible.

Report from literature, company causality assessment compiled by an expert of the literature search project VIGILIT (YES Services):

CAUSALITY ASSESSMENT for amoxicillin:

Temporal Relationship: plausible

Pharmacological Relationship: plausible

Observed reaction is a Sign/Symptom of Underlying Disease: not probable

History of Observed Event without drug involvement: no

Other Risk Factors: probable

Sufficiency of Documentation: sufficient

OVERALL CAUSALITY ASSESSMENT: possible

NOTE: Other risk factor: co-suspect drug paracetamol

REACTION ABATE AFTER STOPPING DRUG: Not applicable

Reporter's comment:

The authors state that they ascribe the patient's symptoms to both amoxicillin and paracetamol because of the clear history in combination with positive skin tests.

AbZ Pharma GmbH, 19.06.2009 DE-ABZ-2009A0605705

Sender diagnosis: not reported

Narrative

A 48-year-old male patient with otitis media was treated with paracetamol and amoxicillin and developed a prolonged course of acute generalised exanthematous pustulosis (AGEP) with liver involvement.

The patient was treated with paracetamol for otitis media, and in addition, amoxicillin was started 2 days later. Within 3 days he developed exanthema, which was progressive despite immediate cessation of both drugs. The patient was hospitalised with pustular suberythrodermic exanthema and oral erosions. Skin lesions were accompanied by fever > 39°C, leukocytosis (25/nl), neutrophilia (87%) and increased C-reactive protein level (250 mg/l).

The skin lesions healed with desquamation. Hepatomegaly and laboratory abnormalities as well as the skin lesions showed a prolonged clinical course over a period of 6 weeks on administration of corticosteroid treatment. Skin testing performed 4 months later revealed positive patch tests for both amoxicillin and paracetamol. The authors state that they ascribe the patient's symptoms to both amoxicillin and paracetamol because of the clear history in combination with positive skin tests. CLASSIFICATION: Adverse Event: No, Adverse Reaction: Yes, Drug / Drug Interaction: No, Abuse / Misuse: No, Pregnancy / Lactation tracking: No

Sender's comment:

The authors conclude that the patient's symptoms were triggered by both Amoxicillin and Paracetamol, because of the "clear history in combination with positive skin tests". However, only positive oral rechallenge with both drugs separately would confirm this assumption.

AGEP is not exactly worded as an undesirable effect in the Amoxicillin RSI. As various skin reactions up to serious reactions like Lyell syndrome are listed, this information is considered to be sufficient and covering also the ADR AGEP.

AGEP is not worded as an undesirable effect in the Paracetamol RSI. Despite the very widespread use of Paracetamol over many years, only single cases concerning AGEP associated with Paracetamol use are available in the literature. In most of these cases, Paracetamol wasn't the only suspected drug. AGEP is characterized by fever and generalized scarlatiniform erythema with small, sterile, nonfollicular pustules. "Hypersensitivity reactions, ranging from erythema to urticaria and anaphylactic shock" is listed as adverse effect of Paracetamol in the RSI. No actions concerning the wording of the RSI are regarded to be necessary based on current data.

Reporter's comment:

ratiopharm GmbH, 19.06.2009 DE-RATIOPH-2009R0605642

Sender diagnosis: not reported

Narrative

A 48-year-old male patient with otitis media was treated with paracetamol and amoxicillin and developed a prolonged course of acute generalised exanthematous pustulosis (AGEP) with liver involvement.

The patient was treated with paracetamol for otitis media, and in addition, amoxicillin was started 2 days later. Within 3 days he developed exanthema, which was progressive despite immediate cessation of both drugs. The patient was hospitalised with pustular suberythrodermic exanthema and oral erosions. Skin lesions were accompanied by fever > 39°C, leukocytosis (25/nl), neutrophilia (87%) and increased C-reactive protein level (250 mg/l).

The skin lesions healed with desquamation. Hepatomegaly and laboratory abnormalities as well as the skin lesions showed a prolonged clinical course over a period of 6 weeks on administration of corticosteroid treatment. Skin testing performed 4 months later revealed positive patch tests for both amoxicillin and paracetamol. The authors state that they ascribe the patient's symptoms to both amoxicillin and paracetamol because of the clear history in combination with positive skin tests. CLASSIFICATION: Adverse Event: No, Adverse Reaction: Yes, Drug / Drug Interaction: No, Abuse / Misuse: No, Pregnancy / Lactation tracking: No

Sender's comment:

The authors conclude that the patient's symptoms were triggered by both Amoxicillin and Paracetamol, because of the "clear history in combination with positive skin tests". However, only positive oral rechallenge with both drugs separately would confirm this assumption.

AGEP is not exactly worded as an undesirable effect in the Amoxicillin RSI. As various skin reactions up to serious reactions like Lyell syndrome are listed, this information is considered to be sufficient and covering also the ADR AGEP.

AGEP is not worded as an undesirable effect in the Paracetamol RSI. Despite the very widespread use of Paracetamol over many years, only single cases concerning AGEP associated with Paracetamol use are available in the literature. In most of these cases, Paracetamol wasn't the only suspected drug. AGEP is characterized by fever and generalized scarlatiniform erythema with small, sterile, nonfollicular pustules. "Hypersensitivity reactions, ranging from erythema to urticaria and anaphylactic shock" is listed as adverse effect of Paracetamol in the RSI. No actions concerning the wording of the RSI are regarded to be necessary based on current data.

Reporter's comment:

SANOFI-SYNTHELABO, 22.06.2009 DE-SANOFI-SYNTHELABO-A04200900810

Sender diagnosis: not reported

Narrative

Reaction as reported: acute generalised exanthematous pustulosis
 LLT: ACUTE GENERALISED EXANTHEMATOUS PUSTULOSIS / 10048799
 PT: ACUTE GENERALISED EXANTHEMATOUS PUSTULOSIS / 10048799

Reaction as reported: elevated liver enzyme levels (2 to 3-fold)
 LLT: ELEVATED LIVER ENZYME LEVELS / 10014480
 PT: HEPATIC ENZYME INCREASED / 10060795

Reaction as reported: hepatomegaly
 LLT: HEPATOMEGALY / 10019842
 PT: HEPATOMEGALY / 10019842

Reaction as reported: oral erosions
 LLT: ORAL ULCERATION / 10031027
 PT: MOUTH ULCERATION / 10028034

Reaction as reported: fever (>39 degrees C)
 LLT: FEVER / 10016558
 PT: PYREXIA / 10037660

Narrative:

Initial report:

This serious case is a literature report about acute generalized exanthematous pustulosis under amoxicillin and paracetamol in a male.
 Case report: A 48-year-old male patient with no history of psoriasis was treated with paracetamol and amoxicillin (no trade names provided) due to otitis media and developed a prolonged course of acute generalised exanthematous pustulosis (AGEP) with elevated liver enzyme levels (nos) and hepatomegaly. The patient was treated with paracetamol for otitis media, and in addition, amoxicillin was started 2 days later. Within 3 days he developed exanthema, which was progressive despite immediate cessation of both drugs. The patient was hospitalised with pustular suberythrodermic exanthema and oral erosions. Skin lesions were accompanied by fever >39 degrees C, leukocytosis (25/nl), neutrophilia (87%) and increased C-reactive protein level (250 mg/l). The skin lesions healed with desquamation. Hepatomegaly and laboratory abnormalities as well as the skin lesions showed a prolonged clinical course over a period of 6 weeks on administration of corticosteroid treatment. Skin testing performed 4 months later revealed positive patch tests for both amoxicillin and paracetamol. The adverse reactions reappeared after reintroduction (details were not provided). The authors state that they ascribe the patient's symptoms to both amoxicillin and paracetamol because of the clear history in combination with positive skin tests.

Addendum for follow-up received 15-Jun-2009

The full article was received and the case report could be amended as follows:

The patient was admitted to hospital on day six after start of therapy and a therapy with prednisolone-succinate (250 mg/day for two days) was initiated. On day seven he was referred to the reporters university hospital with pustular suberythrodermic exanthema and oral erosions. Histology from the trunk revealed slight acanthosis and subcorneal pustules filled with neutrophils. In the papillary dermis there was a mixed perivascular and interstitial infiltrate of neutrophils, eosinophils and lymphocytes.

On day 8, eosinophils and liver enzymes were within normal range but gamma-glutamyl transferase (GGT) and alanine aminotransferase (ALT) then increased two- to threefold, respectively, with a maximum on day 11 and only slight improvement until discharge on day 21. Serology was negative for acute viral infections (hepatitis A, B, C, Epstein-Barr virus, cytomegalovirus, human herpes virus (HHV)-6, parovirus B19). The serum level of interleukin-17 was below the detection level (< 2.5 pg/ml). Ultrasound revealed mild hepatomegaly but no lymphadenopathy. The content of the pustules was sterile, and blood culture was negative.

Treatment included high-dose oral methylprednisolone (starting on day 7 with 160 mg, with slowly decreasing dosages) in combination with topical corticosteroids.

The positive patch tests with both drugs (5% in petrolatum) revealed papulopustular infiltrates reminiscent of typical AGEP lesions. A prick test was positive at late readings for amoxicillin (papular infiltrate of 5 mm at day 2) but not indicative for paracetamol (both pure in aqueous solution). The patient was advised to avoid both drugs in the future.

Sender's comment:

Start of the reaction 3 days after initiation of treatment is too short for primary sensitization, 5 days is borderline. Also duration is unusual: AGEP is considered a self-limited condition with resolution of pustules within less than 15 days. The differential diagnosis of AGEP includes a wide range of diseases or reactions that can cause pustular eruptions of the skin. Based on chronology and the authors' opinion, the causal role of amoxicillin and paracetamol cannot be ruled out. The causal assessment of AGEP in this case is confounded by the concomitant administration of amoxicillin and paracetamol. 15-Jun-2009 Sanofi Aventis Germany: Acute generalised exanthematous pustulosis is considered locally unlabelled for the suspected product. The information presented here, however, shows unusual features for an AGEP. This case does not imply a change in the state of the scientific knowledge for the product. Measures according to Para 29 German Drug Law (AMG), section (1) concerning the approval status are not deemed necessary.

Reporter's comment:Temmler Pharma GmbH & Co. KG, 23.06.2009 DE-TEMMLER06-003457

Sender diagnosis: not reported

Narrative

A 48-year-old male patient with otitis media was treated with paracetamol and amoxicillin and developed a prolonged course of acute generalised exanthematous pustulosis (AGEP) with liver involvement. The patient was treated with paracetamol for otitis media, and in addition, amoxicillin was started 2 days later. Within 3 days he developed exanthema, which was progressive despite immediate cessation of both drugs. The patient was hospitalised with pustular suberythrodermic exanthema and oral erosions. Skin lesions were accompanied by fever > 39°C, leukocytosis (25/nl), neutrophilia (87%) and increased C-reactive protein level (250 mg/l). The skin lesions healed with desquamation. Hepatomegaly and laboratory abnormalities as well as the skin lesions showed a prolonged clinical course over a period of 6 weeks on administration of corticosteroid treatment. Skin testing performed 4 months later revealed positive patch tests for both amoxicillin and paracetamol.

Sender's comment:

Co-suspected Drug is amoxicillin

Reporter's comment:

The authors state that they ascribe the patient's symptoms to both amoxicillin and paracetamol because of the clear history in combination with positive skin tests.

SANOFI-SYNTHELABO, 24.06.2009 DE-SANOFI-SYNTHELABO-A04200900810

Sender diagnosis: not reported

Narrative

Reaction as reported: acute generalised exanthematous pustulosis
 LLT: ACUTE GENERALISED EXANTHEMATOUS PUSTULOSIS / 10048799
 PT: ACUTE GENERALISED EXANTHEMATOUS PUSTULOSIS / 10048799

Reaction as reported: elevated liver enzyme levels (2 to 3-fold)
 LLT: ELEVATED LIVER ENZYME LEVELS / 10014480
 PT: HEPATIC ENZYME INCREASED / 10060795

Reaction as reported: hepatomegaly
 LLT: HEPATOMEGALY / 10019842
 PT: HEPATOMEGALY / 10019842

Reaction as reported: oral erosions
 LLT: ORAL ULCERATION / 10031027
 PT: MOUTH ULCERATION / 10028034

Reaction as reported: fever (>39 degrees C)
 LLT: FEVER / 10016558
 PT: PYREXIA / 10037660

Narrative:

Initial report:

This serious case is a literature report about acute generalized exanthematous pustulosis under amoxicillin and paracetamol in a male.

Case report: A 48-year-old male patient with no history of psoriasis was treated with paracetamol and amoxicillin (no trade names provided) due to

otitis media and developed a prolonged course of acute generalised exanthematous pustulosis (AGEP) with elevated liver enzyme levels (nos) and hepatomegaly. The patient was treated with paracetamol for otitis media, and in addition, amoxicillin was started 2 days later. Within 3 days he developed exanthema, which was progressive despite immediate cessation of both drugs. The patient was hospitalised with pustular suberythrodermic exanthema and oral erosions. Skin lesions were accompanied by fever >39 degrees C, leukocytosis (25/nl), neutrophilia (87%) and increased C-reactive protein level (250 mg/l). The skin lesions healed with desquamation. Hepatomegaly and laboratory abnormalities as well as the skin lesions showed a prolonged clinical course over a period of 6 weeks on administration of corticosteroid treatment. Skin testing performed 4 months later revealed positive patch tests for both amoxicillin and paracetamol. The adverse reactions reappeared after reintroduction (details were not provided). The authors state that they ascribe the patient's symptoms to both amoxicillin and paracetamol because of the clear history in combination with positive skin tests.

Addendum for follow-up received 15-Jun-2009

The full article was received and the case report could be amended as follows:

The patient was admitted to hospital on day six after start of therapy and a therapy with prednisolone-succinate (250 mg/day for two days) was initiated. On day seven he was referred to the reporters university hospital with pustular suberythrodermic exanthema and oral erosions. Histology from the trunk revealed slight acanthosis and subcorneal pustules filled with neutrophils. In the papillary dermis there was a mixed perivascular and interstitial infiltrate of neutrophils, eosinophils and lymphocytes.

On day 8, eosinophils and liver enzymes were within normal range but gamma-glutamyl transferase (GGT) and alanine aminotransferase (ALT) then increased two- to threefold, respectively, with a maximum on day 11 and only slight improvement until discharge on day 21. Serology was negative for acute viral infections (hepatitis A, B, C, Epstein-Barr virus, cytomegalovirus, human herpes virus (HHV)-6, parovirus B19). The serum level of interleukin-17 was below the detection level (< 2.5 pg/ml). Ultrasound revealed mild hepatomegaly but no lymphadenopathy. The content of the pustules was sterile, and blood culture was negative.

Treatment included high-dose oral methylprednisolone (starting on day 7 with 160 mg, with slowly decreasing dosages) in combination with topical corticosteroids.

The positive patch tests with both drugs (5% in petrolatum) revealed papulopustular infiltrates reminiscent of typical AGEP lesions. A prick test was positive at late readings for amoxicillin (papular infiltrate of 5 mm at day 2) but not indicative for paracetamol (both pure in aqueous solution).

The patient was advised to avoid both drugs in the future.

Sender's comment:

Start of the reaction 3 days after initiation of treatment is too short for primary sensitization, 5 days is borderline. Also duration is unusual: AGEP is considered a self-limited condition with resolution of pustules within less than 15 days. The differential diagnosis of AGEP includes a wide range of diseases or reactions that can cause pustular eruptions of the skin. Based on chronology and the authors' opinion, the causal role of amoxicillin and paracetamol cannot be ruled out. The causal assessment of AGEP in this case is confounded by the concomitant administration of amoxicillin and paracetamol. 15-Jun-2009 Sanofi Aventis Germany: Acute generalised exanthematous pustulosis is considered locally unlabelled for the suspected product. The information presented here, however, shows unusual features for an AGEP. This case does not imply a change in the state of the scientific knowledge for the product. Measures according to Para 29 German Drug Law (AMG), section (1) concerning the approval status are not deemed necessary.

Reporter's comment:

NOVARTIS, 02.07.2009 DE-002147023-PHHY2009DE24689

Sender diagnosis: not reported

Narrative

Initial literature report was received on 17 Jun 2009: (This patient was included in the study "Two cases of drug-induced subacute cutaneous lupus erythematosus on simvastatin and carbamazepine"). This patient had a five year history of papular skin changes on areas exposed to light. He had not been previously investigated. Within the two previous years, there had been massive progression, and at the time of his initial consultation he was noted to have marked clinical signs with numerous areas of hyperkeratosis and atrophy. He additionally had epilepsy. Two years previously, he has been started on carbamazepine for this (manufacturer unknown). In view of the typical histology, the autoimmune serology and the history of deterioration in the skin signs (occurring at the same time when patient was put on carbamazepine), the reporter diagnosed drug-induced subacute cutaneous lupus erythematosus (SCLE), or queried possible drug-exacerbated SCLE. When carbamazepine was stopped, there was subsequently good regression of the findings. The authors concluded stating that prescription in patients with SCLE should be critically appraised in relation to a possible exacerbation of the underlying disease.

Sender's comment:

Serious literature report, Cutaneous lupus erythematosus [medically significant], assessed as listed according to the Core Data Sheet.

All literature reports are considered suspected for reporting purposes or in accordance with national regulatory requirements.

Reporter's comment:

Boehringer Ingelheim GmbH, Germany, 13.07.2009 DE-Boehringer Ingelheim GmbH, Germany-2009-DE-04771GD

Sender diagnosis: not reported

Narrative

A case of severe acute generalized exanthematous pustulosis (AGEP) is reported that was elicited by a combined sensitization to amoxicillin and paracetamol. The subject showed unusual liver involvement and prolonged course of the disease (complete recovery after 6 weeks). The 48-year-old man with no history of psoriasis was treated with paracetamol because of otitis media. In addition, amoxicillin was started 2 days later and within 3 more days he developed exanthema, which was progressive despite immediate cessation of both drugs on day 5 after start of drug therapy, hospital admission on day 6 and therapy with prednisolone-succinate (250 mg/day for 2 days). On day 7 he was referred to an university hospital with pustular suberythrodermic exanthema and oral erosions. Histology from the trunk revealed slight acanthosis and subcorneal pustules filled with neutrophils. In the papillary dermis there was a mixed perivascular and interstitial infiltrate of neutrophils, eosinophils and lymphocytes. Skin lesions were accompanied by fever >39 degC, leukocytosis (25/nl), neutrophilia (87%) and increased C-reactive protein level (CRP) (250 mg/l). On day 8, eosinophils and liver enzymes were within normal range, but gamma-gluta-myl transferase (GGT) and alanine aminotransferase (ALT) then increased two- to three-fold, respectively, with a maximum on day 11 and only slight improvement until discharge on day 21. Serology was negative for acute viral infections (hepatitis A, B, C, Epstein-Barr virus, cytomegalovirus, human herpes virus (HHV)-6, parvovirus B19). The serum level of interleukin-17 was below the detection level (<2.5 pg/ml). Ultrasound revealed mild hepatomegaly but no lymphadenopathy. The content of the pustules was sterile, and blood culture was negative. The case was reported to the German Registry of Severe Skin Reactions (<http://www.uniklinik-freiburg.de/hautklinik/live/dzh.html>) and was classified as 'probable' AGEP by an associated expert panel. Treatment included high-dose oral methylprednisolone (starting on day 7 with 160 mg, with slowly decreasing dosages) in combination with topical corticosteroids. The skin lesions healed with desquamation and showed a prolonged clinical course over a period of 6 weeks, together with hepatomegaly and laboratory abnormalities. Skin testing performed 4 months later revealed positive patch tests for both amoxicillin and paracetamol (both 5% in petrolatum, day 3) with papulo pustular infiltrates reminiscent of typical AGEP lesions. A prick test was positive at late readings for amoxicillin (papular infiltrate of 5 mm at day 2) but not indicative for paracetamol (both pure in aqueous solution). The patient was advised to avoid both drugs in the future. It is concluded that the AGEP in this patient was unique because of: (i) the prolonged course of skin and liver involvement; and (ii) the combined sensitization to two drugs, including (iii) paracetamol, which was recently assigned as having no significant association with AGEP.

Sender's comment:

Regional Centre Drug Safety Boehringer Ingelheim
Comment on case 2009-DE-04771GD / PARACETAMOL

The event ACUTE GENERALIZED EXANTHEMATOUS PUSTULOSIS and its consequent signs and symptoms in this case report are not listed adverse drug reactions for the product in question. ACUTE GENERALIZED EXANTHEMATOUS PUSTULOSIS is a listed event for the concomitant medication amoxicillin, and this is a more plausible explanation for the occurrence of the event.

As with all unlisted events, the company will continue to monitor all respective reports received and, based on cumulative experience, will re-evaluate the available evidence on an ongoing basis. No change in the company core safety information or other measures is recommended at this time.

E_SHIBEIJU

Case assessment created by E_SHIBEIJU on 10 Jul 2009

Reporter's comment:

GRUNENTHAL, 21.07.2009 DE-GRUNENTHAL-2009-12149

Sender diagnosis: not reported

Narrative

SUBJECT/PATIENT ID

07.03.2016

BfArM - Pharmacovigilance

579 / 678

Manufacturer Control Number: 2009-12149

ADVERSE EVENT DETAILS

This literature case from GERMANY was reported by a physician via other and was initially received on 10-Jul-2009.

It concerns a 48-year-old male subject/patient (height unknown / weight unknown).

The relevant medical history included otitis media.

Other suspected medication paracetamol.

The patient with no history of psoriasis was treated with paracetamol because of otitis media, in addition, amoxicillin was started 2 days later and within 3 more days he developed exanthema, which was progressive despite immediate cessation of both drugs on day 5 after start of drug therapy, hospital admission on day 6 and therapy with prednisolone-succinate (250 mg/day for 2 days). On day 7 he was referred to hospital with pustular suberythrodermic exanthema and oral erosions. Histology from the trunk revealed slight acanthosis and subcorneal pustules filled with neutrophils. Skin lesions were accompanied by fever, more than 39°C, leukocytosis (25/nl), neutrophilia (87%) and increased C-reactive protein level (CRP) (250 mg/l). On day 8, eosinophils and liver enzymes were within normal range, but gamma-glutamyl transferase (GGT) and alanine aminotransferase (ALT) then increased two- to three-fold, respectively, with a maximum on day 11 and only slight improvement until discharge on day 21. Serology was negative for acute viral infections (hepatitis A, B, C, Epstein-Bart virus, cytomegalovirus, human herpes virus (HHV)-6, parvovirus B 19). The serum level of interleukin-17 was below the detection level (less than 2.5 pg/ml). Ultrasound revealed mild hepatomegaly but no lymphadenopathy. Treatment included high dose oral methylprednisolone (starting on day 7 with 160 mg, with slowly decreasing dosages) in combination with topical corticosteroids. The skin lesions healed with desquamation and showed a prolonged clinical course over a period of 6 weeks, together with hepatomegaly and laboratory abnormalities. Skin testing performed 4 months later revealed positive patch test for both amoxicillin and paracetamol.

Third party case numbers

GRT LitDB AccNo: 624033

Sender's comment:

Acute generalised exanthematous pustulosis and hepatic enzyme increased are assessed as serious.

Acute generalised exanthematous pustulosis and hepatic enzyme increased are listed in the CCDS of amoxicillin.

Acute generalised exanthematous pustulosis and hepatic enzyme increased are assessed to be possibly related to the administration of amoxicillin.

The information in this single case does not appear to alter the favourable benefit to risk ratio of amoxicillin.

There is a plausible temporal relationship as well as a known response pattern of amoxicillin regarding acute generalised exanthematous pustulosis and hepatic enzyme increased.

Therefore, the events are assessed to be possibly related to the administration of amoxicillin.

Reporter's comment:

GlaxoSmithKline, 21.07.2009 DE-GlaxoSmithKline-B0584328A

Sender diagnosis: not reported

Narrative

This case was reported in a literature article and described the occurrence of acute generalized exanthematous pustulosis (AGEP) in a 48-year-old male patient who received amoxicillin trihydrate (Amoxicillin; brand name/trade name unknown) for an otitis media.

Co-suspect medication included Paracetamol. The patient had no history of psoriasis.

On an unknown date, the patient started paracetamol and approximately 2 days later started amoxicillin trihydrate (both unknown dosage regimen). Approximately 3 days after starting amoxicillin trihydrate, the patient developed exanthema. Treatment with paracetamol and amoxicillin trihydrate was discontinued immediately and the following day the patient was hospitalised. Despite treatment with prednisolone succinate at 2 mg daily for 2 days the exanthema became progressively worse. On the same day, the patient was treated with oral methylprednisolone, starting at 160 mg with decreasing dosages, in combination with topical corticosteroids.

The following day, the patient was referred to another hospital with pustular suberythroderma exanthema and oral erosions. Histology from the trunk revealed slight acanthiosis and subcorneal pustules filled with neutrophils. In the papillary dermis there was a mixed perivascular and interstitial infiltrate of neutrophils, eosinophils and lymphocytes. Skin lesions were accompanied by a fever of greater than 39 °C, leukocytosis (25 /nl), neutrophilia (87%) and increased C-reactive protein level (250 mg/l). The following day, eosinophils and liver enzymes were within normal range, but GGT and alanine aminotransferase increased two to three fold respectively. Serology was negative for acute viral infections (hepatitis A, B, C, Epstein Barr virus, cytomegalovirus, human herpes virus-6, parovirus B19). The serum level of interleukin-17 was below the detection level (less than 2.5 pg/ml) and ultrasound revealed mild hepatomegaly but no lymphadenopathy. The content of the pustules was sterile and blood culture was negative. The patient was discharged on day 21 after starting paracetamol.

The skin lesions healed with desquamation and showed a prolonged clinical course over the period of 6 weeks together with hepatomegaly and laboratory abnormalities. 4 months later, skin testing revealed positive patch tests for both amoxicillin and paracetamol with papulopustular infiltrates reminiscent of typical AGEP lesions. A prick test was positive at late readings for amoxicillin but not indicative for paracetamol. At the time of reporting, the events were resolved.

The author considered the events were probably related to treatment with the suspect drugs commenting "The case was reported to the German Registry of Severe Skin Reactions and was classified as probable AGEP by an associated expert panel" and "because of the clear history in combination with positive skin tests, we contributed our patient's symptoms to both amoxicillin and paracetamol".

Sender's comment:

Die UAW ist erwartet

Ein Kausalzusammenhang ist moeglich

Auf der Basis dieses Einzelfalles ergibt sich keine Aenderung des wissenschaftlichen Kenntnisstandes und der Nutzen/Risiko-Relation. Weitergehende eigenverantwortliche Massnahmen halten wir derzeit nicht fuer erforderlich.

Reporter's comment:

NOVARTIS, 22.07.2009 DE-SZ09-GXKR2009DE07972

Sender diagnosis: not reported

Narrative

Initial literature report received on 13 Jul 2009. A patient developed acute generalized exanthematous pustulosis (AGEP) due to sensitization to amoxicillin and paracetamol, with an unusual liver involvement and prolonged course of the disease. The patient with no history of psoriasis received treatment with paracetamol for otitis media. Two days later, amoxicillin was started and within more 3 days patient developed exanthema, which progressed despite cessation of both drugs on day 5 after the start of drug therapy. Patient was hospitalised on day 6 and received prednisolone-succinate for 2 days. On day 7, the patient was referred to university hospital with pustular suberythrodermic exanthema and oral erosions. Histology from the trunk revealed slight acanthosis and subcorneal pustules filled with neutrophils. Skin lesions were accompanied by fever >39 C, leukocytosis, neutrophilia and increased C-reactive protein. On day 8, two- to three-fold increase in gamma-glutamyl transferase and alanine aminotransferase was detected, respectively, with a maximum on day 11 and only slight improvement until discharge on day 21. The serum level of interleukin-17 was below the detection level (<2.5 pg/ml). Ultrasound revealed mild hepatomegaly and no lymphadenopathy. Treatment included high-dose oral methylprednisolone (with slowly decreasing dosages) in combination with topical corticosteroids. Skin lesions healed with desquamation and showed a prolonged clinical course over a period of 6 weeks, together with hepatomegaly and laboratory abnormalities. The patient needed prolonged corticosteroid treatment until complete recovery after 6 weeks. Skin testing performed 4 months later revealed positive patch tests for both amoxicillin and paracetamol with papulopustular infiltrates reminiscent of typical AGEP lesions. A prick test was positive at late readings for amoxicillin but not indicative for paracetamol. The patient was advised to avoid both drugs in the future. The authors state that AGEP was unique in this patient because of the prolonged course of skin and liver involvement, combined sensitization to two drugs.

Sender's comment:

For amoxicillin: Serious literature report (hospitalisation) already listed in the Core Data Sheet Information.

For paracetamol: Serious literature report (hospitalisation) assessed as unlisted according to the Core Data Sheet Information. The information provided in this individual case does not warrant a change to the Core Data Sheet Information text.

All literature reports are considered suspected for reporting purposes.

Reporter's comment:

Bristol-Myers Squibb Company, 03.08.2009 DE-Bristol-Myers Squibb Company-14716021

Sender diagnosis: not reported

Narrative

This case was derived from a literature abstract which described about acute generalised exanthematous pustulosis (AGEP) with liver involvement, due to sensitization to acetaminophen and amoxicillin in a male patient.

A 48-year-old male patient was hospitalized for acute generalised exanthematous pustulosis (AGEP) with liver involvement, due to sensitization to acetaminophen and amoxicillin (dosages not stated). He also developed hepatomegaly, alanine aminotransferase (ALT) increased and gamma-glutamyl transferase (GGT) increased.

The man was commenced on acetaminophen for otitis media, and after 2 days, amoxicillin was initiated (therapeutic indication not stated). Three days later, he developed exanthema. Both drugs were ceased on the same day. The next day, he was hospitalized and started treatment with prednisolone succinate for 2 days. On day 7, he was transferred to another hospital with pustular suberythrodermic exanthema and oral erosions. Histology showed slight acanthosis and subcorneal pustules containing neutrophils. Perivascular and interstitial infiltrate made up of neutrophils, eosinophils and lymphocytes was present in the papillary dermis. He had a fever, and displayed neutrophilia, leucocytosis and an elevated C-reactive protein level. After day 8, there was a two - to three-fold increase in ALT and GGT levels, respectively; this peaked on day 11. An ultrasound revealed mild hepatomegaly. The reaction was classified as a "probable" AGEP by an expert panel of the German Registry of Severe Skin Reactions. Treatment with high-dose methylprednisolone was started on day 7 and slowly tapered. Topical corticosteroids were also administered. The man was discharged on day 21; at that time his GGT and ALT levels had only slightly improved. The hepatomegaly and laboratory abnormalities healed after a period of 6 weeks, as did the skin lesions, with desquamation. Four months later, skin testing yielded positive patch test results on day 3 for both amoxicillin and paracetamol. A prick test was positive for amoxicillin at day 2, but not positive for paracetamol.

Author comment: "Because of the clear history in combination with positive skin tests, we contributed our patient's symptoms to both amoxicillin and paracetamol. Definitive confirmation as the cause of AGEP could only be obtained by re-challenging the patient, which is, however, ethically not justifiable in severe cases like the one reported here."

Literature citation: Treudler R, Grunewald S, Gebhardt C, Simon JC. Prolonged course of acute generalized exanthematous pustulosis with liver involvement due to sensitization to amoxicillin and paracetamol. Acta Dermato-Venereologica 2009 Jan 1;89:314-5.

BMS medical evaluation comment:

This patient developed acute generalised exanthematous pustulosis after therapy with amoxicillin and acetaminophen. In the clinical context of symptom evolution and positive patch test results for both amoxicillin and acetaminophen, a causal role of these medications remains possible.

Sender's comment:

This report is submitted to you in the name of the marketing authorisation holder Bristol-Myers Squibb GmbH & Co. KGaA.

Acetaminophen

Event number; label*; causality**

#1; unexpected; PSB

* according to current Summary of Product Characteristics

** DEF=definite, PRB=probable, PSB=possible, NOL=not likely, NOR=not related

Change of current scientific knowledge: no.

Comments: This patient developed acute generalised exanthematous pustulosis after therapy with amoxicillin and acetaminophen. The occurrence of erythema, pruritus and flush is expected according to the current SPC of PERFALGAN, however AGEP is not specified. In the clinical context of symptom evolution and positive patch test results for both amoxicillin and acetaminophen, a causal role of these medications remains possible.

Reporter's comment:

RANBAXY, 05.08.2009 DE-RANBAXY-2009RR-25985

Sender diagnosis: not reported

Narrative

This case, manufacturer control number 2009RR-25985 from GERMANY refers to a 48 year old male who developed acute generalised exanthematous pustulosis with liver involvement following treatment with amoxicillin and paracetamol that required hospitalisation.

This case was reported in the literature.

The subject received the following suspect products:

Amoxicillin: patch test and prick test positive. The drug was withdrawn.

Paracetamol: for otitis media. Patch test positive, prick test negative. The drug was withdrawn.

The patient received the following treatment products:

Prednisolone succinate: 2 days.

Methylprednisolone: high-dose.

Topical corticosteroids.

The patient developed acute generalised exanthematous pustulosis with liver involvement.

A 48-year-old man developed acute generalised exanthematous pustulosis (AGEP) with liver involvement, due to sensitisation to paracetamol [acetaminophen] and amoxicillin [dosages not stated].

The man was commenced on paracetamol for otitis media, and after 2 days, amoxicillin was initiated [therapeutic indication not stated]. Three days later, he developed exanthema. Both drugs were ceased on the same day. The next day, he was hospitalised and started treatment with prednisolone succinate for 2 days. On day 7, he was transferred to another hospital with pustular suberythrodermic exanthema and oral erosions. Histology showed slight acanthosis and subcorneal pustules containing neutrophils. Perivascular and interstitial infiltrate made up of neutrophils, eosinophils and lymphocytes was present in the papillary dermis. He had a fever, and displayed neutrophilia, leucocytosis and an elevated C-reactive protein level. After day 8, there was a two- to three-fold increase in ALT and -glutamyl transferase (GGT) levels, respectively; this peaked on day 11. An ultrasound revealed mild hepatomegaly. The reaction was classified as a "probable" AGEP by an expert panel of the German Registry of Severe Skin Reactions.

Treatment with high-dose methylprednisolone was started on day 7 and slowly tapered. Topical corticosteroids were also administered. The man was discharged on day 21; at that time his GGT and ALT levels had only slightly improved. The hepatomegaly and laboratory abnormalities healed after a period of 6 weeks, as did the skin lesions, with desquamation.

Four months later, skin testing yielded positive patch test results on day 3 for both amoxicillin and paracetamol. A prick test was positive for amoxicillin at day 2, but not positive for paracetamol.

The outcome of the events was reported as "healed" at the time of this report.

The reporter considered the case serious because the patient was hospitalised.

Author's Comment: "Because of the clear history in combination with positive skin tests, we contributed [sic] our patient's symptoms to both amoxicillin and paracetamol. . . Definitive confirmation as the cause of AGEP could only be obtained by re-challenging the patient, which is, however, ethically not justifiable in severe cases like the one reported here."

Company comment:

The case is rated as serious and unexpected and possible.

Acute generalized exanthematous pustulosis (AGEP) is an uncommon cutaneous eruption that is most often provoked by drugs, by acute infections with enteroviruses, or by mercury. It is characterized by acute, extensive formation of nonfollicular sterile pustules on an erythematous background, fever and elevated numbers of blood neutrophils. Involvement of T cells in drug-induced AGEP was suggested by positive patch tests and lymphocyte transformation tests. Moreover, drug-specific CD4+ and CD8+ T cells could be isolated and propagated in vitro from patch test sites and blood from AGEP patients.

Most cases of AGEP are drug induced, particularly by antibiotics and mainly beta-lactams.

Because of the clear history in combination with positive skin tests, both amoxicillin and paracetamol contributed to this patient's symptoms. Definitive confirmation as the cause of AGEP could only be obtained by re-challenging the patient, which is, however, ethically not justifiable in severe cases like the one reported here.

Reference:

Britschgi M, Pichler WJ. Acute generalized exanthematous pustulosis, a clue to neutrophil-mediated inflammatory processes orchestrated by T cells. Curr Opin Allergy Clin Immunol 2002 Aug;2(4):325-31.

Treudler R, Grunewald S, Gebhardt C, Simon J-C. Prolonged course of acute generalized exanthematous pustulosis with liver involvement due to sensitization to amoxicillin and paracetamol. Acta Dermato-Venereologica 89: 314-315, No. 3, 2009.

Sender's comment:

Reporter's comment:

Generics [UK] Ltd, 07.08.2009 DE-MYLANLABS-2009S1013253

Sender diagnosis: not reported

Narrative

Treudler R, Grunewald S, Gebhardt C, Simon J-C. Prolonged course of acute generalized exanthematous pustulosis with liver involvement due to sensitization to amoxicillin and paracetamol. Acta-Derm-Venereol 2009; 89(3) 314-315

A 48-year-old man developed acute generalised exanthematous pustulosis (AGEP) with liver involvement, due to sensitisation to paracetamol [acetaminophen] and amoxicillin [dosages not stated]. The man was commenced on paracetamol for otitis media, and after 2 days, amoxicillin was initiated [therapeutic indication not stated]. Three days later, he developed exanthema. Both drugs were ceased on the same day. The next day, he was hospitalised and started treatment with prednisolone succinate for 2 days. On day 7, he was transferred to another hospital with pustular suberythrodermic exanthema and oral erosions. Histology showed slight acanthosis and subcorneal pustules containing neutrophils. Perivascular and interstitial infiltrate made up of neutrophils, eosinophils and lymphocytes was present in the papillary dermis. He had a fever, and displayed neutrophilia, leucocytosis and an elevated C-reactive protein level. After day 8, there was a two- to three-fold increase in ALT and gamma-glutamyl transferase (GGT) levels, respectively; this peaked on day 11. An ultrasound revealed mild hepatomegaly. The reaction was classified as a "probable" AGEP by an expert panel of the German Registry of Severe Skin Reactions. Treatment with high-dose methylprednisolone was started on day 7 and slowly tapered. Topical corticosteroids were also administered. The man was discharged on day 21; at that time his GGT and ALT levels had only slightly improved. The hepatomegaly and laboratory abnormalities healed after a period of 6 weeks, as did the skin lesions, with desquamation. Four months later, skin testing yielded positive patch test results on day 3 for both amoxicillin and paracetamol. A prick test was positive for amoxicillin at day 2, but not positive for paracetamol.

Sender's comment:

Non-specific skin rashes are described in paracetamol core safety information. Cutaneous reactions are listed for amoxicillin, AGEP specifically is not. Causality is assessed as probable for both drugs based on the close temporal relationship and response to skin testing.

Reporter's comment:

"Because of the clear history in combination with positive skin tests, we contributed [sic] our patient's symptoms to both amoxicillin and paracetamol. . . . Definitive confirmation as the cause of AGEP could only be obtained by re-challenging the patient, which is, however, ethically not justifiable in severe cases like the one reported here."

Generics [UK] Ltd, 07.08.2009 DE-MYLANLABS-2009S1013253

Sender diagnosis: not reported

Narrative

Treudler R, Grunewald S, Gebhardt C, Simon J-C. Prolonged course of acute generalized exanthematous pustulosis with liver involvement due to sensitization to amoxicillin and paracetamol. Acta-Derm-Venereol 2009; 89(3) 314-315

A 48-year-old man developed acute generalised exanthematous pustulosis (AGEP) with liver involvement, due to sensitisation to paracetamol [acetaminophen] and amoxicillin [dosages not stated]. The man was commenced on paracetamol for otitis media, and after 2 days, amoxicillin was initiated [therapeutic indication not stated]. Three days later, he developed exanthema. Both drugs were ceased on the same day. The next day, he was hospitalised and started treatment with prednisolone succinate for 2 days. On day 7, he was transferred to another hospital with pustular suberythrodermic exanthema and oral erosions. Histology showed slight acanthosis and subcorneal pustules containing neutrophils. Perivascular and interstitial infiltrate made up of neutrophils, eosinophils and lymphocytes was present in the papillary dermis. He had a fever, and displayed neutrophilia, leucocytosis and an elevated C-reactive protein level. After day 8, there was a two- to three-fold increase in ALT and gamma-glutamyl transferase (GGT) levels, respectively; this peaked on day 11. An ultrasound revealed mild hepatomegaly. The reaction was classified as a "probable" AGEP by an expert panel of the German Registry of Severe Skin Reactions. Treatment with high-dose methylprednisolone was started on day 7 and slowly tapered. Topical corticosteroids were also administered. The man was discharged on day 21; at that time his GGT and ALT levels had only slightly improved. The hepatomegaly and laboratory abnormalities healed after a period of 6 weeks, as did the skin lesions, with desquamation. Four months later, skin testing yielded positive patch test results on day 3 for both amoxicillin and paracetamol. A prick test was positive for amoxicillin at day 2, but not positive for paracetamol.

Sender's comment:

Non-specific skin rashes are described in paracetamol core safety information. Cutaneous reactions are listed for amoxicillin, AGEP specifically is not.

Causality is assessed as probable for both drugs based on the close temporal relationship and response to skin testing.

Reporter's comment:

"Because of the clear history in combination with positive skin tests, we contributed [sic] our patient's symptoms to both amoxicillin and paracetamol. . . . Definitive confirmation as the cause of AGEP could only be obtained by re-challenging the patient, which is, however, ethically not justifiable in severe cases like the one reported here."

STADA Arzneimittel AG, 07.08.2009 DE-STADA-019143

Sender diagnosis: not reported

Narrative

The following information was published in the scientific literature.

Reactions Weekly 1263 pp 6-7; 1 August 2009

Amoxicillin/paracetamol: Acute generalised exanthematous pustulosis with liver involvement: case report

Treudler R, Grunewald S, Gebhardt C, Simon J-C. Prolonged course of acute generalized exanthematous pustulosis with liver involvement due to sensitization to amoxicillin and paracetamol. Acta Dermato-Venereologica 89: 314-315, No. 3, 2009 - Germany

Summary:

A 48-year-old man developed acute generalised exanthematous pustulosis (AGEP) with liver involvement, due to sensitisation to paracetamol [acetaminophen] and amoxicillin [dosages not stated].

The man was commenced on paracetamol for otitis media, and after 2 days, amoxicillin was initiated [therapeutic indication not stated]. Three days later, he developed exanthema. Both drugs were ceased on the same day. The next day, he was hospitalised and started treatment with prednisolone succinate for 2 days. On day 7, he was transferred to another hospital with pustular suberythrodermic exanthema and oral erosions. Histology showed slight acanthosis and subcorneal pustules containing neutrophils. Perivascular and interstitial infiltrate made up of neutrophils, eosinophils and lymphocytes was present in the papillary dermis. He had a fever, and displayed neutrophilia, leucocytosis and an elevated C-reactive protein level. After day 8, there was a two- to three-fold increase in ALT and γ -glutamyl transferase (GGT) levels, respectively; this peaked on day 11. An ultrasound revealed mild hepatomegaly. The reaction was classified as a 'probable' AGEP by an expert panel of the German Registry of Severe Skin Reactions.

Treatment with high-dose methylprednisolone was started on day 7 and slowly tapered. Topical corticosteroids were also administered. The man was discharged on day 21; at that time his GGT and ALT levels had only slightly improved. The hepatomegaly and laboratory abnormalities healed after a period of 6 weeks, as did the skin lesions, with desquamation.

Four months later, skin testing yielded positive patch test results on day 3 for both amoxicillin and paracetamol. A prick test was positive for amoxicillin at day 2, but not positive for paracetamol.

Sender's comment:

This case was classified as serious.

Paracetamol

The causal relationship between the suspected drug and the suspected adverse reaction(s) was assessed as probably related based on the temporal association and the known safety profile of paracetamol, which includes hepatic toxicity and severe hypersensitivity reactions.

The suspected adverse reaction(s) are listed, AGEP is not explicitly stated according to the current reference SPC.

Amoxicillin

The causal relationship between the suspected drug and the suspected adverse reaction(s) was assessed as probably related based on the temporal association and the known safety profile of amoxicillin, which includes hepatic toxicity and severe hypersensitivity reactions.

The suspected adverse reaction(s) are listed, AGEP is not explicitly stated according to the current reference SPC.

This case does not change the overall benefit-risk balance of the medicinal products.

Reporter's comment:

Author Comment: Because of the clear history in combination with positive skin tests, we contributed [sic] our patient's symptoms to both amoxicillin and paracetamol... Definitive confirmation as the cause of AGEF could only be obtained by re-challenging the patient, which is, however, ethically not justifiable in severe cases like the one reported here.

Bayer Vital GmbH, 10.08.2009 DE-BAYER-200927350GPV

Sender diagnosis: not reported

Narrative

This report derived from medical literature on 29-Jul-2009 and describes the occurrence of acute generalized exanthematous pustulosis with liver involvement (hepatomegaly and elevation of liver enzymes) in a 48-year-old male who was treated with amoxicillin and paracetamol for otitis media.

****Case Report:**

A 48-year-old man with no history of psoriasis was treated with paracetamol because of otitis media. In addition, amoxicillin was started 2 days later and within 3 more days he developed exanthema, which was progressive despite immediate cessation of both drugs on day 5 after start of drug therapy, hospital admission on day 6 and therapy with prednisolone-succinate (250 mg/day for 2 days). On day 7 he was referred to the university hospital with pustular suberythrodermic exanthema and oral erosions. Histology from the trunk revealed slight acanthosis and subcorneal pustules filled with neutrophils. In the papillary dermis there was a mixed perivascular and interstitial infiltrate of neutrophils, eosinophils and lymphocytes. Skin lesions were accompanied by fever >39°C, leukocytosis (25/nL), neutrophilia (87%) and increased C-reactive protein level (CRP) (250 mg/l). On day 8, eosinophils and liver enzymes were within normal range, but gamma-glutamyl transferase (GGT) and alanine aminotransferase (ALT) then increased two- to three-fold, respectively, with a maximum on day 11 and only slight improvement until discharge on day 21. Serology was negative for acute viral infections (hepatitis A, B, C, Epstein-Barr virus, cytomegalovirus, human herpes virus (HHV)-6, parvovirus B19). The serum level of interleukin-17 was below the detection level (<2.5 pg/ml). Ultrasound revealed mild hepatomegaly but no lymphadenopathy. The content of the pustules was sterile, and blood culture was negative. The case was reported to the German Registry of Severe Skin Reactions and was classified as "probable" acute generalized exanthematous pustulosis (AGEP) by an associated expert panel. Treatment included high-dose oral methylprednisolone (starting on day 7 with 160 mg, with slowly decreasing dosages) in combination with topical corticosteroids. The skin lesions healed with desquamation and showed a prolonged clinical course over a period of 6 weeks, together with hepatomegaly and laboratory abnormalities. Skin testing performed 4 months later revealed positive patch tests for both amoxicillin and paracetamol (both 5% in petrolatum, day 3) with papulopustular infiltrates reminiscent of typical AGEF lesions. A prick test was positive at late readings for amoxicillin (papular infiltrate of 5 mm at day 2) but not indicative for paracetamol (both pure in aqueous solution). The patient was advised to avoid both drugs in the future.

****Authors' Comments:**

The authors report the case of a patient for whom they have good evidence (i.e. by skin testing) that AGEF (acute generalized exanthematous pustulosis) was elicited by a combination of amoxicillin and paracetamol and who showed unusual liver involvement and prolonged course of the disease... Because of the clear history in combination with positive skin tests, the authors contributed the patient's symptoms to both amoxicillin and paracetamol. Positive patch tests in the diagnosis of AGEF have been reported for these two as well as for several other drugs. In this patient, patch testing was more sensitive than prick testing, where paracetamol remained negative. Definitive confirmation as the cause of AGEF could only be obtained by re-challenging the patient, which is, however, ethically not justifiable in severe cases like the one reported here. In conclusion, the AGEF in this patient was unique because of: (i) the prolonged course of skin and liver involvement; and (ii) the combined sensitization to two drugs, including (iii) paracetamol, which was recently assigned as having no significant association with AGEF.

Sender's comment:

Initial 29-Jul-2009: This literature report concerns a paracetamol as well as an amoxicillin containing product of unknown origin. Even if may not be a Bayer Schering Pharma products, this report might convey new findings relevant for company products. The reported serious (hospitalization) events ACUTE GENERALIZED EXANTHEMATOUS PUSTULOSIS with the symptoms of pustular exanthema, oral erosions and fever, HEPATOMEGALY as well as ELEVATION OF LIVER ENZYMES are listed in the International Product Information (IPI) of paracetamol but not listed in the IPI of Amoxicillin. As there is a compatible temporal relationship between the onset of the events and the administration of both suspect drugs, company cannot exclude a causal relationship. No alternative explanation is available.

Reporter's comment:

Dolorgiet GmbH, 11.08.2009 DE-DOLORGIET-2009paracetamol01411

Sender diagnosis: not reported

Narrative

A 48-year-old man developed acute generalised exanthematous pustulosis (AGEP) with liver involvement, due to sensitisation to paracetamol [acetaminophen] and amoxicillin [dosages not stated].

The man was commenced on paracetamol for otitis media, and after 2 days, amoxicillin was initiated [therapeutic indication not stated]. Three days later, he developed exanthema. Both drugs were ceased on the same day. The next day, he was hospitalised and started treatment with prednisolone succinate for 2 days. On day 7, he was transferred to another hospital with pustular suberythrodermic exanthema and oral erosions. Histology showed slight acanthosis and subcorneal pustules containing neutrophils. Perivascular and interstitial infiltrate made up of neutrophils, eosinophils and lymphocytes was present in the papillary dermis. He had a fever, and displayed neutrophilia, leucocytosis and an elevated C-reactive protein level. After day 8, there was a two- to three-fold increase in ALT and γ -glutamyl transferase (GGT) levels, respectively; this peaked on day 11. An ultrasound revealed mild hepatomegaly. The reaction was classified as a "probable" AGEP by an expert panel of the German Registry of Severe Skin Reactions.

Treatment with high-dose methylprednisolone was started on day 7 and slowly tapered. Topical corticosteroids were also administered. The man was discharged on day 21; at that time his GGT and ALT levels had only slightly improved. The hepatomegaly and laboratory abnormalities healed after a period of 6 weeks, as did the skin lesions, with desquamation.

Four months later, skin testing yielded positive patch test results on day 3 for both amoxicillin and paracetamol. A prick test was positive for amoxicillin at day 2, but not positive for paracetamol.

Sender's comment:

Reporter's comment:

Because of the clear history in combination with positive skin tests, we contributed [sic] our patient's symptoms to both amoxicillin and paracetamol... Definitive confirmation as the cause of AGEP could only be obtained by re-challenging the patient, which is, however, ethically not justifiable in severe cases like the one reported here.

RANBAXY, 12.08.2009 DE-RANBAXY-2009RR-25985

Sender diagnosis: not reported

Narrative

This case, manufacturer control number 2009RR-25985 from GERMANY refers to a 48 year old male who developed acute generalised exanthematous pustulosis with liver involvement following treatment with amoxicillin and paracetamol that required hospitalisation.

This case was reported in the literature.

The subject received the following suspect products:

Amoxicillin: patch test and prick test positive. The drug was withdrawn.

Paracetamol: for otitis media. Patch test positive, prick test negative. The drug was withdrawn.

The patient received the following treatment products:

Prednisolone succinate: 250 mg daily, 2 days.

Methylprednisolone: 160 mg, high-dose. The drug dose was decreased.

Topical corticosteroids.

The patient developed acute generalised exanthematous pustulosis with liver involvement.

A 48-year-old man developed acute generalised exanthematous pustulosis (AGEP) with liver involvement, due to sensitisation to paracetamol [acetaminophen] and amoxicillin [dosages not stated].

The man with no history of psoriasis was commenced on paracetamol for otitis media, and after 2 days, amoxicillin was initiated [therapeutic indication not stated]. Three days later, he developed exanthema. Both drugs were ceased on the same day. The next day, he was hospitalised and started treatment with prednisolone succinate 250 mg/day for 2 days. On day 7, he was transferred to another hospital with pustular suberythrodermic exanthema and oral erosions.

Histology showed slight acanthosis and subcorneal pustules containing neutrophils. Perivascular and interstitial infiltrate made up of neutrophils, eosinophils and lymphocytes was present in the papillary dermis. He had a fever, and displayed neutrophilia, leucocytosis and an elevated C-reactive

protein level. After day 8, there was a two- to three-fold increase in ALT and -glutamyl transferase (GGT) levels, respectively; this peaked on day 11.

Serology was negative for acute viral infections (hepatitis a, B, C, Epstein-Barr virus, cytomegalovirus, human herpes virus (HHV-6), parvovirus B19.) An ultrasound revealed mild hepatomegaly. The reaction was classified as a "probable" AGEP by an expert panel of the German Registry of Severe Skin Reactions.

Treatment with high-dose of 160 mg methylprednisolone was started on day 7 and slowly tapered. Topical corticosteroids were also administered. The man was discharged on day 21; at that time his GGT and ALT levels had only slightly improved. The hepatomegaly and laboratory abnormalities healed after a period of 6 weeks, as did the skin lesions, with desquamation.

Four months later, skin testing yielded positive patch test results on day 3 for both amoxicillin and paracetamol (both in 5% in petroleum). A prick test was positive for amoxicillin at day 2, but not positive for paracetamol. The patient was advised to avoid both drugs in the future.

The outcome of the events was reported as "healed" at the time of this report.

The reporter considered the case serious because the patient was hospitalised.

Author's Comment: "Because of the clear history in combination with positive skin tests, we contributed [sic] our patient's symptoms to both amoxicillin and paracetamol. . . Definitive confirmation as the cause of AGEP could only be obtained by re-challenging the patient, which is, however, ethically not justifiable in severe cases like the one reported here."

Follow up report received on 30-JUL-2009 containing further information regarding:

Laboratory data.

Treatment products: dosages.

Reaction text.

Company comment:

The case is rated as serious and unexpected and possible.

Acute generalized exanthematous pustulosis (AGEP) is an uncommon cutaneous eruption that is most often provoked by drugs, by acute infections with enteroviruses, or by mercury. It is characterized by acute, extensive formation of nonfollicular sterile pustules on an erythematous background, fever and elevated numbers of blood neutrophils. Involvement of T cells in drug-induced AGEP was suggested by positive patch tests and lymphocyte transformation tests. Moreover, drug-specific CD4+ and CD8+ T cells could be isolated and propagated in vitro from patch test sites and blood from AGEP patients.

Most cases of AGEP are drug induced, particularly by antibiotics and mainly beta-lactams.

Because of the clear history in combination with positive skin tests, both amoxicillin and paracetamol contributed to this patient's symptoms. Definitive confirmation as the cause of AGEP could only be obtained by re-challenging the patient, which is, however, ethically not justifiable in severe cases like the one reported here.

Reference:

Britschgi M, Pichler WJ. Acute generalized exanthematous pustulosis, a clue to neutrophil-mediated inflammatory processes orchestrated by T cells. Curr Opin Allergy Clin Immunol 2002 Aug;2(4):325-31.

Treudler R, Grunewald S, Gebhardt C, Simon J-C. Prolonged course of acute generalized exanthematous pustulosis with liver involvement due to sensitization to amoxicillin and paracetamol. Acta Dermato-Venereologica 89: 314-315, No. 3, 2009.

Sender's comment:

Reporter's comment:

Bristol-Myers Squibb Company, 13.08.2009 DE-Bristol-Myers Squibb Company-14716021

Sender diagnosis: not reported

Narrative

This case was derived from a literature abstract which described about acute generalised exanthematous pustulosis (AGEP) with liver involvement, due to sensitization to acetaminophen and amoxicillin in a male patient.

A 48-year-old male patient was hospitalized for acute generalised exanthematous pustulosis (AGEP) with liver involvement, due to sensitization to

acetaminophen and amoxicillin (dosages not stated). He also developed hepatomegaly, alanine aminotransferase (ALT) increased and gamma-glutamyl transferase (GGT) increased.

The man was commenced on acetaminophen for otitis media, and after 2 days, amoxicillin was initiated (therapeutic indication not stated). Three days later, he developed exanthema. Both drugs were ceased on the same day. The next day, he was hospitalized and started treatment with prednisolone succinate for 2 days. On day 7, he was transferred to another hospital with pustular suberythrodermic exanthema and oral erosions. Histology showed slight acanthosis and subcorneal pustules containing neutrophils. Perivascular and interstitial infiltrate made up of neutrophils, eosinophils and lymphocytes was present in the papillary dermis. He had a fever, and displayed neutrophilia, leucocytosis and an elevated C-reactive protein level. After day 8, there was a two - to three-fold increase in ALT and GGT levels, respectively; this peaked on day 11. An ultrasound revealed mild hepatomegaly. The reaction was classified as a "probable" AGEF by an expert panel of the German Registry of Severe Skin Reactions. Treatment with high-dose methylprednisolone was started on day 7 and slowly tapered. Topical corticosteroids were also administered. The man was discharged on day 21; at that time his GGT and ALT levels had only slightly improved. The hepatomegaly and laboratory abnormalities healed after a period of 6 weeks, as did the skin lesions, with desquamation. Four months later, skin testing yielded positive patch test results on day 3 for both amoxicillin and acetaminophen. A prick test was positive for amoxicillin at day 2, but not positive for acetaminophen.

Author comment: "Because of the clear history in combination with positive skin tests, we contributed our patient's symptoms to both amoxicillin and paracetamol. Definitive confirmation as the cause of AGEF could only be obtained by re-challenging the patient, which is, however, ethically not justifiable in severe cases like the one reported here."

Literature citation: Treudler R, Grunewald S, Gebhardt C, Simon JC. Prolonged course of acute generalized exanthematous pustulosis with liver involvement due to sensitization to amoxicillin and paracetamol. *Acta Dermato-Venereologica* 2009 Jan 1;89:314-5.

BMS medical evaluation comment:

This patient developed acute generalised exanthematous pustulosis after therapy with amoxicillin and acetaminophen. In the clinical context of symptom evolution and positive patch test results for both amoxicillin and acetaminophen, a causal role of these medications remains possible.

Supplemental information was received on 05-Aug-2009 as full text article. The event of hepatomegaly was deleted and seriousness criterion of hospitalization was added for alanine aminotransferase increased and gamma-glutamyltransferase increased.

Skin lesions were accompanied by fever more than 39 degree celsius, leucocytosis (25/nl), neutrophilia (87%) and increased C-reactive protein level (250 mg/l). The serum level of interleukin-17 was below the detection level (less than 2.5 pg/ml). Treatment included high-dose oral methylprednisolone (starting on day 7 with 160 mg, with slowly decreasing dosages) in combination with topical corticosteroids. The patient was advised to avoid both acetaminophen and amoxicillin in future.

BMS medical evaluation comment:

This patient developed acute generalised exanthematous pustulosis (AGEF), increased alanine aminotransferase (ALT) and increased gamma-glutamyl transferase (GGT) after therapy with amoxicillin and acetaminophen. As part of concurrent pharmacotherapy, the cosuspect acetaminophen could cause increased ALT and increased GGT. In the clinical context of symptom evolution and positive patch test results for both amoxicillin and acetaminophen, these medications possibly resulted in AGEF.

Sender's comment:

This report is submitted to you in the name of the marketing authorisation holder Bristol-Myers Squibb GmbH & Co. KGaA.

Acetaminophen

Event number; label*; causality**

#1; unexpected; PSB

#2; expected; PSB

#3; expected; PSB

* according to current Summary of Product Characteristics

** DEF=definite, PRB=probable, PSB=possible, NOL=not likely, NOR=not related

Change of current scientific knowledge: no.

Comments: This patient developed acute generalised exanthematous pustulosis after therapy with amoxicillin and acetaminophen. The occurrence of erythema, pruritus and flush is expected according to the current SPC of PERFALGAN, however AGEF is not specified. In the clinical context of symptom evolution and positive patch test results for both amoxicillin and acetaminophen, a causal role of these medications remains possible.

Reporter's comment:

Dr. August Wolff Arzneimittel, 21.09.2009 DE-DRWOLFF-2009-029

Sender diagnosis: not reported

Narrative

A case of acute generalized exanthematous pustulosis was reported in a 48-year-old male patient. The patient had a past medical history of otitis media for which he received treatment with paracetamol and two days later with amoxicillin. He developed a progressive erythema within three more days (day 5) and discontinued both drugs. He was hospitalised on day 6 and received prednisolone-succinate. On day 7 he was diagnosed with a pustular suberythrodermic exanthema, oral erosions and fever of 39 degree Celsius. Ultrasound demonstrated mild hepatomegaly. Histological examination of the skin revealed slight acanthosis and subcorneal pustules filled with neutrophils and the papillary dermis showed mixed perivascular and interstitial infiltrate of neutrophils, eosinophils and lymphocytes. Relevant laboratory test results included leukocytes 25/nl, neutrophils 87%, C-reactive protein 250 mg/l and GGT and ALT two- and threefold increased. Skin lesions healed with desquamation and showed a prolonged clinical course over 6 weeks. Skin testing performed 4 months later revealed positive patch and prick test for amoxicillin and a positive patch but negative prick test for paracetamol.

Sender's comment:

This adverse reaction is expected for Amoxicillin. However, the adverse reaction AGEP in this patient was unique because of prolonged course of skin and liver involvement. The individual case does not indicate a change in the assessment of the safety and benefit-risk-ratio of Amoxi-Wolff products. No further relevant measure is currently deemed necessary.

Reporter's comment:

Bausch & Lomb Incorporated, 22.09.2009 DE-BAUSCH-2009BL004524

Sender diagnosis: not reported

Narrative

A report was identified in the literature indicating that a 48-year-old male patient with no history of psoriasis experienced acute generalized exanthematous pustulosis (AGEP) (ACUTE GENERALISED EXANTHEMATOUS PUSTULOSIS) with liver involvement after therapy with PARACETAMOL (route of administration, dose and manufacturer unknown) for otitis media and AMOXICILLIN (route of administration, dose and manufacturer unknown) that was added two days later. Within three more days the patient experienced exanthema. Despite stopping both drugs on day 5 after start of drug therapy, the exanthema was progressive and the patient was hospitalized on day 6. Therapy with prednisolone-succinate (250 mg/day for two days) was started. On day 7 he was referred to a university hospital with pustular suberythrodermic exanthema and oral erosions. Histology from the trunk revealed slight acanthosis and subcorneal pustules filled with neutrophils. In the papillary dermis there was a mixed perivascular and interstitial infiltrate of neutrophils, eosinophils and lymphocytes. Skin lesions were accompanied by fever (PYREXIA), leukocytosis (LEUKOCYTOSIS) (25/nl), neutrophilia (NEUTROPHILIA) (87 %) and increased C-reactive protein (CRP) level (C-REACTIVE PROTEIN INCREASED) (250 mg/l). On day 8, eosinophils and liver enzymes were within normal range, but gamma-glutamyl transferase (GGT) (GAMMA-GLUTAMYLTRANSFERASE INCREASED) and alanine aminotransferase (ALT) (ALANINE AMINOTRANSFERASE INCREASED) then increased two- to three-fold, respectively, with a maximum on day 11 and only slight improvement by discharge on day 21. Acute viral infections (hepatitis A, B, C, Epstein-Barr virus, cytomegalovirus, human herpes virus (HHV)-6, parvovirus B19) could be excluded serologically. The serum level of interleukin 17 was below the detection level (<2.5 pg/ml). Ultrasound revealed mild hepatomegaly (HEPATOMEGALY) but no lymphadenopathy. The content of the pustules was sterile, and blood culture was negative. On day 7 treatment with 160 mg high-dose oral methylprednisolone with slowly decreasing dosages in combination with topical corticosteroids were started. The skin lesions healed with desquamation and showed a prolonged clinical course over a period of six weeks. The patient made a complete recovery. Skin testing performed four months later revealed positive patch tests for both, amoxicillin and paracetamol with papulopustular infiltrates reminiscent of typical AGEP lesions. A prick test was positive at late readings for amoxicillin (papular infiltrate of 5 mm at day 2) but not indicative for paracetamol. The patient was advised to avoid both drugs in future.

Source: TREUDLER R, GRUNEWALD S, GEBHARDT C, SIMON JC. PROLONGED COURSE OF ACUTE GENERALIZED EXANTHEMATOUS PUSTULOSIS WITH LIVER INVOLVEMENT DUE TO SENSITIZATION TO AMOXICILLIN AND PARACETAMOL. ACTA DERM VENEROL 2009; 89 (3): 314-5

Sender's comment:

This literature report describes the prolonged course of an acute generalized exanthematous pustulosis (AGEP) with liver involvement following the application of paracetamol and amoxicillin. As the company also has a product on the market containing paracetamol (vivimed N gegen Fieber und

Kopfschmerzen) this case is reported. In the overall assessment this case is serious, unexpected and probably related to the drug substance paracetamol. Although paracetamol is generally assigned as having no association to AGEF, some rare cases have been reported. In this case skin tests showed positive results for paracetamol therefore this case was assessed probably related. Some of the reported adverse events like increase of liver enzymes or changes in the blood count are mentioned in the SPC and are expected. The signs like fever or CRP increase are considered unexpected and may be due to the severe reaction of the immune system.

As this report is an isolated case no further measures for risk mitigation are planned.

Reporter's comment:

Bausch & Lomb Incorporated, 12.10.2009 DE-BAUSCH-2009BL004524

Sender diagnosis: not reported

Narrative

A report was identified in the literature indicating that a 48-year-old male patient with no history of psoriasis experienced acute generalized exanthematous pustulosis (AGEP) (ACUTE GENERALISED EXANTHEMATOUS PUSTULOSIS) with liver involvement after therapy with PARACETAMOL (route of administration, dose and manufacturer unknown) for otitis media and AMOXICILLIN (route of administration, dose and manufacturer unknown) that was added two days later. Within three more days the patient experienced exanthema. Despite stopping both drugs on day 5 after start of drug therapy, the exanthema was progressive and the patient was hospitalized on day 6. Therapy with prednisolone-succinate (250 mg/day for two days) was started. On day 7 he was referred to a university hospital with pustular suberythrodermic exanthema and oral erosions. Histology from the trunk revealed slight acanthosis and subcorneal pustules filled with neutrophils. In the papillary dermis there was a mixed perivascular and interstitial infiltrate of neutrophils, eosinophils and lymphocytes. Skin lesions were accompanied by fever (PYREXIA), leukocytosis (LEUKOCYTOSIS) (25/nl), neutrophilia (NEUTROPHILIA) (87 %) and increased C-reactive protein (CRP) level (C-REACTIVE PROTEIN INCREASED) (250 mg/l). On day 8, eosinophils and liver enzymes were within normal range, but gamma-glutamyl transferase (GGT) (GAMMA-GLUTAMYLTRANSFERASE INCREASED) and alanine aminotransferase (ALT) (ALANINE AMINOTRANSFERASE INCREASED) then increased two- to three-fold, respectively, with a maximum on day 11 and only slight improvement by discharge on day 21. Acute viral infections (hepatitis A, B, C, Epstein-Barr virus, cytomegalovirus, human herpes virus (HHV)-6, parvovirus B19) could be excluded serologically. The serum level of interleukin 17 was below the detection level (<2.5 pg/ml). Ultrasound revealed mild hepatomegaly (HEPATOMEGALY) but no lymphadenopathy. The content of the pustules was sterile, and blood culture was negative. On day 7 treatment with 160 mg high-dose oral methylprednisolone with slowly decreasing dosages in combination with topical corticosteroids were started. The skin lesions healed with desquamation and showed a prolonged clinical course over a period of six weeks. The patient made a complete recovery.

Skin testing performed four months later revealed positive patch tests for both, amoxicillin and paracetamol with papulopustular infiltrates reminiscent of typical AGEF lesions. A prick test was positive at late readings for amoxicillin (papular infiltrate of 5 mm at day 2) but not indicative for paracetamol. The patient was advised to avoid both drugs in future.

Source: TREUDLER R, GRUNEWALD S, GEBHARDT C, SIMON JC. PROLONGED COURSE OF ACUTE GENERALIZED EXANTHEMATOUS PUSTULOSIS WITH LIVER INVOLVEMENT DUE TO SENSITIZATION TO AMOXICILLIN AND PARACETAMOL. ACTA DERM VENEROL 2009; 89 (3): 314-5

Upon further file review it was noticed that the company's causality assessment at the event level for the reported event gamma-glutamyl transferase increased (LLT: GGT increased and PT: Gamma-glutamyltransferase increased) was entered as "possible related" to the product. Changes were made to the report reflecting the company's causality assessment as "probable related" for the reported event GGT increased.

Sender's comment:

This literature report describes the prolonged course of an acute generalized exanthematous pustulosis (AGEP) with liver involvement following the application of paracetamol and amoxicillin. As the company also has a product on the market containing paracetamol (vivimed N gegen Fieber und Kopfschmerzen) this case is reported. In the overall assessment this case is serious, unexpected and probably related to the drug substance paracetamol. Although paracetamol is generally assigned as having no association to AGEF, some rare cases have been reported. In this case skin tests showed positive results for paracetamol therefore this case was assessed probably related. Some of the reported adverse events like increase of liver enzymes or changes in the blood count are mentioned in the SPC and are expected. The signs like fever or CRP increase are considered unexpected and may be due to the severe reaction of the immune system.

As this report is an isolated case no further measures for risk mitigation are planned.

Reporter's comment:

MIP International Pharma Research GmbH, 24.11.2009 DE-MIP-hamxpo161809-hpaopo160109

Sender diagnosis: not reported

Narrative

A 48-year-old man with no history of psoriasis was treated with paracetamol because of otitis media. In addition, amoxicillin was started 2 days later and within 3 more days he developed exanthema, which was progressive despite immediate cessation of both drugs on day 5 after start of drug therapy, hospital admission on day 6 and therapy with prednisolone-succinate (250 mg/day for 2 days). On day 7 he was referred to university hospital with pustular suberythrodermic exanthema and oral erosions. Skin lesions were accompanied by fever, leukocytosis (25/nl), neutrophilia (87%) and increased C-reactive protein level (CRP) (250 mg/l). On day 8, eosinophils and liver enzymes were within normal range, but GGT and ALT then increased two- to three-fold, with a maximum on day 11 and only slight improvement until discharge on day 21. Serology was negative for acute viral infections, serum level of interleukin-17 was below the detection level. Ultrasound revealed mild hepatomegaly but no lymphadenopathy. Treatment included high-dose oral methylprednisolone in combination with topical corticosteroids. The skin lesions healed with desquamation and showed a prolonged clinical course over a period of 6 weeks, together with hepatomegaly and laboratory abnormalities. Skin testing performed 4 months later revealed positive patch tests for both amoxicillin and paracetamol. A prick test was positive at late readings for amoxicillin but not indicative for paracetamol.

Sender's comment:

The case was detected in the MEDLINE database on 11th November 2009. In the case, no product name or MAH could be identified. The case could theoretically be in connection with amoxicillin- or paracetamol-containing products of the MAH. Therefore, the case was recorded for both active substances in the database of the MAH as case hamxpo161809 and case hpaopo160109. Skin and subcutaneous disorders like exanthema, rash and allergic reactions as well as hepatic disorders like increased GGT and ALT are known ADRs of both drugs. The cases were classified as labelled for both active ingredients. Since the ADR caused hospitalisation, the cases were classified as serious. Based on the available data, the MAH evaluated the causality as probable for both drugs. The reported case does not change the risk/benefit assessment for Amoxi-saar 500/750/1000 mg Tabletten as well as Paedialgon-Saft/Tabletten and Paracetamol-saar.

Reporter's comment:

The case was reported to the German Registry of Severe Skin Reactions (<http://www.uniklinik-freiburg.de/hautklinik/live/dzh.html>) by the authors and was classified as probable acute generalized exanthematous pustulosis by an associated expert panel. Because of the clear history in combination with positive skin tests, the authors assume that the symptoms were released by both amoxicillin and paracetamol.

AXCOUNT, 07.09.2010 DE-AXCOUNT-2010-013

Sender diagnosis: not reported

Narrative

A 48-year-old man was treated with paracetamol and amoxicillin because of otitis media. Amoxicillin was started 2 days after treatment start with paracetamol and within 3 more days, the man developed exanthema, which was progressive despite immediate cessation of both drugs. The man was hospitalised on day 6 after start of drug therapy and treated with prednisolone-succinate (250 mg/day for 2 days). The man had pustular suberythrodermic exanthema and oral erosions. Histology from the trunk revealed slight acanthosis and subcorneal pustules filled with neutrophils. Skin lesions were accompanied by fever ($>39^{\circ}\text{C}$), leukocytosis (25/nl), neutrophilia (87%) and increased C-reactive protein level (250 mg/l). On day 8, the patient's GGT and ALT levels increased two- to three-fold, respectively, with a maximum on day 11 and only slight improvement until discharge on day 21. Serology was negative for viral infections. The content of the pustules was sterile, and blood culture was negative. Acute generalised exanthematous pustulosis (AGEP) was diagnosed and the patient was treated with oral methylprednisolone in combination with topical corticosteroids. The lesions healed within 6 weeks and the laboratory abnormalities resolved. A skin testing was performed 4 months later and revealed positive patch tests for both amoxicillin and paracetamol with papulopustular infiltrates reminiscent of typical AGEP lesions. The patient was advised to avoid both drugs in the future.

Sender's comment:

This case report was classified serious due to hospitalisation. Since this case occurred in Germany and based on the information given on the suspected drugs, it cannot be excluded that Paracetamol axcount or Amoxicillin axcount were involved in this case report. The reaction is expected, since severe allergic reactions are mentioned in both SPCs. The causality was assessed as probable for both drugs. This case report does not affect the benefit/risk ratio of Paracetamol axcount and Amoxicillin axcount.

Reporter's comment:

The authors stated that because of the clear history in combination with positive skin tests, the patient's symptoms are attributed to amoxicillin and paracetamol.

Heumann Pharma GmbH & Co. Generika KG, 07.12.2010 DE-HEUMANN1-000342

Sender diagnosis: not reported

Narrative

This case has been identified in the literature.

A 48-year-old man with no history of psoriasis was treated with paracetamol because of otitis media. In addition, amoxicillin was started 2 days later and within 3 more days he developed exanthema, which was progressive despite immediate cessation of both drugs on day 5 after start of drug therapy, hospital admission on day 6 and therapy with prednisolone-succinate (250 mg/day for 2 days). On day 7 he was referred to our university hospital with pustular suberythrodermic exanthema and oral erosions. Histology from the trunk revealed slight acanthosis and subcorneal pustules filled with neutrophils. In the papillary dermis there was a mixed perivascular and interstitial infiltrate of neutrophils, eosinophils and lymphocytes. Skin lesions were accompanied by fever > 39°C, leukocytosis (25/nl), neutrophilia (87%) and increased C-reactive protein level (CRP) (250 mg/l). On day 8, eosinophils and liver enzymes were within normal range, but gamma-glutamyl transferase (GGT) and alanine aminotransferase (ALT) then increased two- to three-fold, respectively, with a maximum on day 11 and only slight improvement until discharge on day 21. Serology was negative for acute viral infections (hepatitis A, B, C, Epstein-Barr virus, cytomegalovirus, human herpes virus (HHV)-6, parvovirus B19). The serum level of interleukin-17 was below the detection level (< 2.5 pg/ml). Ultrasound revealed mild hepatomegaly but no lymphadenopathy. The content of the pustules was sterile, and blood culture was negative. The case was reported to the German Registry of Severe Skin Reactions (<http://www.uniklinikfreiburg.de/hautklinik/live/dzh.html>) and was classified as "probable" AGEP by an associated expert panel. Treatment included high-dose oral methylprednisolone (starting on day 7 with 160 mg, with slowly decreasing dosages) in combination with topical corticosteroids. The skin lesions healed with desquamation and showed a prolonged clinical course over a period of 6 weeks, together with hepatomegaly and laboratory abnormalities. Skin testing performed 4 months later revealed positive patch tests for both amoxicillin and paracetamol (both 5% in petrolatum, day 3) with papulopustular infiltrates reminiscent of typical AGEP lesions. A prick test was positive at late readings for amoxicillin (papular infiltrate of 5 mm at day 2) but not indicative for paracetamol (both pure in aqueous solution). The patient was advised to avoid both drugs in the future.

Sender's comment:

This case report was identified within a literature search for a PSUR for Amoxicillin. It was classified serious due to hospitalisation. Due to the information given in this report and since this case occurred in Germany, it cannot be excluded that Amoxicillin Heumann or Paracetamol Heumann were involved in this case. The reaction is labelled for Amoxicillin Heumann but not explicitly mentioned in the SPC of Paracetamol Heumann. However, allergic skin reactions to the point of anaphylactic shock are mentioned in the SPC of Paracetamol Heumann. The reaction resolved after both drugs were withdrawn. The relationship between paracetamol and amoxicillin was affirmed by skin tests. Therefore, causality was assessed as probable for both drugs. There are no further cases in the database of Heumann. This case report does not affect the benefit/risk ratio of Amoxicillin Heumann and Paracetamol Heumann.

Reporter's comment:

The authors stated that because of the clear history in combination with positive skin tests, the patient's symptoms are attributed to amoxicillin and paracetamol.

PFIZERINC, 15.10.2013 DE-PFIZER INC-2013292937

Sender diagnosis: not reported

Narrative

This is a literature report from Acta dermato-venereologica, 2009, Vol. 89(3), Pages. 314-5, entitled "Prolonged course of acute generalized exanthematous pustulosis with liver involvement due to sensitization to amoxicillin and paracetamol." Full publication has been requested.

Additional information has been requested and will be provided as it becomes available.

Sender's comment:

The information available in this report is limited, and does not allow a medically meaningful assessment. The missing information includes event onset date, event details, relevant medical history and concomitant medication(s). A contributory role of the suspect products (paracetamol and amoxicillin) to the events acute generalized exanthematous pustulosis with liver involvement and cannot be excluded based on temporal association and known product profiles. The case will be reviewed further once the relevant information becomes available. The impact of this report on the benefit/risk profile of the Pfizer product is evaluated as part of Pfizer procedures for safety evaluation, including the review and analysis of aggregate data for adverse events. Any safety concern identified as part of this review, as well as any appropriate action in response, will be promptly notified to Regulatory Authorities, Ethics Committees and Investigators as appropriate.

Reporter's comment:

sanofi-aventis, 11.11.2014 DE-SA-A04200900810

Sender diagnosis: not reported

Narrative

Initial report was received on 12-Jun-2009 via:

Treudler R, Grunewald S, Gebhardt C, Simon JC. Prolonged Course of Acute Generalized Exanthematous Pustulosis with Liver Involvement due to Sensitization to Amoxicillin and Paracetamol. Acta Derm Venereol 2009; 89: 314-5.

This serious case is a literature report about acute generalized exanthematous pustulosis under amoxicillin and paracetamol in a male.

Case report: A 48-year-old male patient with no history of psoriasis was treated with paracetamol and amoxicillin (no trade names provided) due to otitis media and developed a prolonged course of acute generalised exanthematous pustulosis (AGEP) with elevated liver enzyme levels (nos) and hepatomegaly. The patient was treated with paracetamol for otitis media, and in addition, amoxicillin was started 2 days later. Within 3 days he developed exanthema, which was progressive despite immediate cessation of both drugs. The patient was hospitalized with pustular suberythrodermic exanthema and oral erosions. Skin lesions were accompanied by fever greater than 39 degrees C, leukocytosis (25/nl), neutrophilia (87%) and increased C-reactive protein level (250 mg/l). The skin lesions healed with desquamation. Hepatomegaly and laboratory abnormalities as well as the skin lesions showed a prolonged clinical course over a period of 6 weeks on administration of corticosteroid treatment. Skin testing performed 4 months later revealed positive patch tests for both amoxicillin and paracetamol. The adverse reactions reappeared after reintroduction (details were not provided). The authors state that they ascribe the patient's symptoms to both amoxicillin and paracetamol because of the clear history in combination with positive skin tests.

Addendum for follow-up received 15-Jun-2009

The full article was received and the case report could be amended as follows:

The patient was admitted to hospital on day six after start of therapy and a therapy with prednisolone-succinate (250 mg/day for two days) was initiated. On day seven he was referred to the reporters university hospital with pustular suberythrodermic exanthema and oral erosions. Histology from the trunk revealed slight acanthosis and subcorneal pustules filled with neutrophils. In the papillary dermis there was a mixed perivascular and interstitial infiltrate of neutrophils, eosinophils and lymphocytes.

On day 8, eosinophils and liver enzymes were within normal range but gamma-glutamyl transferase (GGT) and alanine aminotransferase (ALT) then increased two- to threefold, respectively, with a maximum on day 11 and only slight improvement until discharge on day 21. Serology was negative for acute viral infections (hepatitis A, B, C, Epstein-Barr virus, cytomegalovirus, human herpes virus (HHV)-6, parovirus B19). The serum level of interleukin-17 was below the detection level (less than 2.5 pg/ml). Ultrasound revealed mild hepatomegaly but no lymphadenopathy. The content of the pustules was sterile, and blood culture was negative.

Treatment included high-dose oral methylprednisolone (starting on day 7 with 160 mg, with slowly decreasing dosages) in combination with topical corticosteroids.

The positive patch tests with both drugs (5% in petrolatum) revealed papulopustular infiltrates reminiscent of typical AGEP lesions. A prick test was positive at late readings for amoxicillin (papular infiltrate of 5 mm at day 2) but not indicative for paracetamol (both pure in aqueous solution).

The patient was advised to avoid both drugs in the future.

Follow-up was received on 31-Oct-2014 via an article:

Treudler R, Grunewald S, Gebhardt C, Simon J. Prolonged Course of Acute Generalized Exanthematous Pustulosis with Liver Involvement in a patient with combined sensitization to Amoxicillin and Paracetamol. Allergy 2009;64 Suppl 90:289.

Negative medical history added, lab tests added, action taken of suspect drugs changed from drug withdrawn NOS to permanently discontinued, duration captured, last dose to onset removed, first dose to onset in both suspects and events changed to symptoms, additional drugs captured in corrective treatment, complication captured. Additional reporter added.

Sender's comment:

Sanofi Company follow up comment dated 08-11-2014: In this case, as per the follow up information provided and positive skin tests, the role of amoxicillin and paracetamol cannot be denied for the events of acute generalised exanthematous pustulosis and hepatomegaly; however, the lack of detailed information regarding any past or concomitant drugs of the patient precludes a comprehensive assessment in this case.

Start of the reaction 3 days after initiation of treatment is too short for primary sensitization, 5 days is borderline. Also duration is unusual: AGEP is considered a self-limited condition with resolution of pustules within less than 15 days. The differential diagnosis of AGEP includes a wide range of diseases or reactions that can cause pustular eruptions of the skin. Based on chronology and the authors' opinion, the causal role of amoxicillin and paracetamol cannot be ruled out. The causal assessment of AGEP in this case is confounded by the concomitant administration of amoxicillin and paracetamol.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-09108231

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		

Patient

Initials:	Sex: Female	Birth: privacy	Age: 24 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 C-reactive protein increased	200909				Unknown
2 Fuchs' syndrome	200909				Unknown
3 Stevens Johnson syndrome					Unknown
4 Leukocytosis	200909				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 acetylsalicylic acid	Oral	/	-	Common cold
acetylsalicylic acid			-	
2 Amoxicillin	Oral	3000 Mg milligram(s)/Day	-	Pharyngitis
amoxicillin			-	
3 diclofenac	Oral	150 Mg milligram(s)/Day	-	Pharyngitis
diclofenac			-	
4 ibuprofen	Oral	/	-	Common cold
ibuprofen			-	
5 ibuprofen	Unknown	/	-	Fever
ibuprofen			-	
6 Paracetamol	Oral	/	-	Common cold
paracetamol			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Apathy			Unknown	
Rhinitis			Unknown	
Odynophagia			Unknown	
Fever sore			Unknown	
Common cold			Yes	With odynophagy, rhinitis, general apathy and fever

Primary source

Qualification:	Other Health Professional	observestudytype:
Literature:	Jakob M, Stuhmann N, Jordan J, Bootz F, Schröck A. [Stomatitis, cheilitis and conjunctivitis after a common cold.] HNO 2009; doi 10.1007/s00106-009-1931-2.	
Qualification:	Other Health Professional	observestudytype:
Literature:	Jakob M., Stuhmann N., Jordan J., Bootz F., Schröck A. Stomatitis, cheilitis and conjunctivitis after a common cold. HNO 2009 , 57:914-917	

Narratives (nur bei elektronischem Eingang)

JUTA, 18.08.2009 DE-JUTA-1515-09

Sender diagnosis: not reported

Narrative

A 24-year-old female patient developed Stevens Johnson syndrome (Fuchs' syndrome) during treatment with amoxicillin and diclofenac and after treatment with paracetamol, ibuprofen and acetylsalicylic acid. The patient complained of odynophagy, rhinitis, general apathy and fever which had persisted for 5 days. After initial treatment with paracetamol, ibuprofen and acetylsalicylic acid without improvement amoxicillin and diclofenac were prescribed. In addition to leukocytosis and increased C-reactive protein, there were pronounced erosive, reddened edematous and fibrin-covered alterations of the whole oral, pharyngeal and laryngeal mucous membranes as well as conjunctivitis. The lips were red with hemorrhagic scabs.

Sender's comment:

Allergic skin reactions are expected adverse drug reactions for amoxicillin. The comedication and the basic disease are alternative explanations. No further measures are considered to be necessary.

Reporter's comment:

Riemser Arzneimittel AG, 24.11.2009 DE-RIEMSER-20090338

Sender diagnosis: not reported

Narrative

This spontaneous literature case concerned a previous healthy female who experienced Fuchs syndrome while receiving paracetamol, ibuprofen and acetylsalicylic acid for a common cold and amoxicillin and diclofenac for acute pharyngitis. The medical history of the patient was unremarkable especially for allergic skin reactions.

On an unspecified date, the patient suffered from a common cold with odynophagia, rhinitis, fatigue and pyrexia and started treatment with paracetamol, ibuprofen and acetylsalicylic acid without medical advice. Three days later, there was no improvement of her symptoms and she consulted an otorhinolaryngologist. The diagnosis of an acute pharyngitis was made and treatment with amoxicillin (1000 mg tid) and diclofenac (50 mg tid) was initiated. During the following two days, the patient suffered from a painless redness of her eyes and from a progression of odynophagia. Thus, she was referred to the Department of Otorhinolaryngology of the University Hospital Bonn, Germany. On admission, the patient presented in a severely reduced general condition with pyrexia. Nutrition and fluid intake was not possible due to excessive odynophagia. There were pronounced erosive, reddened, edematous and fibrin-covered alterations of the whole oral, pharyngeal and laryngeal mucous membranes. The lips were red with hemorrhagic scabs. In addition, a conjunctivitis was noticed. Laboratory parameters were remarkable for leukocytosis and elevated CRP. The diagnosis of Fuchs syndrome, a special form of Stevens-Johnson syndrome, was made and the patient was treated with systemic corticosteroids (100 mg daily). Furthermore, antiseptic and anaesthetic mouth rinsings as well as dexpanthenol-containing ointments were prescribed. It is obvious, that all drugs were discontinued. During her hospital stay, the patient recovered completely and was discharged three weeks after admission.

Follow up information was received and the following data were added to the case: clinical course and outcome of the patient; causality assessment between the reported event and the used drugs; medical history of the patient.

Sender's comment:

The RIEMSER Arzneimittel AG holds the marketing authorization for paracetamol-containing preparations (e.g. Neopyrin forte) and for amoxicillin-containing preparations (e.g. AmoxiHefa 1000 mg). Because there was no information about the tradenames of the used paracetamol and amoxicillin in this case, it cannot be excluded, that RIEMSER products were administered. Fuchs syndrome is not listed for Neopyrin forte and AmoxiHefa. However, hypersensitivity reactions are listed for Neopyrin forte and for AmoxiHefa 1000 mg, severe allergic skin reactions are listed. The causal relationship between the reported event and paracetamol and amoxicillin was assessed as possible by the company. Alternative explanation is the concomitant medication of ibuprofen, acetylsalicylic acid and diclofenac. Furthermore, the underlying common cold might be a contributing factor for the Fuchs syndrome. The seriousness of this case is assessed as serious because a hospitalization was required. Additionally, the case was classified as medically significant by the company. Based upon this single case, the overall safety profile of the RIEMSER paracetamol- and amoxicillin-containing preparations has not been changed.

Reporter's comment:

The reporting physician assessed, that each of the used drugs might be the causative agent for the occurred Fuchs syndrome in this case.

AXCOUNT, 22.12.2009 DE-AXCOUNT-00019

Sender diagnosis: not reported

Narrative

A 24-year-old patient complained of odynophagy, rhinitis, general apathy and fever (38,5 Â°C) which had persisted for 5 days. Treatment had initially

been a combination of paracetamol, ibuprofen and acetylsalicylic acid and when no improvement occurred, this medication was stopped after 3 days and amoxicillin and diclofenac were prescribed for sore throat. Following, odynophagy was exacerbated and nutrition and fluid intake was not possible. In addition to leukocytosis and increased C-reactive protein, there were oedematous alterations of the whole oral, pharyngeal and laryngeal mucous membranes as well as conjunctivitis. The diagnosis was Fuchs' syndrome. The reaction outcome was not mentioned.

Sender's comment:

This case was classified serious because to its medical importance (since the nutrition and fluid intake were not possible due to excessive odynophagy in this case). Regarding the information on this case, it can not be excluded that Amoxicillin account 1000 mg, Tabletten was involved in the adverse reaction because this case occurred in Germany. Fuchs' syndrome is not covered by the current SPC and will be closely monitored by the MAH in the future.

Reporter's comment:

The reporters stated that Fuchs' syndrome was medication induced. The exact role of amoxicillin in the development of Fuchs' syndrome is not nearly posed.

Bausch & Lomb Incorporated, 05.01.2010 DE-BAUSCH-2009BL006723

Sender diagnosis: not reported

Narrative

A report was identified in a literature search indicating that a 24-year-old female patient complained of odynophagy, rhinitis, general apathy and fever (38.5 degrees C) which had persisted for 5 days. Treatment had initially been a combination of PARACETAMOL, IBUPROFEN and ACETYLSALICYLIC ACID (self-medication, manufacturers and doses unknown) and when no improvement occurred on the third day the medication was changed to 1000 mg AMOXICILLIN three times per day and 50 mg DICLOFENAC three times per day by an ENT doctor who made the diagnosis of angina of the salpingopalatine fold. Nutrition and fluid intake was not possible due to excessive odynophagy. In addition to leukocytosis (LEUCOCYTOSIS) (13.12 g/l) and increased C-reactive protein (C-REACTIVE PROTEIN INCREASED) (193 mg/l), there were pronounced erosive, reddened edematous and fibrin-covered alterations of the whole oral, pharyngeal and laryngeal mucous membranes as well as conjunctivitis. The lips were red with hemorrhagic scabs. The diagnosis was Fuchs' syndrome (FUCHS' SYNDROME) which is a severe medication-induced skin reaction. Abstinence from the agent (medication) responsible and early cortisone therapy are decisive. The patient received cortisone therapy of 100 mg per day. The erosive alterations of oral mucosa were treated with antiseptic, anaesthetizing and caring mouth irrigations, the lip alterations with dexpanthenol containing ointments. The outcome was not reported. No further information is available.

Source:

JAKOB M, STUHRMANN N, JORDAN J, BOOTZ F, SCHROECK A. STOMATITIS, CHEILITIS AND CONJUNCTIVITIS AFTER A COMMON COLD. HNO 2009 SEP; 57(9): 914-7.

Sender's comment:

A literature report was received indicating a serious unexpected case of FUCHS' SYNDROME in connection with the use of several drugs including paracetamol (manufacturer unknown). As Bausch and Lomb also have a marketing authorisation for paracetamol (Vivimed N gegen Fieber und Kopfschmerzen) this case is reported.

This case was assessed as serious (medical significant), unexpected and possibly related as a causal relationship between the application of paracetamol and the occurrence of this adverse drug reaction cannot be excluded. FUCHS' SYNDROME is a sub-type of the Stevens Johnsons Syndrome with a very rare frequency of about 1 out of one million patients. The Fuchs syndrome like Stevens Johnsons Syndrome may be caused by different drugs including antibiotics and NSAIDs. In this case report the patient had used several drugs that are suspected to cause this type of adverse reaction like penicilline, and several NSAIDs (ibuprofen, ASS, diclofenac, paracetamol). Therefore the contribution of paracetamol to this adverse reaction remains unclear but cannot be ruled out. In the SmPC of our paracetamol-containing product a potential of generalised hypersensitivity reactions after the use of paracetamol is already described. However this special drug reaction is not mentioned, therefore this case is assessed unexpected.

As this report is an isolated case no further measures for risk mitigation are considered necessary.

Reporter's comment:

Dr. August Wolff Arzneimittel, 20.01.2010 DE-DRWOLFF-2010-050

Sender diagnosis: not reported

Narrative

A case of Fuchs syndrome with stomatitis, cheilitis and conjunctivitis was reported in a 24-year-old female patient. The patient had a past medical history of common cold with odynophagy, rhinitis, general apathy and fever. Five days before presentation, she had taken paracetamol, ibuprofen and acetylsalicylic acid for treatment. As her symptoms did not improve, amoxicillin (3x1000mg/d) and diclofenac (3x50 mg/d) were prescribed by her treating otorhinolaryngologist based on a diagnosis of acute pharyngitis two days before presentation. The patient presented to the hospital with stomatitis, cheilitis and conjunctivitis. Physical examination revealed pronounced erosive, reddened oedematous and fibrin-covered alterations of the whole oral, pharyngeal and laryngeal mucous membranes. Relevant laboratory test results included leucocytes 13.12 G/L and CRP 193 mg/L. The patient was diagnosed with Fuchs syndrome. No further case details were reported.

Sender's comment:

This adverse reaction is unexpected for Amoxi-Wolff products. Fuchs syndrome is a subtype of Stevens-Johnson-Syndrome. However, as this term (SJS) is not sufficiently reflected in the current SPCs of Amoxi-Wolff products, the company will revise the wording.

Reporter's comment:

Dr. August Wolff Arzneimittel, 20.01.2010 DE-DRWOLFF-2010-050

Sender diagnosis: not reported

Narrative

A case of Fuchs syndrome with stomatitis, cheilitis and conjunctivitis was reported in a 24-year-old female patient. The patient had a past medical history of common cold with odynophagy, rhinitis, general apathy and fever. Five days before presentation, she had taken paracetamol, ibuprofen and acetylsalicylic acid for treatment. As her symptoms did not improve, amoxicillin (3x1000mg/d) and diclofenac (3x50 mg/d) were prescribed by her treating otorhinolaryngologist based on a diagnosis of acute pharyngitis two days before presentation. The patient presented to the hospital with stomatitis, cheilitis and conjunctivitis. Physical examination revealed pronounced erosive, reddened oedematous and fibrin-covered alterations of the whole oral, pharyngeal and laryngeal mucous membranes. Relevant laboratory test results included leucocytes 13.12 G/L and CRP 193 mg/L. The patient was diagnosed with Fuchs syndrome. No further case details were reported.

Sender's comment:

This adverse reaction is unexpected for Amoxi-Wolff products. Fuchs syndrome is a subtype of Stevens-Johnson-Syndrome. However, as this term (SJS) is not sufficiently reflected in the current SPCs of Amoxi-Wolff products, the company will revise the wording.

Reporter's comment:

Dr. August Wolff Arzneimittel, 21.01.2010 DE-DRWOLFF-2010-050

Sender diagnosis: not reported

Narrative

A case of Fuchs syndrome with stomatitis, cheilitis and conjunctivitis was reported in a 24-year-old female patient. The patient had a past medical history of common cold with odynophagy, rhinitis, general apathy and fever. Five days before presentation, she had taken paracetamol, ibuprofen and acetylsalicylic acid for treatment. As her symptoms did not improve, amoxicillin (3x1000mg/d) and diclofenac (3x50 mg/d) were prescribed by her treating otorhinolaryngologist based on a diagnosis of acute pharyngitis two days before presentation. The patient presented to the hospital with stomatitis, cheilitis and conjunctivitis. Physical examination revealed pronounced erosive, reddened oedematous and fibrin-covered alterations of the whole oral, pharyngeal and laryngeal mucous membranes. Relevant laboratory test results included leucocytes 13.12 G/L and CRP 193 mg/L. The

patient was diagnosed with Fuchs syndrome. No further case details were reported.

Sender's comment:

This adverse reaction is unexpected for Amoxi-Wolff products. Fuchs syndrome is a subtype of Stevens-Johnson-Syndrome. However, as this term (SJS) is not sufficiently reflected in the current SPCs of Amoxi-Wolff products, the company will revise the wording.

Reporter's comment:

mibe GmbH Arzneimittel, 08.03.2010 DE-MIBE-N02BA01-01032010-A

Sender diagnosis: not reported

Narrative

A 24-year-old female patient complained of odynophagy, rhinitis, general apathy and fever (38.5 degrees C) which had persisted for 5 days. Treatment had initially been a combination of paracetamol, ibuprofen and acetylsalicylic acid and when no improvement occurred amoxicillin was prescribed. Nutrition and fluid intake was not possible due to excessive odynophagy. In addition to leukocytosis and increased C-reactive protein, there were pronounced erosive, reddened edematous and fibrin-covered alterations of the whole oral, pharyngeal and laryngeal mucous membranes as well as conjunctivitis. The lips were red with hemorrhagic scabs. The diagnosis was Fuchs' syndrome which is a severe medication-induced skin

Sender's comment:

mibe comment:

Fuchs syndrome describes a variant of erythema multiforme majus which mainly involves the mucosal surfaces. Severe skin reactions such as Stevens-Johnson syndrome are known side effects on acetylsalicylic acid and ibuprofen and listed in the current SPC of Temagin ASS, Jenaprofen, Ibu-MTT and Fibralflex. Only allergic skin reactions with rash are listed for paracetamol. This case is assessed as serious and the causal relationship as possible for several suspected drugs. No safety concern is elicited from this case.

Reporter's comment:

MIP International Pharma Research GmbH, 31.05.2011 DE-MIP-hamxpo090111-hdcopo090111

Sender diagnosis: not reported

Narrative

A 24-year-old female patient complained of odynophagy, rhinitis, general apathy and fever (38.5 °C) which had persisted for 5 days. Treatment had initially been a combination of paracetamol, ibuprofen and acetylsalicylic acid and when no improvement occurred amoxicillin and diclofenac were prescribed. Nutrition and fluid intake was not possible due to excessive odynophagy. In addition to leukocytosis and increased C-reactive protein, there were pronounced erosive, reddened edematous and fibrin-covered alterations of the whole oral, pharyngeal and laryngeal mucous membranes as well as conjunctivitis. The lips were red with hemorrhagic scabs. The diagnosis was Fuchs' syndrome which is a severe medication-induced skin reaction. Abstinence from the agent (medication) responsible and early cortisone therapy are decisive.

Sender's comment:

The case was detected in the MEDLINE database on 18th May 2011. In the case, no product name or MAH could be identified. The case could theoretically be in connection with amoxicillin- or diclofenac-containing products of the MAH. Therefore, the case was recorded for both active substances in the database of the MAH as case hamxpo090111 and case hdcopo090111. The diagnosed Fuchs' syndrome is a special form of

Stevens-Johnson-Syndrom concerning the eye and oral mucosa. According to the treating physician, the Fuchs' Syndrom belongs to serious drug-induced skin reactions. Skin and subcutaneous disorders like exanthema, rash and allergic reactions are known ADRs of both drugs. The cases were classified as labelled for diclofenac and as unlabelled for amoxicillin. The case was classified as serious due to the medically important condition. Based on the available data, the MAH evaluated the causality as possible for both drugs. The reported case does not change the risk/benefit assessment for Amoxi-saar 500/750/1000 mg Tabletten as well as Diclo-saar 50/100 mg Tabletten. Stevens-Johnson Syndrome is also labelled for the co-medication ASS and Ibuprofen.

Reporter's comment:

McNeil Germany, Local Safety Officers, 17.04.2013 DE-JNJFOC-20130407230

Sender diagnosis: not reported

Narrative

Report was received from literature.

Jakob M, Stuhmann N, Jordan J, Bootz F, Schrock A. Stomatitis, cheilitis and conjunctivitis after a common cold. Hno 2009; 57(9):914-917

This report concerns a 24-year-old female patient. The patient's height, weight and medical history were not reported. The patient had complained of odynophagy, rhinitis, general apathy and fever (38.5degreeC) which had persisted for 5 days. The patient was treated with the combination of Ibuprofen (unspecified formulation and route, batch unspecified) dose and frequency unspecified and non-company suspect drug(s) paracetamol (unspecified formulation and route, batch unspecified) dose and frequency unspecified and acetylsalicylic acid (unspecified formulation and route, batch unspecified) dose and frequency unspecified and when no improvement occurred amoxicillin (unspecified formulation and route, batch unspecified) dose and frequency unspecified was prescribed for odynophagy, rhinitis, general apathy and fever. The patient's nutrition and fluid intake was not possible due to excessive odynophagy. In addition to leukocytosis and increased C-reactive protein, there were pronounced erosive, reddened edematous and fibrin-covered alterations of the whole oral, pharyngeal and laryngeal mucous membranes as well as conjunctivitis. The lips were red with hemorrhagic scabs. The diagnosis was Fuchs syndrome which was a severe medication induced skin reaction. Abstinence from the agent (medication) responsible and early cortisone therapy were decisive. Action taken with ibuprofen, paracetamol, acetylsalicylic acid and amoxicillin were not reported. The patient's outcome was not reported for Fuchs syndrome at the time of this report.

This report is serious (medically significant).<dbtext>To_Char_NARRATIVE_2</dbtext>

Sender's comment:

Reporter's comment:

PFIZERINC, 23.04.2013 DE-PFIZER INC-2013120405

Sender diagnosis: not reported

Narrative

This is a literature report from the Hno 2009; 57(9):914-917, entitled "Stomatitis, cheilitis and conjunctivitis after a common cold", based on information received by Pfizer from Johnson and Johnson (manufacturer control number DE-JNJFOC-20130407230), license party for ibuprofen.

Report was received from literature. Jakob M, Stuhmann N, Jordan J, Bootz F, Schrock A. Stomatitis, cheilitis and conjunctivitis after a common cold. Hno 2009; 57(9):914-917. This report concerns a 24-year-old female patient. The patient's height, weight and medical history were not reported. The patient had complained of odynophagy, rhinitis, general apathy and fever (38.5degreeC) which had persisted for 5 days. The patient was treated with the combination of Ibuprofen (unspecified formulation and route, batch unspecified) dose and frequency unspecified and non-company suspect drug (s) paracetamol (unspecified formulation and route, batch unspecified) dose and frequency unspecified and acetylsalicylic acid (unspecified formulation and route, batch unspecified) dose and frequency unspecified and when no improvement occurred amoxicillin (unspecified formulation and route, batch unspecified) dose and frequency unspecified was prescribed for odynophagy, rhinitis, general apathy and fever. The patient's nutrition and fluid intake was not possible due to excessive odynophagy. In addition to leukocytosis and increased C-reactive protein, there were pronounced erosive, reddened edematous and fibrin-covered alterations of the whole oral, pharyngeal and laryngeal mucous membranes as well as conjunctivitis.

The lips were red with hemorrhagic scabs. The diagnosis was Fuchs syndrome which was a severe medication induced skin reaction. Abstinence from the agent (medication) responsible and early cortisone therapy were decisive. Action taken with ibuprofen, paracetamol, acetylsalicylic acid and amoxicillin were not reported. The patient's outcome was not reported for Fuchs syndrome at the time of this report. This report is serious (medically significant).

Causality Assessment per reporter for the event is "Possible".

Johnson and Johnson Causality assessment: the event of Fuchs syndrome was considered possibly related to ibuprofen.

Pfizer is Marketing Authorization Holder of ibuprofen in the reporter's country. This may be a duplicate report in situations where another Marketing Authorization Holder of ibuprofen has submitted the same report to the regulatory authorities.

Sender's comment:

Based on the information provided, a possible contributory role of the suspect drug to the reported event Fuch's syndrome cannot be excluded. The impact of this report on the benefit/risk profile of the Pfizer product is evaluated as part of Pfizer procedures for safety evaluation, including the review and analysis of aggregate data for adverse events. Any safety concern identified as part of this review, as well as any appropriate action in response, will be promptly notified to Regulatory Authorities, Ethics Committees and Investigators, as appropriate.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Report from study

case: DE-BFARM-10024784

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	Yes		

Patient

Initials:	Sex: Female	Birth: privacy	Age: 53 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Mucosal erosion	20070726			36 Day	Unknown
2 Chronic obstructive pulmonary disease	200707			11 Day	Unknown
3 Oral mucosa erosion	20070724			34 Day	Unknown
4 Lip erosion	20070723			33 Day	Unknown
5 Oral mucosal erythema	200707			11 Day	Unknown
6 Eye red	20070719			29 Day	Unknown
7 Macule	20070718			28 Day	Unknown
8 Lip swelling	20070717			27 Day	Unknown
9 Eye swelling	20070717			27 Day	Unknown
10 Prurigo	20070717			27 Day	Unknown
11 Genital erosion	20070726			36 Day	Unknown
12 Fever	20070714	20070718	5 Day	24 Day	recovered/resolved
13 Lobar pneumonia	20070714			17 Day	Unknown
14 Stevens Johnson syndrome	20070717	20070809	24 Day	11 Day	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
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DE-BFARM-10024784

1	Ambroxol ambroxol	Oral	60 Gtt drop(s)/Day 158	20070706-20070714 9 Day	
2	Berodual fenoterol hydrobromide; ipratropium bromide	Respiratory (inhalation)	6 Df dosage form/Day 110	20070715- -	Chronic obstructive pulmonary disease
3	Cefuroxim cefuroxime sodium	Intravenous (not otherwise)	4.5 G gram(s)/Day 120	20070714-20070714 1 Day	Pneumonia
4	Ergenyl valproate sodium	Oral	2400 Mg milligram(s)/Day	20070714- -	Epilepsy
5	Heparin heparin	Subcutaneous	15000 Iu international unit 120	20070714-20070717 4 Day	Thrombosis prophylaxis
6	inflanefran prednisolone acetate	Intraocular	/ 158	20070620-20070706 17 Day	Conjunctivitis
7	kalinor potassium chloride	Oral	/Day	20070621- -	Hypokalemia
8	Lamictal lamotrigine	Oral	/ 245	20070703-20070719 17 Day	Epilepsy
9	mono-embolex certoparin sodium	Subcutaneous	1 Df dosage form/Day 120	20070620-20070706 17 Day	Thrombosis prophylaxis
10	Mucosolvan ambroxol	Oral	/ 231	20070629-20070703 5 Day	
11	nac 600 acetylcysteine	Oral	600 Mg milligram(s)/Day	20070714- -	
12	novalgin metamizole	Intravenous (not otherwise)	1 G gram(s)/Day	20070717-20070717 1 Day	Fever
13	Orfiril valproate sodium	Oral	/ 170	20070620-20070706 17 Day	Epilepsy
14	Paracetamol paracetamol	Oral	500 Mg milligram(s)/Day	20070714-20070716 3 Day	Fever
15	Rocephin ceftriaxone sodium	Intravenous (not otherwise)	2 G gram(s)/Day 120	20070715-20070717 3 Day	Pneumonia
16	Rulid roxithromycin	Oral	300 Mg milligram(s)/Day	20070715-20070717 3 Day	Pneumonia
17	Seroquel quetiapine fumarate	Oral	50 Mg milligram(s)/Day	20070627- -	Schizophrenia
18	Valproinsaeure valproic acid	Oral	2400 Mg milligram(s)/Day	20070706-20070714 9 Day	Epilepsy

Drugs (concomitant)

19	multiple medication homeopaths nos	Unknown	/	- -	
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Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Prophylaxis			Unknown	
Encephalitis			Unknown	
Thrombosis prophylaxis			No	
Productive cough			Yes	
Hypokalemia			Yes	
Conjunctivitis			Yes	
Schizophrenia			Unknown	

Primary source

Qualification:	Physician	observestudytype:	Other studies
Literature:			
Qualification:	Other Health Professional	observestudytype:	Other studies
Literature:			

Narratives (nur bei elektronischem Eingang)

GlaxoSmithKline, 17.02.2010 DE-GlaxoSmithKline-D0066487A

Sender diagnosis: not reported

Narrative

This case was reported by a physician and described the occurrence of Stevens-johnson syndrome in a 53-year-old female patient who received Lamotrigine (Lamictal) tablet over a period of 17 days for epilepsy.

This report was received from the Intensified Monitoring from the Documentation Centre for Severe Skin Reactions in Freiburg.

Concurrent medical conditions included suspected encephalitis, conjunctivitis, gastric protection, hypokalemia, productive cough, schizophrenia and thrombosis prophylaxis.

Co-suspect medication included cefuroxime sodium (Cefuroxim), ceftriaxone sodium (Rocephin), roxithromycin (Rulid), dipyrone (Novalgin), quetiapine fumarate (Seroquel), Valproic acid, Ambroxol, Ambroxol (Mucosolvan), Paracetamol, Heparin, valproate sodium (Ergenyl), acetylcysteine (NAC 600), Berodual, valproate sodium (Orfiril), low molecular weight heparin (Mono-embolex), potassium chloride (Kalinor) and prednisolone (Inflanefran).

The patient also received 13 concurrent or treatment drugs.

On 3 July 2007 the patient started Lamotrigine (oral).

Approximately 11 days after starting Lamotrigine, on 14 July 2007, the patient experienced lobar pneumonia and fever.

Additionally the patient was treated for chronic obstructive pulmonary disease, but the start date of this event was not reported.

The patient was hospitalized from 14 - 26 July 2007 at the hospital for internal medicine.

The patient was treated with cefuroxime sodium, Paracetamol, Heparin, acetylcysteine, ceftriaxone sodium, roxithromycin, Berodual and dipyrone.

On 17 July 2007, the patient experienced pruriginous exanthema, eye and lip swelling. On 18 July 2007, the patient experienced macule. On 19 July 2007, the patient experienced redness of eyes.

Additionally the patient was treated for oral mucosal erythema, but the start date of this event was not reported.

Treatment with Lamotrigine was discontinued on 19 July 2007.

On 23 July 2007, the patient experienced lip erosion. On 24 July 2007, the patient experienced oral mucosa erosion and on 26 July 2007, the patient experienced erosion of genital mucosa and nasal mucosa. Stevens-johnson syndrome was diagnosed.

From 26 July to 9 August 2007 the patient stayed at the dermatological hospital. Lamotrigine was reintroduced on the same day.

Stevens-johnson syndrome resolved on 9 August 2007.

At the time of reporting, at the latest on September 2007, the outcome of the other events was unknown.

The reporting physician considered Stevens-johnson syndrome was probably related to treatment with Lamotrigine and possibly related to all co-suspect medication. Lobar pneumonia was also considered a possible cause of Stevens-johnson syndrome.

No further information will be available.

Sender's comment:

Die UAW ist erwartet.

Wir sehen hier einen kausalen Zusammenhang als moeglich.

Auf der Basis dieses Einzelfalles ergibt sich keine Aenderung des wissenschaftlichen Kenntnisstandes und der Nutzen/Risiko-Relation. Weitergehende eigenverantwortliche Massnahmen halten wir derzeit nicht fuer erforderlich.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-10021913

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	Yes	Yes	No	No	No		Yes

Patient

Initials:	Sex: Male	Birth: privacy	Age: 45 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Gastritis	200901			0 Month	Unknown
2 Hypertension	200901			0 Month	Unknown
3 Macule	20090121			20 Day	Unknown
4 TEN	20090121			15 Day	not recovered/not resolved
5 Conjunctivitis	20090129			28 Day	Unknown
6 Blister	20090121			20 Day	Unknown
7 Pneumonia aspiration	20090103				Unknown
8 Toxic epidermal necrolysis	20090121				not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
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DE-BFARM-10021913

1	albuterol salbutamol	Oral	20 Gtt drop(s)/Day	20090106-20090113 Cough 8 Day
2	BELOC metoprolol tartrate	Oral	/ 245	20090101-20090105 Hypertension arterial 5 Day
3	BELOC metoprolol tartrate	Oral	/ 245	20090112-20090119 Hypertension arterial 8 Day
4	berlosin metamizole	Intravenous (not otherwis	/ 120	20090113-20090113 Pain 1 Day
5	berlosin metamizole	Intravenous (not otherwis	/ 120	20090101-20090104 Pain 4 Day
6	berlosin metamizole	Intravenous (not otherwis	/ 120	20090106-20090110 Pain 5 Day
7	CIPRALEX escitalopram oxalate	Oral	10 Mg milligram(s)/Day 245	20090103-20090121 Sedation 19 Day
8	clonidine clonidine	Intravenous (not otherwis	/ 120	20090110-20090116 Withdrawal syndrome 7 Day
9	COMBACTAM sulbactam sodium	Intravenous (not otherwis	1.5 G gram(s)/Day 120	20090107-20090113 Pneumonia aspiration 7 Day
10	DELIX (ramipril) ramipril	Oral	/ 245	20090112-20090121 Hypertension arterial 10 Day
11	DELIX (ramipril) ramipril	Oral	/ 245	20090103-20090105 Hypertension arterial 3 Day
12	DORMICUM (midazolam) midazolam	Intravenous (not otherwis	/ 120	20090104-20090114 Sedation 11 Day
13	EBRANTIL urapidil	Intravenous (not otherwis	/ 120	20090115-20090119 Hypertension arterial 5 Day
14	erythromycin erythromycin	Intravenous (not otherwis	600 Mg milligram(s)/Day 120	20090111-20090113 Pneumonia aspiration 3 Day
15	erythromycin erythromycin	Intravenous (not otherwis	600 Mg milligram(s)/Day 120	20090105-20090109 Pneumonia aspiration 5 Day
16	FRAXIPARIN nadroparin calcium	Subcutaneous	/ 120	20090114- - Thrombosis prophylaxis
17	gastrozepin pirenzepine dihydrochloride	Oral	75 Mg milligram(s)/Day 245	20090116-20090119 Gastritis 4 Day
18	hydrochlorothiazide hydrochlorothiazide	Oral	/ 245	20090101-20090105 Hypertension arterial 5 Day
19	insulin insulin	Intravenous (not otherwis	/ 120	20090104-20090119 Hyperglycaemia 16 Day
20	ipratropium bromide ipratropium bromide	Oral	20 Gtt drop(s)/Day	20090106-20090113 Cough 8 Day
21	L-THYROXIN levothyroxine sodium	Oral	/ 245	20090106-20090119 Hyperthyroidism 14 Day
22	MONO EMBOLEX certoparin sodium	Subcutaneous	/ 120	20090102-20090114 Thrombosis prophylaxis 13 Day
23	MOTILUM domperidone	Oral	10 Mg milligram(s)/Day 245	20090105-20090116 Vomiting 12 Day
24	MUCOSOLVAN ambroxol hydrochloride	Respiratory (inhalation)	3 Df dosage form/Day	20090105-20090115 Respiratory tract congestion 11 Day
25	NEXIUM esomeprazole magnesium	Oral	20 Mg milligram(s)/Day 245	20090102-20090121 Gastric ulcer prophylaxis 20 Day
26	norvasc amlodipine besilate	Oral	/ 245	20090115- - Hypertension arterial
27	Paracetamol paracetamol	Intravenous (not otherwis	/ 120	20090109-20090112 Pain 4 Day
28	Paracetamol paracetamol	Intravenous (not otherwis	/ 120	20090113-20090118 Pain 6 Day

DE-BFARM-10021913

29	Paracetamol paracetamol	Intravenous (not otherwis / 120	20090101-20090105 Pain 5 Day
30	PASPERTIN metoclopramide hydrochloride	Intravenous (not otherwis 20 Mg milligram(s)/Day 120	20090105-20090115 Vomiting 11 Day
31	piperacillin piperacillin	Intravenous (not otherwis 12 G gram(s)/Day 120	20090107-20090113 Pneumonia aspiration 7 Day
32	propofol propofol	Intravenous (not otherwis / 120	20090104-20090114 Sedation 11 Day
33	sufentanil sufentanil	Intravenous (not otherwis / 120	20090104-20090119 Pain 16 Day
34	tavanic levofloxacin	Oral 1500 Mg milligram(s)/Day 245	20090116- - Pneumonia aspiration
35	UNACID ampicillin sodium; sulbactam sodium	Intravenous (not otherwis 9 G gram(s)/Day 120	20090114-20090118 Pneumonia aspiration 5 Day
36	ZOCOR simvastatin	Oral 20 Mg milligram(s)/Day 245	20090102-20090114 Hyperlipidaemia 13 Day

Drugs (concomitant)					
37	ACC (acetylcysteine) acetylcysteine	Oral	600 Mg milligram(s)/Day 245	20090105- -	Respiratory tract congestion
38	allopurinol allopurinol	Oral	300 Mg milligram(s)/Day 245	20050101-20090107 4 Year	Hyperuricaemia
39	ATACAND candesartan cilexetil	Oral	/ 245	20090119- -	Hypertension arterial
40	BELOC-ZOK metoprolol succinate	Oral	2 Df dosage form/Day 245	20090119- -	Hypertension arterial
41	BERLTHYROX levothyroxine sodium	Oral	75 µg microgram(s)/Day 245	20090119- -	Hyperthyroidism
42	BIFITERAL lactulose	Oral	30 ml millilitre(s)/Day 244	20090102- -	Constipation
43	calcium chloride / magnesium chloride / potassi	Intravenous (not otherwis	/ 230	20090102-20090119 18 Day	
44	dobutamine dobutamine	Intravenous (not otherwis	/ 120	20090105-20090107 3 Day	Decompensation cardiac
45	fresubin carbohydrates nos; fatty acids nos; minerals nos; proteins nos; vitamins nos	Oral	/	20090102-20090119 18 Day	Nutritional supplement
46	haloperidol haloperidol	Intravenous (not otherwis	/ 120	20090109-20090109 1 Day	Withdrawal syndrome
47	INEGY ezetimibe; simvastatin	Oral	Mg milligram(s)/Day 245	20010101-20090101 8 Year	Hyperlipidaemia
48	Kalium potassium	Oral	/ 245	20090102-20090104 3 Day	Hypokalaemia
49	Kalium potassium	Oral	/ 245	20090116-20090119 4 Day	Hypokalaemia
50	Kalium potassium	Oral	/ 245	20090108-20090112 5 Day	Hypokalaemia
51	Kaliumchlorid potassium chloride	Intravenous (not otherwis	/ 230	20090101-20090116 16 Day	Hypokalaemia
52	norepinephrine norepinephrine	Intravenous (not otherwis	/ 120	20090102-20090109 8 Day	Cardiovascular disorder
53	norepinephrine norepinephrine	Unknown	/ 120	- -	Cardiovascular disorder
54	Paracetamol paracetamol	Rectal	/ 242	20090119- -	
55	periplasmal amino acids nos; electrolytes nos; sorbitol; xylitol	Intravenous (not otherwis	/ 230	20090106-20090110 5 Day	
56	periplasmal amino acids nos; electrolytes nos; sorbitol; xylitol	Intravenous (not otherwis	/ 230	20090114-20090115 2 Day	
57	periplasmal amino acids nos; electrolytes nos; sorbitol; xylitol	Intravenous (not otherwis	/ 230	20090102-20090102 1 Day	
58	scopoderm tts hyoscine	Topical	/ 250	20090119- -	Pain
59	structokabiven amino acids nos	Intravenous (not otherwis	/ 230	20090109-20090112 4 Day	Nutritional supplement

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Hyperthyroidism			Yes	
Hyperuricaemia			Yes	
Hypokalaemia			Yes	
Pain			Yes	
Respiratory tract congestion			Yes	
Vomiting			Yes	
Withdrawal syndrome			Yes	
Anemia			Unknown	
Cerebral edema			Unknown	
Hypertension arterial			Yes	
Hyperglycaemia			Yes	
Hospitalisation			Yes	
Gastritis			Yes	
Decompensation cardiac			Yes	
Cough			Yes	
Constipation			Yes	
Cardiovascular disorder			Yes	

Primary source

Qualification: Physician observestudytype:
 Literature:

Narratives (nur bei elektronischem Eingang)

009507513, 19.02.2010 DE-MERCK-1002DEU00076

Sender diagnosis: not reported

Narrative

Information has been received from a physician of a registry center (documenting serious skin reactions that occur in Germany) concerning a 45-year-old male patient (hospitalized since 01-JAN-2009; neurology clinic) with pain, arterial hypertension, hyperglycemia, respiratory tract congestion, vomiting, cough, hyperthyroidism, withdrawal syndrome, gastritis, hyperuricemia, cardiovascular disorder, hypokalemia, constipation, cardiac decompensation who, on 02-JAN-2009, was placed on therapy with simvastatin, 20 mg tablet, 20 mg, once a day for the treatment of hyperlipidemia. Prior therapy included ezetimibe (+) simvastatin (MSD). Concomitant suspect therapy included dipyrone (BERLOSIN), 1 gm injection (form), IV for the treatment of pain (therapy started on 01-JAN-2009; total daily dose not reported), acetaminophen, 1 gm injection (form), IV for the treatment of pain (therapy started on 01-JAN-2009; total daily dose not reported), metoprolol tartrate (BELOC), tablet for the treatment of arterial hypertension (therapy started on 01-JAN-2009; total daily dose not reported), hydrochlorothiazide (not MSD), 12.5 mg tablet for the treatment of arterial hypertension (therapy started on 01-JAN-2009; total daily dose not reported), certoparin sodium (MONO EMBOLIX), injection (form), SC for thrombosis prophylaxis (therapy started on 02-JAN-2009; total daily dose not reported) and esomeprazole magnesium (NEXIUM), 20 mg tablet, 20 mg, once a day for gastric ulcer protection (therapy started on 02-JAN-2009; duration not reported). Other concomitant therapy included allopurinol, potassium chloride, norepinephrine, amino acids (unspecified) (+) dextrose (+) electrolytes (unspecified) (PERIPLASMAL), potassium (unspecified), nutritional supplements (FRESUBIN), calcium chloride (+) magnesium chloride (+) potassium chloride (+) sodium acetate (+) sodium chloride, lactulose (BIFITERAL), dobutamine, acetylcysteine (ACC), haloperidol, amino acids (unspecified) (+) dextrose (+) electrolytes (unspecified) (+) lipids (unspecified) (STRUCTOKABIVEN), acetaminophen, candesartan cilexetil (ATACAND), metoprolol succinate (BELOC-ZOK), levothyroxine Na (BERLTHYROX) and scopolamine (SCOPODERM TTS).

On 03-JAN-2009, the patient experienced aspiration pneumonia. That same day, he was placed on suspect therapy with ramipril (DELIX), 5 mg tablet for the treatment of arterial hypertension (total daily dose not reported) and escitalopram oxalate (CIPRALEX), 10 mg tablet, 10 mg, once a day for sedation (duration not reported). On 04-JAN-2009, therapy with dipyrone was interrupted; the patient was placed on suspect therapy with midazolam (DORMICUM), 100 mg injection (form), IV for sedation (total daily dose not reported), propofol, 2% injection (form), IV for sedation (total daily dose not reported), insulin, injection (form), IV for the treatment of hyperglycemia (total daily dose not reported) and sufentanil, 500 mcg injection (form), IV for the treatment of pain (total daily dose not reported). On 05-JAN-2009, therapy with acetaminophen, metoprolol tartrate, hydrochlorothiazide and ramipril was interrupted; the patient was placed on suspect therapy with erythromycin, 200 mg injection (form), 200 mg, three times a day, IV for the treatment of aspiration pneumonia, ambroxol hydrochloride (MUCOSOLVAN), liquid for inhalation, 1 dose, three times a day for the treatment of respiratory tract congestion, metoclopramide hydrochloride (PASPERTIN), 10 mg injection (form), 10 mg, twice a day, IV for the treatment of vomiting and domperidone (MOTILIUM), 10 mg tablet, 10 mg, once a day for the treatment of vomiting.

On 06-JAN-2009, therapy with dipyrone was restarted; the patient was placed on suspect therapy with ipratropium bromide, liquid, 5 drops, four times a day for the treatment of cough, albuterol, liquid, 5 drops, four times a day for the treatment of cough and levothyroxine Na (L-THYROXIN), tablet for

the treatment of hypothyroidism (total daily dose not reported). On 07-JAN-2009, the patient was placed on suspect therapy with sulbactam sodium (COMBACTAM), 0.5 gm injection (form), 0.5 gm, three times a day, IV for the treatment of aspiration pneumonia and piperacillin, 4 gm injection (form), 4 gm, three times a day, IV for the treatment of aspiration pneumonia. On 09-JAN-2009, therapy with acetaminophen was restarted and therapy with erythromycin was interrupted. On 10-JAN-2009, therapy with dipyrone was interrupted; the patient was placed on suspect therapy with clonidine, 750 mcg injection (form), IV for the treatment of withdrawal syndrome (total daily dose not reported). On 11-JAN-2009, therapy with erythromycin was restarted. On 12-JAN-2009, therapy with acetaminophen was interrupted and therapy with metoprolol tartrate and ramipril was restarted (ramipril duration not reported). On 13-JAN-2009, therapy with dipyrone was restarted and stopped that same day; therapy with acetaminophen was restarted; therapy with erythromycin, ambroxol hydrochloride, ipratropium bromide, albuterol, sulbactam sodium and piperacillin was stopped.

On 14-JAN-2009, therapy with simvastatin, certoparin sodium, midazolam and propofol was stopped (reason not reported); the patient was placed on suspect therapy with ampicillin sodium (+) sulbactam sodium (UNACID), 3 gm injection (form), 3 gm, three times a day for the treatment of aspiration pneumonia and nadroparin calcium (FRAXIPARIN), 0.4 ml injection (form), SC for thrombosis prophylaxis (total daily dose not reported). On 15-JAN-2009, therapy with hydrochlorothiazide was restarted (duration not reported); therapy with metoclopramide hydrochloride was stopped; the patient was placed on suspect therapy with urapidil (EBRANTIL), 250 mg injection (form), IV for the treatment of arterial hypertension (total daily dose not reported) and amlodipine besylate (NORVASC), 10 mg tablet for the treatment of arterial hypertension (total daily dose not reported). On 16-JAN-2009, therapy with domperidone (MOTILIUM), clonidine and ampicillin sodium (+) sulbactam sodium was stopped; the patient was placed on suspect therapy with pirenzepine hydrochloride (GASTROZEPIN), 25 mg tablet, 25 mg, three times a day for the treatment of gastritis, and levofloxacin (TAVANIC), 500 mg tablet, 500 mg, three times a day for the treatment of aspiration pneumonia (duration not reported). On 18-JAN-2009, therapy with acetaminophen was stopped. On 19-JAN-2009, therapy with metoprolol tartrate, insulin, sufentanil, levothyroxine Na, urapidil and pirenzepine hydrochloride was stopped. That same day, the patient was transferred to a rehabilitation clinic.

On 21-JAN-2009 the patient developed target lesions, spots and blisters on his skin. On 26-FEB-2009, he developed erosions of his genital mucosa. On 29-JAN-2009, he developed conjunctivitis. A diagnosis of toxic epidermal necrolysis was made. The patient's toxic epidermal necrolysis persisted.

The reporting physician felt that toxic epidermal necrolysis was quite possibly related to therapy with dipyrone (BERLOSIN), erythromycin, sulbactam sodium (COMBACTAM), piperacillin, ampicillin sodium (+) sulbactam sodium (UNACID) and levofloxacin (TAVANIC), and possibly related to therapy with simvastatin, acetaminophen, metoprolol tartrate (BELOC), hydrochlorothiazide, certoparin sodium (MONO EMBOLEX), esomeprazole magnesium (NEXIUM), ramipril (DELIX), escitalopram oxalate (CIPRALEX), midazolam (DORMICUM), propofol, insulin, sufentanil, ambroxol hydrochloride (MUCOSOLVAN), metoclopramide hydrochloride (PASPERTIN), domperidone (MOTILIUM), ipratropium bromide, albuterol, levothyroxine Na (L-THYROXIN), clonidine, nadroparin calcium (FRAXIPARIN), urapidil (EBRANTIL), amlodipine besylate (NORVASC) and pirenzepine hydrochloride (GASTROZEPIN). She also took the patient's aspiration pneumonia into consideration as a possible cause.

Toxic epidermal necrolysis prolonged hospitalization and was considered to be immediately life-threatening.

Information received on 15-FEB-2010 contained the following adverse experience: aspiration pneumonia (03-JAN-2009) which was treated with metronidazole, cefotaxime sodium (CLAFORAN), erythromycin, sulbactam sodium (COMBACTAM), piperacillin, ampicillin sodium (+) sulbactam sodium (UNACID) and levofloxacin (TAVANIC)).

Additional information is not expected.

Sender's comment:

Toxic epidermal necrolysis is a serious unlabeled event for therapy with Zocor, aspiration pneumonia is nonserious and unlabeled. This case is confounded by the patient's co-morbidity and the numerous concomitant medications. Whether Zocor played a causative role in this complex scenario, cannot be assessed based on the available data. The safety profile remains unchanged. Self-initiated measures are not indicated.

Reporter's comment:

009507513, 23.02.2010 DE-MERCK-1002DEU00076

Sender diagnosis: not reported

Narrative

Information has been received from a physician of a registry center (documenting serious skin reactions that occur in Germany) concerning a 45-year-old male patient (hospitalized since 01-JAN-2009; neurology clinic) with pain, arterial hypertension, hyperglycemia, respiratory tract congestion, vomiting, cough, hyperthyroidism, withdrawal syndrome, gastritis, hyperuricemia, cardiovascular disorder, hypokalemia, constipation, cardiac decompensation who, on 02-JAN-2009, was placed on therapy with simvastatin, 20 mg tablet, 20 mg, once a day for the treatment of hyperlipidemia. Prior therapy included ezetimibe (+) simvastatin (MSD). Concomitant suspect therapy included dipyrone (BERLOSIN), 1 gm injection (form), IV for the

treatment of pain (therapy started on 01-JAN-2009; total daily dose not reported), acetaminophen, 1 gm injection (form), IV for the treatment of pain (therapy started on 01-JAN-2009; total daily dose not reported), metoprolol tartrate (BELOC), tablet for the treatment of arterial hypertension (therapy started on 01-JAN-2009; total daily dose not reported), hydrochlorothiazide (not MSD), 12.5 mg tablet for the treatment of arterial hypertension (therapy started on 01-JAN-2009; total daily dose not reported), certoparin sodium (MONO EMBOLEX), injection (form), SC for thrombosis prophylaxis (therapy started on 02-JAN-2009; total daily dose not reported) and esomeprazole magnesium (NEXIUM), 20 mg tablet, 20 mg, once a day for gastric ulcer protection (therapy started on 02-JAN-2009; duration not reported). Other concomitant therapy included allopurinol, potassium chloride, norepinephrine, amino acids (unspecified) (+) dextrose (+) electrolytes (unspecified) (PERIPLASMAL), potassium (unspecified), nutritional supplements (FRESUBIN), calcium chloride (+) magnesium chloride (+) potassium chloride (+) sodium acetate (+) sodium chloride, lactulose (BIFITERAL), dobutamine, acetylcysteine (ACC), haloperidol, amino acids (unspecified) (+) dextrose (+) electrolytes (unspecified) (+) lipids (unspecified) (STRUCTOKABIVEN), acetaminophen, candesartan cilexetil (ATACAND), metoprolol succinate (BELOC-ZOK), levothyroxine Na (BERLTHYROX) and scopolamine (SCOPODERM TTS).

On 03-JAN-2009, the patient experienced aspiration pneumonia. That same day, he was placed on suspect therapy with ramipril (DELIX), 5 mg tablet for the treatment of arterial hypertension (total daily dose not reported) and escitalopram oxalate (CIPRALEX), 10 mg tablet, 10 mg, once a day for sedation (duration not reported). On 04-JAN-2009, therapy with dipyrone was interrupted; the patient was placed on suspect therapy with midazolam (DORMICUM), 100 mg injection (form), IV for sedation (total daily dose not reported), propofol, 2% injection (form), IV for sedation (total daily dose not reported), insulin, injection (form), IV for the treatment of hyperglycemia (total daily dose not reported) and sufentanil, 500 mcg injection (form), IV for the treatment of pain (total daily dose not reported). On 05-JAN-2009, therapy with acetaminophen, metoprolol tartrate, hydrochlorothiazide and ramipril was interrupted; the patient was placed on suspect therapy with erythromycin, 200 mg injection (form), 200 mg, three times a day, IV for the treatment of aspiration pneumonia, ambroxol hydrochloride (MUCOSOLVAN), liquid for inhalation, 1 dose, three times a day for the treatment of respiratory tract congestion, metoclopramide hydrochloride (PASPERTIN), 10 mg injection (form), 10 mg, twice a day, IV for the treatment of vomiting and domperidone (MOTILIUM), 10 mg tablet, 10 mg, once a day for the treatment of vomiting.

On 06-JAN-2009, therapy with dipyrone was restarted; the patient was placed on suspect therapy with ipratropium bromide, liquid, 5 drops, four times a day for the treatment of cough, albuterol, liquid, 5 drops, four times a day for the treatment of cough and levothyroxine Na (L-THYROXIN), tablet for the treatment of hypothyroidism (total daily dose not reported). On 07-JAN-2009, the patient was placed on suspect therapy with sulbactam sodium (COMBACTAM), 0.5 gm injection (form), 0.5 gm, three times a day, IV for the treatment of aspiration pneumonia and piperacillin, 4 gm injection (form), 4 gm, three times a day, IV for the treatment of aspiration pneumonia. On 09-JAN-2009, therapy with acetaminophen was restarted and therapy with erythromycin was interrupted. On 10-JAN-2009, therapy with dipyrone was interrupted; the patient was placed on suspect therapy with clonidine, 750 mcg injection (form), IV for the treatment of withdrawal syndrome (total daily dose not reported). On 11-JAN-2009, therapy with erythromycin was restarted. On 12-JAN-2009, therapy with acetaminophen was interrupted and therapy with metoprolol tartrate and ramipril was restarted (ramipril duration not reported). On 13-JAN-2009, therapy with dipyrone was restarted and stopped that same day; therapy with acetaminophen was restarted; therapy with erythromycin, ambroxol hydrochloride, ipratropium bromide, albuterol, sulbactam sodium and piperacillin was stopped.

On 14-JAN-2009, therapy with simvastatin, certoparin sodium, midazolam and propofol was stopped (reason not reported); the patient was placed on suspect therapy with ampicillin sodium (+) sulbactam sodium (UNACID), 3 gm injection (form), 3 gm, three times a day for the treatment of aspiration pneumonia and nadroparin calcium (FRAXIPARIN), 0.4 ml injection (form), SC for thrombosis prophylaxis (total daily dose not reported). On 15-JAN-2009, therapy with hydrochlorothiazide was restarted (duration not reported); therapy with metoclopramide hydrochloride was stopped; the patient was placed on suspect therapy with urapidil (EBRANTIL), 250 mg injection (form), IV for the treatment of arterial hypertension (total daily dose not reported) and amlodipine besylate (NORVASC), 10 mg tablet for the treatment of arterial hypertension (total daily dose not reported). On 16-JAN-2009, therapy with domperidone (MOTILIUM) and clonidine was stopped; the patient was placed on suspect therapy with pirenzepine hydrochloride (GASTROZEPIN), 25 mg tablet, 25 mg, three times a day for the treatment of gastritis, and levofloxacin (TAVANIC), 500 mg tablet, 500 mg, three times a day for the treatment of aspiration pneumonia (duration not reported). On 18-JAN-2009, therapy with acetaminophen and ampicillin sodium (+) sulbactam sodium was stopped. On 19-JAN-2009, therapy with metoprolol tartrate, insulin, sufentanil, levothyroxine Na, urapidil and pirenzepine hydrochloride was stopped. That same day, the patient was transferred to a rehabilitation clinic.

On 21-JAN-2009 the patient developed target lesions, spots and blisters on his skin. On 26-FEB-2009, he developed erosions of his genital mucosa. On 29-JAN-2009, he developed conjunctivitis. A diagnosis of toxic epidermal necrolysis was made. The patient's toxic epidermal necrolysis persisted.

The reporting physician felt that toxic epidermal necrolysis was quite possibly related to therapy with dipyrone (BERLOSIN), erythromycin, sulbactam sodium (COMBACTAM), piperacillin, ampicillin sodium (+) sulbactam sodium (UNACID) and levofloxacin (TAVANIC), and possibly related to therapy with simvastatin, acetaminophen, metoprolol tartrate (BELOC), hydrochlorothiazide, certoparin sodium (MONO EMBOLEX), esomeprazole magnesium (NEXIUM), ramipril (DELIX), escitalopram oxalate (CIPRALEX), midazolam (DORMICUM), propofol, insulin, sufentanil, ambroxol hydrochloride (MUCOSOLVAN), metoclopramide hydrochloride (PASPERTIN), domperidone (MOTILIUM), ipratropium bromide, albuterol, levothyroxine Na (L-THYROXIN), clonidine, nadroparin calcium (FRAXIPARIN), urapidil (EBRANTIL), amlodipine besylate (NORVASC) and pirenzepine hydrochloride (GASTROZEPIN). She also took the patient's aspiration pneumonia into consideration as a possible cause.

Toxic epidermal necrolysis prolonged hospitalization and was considered to be immediately life-threatening.

Information received on 15-FEB-2010 contained the following adverse experience: aspiration pneumonia (03-JAN-2009) which was treated with metronidazole, cefotaxime sodium (CLAFORAN), erythromycin, sulbactam sodium (COMBACTAM), piperacillin, ampicillin sodium (+) sulbactam sodium (UNACID) and levofloxacin (TAVANIC)).

Additional information is not expected.

Sender's comment:

Toxic epidermal necrolysis is a serious unlabeled event for therapy with ZOCOR®, aspiration pneumonia is nonserious and unlabeled. This case is confounded by the patient's co-morbidity and the numerous concomitant medications. Whether ZOCOR® played a causative role in this complex scenario, cannot be assessed based on the available data. The safety profile remains unchanged. Self-initiated measures are not indicated.

Reporter's comment:

GlaxoSmithKline, 17.03.2010 DE-GlaxoSmithKline-D0066740A

Sender diagnosis: not reported

Narrative

This case was reported by a physician and described the occurrence of toxic epidermal necrolysis in a 45-year-old male patient who received Nadroparine calcium (Fraxiparin) injection for prophylactic antithrombose.

This report was received from the Intensified Monitoring from the Documentation Centre for Severe Skin Reactions in Freiburg.

The patient's past medical history included angiography, computerized tomography and nuclear magnetic resonance imaging. Concurrent medical conditions included anemia, antiemesis, aspiration pneumonia, cardiac decompensations, cerebral edema, constipation, cough, diuresis, gastric protection, hospitalization, hyperglycemia, hyperlipidemia, hypertension arterial, hyperuricemia, hypopotassemia, hypothyroidism, mucolysis, nutrition, pain, pneumonia, prophylactic antithrombose, sedation, substitution therapy, unstable circulation and volume substitution.

Co-suspect medication included Berlosin, Erythromycin, Combactam, Piperacillin, Unacid, Tavanic and 37 other drugs. The patient also received 14 concurrent drugs.

On 14 January 2009 the patient started Nadroparine calcium (subcutaneous) at 0.4 ml, unknown dosing.

Approximately 7 days later, on 21 January 2009, the patient experienced toxic epidermal necrolysis, macule and skin blister.

On 26 February 2009, 15 days after starting of Fraxiparin, the patient developed erosion of genital mucosa.

On 29 January 2009, 43 days after starting of Fraxiparin, the patient developed conjunctivitis. Additionally the patient was treated since 15 January 2009 for hypertension and since 16 January 2009 for possible gastritis, but the start date of these events was not reported.

The physician considered the events to be life threatening.

At the time of reporting, toxic epidermal necrolysis was unresolved. It was also reported that toxic epidermal necrolysis lasted up to 04 March 2009 (inconsistent data). The outcome of the other events was not specified.

The reporting physician considered the events were possibly related to treatment with Nadroparine calcium and to all co-suspect medication.

Sender's comment:

Die UAW ist nicht erwartet.

Wir sehen einen kausalen Zusammenhang als unwahrscheinlich. Es gibt weitere Arzneimittel, die als ursächlich in Frage kommen.

Auf der Basis dieses Einzelfalles ergibt sich keine Aenderung des wissenschaftlichen Kenntnisstandes und der Nutzen/Risiko-Relation. Weitergehende eigenverantwortliche Massnahmen halten wir derzeit nicht fuer erforderlich.

Reporter's comment:

PFIZERINC, 14.05.2010 DE-PFIZER INC-2010055676

Sender diagnosis: not reported

Narrative

This is a spontaneous report from a project, sponsored by the German Regulatory Authority, set up to collect suspected drug induced severe skin reactions leading to hospital admission. Reference number of the documentation center for severe skin reactions (dZh): 9170788. The patient had been hospitalized for unknown reason at the ward for neurology from 01Jan2009 until 19Jan2009 and at a rehabilitation center from 19Jan2009 until 25Jan2009.

A contactable physician reported that this 45 year old male patient was treated with the following suspect drugs: intravenous sulbactam sodium

(COMBACTAM) 0.5 g 3x/day from 07Jan2009 to 13Jan2009 and intravenous ampicillin sodium, sulbactam sodium (UNACID) 3 g 3x/day from 14Jan2009 to 18Jan2009, both due to aspiration pneumonia which had been diagnosed on 03Jan2009, oral amlodipine besilate (NORVASC) initially 10 mg 1x/day, then 20 mg 1x/day due to arterial hypertension from 15Jan2009 on, intravenous metazolol sodium (BERLOSIN) 1 g 2x/day (morning, evening) from 01Jan2009 to 04Jan2009, 1g 1x/day (morning) from 06Jan2009 to 10Jan2009 and 1 g 2x/day (morning, noon) on 13Jan2009 due to pain, intravenous potassium chloride (KALIUMCHLORID) for substitution therapy from 01Jan2009 to 16Jan2009, intravenous paracetamol (Marketing Authorization Holder, MAH, unknown) 1g 3x/day, or 1g 1x/day, or 1g 2x/day, or 0.5g 1x/day due to pain from 01Jan2009 to 05Jan2009, 09Jan2009 to 12Jan2009 and 13Jan2009 to 18Jan2009, oral metoprolol succinate (BELOC ZOK) 47.5mg 1x/day or 23.75 mg 1x/day or 95mg 1x/day due to arterial hypertension from 01Jan2009 to 05Jan2009 and 12Jan2009 to 19Jan2009 and 2x/day with unknown dose from 19Jan2009 on, oral hydrochlorothiazide (manufacturer unknown) 12.5 mg 1x/day from 01Jan2009 to 05Jan2009 and 12.5mg 2x/day from 15Jan2009 on due to arterial hypertension, intravenous norepinephrine (NORADRENALIN) for stabilization of circulation from 02Jan2009 to 09Jan2009, subcutaneous heparin fraction, sodium salt (MONO-EMBOLEX) 0.8 ml or 0.3 ml for prophylaxis of thrombosis from 02Jan2009 to 14Jan2009, oral simvastatin (ZOCOR) 20 mg 1x/day due to hyperlipidemia from 02Jan2009 to 14Jan2009, intravenous amino acids NOS, electrolytes NOS, glucose, sorbitol, xylitol (PERIPLASMAL) 3.5% for substitution on 02Jan2009, 06Jan2009 to 10Jan2009 and 14Jan2009 to 15Jan2009, oral potassium (MAH unknown) 40 mmol due to hypopotassemia from 02Jan2009 to 04Jan2009, 08Jan2009 to 12Jan2009 and 16Jan2009 to 19Jan2009, oral carbohydrates NOS, fatty acids NOS, minerals NOS, proteins NOS, vitamins NOS (FRESUBIN) for nutrition from 07Jan2009 to 19Jan2009, intravenous calcium chloride hexahydrate, magnesium chloride anhydrous, potassium chloride, sodium acetate, sodium chloride (ELEKTROLYT 153) for substitution of volume from 07Jan2009 to 19Jan2009, oral lactulose (BIFITERAL) 10ml 3x/day due to constipation from 02Jan2009 on, oral esomeprazole (NEXIUM) 20 mg 1x/day for stomach protection from 02Jan2009 on, oral ramipril (DELIX) 5mg 1x/day from 03Jan2009 to 05Jan2009 and 2.5 mg 1x/day from 12Jan2009 on due to arterial hypertension, oral escitalopram (CIPRALEX) 10mg 1x/day for sedation from 03Jan2009 on, intravenous midazolam hydrochloride (DORMICUM) 100 mg and intravenous propofol (PROPOFOL) 2%, both for sedation from 04Jan2009 to 14Jan2009, intravenous insulin (MAH unknown) due to hyperglycemia from 04Jan2009 to 19Jan2009, intravenous sufentanil (SUFENTANIL) 500 microgram due to pain from 04Jan2009 to 19Jan2009, intravenous erythromycin (MAH unknown) 200mg 3x/day due to aspiration pneumonia from 05Jan2009 to 09Jan2009 and 11Jan2009 to 13Jan2009, respiratory therapy with ambroxol hydrochloride (MUCOSOLVAN) 3x/day due to secretolysis from 05Jan2009 to 13Jan2009, intravenous metoclopramide hydrochloride (PASPERTIN) 10mg 2x/day for antiemesis from 05Jan2009 to 15Jan2009, oral domperidone (MOTILIUM) 10mg 1x/day for antiemesis from 05Jan2009 to 16Jan2009, oral ipratropium (IPRATROPIUM) 5drops 4x/day and oral salbutamol (SALBUTAMOL) 5drops 4x/day, both due to cough from 06Jan2009 to 13Jan2009, oral levothyroxine sodium (L-THYROXIN) 25 microgram or 50 microgram or 75 microgram due to hypothyroidism from 06Jan2009 to 19Jan2009, intravenous piperacillin (MAH unknown) 4g 3x/day due to aspiration pneumonia from 07Jan2009 to 13Jan2009, intravenous haloperidol (MAH unknown) 5 mg for withdrawal on 09Jan2009, intravenous amino acids, glucose, sodium acetate, potassium chloride, soy oil (STRUCTOKABIVEN) for nutrition from 09Jan2009 to 12Jan2009, intravenous clonidine (CLONIDIN) 750 microgram for withdrawal from 10Jan2009 to 16Jan2009, subcutaneous heparin-fraction, calcium salt (FRAXIPARIN) 0.4ml for prophylaxis of thrombosis from 14Jan2009 on, intravenous urapidil (EBRANTIL) 250 mg due to hypertension from 15Jan2009 to 19Jan2009, oral pirenzepine dihydrochloride (GASTROZEPIN) 25mg 3x/day due to gastritis (questionable) from 16Jan2009 to 19Jan2009, oral levofloxacin (TAVANIC) 500 mg 3x/day due to aspiration pneumonia from 16Jan2009 on, paracetamol suppository (MAH unknown) for unknown indication from 19Jan2009 on, oral candesartan cilexetil (ATACAND) due to arterial hypertension from 19Jan2009 on, oral levothyroxine sodium (BERLTHYROX) 75 mg 1x/day due to hypothyroidism from 19Jan2009 on and transdermal plaster hyoscine (SCOPODERM) due to pain from 19Jan2009 on. The date when the dose of suspect drugs was changed was not specifically documented.

Medical history included hyperuricemia, anemia, cerebral edema and cardiac decompensation.

Concomitant medication included oral metoprolol succinate/hydrochlorothiazide (BELOC-ZOK comp) 95mg/12.5mg; 1/2DF 1x/day due to arterial hypertension from 01Jan2001 to 01Jan2009, oral simvastatin, ezetimibe (INEGY) 10mg/20mg 1DF 1x/day due to hyperlipidemia from 01Jan2001 to 01Jan2009, oral allopurinol (ALLOPURINOL) 300 mg 1x/day due to hyperuricemia from 01Jan2005 to 07Jan2009, intravenous contrast medium NOS for diagnostic examination (computed tomography, nuclear magnetic resonance imaging, angiography) on 01Jan2009, intravenous erythrocyte concentration due to anemia on 02Jan2009, intravenous metronidazole (METRONIDAZOL) 500mg 3x/day due to aspiration pneumonia from 03Jan2009 to 06Jan2009, intravenous cefotaxime sodium (CLAFORAN) 2g 3x/day due to pneumonia from 03Jan2009 to 06Jan2009, intravenous mannitol (MANNITOL) 20% 125 ml due to cerebral edema on 04Jan2009, intravenous fentanyl (FENTANYL) due to pain from 04Jan2009 to 05Jan2009, intravenous furosemide (FUROSEMID) 5mg for diuresis on 05Jan2009, intravenous dobutamine (DOBUTAMIN) 250 mg due to cardiac decompensation from 05Jan2009 to 07Jan2009, oral acetylcysteine (ACC) 600mg for secretolysis from 05Jan2009 on, intravenous ceruletide diethylamine thiomalate sodium (TAKUS) for substitution on 06Jan2009, glycerol (MILAX) suppository and bisacodyl (PYRILAX) suppository due to constipation on 06Jan2009.

Past drug history included intravenous treatment with paracetamol, unknown if tolerated well. He developed cockades, spots and first blisters of the skin on 21Jan2009. A conjunctivitis on 29Jan2009 and erosions of the genital mucosa on 26Feb2009. A toxic epidermal necrolysis (ensured case), with onset date 21Jan2009, was diagnosed. The event was considered life-threatening. Per documentation, the event persisted until 04Mar2009, however outcome of the event was documented as not recovered (obvious discrepancy of reporting). Information on medication is available from 24Dec2008 until 21Jan2009. In March2009 the project experts assessed the causal relationship as "very possible" with sulbactam sodium; ampicillin sodium/sulbactam sodium; piperacillin; levofloxacin; metazolol and erythromycin as "possible" with paracetamol; metoprolol succinate; hydrochlorothiazide; heparin fraction, sodium salt; simvastatin; esomeprazole; ramipril; escitalopram; midazolam hydrochloride; propofol; insulin; sufentanil; ambroxol hydrochloride; metoclopramide hydrochloride; domperidone; ipratropium; salbutamol; levothyroxine sodium; clonidine; heparin-fraction, calcium salt; urapidil; amlodipine besilate; pirenzepine dihydrochloride and as "dubious" with potassium chloride; norepinephrine; amino acids NOS, electrolytes NOS, glucose, sorbitol, xylitol; potassium; carbohydrates NOS, fatty acids NOS, minerals NOS, proteins NOS, vitamins NOS; calcium chloride hexahydrate, magnesium chloride anhydrous, potassium chloride, sodium acetate, sodium chloride; lactulose; haloperidol; amino acids, glucose, sodium acetate, potassium chloride, soy oil; paracetamol suppository; candesartan cilexetil; metoprolol succinate; levothyroxine sodium and hyoscine. Aspiration pneumonia as alternative cause was documented as possible by the project experts. Metoprolol succinate was assessed as possible (therapy until 19Jan2009) and also as "dubious" (therapy from 19Jan2009 on).

This case contains Wyeth co-suspect products. The Wyeth case may qualify for regulatory reporting for the Wyeth co-suspect drug. This case may therefore be a duplicate report.

Follow-up status: Case closed (04May2010).

Sender's comment:

Based on the information received the Company (Pfizer) cannot exclude at this time that the suspect medications contributed to the reported event. This report would not seem to modify the risk-benefit profile of the subject drugs. The case will be reassessed if additional information becomes available. Die toxisch epidermale Nekrolyse ist in der Produktinformation von Unacid, jedoch nicht von Combactam und Norvasc als mögliche Nebenwirkung beschrieben. Wir sehen zum gegenwärtigen Zeitpunkt keinen Anlass für eine geänderte Bewertung des Nutzen-/Risikoverhältnisses.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Report from study

case: DE-BFARM-10025329

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	Yes	Yes	No	No	No		

Patient

Initials:	Sex: Female	Birth: privacy	Age: 3 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Constipation	200712			0 Month	Unknown
2 Protein abnormal NOS				0 Month	Unknown
3 Dyspnea	200712			0 Month	Unknown
4 Pain	200712			0 Month	Unknown
5 Mucosal erosion	20071230			0 Month	Unknown
6 Genital erosion	20071230			0 Month	Unknown
7 Oral mucosa erosion	20071230			0 Month	Unknown
8 Lip erosion	20071230			0 Month	Unknown
9 Blister	20071226			0 Month	Unknown
10 Macule	20071224			0 Month	Unknown
11 Stevens Johnson syndrome	20071224	20080804	225 Day	0 Month	recovered/resolved
12 Toxic epidermal necrolysis	20071224	20080804	225 Day	0 Month	recovered/resolved
13 Pneumonia	20071128			0 Month	Unknown
14 Acute bronchitis	20071207			0 Month	Unknown
15 Exanthema	20071219			0 Month	Unknown
16 Fever	20071224			0 Month	Unknown
17 Eye red	20071224			0 Month	Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
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DE-BFARM-10025329

1	acc acetylcysteine	Oral	200 Mg milligram(s)/Day	20071218-	-
2	anderes Arzneimittel nnb	Oral	/Day	20071213-	-
3	Bifiteral lactulose	Oral	10 ml millilitre(s)/Day 244	20071213-20071224	Constipation 12 Day
4	bronchoparat theophylline sodium aminoacetate	Intravenous (not otherwis /	120	20071209-20071211	Respiratory insufficiency 3 Day
5	cefazolin cefazolin	Intravenous (not otherwis	900 Mg milligram(s)/Day 120	20071207-20071221	Pneumonia 15 Day
6	Chloralhydrat chloral hydrate	Unknown	/Day 242	200712-	Sedation -
7	gentamycin gentamicin sulfate	Intravenous (not otherwis /		20071210-20071215	Pneumonia 6 Day
8	Glukose 5% glucose	Intravenous (not otherwis /		20071209-20071215	7 Day
9	moronal nystatin	Other	5* /Day 77	20071212-	Antifungal prophylaxis -
10	Morphin morphine	Intravenous (not otherwis /	120	20071211-20071215	Pain 5 Day
11	natriumchlorid 0,9% sodium chloride	Intravenous (not otherwis /		20071207-	Fluid replacement -
12	Nexium esomeprazole	Intravenous (not otherwis	10 Mg milligram(s)/Day 120	20071212-20071215	Prophylaxis 4 Day
13	Paracetamol paracetamol	Unknown	/ 242	20071201-	Fever -
14	Singulair montelukast sodium	Oral	4 Mg milligram(s)/Day	20071209-	Dyspnea -
15	solu-decortin-h prednisolone sodium succinate	Intravenous (not otherwis /	120	20071207-20071216	Respiratory insufficiency 10 Day
16	sultanol salbutamol	Respiratory (inhalation)	/ 110	20071207-	Respiratory insufficiency -
17	Viani fluticasone propionate; salmeterol xinafoate	Respiratory (inhalation)	2* /Day 110	20071209-	Respiratory insufficiency -

Drugs (concomitant)

18	dormicum midazolam maleate	Intravenous (not otherwis	50 Mg milligram(s)/Day 120	20071210-20071210	Sedation 1 Day
19	fenistil dimetindene maleate	Oral	3* /Day 158	20071225-	Allergy -
20	flutide mite fluticasone propionate	Respiratory (inhalation)	2* /Day 110	20060101-20071208	Muscular dystrophy 707 Day
21	ibuprofen ibuprofen	Oral	/	20071225-	-
22	novalgin metamizole	Intravenous (not otherwis	100 Mg milligram(s)/Day 120	20071225-20071225	Fever 1 Day
23	Orelox cefpodoxime proxetil	Oral	2* /Day	20071201-20071207	Respiratory infection 7 Day
24	salbutamol salbutamol	Respiratory (inhalation)	/ 110	20071127-20071207	Cough 11 Day
25	zithromax azithromycin	Oral	100 Mg milligram(s)/Day	-	Infection -

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Fever			Unknown	
Cough			Unknown	
Muscular dystrophy			Unknown	
Respiratory insufficiency			Unknown	
Respiratory infection			Unknown	

Primary source

Qualification:	Physician	observestudytype:	Other studies
Literature:			
Qualification:	Other Health Professional	observestudytype:	Other studies
Literature:			

Narratives (nur bei elektronischem Eingang)

GlaxoSmithKline, 19.02.2010 DE-GlaxoSmithKline-D0066473A

Sender diagnosis: not reported

Narrative

This case was reported by a physician and described the occurrence of stevens-johnson syndrome in a 3-year-old female patient who received Salbutamol sulphate (Sultanol) inhaler and Viani inhalation for respiratory insufficiency. This report was received from the Intensified Monitoring from the Documentation Centre for Severe Skin Reactions in Freiburg.

Co-suspect medication included Paracetamol, Solu-Decortin-H, Chloralhydrat, Cefazolin, Sodium chloride 0.9%, Bronchoparat, 5% glucose, Singulair, Gentamycin, Morphin, Nexium, Moronal, Bifiteral, ACC and LGG as an unspecified medication.

Concurrent medications included fluticasone propionate (Flutide mite), salbutamol sulphate (Salbutamol), cefpodoxime proxetil (Orelox), Dormicum, dipyrone (Novalgin), Ibuprofen, dimethindene maleate (Fenistil) and azithromycin (Zithromax).

Concurrent medical conditions included cough, fever, muscular dystrophy, respiratory infection and respiratory insufficiency.

On 7 December 2007 the patient started Salbutamol sulphate (inhaled) three times per day.

On 9 December 2007 treatment with Sultanol was interrupted till 13 December 2007. The dosage of Sultanol inhalation was variable.

On 9 December 2007 the patient started Viani inhalation twice per day.

On 28 November 2007, the patient experienced pneumonia. On 7 December 2007 the patient was hospitalised with acute bronchitis.

On 24 December 2007 the patient experienced stevens-johnson syndrome and toxic epidermal necrolysis in transition state until 4 August 2008.

These events were considered to be life-threatening.

On 19 December 2007 the patient developed exanthema. On 24 December 2007 the patient developed fever, macules and redness of eyes.

On 26 December 2007 the patient was hospitalised with skin blisters.

On 30 December 2007 the patient developed lip erosion, oral mucosa erosion, erosion of genital mucosa and erosion of nasal mucosa.

On an unspecified date in December 2007 the patient developed pain, dyspnea, constipation and alpha-2-laminine deficiency.

At the time of reporting, the events were unresolved.

The reporting physician considered the events were possibly related to treatment with Sultanol and Viani and to all other co-suspect drugs.

Sender's comment:

Die UAW ist unerwartet.

Wir sehen hier einen kausalen Zusammenhang mit Viani und Salbutamol als unwahrscheinlich. Es kommen weitere Arzneimittel als ursächlich in Frage.

Auf der Basis dieses Einzelfalles ergibt sich keine Aenderung des wissenschaftlichen Kenntnisstandes und der Nutzen/Risiko-Relation. Weitergehende eigenverantwortliche Massnahmen halten wir derzeit nicht fuer erforderlich.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-10029852

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		Yes

Patient

Initials:	Sex: Female	Birth: privacy	Age: 62 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	20070911	20071001	21 Day	35 Day	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 BAYOTENSIN nitrendipine	Oral	/	20070908-20070908 1 Day	Hypertensive crisis
2 DIPIPERON pipamperone		/	20070826-20070909 15 Day	Restlessness
3 EUTHYROX levothyroxine sodium	Oral	75 µg microgram(s)/	20070625- -	
4 JELLIN fluocinolone acetonide; neomycin sulfate	Topical	2* /Day	20070827-20070911 16 Day	Skin lesion
5 LAMICTAL lamotrigine	Oral	25 Mg milligram(s)/Day	20070831-20070911 12 Day	Depression
6 PARACETAMOL paracetamol	Oral	500 Mg milligram(s)/	20070820-20070821 2 Day	Headache
7 PARACETAMOL paracetamol	Oral	500 Mg milligram(s)/Day	20070912-20070913 2 Day	
8 PARACETAMOL paracetamol	Oral	1500 Mg milligram(s)/Day	20070910-20070910 1 Day	Fever
9 REMERGIL mirtazapine	Oral	7.5 Mg milligram(s)/Day	20070813- -	Depression
10 REMERGIL mirtazapine	Oral	15 Mg milligram(s)/Day	20070813- -	
11 VALIQUID diazepam	Oral	7.5 Mg milligram(s)/	20070829-20070829 1 Day	Restlessness
12 zoloft sertraline hydrochloride	Oral	25 Mg milligram(s)/ 82	20070808- -	
13 zoloft sertraline hydrochloride	Oral	25 Mg milligram(s)/Day 82	20070808- -	Depression
14 zoloft sertraline hydrochloride	Oral	25 Mg milligram(s)/ 82	20070808- -	

Drugs (concomitant)

15	advantan methylprednisolone aceponate	Topical	2* /Day	20070912-	Skin lesion
16	fenistil dimetindene maleate	Oral	/	20070913-	Skin lesion
17	NORVASC amlodipine besilate	Oral	10 Mg milligram(s)/Day	-	Hypertension arterial
18	NORVASC amlodipine besilate	Oral	5 Mg milligram(s)/Day	-	
19	tavor lorazepam	Oral	0.25 Mg milligram(s)/Day	-	
20	tavor lorazepam	Oral	0.5 Mg milligram(s)/Day	-20070824	Restlessness

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Hypertension arterial			k.A.	

Primary source

Qualification: Physician observestudytype:
Literature:

Narratives (nur bei elektronischem Eingang)

PFIZERINC, 26.02.2010 DE-PFIZER INC-2010019327

Sender diagnosis: not reported

Narrative

This is a spontaneous report from a project, sponsored by the German Regulatory Authority, set up to collect suspected drug induced severe skin reactions leading to hospital admission. Reference number from the documentation center for severe skin reactions (dZh): 9170568.

A contactable physician reported about a 62 year old female patient with a medical history of arterial hypertension who was admitted to a psychiatric hospital on 07Aug2008. She started treatment with 25 mg/day oral sertraline HCl (ZOLOFT) due to depression from 08Aug2007 onwards, on unspecified dates the patient started taking 2 x 25 mg/day, then 4 x 25 mg/day sertraline HCl. She started treatment with once daily 7.5 mg oral mirtazapine (REMERGIL) due to depression on 13Aug2007 and continued on an unspecified date with 15 mg once daily mirtazapine. Paracetamol was taken due to headache and fever from 20Aug2007 to 21Aug2007, on 10Sep2007 and from 12Sep2007 to 13 Sep2007. She started with 1 x 500 mg oral paracetamol at noon, continued with 1 x 500 mg in the evening, took on 10Sep2007 thrice daily 500 mg and from 12Sep2007 to 13Sep2007 1 x 500 mg in the morning. On 25Jun2007 she started treatment with 75 ug oral levotyroxine sodium (EUTHYROX). On 29Aug2007 she was treated with 7.5 mg oral diazepam (VALIQUID) due to restlessness. Due to the same indication was treated on 26Aug2007 and on 09Sep2007 with oral pipamperone (DIPIPERON), starting with 20 mg/day continued on an unspecified date with 30 mg in the morning and 40 mg in the evening and then continued with daily 40 mg pipamperone. On 08Sep2007 the patient was treated with nitrendipine (BAYOTENSIN) due to hypertensive crisis. From 31Aug2007 to 11Sep2007 the patient was treated with 25 mg/day oral lamotrigine (LAMICTAL) due to depression. Concomitant medication included oral amlodipine besilate (NORVASC) on 19Aug2007 and oral lorazepam (TAVOR) from unknown start date until 24Aug2007. On 28Aug2007 the patient was diagnosed with dishydrosiformous eczema of the hands (differential diagnosis of tinea: drug induced toxicity). Treatment of skin lesions had been started on 27Aug2007 and was continued until 11Sep2007 with twice daily topical fluocinolone acetone, neomycin sulfate (JELLIN). On 11Sep2007 the patient experienced fever and spots, on 12Sep2007 she experienced painful skin and on 14Sep2007 Stevens Johnson syndrome was diagnosed with symptoms of blistering of skin and erosions of lips and oral mucosa. She received treatment with topical methylprednisolone aceponate (ADVANTAN) from 12Sep2007 onwards and oral dimetindene (FENISTIL) from 13Sep2007 onwards. Outcome of the event was documented as recovered on 01Oct2007. The event was documented as not life-threatening. Information on medication was available from 17Aug2007 until diagnosis of the severe skin reaction on 14Sep2007. On 11Sep2007 the project experts assessed the causal relationship as "very probable" with lamotrigine, as "possible" with mirtazapine and sertraline, as "questionable" with levothyroxine, paracetamol, pipamperone, fluocinolone, diazepam and nitrendipine. Fluconicolone had been administered in the past and had been well tolerated. The category "infection" was documented as "unknown" by the project experts.

Pfizer is not the Marketing Authorization Holder for paracetamol in the reporter's country. This may be a duplicate report in situations where another Marketing Authorization Holder submitted the same report to the regulatory authorities.

Follow-up status: Case closed (15Feb2010).

Sender's comment:

Based on the information provided in the case, this individual report would not seem to modify the risk-benefit profile of the subject drug. Bei Stevens-Johnson-Syndrom handelt es sich um eine bekannte, in der Produktinformation ausreichend praezise deklarierte Nebenwirkung. Wir sehen zum gegenwaertigen Zeitpunkt keinen Anlass fuer eine geaenderte Bewertung des Nutzen-/Risikoverhaeltnisses.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Report from study

case: DE-BFARM-10030567

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		

Patient

Initials:	Sex: Male	Birth: privacy	Age: 21 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Gastric erosions	20060927	20061013	17 Day		recovered/resolved
2 Lip erosion	20060927	20061013	17 Day		recovered/resolved
3 Stevens-Johnson syndrome	20060927	20061013	17 Day		recovered/resolved
4 Pain	20060927	20061013	17 Day		recovered/resolved
5 Skin pigmentation	20060927	20061013	17 Day		recovered/resolved
6 Pruritus	20060927	20061013	17 Day	44 Day	recovered/resolved
7 Vesicles	20060927	20061013	17 Day		recovered/resolved
8 Fever	20060927	20061013	17 Day		recovered/resolved
9 Mucosal erosion					recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 cotrim forte heumann sulfamethoxazole; trimethoprim		/	20060829- -	Pneumocystis pneumonia
2 fortecortin dexamethasone	Oral	/	20060815- -	Cerebral edema
3 METOCLOPRAMID metoclopramide hydrochloride	Oral	/	20060829- -	Nausea
4 novalgin metamizole	Oral	/	20060829- -	Pain
5 OMEPRAZOLE omeprazole		/	20060830- -	Stomach discomfort
6 PARACETAMOL paracetamol	Oral	/	20060822- -	
7 TEGRETAL carbamazepine	Oral	/	20090815- -	Seizure

Drugs (concomitant)

8 amphotericin b amphotericin b	/	20060928- -
9 CETRIZIN cetirizine hydrochloride	/	20060928- -
10 TEMODAL "ESSEX" temozolomide	/	20060829- -

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Glioneuronal tumor			k.A.	

Primary source

Qualification:	Physician	observestudytype:	Other studies
Literature:			

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 01.03.2010 DE-002147023-PHHY2010DE08998

Sender diagnosis: not reported

Narrative

Initial report received from the international RegiSCAR project (European Registry of Severe Cutaneous Adverse Reactions, non-Novartis patient registry), (dZh Dokumentationszentrum schwerer Hautreaktionen, case number 9170473) on 15 Feb 2010. This polymedicated patient with medical history of glioblastoma started treatment with Tegretol (carbamazepine) on 15 Aug 2006, for seizure prophylaxis. The patient was also on Novalgin, paracetamol, Fortecortin, MCP drops, omeprazol, Cotrim forte at the same time. On 27 Sep 2006, patient showed pains of skin and spots. One day later, fever, pruritus and lip and gastric mucosal erosions occurred. On 27 Sep 2009, after skin vesicles and nasal mucosal erosions were seen, the diagnosis of Stevens Johnson syndrome was made, and the patient was hospitalised on the same day. The patient made a full recovery, and the event lasted until 13 Oct 2006. The reporter assessed the causality as suspected. As other potential or contributing cause, the glioblastoma has explicitly been mentioned.

Sender's comment:

Serious adverse drug reaction report, Stevens-Johnson syndrome [hospitalisation], assessed as listed according to the Core Data Sheet. Reporting healthcare professional causality is suspected.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-10028893

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		Yes

Patient

Initials:	Sex: Female	Birth: privacy	Age: 57 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Genital erosion	20061009	20061025	17 Day	34 Day	recovered/resolved
2 Oral mucosa erosion	20061009	20061025	17 Day	34 Day	recovered/resolved
3 Stevens-Johnson syndrome	20061006	20061025	20 Day	31 Day	recovered/resolved
4 Stevens-Johnson syndrome	20061006	20061025	20 Day	34 Day	recovered/resolved
5 Bullous eruption	20061009	20061025	17 Day	34 Day	recovered/resolved
6 Edema face	20061006	20061025	20 Day	31 Day	recovered/resolved
7 Erythema	20061006	20061025	20 Day	31 Day	recovered/resolved
8 Fever	20061009	20061025	17 Day	34 Day	recovered/resolved
9 Pruritus	20061007	20061025	19 Day	32 Day	recovered/resolved
10 Red blotches	20061006	20061025	20 Day	31 Day	recovered/resolved
11 Redness of eyes	20061007	20061025	19 Day	32 Day	recovered/resolved
12 Lip erosion	20061009	20061025	17 Day	34 Day	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 DEXAMETHASONE dexamethasone	Oral	/	-	-
2 DEXAMETHASONE dexamethasone	Oral	/	-	-
3 DEXAMETHASONE dexamethasone	Oral	/	20060902-	Cerebral edema
4 NOVAMINSULFON metamizole	Oral	/	20060926-	Pain
5 PANTOZOL pantoprazole	Oral	/Day	20060902-	Gastrointestinal disorder
6 PARACETAMOL paracetamol	Oral	/	20060927-	Pain
7 TEGRETAL carbamazepine	Oral	/	20060906-	Convulsion prophylaxis

Drugs (concomitant)

8 CALCIUM CARBONATE calcium carbonate	Oral	/	20060918-	Supplementation therapy
9 CLEXANE enoxaparin	Subcutaneous	/	20060905-	Thrombosis
10 kalinor-brausetabletten potassium carbonate; potassium citrate		/	-	-
11 Kontrastmittel		/	-	-
12 NITRAZEPAM nitrazepam	Oral	/	20060912-	Difficulty sleeping
13 spirulina spirulina		/	-	-

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Pain			k.A.	
Thrombosis			k.A.	
Sleep disorder			k.A.	
Metastases to central nervous system			k.A.	Cerebral metastases with unknown primary tumor

Primary source

Qualification:	Physician	observestudytype:
Literature:		
Qualification:	Other Health Professional	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 01.03.2010 DE-002147023-PHHY2010DE09016

Sender diagnosis: not reported

Narrative

Initial investigator report from the international RegiSCAR project (European Registry of Severe Cutaneous Adverse Reactions), a scientific network dedicated to severe cutaneous adverse reactions, in collaboration with the German documentation center of severe skin reactions (dZh-Dokumentationszentrum schwerer Hautreaktionen) received on 15 Feb 2010. The patient had a concurrent condition with cerebral metastases with unknown primary tumor. The patient was treated with Tegretal (carbamazepine) 400 mg (half or one tablet a day) since 06 Sep 2006. On 06 Oct 2006 she has developed Stevens-Johnson-Syndrome (SJS) with facial edema, erythema and spots on 6 Oct 2006, followed by pruritus and redness of the eyes on 07 Oct 2006. The patient was admitted to hospital on 09 Oct 2006 with additional symptoms of fever, bullous eruptions on skin, erosion of the lips and of mucosa membranes (mouth, nose, genitale). The events resolved on 25 Oct 2006. This event was assessed as probably related to Tegretal. Other medicinal products were also reported as potential cause for the SJS: metamizole natrium as very possible, dexamethasone as possible, pantoprazole as possible and acetaminophen as possible related to the event. It was further mentioned that the patient underwent radiation therapy due to cerebral metastases in the period from 08 Sep 2006 to 26 Sep 2006 on an inpatient basis. The cerebral metastases (with unknown primary tumor) were considered as an additional risk factor for the occurrence of the Stevens-Johnson-Syndrome. These metastases were not assessed in terms of causality and outcome.

Sender's comment:

For Tegretal and Pantozol review,

Serious adverse drug reaction report, Stevens-Johnson syndrome [hospitalisation], assessed as listed according to the Core Data Sheet. Reporting healthcare professional causality is suspected.

Reporter's comment:

Nycomed, 01.03.2010 DE-Nycomed-0210211

Sender diagnosis: not reported

Narrative

This case was received by our licensee Novartis (ref. no. PHHY2010DE09016) on 15-FEB-2010 and forwarded to Nycomed on 26-FEB-2010. This is an initial investigator report from the international RegiSCAR project (European Registry of Severe Cutaneous Adverse Reactions), a scientific network dedicated to severe cutaneous adverse reactions, in collaboration with the German documentation center of severe skin reactions (Dokumentationszentrum schwerer Hautreaktionen).

It concerned a 57-year-old female patient started therapy treated with Pantozol (pantoprazole), dosage unknown, once daily, oral, on 02-SEP-2006, for stomach protection. Additional suspect medications were carbamazepine (introduced on 06-SEP-2006, for convulsions prophylaxis), metamizole sodium and paracetamol (introduced on 26 and 27-SEP-2006, respectively, due to pain) and dexamethasone (introduced on 02-SEP-2006, for prophylaxis of cerebral oedema). The patient had a medical history of sleep disorder, thrombosis and metastases to central nervous system with unknown primary tumor. She was concomitantly treated with nitrazepam, calcium carbonate, enoxaparin sodium, potassium chloride, an unspecified

contrast media and an unspecified medication. The patient had undergone radiation therapy due to cerebral metastases from 08-SEP-2006 to 26-SEP-2006 on an inpatient basis. On 06-OCT-2006 (1 month after the first dose of pantoprazole, carbamazepine and dexamethasone; 1.5 weeks after first dose of metamizole sodium and paracetamol), the patient developed Stevens-Johnson-Syndrome (SJS) with facial oedema, erythema and spots, followed by pruritus and redness of the eyes on 07-OCT-2006. The patient was admitted to hospital on 09-OCT-2006 with additional symptoms of fever, bullous eruptions on skin, erosion of the lips and of mucosa membranes (mouth, nose, genital).

Therapy with pantoprazole continued.
The reactions resolved on 25-OCT-2006.

The investigator assessed the Stevens-Johnson-Syndrome as probably related to carbamazepine and possibly related to pantoprazole and other co-suspect medications. The cerebral metastases with unknown primary tumour were considered as an additional risk factor for the occurrence of the Stevens-Johnson-Syndrome.

Sender's comment:

Stevens-Johnson-Syndrome is listed in the current reference safety information on pantoprazole.

Stevens - Johnson syndrome (SJS) is a rare but severe dermatological condition, considered as a severe type of erythema exsudativum multiforme. It is characterized by erythema with bullous and eroded lesions of skin and mucous membranes. Additional symptoms include fever, general ill feeling and joint aches. SJS is an immune-complex-mediated hypersensitivity reaction that occurs in response to medications (e.g., sulfonamides, penicillins, barbiturates, and phenytoin), infections (e.g., herpes simplex, Mycoplasma and malaria infections), or malignancies (various carcinomas and lymphomas). Stevens-Johnson syndrome (SJS) is idiopathic in 25-50% of cases.

This patient had other risk factors that could have caused or contributed to the development of the reaction e.g. concomitant treatment with carbamazepine, metamizole sodium and paracetamol and concurrent condition of cerebral metastases with unknown primary tumour.

This single case report is not considered to change the current knowledge of the safety profile of pantoprazole.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-10027526

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		

Patient

Initials:	Sex: Male	Birth: privacy	Age: 60 Year	weight (kg): 55	BMI_int: 18.59	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Facial swelling					recovered/resolved
2 Ear swelling	20100104	20100106	3 Day	5 Day	recovered/resolved
3 Redness					recovered/resolved
4 Vomiting					recovered/resolved
5 Allergic reaction	20100104	20100106	3 Day	5 Day	recovered/resolved
6 C-reactive protein increased	20100104			5 Day	Unknown
7 Congestion pulmonary					recovered/resolved
8 Sebostasis					recovered/resolved
9 Vomiting	20100104	20100106	3 Day	5 Day	recovered/resolved
10 Swelling of face	20100104	20100106	3 Day	5 Day	recovered/resolved
11 Swelling of eyelid	20100104	20100106	3 Day	5 Day	recovered/resolved
12 Sweating					recovered/resolved
13 Skin peeling					recovered/resolved
14 Nausea	20100104			6 Day	recovered/resolved
15 Face red	20100104	20100106	3 Day	5 Day	recovered/resolved
16 Nausea					recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac	Rectal	/	20091231-20091231	Limb discomfort
diclofenac		242	1 Day	
2 paracetamol 500	Oral	500 Mg milligram(s)/	20091230-20100102	Viral upper respiratory tract
paracetamol		245	4 Day	infection

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Viral upper respiratory tract infection	200912		Unknown	viraler Infekt mit Fieber und Gliederschmerzen
Limb discomfort	200912		Unknown	Gliederschmerzen bei viralem Infekt mit Fieber
Fever	200912		Unknown	
Headache	200912		Unknown	

Primary source

Qualification:	Pharmacist	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

HELIOS Klinikum Wuppertal, 03.03.2010 DE-PVZIBE-02-10251

Sender diagnosis: not reported

Narrative

ANMERKUNGEN ZU FOLGENDER UAW:
allergische Reaktion

KURZZUSAMMENFASSUNG:
allergischen Reaktion auf Paracetamol und Diclofenac
BESCHREIBUNG:

seit dem 30.12.2009 Kopfschmerzen und Fieber bis 38,5°C (im Sinne eines viralen Infektes), daraufhin Einnahme von Paracetamol (vom 30.12.2009 bis 02.01.2010) und Einnahme von Diclofenac einmalig am 31.12.2009;

bei Einweisung Schwellung des Gesichtes, der Ohren, der Augenlider und der Wangen, starke Rötung des Gesichtsbereiches, leichte Übelkeit, einmaliges Erbrechen;

bei Aufnahme kein weiteres Exanthem oder Erythem am Körper, keine bronchiale Spastik, fieberfrei, kein Meningismus, Blutdruck 110/60mmHg; vor dem 30.12.2009 kein Paracetamol eingenommen, so dass dies die erstmalige Anwendung war

THERAPIE:

beim Hausarzt: 100mg Prednisolut i.v. und eine Ampulle Fenistil i.v.;

stationär: parenteraler Volumenersatz, Kreislaufkontrollen

VERLAUF:

laut Beschreibung des Hausarztes Patient bisher völlig gesund;

während des stationären Aufenthaltes starkes Schwitzen, das langsam im Verlauf abklingt;

04.01.2010 Röntgen-Thorax: Zwerchfell glatt abzugrenzen, beginnende peribronchitische Anschoppung nicht ausgeschlossen, sonst kein Nachweis pulmonaler Infiltrationen, kein Anhalt für Pneumonie;

06.01.2010 rückläufiges Erythem der Gesichtshaut, teilweise Ablösen der Gesichtshaut bei insgesamt Neigung des Patienten zur Seborrhoe;

bei symptomatologischer Behandlung allmähliche Befund- und Beschwerderegress

ERLÄUTERUNGEN ZUR UAW:

Krankenhausaufenthalt: 04.-06.01.2010;

Krankensein aufgrund einer allergischen Reaktion auf Paracetamol und Diclofenac;

Besserung nach Absetzen der Medikamente und antiallergisch wirksamer Therapie

ERLÄUTERUNGEN ZU DEN KAUSALITÄTEN:

MEDIKAMENT: paracetamol 500

BETREFFENDE UAW: allergische Reaktion

ERLÄUTERUNG: Überempfindlichkeitsreaktionen auf Paracetamol mit der Ausprägung der beschriebenen UAW sind in der Fachinfo beschrieben.

MEDIKAMENT: diclofenac

BETREFFENDE UAW: allergische Reaktion

ERLÄUTERUNG: Überempfindlichkeitsreaktionen auf Diclofenac sind bekannt und in der Fachinfo als häufig auftretend beschrieben.<dbtext>

To_Char_NARRATIVE_2</dbtext>

Sender's comment:

Initialmeldung - Zusatzinformationen werden erwartet.

Reporter's comment:

sanofi-aventis, 12.03.2010 DE-SANOFI-AVENTIS-2010SA012485

Sender diagnosis: not reported

Narrative

This ICSR had been created by a regional pharmacovigilance centre and was extracted from the website of the German health authority (authority number DE-BFARM-10027526) on 04-Mar-2010. The pharmacovigilance (PV) centre is conducting a hospitalisation registry.

Within this registry, the patient's medical records are screened for adverse drug reactions that could have led to hospital admission; this screening takes place after the patient's discharge from hospital.

The case involves a 60-year-old male patient. Since 30-Dec-2009 the patient suffered from headache and fever 38.5 °C (in the sense of a viral infection). Therefore, he took paracetamol (no tradename provided) at a dose of 500 mg for viral upper respiratory tract infection from 30-Dec-2009 to 02-Jan-2010 and diclofenac (no tradename provided) once for limb discomfort on 31-Dec-2009. On 04-Jan-2010 the patient experienced a facial swelling (ear, eye lid, cheeks), severe redness of face, mild nausea and unique vomiting. The family doctor administered prednisolone sodium succinate at a dose of 100 mg i.v. and one ampoule of dimetindene maleate i.v. to treat the events. The patient was hospitalised due to an allergic reaction to paracetamol and diclofenac on the same day. On admission no further exanthema on body, no bronchial spasm, no fever and no meningism was observed. The blood pressure was 110/60 mmHg. The patient received volume replacement. During the hospitalisation the patient experienced severe sweating that slowly decays over time. On 04-Jan-2010 an x-ray thorax was performed with the following findings: Delineate

diaphragm smooth, incipient peri bronchial engorgement not excluded, no evidence of pulmonary infiltration or pneumonia. On 06-Jan-2010 a declining erythema of face and partial detachment of the facial skin in tendency to seborrhea were observed. The condition of the patient improved after effective anti-allergic therapy. The patient recovered and was discharged from hospital on 06-Jan-2010.

The reporter assessed the causal relationship between the events and the suspected drugs according to the WHO score as possible.

Explanatory notes to causality:

Paracetamol: The adverse drug reactions are well known and described in the physician's information.

Diclofenac: The adverse drug reactions are well known and described in the physician's information as frequently occurring.

Sender's comment:

Sanofi-aventis Company Comment dated 05-MAR-2010:

Due to temporary relationship the causal role of paracetamol and diclofenac cannot be excluded.

12-MAR-2010 Sanofi-Aventis Germany

All reported clinical diagnoses are considered locally labelled for all suspected products and thus consistent with the known benefit risk ratio of the product(s). Measures according to Paragraph 29 German Drug Law (AMG), section (1) concerning the approval status are not deemed necessary.

Reporter's comment:

NOVARTIS, 17.03.2010 DE-002147023-PHHY2010DE13353

Sender diagnosis: not reported

Narrative

Initial report received from a regional Pharmacovigilance center (intensive monitoring project) via Health Authority (DE-BFARM-10027526) on 03 Mar 2010: This patient with medical history of viral upper respiratory infection and Limb discomfort received diclofenac (manufacturer unknown) suppository and paracetamol (manufacturer unknown) and was hospitalized for allergic reaction. He had headache and fever up to 38.5 degree Celsius (consistent with a viral infection) since 30 Dec 2009, followed by intake of paracetamol, 500 mg tablet daily, orally from 30 Dec 2009 to 02 Jan 2010 and single intake of diclofenac on 31 Dec 2009. On admission to hospital on 04 Jan 2010, he had swelling of the face, ears, eyelids and cheeks, severe reddening of the facial area, slight nausea, single episode of vomiting. After admission to hospital, he had no further exanthema or erythema on the body, no bronchial spasticity, afebrile, no meningism, blood pressure was 110/80 mmHg. He had not taken paracetamol before 30 Dec 2009 and this was therefore the first use. According to the primary health care provider's description, the patient had previously been completely healthy. During hospitalization, severe sweating gradually subsiding during the course. On 04 Jan 2010, Chest radiography showed diaphragm clearly distinguishable, incipient peribronchitic congestion not ruled out, otherwise no evidence of pulmonary infiltrations, no sign of pneumonia. C-reactive protein was 158.3 mg/dL (Reference range: 0 to 5 mg/L), absolute neutrophil count was $9.11 \times 10^9/L$ (Reference range: 1.5 to 7.7) and WBC was $15.32 \times 10^9/L$ (reference range: 4 to 10). He received Prednisolut (prednisolone-21-hydrogen succinate, sodium salt) 100 mg, intravenously and one ampoule of Fenistil (dimetinden) intravenously. He also received parenteral volume replacement and had a circulatory monitoring. On 06 Jan 2010, he had regressive erythema of the facial skin, partial detachment of the facial skin with overall tendency to seborrhea. He had general regression of the findings and complaints with symptomatic treatment. The hospitalization duration was from 04 Jan 2010 to 06 Jan 2010. The event outcome was reported as completely recovered. The regional Pharmacovigilance center assessed as possibly related to diclofenac and paracetamol.

Sender's comment:

Serious adverse drug reaction report, hypersensitivity [hospitalization], assessed as listed according to the Core Data Sheet. Reporting healthcare professional causality is suspected.

Reporter's comment:

HELIOS Klinikum Wuppertal, 07.04.2010 DE-PVZIBE-02-10251

Sender diagnosis: not reported

Narrative

ANMERKUNGEN ZU FOLGENDER UAW:

07.03.2016

BfArM - Pharmacovigilance

634 / 678

allergische Reaktion

KURZZUSAMMENFASSUNG:

allergischen Reaktion auf Paracetamol und Diclofenac

BESCHREIBUNG:

seit dem 30.12.2009 Kopfschmerzen und Fieber bis 38,5°C (im Sinne eines viralen Infektes), daraufhin Einnahme von Paracetamol (vom 30.12.2009 bis 02.01.2010) und Einnahme von Diclofenac einmalig am 31.12.2009; seit 2.01. zunächst Rötung des Gesichts, dann zunehmende Schwellung bis 04.01.. daraufhin Arzt aufgesucht

bei Einweisung Schwellung des Gesichtes, der Ohren, der Augenlider und der Wangen, starke Rötung des Gesichtsbereiches, leichte Übelkeit, einmaliges Erbrechen;

bei Aufnahme kein weiteres Exanthem oder Erythem am Körper, keine bronchiale Spastik, fieberfrei, kein Meningismus, Blutdruck 110/60mmHg; vor dem 30.12.2009 kein Paracetamol eingenommen, so dass dies die erstmalige Anwendung war

THERAPIE:

beim Hausarzt: 100mg Prednisolol i.v. und eine Ampulle Fenistil i.v.;

stationär: parenteraler Volumenersatz, Kreislaufkontrollen

VERLAUF:

laut Beschreibung des Hausarztes Patient bisher völlig gesund;

während des stationären Aufenthaltes starkes Schwitzen, das langsam im Verlauf abklingt;

04.01.2010 Röntgen-Thorax: Zwerchfell glatt abzugrenzen, beginnende peribronchitische Anschoppung nicht ausgeschlossen, sonst kein Nachweis pulmonaler Infiltrationen, kein Anhalt für Pneumonie;

06.01.2010 rückläufiges Erythem der Gesichtshaut, teilweise Ablösen der Gesichtshaut bei insgesamt Neigung des Patienten zur Seborrhoe;

bei symptomatologischer Behandlung allmähliche Befund- und Beschwerderegress

ERLÄUTERUNGEN ZUR UAW:

Krankenhausaufenthalt: 04.-06.01.2010;

Krankenhauseinweisung aufgrund einer allergischen Reaktion auf Paracetamol und Diclofenac;

Besserung nach Absetzen der Medikamente und antiallergisch wirksamer Therapie

ERLÄUTERUNGEN ZU DEN KAUSALITÄTEN:

MEDIKAMENT: paracetamol 500

BETREFFENDE UAW: allergische Reaktion

ERLÄUTERUNG: Überempfindlichkeitsreaktionen auf Paracetamol mit der Ausprägung der beschriebenen UAW sind in der Fachinfo beschrieben.

MEDIKAMENT: diclofenac

BETREFFENDE UAW: allergische Reaktion

ERLÄUTERUNG: Überempfindlichkeitsreaktionen auf Diclofenac sind bekannt und in der Fachinfo als häufig auftretend beschrieben.<dbtext>

To_Char_NARRATIVE_2</dbtext>

Sender's comment:

case closed; es werden keine weiteren Zusatzinformationen mehr erwartet.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Report from study

case: DE-BFARM-10035300

Serious Yes	Death Yes	Lifethr. Yes	Hospital. Yes	Disabling No	Congenital No	Other No	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Male	Birth: privacy	Age: 25 Year	weight (kg):	BMI_int:	Age fetus:
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Patientdeath

deathdate: 20090810	deathcause: Toxic epidermal necrolysis
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Genital erosion	20090602			88 Day	Unknown
2 Lip erosion	20090529			84 Day	Unknown
3 Eye red	20090529			84 Day	Unknown
4 Blister	20090526			81 Day	Unknown
5 Oral mucosa erosion	20090522			77 Day	Unknown
6 Macule	20090522			77 Day	Unknown
7 Toxic epidermal necrolysis	20090522	20090810	81 Day	77 Day	fatal
8 Ulcerative stomatitis	20090519			74 Day	Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ampho-moronal amphotericin b	Unknown	4*2 Df dosage form/ Day	20090325- -	Stomatitis
2 cotrim forte sulfamethoxazole; trimethoprim	Unknown	/ Day	20090311- -	Prophylaxis
3 Daraprim pyrimethamine	Oral	2*25 Mg milligram(s)/ Day 245	20090306- -	Toxoplasmosis
4 leucovorin folinic acid	Unknown	/	20090306- -	Electrolyte substitution therapy
5 Paracetamol paracetamol	Oral	/	20090326- -	Pain
6 Valtrex valaciclovir hydrochloride	Oral	3*1000 Mg milligram(s)/ D 245	20090519- -	Ulcerative stomatitis

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Acquired immunodeficiency syndrome			Yes	
Pain			Yes	
Cerebral toxoplasmosis			Yes	
Stomatitis			Yes	

Primary source

Qualification:	Physician	observestudytype:	Other studies
Literature:			
Qualification:	Other Health Professional	observestudytype:	Other studies
Literature:			

Narratives (nur bei elektronischem Eingang)

GlaxoSmithKline, 10.03.2010 DE-GlaxoSmithKline-D0066742A

Sender diagnosis: not reported

Narrative

This case was reported by a physician and described the occurrence of toxic epidermal necrolysis in a 25-year-old male patient who received Pyrimethamine (Daraprim) tablet for toxoplasmosis and Valaciclovir hydrochloride (Valtrex) for the treatment of stomatitis.

This report was received from the Intensified Monitoring from the Documentation Centre for Severe Skin Reactions in Freiburg.

Concurrent medical conditions included acquired immune deficiency syndrome, cerebral toxoplasmosis, pain and stomatitis. Co-suspect medication included Co-trimoxazole (Cotrim forte), Paracetamol, Calcium folinate (Leucovorin) and Amphotericin (Amphomoronal).

On 6 March 2009 the patient started Pyrimethamine (oral) at 25 mg twice per day. Approximately 74 days after starting Pyrimethamine, on 19 May 2009, the patient experienced ulcerative stomatitis.

The patient was treated with Valaciclovir hydrochloride.

On 22 May 2009 the patient developed macule and mucosa erosion of mouth and throat.

The patient was hospitalised and the physician considered the events to be life threatening.

On 26 May 2009 the patient developed skin blister, followed by redness of eyes, lip erosion and oral mucosa erosion on 29 May 2009 and erosion of genital mucosa on 02 June 2009.

The patient was diagnosed with toxic epidermal necrolysis.

The reporting physician considered the events were questionably related to treatment with Pyrimethamine, Valaciclovir hydrochloride, Paracetamol, Calcium folinate (Leucovorin) and Amphotericin, but probably related to treatment with Co-trimoxazole.

The patient died on 10 August 2009 from toxic epidermal necrolysis. It is unknown whether an autopsy was performed.

No further information will be available.

Sender's comment:

Die UAW ist unerwartet.

Wir sehen hier einen kausalen Zusammenhang als unwahrscheinlich. Es kommen weitere Arzneimittel in Frage.

Auf der Basis dieses Einzelfalles ergibt sich keine Aenderung des wissenschaftlichen Kenntnisstandes und der Nutzen/Risiko-Relation. Weitergehende eigenverantwortliche Massnahmen halten wir derzeit nicht fuer erforderlich.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-10038990

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes	Yes	No	No	No	No		Yes

Patient

Initials:	Sex: Male	Birth: privacy	Age: 42 Year	weight (kg):	BMI_int:	Age fetus:
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Patientdeath

deathdate: 20080324	deathcause: Stevens Johnson syndrome
deathdate: 20080324	deathcause: Toxic epidermal necrolysis
deathdate: 20080324	deathcause: TEN

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	20080304	20080524	82 Day	12 Day	fatal
2 TEN	20080304	20080524	82 Day	7 Day	fatal
3 Oral mucosa erosion	20080309			17 Day	Unknown
4 Blister	20080309			17 Day	Unknown
5 Fever	20080306			14 Day	Unknown
6 Hyponatremia					Unknown
7 Dyspnea					Unknown
8 Toxic epidermal necrolysis	20080304	20080524	82 Day	12 Day	fatal
9 Septic shock					Unknown
10 Acne					Unknown
11 Restlessness					Unknown
12 Macule	20080304			12 Day	Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
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DE-BFARM-10038990

1	aspisol acetylsalicylate lysine	Intravenous (not otherwis / 120	20080221-20080225 Thrombosis prophylaxis 5 Day
2	ciloxan ciprofloxacin hydrochloride	3 Df dosage form/Day	20080227-20080306 Otitis 9 Day
3	clindamycin clindamycin	Intravenous (not otherwis 1800 Mg milligram(s)/Day	20080306- Sepsis -
4	Daraprim pyrimethamine	Oral 50 Mg milligram(s)/Day 245	20080225- Toxoplasmosis -
5	fortum ceftazidime	Intravenous (not otherwis 3*2 G gram(s)/ Day 120	20080221-20080226 Sepsis 6 Day
6	Fragmin P dalteparin sodium	Subcutaneous .2 ml millilitre(s)/ Day 120	20080221-20080306 Thrombosis prophylaxis 15 Day
7	Haldol haloperidol	Intravenous (not otherwis / 120	20080221-20080302 Hallucination 11 Day
8	HALDOL haloperidol	Oral 15 Mg milligram(s)/Day	20080302-20080306 5 Day
9	HALDOL haloperidol	Intravenous (not otherwis 15 Mg milligram(s)/Day	20080221-20080222 Hallucination 2 Day
10	HALDOL haloperidol	Intravenous (not otherwis 10 Mg milligram(s)/Day	20080224-20080301 7 Day
11	insuman rapid insulin human; isophane insulin	Intravenous (not otherwis / 120	20080221-20080308 Hyperglycemia 17 Day
12	Kontrastmittel	Intravenous (not otherwis / 120	20080222- -
13	multiple medication homeopatics nos	Unknown /	- -
14	Nexium esomeprazole	Intravenous (not otherwis / Day 120	20080222- Prophylaxis -
15	Paracetamol paracetamol	Oral /	20080221-20080306 Fever 15 Day
16	sulfadiazin sulfadiazine	Intravenous (not otherwis 3*1.5 G gram(s)/ Day 120	20080225-20080306 Toxoplasmosis 11 Day
17	tavanic levofloxacin	Intravenous (not otherwis 2*500 Mg milligram(s)/ Da 231	20080301-20080306 Sepsis 6 Day
18	tavor lorazepam	Intravenous (not otherwis 3* / Day 120	20080221- Hallucination -
19	Tazobac piperacillin sodium; tazobactam sodium	Intravenous (not otherwis 3*4.5 Mg milligram(s)/ Da 120	20080226-20080306 Sepsis 10 Day
20	vancomycin vancomycin	Intravenous (not otherwis 2 G gram(s)/Day	20080306- -
21	vancomycin vancomycin	Intravenous (not otherwis 2 G gram(s)/Day	20080221-20080301 Otitis 10 Day
22	zovirax aciclovir	Intravenous (not otherwis 3*750 Mg milligram(s)/ Da 120	20080221-20080225 Encephalitis herpes 5 Day

Drugs (concomitant)

23	advantan methylprednisolone aceponate	Topical	/	20080225-20080227	Acne
				3 Day	
24	AMINOVEL amino acids nos; electrolytes nos; sorbitol; vitamins nos		/	20080221-20080306	
				15 Day	
25	ARTERENOL norepinephrine hydrochloride	Intravenous (not otherwis	/	20080306-20080308	
				3 Day	
26	calcium folinate calcium folinate	Oral	15 Mg milligram(s)/Day	20080225-20080306	
				11 Day	
27	EUNERPAN melperone hydrochloride	Oral	5 ml millilitre(s)/Day	20080304-20080305	Unrest
				2 Day	
28	KONAKION phytomenadione	Intravenous (not otherwis	/	20080228-20080305	
				7 Day	
29	KONTRAST U methiodal sodium	Intravenous (not otherwis	/	20080304-20080305	
				2 Day	
30	KONTRAST U methiodal sodium	Intravenous (not otherwis	/	20080228-20080228	
				1 Day	
31	KONTRAST U methiodal sodium	Intravenous (not otherwis	/	20080222-20080222	
				1 Day	
32	lavasept polihexanide	Topical	/	20080223-20080226	Acne
				4 Day	
33	lavasept polihexanide		3 Df dosage form/Day	20080227-20080306	Otitis
				9 Day	
34	MAGNESIOCARD magnesium aspartate hydrochloride	Intravenous (not otherwis	2 Df dosage form/Day	20080306-20080306	
				1 Day	
35	multiple medication homeopatics nos	Unknown	/	-	
				-	
36	Natriumchlorid sodium chloride	Oral	12 Df dosage form/Day	20080305-20080306	Hyponatraemia
				2 Day	
37	nizoral ketoconazole	Topical	/	20080225-20080306	Acne
				11 Day	
38	sultanol salbutamol	Respiratory (inhalation)	/	-	Dyspnea
			110	-	
39	THIAMIN HYDROCHLORID thiamine hydrochloride	Oral	/	20080229-20090306	
			245	12 Month	
40	VITAMIN B1 thiamine hydrochloride	Intravenous (not otherwis	100 Mg milligram(s)/Day	20080221-20080229	Vitamin B1 deficiency
				9 Day	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Delirium			Yes	
Encephalitis herpes			Yes	
Fever			Yes	
Hallucination			Yes	
Hospitalisation			Yes	
Hyperglycemia			Yes	
Mastoiditis			Yes	
Otitis			Yes	
Sepsis			Yes	
HIV infection			Yes	

Primary source

Qualification:	Physician	observestudytype:
Literature:		
Qualification:	Other Health Professional	observestudytype:

Literature:

Narratives (nur bei elektronischem Eingang)

GlaxoSmithKline, 11.03.2010 DE-GlaxoSmithKline-D0066743A

Sender diagnosis: not reported

Narrative

This case was reported by a physician and described the occurrence of Stevens Johnson syndrome in a male patient who received Ceftazidime sodium (Fortum) injection over a period of 6 days for sepsis. This report was received from the Intensified Monitoring from the Documentation Centre for Severe Skin Reactions in Freiburg.

Concurrent medical conditions included human immunodeficiency virus. Concurrently the patient was treated for delirium, fever, hallucination, herpetic encephalitis suspected, hyperglycemia, mastoiditis, otitis and sepsis. The patient was in hospital, at first at a stroke unit, later to an intensive care unit.

Co-suspect medication included Acyclovir (Zovirax), Pyrimethamine (Daraprim), Sulphadiazine (Sulfadiazin), Piperacillin sodium + Tazobactam sodium (Tazobac), Lysine aspirin (Aspisol), Haloperidol (Haldol), Paracetamol (Paracetamol), Dalteparin sodium (Fragmin P), Human Insulin (Insuman rapid), Vancomycin (Vancomycin), Lorazepam (Tavor), Contrast medium, Esomeprazole (Nexium), Levofloxacin (Tavanic) and Multiple other medication. Concurrent medications included salbutamol sulphate (Sultanol) and Multiple other medication.

On 21 February 2008 the patient started Ceftazidime sodium (intravenous) at 2 gram(s) three times per day and Acyclovir at 750 mg three times daily. Pyrimethamine was started on 25 February 2008 at 25 mg twice daily.

Approximately 12 days after starting Ceftazidime sodium, on 04 March 2008 the patient experienced macule, followed by fever on 06 March 2008 and by skin blister and oral mucosa erosion on 09 March 2008.

The subject was diagnosed with transitional form between Stevens Johnson syndrome and toxic epidermal necrolysis. The physician considered the events to be life threatening.

Additionally the patient was treated for septic shock, acne, restlessness, hyponatremia and dyspnea, but the start date of these events was not reported.

The reporting physician considered transitional form between Stevens Johnson syndrome and toxic epidermal necrolysis was probably related to treatment with Sulphadiazine and possibly related to all other suspect drugs. Otitis with mastoiditis and septic shock as well as HIV infection were also considered a possible cause of the events.

The patient died on 24 May 2008 from transitional form between Stevens Johnson syndrome and toxic epidermal necrolysis. It is unknown whether an autopsy was performed.

No further information will be available.

Sender's comment:

Die UAW ist unerwartet fuer Zovirax, Daraprim, Sultanol

Wir sehen hier einen kausalen Zusammenhang als unwahrscheinlich. Es kommen weitere Arzneimittel in Betracht.

Auf der Basis dieses Einzelfalles ergibt sich keine Aenderung des wissenschaftlichen Kenntnisstandes und der Nutzen/Risiko-Relation. Weitergehende eigenverantwortliche Massnahmen halten wir derzeit nicht fuer erforderlich.

Reporter's comment:

Wyeth-Pharma GmbH, 23.03.2010 DE-WYE-G05738710

Sender diagnosis: not reported

Narrative

This case has been received from Pfizer (Pfizer Case ID 2010030766) and contains a Pfizer co-suspect product. The Pfizer case may qualify for reporting for the Pfizer co-suspect product. This case may therefore be a duplicate report. Information was received from a healthcare professional via a registry entitled "European Registry of Severe Cutaneous Adverse Reactions to Drugs

(RegiSCAR)" regarding a 42-year-old male patient who received Tazobac (piperacillin/tazobactam injection) therapy and died of TEN and Stevens Johnson syndrome.

MEDICAL HISTORY:

Past therapies included Dormicum (midazolam maleate). The patient has been hospitalized since 21-Feb-2008.

PRODUCT DETAILS:

Indication for Tazobac was sepsis. Therapy began on 26-Feb-2008 and was permanently discontinued on 06-Mar-2008. Dose regimen was 4.5 mg 3 times per day (intravenous).

Additional suspect medication included Aspisol (acetylsalicylate lysine), Zovirax (aciclovir), Fortum (ceftazidime pentahydrate), Haldol (haloperidol), Paracetamol, Fragmin (dateparin sodium), Insuman (insulin human injection, isophane/insulin zinc protamine injection), Vancomycin, Tavor (lorazepam injection), Nexium (esomeprazole), Sulfadiazine, Daraprim (pyrimethamine), Ciloxan (ciprofloxacin hydrochloride), Tavanic (levofloxacin) and Clindamycin.

CONCOMITANT THERAPY:

Concomitant therapy included Vitamin B1 (thiamine hydrochloride), Aminovel (amino acids nos/electrolytes nos/sorbitol/vitamins nos), Kontrast U (methiodal sodium), Lavasept (epirizole), Advantan (methylprednisolone aceponate), Calcium Folate, Nizoral (ketoconazole), Lavasept (epirizole), Konakion (phytomenadione), Betabion (thiamine hydrochloride tablet), Eunerpan (melperone hydrochloride), Sodium Chloride, Magnesiacard (magnesium aspartate hydrochloride) and Arterenol (norepinephrine hydrochloride).

EVENT DETAILS:

On 21-Feb-2008 the patient experienced otitis and on 04-Mar-2008 the patient experienced cockade and spots as symptoms of TEN (toxic epidermal necrolysis) and Stevens Johnson syndrome (Stevens-Johnson syndrome). On 06-Mar-2008 the patient experienced fever and on 09-Mar-2008 the patient experienced vesicles of skin and erosions of oral mucosa. After the events occurred the patient received following drugs: Meronem (meropenem), Liquemin (heparin-sodium), KCL (potassium chloride), Lasix (furosemide), Disoprivan (propofol), AKE (isoleucine, leucine, lysine, methionine, phenylalanine), Sultanol (salbutamol), Atrovent (ipratropium bromide), Solu-Decortin H (prednisolone), Perfalgan (propacetamol), Addele (electrolytes), Glycerolphosphate, Deltamin (acetylcystein, arginine, serine, glycine, leucine), Clinoleic (soybean oil, glucose, calcium chloride, amino acids), Cernevit (retinol, colecalciferol, tocopherol, ascorbic acid, cocarboxylase), Tavegil (clemastine) and Decortin (prednisone). It was unknown which drugs were administered for treatment and which drugs were administered for other reasons. The patient died on 24-May-2008.

The cause of death was reported as Stevens-Johnson syndrome and toxic epidermal necrolysis.

RELATEDNESS ASSESSMENT:

The investigator considered Stevens-Johnson syndrome and toxic epidermal necrolysis probably related to Sulfadiazine and quite possibly related to Fortum (ceftazidime pentahydrate) and Tazobac (piperacillin/tazobactam injection) and possibly related to Aspisol (acetylsalicylate lysine), Zovirax (aciclovir), Haldol (haloperidol), Paracetamol, Fragmin (dateparin sodium), Insuman (insulin human injection, isophane/insulin zinc protamine injection), Vancomycin, Tavor (lorazepam injection), Nexium (esomeprazole), Daraprim (pyrimethamine), Ciloxan (ciprofloxacin hydrochloride), Tavanic (levofloxacin) and Clindamycin.

Sender's comment:

Wyeth comment:

Initial (v1) 15-Mar-2010

STEVENS-JOHNSON-SYNDROM unter Tazobac und Tavor

Dieser Fall wurde von Pfizer erhalten (Fallnummer 2010030766) und enthält ein verdächtiges Pfizer-Präparat, weshalb es zu einer Doppelmeldung kommen kann.

Fall aus dem RegiSCAR-Register der Uni Freiburg:

Es wurde über einen 42-jährigen männlichen Patienten berichtet, der unter Tazobac (Einnahme seit 26.02.08), Tavor pro Injektione (seit 21.02.08), Aspisol (Acetylsalicylsäure), Ciloxan (Ciprofloxacin), Clindamycin, Daraprim (Pyrimethamin), Fortum (Ceftazidin), Fragmin (Dalteparin), Haldol (Haloperidol), Insuman (Humaninsulin), Nexium (Esomeprazol), Paracetamol, Sulfadiazin, Tavanic (Levofloxacin), Vancomycin und Zovirax (Aciclovir) neben diverser Begleitmedikation ein Stevens-Johnson-Syndrom mit toxisch-epidermaler Nekrolyse entwickelte. Die Symptomatik begann am 04.03.08 mit Kokarden und Flecken, ab dem 06.03. trat Fieber auf und am 09.03. traten Blasen der Haut und Erosionen der Mundschleimhaut auf. Nach Auftreten wurden diverse Medikamente verabreicht, wobei nicht bekannt ist, welche zur Therapie und welche aus anderen Gründen verabreicht wurden. Am 24.05.08 verstarb der Patient aufgrund der Ereignisse.

Vom RegiSCAR-Register wird ein wahrscheinlicher Zusammenhang zwischen Sulfadiazin und den Ereignissen angenommen. Für Fortum und Tazobac wird ein gut möglicher Zusammenhang vermutet und für die anderen o.g. Arzneimittel ein möglicher Zusammenhang.

Die berichteten Ereignisse sind in der FI zu Tazobac aufgeführt. Aufgrund des tödlichen Verlaufs, auf den nicht explizit in der FI hingewiesen wird, wurden die Ereignisse sowohl unter Tazobac als auch unter Tavor als unbekannt bewertet.

Basierend auf der aktuellen Information führt dieser Bericht nicht zu einer Änderung des Nutzen-Risiko-Verhältnisses. Zu diesem Zeitpunkt sind keine weiteren Maßnahmen vorgesehen. Sobald Änderungen hierzu bekannt werden, werden wir darüber unverzüglich und unaufgefordert informieren.

Birte Kruse

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Report from study

case: DE-BFARM-10039872

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	Yes		

Patient

Initials:	Sex: Female	Birth: privacy	Age: 70 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	20070919	20071026	38 Day	13 Day	recovered/resolved
2 Blisters	20070925	20071026	32 Day	19 Day	recovered/resolved
3 Eye redness	20070925	20071026	32 Day	19 Day	recovered/resolved
4 Skin lesion	20070919	20071026	38 Day	13 Day	recovered/resolved
5 Pruritus	20070921	20071026	36 Day	15 Day	recovered/resolved
6 Oral mucosa redness	20070925	20071026	32 Day	19 Day	recovered/resolved
7 Fever	20070917	20071026	40 Day	11 Day	recovered/resolved
8 Nikolsky's sign	20070925	20071026	32 Day	19 Day	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
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DE-BFARM-10039872

1	acetylcysteine acetylcysteine	Oral	600 Mg milligram(s)/Day	20070905-20070918 14 Day	
2	acetylcysteine acetylcysteine	Oral	900 Mg milligram(s)/Day	20070828-20070828 1 Day	
3	acetylcysteine acetylcysteine	Intravenous (not otherwis	1200 Mg milligram(s)/Day	20070829-20070829 1 Day	
4	ADALAT nifedipine	Oral	/	20070911-20070914 4 Day	Hypertension
5	ALDACTONE potassium canrenoate	Oral	100 Mg milligram(s)/Day 120	20070903-20070903 1 Day	Hypertension
6	AMBROXOL ambroxol	Oral	45 Mg milligram(s)/Day	20070913-20070918 6 Day	Bronchospasm
7	avalox moxifloxacin hydrochloride	Oral	400 Mg milligram(s)/Day	20070916-20070919 4 Day	Psoas abscess
8	BELOC ZOK metoprolol succinate	Oral	2 Df dosage form/Day	20070902- -	Hypertension
9	BERODUAL fenoterol hydrobromide; ipratropium bromide	Respiratory (inhalation)	4 Df dosage form/Day	20070906- -	Respiratory obstruction unspecified
10	catapresan clonidine	Oral	450 Mg milligram(s)/Day	20070912- -	
11	catapresan clonidine	Intravenous (not otherwis	/	20070830-20070831 2 Day	Hypertension
12	CLEXANE heparin-fraction, sodium salt	Subcutaneous	/	20070908- -	Thrombosis prophylaxis
13	COMBACTAM sulbactam sodium	Intravenous (not otherwis	2 G gram(s)/Day	20070828-20070828 1 Day	Psoas abscess
14	diclac diclofenac sodium	Oral	150 Mg milligram(s)/Day	20070908-20070912 5 Day	Pain
15	DIOVAN valsartan	Oral	80 Mg milligram(s)/Day	20070907-20070912 6 Day	Hypertension
16	DIPIDOLOR piritramide	Intravenous (not otherwis	6 Mg milligram(s)/Day	20070828-20070828 1 Day	Pain
17	DIPIDOLOR piritramide	Intravenous (not otherwis	8 Mg milligram(s)/Day	20070908-20070909 2 Day	
18	DIPIDOLOR piritramide	Intravenous (not otherwis	9 Mg milligram(s)/Day	20070830-20070905 7 Day	
19	HYDROCHLOROTHIAZIDE hydrochlorothiazide	Oral	25 Mg milligram(s)/Day	20070907-20070911 5 Day	Diuresis
20	hydrocortisone hydrocortisone	Intravenous (not otherwis	/	20070829-20070901 4 Day	Sepsis
21	hydrocortisone hydrocortisone	Intravenous (not otherwis	/	20070907-20070914 8 Day	
22	INSULIN insulin	Subcutaneous	/	20070905-20070914 10 Day	Hyperglycaemia
23	L-THYROXIN levothyroxine sodium	Oral	75 µg microgram(s)/Day	20070910- -	Hypothyroidism
24	MUCOSOLVAN ambroxol hydrochloride	Oral	45 Mg milligram(s)/Day	20070913- -	
25	MUCOSOLVAN ambroxol hydrochloride	Intravenous (not otherwis	/	20070907-20070912 6 Day	
26	NORVASC amlodipine besilate	Oral	10 Mg milligram(s)/Day	20070902- -	Hypertension
27	NOVALGIN metamizole	Oral	30 Gtt drop(s)/Day	20070923-20070925 3 Day	
28	NOVALGIN metamizole		3 G gram(s)/Day	20070829-20070914 17 Day	Pain

DE-BFARM-10039872

29	PANTOZOL pantoprazole	Intravenous (not otherwis 40 Mg milligram(s)/Day	20070828-20070901	Gastric ulcer prophylaxis
30	PANTOZOL pantoprazole	Oral 40 Mg milligram(s)/Day	5 Day 20070905-	
31	PARACETAMOL paracetamol	Oral /	20070912-20070916	Fever
32	PIPERACILLIN piperacillin	Intravenous (not otherwis 8 G gram(s)/Day	5 Day 20070828-20070828	Psoas abscess
33	RIFAMPICIN rifampicin	Intravenous (not otherwis 600 Mg milligram(s)/Day	1 Day 20070901-20070916	Psoas abscess
34	ROCEPHIN ceftriaxone sodium	Intravenous (not otherwis 4 G gram(s)/Day	16 Day 20070828-20070831	Psoas abscess
35	SAROTEN amitriptyline hydrochloride	Oral 75 Mg milligram(s)/Day	4 Day 20070907-20070914	Pain
36	staphylex flucloxacillin sodium	Intravenous (not otherwis 6 G gram(s)/Day	8 Day 20070831-20070916	Psoas abscess
37	tavor lorazepam	Oral 1 Mg milligram(s)/Day	17 Day 20070920-20070920	
38	tavor lorazepam	Oral 1 Mg milligram(s)/Day	1 Day 20070918-20070918	
39	tavor lorazepam	Oral 1 Mg milligram(s)/Day	1 Day 20070907-20070914	Restlessness
40	tavor lorazepam	Oral 1 Mg milligram(s)/Day	8 Day 20070922-20070922	
41	TRAMAL tramadol hydrochloride	Oral 40 Gtt drop(s)/Day	1 Day 20070908-20070909	Pain
42	TRAMAL tramadol hydrochloride	Oral 60 Gtt drop(s)/Day	2 Day 20070912-20070913	
			2 Day	

Drugs (concomitant)			
43	actrapid mc insulin porcine	Intravenous (not otherwis /	20070829-20070905 Hyperglycaemia 8 Day
44	AMINO ACIDS NOS	Intravenous (not otherwis /	20070828-20070828 1 Day
45	AMINO ACIDS NOS/ELECTROLYTES NOS/S amino acids nos; electrolytes nos; sorbitol; xylitol	Intravenous (not otherwis /	20070903-20070905 3 Day
46	AMINO ACIDS NOS/ELECTROLYTES NOS/S amino acids nos; electrolytes nos; sorbitol; xylitol	Intravenous (not otherwis /	20070829-20070831 Volume depletion 3 Day
47	aminoven amino acids nos	Intravenous (not otherwis /	20070828-20070828 Parenteral fluid replacement 1 Day
48	ARTERENOL norepinephrine hydrochloride	Intravenous (not otherwis /	20070828-20070831 Instability cardiovascular 4 Day
49	ASCORBIC ACID/CALCIUM PANTOTHENATE ascorbic acid; calcium pantothenate; cyanocobalamin; nicotinamide; pyridoxine hydrochloride; retinol palmitate; riboflavin; rutoside; thiamine hydrochloride; tocopheryl acetate	Intravenous (not otherwis 1 Df dosage form/Day	20070902-20070902 Parenteral nutrition 1 Day
50	BELOC metoprolol tartrate	Intravenous (not otherwis 25 Mg milligram(s)/Day	20070830-20070901 3 Day
51	BELOC metoprolol tartrate	Intravenous (not otherwis 5 Mg milligram(s)/Day	20070828-20070828 Hypertension 1 Day
52	clindamycin clindamycin	Intravenous (not otherwis 1800 Mg milligram(s)/Day	20070828-20070831 Psoas abscess 4 Day
53	clindamycin clindamycin	Oral 2 Df dosage form/Day	20070920- -
54	CYANOCOBALAMIN cyanocobalamin	Intravenous (not otherwis 1 Df dosage form/Day	20090902-20090902 1 Day
55	DIAZEPAM diazepam	Intravenous (not otherwis 12.5 Mg milligram(s)/Day	20070828-20070828 Unrest 1 Day
56	DOLANTIN pethidine hydrochloride	Intravenous (not otherwis 25 Mg milligram(s)/Day	20070902-20070902 Pain 1 Day
57	EBRANTIL urapidil	Intravenous (not otherwis 250 Mg milligram(s)/Day	20070831-20070901 Hypertension 2 Day
58	EUGALAC lactulose	Oral /	20070904-20070905 Constipation 2 Day
59	fenistil dimetindene maleate	Oral 1 Df dosage form/Day	20070919- -
60	fenistil dimetindene maleate	Intravenous (not otherwis 1 Df dosage form/Day	20070908-20070908 Respiratory obstruction 1 Day unspecified
61	Folsaeure folic acid	Intravenous (not otherwis 1 Df dosage form/Day	20070902-20070902 1 Day
62	Glyceroltrinitrat glyceryl trinitrate	Respiratory (inhalation) 2 Df dosage form/Day	20070917-20070917 Hypertension 1 Day
63	HALDOL haloperidol	Intravenous (not otherwis 5 Mg milligram(s)/Day	20070828-20070828 Unrest 1 Day
64	HEPARIN heparin	Intravenous (not otherwis 15000 Iu international unit	20070828-20070828 Thrombosis prophylaxis 1 Day
65	HETASTARCH hetastarch	Intravenous (not otherwis /	20070828-20070829 Volume depletion 2 Day
66	HETASTARCH hetastarch	Intravenous (not otherwis /	20070907-20070913 7 Day
67	INSULIN GLARGINE insulin glargine	Subcutaneous /	20070902-20070902 Hyperglycaemia 1 Day
68	INSUMAN insulin human injection, isophane; insulin zinc protamine injection	Intravenous (not otherwis 8 Iu international unit(s)/D	20070828-20070828 Hyperglycaemia 1 Day

Drugs (concomitant)

69	INZOLEN aspartate zinc; cobalt hydrogenaspartate; copper hydrogenaspartate; magnesium aspartate; manganese hydrogenaspartate; potassium hydrogen aspartate	Intravenous (not otherwis /		20070912-20070914 3 Day
70	JONOSTERIL calcium acetate; magnesium acetate; potassium acetate; sodium acetate; sodium chloride	Intravenous (not otherwis /		20070906-20070911 6 Day
71	JONOSTERIL calcium acetate; magnesium acetate; potassium acetate; sodium acetate; sodium chloride	Intravenous (not otherwis /		20070828-20070903 Volume depletion 7 Day
72	JONOSTERIL calcium acetate; magnesium acetate; potassium acetate; sodium acetate; sodium chloride	Intravenous (not otherwis /		20070920-20070921 2 Day
73	JONOSTERIL calcium acetate; magnesium acetate; potassium acetate; sodium acetate; sodium chloride	Intravenous (not otherwis /		20070914-20070916 3 Day
74	kalinor potassium chloride	Oral	300 Mg milligram(s)/Day	20070912-20070920 9 Day
75	Kaliumchlorid potassium chloride	Intravenous (not otherwis /		20070828-20070913 Electrolyte replacement 17 Day
76	Kaliumzitrat potassium citrate	Intravenous (not otherwis 2 Df dosage form/Day		20070902-20070903 2 Day
77	LASIX furosemide	/		20070828-20070905 Diuresis 9 Day
78	magnetrans magnesium aspartate hydrochloride	Oral	900 Mg milligram(s)/Day	20070911-20070911 1 Day
79	METOCLOPRAMIDE metoclopramide	Intravenous (not otherwis 30 Mg milligram(s)/Day		20070828-20070901 Nausea 5 Day
80	MONO-EMBOLEX heparin-fraction, sodium salt	Subcutaneous	3000 Iu international unit(s)	20070830-20070907 Thrombosis prophylaxis 9 Day
81	moronal nystatin	/		20070914-20070918 Prophylaxis 5 Day
82	Natriumchlorid sodium chloride	/		- -
83	neostigmine neostigmine	Intravenous (not otherwis /		20070829-20070901 4 Day
84	PERENTEROL yeast dried	Oral	6 Df dosage form/Day	20070916-20070919 Diarrhea 4 Day
85	PERFALGAN paracetamol	Intravenous (not otherwis /		20070902-20070903 2 Day
86	PERFALGAN paracetamol	Intravenous (not otherwis /		20070828-20070828 Fever 1 Day
87	PERFALGAN paracetamol	Intravenous (not otherwis /		20070922-20070922 1 Day
88	PERFALGAN paracetamol	Intravenous (not otherwis /		20070924-20070924 1 Day
89	PHYTOMENADIONE phytomenadione	Intravenous (not otherwis 1 Df dosage form/Day		20070902-20070902 1 Day
90	prednisolone prednisolone	Oral	/	20070921-20070924 Exanthema 4 Day
91	PROPOFOL propofol	Intravenous (not otherwis /		20070828-20070831 Narcosis 4 Day
92	sobelin solubile clindamycin phosphate	Intravenous (not otherwis 1800 Mg milligram(s)/Day		20070828-20070828 Psoas abscess 1 Day
93	solu-decortin-h prednisolone sodium succinate	Intravenous (not otherwis 250 Mg milligram(s)/Day		20070906-20070906

Drugs (concomitant)

			Exanthema
		1 Day	
94	solu-decortin-h prednisolone sodium succinate	Intravenous (not otherwis 250 Mg milligram(s)/Day	20070920-20070920
		1 Day	
95	SUFENTA sufentanil citrate	Intravenous (not otherwis /	20070829-20070829 Narcosis
		1 Day	
96	Vitamin B calcium pantothenate; nicotinamide; pyridoxine hydrochloride; riboflavin; thiamine hydrochloride	Intravenous (not otherwis 1 Df dosage form/Day	20070911-20070911 Oral fluid replacement
		1 Day	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Sepsis			k.A.	
Psoas abscess			k.A.	
Intervertebral discitis			k.A.	
Spondylodiscitis			k.A.	
Spondylodiscitis			Yes	
Psoas abscess			Yes	
Diuresis			k.A.	
Fever			k.A.	
Hyperglycaemia			k.A.	
Hypertension			k.A.	
Hypothyroidism			k.A.	
Pain			k.A.	
Respiratory obstruction unspecified			k.A.	
Restlessness			k.A.	

Primary source

Qualification:	Physician	observestudytype:	Other studies
Literature:			

Narratives (nur bei elektronischem Eingang)

Nycomed, 19.03.2010 DE-Nycomed-0210604

Sender diagnosis: not reported

Narrative

The case was received by Pfizer (ref. no. 2010031955) /Wyeth (ref. no. DE-WYE-G05756510) Germany on 12-MAR-2010 from the German Documentation centre for severe skin reactions (RegiSCAR) and was forwarded to Nycomed on 17-MAR-2010.

It concerned a 70-year-old female patient started therapy with Pantozol (pantoprazole), 40 mg daily, intravenous, on 28-AUG-2007, for gastric ulcer prophylaxis. On 05-SEP-2007, intravenous pantoprazole was switched to oral pantoprazole, 20 mg twice daily. Additional suspect medications were piperacillin, sulbactam, ceftriaxone, piritramide, acetylcysteine, metamizole, hydrocortisone, clonidine, flucloxacillin, rifampicin, metoprolol, amlodipine besilate), potassium canrenoate, insulin, fenoterol /ipratropium, hydrochlorothiazide, valsartan, ambroxol, amitriptyline, diclofenac, tramadol heparin-fraction, levothyroxine sodium, nifedipine, paracetamol and moxifloxacin. The patient had a medical history of restlessness, intervertebral discitis, psoas abscess, hypertension, pain, hypothyroidism, unspecified respiratory obstruction, hyperglycaemia, diuresis and fever. She was concomitantly treated with clindamycin, human insulin, haloperidol, diazepam, furosemide, neostigmine, urapidil and prednisolone, amongst other numerous medications. From 19-SEP-2007 (22 days after the first dose of pantoprazole), the patient suffered from exanthema. On 21-SEP-2007, she experienced pruritus and on 22-SEP-2007, she presented with fever. On 25-SEP-2007, she was diagnosed with Stevens-Johnson syndrome. At that time she was additionally suffered from vesicles on the skin, redness of eyes and as well as redness oral mucosa and Nikolsky's signs. The patient was hospitalised. No details regarding clinical course or treatment were provided.

Action taken concerning therapy with pantoprazole was unknown.
The patient recovered on 26-OCT-2007.

The investigator considered Stevens-Johnson syndrome possibly related to pantoprazole and all other suspect medications.

Sender's comment:

Stevens-Johnson syndrome is listed in the current reference safety information on pantoprazole.

Stevens-Johnson Syndrome (SJS) is a rare but severe dermatological condition, considered as a severe type of erythema exsudativum multiforme. It is characterized by erythema with bullous and eroded lesions of skin and mucous membranes. Additional symptoms include fever, general ill feeling and joint aches. SJS is an immune-complex-mediated hypersensitivity reaction that occurs in response to medications (e.g., sulfonamides, penicillins, barbiturates, and phenytoin), infections (e.g., herpes simplex, Mycoplasma and malaria infections), or malignancies (various carcinomas and lymphomas). Stevens-Johnson syndrome (SJS) is idiopathic in 25-50% of cases.

This patient had other risk factors that could have caused or contributed to the development of the reaction e.g. co-suspect medications and several concomitant medications.

This single case report is not considered to change the current knowledge of the safety profile of pantoprazole.

Reporter's comment:

Wyeth-Pharma GmbH, 23.03.2010 DE-WYE-G05756510

Sender diagnosis: not reported

Narrative

This case has been received from Pfizer (Pfizer Case ID #2010031955) and contains a Pfizer co-suspect product. The Pfizer case may qualify for regulatory reporting for the Pfizer co-suspect product. This case may therefore be a duplicate report. Information was received from a healthcare professional via a registry entitled "European Registry of Severe Cutaneous Adverse Reactions to Drugs (RegiSCAR)" regarding a 70-year-old female patient who received Tavor (lorazepam unspc) therapy and experienced Stevens Johnson syndrome.

MEDICAL HISTORY:

The patient's concurrent illnesses include intervertebral discitis and psoas abscess.

PRODUCT DETAILS:

Indication for Tavor was restlessness. The dose regimen was 1 mg 1 time per day (oral) from 07-Sep-2007 to 14-Sep-2007, then was 1 mg (oral) on 18-Sep-2007, then was 1 mg (oral) on 20-Sep-2007 and then was 1 mg (oral) on 22-Sep-2007.

Additional suspect medication included Piperacillin, Combactam (sulbactam sodium), Rocephin (ceftriaxone sodium), Pantozol (pantoprazole unspc), Dipidolor (piritramide), Acetylcysteine, Novalgin (metamizole sodium), Hydrocortisone, Catapresan (clonidine), Staphylex (flucloxacillin sodium), Rifampicin, Beloc Zok (metoprolol succinate), Norvasc (amlodipine besilate), Aldactone (potassium canrenoate injection), Insulin (unspc), Berodual (fenoterol hydrobromide/ipratropium bromide), Hydrochlorothiazide, Diovan (valsartan), Mucosolvan (ambroxol hydrochloride), Saroten (amitriptyline hydrochloride), Diclac (diclofenac sodium), Tramal (tramadol hydrochloride), Clexane (heparin-fraction, sodium salt), L-Thyroxin (levothyroxine sodium), Adalat (nifedipine), Paracetamol and Avalox (moxifloxacin).

CONCOMITANT THERAPY:

Concomitant therapy included Sobelin Solubile (clindamycin phosphate unspc), Insuman (insulin human injection, isophane/insulin zinc protamine injection), Heparin, Haldol (haloperidol), Diazepam, Hetastarch, Amino Acids Nos (unspc), Clindamycin, Arterenol (norepinephrine hydrochloride), Disoprivan (propofol), Beloc (metoprolol tartrate), Metoclopramide, Lasix (furosemide), Potassium Chloride, Jonosteril (calcium acetate/magnesium acetate/potassium acetate/sodium chloride), Perfalgan (paracetamol), Neostigmine, Sufenta (sufentanil citrate), Multibionta (ascorbic acid/calcium pantothenate/cyanocobalamin/nicotinamide/pyridoxine hydrochloride/retinol palmitate/riboflavin/rutoside/thiamine hydrochloride/tocopheryl acetate), Actrapid Mc (insulin), Periplasol (amino acids nos/electrolytes nos/sorbitol/xylitol), Mono-Embolex (heparin-fraction, sodium salt), Ebrantil (urapidil), Lantus (insulin glargine), Dolantin (pethidine hydrochloride), Folic Acid, Vitamin B12 (cyanocobalamin), Vitamin K (phytomenadione), Potassium Citrate, Eugalac (lactulose), Solu-Decortin-H (prednisolone sodium succinate), Fenistil (dimetindene maleate unspc), Magnetrans (magnesium aspartate), Vitamin B, Sodium Chloride (unspc), Inzolen (aspartate zinc/cobalt hydrogenaspartate/copper hydrogenaspartate/magnesium aspartate/manganese hydrogenaspartate/potassium hydrogen aspartate), Kalinor (potassium chloride), Moronal (nystatin), Perenterol (yeast dried), Nitro (glyceryl trinitrate unspc) and Prednisolon (prednisolone).

EVENT DETAILS:

Starting from 19-Sep-2007 the patient suffered from exanthema. On 21-Sep-2007 she experienced pruritus and on 22-Sep-2007 additionally fever. Then on 25-Sep-2007 she was diagnosed with Stevens-Johnson syndrome (Stevens-Johnson syndrome). At this time she was additionally suffering from vesicles of skin, redness of eyes and as well as redness oral mucosa and Nikolsky's signs. No details regarding clinical course or treatment were provided. However she has recovered by 26-Oct-2007.

RELATEDNESS ASSESSMENT:

The investigator considered Stevens-Johnson syndrome possibly related to Tavor as well as to Piperacillin, Combactam, Rocephin, Pantozol, Dipidolor, Acetylcysteine, Novalgin, Hydrocortisone, Catapresan, Staphylex, Rifampicin, Beloc Zok, Norvasc, Aldactone, Insulin (unspc), Berodual, Hydrochlorothiazide, Diovan, Mucosolvan, Saroten, Diclac, Tramal, Clexane, L-Thyroxin, Adalat, Paracetamol and Avalox.

Sender's comment:

Wyeth comment:

Initial (v1) 16-Mar-2010

STEVENS-JOHNSON-SYNDROM nach Tavor

Dieser Fall wurde von Pfizer erhalten (Fallnummer 2010031955) und enthält ein verdächtiges Pfizer-Präparat, weshalb es zu einer Doppelmeldung kommen kann.

Fall aus dem RegiSCAR-Register der Uni Freiburg:

Es wurde über eine 70-jährige Patientin berichtet, die während eines stationären Aufenthaltes bei Spondylodisitis mit Psoasabszessen aufgrund von Unruhe vom 07.09.07 bis zum 15.09.07 sowie am 18., 20. und 22.09.07 Tavor erhielt. Sie bekam eine umfangreiche weitere medikamentöse Behandlung. Ab dem 19.09.07 entwickelte sie dermale Symptome, die sich bis zum 25.09.07 zu einem Steven-Johnson-Syndrom verschlimmerten. Ausser zu Tavor wurde ein möglicher Zusammenhang zur Behandlung mit Piperacillin, Combactam (Sulbactam), Rocephin (Ceftriaxon), Pantozol, Dipidolor (Piriramid), Acetylcystein, Novalgin (Metamizole), Hydrocortison, Catapresan (Clonidin), Staphylex (Flucloxacillin), Rifampicin, Beloc Zok (Metoprolol), Norvasc (Amlodipin), Aldactone, Insulin (unspez.), Berodual (Fenoterol/Ipratropiumbromid), Hydrochlorothiazid, Diovan (Valsartan), Mucosolvan (Ambroxol), Saroten (Amitriptyline), Diclac (Diclofenac), Tramal (Tramadol), Clexane (Heparin-Fraktion), L-Thyroxin (Levothyroxin), Adalat (Nifedipin), Paracetamol und Avalox (Moxifloxacin) gesehen. Weitere Informationen zum klinischen Verlauf und zur Behandlung lagen nicht vor. Bis zum 26.10.07 erholte sie sich jedoch von den Ereignissen.

Vom RegiSCAR-Register wird ein möglicher Zusammenhang für die o.g. Arzneimittel angenommen.

Die berichteten Ereignisse sind in der FI zu Tavor nicht aufgeführt und somit unbekannt.

Basierend auf der aktuellen Information führt dieser Bericht nicht zu einer Änderung des Nutzen-Risiko-Verhältnisses. Zu diesem Zeitpunkt sind keine weiteren Maßnahmen vorgesehen. Sobald Änderungen hierzu bekannt werden, werden wir darüber unverzüglich und unaufgefordert informieren.

Dr. D. Kracht

Reporter's comment:

NOVARTIS, 30.06.2010 DE-002147023-PHHY2010DE40691

Sender diagnosis: not reported

Narrative

Initial report received from a physician on 15 Jun 2010: This patient was enrolled in the non-Novartis multinational patient Registry RegiSCAR collecting data about Severe Cutaneous Adverse Reactions (SCAR). This patient with a history of a spondylodisitis with an psoas abscess was admitted to an intensive care unit on 24 Aug 2008. She was treated with a multiple drug regimen including Pipril iv (piperacillin) for psoasabscess, Sobelin iv (clindamycine) for psoasabscess, Combactam iv (sulbactam) for psoasabscess, Insuman Rapid iv (insulin human) for hyperglycaemia, heparin sodium iv for thrombosis prophylaxis, Haldol iv (haloperidol) for unrest, diazepam iv for unrest, Haes iv (polyhydroxyethyl starch, sodium chloride) for volume substitution, Aminoven iv (isoleucine, leucine, lysine, methionine, phenylalanine) for substitution, Rocephin iv (ceftriaxone) for psoasabscess, Arterenol iv (nor epinephrine) for stabilization of the circulatory system, Disoprivan iv (propofol) for narcosis, Beloc iv (metoprolol) for hypertension, MCP iv (metoclopramide) for nausea, Pantozol iv (pantoprazole) for stomach protection and Lasix iv (furosemide) for diuresis. From 28 Aug 2007 until 29 Aug 2007 the patient had to be transferred to an area for shock treatment. She was also given Dipidolor iv (piritramide) for pain, potassium chloride iv for substitution, Voluven iv (polyhydroxyethyl starch, sodium chloride) for substitution, NAC oral (acetylcysteine) for secretolysis, Jonosteril iv (sodium chloride, sodium acetate, potassium acetate, calcium acetate, magnesium acetate) for volume substitution, Perfalgan iv (paracetamol) for fever, ACC iv (acetylcysteine) for secretolysis, neostigmine iv for stimulation of the gut, Sufenta iv (sufentanil) for narcosis, Novalgin iv (metamizole) for pain, Actrapid iv (insulin) for hyperglycaemia, Periplasma iv (amino acids, glucose, sodium chloride, potassium chloride, magnesium chloride) for volume substitution, hydrocortisone iv for sepsis/stridor and a contrast media for a CT and angiography. On 30 Aug 2007 Catapresan iv (clonidine) for hypertension and Mono-Embolex 3000 IU sc (certoparin) for thrombosis prophylaxis were started. On 31 Aug 2007 Catapresan was stopped, Ebrantil iv (urapidil) for hypertension, Staphylex iv (flucloxacillin) for psoasabscess and rifampicin iv for psoasabscess were started. On 01 Sep Ebrantil was stopped, a single dose of Lantus sc (insulin glargine) for hyperglycaemia, beloc zok oral (metoprolol) for hypertension, Norvasc oral (amlodipine) for hypertension and Aldactone oral (spironolactone) for hypertension were started. On 02 Sep the patient received a single dose of Dolantin iv (pethidine) for pain, of folic acid iv, of vitamin B 12 iv and of vitamin K iv for substitution. Furthermore potassium oral and Lasix oral (furosemide) for diuresis were started. Potassium oral was stopped on 03 Sep. On 04 Sep 2007 the patient received single doses of Novalgin oral (metamizole), of Kitiva iv (unknown ingredients) for pain and of Eugalac oral (lactulose) for constipation. On 05 Sep Novalgin oral for pain, insulin sc for hyperglycaemia and Pantozol oral for stomach protection were started. On 06 Sep 2007 a single dose of Solu-Decortin H iv (prednisolone) for stridor and an exanthema was given. Berodual inhalation (ipratropium bromide/fenoterol) for pulmonary obstruction was started. On 07 Sep 2007 Mono-Embolex was stopped and HCT oral (hydrochlorothiazide) for diuresis, Diovan oral (valsartan) for hypertension, Mucosolvan iv (ambroxol) for secretolysis, Tavor oral (lorazepam) for unrest and Saroten oral (amitriptyline) for pain were started. On 08 Sep single doses of Fenistil iv (dimetidine) and erythrocyte concentrate iv for anaemia were given. Also Diclac oral (diclofenac) for pain, Tramal oral (tramadol) for pain and Clexane sc (enoxaparin) for thrombosis prophylaxis were started. On 10 Sep 2007 levothyroxine oral for hypothyroidism was started. On 11 Sep 2007 the patient was given a single dose of Magnetran forte oral (magnesium oxide) and of vitamin B iv for substitution. Furthermore Adalat oral (nifedipine) for

hypertension and sodium chloride iv to keep the injection entry open were started. HCT was stopped on the same day. On 12 Sep 2007 Diovan, Mucosolvan and Diclac were stopped. Inzolen iv (potassium aspartate, magnesium aspartate, copper hydrogenaspartate, zinc aspartate, manganese hydrogenaspartate) for substitution, paracetamol oral for fever, Kalinor oral (potassium citrate, potassium hydrogencarbonate, citric acid) for substitution and Catapresan oral (clonidine) for hypertension were started. On 13 Sep 2007 Tramal was stopped, ambroxol oral for secretolysis was started. On 14 Sep 2007 Novalgin oral, insulin sc, Saroten oral, Adalat oral and Inzolen iv were stopped, Moronal top (nystatin) for prophylaxis of soor and Xaramitis oral (unknown ingredients) were started. The patient was transferred on this day to a surgical unit. On 15 Sep 2007 Perenterol oral (saccharomyces boulardii) for diarrhoea was started. On 16 Sep 2007 Staphylex iv, rifampicin iv and paracetamol oral were stopped, Avalox oral (moxifloxacin) for psoasabscess was started. On 17 Sep 2007 the patient developed fever which lasted until 20 Sep 2007. She was given a single dose of Nitro oral (glyceryl trinitrate) for hypertension. On 18 Sep 2007 ambroxol oral and Moronal oral were stopped. On 19 Sep the patient developed skin lesions. A Stevens-Johnson syndrome was diagnosed. Perenterol and Avalox were stopped. Fenistil oral (dimetidine) for rash was started. On 20 Sep 2007 the patient was given a single dose of Solu- Decortin H iv (prednisolone) for pulmonary obstruction. Kalinor oral and Xaramitis were stopped, Clinda saar oral (clindamycin) for psoasabscess was started. On 21 Sep 2007 the patient developed pruritus, prednisolone oral for exanthema was started. On 22 Sep 2007 the patient developed again fever. On 23 Sep 2007 Novaminsulfon drops (metamizole) for fever was started and was given for two days. On 24 Sep 2007 prednisolone oral was stopped. On 25 Sep 2007 the patient developed also blisters on the skin, Nikolskys sign and redness of the eyes and oral mucosa. The Stevens-Johnson syndrome lasted until 26 Oct 2007. The patient had fully recovered. The causal relationship between the Stevens-Johnson syndrome and the medications was assessed by a dermatology expert panel as follows.: To Pipril iv, Sobelin iv, Combactam iv, Insuman Rapid iv, heparin iv, Haldol iv, diazepam iv, Haes iv, Aminoven iv, Rocephin iv, clindamycin iv, Arterenol iv, Disoprivan iv, Beloc iv, MCP iv, Pantozol iv, Lasix iv, Perfalgan iv, ACC iv, neostigmine iv, Sufenta iv, Novalgin iv, Actrapid iv, PeriPlasmasol iv, Ebrantil iv, Lantus sc, Dolantin iv, folic acid iv, vitamin B12 iv, vitamin K iv, potassium oral, Novalgin oral, Kitiva iv, Eugalac oral, Fenistil oral, Clinda Saar oral, prednisolone oral and Novaminsulfon oral not related. To potassium chloride iv, Voluven iv, Jonosteril iv, Catapresan iv, Mono-Embolex sc, Solu-Decortin H iv, Fenistil iv, erythrocyte concentrate iv, Magnetrans forte oral, vitamin B iv, sodium chloride iv, Inzolen iv, Kalinor oral, Moronal top, Perenterol oral and Nitro oral as doubtful. To Dipidolor iv, NAC oral, Tavor oral, hydrocortisone iv, contrast media iv, Beloc zok oral, Norvasc oral, Aldactone oral, Lasix oral, insulin sc, Pantozol oral, Berodual inh, HCT oral, Diovan oral, Mucosolvan oral, Saroten oral, Clexane sc, levothyroxine oral, Adalat oral, paracetamol oral, Catapresan oral, ambroxol oral, Xaramitis oral and Avalox oral as possible. To Staphylex iv, rifampicin iv, Novalgin drops oral, Diclac oral and Tramal drops oral as quite possible. In the absence of a health care professional's causality, the Novartis Medical Safety Physician has provisionally assessed a relationship between the event Stevens Johnson syndrome " and " potassium chloride " as suspected based on the current available information.

Sender's comment:

For Diovan and hydrochlorothiazide

Serious adverse drug reaction report: Stevens Johnson syndrome [hospitalization], assessed as unlisted according to the Core Data Sheet. However other alternative causes [other drugs] provide a possible explanation for the reported adverse event. Reporting healthcare professional causality is suspected.

For potassium chloride

Stevens Johnson syndrome [hospitalization], assessed as unlisted according to the Core Data Sheet . However other alternative causes [other drugs] provide a possible explanation for the reported adverse event. In the absence of a health care professional's causality, the Novartis Medical Safety Physician has provisionally assessed a relationship between the event Stevens Johnson syndrome " and " potassium chloride " as suspected based on the current available information.

For Pantozol

Stevens Johnson syndrome [hospitalization], assessed as listed according to the Core Data Sheet. Reporting healthcare professional causality is suspected.

Reporter's comment:

NOVARTIS, 01.07.2010 DE-NOVNCHHQ-PHHY2010DE40691

Sender diagnosis: not reported

Narrative

Initial report received from a physician on 15 Jun 2010: This patient was enrolled in the non-Novartis multinational patient Registry RegiSCAR collecting data about Severe Cutaneous Adverse Reactions (SCAR). This patient with a history of a spondylodiscitis with an psoas abscess was admitted to an intensive care unit on 24 Aug 2008. She was treated with a multiple drug regimen including Pipril iv (piperacillin) for psoasabscess,

Sobelin iv (clindamycine) for psoasabscess, Combactam iv (sulbactam) for psoasabscess, Insuman Rapid iv (insulin human) for hyperglycaemia, heparin sodium iv for thrombosis prophylaxis, Haldol iv (haloperidol) for unrest, diazepam iv for unrest, Haes iv (polyhydroxyethyl starch, sodium chloride) for volume substitution, Aminoven iv (isoleucine, leucine, lysine, methionine, phenylalanine) for substitution, Rocephin iv (ceftriaxone) for psoasabscess, Arterenol iv (nor epinephrine) for stabilization of the circulatory system, Disoprivan iv (propofol) for narcosis, Beloc iv (metoprolol) for hypertension, MCP iv (metoclopramide) for nausea, Pantozol iv (pantoprazole) for stomach protection and Lasix iv (furosemide) for diuresis. From 28 Aug 2007 until 29 Aug 2007 the patient had to be transferred to an area for shock treatment. She was also given Dipidolor iv (piritramide) for pain, potassium chloride iv for substitution, Voluven iv (polyhydroxyethyl starch, sodium chloride) for substitution, NAC oral (acetylcysteine) for secretolysis, Jonosteril iv (sodium chloride, sodium acetate, potassium acetate, calcium acetate, magnesium acetate) for volume substitution, Perfalgan iv (propacetamol) for fever, ACC iv (acetylcysteine) for secretolysis, neostigmine iv for stimulation of the gut, Sufenta iv (sufentanil) for narcosis, Novalgin iv (metamizole) for pain, Actrapid iv (insulin) for hyperglycaemia, Periplasma iv (amino acids, glucose, sodium chloride, potassium chloride, magnesium chloride) for volume substitution, hydrocortisone iv for sepsis/stridor and a contrast media for a CT and angiography. On 30 Aug 2007 Catapresan iv (clonidine) for hypertension and Mono-Embolex 3000 IU sc (certoparin) for thrombosis prophylaxis were started. On 31 Aug 2007 Catapresan was stopped, Ebrantil iv (urapidil) for hypertension, Staphylex iv (flucloxacillin) for psoasabscess and rifampicin iv for psoasabscess were started. On 01 Sep Ebrantil was stopped, a single dose of Lantus sc (insulin glargine) for hyperglycaemia, belok zok oral (metoprolol) for hypertension, Norvasc oral (amlodipine) for hypertension and Aldactone oral (spironolactone) for hypertension were started. On 02 Sep the patient received a single dose of Dolantin iv (pethidine) for pain, of folic acid iv, of vitamin B 12 iv and of vitamin K iv for substitution. Furthermore potassium oral and Lasix oral (furosemide) for diuresis were started. Potassium oral was stopped on 03 Sep. On 04 Sep 2007 the patient received single doses of Novalgin oral (metamizole), of Kitiva iv (unknown ingredients) for pain and of Eugalac oral (lactulose) for constipation. On 05 Sep Novalgin oral for pain, insulin sc for hyperglycaemia and Pantozol oral for stomach protection were started. 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Furthermore Adalat oral (nifedipine) for hypertension and sodium chloride iv to keep the injection entry open were started. HCT was stopped on the same day. On 12 Sep 2007 Diovan, Mucosolvan and Diclac were stopped. Inzolen iv (potassium aspartate, magnesium aspartate, copper hydrogenaspartate, zinc aspartate, manganese hydrogenaspartate) for substitution, paracetamol oral for fever, Kalinor oral (potassium citrate, potassium hydrogencarbonate, citric acid) for substitution and Catapresan oral (clonidine) for hypertension were started. On 13 Sep 2007 Tramal was stopped, ambroxol oral for secretolysis was started. On 14 Sep 2007 Novalgin oral, insulin sc, Saroten oral, Adalat oral and Inzolen iv were stopped, Moronal top (nystatin) for prophylaxis of soor and Xaramitis oral (unknown ingredients) were started. The patient was transferred on this day to a surgical unit. On 15 Sep 2007 Perenterol oral (saccharomyces boulardii) for diarrhoea was started. On 16 Sep 2007 Staphylex iv, rifampicin iv and paracetamol oral were stopped, Avalox oral (moxifloxacin) for psoasabscess was started. On 17 Sep 2007 the patient developed fever which lasted until 20 Sep 2007. She was given a single dose of Nitro oral (glyceryl trinitrate) for hypertension. On 18 Sep 2007 ambroxol oral and Moronal oral were stopped. On 19 Sep the patient developed skin lesions. A Stevens-Johnson syndrome was diagnosed. Perenterol and Avalox were stopped. Fenistil oral (dimetidine) for rash was started. On 20 Sep 2007 the patient was given a single dose of Solu- Decortin H iv (prednisolone) for pulmonary obstruction. Kalinor oral and Xaramitis were stopped, Clinda saar oral (clindamycin) for psoasabscess was started. On 21 Sep 2007 the patient developed pruritus, prednisolone oral for exanthema was started. On 22 Sep 2007 the patient developed again fever. On 23 Sep 2007 Novaminsulfon drops (metamizole) for fever was started and was given for two days. On 24 Sep 2007 prednisolone oral was stopped. On 25 Sep 2007 the patient developed also blisters on the skin, Nikolskys sign and redness of the eyes and oral mucosa. The Stevens-Johnson syndrome lasted until 26 Oct 2007. The patient had fully recovered. The causal relationship between the Stevens-Johnson syndrome and the medications was assessed by a dermatology expert panel as follows.: To Pipril iv, Sobelin iv, Combactam iv, Insuman Rapid iv, heparin iv, Haldol iv, diazepam iv, Haes iv, Aminoven iv, Rocephin iv, clindamycin iv, Arterenol iv, Disoprivan iv, Beloc iv, MCP iv, Pantozol iv, Lasix iv, Perfalgan iv, ACC iv, neostigmine iv, Sufenta iv, Novalgin iv, Actrapid iv, Periplasma iv, Ebrantil iv, Lantus sc, Dolantin iv, folic acid iv, vitamin B12 iv, vitamin K iv, potassium oral, Novalgin oral, Kitiva iv, Eugalac oral, Fenistil oral, Clinda Saar oral, prednisolone oral and Novaminsulfon oral not related. To potassium chloride iv, Voluven iv, Jonosteril iv, Catapresan iv, Mono-Embolex sc, Solu-Decortin H iv, Fenistil iv, erythrocyte concentrate iv, Magnetran forte oral, vitamin B iv, sodium chloride iv, Inzolen iv, Kalinor oral, Moronal top, Perenterol oral and Nitro oral as doubtful. To Dipidolor iv, NAC oral, Tavor oral, hydrocortisone iv, contrast media iv, Beloc zok oral, Norvasc oral, Aldactone oral, Lasix oral, insulin sc, Pantozol oral, Berodual inh, HCT oral, Diovan oral, Mucosolvan oral, Saroten oral, Clexane sc, levothyroxine oral, Adalat oral, paracetamol oral, Catapresan oral, ambroxol oral, Xaramitis oral and Avalox oral as possible. To Staphylex iv, rifampicin iv, Novalgin drops oral, Diclac oral and Tramal drops oral as quite possible. 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Sender's comment:

For Diovan and hydrochlorothiazide

Serious adverse drug reaction report: Stevens Johnson syndrome [hospitalization], assessed as unlisted according to the Core Data Sheet. However other alternative causes [other drugs] provide a possible explanation for the reported adverse event. Reporting healthcare professional causality is suspected.

For potassium chloride

Stevens Johnson syndrome [hospitalization], assessed as unlisted according to the Core Data Sheet . However other alternative causes [other drugs] provide a possible explanation for the reported adverse event.

In the absence of a health care professional's causality, the Novartis Medical Safety Physician has provisionally assessed a relationship between the event Stevens Johnson syndrome " and " potassium chloride " as suspected based on the current available information.

For Pantozol

Stevens Johnson syndrome [hospitalization], assessed as listed according to the Core Data Sheet. Reporting healthcare professional causality is suspected.

Reporter's comment:

NOVARTIS, 01.07.2010 DE-002147023-PHHY2010DE40691

Sender diagnosis: not reported

Narrative

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On 24 Sep 2007 prednisolone oral was stopped. On 25 Sep 2007 the patient developed also blisters on the skin, Nikolskys sign and redness of the eyes and oral mucosa. The Stevens-Johnson syndrome lasted until 26 Oct 2007. The patient had fully recovered. The causal relationship between the Stevens-Johnson syndrome and the medications was assessed by a dermatology expert panel as follows.: To

Pipril iv, Sobelin iv, Combactam iv, Insuman Rapid iv, heparin iv, Haldol iv, diazepam iv, Haes iv, Aminoven iv, Rocephin iv, clindamycin iv, Arterenol iv, Disoprivan iv, Beloc iv, MCP iv, Pantozol iv, Lasix iv, Perfalgan iv, ACC iv, neostigmine iv, Sufenta iv, Novalgin iv, Actrapid iv, PeriPlasma iv, Ebrantil iv, Lantus sc, Dolantin iv, folic acid iv, vitamin B12 iv, vitamin K iv, potassium oral, Novalgin oral, Kitiva iv, Eugalac oral, Fenistil oral, Clinda Saar oral, prednisolone oral and Novaminsulfon oral not related. To potassium chloride iv, Voluven iv, Jonosteril iv, Catapresan iv, Mono-Embolex sc, Solu-Decortin H iv, Fenistil iv, erythrocyte concentrate iv, Magnetran forte oral, vitamin B iv, sodium chloride iv, Inzolen iv, Kalinor oral, Moronal top, Perenterol oral and Nitro oral as doubtful. To Dipidolor iv, NAC oral, Tavor oral, hydrocortisone iv, contrast media iv, Beloc zok oral, Norvasc oral, Aldactone oral, Lasix oral, insulin sc, Pantozol oral, Berodual inh, HCT oral, Diovan oral, Mucosolvan oral, Saroten oral, Clexane sc, levothyroxine oral, Adalat oral, paracetamol oral, Catapresan oral, ambroxol oral, Xaramitis oral and Avalox oral as possible. To Staphylex iv, rifampicin iv, Novalgin drops oral, Diclac oral and Tramal drops oral as quite possible. In the absence of a health care professional's causality, the Novartis Medical Safety Physician has provisionally assessed a relationship between the event Stevens Johnson syndrome " and " potassium chloride " as suspected based on the current available information.

Sender's comment:

For Diovan and hydrochlorothiazide

Serious adverse drug reaction report: Stevens Johnson syndrome [hospitalization], assessed as unlisted according to the Core Data Sheet. However other alternative causes [other drugs] provide a possible explanation for the reported adverse event. Reporting healthcare professional causality is suspected.

For potassium chloride

Stevens Johnson syndrome [hospitalization], assessed as unlisted according to the Core Data Sheet . However other alternative causes [other drugs] provide a possible explanation for the reported adverse event. In the absence of a health care professional's causality, the Novartis Medical Safety Physician has provisionally assessed a relationship between the event Stevens Johnson syndrome " and " potassium chloride " as suspected based on the current available information.

For Pantozol

Stevens Johnson syndrome [hospitalization], assessed as listed according to the Core Data Sheet. Reporting healthcare professional causality is suspected.

Reporter's comment:

NOVARTIS, 22.08.2011 DE-002147023-PHHY2010DE40691

Sender diagnosis: not reported

Narrative

Initial report received from a physician on 15 Jun 2010: This patient was enrolled in the non-Novartis multinational patient Registry RegiSCAR collecting data about Severe Cutaneous Adverse Reactions (SCAR). This patient with a history of a spondylodiscitis with an psoas abscess was admitted to an intensive care unit on 24 Aug 2008. She was treated with a multiple drug regimen including Pipril iv (piperacillin) for psoasabscess, Sobelin iv (clindamycin) for psoasabscess, Combactam iv (sulbactam) for psoasabscess, Insuman Rapid iv (insulin human) for hyperglycaemia, heparin sodium iv for thrombosis prophylaxis, Haldol iv (haloperidol) for unrest, diazepam iv for unrest, Haes iv (polyhydroxyethyl starch, sodium chloride) for volume substitution, Aminoven iv (isoleucine, leucine, lysine, methionine, phenylalanine) for substitution, Rocephin iv (ceftriaxone) for psoasabscess, Arterenol iv (nor epinephrine) for stabilization of the circulatory system, Disoprivan iv (propofol) for narcosis, Beloc iv (metoprolol) for hypertension, MCP iv (metoclopramide) for nausea, Pantozol iv (pantoprazole) for stomach protection and Lasix iv (furosemide) for diuresis. From 28 Aug 2007 until 29 Aug 2007 the patient had to be transferred to an area for shock treatment. She was also given Dipidolor iv (piritramide) for pain, potassium chloride iv for substitution, Voluven iv (polyhydroxyethyl starch, sodium chloride) for substitution, NAC oral (acetylcysteine) for secretolysis, Jonosteril iv (sodium chloride, sodium acetate, potassium acetate, calcium acetate, magnesium acetate) for volume substitution, Perfalgan iv (paracetamol) for fever, ACC iv (acetylcysteine) for secretolysis, neostigmine iv for stimulation of the gut, Sufenta iv (sufentanil) for narcosis, Novalgin iv (metamizole) for pain, Actrapid iv (insulin) for hyperglycaemia, PeriPlasma iv (amino acids, glucose, sodium chloride, potassium chloride, magnesium chloride) for volume substitution, hydrocortisone iv for sepsis/stridor and a contrast media for a CT and angiography. On 30 Aug 2007 Catapresan iv (clonidine) for hypertension and Mono-Embolex 3000 IU sc (certoparin) for thrombosis prophylaxis were started. On 31 Aug 2007 Catapresan was stopped, Ebrantil iv (urapidil) for hypertension, Staphylex iv (flucloxacillin) for psoasabscess and rifampicin iv for psoasabscess were started. On 01 Sep Ebrantil was stopped, a single dose of Lantus sc (insulin glargine) for hyperglycaemia, beloc zok oral (metoprolol) for hypertension, Norvasc oral (amlodipine) for hypertension and Aldactone oral (spironolactone) for hypertension were started. On 02 Sep the patient received a single dose of Dolantin iv (pethidine) for pain, of folic acid iv, of vitamin B 12 iv and of vitamin K iv for substitution. Furthermore potassium oral and Lasix oral

(furosemide) for diuresis were started. Potassium oral was stopped on 03 Sep. On 04 Sep 2007 the patient received single doses of Novalgin oral (metamizole), of Kitiva iv (unknown ingredients) for pain and of Eugalac oral (lactulose) for constipation. On 05 Sep Novalgin oral for pain, insulin sc for hyperglycaemia and Pantozol oral for stomach protection were started. On 06 Sep 2007 a single dose of Solu-Decortin H iv (prednisolone) for stridor and an exanthema was given. Berodual inhalation (ipratropium bromide/fenoterol) for pulmonary obstruction was started. On 07 Sep 2007 Mono-Embolex was stopped and HCT oral (hydrochlorothiazide) for diuresis, Diovan oral (valsartan) for hypertension, Mucosolvan iv (ambroxol) for secretolysis, Tavor oral (lorazepam) for unrest and Saroten oral (amitriptyline) for pain were started. On 08 Sep single doses of Fenistil iv (dimetidine) and erythrocyte concentrate iv for anaemia were given. Also Diclac oral (diclofenac) for pain, Tramal oral (tramadol) for pain and Clexane sc (enoxaparin) for thrombosis prophylaxis were started. On 10 Sep 2007 levothyroxine oral for hypothyroidism was started. On 11 Sep 2007 the patient was given a single dose of Magnettrans forte oral (magnesium oxide) and of vitamin B iv for substitution. Furthermore Adalat oral (nifedipine) for hypertension and sodium chloride iv to keep the injection entry open were started. HCT was stopped on the same day. On 12 Sep 2007 Diovan, Mucosolvan and Diclac were stopped. Inzolen iv (potassium aspartate, magnesium aspartate, copper hydrogenaspartate, zinc aspartate, manganese hydrogenaspartate) for substitution, paracetamol oral for fever, Kalinor oral (potassium citrate, potassium hydrogencarbonate, citric acid) for substitution and Catapresan oral (clonidine) for hypertension were started. On 13 Sep 2007 Tramal was stopped, ambroxol oral for secretolysis was started. On 14 Sep 2007 Novalgin oral, insulin sc, Saroten oral, Adalat oral and Inzolen iv were stopped, Moronal top (nystatin) for prophylaxis of soor and Xaramitis oral (unknown ingredients) were started. The patient was transferred on this day to a surgical unit. On 15 Sep 2007 Perenterol oral (saccharomyces boulardii) for diarrhoea was started. On 16 Sep 2007 Staphylex iv, rifampicin iv and paracetamol oral were stopped, Avalox oral (moxifloxacin) for psoasabscess was started. On 17 Sep 2007 the patient developed fever which lasted until 20 Sep 2007. She was given a single dose of Nitro oral (glyceryl trinitrate) for hypertension. On 18 Sep 2007 ambroxol oral and Moronal oral were stopped. On 19 Sep the patient developed skin lesions. A Stevens-Johnson syndrome was diagnosed. Perenterol and Avalox were stopped. Fenistil oral (dimetidine) for rash was started. On 20 Sep 2007 the patient was given a single dose of Solu-Decortin H iv (prednisolone) for pulmonary obstruction. Kalinor oral and Xaramitis were stopped, Clinda saar oral (clindamycin) for psoasabscess was started. On 21 Sep 2007 the patient developed pruritus, prednisolone oral for exanthema was started. On 22 Sep 2007 the patient developed again fever. On 23 Sep 2007 Novaminsulfon drops (metamizole) for fever was started and was given for two days. On 24 Sep 2007 prednisolone oral was stopped. On 25 Sep 2007 the patient developed also blisters on the skin, Nikolskys sign and redness of the eyes and oral mucosa. The Stevens-Johnson syndrome lasted until 26 Oct 2007. The patient had fully recovered. The causal relationship between the Stevens-Johnson syndrome and the medications was assessed by a dermatology expert panel as follows.: To Pipril iv, Sobelin iv, Combactam iv, Insuman Rapid iv, heparin iv, Haldol iv, diazepam iv, Haes iv, Aminoven iv, Rocephin iv, clindamycin iv, Arterenol iv, Disoprivan iv, Beloc iv, MCP iv, Pantozol iv, Lasix iv, Perfalgan iv, ACC iv, neostigmine iv, Sufenta iv, Novalgin iv, Actrapid iv, PeriPlasmasol iv, Ebrantil iv, Lantus sc, Dolantin iv, folic acid iv, vitamin B12 iv, vitamin K iv, potassium oral, Novalgin oral, Kitiva iv, Eugalac oral, Fenistil oral, Clinda Saar oral, prednisolone oral and Novaminsulfon oral not related. To potassium chloride iv, Voluven iv, Jonosteril iv, Catapresan iv, Mono-Embolex sc, Solu-Decortin H iv, Fenistil iv, erythrocyte concentrate iv, Magnettrans forte oral, vitamin B iv, sodium chloride iv, Inzolen iv, Kalinor oral, Moronal top, Perenterol oral and Nitro oral as doubtful. To Dipidolor iv, NAC oral, Tavor oral, hydrocortisone iv, contrast media iv, Beloc zok oral, Norvasc oral, Aldactone oral, Lasix oral, insulin sc, Pantozol oral, Berodual inh, HCT oral, Diovan oral, Mucosolvan oral, Saroten oral, Clexane sc, levothyroxine oral, Adalat oral, paracetamol oral, Catapresan oral, ambroxol oral, Xaramitis oral and Avalox oral as possible. To Staphylex iv, rifampicin iv, Novalgin drops oral, Diclac oral and Tramal drops oral as quite possible. In the absence of a health care professional's causality, the Novartis Medical Safety Physician has provisionally assessed a relationship between the event Stevens Johnson syndrome " and " potassium chloride " as suspected based on the current available information.

Following an internal review on 10 Aug 2011, of the information received on 15 Jun 2010: Following significant correction was done. Mono-embolex was recoded with Novartis dictionary.

Sender's comment:

For Diovan and hydrochlorothiazide

Serious adverse drug reaction report: Stevens Johnson syndrome [hospitalization], assessed as unlisted according to the Core Data Sheet. However other alternative causes [other drugs] provide a possible explanation for the reported adverse event. Reporting healthcare professional causality is suspected.

For potassium chloride

Stevens Johnson syndrome [hospitalization], assessed as unlisted according to the Core Data Sheet . However other alternative causes [other drugs] provide a possible explanation for the reported adverse event. In the absence of a health care professional's causality, the Novartis Medical Safety Physician has provisionally assessed a relationship between the event Stevens Johnson syndrome " and " potassium chloride " as suspected based on the current available information.

For Pantozol

Stevens Johnson syndrome [hospitalization], assessed as listed according to the Core Data Sheet. Reporting healthcare professional causality is suspected.

For Mono-Embolex,

Stevens Johnson syndrome [hospitalization], assessed as unlisted according to the Core Data Sheet . However other alternative causes [other drugs] provide a possible explanation for the reported adverse event.

In the absence of the reporting healthcare professional causality, Novartis has processed the case as suspected for reporting purposes. The case will be reassessed on receipt of follow-up information.

MR: The event could have been confounded by pantozol.

Reporter's comment:

NOVARTIS, 22.08.2011 DE-NOVNCHHQ-PHHY2010DE40691

Sender diagnosis: not reported

Narrative

Initial report received from a physician on 15 Jun 2010: This patient was enrolled in the non-Novartis multinational patient Registry RegiSCAR collecting data about Severe Cutaneous Adverse Reactions (SCAR). This patient with a history of a spondylodiscitis with an psoas abscess was admitted to an intensive care unit on 24 Aug 2008. She was treated with a multiple drug regimen including Pipril iv (piperacillin) for psoasabscess, Sobelin iv (clindamycine) for psoasabscess, Combactam iv (sulbactam) for psoasabscess, Insuman Rapid iv (insulin human) for hyperglycaemia, heparin sodium iv for thrombosis prophylaxis, Haldol iv (haloperidol) for unrest, diazepam iv for unrest, Haes iv (polyhydroxyethyl starch, sodium chloride) for volume substitution, Aminoven iv (isoleucine, leucine, lysine, methionine, phenylalanine) for substitution, Rocephin iv (ceftriaxone) for psoasabscess, Arterenol iv (nor epinephrine) for stabilization of the circulatory system, Disoprivan iv (propofol) for narcosis, Beloc iv (metoprolol) for hypertension, MCP iv (metoclopramide) for nausea, Pantozol iv (pantoprazole) for stomach protection and Lasix iv (furosemide) for diuresis. From 28 Aug 2007 until 29 Aug 2007 the patient had to be transferred to an area for shock treatment. She was also given Dipidolor iv (piritramide) for pain, potassium chloride iv for substitution, Voluven iv (polyhydroxyethyl starch, sodium chloride) for substitution, NAC oral (acetylcysteine) for secretolysis, Jonosteril iv (sodium chloride, sodium acetate, potassium acetate, calcium acetate, magnesium acetate) for volume substitution, Perfalgan iv (propacetamol) for fever, ACC iv (acetylcysteine) for secretolysis, neostigmine iv for stimulation of the gut, Sufenta iv (sufentanil) for narcosis, Novalgin iv (metamizole) for pain, Actrapid iv (insulin) for hyperglycaemia, Peri plasma iv (amino acids, glucose, sodium chloride, potassium chloride, magnesium chloride) for volume substitution, hydrocortisone iv for sepsis/stridor and a contrast media for a CT and angiography. On 30 Aug 2007 Catapresan iv (clonidine) for hypertension and Mono-Embolex 3000 IU sc (certoparin) for thrombosis prophylaxis were started. On 31 Aug 2007 Catapresan was stopped, Ebrantil iv (urapidil) for hypertension, Staphylex iv (flucloxacillin) for psoasabscess and rifampicin iv for psoasabscess were started. On 01 Sep Ebrantil was stopped, a single dose of Lantus sc (insulin glargine) for hyperglycaemia, beloc zok oral (metoprolol) for hypertension, Norvasc oral (amlodipine) for hypertension and Aldactone oral (spironolactone) for hypertension were started. On 02 Sep the patient received a single dose of Dolantin iv (pethidine) for pain, of folic acid iv, of vitamin B 12 iv and of vitamin K iv for substitution. Furthermore potassium oral and Lasix oral (furosemide) for diuresis were started. Potassium oral was stopped on 03 Sep. On 04 Sep 2007 the patient received single doses of Novalgin oral (metamizole), of Kitiva iv (unknown ingredients) for pain and of Eugalac oral (lactulose) for constipation. On 05 Sep Novalgin oral for pain, insulin sc for hyperglycaemia and Pantozol oral for stomach protection were started. 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Saar oral, prednisolone oral and Novaminsulfon oral not related. To potassium chloride iv, Voluven iv, Jonosteril iv, Catapresan iv, Mono-Embolex sc, Solu-Decortin H iv, Fenistil iv, erythrocyte concentrate iv, Magnetrans forte oral, vitamin B iv, sodium chloride iv, Inzolen iv, Kalinor oral, Moronal top, Perenterol oral and Nitro oral as doubtful. To Dipidolor iv, NAC oral, Tavor oral, hydrocortisone iv, contrast media iv, Beloc zok oral, Norvasc oral, Aldactone oral, Lasix oral, insulin sc, Pantozol oral, Berodual inh, HCT oral, Diovan oral, Mucosolvan oral, Saroten oral, Clexane sc, levothyroxine oral, Adalat oral, paracetamol oral, Catapresan oral, ambroxol oral, Xaramitis oral and Avalox oral as possible. To Staphylex iv, rifampicin iv, Novalgin drops oral, Diclac oral and Tramal drops oral as quite possible. In the absence of a health care professional's causality, the Novartis Medical Safety Physician has provisionally assessed a relationship between the event Stevens Johnson syndrome " and " potassium chloride " as suspected based on the current available information.

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Sender's comment:

For Diovan and hydrochlorothiazide

Serious adverse drug reaction report: Stevens Johnson syndrome [hospitalization], assessed as unlisted according to the Core Data Sheet. However other alternative causes [other drugs] provide a possible explanation for the reported adverse event. Reporting healthcare professional causality is suspected.

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For Pantozol

Stevens Johnson syndrome [hospitalization], assessed as listed according to the Core Data Sheet. Reporting healthcare professional causality is suspected.

For Mono-Embolex,

Stevens Johnson syndrome [hospitalization], assessed as unlisted according to the Core Data Sheet . However other alternative causes [other drugs] provide a possible explanation for the reported adverse event.

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MR: The event could have been confounded by pantozol.

Reporter's comment:

NOVARTIS, 22.08.2011 DE-SZ09-PHHY2010DE40691

Sender diagnosis: not reported

Narrative

Initial report received from a physician on 15 Jun 2010: This patient was enrolled in the non-Novartis multinational patient Registry RegiSCAR collecting data about Severe Cutaneous Adverse Reactions (SCAR). This patient with a history of a spondylodiscitis with an psoas abscess was admitted to an intensive care unit on 24 Aug 2008. She was treated with a multiple drug regimen including Pipril iv (piperacillin) for psoasabscess, Sobelin iv (clindamycine) for psoasabscess, Combactam iv (sulbactam) for psoasabscess, Insuman Rapid iv (insulin human) for hyperglycaemia, heparin sodium iv for thrombosis prophylaxis, Haldol iv (haloperidol) for unrest, diazepam iv for unrest, Haes iv (polyhydroxyethyl starch, sodium chloride) for volume substitution, Aminoven iv (isoleucine, leucine, lysine, methionine, phenylalanine) for substitution, Rocephin iv (ceftriaxone) for psoasabscess, Arterenol iv (nor epinephrine) for stabilization of the circulatory system, Disoprivan iv (propofol) for narcosis, Beloc iv (metoprolol) for hypertension, MCP iv (metoclopramide) for nausea, Pantozol iv (pantoprazole) for stomach protection and Lasix iv (furosemide) for diuresis. From 28 Aug 2007 until 29 Aug 2007 the patient had to be transferred to an area for shock treatment. She was also given Dipidolor iv (piritramide) for pain,

potassium chloride iv for substitution, Voluven iv (polyhydroxyethyl starch, sodium chloride) for substitution, NAC oral (acetylcysteine) for secretolysis, Jonosteril iv (sodium chloride, sodium acetate, potassium acetate, calcium acetate, magnesium acetate) for volume substitution, Perfalgan iv (paracetamol) for fever, ACC iv (acetylcysteine) for secretolysis, neostigmine iv for stimulation of the gut, Sufenta iv (sufentanil) for narcosis, Novalgin iv (metamizole) for pain, Actrapid iv (insulin) for hyperglycaemia, PeriPlasma iv (amino acids, glucose, sodium chloride, potassium chloride, magnesium chloride) for volume substitution, hydrocortisone iv for sepsis/stridor and a contrast media for a CT and angiography. On 30 Aug 2007 Catapresan iv (clonidine) for hypertension and Mono-Embolex 3000 IU sc (certoparin) for thrombosis prophylaxis were started. 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To potassium chloride iv, Voluven iv, Jonosteril iv, Catapresan iv, Mono-Embolex sc, Solu-Decortin H iv, Fenistil iv, erythrocyte concentrate iv, Magnetran forte oral, vitamin B iv, sodium chloride iv, Inzolen iv, Kalinor oral, Moronal top, Perenterol oral and Nitro oral as doubtful. To Dipidolor iv, NAC oral, Tavor oral, hydrocortisone iv, contrast media iv, Beloc Zok oral, Norvasc oral, Aldactone oral, Lasix oral, insulin sc, Pantozol oral, Berodual inh, HCT oral, Diovan oral, Mucosolvan oral, Saroten oral, Clexane sc, levothyroxine oral, Adalat oral, paracetamol oral, Catapresan oral, ambroxol oral, Xaramitis oral and Avalox oral as possible. To Staphylex iv, rifampicin iv, Novalgin drops oral, Diclac oral and Tramal drops oral as quite possible. In the absence of a health care professional's causality, the Novartis Medical Safety Physician has provisionally assessed a relationship between the event Stevens Johnson syndrome " and " potassium chloride " as suspected based on the current available information.

Following an internal review on 10 Aug 2011, of the information received on 15 Jun 2010: Following significant correction was done. Mono-embolex was recorded with Novartis dictionary.

Sender's comment:

For Diovan and hydrochlorothiazide

Serious adverse drug reaction report: Stevens Johnson syndrome [hospitalization], assessed as unlisted according to the Core Data Sheet. However other alternative causes [other drugs] provide a possible explanation for the reported adverse event. Reporting healthcare professional causality is suspected.

For potassium chloride

Stevens Johnson syndrome [hospitalization], assessed as unlisted according to the Core Data Sheet .

However other alternative causes [other drugs] provide a possible explanation for the reported adverse event.

In the absence of a health care professional's causality, the Novartis Medical Safety Physician has provisionally assessed a relationship between the event Stevens Johnson syndrome " and " potassium chloride " as suspected based on the current available information.

For Pantozol

Stevens Johnson syndrome [hospitalization], assessed as listed according to the Core Data Sheet. Reporting healthcare professional causality is suspected.

For Mono-Embolex,

Stevens Johnson syndrome [hospitalization], assessed as unlisted according to the Core Data Sheet .
However other alternative causes [other drugs] provide a possible explanation for the reported adverse event.

In the absence of the reporting healthcare professional causality, Novartis has processed the case as suspected for reporting purposes. The case will be reassessed on receipt of follow-up information.

MR: The event could have been confounded by pantozol.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-10038846

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		Yes

Patient

Initials:	Sex: Male	Birth: privacy	Age: 11 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Erythema exsudativum multiforme	20070915	20071001	17 Day	4 Day	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 CAPVAL noscipine resin	Oral	2* /Day	20070914-20070914	Cough
2 PARACETAMOL paracetamol	Oral	/	20070908-20070914	Febrile infection
3 zithromax azithromycin	Oral	1 Df dosage form/Day 82	20070912-20070915	Febrile infection

Drugs (concomitant)

4 SEDOTUSSIN "UCB" pentoxyverine citrate	Oral	/Day	20070915-20070915	Cough
5 xylocain lidocaine hydrochloride	Oral	/	20070915-20070915	Stomatitis

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Stomatitis			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

PFIZERINC, 23.03.2010 DE-PFIZER INC-2010029185

Sender diagnosis: not reported

Narrative

This is a spontaneous report from a project, sponsored by the German Regulatory Authority, set up to collect suspected drug induced severe skin reactions leading to hospital admission. Reference number of the Documentation Center for Severe Skin Reactions (dZh): 9170594.

A contactable physician reported that this 11-year old male patient was treated with azithromycin (ZITHROMAX) one tablet once daily from 12Sep2007 to 15Sep2007 for febrile infection, paracetamol (manufacturer unknown) one to three tablets daily from 08Sep2007 to 14Sep2007 for febrile infection and noscapine (CAPVAL) liquid twice on 14Sep2007 for cough. Concomitant medication included pentoxyverine (SEDOTUSSIN) once on 15Sep2007 for cough and lidocaine (XYLOCAIN) solution on 15Sep2007 for stomatitis.

The patient experienced fever from 09Sep2007 to 13Sep2007 with fever increase on 12Sep2007, tonsillitis from 09Sep2007 to 15Sep2007, cough on 13Sep2007. On 15Sep2007 the patient experienced lip and oral mucosa erosions and was diagnosed with erythema exsudativum multiforme majus. On 16Sep2007 the patient experienced conjunctivitis and was hospitalized. On 17Sep2007 the patient experienced cockades and genital mucosa erosions. The event was not considered life-threatening. In the past the patient had used paracetamol and tolerated it well. Outcome was reported as recovered on 01Oct2007. Observation period including information on medication was available from 19Aug2007 to 16Sep2007. In Feb2008 the project experts assessed the causal relationship as possible with paracetamol and azithromycin and as dubious with noscapine. Infection as alternative cause was documented as probable: Mycoplasma infection by the project experts.

Follow-up status: case closed (08Mar2010).

Sender's comment:

There is a reportedly positive temporal relationship between treatment administration and onset of the event; causality therefore cannot be completely excluded. Bei Erythema multiforme handelt es sich um eine bekannte, in der Produktinformation ausreichend praezise deklarierte Nebenwirkung. Wir sehen zum gegenwaertigen Zeitpunkt keinen Anlass fuer eine geaenderte Bewertung des Nutzen-/Risikoverhaeltnisses.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-10096009

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	Yes		Yes

Patient

Initials:	Sex: Male	Birth: privacy	Age: 4 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens-Johnson syndrome	20080521			11 Day	recovered/resolved
2 Skin discoloration	20080521			11 Day	recovered/resolved
3 Blister	20080521			11 Day	recovered/resolved
4 Oral mucosa erosion	20080522			12 Day	recovered/resolved
5 Nasal disorder	20080523			13 Day	recovered/resolved
6 Vomiting	20080519			9 Day	recovered/resolved
7 Itching	20080521			11 Day	recovered/resolved
8 Genital erosion	20080522			12 Day	recovered/resolved
9 Fever	20080519			9 Day	recovered/resolved
10 Ear pain	20080519			11 Day	recovered/resolved
11 Ear disorder	20080523			13 Day	recovered/resolved
12 Lip erosion	20080522			12 Day	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 BENURON paracetamol	Rectal	/	20080519-20080521 3 Day	Fever
2 cotrim forte heumann sulfamethoxazole; trimethoprim	Unknown	/	-	-
3 LAMICTAL "BURROUGHS WELLCOME" lamotrigine	Unknown	/	-	-
4 olynth SASSE FRIEDRICH xylometazoline hydrochloride	Nasal	/	20080519- -	Sinusitis
5 otriven xylometazoline hydrochloride	Nasal	/ 146	20080511-20080514 4 Day	Rhinitis
6 TEGRETAL carbamazepine	Unknown	/	-	-
7 VOMEX A "ENDOPHARM" dimenhydrinate	Oral	/	20080519- -	Vomiting

Primary source

Qualification: Other Health Professional

observestudytype:

Literature:

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 25.06.2010 DE-NOVNCHHQ-CHPA2010DE10165

Sender diagnosis: not reported

Narrative

initial report received from systematically screening of the German documentation centre for severe skin reactions dZh, Freiburg, (Case No: 9170688, case was not received from BfArM, case was assessed by the experts in Sep 2009) on 15 Jun 2010. A 4 year old male patient was hospitalised on 21 May 2008 (end date not reported). The treatment was graphically recorded for the time period from 23 Apr 2008 to 21 May 2008. The patient cosuspect medications included: OTRIVEN Nasenspray (xylometazoline) to treat rhinitis from 11 May 2008 to 14 May 2008. Concurrently, the patient was being treated rectally with Benuron Supp (paracezamol) to treat fever from 19 May 2008 to 21 May 2008, orally with Vomex Saft (dimenhydrinate) to treat vomiting from 19 May 2008 to an unknown date, and nasally with Olynth Nasenspray (xylometazoline) several times to treat suspected sinusitis from 19 May 2008 to an unknown date, Lamictal (lamotrigine), Cotrim forte (sulfamethoxazole, trimethoprim) and TEGRETAL

(carbamazepine). The patient was suffering from 'rhinitis' from unknown date to 14 May 2008, fever, vomiting, and ear pain on 19 May 2008, itching, skin spots, skin blister on 21 May 2008, lip erosions, oral mucosa erosions, genital mucosa erosions on 22 May 2008, and nasal and auricular erosions on 23 May 2008. The index day for first occurrence of blisters and erosions was reported as 21 May 2008. The reaction was specified as 'Stevens-Johnson syndrome (probable case)'. The onset date of reaction was specified as 21 May 2008. The reaction was rated as non life-threatening. The intensive monitoring project assessed the causality with regard to the patient's medication (using an algorithm with 7 different levels, ranging from definite to no causal relationship) as follows: Otriven Nasenspray, Benuron Supp, Vomex Saft and Olynth Nasenspray were rated as '6 =questionable'. Additionally, Otriven Nasenspray, Benuron Supp, and Olynth Nasenspray were classified as 'used before' and 'tolerated well before', respectively. Vomex Saft was classified as 'not used before'. It was added that the patient was suffering from infection (rhinitis) as other possible risk factors and that there were no further known pre-existing conditions. According to overall rating for Lamictal, Cotrim forte and TEGRETAL (carbamazepine) were rated as 'highly suspicious' with regard to causality assessment. Outcome of the events was reported as recovery. No additional information was available.

Sender's comment:

OTRIVEN (NCH): All spontaneous reports are considered suspect for reporting purposes. This is a serious spontaneous report (hospitalization). The events Stevens-Johnson syndrome, and related events Pyrexia, Vomiting, Ear pain, Pruritus, Skin discolouration, Blister, Lip erosion, Oral mucosa erosion, Genital erosion, Nasal disorder, and Ear disorder were assessed as unlabelled according to the product information. The available information was considered inadequate to fully assess the case.

TEGRETAL: Serious spontaneous report; The events Stevens-Johnson syndrome, Stevens-Johnson syndrome [hospitalisation] was already listed in the Core Data Sheet. All spontaneous reports are considered suspected for reporting purposes or in accordance with national regulatory requirements.

Reporter's comment:

NOVARTIS, 25.06.2010 DE-002147023-CHPA2010DE10165

Sender diagnosis: not reported

Narrative

Initial report received from systematically screening of the German documentation centre for severe skin reactions dZh, Freiburg, (Case No: 9170688, case was not received from BfArM, case was assessed by the experts in Sep 2009) on 15 Jun 2010. A 4 year old male patient was hospitalised on 21 May 2008 (end date not reported). The treatment was graphically recorded for the time period from 23 Apr 2008 to 21 May 2008. The patient cosuspect medications included: OTRIVEN Nasenspray (xylometazoline) to treat rhinitis from 11 May 2008 to 14 May 2008. Concurrently, the patient was being treated rectally with Benuron Supp (paracetamol) to treat fever from 19 May 2008 to 21 May 2008, orally with Vomex Saft (dimenhydrinate) to treat vomiting from 19 May 2008 to an unknown date, and nasally with Olynth Nasenspray (xylometazoline) several times to treat suspected sinusitis from 19 May 2008 to an unknown date, Lamictal (lamotrigine), Cotrim forte (sulfamethoxazole, trimethoprim) and TEGRETAL (carbamazepine). The patient was suffering from 'rhinitis' from unknown date to 14 May 2008, fever, vomiting, and ear pain on 19 May 2008, itching, skin spots, skin blister on 21 May 2008, lip erosions, oral mucosa erosions, genital mucosa erosions on 22 May 2008, and nasal and auricular erosions on 23 May 2008. The index day for first occurrence of blisters and erosions was reported as 21 May 2008. The reaction was specified as 'Stevens-Johnson syndrome (probable case)'. The onset date of reaction was specified as 21 May 2008. The reaction was rated as non life-threatening. The intensive monitoring project assessed the causality with regard to the patient's medication (using an algorithm with 7 different levels, ranging from definite to no causal relationship) as follows: Otriven Nasenspray, Benuron Supp, Vomex Saft and Olynth Nasenspray were rated as '6 =questionable'. Additionally, Otriven Nasenspray, Benuron Supp, and Olynth Nasenspray were classified as 'used before' and 'tolerated well before', respectively. Vomex Saft was classified as 'not used before'. It was added that the patient was suffering from infection (rhinitis) as other possible risk factors and that there were no further known pre-existing conditions. According to overall rating for Lamictal, Cotrim forte and TEGRETAL (carbamazepine) were rated as 'highly suspicious' with regard to causality assessment. Outcome of the events was reported as recovery. No additional information was available.

Sender's comment:

OTRIVEN (NCH): All spontaneous reports are considered suspect for reporting purposes. This is a serious spontaneous report (hospitalization). The events Stevens-Johnson syndrome, and related events Pyrexia, Vomiting, Ear pain, Pruritus, Skin discolouration, Blister, Lip erosion, Oral mucosa erosion, Genital erosion, Nasal disorder, and Ear disorder were assessed as unlabelled according to the product information. The available information was considered inadequate to fully assess the case.

TEGRETAL: Serious spontaneous report; The events Stevens-Johnson syndrome, Stevens-Johnson syndrome [hospitalisation] was already listed in the Core Data Sheet. All spontaneous reports are considered suspected for reporting purposes or in accordance with national regulatory requirements.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-11236823

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	Yes		

Patient

Initials:	Sex: Male	Birth: privacy	Age: 19 Month	weight (kg): 13	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Angioedema	19961211			28 Day	recovering/resolving
2 Stevens Johnson syndrome	19961213			30 Day	recovering/resolving
3 Airway obstruction NOS	19961207			24 Day	Unknown
4 Ascites	19961204			18 Day	recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
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1	ALDACTONE	/	19961204-	
	spironolactone		-	
2	ambisome	/	19961124-	
	amphotericin b		-	
3	ARTERENOL	/	19961210-	
	norepinephrine hydrochloride		-	
4	BACTRIM	/	19961129-	
	sulfamethoxazole; trimethoprim		-	
5	diflucan	/	19961117-19961125	
	fluconazole		9 Day	
6	DOPAMIN	/	19961210-	
	dopamine hydrochloride		-	
7	DORMICUM	/	19961209-	
	midazolam maleate		-	
8	etoposide	/	19961122-19961122	
	etoposide		1 Day	
9	FENTANYL	/	19961209-	
	fentanyl		-	
10	FORTUM	/	19961122-19961130	
	ceftazidime pentahydrate		9 Day	
11	GAMMA GLOBULIN	/	19961127-19961210	
	immunoglobulin human normal		14 Day	
12	HYDROMEDIN	/	19961212-	
	etacrynic acid		-	
13	LASIX	/	19961124-	
	furosemide		-	
14	luminal	/	19961209-	
	phenobarbital		-	
15	methotrexate	/	19961127-19961129	
	methotrexate sodium		3 Day	
16	micronefrin	/	19961206-	
	epinephrine hydrochloride		-	
17	Morphin	/	19961130-	
	morphine hydrochloride		-	
18	MUCOSOLVAN	/	19961211-	
	ambroxol hydrochloride		-	
19	neupogen	Intravenous (not otherwise specified)	19961125-	Bone marrow transplant NOS
	filgrastim	120	-	
20	NOVALGIN	/	19961120-19961126	
	metamizole sodium		7 Day	
21	PANCURONIUM BROMIDE	/	19961213-19961213	
	pancuronium bromide		1 Day	
22	paracetamol	/	19961120-19961126	
	paracetamol		7 Day	
23	phenytoin	/	19961114-19961124	
	phenytoin		11 Day	
24	phenytoin	/	19961114-19961124	
	phenytoin		11 Day	
25	prostavaslin	/	19961201-	
	alprostadil		-	
26	sobelin	/	19961203-	
	clindamycin hydrochloride		-	
27	soluvit	/	19961126-	
	vitamins nos		-	
28	tobramycin	/	19961124-	
	tobramycin		-	

29	TRIGLOBE	/	19961116-19961127
	sulfadiazine; trimethoprim		12 Day
30	Urbason	/	19961124-
	methylprednisolone		-
31	VITINTRA	/	19961127-
	ergocalciferol; phytomenadione; retinol; retinol palmitate		-
32	ZANTIC	/	-19961211
	ranitidine hydrochloride		-
33	ZIENAM	/	19961201-
	cilastatin sodium; imipenem		-
34	zovirax	/	19961127-
	aciclovir		-

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Autosomal chromosome anomaly			Unknown	
Allogenic bone marrow transplantation therapy	19961126		Unknown	experienced graft versus host disease
Familial haemophagocytic lymphohistiocytosis			Unknown	
Upper respiratory infection			Unknown	

Primary source

Qualification: Physician
 Literature: observestudytype:

Narratives (nur bei elektronischem Eingang)

PFIZERINC, 05.12.2011 DE-PFIZER INC-049-0073-990003

Sender diagnosis: not reported

Narrative

Following the integration of the legacy safety adverse event reporting databases, this case has been migrated from the legacy company database into the current safety database for processing follow-up information. As a consequence of this migration, the follow-up CIOMS I or MedWatch report may indicate in the appropriate field that it is an initial report.

This is a spontaneous report from a contactable physician in Germany via Amgen pharmaceuticals, Mfr. Report #303385. A health care professional reported on 06Apr1999 that a 19-month-old male patient developed angioedema, ascites, and an airway obstruction, events considered medically significant hospitalization, and Stevens Johnson Syndrome, prolonging hospitalization eight days after receiving phenytoin. The patient was also receiving filgrastim (NEUPOGEN) intravenously, acetylcysteine, spironolactone (ALDACTONE), amphotericine B/ liposome (AMBISOME) 40 mg daily, norepinephrine hydrochloride (ARTERENOL), sulfamethoxazole trimethoprim (BACTRIM), fluconazole (DIFLUCAN), dopamine hydrochloride (DOPAMIN), midazolam maleate (DORMICUM), etoposide, fentanyl, ceftazidime pentahydrate (FORTUM), immunoglobulin human normal (GAMMA GLOBULIN), etacrynic acid (HYDROMEDIN), furosemide (LASIX), phenobarbital (LUMINAL), methotrexate, epinephrine hydrochloride (MICRONEFRIN), morphine hydrochloride, ambroxol hydrochloride (MUCOSOLVAN), Novalgine (metamizole sodium), pancuronium bromide, paracetamol, alprostadil (PROSTAVASIN), clindamycin hydrochloride (SOBELIN), vitamins nos (SOLUVIT), tobramycin, sulfadiazine/ trimethoprim (TRIGLOBE), methylprednisolone (URBASON), ergocalciferol/ retinol/ retinol palmitate/ phytomenadione (VITINTRA), ranitidine hydrochloride (ZANTIC), imipenem/ cilastatin sodium (ZIENAM), and aciclovir (ZOVIRAX). He started taking phenytoin (dosage unknown) on 14Nov1996. He started sulfadiazine (TRIGLOBE) on 16Nov1996. He began treatment with fluconazole on 17Nov1996. On 20Nov1996, he started taking metamizole and paracetamol. On 22Nov1996, started ceftazidime pentahydrate and took etoposide for only that day. On 24Nov1996, he started taking amphotericine B/ liposome, furosemide, tobramycin, and methylprednisolone, and phenytoin therapy was discontinued. Fluconazole treatment was discontinued on 25Nov1996 and filgrastim therapy was started for bone marrow transplantation. The patient underwent an allogenic bone marrow transplant on 26Nov1996. Vitamins were started and metamizole sodium and paracetamol therapies were discontinued on 26Nov1996. On 27Nov1996, he started methotrexate, aciclovir, immunoglobulin human normal, and Ergocalciferol/ retinol/ retinol palmitate/ phytomenadione. Sulfadiazine (TRIGLOBE) therapy was discontinued on 27Nov1996. On 29Nov1996, sulfamethoxazole trimethoprim therapy was started and methotrexate therapy was discontinued. He experienced graft versus host disease on an unspecified date. Acetylcysteine and morphine hydrochloride therapies were started on 30Nov1996, and ceftazidime pentahydrate therapy was discontinued. On 01Dec1996, he developed hemorrhagic mucositis and started treatment with alprostadil and imipenem. On 03Dec1996, he developed erosions and began treatment with clindamycin hydrochloride. On 04Dec1996, he experienced erosive oral mucosa and ascites and was started on spironolactone therapy. He developed inspiratory stridor on 06Dec1996, and was treated with epinephrine hydrochloride. The following day he experienced an acute airway obstruction. On 09Dec1996, he started taking midazolam, fentanyl, and phenobarbital. He started taking norepinephrine hydrochloride and dopamine hydrochloride on 10Dec1996, and discontinued immunoglobulin human normal that day. He developed angioedema on 11Dec1996 and was started on ambroxol hydrochloride. Ranitidine hydrochloride was discontinued on 11Dec1996. Etacrynic acid was started on 12Dec1996. On 13Dec1996, he experienced blistering and erosions of his skin and was diagnosed with Stevens Johnson syndrome. He was given pancuronium bromide only on 13Dec1996. By 05Jan1997, the Stevens Johnson syndrome had improved. It was unknown if filgrastim, vitamins, tobramycin, sulfamethoxazole trimethoprim, acetylcysteine, spironolactone,

methylprednisolone, alprostadil, clindamycin, imipenem, aciclovir, fentanyl, phenobarbital, amphotericine B/ liposome, norepinephrine hydrochloride, dopamine, midazolam, ambroxol, ergocalciferol/ retinol/ retinol palmitate/ phytomenadione, etacrynic acid, and furosemide therapies continued. It was unknown if the patient recovered from angioedema, ascites, or the airway obstruction. Per case 0015486501PHAMED, the patient was recovering from angioedema, ascites and Stevens Johnson syndrome. His medical history was remarkable for autosomal anomalies, upper respiratory infection, and familial history of hemophagocytic lymphohistiocytosis with cerebral involvement. He had no known allergies. The reporting physician determined the Stevens Johnson syndrome was possibly related to filgrastim.

Pfizer is the Marketing Authorization Holder of phenytoin and methotrexate sodium in the reporter's country. This may be a duplicate report in situations where another Marketing Authorization Holder of phenytoin and methotrexate sodium has submitted the same report to the regulatory authorities.

Follow-up (21Nov2011): This is a follow-up report combining information from duplicate reports case 990408-SK015, case 0015486501PHAMED and case 049-0073-990003. The current and all subsequent follow-up information will be reported under report number case 049-0073-990003. The new information reported from a physician includes.

Sender's comment:

The ascites and acute airway obstruction most likely represented intercurrent medical conditions in this critically ill patient with graft versus host disease after the bone marrow transplantation. The angioedema and Steven Johnson Syndrome represented severe allergic reaction to administered medications. The reported events occurred after discontinuation of phenytoin, fluconazole and methotrexate treatment, hence, these events were not likely related to phenytoin, fluconazole and methotrexate treatment. The ascites most likely was the indication of spironolactone, not related to the suspect drug. A possible contributory role of spironolactone can not be totally excluded for other events due to temporal relationship. Other co-administered medications could also contribute to the reported events.

The impact of this report on the benefit/risk profile of the Pfizer product is evaluated as part of Pfizer procedures for safety evaluation, including the review and analysis of aggregate data for adverse events. Any safety concern identified as part of this review, as well as any appropriate action in response, will be promptly notified to Regulatory Authorities, Ethics Committees and Investigators, as appropriate. In der Konzerndatenbank wurden die Faele unter den Nummern 990408-SK015, 0015486501PHAMED und 049-0073-990003 als Triplikate identifiziert. Die Meldung wird nun lediglich unter der Nummer 049-0073-990003 gefuehrt. Keine neuen Safety-Informationen, daher keine Aenderungen zu den zuvor uebermittelten Stellungnahmen.

Reporter's comment:

PFIZERINC, 13.12.2011 DE-PFIZER INC-049-0073-990003

Sender diagnosis: not reported

Narrative

Following the integration of the legacy safety adverse event reporting databases, this case has been migrated from the legacy company database into the current safety database for processing follow-up information. As a consequence of this migration, the follow-up CIOMS I or MedWatch report may indicate in the appropriate field that it is an initial report.

This is a spontaneous report from a contactable physician in Germany via Amgen pharmaceuticals, Mfr. Report #303385. A health care professional reported on 06Apr1999 that a 19-month-old male patient developed angioedema, ascites, and an airway obstruction, events considered medically significant hospitalization, and Stevens Johnson Syndrome, prolonging hospitalization eight days after receiving phenytoin. The patient was also receiving filgrastim (NEUPOGEN) intravenously, acetylcysteine, spironolactone (ALDACTONE), amphotericine B/ liposome (AMBISOME) 40 mg daily, norepinephrine hydrochloride (ARTERENOL), sulfamethoxazole trimethoprim (BACTRIM), fluconazole (DIFLUCAN), dopamine hydrochloride (DOPAMIN), midazolam maleate (DORMICUM), etoposide, fentanyl, ceftazidime pentahydrate (FORTUM), immunoglobulin human normal (GAMMA GLOBULIN), etacrynic acid (HYDROMEDIN), furosemide (LASIX), phenobarbital (LUMINAL), methotrexate, epinephrine hydrochloride (MICRONEFRIN), morphine hydrochloride, ambroxol hydrochloride (MUCOSOLVAN), metamizole sodium (NOVALGIN), pancuronium bromide, paracetamol, alprostadil (PROSTAVASIN), clindamycin hydrochloride (SOBELIN), vitamins nos (SOLUVIT), tobramycin, sulfadiazine/ trimethoprim (TRIGLOBE), methylprednisolone (URBASON), ergocalciferol/ retinol/ retinol palmitate/ phytomenadione (VITINTRA), ranitidine hydrochloride (ZANTIC), imipenem/ cilastatin sodium (ZIENAM), and aciclovir (ZOVIRAX). He started taking phenytoin (dosage unknown) on 14Nov1996. He started sulfadiazine (TRIGLOBE) on 16Nov1996. He began treatment with fluconazole on 17Nov1996. On 20Nov1996, he started taking metamizole and paracetamol. On 22Nov1996, started ceftazidime pentahydrate and took etoposide for only that day. On 24Nov1996, he started taking amphotericine B/ liposome, furosemide, tobramycin, and methylprednisolone, and phenytoin therapy was discontinued. Fluconazole treatment was discontinued on 25Nov1996 and filgrastim therapy was started for bone marrow transplantation. The patient underwent an allogenic bone marrow transplant on 26Nov1996. Vitamins were started and metamizole sodium and paracetamol therapies were discontinued on 26Nov1996. On 27Nov1996, he started

methotrexate, aciclovir, immunoglobulin human normal, and Ergocalciferol/ retinol/ retinol palmitate/ phytomenadione. Sulfadiazine (TRIGLOBE) therapy was discontinued on 27Nov1996. On 29Nov1996, sulfamethoxazole trimethoprim therapy was started and methotrexate therapy was discontinued. He experienced graft versus host disease on an unspecified date. Acetylcysteine and morphine hydrochloride therapies were started on 30Nov1996, and ceftazidime pentahydrate therapy was discontinued. On 01Dec1996, he developed hemorrhagic mucositis and started treatment with alprostadil and imipenem. On 03Dec1996, he developed erosions and began treatment with clindamycin hydrochloride. On 04Dec1996, he experienced erosive oral mucosa and ascites and was started on spironolactone therapy. He developed inspiratory stridor on 06Dec1996, and was treated with epinephrine hydrochloride. The following day he experienced an acute airway obstruction. On 09Dec1996, he started taking midazolam, fentanyl, and phenobarbital. He started taking norepinephrine hydrochloride and dopamine hydrochloride on 10Dec1996, and discontinued immunoglobulin human normal that day. He developed angioedema on 11Dec1996 and was started on ambroxol hydrochloride. Ranitidine hydrochloride was discontinued on 11Dec1996. Etacrynic acid was started on 12Dec1996. On 13Dec1996, he experienced blistering and erosions of his skin and was diagnosed with Stevens Johnsons syndrome. He was given pancuronium bromide only on 13Dec1996. By 05Jan1997, the Stevens Johnson syndrome had improved. It was unknown if filgrastim, vitamins, tobramycin, sulfamethoxazole trimethoprim, acetylcysteine, spironolactone, methylprednisolone, alprostadil, clindamycin, imipenem, aciclovir, fentanyl, phenobarbital, amphotericine B/ liposome, norepinephrine hydrochloride, dopamine, midazolam, ambroxol, ergocalciferol/ retinol/ retinol palmitate/ phytomenadione, etacrynic acid, and furosemide therapies continued. It was unknown if the patient recovered from angioedema, ascites, or the airway obstruction. Per case 0015486501PHAMED, the patient was recovering from angiodema, ascites and Stevens Johnson syndrome. His medical history was remarkable for autosomal anomalies, upper respiratory infection, and familial history of hemophagocytic lymphohistiocytosis with cerebral involvement. He had no known allergies. The reporting physician determined the Stevens Johnson syndrome was possibly related to filgrastim.

Pfizer is the Marketing Authorization Holder of phenytoin and methotrexate sodium in the reporter's country. This may be a duplicate report in situations where another Marketing Authorization Holder of phenytoin and methotrexate sodium has submitted the same report to the regulatory authorities.

Follow-up (21Nov2011): This is a follow-up report combining information from duplicate reports case 990408-SK015, case 0015486501PHAMED and case 049-0073-990003. The current and all subsequent follow-up information will be reported under report number case 049-0073-990003. The new information reported from a physician includes.

Follow-up (30Nov2011): This is a follow-up spontaneous report based on information received by Pfizer from JNJ. The above mentioned case (Local Reference 049-0073-990003) had been assigned SCEPTRE AER Number 049-0073-990003 and was designated as serious.

Sender's comment:

The ascites and acute airway obstruction most likely represented intercurrent medical conditions in this critically ill patient with graft versus host disease after the bone marrow transplantation. The angioedema and Steven Johnson Syndrome represented severe allergic reaction to administered medications. The reported events occurred after discontinuation of phentoin, fluconazole and methotrexate treatment, hence, these events were not likely related to phenytoin, fluconazole and methotrexate treatment. The ascites most likely was the indication of spironolactone, not related to the suspect drug. A possible contributory role of spironolactone can not be totally excluded for other events due to temporal relationship. Other co-administered medications could also contribute to the reported events. The follow-up information received does not alter the previous company clinical evaluation.

The impact of this report on the benefit/risk profile of the Pfizer product is evaluated as part of Pfizer procedures for safety evaluation, including the review and analysis of aggregate data for adverse events. Any safety concern identified as part of this review, as well as any appropriate action in response, will be promptly notified to Regulatory Authorities, Ethics Committees and Investigators, as appropriate. In der Konzerndatenbank wurden die Faelle unter den Nummern 990408-SK015, 0015486501PHAMED und 049-0073-990003 als Triplikate identifiziert. Die Meldung wird nun lediglich unter der Nummer 049-0073-990003 gefuehrt. Keine neuen Safety-Informationen, daher keine Aenderungen zu den zuvor uebermittelten Stellungnahmen.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-14248672

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		Yes

Patient

Initials:	Sex: Female	Birth: privacy	Age: 11 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Toxic epidermal necrolysis					recovered/resolved with sequel
2 Hypertrophic scar					Unknown
3 Withdrawal syndrome					Unknown
4 Scarring					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 AMOXICILLIN		/	-	Scarlet fever
amoxicillin			-	
2 cromoglicat sodium		/	-	Allergic rhinitis
cromoglicate sodium		146	7 Day	
3 PARACETAMOL		/	-	Scarlet fever
paracetamol			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Allergic rhinitis			Unknown	
Mental disorder			Unknown	
Behavioural disorder			Unknown	

Primary source

Qualification:	Other Health Professional	observestudytype:
Literature:	Kreft, B. Extensive hypertrophic scarring after toxic epidermal necrolysis in a child. Pediatric Dermatology. 2014;31 (4):527-528	

Narratives (nur bei elektronischem Eingang)

PFIZERINC, 18.08.2014 DE-PFIZER INC-2014223930

Sender diagnosis: not reported

Narrative

This is a literature report from the Pediatric Dermatology, 2014, volume 31 (4); page 527-528, entitled "Extensive hypertrophic scarring after toxic epidermal necrolysis in a child". The full publication has been requested.

Stevens-Johnson syndrome and toxic epidermal necrolysis are some of the most serious, usually drug-induced, skin reactions. We report a case of severe toxic epidermal necrolysis in a child, which in addition to ophthalmic sequelae, caused extensive hypertrophic scarring of the skin. Such a course is uncommon and has rarely been described in the literature. 2012 Wiley Periodicals, Inc.

Follow-up (08Aug2014): The initial case was missing the following minimum criteria: unspecified drug. Upon receipt of follow up information on 08Aug2014, this case now contains all required information to be considered valid. This case refers to Patient (11 year old female) taking Paracetamol and experienced Toxic epidermal necrolysis, hypertrophic scarring of the skin, distinctive analgesic withdrawal syndrome.

BRIEF REPORT: An 11-year-old girl without any known diseases except slight allergic rhinitis, for which she was taking cromoglycate-containing nasal spray for 7 days, developed a patchy exanthem with fever and conjunctivitis. Under clinical suspicion of scarlet fever, she was prescribed amoxicillin and paracetamol. Within 24 hours the initially livid, erythematous macular lesions limited to the trunk turned into a generalized bullous exanthem, with her general condition significantly deteriorating. In addition to conjunctivitis, painful erosions of the oral and genital mucous membranes appeared. Histologic examination of skin tissue showed extensive hyaline necrosis of keratinocytes in the basal epidermis and minor perivascular lymphocytic inflammatory infiltration in the upper part of the dermis. Based on the clinical picture and the histologic findings, the diagnosis of toxic epidermal necrolysis (TEN) was made. Amoxicillin, paracetamol, and the cromoglycate-containing nasal spray were discontinued. A definite drug trigger of TEN could not be identified. There was neither clinical nor paraclinical evidence of an infectious etiology (including testing for immunoglobulin [Ig] G and

IgM class antibodies to *Mycoplasma pneumoniae*) or autoimmune disease. Appropriate ophthalmologic and rigorous dermatologic topical treatment under sterile conditions with octenidine dihydrochloride/phenoxyethanol-moistened compresses, subsequent application of nonadhesive paraffin gauze dressings on the erosive skin areas, and supportive intensive care were initiated. Because of the extent of the skin lesions, intravenous prednisolone (2 mg/kg) was administered for two consecutive days and immunoglobulins (0.6 mg/kg) over 6 days. Because of further progression of the inflammatory lesions, a single intravenous (IV) dose of the tumor necrosis factor alpha antibody infliximab (250 mg) was given after 3 days of IV Ig therapy. Within 2 weeks her skin and oral mucous membranes reepithelialized to a large extent, although the child developed a posttraumatic mental behavioral disorder with a distinctive analgesic withdrawal syndrome, nocturnal enuresis, and trichotillomania. Strikingly and unexpectedly, in addition to ophthalmic complications with entropion and increasing corneal neovascularization, hypertrophic, partially keloidal scarring on the skin was observed within 4 months with the first clinical evidence of keloid formation approximately 2 weeks after discharge from inpatient care. The upper part of the body was predominantly involved. Such a course is uncommon and has rarely been described in the literature.

DISCUSSION: The possible causes of the hypertrophic scarring in the presented case are speculative and could possibly be due to an individual predisposition to hypertrophic scarring or repeated compulsive scratching because of pruritus and posttraumatic mental behavioral disorder. Because self-mutilation was suspected, the posttraumatic mental behavioral disorder could have played a role in scarring in this case. Based on this reported case, we would like to emphasize that TEN may occasionally result in permanent scars.

Pfizer is marketing Authorization Holder of paracetamol and amoxicillin in the reporter's country. This may be a duplicate report in situations where another marketing Authorization Holder of amoxicillin has submitted to regulatory authorities.

Sender's comment:

Based on the information available, and on a close temporal association a causal relationship between the reported event of toxic epidermal necrolysis (TEN) and suspected products paracetamol and amoxicillin cannot be excluded. Causality is also confounded by the use of cromoglycate-containing nasal spray for 7 days prior to the onset of TEN signs/symptoms. The impact of this report on the benefit/risk profile of the Pfizer product is evaluated as part of Pfizer procedures for safety evaluation, including the review and analysis of aggregate data for adverse events. Any safety concern identified as part of this review, as well as any appropriate action in response, will be promptly notified to Regulatory Authorities, Ethics Committees and Investigators, as appropriate.

Reporter's comment:

PANPHARMAP, 22.08.2014 DE-PANPHARMAP-201402652

Sender diagnosis: not reported

Narrative

The initial information was published in the scientific literature and identified on 11-Aug-2014.

Kreft B. Lieser U. Haase R. Marsch W.C. Wohlrab J. Extensive hypertrophic scarring after toxic epidermal necrolysis in a child. *Pediatr Dermatol.* 2014 Jul; 31 (4): 527-8.

Abstract: Stevens-Johnson syndrome and toxic epidermal necrolysis are some of the most serious, usually drug-induced, skin reactions. We report a case of severe toxic epidermal necrolysis in a child, which in addition to ophthalmic sequelae, caused extensive hypertrophic scarring of the skin. Such a course is uncommon and has rarely been described in the literature.

Case report:

An 11-year-old girl without any known diseases except slight allergic rhinitis, for which she was taking cromoglycate-containing nasal spray for 7 days, developed a patchy exanthema with fever and conjunctivitis. Under clinical suspicion of scarlet fever, she was prescribed amoxicillin and paracetamol. Within 24 hours the initially livid, erythematous macular lesions limited to the trunk turned into a generalized bullous exanthema, with her general condition significantly deteriorating.

In addition to conjunctivitis, painful erosions of the oral and genital mucous membranes appeared. Histologic examination of skin tissue showed extensive hyaline necrosis of keratinocytes in the basal epidermis and minor perivascular lymphocytic inflammatory infiltration in the upper part of the dermis. Based on the clinical picture and the histologic findings, the diagnosis of toxic epidermal necrolysis (TEN) was made.

Amoxicillin, paracetamol and the cromoglycate-containing nasal spray were discontinued. A definite drug trigger of TEN could not be identified. There was neither clinical nor paraclinical evidence of an infectious etiology (including testing for immunoglobulin IgG and IgM class antibodies to *Mycoplasma pneumoniae*) or autoimmune disease. Appropriate ophthalmologic and rigorous dermatologic topical treatment under sterile conditions with octenidine dihydrochloride/phenoxyethanol-moistened compresses, subsequent application of nonadhesive paraffin gauze dressings on the erosive skin areas, and supportive intensive care were initiated. Because of the extent of the skin lesions, intravenous prednisolone (2 mg/kg) was

administered for two consecutive days and immunoglobulins (0.6 mg/kg) over 6 days. Because of further progression of the inflammatory lesions, a single intravenous (IV) dose of the tumor necrosis factor alpha antibody infliximab (250mg) was given after 3 days of IV Ig therapy. Within 2 weeks, her skin and oral mucous membranes reepithelialized to a large extent, although the child developed a posttraumatic mental behavioral disorder with a distinctive analgesic withdrawal syndrome, nocturnal enuresis, and trichotillomania. Strikingly and unexpectedly, in addition to ophthalmic complications with entropion and increasing corneal neovascularization, hypertrophic, partially keloidal scarring on the skin was observed within 4 months, with the first clinical evidence of keloid formation approximately 2 weeks after discharge from inpatient care. The upper part of the body was predominantly involved.

Outcome: The patient had not yet completely recovered from entropion and corneal neovascularization and had recovered with sequelae from toxic epidermal necrolysis.

Sender's comment:

Serious case

According to the German SmPC used as Reference Safety Information, toxic epidermal necrolysis, entropion and corneal neovascularization are unlisted

Based on the information currently available, the company assessed the causal relationship of all suspect drugs as possible according to the WHO-UMC causality assessment (According to chronology, cromoglycate seems be the most suspect drug but in case of first exposure to amoxicillin and paracetamol, delay less twenty hours can not be excluded these other iatrogenous etiologies. Investigations excluded Mycoplasma infection and autoimmune disease but herpes status was not available. However, no allergic check-up has been performed to confirm or not the causal role of the administered drugs).

Reporter's comment:

HOSPIRAP, 01.09.2014 DE-HOSPIRA-2509936

Sender diagnosis: not reported

Narrative

Literature report from Germany describing a case of toxic epidermal necrolysis (TEN) and extensive hypertrophic scarring.

An 11-year-old female patient received paracetamol (dose, route of administration, lot number, frequency, and formulation not reported) for scarlet fever on an unknown date. Patient's medical history included slight allergic rhinitis. Concomitant medications were not reported.

On an unknown date, the patient without any known diseases except slight allergic rhinitis, for which she was taking cromoglycolate-containing spray (dose not reported, nasal spray) for 7 days, developed a patchy exanthem with fever and conjunctivitis. Under clinical suspicion of scarlet fever, she was prescribed amoxicillin (dose and route of administration not reported) and paracetamol. Within 24 hours, the initially livid, erythematous macular lesions limited to the trunk turned into a generalized bullous exanthem, with her general condition significantly deteriorating. In addition to conjunctivitis, painful erosions of the oral and genital mucous membranes appeared. Histologic examination of skin tissue showed extensive hyaline necrosis of keratinocytes in the basal epidermis and minor perivascular lymphocytic inflammatory infiltration in the upper part of the dermis. Based on the clinical picture and the histologic findings, the diagnosis of TEN was made. Amoxicillin, paracetamol, and cromoglycolate-containing containing nasal spray were discontinued. A definite drug trigger of TEN could not be identified. There was neither clinical nor paraclinical evidence of an infectious etiology [including testing for immunoglobulin (Ig) G and IgM class antibodies to Mycoplasma pneumoniae] or autoimmune disease. Appropriate ophthalmologic and rigorous dermatologic topical treatment under sterile conditions with octenidine dihydrofolate/phenoxyethanol-moistened compresses, subsequent application of nonadhesive paraffin gauze dressings on the erosive skin areas, and supportive intensive care were initiated. Because of the extent of the skin lesions, prednisolone (2 mg/kg, intravenous) was administered for two consecutive days and immunoglobulins (0.6 mg/kg, route of administration not reported) over 6 days. Because of further progression of the inflammatory lesions, a single dose of tumor necrosis factor alpha antibody infliximab (250 mg, intravenous) was given after 3 days of IV Ig therapy. Within 2 weeks, her skin and oral mucous membranes reepithelialized to a large extent, although the child developed a posttraumatic mental behavioral disorder with a distinctive analgesic withdrawal syndrome, nocturnal enuresis, and trichotillomania. Strikingly and unexpectedly, in addition to ophthalmic complications with entropion and increasing corneal neovascularization, hypertrophic, partially keloidal scarring on the skin was observed within 4 months with the first clinical evidence of keloid formation approximately 2 weeks after discharge from inpatient care. The upper part of the body was predominantly involved.

Outcome for the event extensive hypertrophic scarring was not reported.

The author attributed the adverse events to the drug as stated in the article title, "Extensive Hypertrophic Scarring After Toxic Epidermal Necrolysis in a Child". The author stated, "Under clinical suspicion of scarlet fever, she was prescribed amoxicillin and paracetamol... Amoxicillin, paracetamol, and the cromoglycate-containing nasal spray were discontinued. A definite drug trigger of TEN could not be identified."

Amoxicillin and cromoglycolate-containing spray were also considered suspects.

Kreft B, Lieser U, Haase R, Marsch WC, Wohlrab J. Extensive Hypertrophic Scarring After Toxic Epidermal Necrolysis in a Child. *Pediatr Dermatol*. 2014 Jul-Aug; 31(4): 527-528

Sender's comment:

Overall case causality: Possible

While temporally related, consider also other suspect drugs (especially amoxicillin).

- R. Jacot Eine Änderung zum wissenschaftlichen Erkenntnisstand der/s involvierten Präparate/s ergibt sich zum gegenwärtigen Zeitpunkt nicht

Reporter's comment:

HOSPIRAP, 04.12.2014 DE-HOSPIRA-2509936

Sender diagnosis: not reported

Narrative

Literature report from Germany describing a case of toxic epidermal necrolysis (TEN) and extensive hypertrophic scarring.

An 11-year-old female patient received paracetamol (dose, route of administration, lot number, frequency, and formulation not reported) for scarlet fever on an unknown date. Patient's medical history included slight allergic rhinitis. Concomitant medications were not reported.

On an unknown date, the patient without any known diseases except slight allergic rhinitis, for which she was taking cromoglycolate-containing spray (dose not reported, nasal spray) for 7 days, developed a patchy exanthem with fever and conjunctivitis. Under clinical suspicion of scarlet fever, she was prescribed amoxicillin (dose and route of administration not reported) and paracetamol. Within 24 hours, the initially livid, erythematous macular lesions limited to the trunk turned into a generalized bullous exanthem, with her general condition significantly deteriorating. In addition to conjunctivitis, painful erosions of the oral and genital mucous membranes appeared. Histologic examination of skin tissue showed extensive hyaline necrosis of keratinocytes in the basal epidermis and minor perivascular lymphocytic inflammatory infiltration in the upper part of the dermis. Based on the clinical picture and the histologic findings, the diagnosis of TEN was made. Amoxicillin, paracetamol, and cromoglycate-containing containing nasal spray were discontinued. A definite drug trigger of TEN could not be identified. There was neither clinical nor paraclinical evidence of an infectious etiology [including testing for immunoglobulin (Ig) G and IgM class antibodies to *Mycoplasma pneumoniae*] or autoimmune disease. Appropriate ophthalmologic and rigorous dermatologic topical treatment under sterile conditions with octenidine dihydrofolate/phenoxylethanol-moistened compresses, subsequent application of nonadhesive paraffin gauze dressings on the erosive skin areas, and supportive intensive care were initiated. Because of the extent of the skin lesions, prednisolone (2 mg/kg, intravenous) was administered for two consecutive days and immunoglobulins (0.6 mg/kg, route of administration not reported) over 6 days. Because of further progression of the inflammatory lesions, a single dose of tumor necrosis factor alpha antibody infliximab (250 mg, intravenous) was given after 3 days of IV Ig therapy. Within 2 weeks, her skin and oral mucous membranes

reepithelialized to a large extent, although the child developed a posttraumatic mental behavioral disorder with a distinctive analgesic withdrawal syndrome, nocturnal enuresis, and trichotillomania. Strikingly and unexpectedly, in addition to ophthalmic complications with entropion and increasing corneal neovascularization, hypertrophic, partially keloidal scarring on the skin was observed within 4 months with the first clinical evidence of keloid formation approximately 2 weeks after discharge from inpatient care. The upper part of the body was predominantly involved.

Outcome for the event extensive hypertrophic scarring was not reported.

The author attributed the adverse events to the drug as stated in the article title, "Extensive Hypertrophic Scarring After Toxic Epidermal Necrolysis in a Child". The author stated, "Under clinical suspicion of scarlet fever, she was prescribed amoxicillin and paracetamol... Amoxicillin, paracetamol, and the cromoglycate-containing nasal spray were discontinued. A definite drug trigger of TEN could not be identified."

Amoxicillin and cromoglycolate-containing spray were also considered suspects.

Kreft B, Lieser U, Haase R, Marsch WC, Wohlrab J. Extensive Hypertrophic Scarring After Toxic Epidermal Necrolysis in a Child. *Pediatr Dermatol*. 2014 Jul-Aug; 31(4): 527-528

Sender's comment:

Overall case causality: Possible

While temporally related, consider also other suspect drugs (especially amoxicillin).

- R. Jacot Eine Änderung zum wissenschaftlichen Erkenntnisstand der/s involvierten Präparate/s ergibt sich zum gegenwärtigen Zeitpunkt nicht

Reporter's comment:

MENARINI, 27.01.2015 DE-MENARINI-DE-MEN-038665

Sender diagnosis: not reported

Narrative

This case was identified within international literature and reported by Pediatric Dermatology.

It has been reported as involving a case of a 11-year-old girl without any known diseases except slight allergic rhinitis, for which she was taking cromoglycate-containing nasal spray for 7 days. She developed a patchy exanthem with fever and conjunctivitis. Under clinical suspicion of scarlet fever, she was prescribed amoxicillin and paracetamol. Within 24 hours the initially livid, erythematous macular lesions limited to the trunk turned into a generalized bullous exanthem, with her general condition significantly deteriorating. In addition to conjunctivitis, painful erosions of the oral and genital mucous membranes appeared. Histologic examination of skin tissue showed extensive hyaline necrosis of keratinocytes in the basal epidermis and minor perivascular lymphocytic inflammatory infiltration in the upper part of the dermis. Based on the clinical picture and the histologic findings, the diagnosis of toxic epidermal necrolysis (TEN) was made.

Amoxicillin, paracetamol, and the cromoglycate-containing nasal spray were discontinued. A definite drug trigger of TEN could not be identified. There was neither clinical nor paraclinical evidence of an infectious etiology (including testing for immunoglobulin [Ig] G and IgM class antibodies to *Mycoplasma pneumoniae*) or autoimmune disease. Appropriate ophthalmologic and rigorous dermatologic topical treatment under sterile conditions with octenidine dihydrochloride/phenoxylethanol-moistened compresses, subsequent application of nonadhesive paraffin gauze dressings on the erosive skin areas, and supportive intensive care were initiated. Because of the extent of the skin lesions, intravenous prednisolone (2 mg/kg) was administered for two consecutive days and immunoglobulins (0.6 mg/kg) over 6 days. Because of further progression of the inflammatory lesions, a single intravenous (IV) dose of the tumor necrosis factor alpha antibody infliximab (250 mg) was given after 3 days of IV Ig therapy. Within 2 weeks her skin and oral mucous membranes reepithelialized to a large extent, although the child developed a posttraumatic mental behavioral disorder with a distinctive analgesic withdrawal syndrome, nocturnal enuresis, and trichotillomania. Strikingly and unexpectedly, in addition to ophthalmic complications with entropion and increasing corneal neovascularization, hypertrophic, partially keloidal scarring on the skin was observed within 4 months, with the first clinical evidence of keloid formation approximately 2 weeks after discharge from inpatient care. The upper part of the body was

predominantly involved. Such a course is uncommon and has rarely been described in the literature.

Sender's comment:

Based on the received information the reported event can be considered as possibly related to both suspect drugs. The case was considered as serious due to the importance of the medical event.

Reporter's comment:

MENARINI, 30.01.2015 DE-MENARINI-DE-MEN-038665

Sender diagnosis: not reported

Narrative

This case was identified within international literature and reported by Pediatric Dermatology.

It has been reported as involving a case of a 11-year-old girl without any known diseases except slight allergic rhinitis, for which she was taking cromoglycate-containing nasal spray for 7 days. She developed a patchy exanthem with fever and conjunctivitis. Under clinical suspicion of scarlet fever, she was prescribed amoxicillin and paracetamol. Within 24 hours the initially livid, erythematous macular lesions limited to the trunk turned into a generalized bullous exanthem, with her general condition significantly deteriorating. In addition to conjunctivitis, painful erosions of the oral and genital mucous membranes appeared. Histologic examination of skin tissue showed extensive hyaline necrosis of keratinocytes in the basal epidermis and minor perivascular lymphocytic inflammatory infiltration in the upper part of the dermis. Based on the clinical picture and the histologic findings, the diagnosis of toxic epidermal necrolysis (TEN) was made.

Amoxicillin, paracetamol, and the cromoglycate-containing nasal spray were discontinued. A definite drug trigger of TEN could not be identified. There was neither clinical nor paraclinical evidence of an infectious etiology (including testing for immunoglobulin [Ig] G and IgM class antibodies to Mycoplasma pneumoniae) or autoimmune disease. Appropriate ophthalmologic and rigorous dermatologic topical treatment under sterile conditions with octenidine dihydrochloride/phenoxyethanol-moistened compresses, subsequent application of nonadhesive paraffin gauze dressings on the erosive skin areas, and supportive intensive care were initiated. Because of the extent of the skin lesions, intravenous prednisolone (2 mg/kg) was administered for two consecutive days and immunoglobulins (0.6 mg/kg) over 6 days. Because of further progression of the inflammatory lesions, a single intravenous (IV) dose of the tumor necrosis factor alpha antibody infliximab (250 mg) was given after 3 days of IV Ig therapy. Within 2 weeks her skin and oral mucous membranes reepithelialized to a large extent, although the child developed a posttraumatic mental behavioral disorder with a distinctive analgesic withdrawal syndrome, nocturnal enuresis, and trichotillomania. Strikingly and unexpectedly, in addition to ophthalmic complications with entropion and increasing corneal neovascularization, hypertrophic, partially keloidal scarring on the skin was observed within 4 months, with the first clinical evidence of keloid formation approximately 2 weeks after discharge from inpatient care. The upper part of the body was predominantly involved. Such a course is uncommon and has rarely been described in the literature.

Sender's comment:

Based on the received information the reported event can be considered as possibly related to both suspect drugs. The case was considered as serious due to the importance of the medical event.

Reporter's comment:

ACTAVISPR, 11.06.2015 DE-ACTAVIS-2015-11565

Sender diagnosis: not reported

Narrative

Date of initial report: 03-JUN-2015

This report originated in the scientific literature.

A 11 years old female patient (Patient Initials: UNKNOWN) experienced toxic epidermal necrolysis whilst being treated with cromoglycate sodium nasal spray, unknown dosage, for allergic rhinitis and with amoxicillin and paracetamol, unknown dosages, frequencies and routes of administration, for supposed scarlet fever. Concomitant medication was not used. Besides allergic rhinitis, the patient didn't have any other medical conditions. The

suspect drugs were withdrawn and the patient was admitted to the hospital and treated with intravenous prednisolone, immunoglobulin and infliximab and she also received ophthalmological treatment . The patient recovered with sequels , hypertrophic , partially keloidal scarring on the skin, and ophthalmic complications were still present.

The girl , with no previous known disease except slight allergic rhinitis was taking cromoglycate-containing nasal spray for 7 days and she developed a patchy exanthema with fever and conjunctivitis. Because of the clinical suspicion of scarlet fever, amoxicillin and paracetamol were prescribed. Within 24 hours , the erythematous macular lesion limited to the trunk turned into generalized bullous exanthema , with her general condition significantly deteriorating. In addition to conjunctivitis , painful erosions of the oral and genital mucous membranes appeared. Histological examination of skin tissue revealed extensive hyaline necrosis of keratinocytes in the basal epidermis and perivascular lymphocytic infiltration in the upper part of dermis. A diagnosis of toxic epidermal necrolysis was made and the treatment was discontinued. Appropriate ophthalmologic and rigorous dermatological topical treatment under sterile conditions with octenidine dihydrochloride/phenoxylethanol-moistened compress, subsequent application of nonadhesive paraffin gauze dressings on the erosive skin areas and supportive intensive care were initiated. Intravenous prednisolone(2 mg/kg) was administered for two consecutive days and immunoglobulins (0.6 mg/kg) over 6 days. Because of further progression of the inflammatory lesions , a single intravenous dose of infliximab (250 mg) was given after 3 days of iv Ig therapy. Within 2 weeks her skin and oral mucous membranes reepithelialized to a large extent , although the child developed a posttraumatic mental behavioral disorder with distinctive analgesic withdrawal syndrome , nocturnal enuresis and trichotillomania. In addition to ophthalmological complications with entropion and increasing corneal neovascularization , hypertrophic , partially keloidal scarring on the skin was observed within 4 months.

Sender's comment:

Reporter's comment:

number of reports: 310